

Narayana Hrudayalaya Ltd. (NH)

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Call: Aug 2022

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To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code – 539551 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051
Scrip Code- NH

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter ended 30th June, 2022

This is further to our earlier letter dated 12th August, 2022 regarding audio/ video recording of Earnings Call of the Company for the quarter ended 30th June 2022, held on 11th August, 2022, please find enclosed herewith the transcript of the said Earnings Call.

The same is also available on the website of the Company at
<https://www.narayanahealth.org/stakeholder-relations/earning-call-transcripts>.

This is for your information and record.

Thanking you.

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q1 FY23 Earnings Conference Call"

August 11 , 2022

MANAGEMENT : MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – CHIEF FINANCIAL OFFICER

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS
SUBSIDIARY HCCI

MR. DEBANGSHU SARKAR – HEAD, MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. DURGA PRASAD – SENIOR MANAGER , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

Debangshu Sarkar: Hello everyone. Good afternoon to all of you. Myself Debangshu and as most of you are aware, I run the Investor Relations and Mergers and Acquisition Practices at Narayana Hrudayalaya. On behalf of the company, I welcome you all to the first quarter fiscal year 2023 earnings call of our company. To discuss our performance and address all your queries today, we have with us Mr. Viren Shetty -- our Vice Chairman, Dr. Emmanuel Rupert -- our CEO and MD, Ms. Sandhya -- our CFO, Dr Anesh Shetty -- MD of our Overseas Subsidiary HCCI, and Durga Prasad from the team.

I'm sure you have gone through the investor collaterals , which have been uploaded on the stock exchanges as well as on our website.

Before we proceed with this call, I would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website at a subsequent date. I would also like to remind you that everything that is being said on this call that reflects any outlook for the future or which can be construed as a forward -looking statement must be viewed in conjunction with the uncertainties and the risk that they face. These uncertainties and risks are included, but not limited to what we have already mentioned in our prospectus filed with SEBI before our Initial Public Offer in late 2015 and subsequent annual reports on our website.

On that account, I'm sure you guys probably would have also gone through our annual report as that has also been very recently updated for the last fiscal.

Post the call, in case you have any further queries please do feel free to get in touch with us. With that, I would now like to hand over the call to Dr. Rupert.

Dr. Emmanuel Rupert: Good afternoon to all. We are pleased to report the highest profitability for our Indian operations during the quarter gone by which surpassed the previous highs witnessed in Q3 FY22. Aided by steady operations at Cayman, we managed to achieve a consolidated EBITDA of INR 2.0 billion at a margin of 19.4% and PAT of INR 1.1 billion at a margin of 10.7% for the period Q1 FY23. Since our planned capital outlay for India and Cayman is still running behind schedule due to external factors , we incurred a cash capex of over INR 1.1 billion during the period. Our overall balance sheet and liquidity profile remains strong with INR 5.2 billion of gross borrowings against a consolidated cash and liquid investments of over INR 4.4 billion as on 30th June, 2022. Separately, we have reorganized our operating regions to ensure greater alignment among our hospitals. From this financial year onwards, Jaipur hospital will be part of the Northern group and Raipur hospital would form part of the Eastern Peripheral grouping.

With negligible contribution of Covid -19 and vaccine volumes combined with recovery in elective volumes, we registered the highest ever EBITDA margin of 16.2% for Q1 FY23 for our Indian operations. Compared to previous high in Q3 FY22, the domestic operations ' EBITDA grew 27.5% while registering an absolute increase of around INR 300 million and it registered 46.8% growth compared to Q4 FY22, which was impacted by the omicron wave. The Indian operations also registered the highest percentage contributions from the international patient business since the onset of the pandemic at 6.5%. While we are working on new marketing channels to grow this further, we don't believe we will reach our pre pandemic figures of around 10% -11% till international travel reverts to pre pandemic behavior.

It is heartening to note that the strong India performance has been achieved through all round performance across the network while continuing to be led by our flagship centres which registered an EBITDA margin of 30.0% in Q1 FY23 as against 28.9% in Q3 FY22. Our three new hospitals across NCR and Mumbai registered a revenue of over INR 1.05 billion.

on and delivered a positive

EBITDA R margin of 6.5%. The Non -Flagship Hospitals excluding Jammu achieved a healthy EBITDA R margin of 18.8% in Q1 FY23. We remain encouraged with the resilience of our business during these uncertain times and we shall continue to invest in brownfield expansion across these units over a period of time.

Moving on to Cayman, our hospital was affected by Covid restrictions and strict quarantine rules placed by the authorities for most of the quarter and managed to register an EBITDA of USD 8.0 million in Q1 FY23. As our expansion slowly continues, we plan to onboard key clinical talent ahead of the anticipated patient volumes since there is a long lead time for getting visas and work permits. With Covid disruptions minimizing from Q2 and with our Camana Bay Clinic making us more accessible, we are confident of sustaining solid performance of this unit going forward.

We continue to focus on various technology initiatives to drive performance.

During the quarter gone by, we implemented a cost estimation module in ATHMA HIS to provide accurate estimates for planned admissions and we also implemented a checklist tool for clinical staff to improve patient safety. Our software team also developed a new user interface for AADI Mobile App to improve user adoption and drive productivity of our doctors across the network. Our efforts in revamping the marketing team have paid off and digital marketing channels now contribute almost 1/4th of the overall India business.

On the ESG front, for the period Q1 FY23, we achieved a net carbon reduction equivalent of 3,705 tons along with savings worth 9.1 million through energy optimization and switching to green energy and another 3.6 million by upgrading to high efficiency equipment across the network. To improve our focus, we have also initiated the business responsibility and sustainability report ESG framework across the group.

On the clinical front, we have continued to differentiate our services by focusing on advanced quaternary work in cardiac, Oncology, and GI sciences across the network. Some of the key highlights for the quarter gone by are

- We added three additional operating rooms to the Narayana Institute of Cardiac Sciences at Bangalore in the Health City campus, thereby increasing the surgical throughput in this unit. This unit did 2,027 cardiac surgeries during the quarter and performed 1,687 Cath lab procedures in May 2022, which is the all-time highest monthly volume at this center.
- Our flagship RTIICS at Kolkata successfully bridged a patient with end stage heart failure to transplant with CentriMag biventricular assist device (BiVAD). The unit also successfully implanted a suture-less aortic valve, which is the first such procedure in the eastern India.
- Our focus on transplant programs generated strong momentum which resulted in 75 successful Bone Marrow Transplants, 20 Liver Transplants, 8 Heart Transplants, and 192 Renal Transplants across the group in Q1 FY23.
- We also managed to perform 30 TAVI procedures as well as 125 robotic surgeries across the group in the same period.

While continuing to consolidate our operations, we would pursue growth opportunities both in India and overseas to derive synergies from our robust existing operations and maximize value for all our stakeholders. We are confident about the prospects of the healthcare landscape across the world and are taking steps to transform our business to become more patient oriented digitally native and operationally efficient. We see our overall business being well poised to continue Dr. Shetty's mission of delivering affordable and high-quality healthcare to all sections of our society. Thank you.

Debangshu Sarkar: Thanks, Dr. Rupert. With that, we will now open the floor for Q&As.

Prithviraj: Mr. Viren I just have a couple of questions.

First on your domestic business

new hospitals, you mention they have done 6.5% EBITDA R margin this quarter, so how do we see this margin trajectory over the next couple of years and when can we expect them to touch that 18% or 20%, which currently your non-flagship hospitals are delivering?

Viren Shetty: These hospitals, two in Delhi, one in Mumbai are of different sizes and configurations. The Delhi ones we feel pretty confident that ultimately, they will reach the normal trajectory that the rest of the hospitals have given that these are multispecialty quaternary and tertiary care hospitals. So, as for the

timeline, it's hard for us to give guidance. I would say normally for our hospital they take anywhere from 3-4 years to break even, post that another four 3-4 to reach a sort of maturity and then depending on how we expand them, it will take its time. So, both the hospitals will be due for expansion, not immediately, but some point over the next two years. In Gurugram, we want to add two more floors, we have the capacity to do so and Dharamshila also we're looking to talk to the hospital owner, the trustees to expand another wing over there. So, what happens with expansion is that again you incur a lot of manpower to get the things done, but it does reach a very steady state EBITDA level, when you don't do anymore expansion, you're just going up the volume growth. So, long answer to your question, I would say to get to that 18%-20%, it would take the normal trajectory that all the other hospitals, which is usually from 5 to 8 years barring certain exceptions that come about. The exception will be our Bombay Children Hospital given that it is highly specialized around children's care and this has a very different earnings profile, which is quite high in cost and as it's trying to differentiate itself, it's taking much longer to break even, so there it's not easy for us to say whether it will match the performance of the other hospitals.

Prithviraj: And just one more question from my side, again on Cayman business, you mentioned that you know business is now coming back to normal, so can we expect again the margin and the volume trend to move to historical levels even here?

Viren Shetty: For this question, I'll pass it on to Anesh, who is based over there and will be better able to flesh this out. Anesh.

Anesh Shetty: Yeah. Thanks, Viren. Hi Prithviraj. So, to your question, yes during the first quarter of the financial year we were severely impacted by COVID. Lot of patients were not sick, but the government rules are very strict around quarantining and testing and even a single family member being tested positive and the whole family needs to isolate. So, lot of our doctors and nurses were unfortunately out of action. Fortunately, that is behind us and even in the first month that's July of the new quarter, we have seen both revenue and margins revert to normal.

Nitin: Anesh, if I can take it forward from the Cayman Island question. Cayman Island essentially has been a big success story for a turnaround story for NH over the last 2½-3 years especially post COVID, since you are closer to the ground can you just help us understand couple of things? One is, what change in Cayman Islands all of a sudden that our business performance really took off? And two there's always concerns that some of this may not sustain, so what would your thoughts be on that account?

Anesh Shetty: Thanks, Nitin. So, there are two questions there. So, to the first one about what suddenly changed?—we don't think it's sudden, of course we've been operating the hospital for about eight years. For the first, I would say 2½ years or so we had a lot of problems finding our fit in the market. We were actually chasing a very aspirational target in terms of

an expectation of medical tourists

coming from overseas, which we realize is a game that will play out over years if not decades and we are getting there, but I think the initial insight we had was that we had to be built on the foundation of a very strong local and Caribbean presence and markets in the US Canada and North America would play out over a longer time frame. So, I would say from year 3 onwards that's when we really made that switch and focus in our efforts and things have like you said have been working out well for us so far. We don't see any sudden swing or we don't see any need for any sudden deterioration that would happen because of any external event in most situations. To your second question about how sustainable is it? We did have COVID restrictions like everybody else in the early half of 2020 that did mandate for a lot of people to remain on island and people here we're happy to be on the island because this was something like a paradise where there was negligible COVID and people couldn't come in, most people couldn't go out. So, a lot of patients especially the privately insured and mobile patients who traditionally would go to the US centers of excellence for their care had to try us out. These people would come to us for certain procedures, but in most cases would go overseas. So, for a good 1½ to 2-year period we had in a sense a group of privately insured local people who had to experience our services and travel restrictions have been rapidly going away in fact right now if you're fully vaccinated, there are almost no restrictions including testing requirements as long as you're fully

vaccinated, which most of the population is. It's hard to say whether those people who used to go out will continue to stay with us, but so far, the trends look very encouraging and very positive. It appears that they've liked what they've seen, they enjoyed the experience and they are sticking with us, but we'll know in the next two or three quarters or so if that is a permanent shift to us or there may be some rebalancing, but so far we're very encouraged by what we're seeing.

Nitin: If I were to just probably take that little forward again, now we have Oncology setup coming through in the Camana Bay units. I mean two things, one is – How does it change the proposition for NH's business in Cayman? And two does it do anything more for you from overall Cayman Island drop beyond Cayman Islands in the broader Caribbean? I mean does it do anything to enhance our proposition with broader Caribbean per say?

Anesh Shetty: Yeah Nitin, absolutely no. You are right on the mark over there. See when we've been working with other islands, other governments, other players to get them to move their patients towards us for the value proposition is very clear, we are a Joint Commission accredited hospital, we offer tertiary care at unbeatable price. The challenge that a lot of the client face or a lot of players face is that they have existing relationships, so it's difficult for them to move some business to us and maintain their relationships elsewhere. One of the biggest gaps we always had was radiotherapy and Oncology in general. This is a very big spend area, if not in volume, but big in spend for everyone. So, what we're seeing now is we're on track to have our radiotherapy center commissioned by Q4 of the current fiscal year that was what we had committed to do and things look to be on track, with that not only do we fill in perhaps the last reason for local people to go overseas for Oncology and radiotherapy, but we become that much more attractive as a full service, a truly full service option for other governments and peers in other jurisdictions where they can now say that alright I don't have to maintain relationships elsewhere, this is a one stop solution for me. So, that really i

s a very positive

sign for us and that's why we've been pursuing this radiotherapy project for quite some time now.

Nitin: Thanks, Anesh. Again, associated point is for us, from our overseas expansion perspective Cayman obviously has turned out to be a significant success, so does now Cayman become an anchor for us to build a business around or just remain essentially one hospital story or one market story from an ex-India perspective?

Anesh Shetty: No, definitely not. As long as I can remember at least 5-6 years onwards we've been wooed by every head of state, every pair in most Caribbean islands, I mean I traveled to all of them, all the English-speaking Caribbean islands, there's not a single market where people don't want us to replicate what we've done in Cayman, naturally, that's not possible. There are some very favorable dynamics in the market that allow us to do what he did. Having said that, Cayman is already on track and will very much be becoming a hub for what we're doing in the region. We for the past year and a half, we've been engaged with the government of Saint Lucia to help them operationalize their National Hospital. The intention of that project always was to learn more about the market and take that to the next level if we wanted to. We have similar early stage discussions going on in several other islands, but these are mostly arrangements where we would be a consultant, we would be an operator, we would not be coming to the table with capital initially, but we always have that option to do that and we still have a lot of unused capacity in our Cayman Hospital. It is still very much in our favor to use this as a hub and establish relationships etc. to better utilize our infrastructure, especially with the new hospital and the Oncology project coming in, but we are very well-known brand in the region and we have proven that this model of having a tertiary joint commission accredited hospital in a small island can work. So, there is opportunity to do something along the same lines elsewhere, but not with so much of an investment and not with such a big presence.

Harith Ahamed: Thanks for the opportunity. Will you be able to share the EBITDA R number for the new hospital cohort? I think I missed that in your opening remark?

Debangshu Sarkar: So, Dr. Rupert had mentioned in his opening remarks that the cohort has delivered a positive EBITDA R margin of 6.5% for the period under review.

Harith Ahamed: Okay, got it. And if I take a step back, I think, maybe the last couple of years or pre-COVID we had embarked on efforts to improve the case mix and our

ARPOBs in India hospitals network and then we've seen some of that reflecting in much improved ARP OBs when I look at the numbers today. So, where are we in that journey? And, if you could give some color on whether we should expect the trajectory in terms of ARPOB improvement to continue?

Dr. Emmanuel Rupert: If you see the clinical work that is happening, apart from routine work, most of the work is happening on the high-end Tertiary and Quaternary work which is actually driving the ARPOBs and the margins as well. You would have seen the transplants numbers which we have done across the group and also the kind of the Quaternary work that is routinely going and we are constantly working with clinical teams to improve throughput and the efficiencies and gradually trying to reduce the ALOS. Currently it is around 4.6 but we are in the journey where we will start reducing it closer to 4.2 to 4.3 and then once we stabilized, we will be further coming closer to 4. And despite of the Quaternary work we are able to maintain a very steady ALOS numbers but it's a combination of different things, We are working a lot on the oncology front – Daycare work, chemo and radiations. Some of our machine

s in Bangalore

have just got upgraded. We were down to only one machine. Similar this story in Howrah. We will get that up by October and also we will be commissioning by closer to the end of this financial year the unit in Jaipur as well. So, all this will you know contribute to reductions of ALOS as well while contributing significantly to the high-end Tertiary and the Quaternary work which will continue to happen in this sector. That is what we are working on and then a lot of other efficiencies and a lot of digital initiatives are also bringing in the efficiencies and the throughput which we have started to work on and which will start bearing fruit in the coming quarters.

Harith Ahamed: Thanks Dr. Rupert and last one for my side on the M & A front. I think, last time you had mentioned that we're evaluating a couple of assets, so where are we in that process? And in terms of geographic priorities which are the key markets that we should expect on M&A front?

Viren Shetty: Priorities are our key markets which are Bangalore and Calcutta. There we are evaluating opportunities but those take a long time to come and it's not always that economics favor us. So, we'll be taking up Brownfield expansion for our Health city in Bangalore and in Calcutta we are not able to get land nearby. We bought a few small parcels that's adjacent to our property but that's a very crowded space. So, we have to buy land a little bit further away around 20-25 minutes driving distance and build a new campus over there. It won't operate traditionally like a Greenfield given that there's so much patient volume that would be shifted and certain clinical departments we will move there. So, it will function more or less like a Brownfield. So, that is our highest priority for expansion. Also adding floors to the existing building, adding Onco units to all our hospitals, changing the bed configuration, and eventually Brownfield expansion for Delhi, Bombay, Mysore, Raipur. M&A wise, there are few things we're looking at that are two hospitals under liquidation. A few things that come up on asset-light basis with certain trusts, some people that we are talking to for management contracts at the hospital but those are still very slow burn, long late projects that will come as and when materialize but our growth numbers are not dependent on that. Those will be coming over and above everything we have planned.

Ahmed Madha: Thank you. So my question was related to the last participant's question, is it fair to assume that there is nothing M&A coming up in this financial year?

Viren Shetty: No, I mean there are a lot of things we're pursuing. It's just that our ability to close them and coming to an agreement on the right price means that it's hard for us to tell you exactly that, you know, exactly one is coming up next quarter, one after that, two beyond that. So, that's the kind of guidance we're not able to give. We are evaluating 3-4 opportunities. We don't know exactly when they'll come up but when they do they're all part of the plan, what we had said earlier, which is to strengthen our existing hospitals, have huge focus on Bangalore and Calcutta and look at the current geographies as areas for expansion and asset-light things in other places.

Ahmed Madha: Got it. And one more question related to ALOS where there's commentary that it will reduce from 4.6 to 4. Now we understand that it is a day in and day out process and you need to improve therapies gradually, but can you give broadly a guideline or how long does it take to improve this, a year, 2 years or longer than that?

Dr. Emmanuel Rupert: Closer to eight quarters to come to that 4.1 figure. But we are constantly

working on that. It's a gradual process which we will see on a quarter -on-qua

ter basis; a step -by-step reduction.

Sameer Baisiwala: Thank you everyone and a very good afternoon. So, just on the previous question, you had a Cap ex target of Rs. 10 billion for the current year and have done Rs. 1 billion. So are we on track ? And is it possible to break it up, especially for the Kolkata and the Bangalore expansions?

Sandhya J: We have guided that we are looking at Rs. 700 crores - Rs.1000 crores over the 3 to 5 years. So, there are a couple of aspects on Cap ex. One is, that it's a mix of Greenfield and Brownfield some of which could be inorganic. So, on the inorganic, Viren, has already given a commentary that depending on the timing it will materialize. As far as the Greenfield opportunities are concerned, like we had indicated, the big expansions will happen in Calcutta and in Bangalore. In addition, we are looking at, for example Oncology expansion in Jaipur, Ahmedabad and Mysore. So, there would be specialties and capabilities that we will be building across different units based on the demand and traction that we are seeing in those specialties. So, those are on track. The Greenfields are subject to land acquisition etc .., The one that Viren explained, those will be timed accordingly. And the inorganic, based on the timing, the Cap ex will be done. So, I am not able to guide a particular number but this is the trajectory in which we are on.

Sameer Baisiwala: Thanks for this. So, did you say 700-1000 crore over the next five -year period? Is that what you said?

Sandhya J: Yeah, 3 to 5 years.

Sameer Baisiwala: Okay. And can you quantify how much would be Bangalore and Kolkata? If you have something in mind?

Sandhya J: Other than the Cayman one, which we've already spoken about which is a big investment, a significant chunk of this is to be investing in Bangalore and Kolkata and in other high performing regions based on return that we will be able to do generate. I'm not able to quantify that number because, like I said, some of these are inorganic opportunities and they will have to materialize. So, therefore it is not fair to put a number to it at this time.

Viren Shetty: We can say the bulk of that amount will be for these two geographies.

Debangshu Sarkar: Sandhya, just to, I mean, probably get this right what Sameer was wanting to understand because the 7 -10 billion is actually 700-1000 crores . That's surely not over five-year period , that's what Sameer's question was. Since we had guided a figure of north of 1000 crores for the current fiscal including all the activities that we're pursuing across organic, inorganic, greenfield and everything (inclusive of Cayman) . So, that remains on track albeit that we are running a little behind the schedule, in terms of what you have seen us incur in the first quarter. But if all the things were to go through in terms of what we have planned, for the current fiscal we should be looking at the kind of numbers that we have previously guided upon.

Sameer Baisiwala: Yeah exactly, Debangshu. That's what my understanding was given your guidance during last call.

Viren Shetty: No, sorry, we got confused because we forgot to add Cayman into that.

Sameer Baisiwala: Okay, got it. So, it is Rs. 1000 crores for the current fiscal .

Debangshu Sarkar: There will be upward bias to the Rs. 1000 crores. So, don't hold us on that number, if at all

Sameer Baisiwala: Okay. Got it. And the other question was on the Mumbai hospital. Can you talk a bit more about, how is the asset utilization in whichever way you want to quantify and talk about? And is it really taking longer than what you had initially planned? And what really needs to be done to make it a very optimal usage?

Viren Shetty : We've seen a lot of growth in Cardiac Sciences Program and the Oncology Program in addition to the very susta

ined and a very steady run of our

Pediatrics Surgical Program as well Pediatric Orthopedics and the other specialties which has been going on. So, we are focusing a lot on the high -risk pregnancies because that is a need which not many wanting to focus on our Level 3 NICUs and the high -risk pregnancies all in under one roof. That is something which we are running and, we're trying to focus on that and move that in that direction in addition to focusing significantly on the very high -end NICU and the Cardiac Sciences Program and various other things. So, for all these expansions with the good traction in the Bone Marrow as well as the

Liver Transplant programs going, we do need additional capacity for which we have mentioned about that in the opening remarks and that is something which we are working on. But this is something which we are utilizing the capacity over a period of time, if we do not expand over a period of time then an upper limit for running the services because Pediatric is something which we can't be, you know, unlike other Adult Program where a lot of efficiency and ALOS can be reduced significantly, we may not be in a position to do that when we are doing very high-end tertiary and quaternary care. So, we do need a little bit more of the capacity there. The capacity will be funded through fundraising from our trustees for that hospital.

Sameer Baisiwala: Sir, what's the utilization over there at the moment?

Dr. Emmanuel Rupert: The bed utilization is close to 70% of the beds commissioned and not capacity (we do have headroom to operationalize few more beds basis the uptick in occupancy). That is as per the midnight occupancy but if you look at the Daycare occupancies will be a little higher.

Sameer Baisiwala: With such high occupancy, I would say, it should be very profitable hospital or operations that you're running over there which is not. So, is there something that's holding you back?

Viren Shetty: Rich people don't have sick kids. This hospital was built with the idea that it could be a center of excellence for paediatric cases modeled on the lines of Children's Hospital of Philadelphia, Boston Children's and so on. But in a country like India, given that most of the patients who come with the worst symptoms tend to be from poorer backgrounds, we rely on a lot of government funding, charitable funding to support the hospital operations. So, in time, once it becomes known as this Number one Center of Excellence and people from all over the country come, the patient demographics of the hospital will change a lot, which will improve the finances. But until that time comes, it's going to take some time.

Sameer Baisiwala: Yeah, thanks a lot, Viren. That's very helpful. And let me ask one more question and this is for the Cayman Islands. So, not worrying about the quarter to quarter fluctuation if I say that you are doing roughly 25 million on the top line and 10 million dollars of say about on EBITDA line, how should now this trajectory be going forward excluding the new Oncology block? Until that comes up, how much potential does it have?

Anesh Shetty: Thanks, Sameer. So, the numbers you referenced were, like you said they've already been achieved in say Q3 of the previous year and they're very much our current expectations, so excluding the Oncology block which will be starting soon, we have a lot of room to grow both in Cayman Islands as well as in the immediate vicinity that is for us the Caribbean and certain Latin American countries. There are a few service lines which we have yet to commission. You know, we don't do the bulk of primary and secondary care. We restrict ourselves to tertiary care in most instances. There are certain very high value niche areas such as Neonatology and Paediatric Intensive Care which we are planning on commissioning soon, hopefully in the next year or so, but those are currently

not offered. These are services for which patients go overseas and locally they are frequently dubbed, the million-dollar fee because Neonatal care in the US is extremely expensive. So, there are a fair number of specialties which we have yet to commence and we have a clear path. We have the infrastructure, it is just putting the teams in place, getting the right approvals and getting started. Having said that, our entry into the retail high street location with our clinic has given us a good strong foothold into the privately insured market. In fact, in the past few quarters since that clinic has been active, we've seen a good improvement in our Market Share in the private insurance space as well. So, we don't have an exact figure to share at this time, we're comfortably seeing a good pathway to easily increasing revenues from the existing facility we have and with adding new service lines to the existing infrastructure. And this is not to mention that a lot of our recent growth over the last two years was achieved with, I would say, the negative performance of our international business because of COVID restrictions. So as the restrictions abate, those are all channels that we will slowly restart. We had to defer a lot of those relationships because of the COVID restrictions. We will be restarting a lot of those and that is a much larger pool. Of course, more challenges to that, but that's a much larger pool that we have to tap into.

Sameer Baisiwala: Okay, this is useful. And for the fully vaccinated, are there any travel restrictions now in Cayman inbound or it's still pretty much locked up?

Anesh Shetty: No, I mentioned this as well earlier that if you're fully vaccinated you can freely enter and exit but there's just an online portal and you have to fill up some information. It's something like our Air Suvidha. So, that's the only requirement and that was as of a few weeks ago. So, if you're fully vaccinated you're good to go.

Damayanti Kerai: I'm looking at your presentation slide where you have given operational review for India. So, your ARPOB for the quarter was INR 12.2 million which is around 9% higher than the previous year. So, how should we look at growth on the ARPOB side and what will be key driver? And my other question will be, have you taken price hike for your healthcare services for the fiscal and by how much?

Viren Shetty: As Rupert was earlier saying, the ARPOB is driven by the mix change or the type of procedures that we've been performing and because of the faster turnaround operations and while overall the ALOS has remained the same but we have been able to turn around a lot of the segments of patients faster. So, as far as the price hike is concerned, as you are aware, price hike is not our first lever. We normally look at costs. However, to counterbalance the heavy inflationary pressures, we do take price hikes depending from unit to unit and that happens in January. So, our last price hike happened in January of this year and the next price hike will be scheduled in next January. It is very low single digits if the broad average of price hike which we have taken this year. Next year as we go through the cycle and we roll up our costs and we see how this all stacks up and the efficiencies that are coming in through the significant investments we've made in technology, we will then see what we have to do from a price point of view.

Damayanti Kerai: Also, key driver, as you said, is mainly the mix improvement from the current level and scale up of some new units and not much reliance on the pricing part?

Viren Shetty: Mix change, volumes and throughput drives the ARPOB.

Gagan Thareja: Thanks for taking my question. You did give a detailed response to what's the headroom for growth at Cayman. Likewise, if you could give some idea on what's the headroom for growth in India? And,

if you could sort of segregate

your northern and western markets and eastern and southern markets and give your comments specifically for both the markets? What I'm trying to access is, till your capex plans come into effective, how much potential is there for the existing capacities to take you further ahead?

Viren Shetty: So, a lot of the capex what we are doing, Onco, for example is done to add a new service line. So, that is revenue that is additive which is not always already happening. Another kind of capex what we're doing is the room upgrades, the infrastructure refresh, lot of the equipment upgradation that is for increasing the yield per room, increasing the throughput, new service lines like adding new diagnostic therapies and faster scans. But, even without that, on a like to like basis, on the same infrastructure, simple things we're doing on the process side, on the service excellence side or on the digital side are done. So, I'll give an example. A big problem all our hospitals have is, whenever we deal with insurance companies or with government schemes, it takes a very long time to get a patient discharged, whereas everyone promptly shows up for admission at 9 am in the morning. So, there's a big mismatch where most of the admissions happen in the morning, but the discharge happens in the evening. So, that's leading to squeeze in beds. Now, lot of the process change what we were doing and on the coordination between doctors, labs and test results and so on, so that we can discharge patients faster, earlier in the morning. So, with the same number of beds, reducing the discharge time increase the admissions of more patients and increase the occupancy.

Otherwise, when my crunch is happening in the day time and occupancy I'm measuring in the night, you may see it as 65-68%, but functionally it acts more like a 75-80%, which is full occupancy. That is one aspect of that. The other is, the clinical team that we bring in. Now, a lot of our clinicians are travelling across. We've done a large number of these procedures called TAVI, which is a very advanced cardiac procedure for valve defects. A lot of them are travelling for doing liver transplants and training our people also. Then, there are robotic surgeries where patients are coming from different cities to get it done. So that is happening on the same infrastructure. The levers for that are just again, landing up patients, lot of communication, some marketing effort and having doctors talk to one another and manage the patients well. The third one is, as

Dr. Rupert mentioned, which is, the clinical teams working to reduce the ALOS, which increases the throughput, which in the same infrastructure, will allow us to see more patients. So, on a like to like basis, these levers will always be there. That is something which is pursued independent of everything else we will do. But, over and above that, in Bangalore, in Kolkata, in Raipur, in a lot of places, even in our Delhi hospitals, we are really still choked for space. So, our bone marrow transplant is usually always full because a lot of patients require that, and so they need more rooms for that. Or similarly, MRIs and CT scans are always choked in the morning time, or in the OT you cannot get a slot in the daytime, our ICUs are always full. So, for that, you would have to also do a little capacity addition to address these minor things as well. Then of course, over and above that, we are looking at brownfield expansion, adding lots of beds to Bangalore and Kolkata because there's just a natural flow. At this stage of our company's life, lot of patients come because their family members got treated here. So, it's not that this is the first time they've ever heard of NH, so they've come. Some family members, maybe even their parents have come here, gotten treated, had a good experience and so these are repeat patients for us. And so, that

thing just keeps growing the more mature you become as a hospital. So, we have to have enough capacity to take care of them.

Gagan Thareja: While I appreciate your very detailed answer, the issue is, all of these steps, as you say, to start with, in a way help you increase throughput and efficiency.

I'm simply grappling with what effective sort of a number or even a ballpark number, in terms of additive capacity it can create or increase throughput by. If you could help us somewhat there? Otherwise, it's very qualitative and very difficult for us to assess a model. And secondly, as you increase throughput, I'm sure it also implies some sort of an efficiency, and therefore improvement in margins. So, would it be a right assessment that, as and when this moves up, there's also margin benefits to be had? If you could give us some idea on both additive capacity and throughput and also maybe enumerate the outcomes of these efforts, and second, the consequences of these on the margins?

Viren Shetty: Gagan, I'm sure you know we don't give guidance on that, and it's not something that we would be able to give with any kind of certainty either in terms of guidance on how much all these digitization activities are going to do. I think, the best just to help you, what I've seen other people do in the model is, take where we are and take what is best in class. So, for hospitals in the certain region, best in class occupancy is, let's say 70-75% or best in class ALOS is that, whereas we are at 50% and the ALOS is where it is. And so, in the gap between the two, you would just need to make a reasonable assumption that this will happen organically over the next 2-3 years for you to reach that and then assign some kind of confidence interval on that. But, I don't think we'd be able to give you any guidance on just by putting a number.

Gagan Thareja: Yeah, thanks, I get what you're saying. Just one final one. The brownfield, if you could just help us understand both, the timeline over which it materializes and to what extent does it add to the existing bed capacity?

Viren Shetty: These are still in the planning stage. We're still talking with the contractors and architects and designers for doing all of that. These will be little, long laid time projects. This construction alone takes 2 to 2½ years, and getting the planning permission and so on would take another 6 months. The Capacity that it would be added in phases. So, ultimately, we want to double the capacity of both hospitals, but that will happen in phases. So, we may start with 200 beds and then keep adding that every couple of years as and when the occupancy reaches a certain level.

Gagan Thareja: So, it would be reasonable to assume that the significant capex budgeted for this year, except Cayman, a large part of the balance will actually bear fruit only 2½ to 3 years out? Or, am I completely wrong there?

Viren Shetty: No. The significant capex that is budgeted for this year, that will actually come online. It won't lead to any bed increase. This is mostly medical equipment and refurbishments; that's what we're able to spend. For bed capacity addition, those are long laid projects which will take 2-3 years. So, it won't show up in changing the overall beds. These things what we're spending now, apart from Cayman, are most quick wins like refurbishment of OT, adding MRIs, adding some robots, and adding Onco, which doesn't really change the bed mix much, but increases your realization; these are very high margin departments.

Gagan Thareja: Thanks a lot for taking my questions. I'll get back in the queue.

Alankar Garude: Thanks for the opportunity. 2 -3 questions. So, one is a follow up to Sameer's question on the hospital in Mumbai. If you see, there's another paediatric hospital in Mumbai which is operating at more than 20% EBITDA margins. We also have the largest mother and child hospital

chain in India which is

operating at 25 -28% EBITDA margins. So, it doesn't seem to be a city problem or maybe an acceptance issue for paediatric hospitals in general in India. So, can you help elaborate on what are the issues which you are facing? Is your mix a larger issue for us compared to some of our other peers, or any other issue which you can highlight?

Viren Shetty: I'll start and Dr. Rupert can elaborate. The key issue what you have identified, and we are mentioning the hospitals which are going in for listing soon. Those are mother and child hospitals, they don't do really high -end paediatric work. Most of it is birthing and lot of birthing related. Since our birthing program has just started, we're yet to get those sorts of numbers. Whereas, we started from day 1 with a focus on very high -end work on newborns, neonates, young children with serious congenital disease. So, that is leading to the difference. The patient profile of our two businesses are very different.

Dr. Emmanuel Rupert: I think Viren has covered it all. But, that is one more area where we are focusing on the routine general paediatric work; we're putting lot of focus on that as well where there is reasonably larger margins compared to some of the higher quaternary work where there's a different your mix which we are catering to as of now.

Alankar Garude: Understood. But, I'm just curious on that. If we look at say the largest chain in India, I mean, 70% of their business comes in from paediatrics, 30% only comes in from maternity. And, when I meant about the hospital in Mumbai, I didn't mention the one which is filed; there's another hospital which I was referring to, which I guess has a higher pediatric mix. So, can you help me understand if you just compare it to the largest chain in India, because the mix is broadly similar, 70% coming in from paediatrics. Is it just because of the mix issue, or is there something else?

Dr. Emmanuel Rupert: The clinical mix, because they have just started doing some super -specialty work, apart from doing the NICU work which they are very well known for. They do lot of retrievals and things like that. But, they have just started to go towards the tertiary and the quaternary work, which we are already well established as a provider of those services in Mumbai. But, we need to have the right mix, and that is what we're working on; the right mix of proper birthing and the right mix of the routine paediatric work, and that is what we are focusing on and trying to get that thing done. the daycare surgeries and things like that.

Alankar Garude: Okay. So, say in a steady state kind of timeline, would these high -end procedures which we are currently doing would be profitable procedures for us?

Dr. Emmanuel Rupert: Yeah, these high -end procedures work well. But, it just occupies too much of your manpower as well, because the more tertiary and the more quaternary work you do, lot of support services also proportionately rises, while the basic secondary level care and the primary kind of care, you don't need that kind of a manpower support. And, that is where we are, and we need to get the right balance in there and that is what we're working towards.

Alankar Garude: Okay. The second question is on Delhi and the eastern peripheral. I guess, there's been some change in how we have reported our clusters in this quarter. So, can you provide some numbers on the margins of Delhi as well as the eastern cluster, excluding Jaipur and Raipur?

Sandhya J: If you look at the Delhi cluster, we have reported EBITDA margin of about 10.4 % including Jaipur. If you look at the Delhi cluster, we would be around the range of 10-11% only in terms of EBITDA margin, because one of the hospitals is already at 15-16% margin and the other hospital is on track. So therefore, because Jaipur has got added to the Delhi cluster, we have not had a

positive bias to our margin numbers. In fact, Delhi performance has been, the northern cluster has recorded one of the highest ever revenue, as well as highest ever margins this quarter. So, the performance has been positive even excluding Jaipur. As far as eastern cluster is concerned, again our commentary is similar. Raipur comes with a slightly higher margin profile, but there isn't a significant positive bias because of Raipur was in the eastern peripheral as

well.

Alankar Garude: Understood. So, basically from almost 1% odd EBITDA margin for the Delhi cluster in FY 22, we are now, as of this quarter around 19%, even excluding Jaipur.

Sandhya J: Not Delhi cluster, one of the hospitals in Delhi is at around 15% EBITDA R and the other hospital is recovering. As a northern cluster, we will be around 10 %.

And, there is positive bias that is coming because of Jaipur getting added to .

Alankar Garude: Fair enough. And one final question. Can you comment on capex plans beyond FY23, I mean, very broadly, given that we have not firmed up our exact plans?

And also, is it possible to split the broad capex plan beyond FY23 between India, Cayman and US?

Sandhya J: So, Cayman the capex is pretty much done. We will finish the existing capex in Cayman and we will have adequate capacity that will run us for the next short to medium term. As far as India is concerned, like we said, there are various initiatives which are work in progress and they have to materialize. Like we guided, maybe we would invest 700 -1,000 crores over the next 3 -5 years in India. But, it is not possible to give a more detailed split of this profile right now because there are various moving parts, and some of it will convert. So, it's not fair to put a number on that right now. We are intending to invest among all the options that we spoke about so far in the call, and as we go through further calls, we can give you more commentary when things fructify and we have greater clarity.

Alankar Garude: Basically, even though the exact number are not finalized as yet, but very broadly, the capex intensity is going to be higher over the next few years compared to what we have seen in the past. That would be a fair assumption , right?

Viren Shetty: That is. But, that's also because we didn't invest anything during the Covid time. So this is catch -up investment that had to happen. The growth capex is driven primarily by Bangalore and Calcutta expansions , but they would have happened regardless.

Gagan Thareja: Just one follow up on the capex, how would you be funding it? What proportion do you intend to fund from internal accruals and how much pre cost to debt would you require here?

Sandhya J: It depends on certain types of capex. For eg. If we are acquiring land, we will be funding them from internal accruals. But say, if we're buying machinery or putting up a building, etc, that would have more skewness towards there.

While we don't have an exact number of how much will be internal accruals v/s debt, but we do not intend to breach the debt equity of 1 at the outer limit; so we would stay in that range. As the propositions come up and we start building our plan, I think we'll be in a position to give better visibility on that number.

Gagan Thareja: Okay. So, would it be reasonable to summarize that towards the end of FY23 our debt to equity would have changed significantly, or we would have levered up significantly from where we are?

Sandhya J: I don't think we would have levered up significantly. We are, like you are aware, not very aggressive in terms of debt equity. We will lever up in the short term, yes. But, I don't know if I will call it significantly, we will try to keep our debt equity under 1, that's the aspiration.

Debangshu Sarkar: So, I think there were some issues with my microphone out here. I was trying to say that, with that we will wrap up our session today. Thanks

once again

everyone for your active participation just like the previous calls. Like we mentioned at the outset, do feel free to reach out to us in case of any further queries or clarifications that you might need to address. Thanks once again for participating and being there with us on this forum.

Call: Nov 2022

Date of submission: 14th November 2022

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code – 539551 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051
Scrip Code- NH

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter ended 30th September 2022

This is further to our earlier letter dated 11th November 2022 regarding audio recording of Earnings Call of the Company for the quarter and half year ended 30th September 2022, held on 11th November 2022.

Please find enclosed herewith the transcript of the said Earnings Call. The same is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earning-call-transcripts>.

This is for your information and record.

Thanking you.

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q2 FY23 Earnings Conference Call"

November 11, 2022

MANAGEMENT : MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – CHIEF OPERATING OFFICER , EAST AND SOUTH REGIONS

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS SUBSIDIARY HCCI

MR. DEBANGSHU SARKAR – HEAD, MERGERS & ACQUISITIONS & INVESTOR RELATIONS

MR. DURGA PRASAD – SENIOR MANAGER , MERGERS & ACQUISITIONS & INVESTOR RELATIONS

Debangshu Sarkar: Good afternoon , everyone. On behalf of Narayana Hrudayalaya, I welcome you all to the Q2 and H1 FY 23 earnings call of the company. To discuss our performance and address all your queries today, we also have with us Mr. Viren Shetty, our Vice Chairman, Dr. Emmanuel Rupert, our CEO and MD, Ms. Sandhya, our CFO, Mr. Venkatesh - COO of our Eastern and Southern domestic operations, Dr. Anesh Shetty, MD of our overseas subsidiary HCCI and Durga Prasad from the team.

I am sure you have gone through the investor collaterals which have been uploaded on the stock exchanges as well as on our website. As usual, before we proceed with this call, I would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website as well as on the stock exchange at a later date. I would also like to remind you that everything that is being said on this call that reflects any outlook for the future or which can be construed as a forward looking statement, must be viewed in conjunction with the uncertainties and the risks that they face. These uncertainties and risks are included but not limited to what we have already mentioned in our prospectus filed with SEBI before our initial public offer in late 2015 and subsequent annual reports on our website. Post the call, should you have any further query, please do not hesitate to get in touch with us. We would like to address it to the best of our ability.

With that now, I would like to hand over the call to Dr. Rupert.

Dr. Emmanuel Rupert: Good afternoon to everyone. I cordially welcome you all to the Q2 FY23 earnings call conference of Narayana Hrudayalaya Limited.

The second quarter of fiscal year exhibited robust performance and maintained the momentum set by the first quarter due to an improvement in patient footfalls , case mix and payer mix. Consolidated revenue for the current quarter stood at INR 11,416 mn a quarter -on-quarter growth of 21.4% aided by steady performance of India business and strong performance of Cayman business.

NHL generated consolidated EBITDA of INR 2,749 mn in Q2 FY23 at a margin of 24.1% which when adjusted for one-time other income stood at 22.3% as against a margin of 19.4% in Q1 FY23. This margin improvement was attributed to improving payor mix and procedure mix.

Our Cayman business demonstrated continued growth due to lifting of restrictions and HCCI revenue increased by 28% to US D 29.1 mn.

Our overall balance sheet and liquidity profile remains strong with INR 7.2 billion of gross borrowings against a consolidated cash and liquid investments of over INR 4.6 billion as of 30th September 2022. Our debt -to-equity ratio remains comfortably low at 0.14 , giving us room to fund our expansions through borrowing and internal accruals.

Including a recent acquisition at Bangalore, we have incurred capital outlay of close to INR 5.5 billion for the 6 months ending 30th September 2022. We will continue to invest as per our strategic priorities , and we believe that our capex should be north of INR 10 billion in FY24 as well.

What gives us immense pride is bringing access to advanced quaternary care to patients from all sectors of society. In line with our vision to continue to invest in state of the art medical equipment, our unit in Howrah commissioned the latest model of linear accelerator, which is the Varian True Beam in Q2 FY23. With this strong momentum in high -end cardiac sciences work, in both congenital and adult segments and also in oncology work in robotic procedures and also in the fields of transplantation, which is both solid organs as well as bone marrow transplants . we are proud to share that

roud to share that

Narayana Institute of Cardiac Sciences in Health City, Bangalore has consistently performed more than 750 cardiac surgeries since March of 2022 and did the highest ever procedures in the cath lab of more than 1850 procedures. The unit was also successfully conducted more than 28 percutaneous valve implantations of the Aortic valve in this Quarter. Mazumdar Shaw Hospital in Health City Bangalore also successfully performed more than 74 robotic procedures for very complex oncology procedures. It

is very heartening for me to share with you that we have been able to perform 2000 bone marrow transplants to date across our network hospitals.

We are increasing our investments in digital engagement channels to improve operational efficiencies and patient experience. The digital traffic grew by 49% to 2.75 million users in the current quarter as against Q1 FY23. As India's leading cardiac and oncology provider we have been able to meaningfully contribute to cutting-edge medical research. We have already published more than 192 scientific papers in this current year and for the last two decades, we have published over 1640 scientific papers.

One of the interesting research activities that is currently undergoing along with the University of Arizona is the multimodal intraoral imaging system which is used for only detection of oral cancer with a smartphone and a specialized app that comes along with that.

Another area of cutting-edge research is in the field of bone marrow transplants, specifically the rejection of transplanted cells. The body can sometimes reject the transplanted cells as they come from a different person. This is known as Graft-versus-host disease (GVHD). At NH we are currently studying the potential of Itolizumab in treating patients who suffer from Graft-versus-host-disease. If successful, this will help thousands of patients across the globe.

We have previously spoken about our collaboration with Immuneel which is a Biocon start-up. It is a GMP certified processing centre which is in-house in the Health City campus for making what is called the CAR T therapy. CAR T stands for Chimeric Antigen Receptor. These are nothing but receptive proteins that are present on the cells, on the T-cells and they are re-engineered in the GMP certified process. The blood is removed through an apheresis machine and separates out the T-cells from the patients. And it is re-engineered by ingesting a gene for Chimeric Antigen Receptors and then we grow millions of these CAR T cells in the artificial lab, and this is reinfused into the patient, and you have a specific targeted destruction of the cancer cells. This is a major new dimension of therapy for refractory cancers across the world, which costs up to half a million to 1 million cost in the Western world. We are participating in the trial with Immuneel and following the regulatory approvals, which will take some time. We should be in a position to be able to provide this form of therapy to many patients in the Health City campus.

Our Health for all is our guiding beacon, and it has always been an integral part of everything that we do. We are proud to have contributed to discounts, and subsidies of greater than INR 300 million towards providing subsidized or free treatment for indigent patients in H1 FY23. Our CSR team continues to focus on providing scholarships for underprivileged children to pursue medical education.

While the results do bear about our focus on execution, as articulated, we are looking to also pursue strategic growth opportunities as reflected in recent acquisitions across our flagship region as well as Cayman Islands. We remain confident about the demand for health care and our ability to deliver high quality solutions to all sections of society. Thank you.

Debangshu Sarkar: I would request everyone to now use the 'raise hand' feature to start posing their questions, and we would try to address them in this forum.

Dheeresh Pathak: What was the EBITDA in the Cayman Hospital asset for the quarter?

Debangshu Sarkar: USD 12.7 million.

Dheeresh Pathak: Okay. This is post-Ind AS, right? And what is the adjustment needed between post to pre?

Debangshu Sarkar: Yes, it is USD 0.5 million, pre-Ind AS EBITDA is USD 12.2 million.

Dheeresh Pathak: For the year FY23, INR 1000 crore capex, I think about 100 million is in Cayman. What would be the breakup of 1000 crore capex which projects it is going into in FY23?

Debangshu Sarkar: We have already incurred around INR 300 odd crores towards our complete acquisition, including the primary investment or subscription of the OCD that we have done over there. Alongside that, while we had initially expected capex outlay of more than 1600 million dollars for Cayman, but given the run rate and some supply chain issues, we now foresee that number to be hovering a little shy of \$40 million. So around \$30 million of capex is possibly pending in over the H2 period in Cayman itself. Other than these, all the capex that you see would be routine maintenance, upgradation and brownfield expansions that we are incurring at multiple locations of ours.

Dheeresh Pathak: Okay. So just to summarize, 300 crores in the acquisition that you did and \$30 to \$40 million for the full year in Cayman and balance on maintenance and de-bottlenecking, right?

Debangshu Sarkar: Yeah. And additionally, that \$5 million of upfront consideration that we need to pay for that acquisition that we have announced at Cayman also, and that's the transaction

pending to be closed yet.

Dheeresh Pathak: Okay. And in FY 24 you will do the balance, \$60 million of Cayman and you also said that INR 1000 crore capex we will have in FY24 as well, so what would be the balance in FY 24? What I want to understand is in Indian as sets, is there meaningful capex ? Because you talked about in the last few calls in terms of the legacy flagship locations of Bangalore and Kolkata , wanting to expand more there. So I just want to know if you can give some more details in terms of how much capex you are doing in those two locations.

Debangshu Sarkar: Viren, you would want to elaborate on that? Other than the normal maintenance and the Cayman thing, which we have already elaborated or Brownfield capex anything else?

Viren Shetty: Yeah, it's more of the same as we described. In Bangalore, we'll be adding additional infrastructure to the Health City and expansion to the heart hospital. One in partnership with the landlord, one we will be doing Greenfield. Kolkata , we are looking to acquire some land and add new infrastructure close to our existing hospital. These are all work -in-progress. We budgeted about INR 1000 crores, which includes work -in-progress as well as some expansion works. But in the time it takes for us to find nice contractors and start doing this work. So conservatively, we can say about INR 1000 crore for FY24 also, and these projects are 2 to 3 year duration builds. The exact details, we will start detailing about from the next quarter onwards. But for now, it's looking like there will be significant capex investment in Bangalore and Kolkata as well.

Dheeresh Pathak: Okay, last question from my side. On the three assets, which earlier we used to disclose separately the revenue and the EBITDA, so SRCC, Dharmasala and Gurugram. If you can just highlight either individually or collectively, what has been the EBITDA and revenue in those assets?

Sandhya J: The three hospitals have generated highest ever revenue of INR 1078 million in Q2 FY23 compared to 944 million in the previous year. So it's a 14% Y-o-Y growth. We have registered a positive EBITDA margin of about 8.3%. Dharmasala

has broke even .

Gurugram has almost break even . Mumbai is on the verge of break even. So all of them are going in a very positive direction.

Dheeresh Pathak: Okay. So when you say break even, this would be after the rental and after the various other cost overheads. This would be like at the EBITDA level for the asset because EBITDA R, you are showing positive of 8.3%?

Sandhya J: Exactly you are correct about it. Broke even for the EBITDA level as well as for the cash flow level.

Dhara Patwa: So I had a question regarding the CAR T therapy, which you were talking about with Immuneel Pharma. So I understand currently there are only few drugs like Kymriah, Yescarta from BMS, which they are in the market. So what exactly were you talking about the Immuneel Pharma because that's a generic company, right?

Viren Shetty: No, it's a new GMP production facility within the Health City campus. It is still in a trial phase. We need to give the results of the detailed trials to the regulatory authorities before any kind of commercialization of this entire thing will be taking place. And currently there is no other GMP certified CAR T therapy production centre in India, which is made in India. Even if something is being done, they are procuring it, which is processed by some labs outside of the country.

Dhara Patwa: When can we expect our Ahmedabad and Mumbai cluster to deliver positive EBITDA margin? Like the one we have seen in Gurugram, one year back, it was around negative EBITDA and now currently it is contributing around 12%. So can we see some improvements for this cluster, Ahmedabad and Mumbai to be particular?

Sandhya J: Sure. Ahmedabad is already cash and EBITDA positive. Mumbai has almost turned around because of declining losses over a period of time, In H1 it was a negative EBITDA of about INR 3.3 crore. Mumbai hospital may break even in another 6 to 9 months

Ashish: Congratulations for a good set of numbers. I just wanted to try and understand that we are doing a INR 2000 crore capex over this year and next year. And what would it imply in terms of the balance sheet and return ratios? So currently we have INR 300 odd crores of debt. What's your estimate of the peak debt for the company let's say, over the next couple of years?

Sandhya J: So, as you're aware, we are cautious in terms of borrowing. Our desired level of debt equity is less than 1 or around 1; that is the desired level. Debt EBITDA maybe for a short period may be in the range of 2.5 to 3 and then come back. Having said that, we are hyper focused on cash flows. You would have seen that there is a tremendous amount of cash outflow that has happened in the current quarter , but that kind of borrowing has not happened because we've been very tight on our cash management. So we would like to see as much as possible that we can fund our expansion from our internal accruals and that we have to borrow only as much as required. So we will aspire to keep our

borrowings at that level.

Ashish: Sandhya sorry, I didn't get the numbers right. Because our EBITDA run rate is roughly a INR 1000 crore run rate and our equity is around 1800 -1900 crores. So what you are saying is that, you are talking about a 2 x debt to EBITDA or a 2.5 x times means the peak debt of 2.5 thousand crores as compared to less than 300 crores today net debt, right?

Sandhya J: Yes.

Ashish: So we are saying that we are going to borrow 1800 crores more in an environment of rising interest rates. Is that what we are looking at? Because when I look at it from a 6 quarter perspective based on the cash flows, we should be generating at least a 1000 crore of cash flows and our capex is around 79 crores, which means they don't seem to tie from my perspective.

Sandhya J: Currently our gross borrowing is about 750 crores

Ashish: I am looking at the net debt number.

Sandhya J: I understand. Net debt is the matter

of the cash flows that we are retaining and the cash flows that we are investing. So the way we've looked at it, I'm giving a gross number because that's a better representative, right now our borrowing is at 750 crores, our equity is currently 1 800 crores. So we do want to keep our debt equity at less than 1 , So, even though we would be incurring, say more than 1000 crores of capex , we don't want to keep debt equity at that level. That's a desire of where we want to be. Some of it would come from this cash flow. That's why you are seeing the benefit in the net debt because there are accruals that we would use and some of it will come from better management of cash on the ongoing basis.

As far as the debt to EBITDA is concerned, what I gave you was an outer -outer limit because this is a cap that we want to work with over the next 2 -3 years and depending on how our cash flows and EBITDA turns out. So that is an outer limit. I think these numbers give you enough guidance on where we are headed.

Ashish: Yeah. The second question on the cash flow again, because my understanding is the reason why we had a large cash balance was because money had got trapped in Cayman and they would have been a tax incidence. Now that we are doing a large capex in Cayman, hopefully our cash balances which are running at 400 crores would kind of come down. Where I'm coming from is, if I put in a 2000 crore debt number and I project the numbers forward, our ROCEs will fall significantly from where we are seeing today, at least in the short term before these new projects start turning around the ROCEs.

Viren Shetty: Yes you are right, in the short term. But we can't sit on the existing capacity forever. So we have to grow

Ashish: Absolutely Viren! I'm completely with you in terms of the growth. When we ran our numbers, we were looking at a net debt of less than a 1000 crores, even assuming 1000 crore net debt. So the question is, if you are saying that instead of 1000 it is going to be 2000, there is a larger capex that is kind of missing though , which might not be frozen, but it's something which you are thinking about ?

Viren Shetty: Yeah. Capex planned is always prior to construction and spread over a duration, depends on the situation we can always take a call on increasing or decreasing the capex spend to match the cash flows and revisit the plans depends on cash flows situation ..

Ashish: How much is the cash which is trapped in Cayman as of date?

Sandhya J: We have about USD20 million of cash in addition to mutual fund investment of USD 20 million . I just want to clarify that, I don't know if it is right to see the cash trapped in Cayman. We can always bring it back. We just have to pay tax and bring it back .

Ashish: That's what I meant Sandhya that there is a tax incidence.

Sandhya J: Yeah. Every income has a tax incidence. So that is a decision we will take. If we need the cash, we will bring the cash. It's not locked there.

Ashish: No, I think Sandhya, the fact is that you're doing a large capex , so you might as well use the cash. Why do you pay extra dividend tax?

Sandhya J: Yeah which we intend to do here. We intend to use for our capex and if there is need in India, we will bring back the Cayman cash.

Ashish: Okay. Thanks a lot.

Debangshu Sarkar: Thank you. Anyone else? Any question? Dheeresh, do you want to ask any follow up question?

Dheeresh Pathak: Yes, I do. So I'm just trying to better understand the capex that we've outlined for Cayman. So if I understand correctly, please correct me if my understanding is wrong, so \$100 million for a 50 bed asset and then there are a few clinics and other things that we want to do. So I'm just trying to understand like, what is the revenue potential of this capex that we have in mind? Because this is not just going into hospitals it might go in to a few other clinics and other off hospital

kind of assets as well. So what kind of an asset turn and margin profiling do we have in mind at maturity for this incremental \$100 million?

Anesh Shetty: So just for clarity, the \$ 100 million will be invested as we previously outlined over a period of 2 years, some of which will be incurred in the current year . it's about 40 million or so, we project to incur in the current year and the balance in the next year. As you rightly said, most of it will be towards a single cancer focused hospital with an attached radio therapy block.

The hospital will be commissioned in two phases. The first phase is the radiotherapy facility, which we hope to commission in the fourth quarter of this year. So that's within the January to March of 2023. So that's the standalone radiotherapy block, which is part of this facility and part of the capex number you mentioned , That will start treating patients immediately, from January to March of 2023. Thereafter, we hope to commission the rest of the hospital, which will have the in -patient facility, the operating rooms and the other ancillary services with the cath lab and things like that. Over the next, 12 months subsequent to that commissioning of the radiotherapy facility.

In terms of margin expectation, in the short term, obviously, because the hospital will have an initial ramp up period, it's not going to be anything in several years what we saw when we first entered Cayman because this is now unknown markets to us. There will be some amount of dilution of margin simply because there will be some amount of business shifting from the existing facility to two facilities. But in the long run, we absolutely intend this facility to be on the whole , Our margin should not be diluted to where they are currently. So we currently are in the range of 40% to 45%, and that is our long -term target as well to preserve that, of course, on a much larger base, given both the hospitals and given the dedicated cancer focus, which is an offering we currently do not offer.

Dheeresh Pathak: Okay. So this is very useful. But given that it's a cancer hospital, even otherwise, you have told us not to look at ARPOB and bed occupancy and all that, because that might not be a right metric, especially since it is a cancer hospital, because a lot of it would be day care procedures as well. So what is also important apart from the margin number that you share, to understand the revenue potential that whatever cancer block that you are putting, the radio block as well as the other assets that you're putting, how much revenue potential? Because the current asset, I think if it is properly utilized, is generating about \$120 mn annualized. So will this \$100 million have similar potential or will have much higher potential of the absolute revenue that it can generate at stability?

Anesh Shetty: Sure. So you are right about approximately in the ballpark of the current asset, give or take, plus or minus \$10 million here and there. We have refrained from committing on an exact revenue guidance number for the new hospital. But what I can share is, from a margin perspective, after the initial ramp up period, we do not expect it to dilute our existing margins.

Dheeresh Pathak : Okay, thank you so much.

Debangshu Sarkar: Anybody else any questions? Yes Kapil, can go ahead.

Kapil Marwaha: Yes. Congrats on robust performance , as Dr. Rupert mentioned. Can you give us some broad indication on whether this robust performance has continued till date?

Sandhya J: As you are aware, Q3 is normally a seasonally weak quarter. In the month of October there was Diwali, there was also Durga Puja/Dussehra across the country. So there would be some moderation that would happen like it happens every year in Q3 in the numbers , and we have seen that moderation happening through October numbers. But, I think some of the underlying work that we have done, which has created

the traction

that we have achieved, those are here to stay. And therefore, to that extent, I think the momentum you would be able to see, but Q3 is normally a weak quarter in general for healthcare.

Ahmed Madha: I wanted to understand a little better on our matured hospitals, or rather our flagship hospitals in Bangalore and Kolkata. So, even if I see from Q1 to Q2, there is some margin expansion. So, from here on, from the current base, is there further scope for volume expansion and margin expansion in those hospitals?

Viren Shetty : Volume expansion may not be much because the buildings are quite full. The margin expansion would come through a lot of the efficiency work that we are doing – reducing length of stay, changing the payer profile of the patients focusing on high -end surgeries , But, it won't swing that much over each individual quarter. So, for example, as Sandhya mentioned, this quarter now, because we have less patients coming from east because of Durga Puja, less patients coming for checkups because of Diwali, there would be some moderation there , But, it picks up after that; in December time again it starts to pick up. But ultimately, for these, the next level of growth will ultimately only be driven by addition of capacity. There's only so much tweaking that can be done, that will give the single digit increments here and there. If you're looking at the big numbers, we'll have

to do full -scale capacity expansion, which is what our plans have taken into account for the next 2 -3 years.

Prashant Nair: Good afternoon. Thanks for the opportunity. I have two questions. Firstly, If you compare to pre -COVID levels, where are we now on out -patient flow and on international patients?

Sandhya J: So, on international patients, we are around 7.5%. We have already given commentary earlier saying there have been changes in the way we our business model for international patients is. And therefore, recovering back to the pre -COVID levels, we'd have to see how it goes. I think we are reaching a good improvement quarter on quarter in our international patient volumes.

Now, as far as IP -OP is concerned, I think we have largely returned to our pre -COVID volumes. In fact, we have been able to exceed them as well.

Prashant Nair: Thanks. My second question is on , in earlier calls you had mentioned that you would be looking at opportunities outside India other than Cayman. If you can just elaborate what form these could take in terms of capital deployment, what kind of assets you're looking for, that would be useful?

Anesh Shetty: We've been in the Caribbean region for about close to 8 to 10 years now, and through this time aside from Cayman , we've naturally developed an interest in the other islands to look for a similar market elsewhere. For the past 2 years we have been engaged with the government of Sr. Lucia for the management contract for an asset for them, that has recently concluded. It is challenging to find another market like Cayman in terms of revenue potential, stability, geopolitical risk and other associated factors. So, our choice, our preferred method of expansion, I would say, Cayman would be initially in an exploratory fashion, then maybe a consultancy fashion, followed by very very capital light model, if at all we do get there. In the foreseeable future, we do not have any clear visibility in deploying anything in the range of \$50 plus million or large sums of money outside Cayman for now. And, we already have significant amount of outlay within Cayman itself.

Having said that, there are multiple early opportunities that we are pursuing, but these would have a long gestation period for us to learn more about the market, to develop the relationships and have the confidence to essentially pull the trigger over there.

Prashant Nair: Thanks a lot. That's it from me.

Debangshu Sarkar: Thanks Prashant. Gagan, you can go a

head with your question.

Gagan Thareja: Thanks for taking my question, and apologies if this is a repetition as there was another call to join. So, the first one is on gross margins. Your gross margins for the quarter end for the 1st half are up by a reasonable amount. Is that sustainable, and what is behind it?

Sandhya J: So, a lot of the improvement in gross margin, after you eliminate the one time we spoke about, is coming from underlying improvement in the cost structures. We've also got a favourable impact of the mix which is playing there. So, from what we have done in terms of sustained level of costs from an efficiency point of view, I think it is sustainable. But what I want to definitely call out is that the cost situation is exceptionally volatile for us. One is, currency is playing a big role in terms of both medical imports as well as biomedical equipment. So, that is one aspect which is playing out. The second also is, the stability in the supply chain has not been achieved at all. So, there is tremendous disruption that we are seeing in the supply chain in terms of various input material. And so, while we are holding on to what best we can do in terms of cost and efficiency, the situation is a little volatile for us, and I think it's true for all other healthcare organizations as well. So, it's a little difficult to give a very strong forecast that, yes, these cost levels we can maintain. These efficiency levels, yes, I think we can maintain.

Gagan Thareja: And, your cash flow from operations has also jumped significantly. There's release of working capital. Can you elaborate a bit on that? And from a net working capital base standpoint, what we've seen for the 1st half via your cash flow, is that representative of a sustainable future?

Sandhya J: Yeah okay. As I've mentioned earlier in our commentary, one is that we've been hyper focused on being able to manage our investments within our cashflows. But, I think one of the reasons why we have the release of working capital, is because we were able to improve on some of the collections from the government payers. So, a lot of the backlog that had got accumulated with some of the government payers got released in the current quarter, and that helped us with cash flows and working capital. But, like it is with every government payer, it depends on when the government releases funds to them, that's when they pay us. So, the underlying operating cash flow is sustainable. The timing of some of the government collections coming exactly as on 31st December and therefore being able to maintain the same number on 31st December, that is a little difficult to pull us.

Gagan Thareja: Right. And, on the India part for your capex program, your total is around INR 2,000 crores, INR 800 crores goes to Cayman. Can you just explain what's the part of capex that comes in this year, what comes in next year, and how does it commission, over what timeframe does it commission or what timeframe do you see that getting utilized optimally?

Sandhya J: Like we'd explained earlier, about north of 1,000 crores is what we think we will spend this year, of which around 300 crores was the Sparsh asset itself and Cayman expansion is about upwards \$30 million. The rest of it is on both, brownfield, replacement maintenance, capacity expansion, etc. Next year also, I think, Cayman will be about \$60 million, and then we would have the expansion, as Viren explained, we would like to do expansion within Health City, as well, in Kolkata. These are all construction linked. So, while we are saying we will spend that money, it depends on how we do the project and build up and availability of input material and stuff like that. So, we believe that, including Cayman, we'll spend upwards of 1,000 crores over next year. Some of the greenfield construction will go into FY25 also. Over this duration, because the equipment ordering and al

l of these long-dated thing would be matched to cash flows and the requirement on site.

Gagan Thareja: So, when do you see the new greenfield assets coming on board, operationally for you?

Viren Shetty: It will be around 30 months plus. A new construction takes about 24 months. Add another 6 months to commission and any kind of contingency.

Gagan Thareja: Right. And in the interim, with addition of machinery on your existing assets in Kolkata and Bangalore, what sort of ramp up is possible from the same hospitals?

Viren Shetty: There is some minor work that is going on on room refurbishment, adding one extra OT in the heart hospital Kolkata, adding a few OPD rooms. So, those things are marginal at best. They will have a minor contribution, but it won't swing the needle too much.

Sandhya J: Just to add, there is expansion work happening in specialties across. So, we are setting, for example, oncology capability in many of our hospitals, key hospitals. So, there will be some of the routine capex which we are talking about, is not just room addition, but also specialty, capability addition. So, that will give us the volume uplift and flow through, even though we may not be increasing the number of beds.

Gagan Thareja: Okay. So on the India assets, would it then be reasonable to assume that the first half margin trajectory can only be a little better? I understand, you talked of some volatility in your gross COGS line. But ceteris paribus, should margins improve at least for the India piece? I understand, when new assets come in, there's a period where they consolidate before you come back to your optimal margin. And, I'm talking about India till the time the new assets come in?

Sandhya J: You can assume that, subject to of course, what we have said that there's a lot of volatility on the costs side which we are not able to forecast.

Viren Shetty: Yeah, that's an important disclaimer you added there – all things being the same. Things that we project could happen. One is, obviously the macroeconomic situation, interest rates, things we don't control, government receivables stretching beyond, any kind of margin capping, adding more drugs to the price control list, salaries going up by quite a bit. There is a huge amount of demand, for example, for nurses and doctors in US and Canada. And usually, when that happens, a lot of senior nurses and doctors tend to leave the Indian system, and we need to pay a lot higher to either retain or to get more people there. But, these are part and parcel of any business, and ours is a business that does have as many external shots as the others. But, the thing that we are working towards, that we will strive for, is to deliver a sustainable growth to be able to increase our utilization, to be able to treat more patients and go into very high end surgeries, and that we'll continue to drive. And, if it so happens that these things have an impact, the impact will be there, it will be limited, and we will be able to find a way to get around it in the next quarter or after.

Gagan Thareja: And finally, what's the headroom for improving your ALOS and ARPOB. I think, in the last call you did indicate that the aspiration is to take ALOS to the level where it's benchmarked to your peers over a period of 2 to 3 years, if I recall correctly. Do you stand by that? Do you feel in the coming 2-3 years you have room to reduce ALOS, and with your case mix, what should be a possible trajectory for your ARPOB?

Dr. Emmanuel Rupert: There will be only a marginal improvement, because we will be mainly working on efficiencies in reducing the ALOS and working towards the kind of procedures that can be moved in. And, a lot of things are changing in the way the clinical pathways can be worked on, and that is something which we are working with the clinical teams and we are working in tandem with them to get this done. We will be seeing, on

a quarter on

quarter, marginal improvements, and whatever guidance we'd given in the last earnings

call, I think that could be achievable. We are working towards our ARPOB improvements, but the fact is, ours is a brand that's more targeting the affordable class, and our room configuration and our pay or mix is skewed more towards middle and low-middle class. We will always be at a ARPOB disadvantage compared to our peers, but how we seek to improve the gap is through the efficiencies, addressing the A LOS, increasing the throughput utilization which will get there. But, we are looking at changing the pay or mix. Like we said, in the capacity expansion or the investments we are making on our infrastructure, adding new service lines, adding new capacity to Bangalore and Kolkata, is built at addressing a lot of those structural differences that exist between our hospitals and the rest. But, it will take time.

Gagan Thareja: And, for the asset that you acquired in Bangalore, if you could give us some idea, both from an operational standpoint, from a business mix standpoint, and from key financial numbers on that one? And, what are your plans for that?

Viren Shetty: So, this is an orthopedic only hospital that is adjacent to our Bangalore Health City.

Under the terms of agreement, they would only do orthopedics, and we would do everything but orthopedics. So, the biggest advantage this brought to us was that it unlocked the ability to do orthopedic procedures. This would be added to our Mazumdar Shaw numbers, because the unit by itself, we didn't want to report it independently because it's not as big as the Mazumdar Shaw, and it would integrate very closely with that, so that would operate as a specialty. The building itself is about 110 bed infrastructure, it can go up by another 100 beds over there, but that we won't be taking up right now, we would be taking it up in time. But essentially, this is something that was an add on. It immediately decongests a lot of the beds that we need. It immediately adds more capacity for us, which can be used in the run of the mill business. But over time, because of the additional orthopedics, we'll add a new service line to our Bangalore numbers, and orthopedics in most hospitals is the 2nd or 3rd largest department for them. So, to give us that platform to be able to build and grow this as well.

Gagan Thareja: How much capacity does it release for you if you shift the entire orthopedic payload to this one? The one that you're handling right now, when you transfer it to Sparsh?

Viren Shetty: I mean, in theory 100, but that's not practically how it's going to work out, because you'll have to build up a specialty from ground up when we acquired it. We have to get new doctors, we have to equip and get all the latest equipment, get a new clinical team, because we acquired it from an existing Bangalore orthopedic group, and so it didn't come with any doctors, and the patients also left with the business, for us, we'll be taking it because of its strategic proximity to our Health City, and would allow us to build that up. So over time, it would get full very quickly and will get expanded. But for right now, it was more about adding orthopedic and the trauma specialty to all our offerings.

Gagan Thareja: And, how does the margin profile or A LOS and ARPOB compare to your metrics?

Sandhya J: Sparsh, last year they had done about INR 49 crores of revenue, but that was COVID impacted. If you analyze the period for Sparsh for this year before we acquired, it would be about 55 crores of revenue. They were having a margin profile upward of 20%. I think, we will be able to maintain their margin profile. I also think we will be able to reach the revenue thresholds, in fact, grow from where Sparsh was. A LOS, because this would be part of MSMC, in the overall volumes that MSMC does what comes from Sparsh, I don't think it will

I significantly move the A LOS number for MSMC or for Health City.

Gagan Thareja: Thanks. That's all from my side. Thanks for taking my questions.

Dheeresh Pathak: Yes, on bookkeeping, this Cayman asset in the preceding June quarter of this year, was how much EBITDA? If you have it handy.

Anesh Shetty: EBITDA of USD 8 million on 22.7 million revenue.

Dheeresh Pathak: Okay, for Q1 FY23. There were 3 assets that we were disclosing earlier. So, SRCC and Ahamadabad you had explained in the last call the reasons for delay in the maturity timeline. For Gurugram as well as Dharamshala, if I look at the way you have disclosed the western block numbers, maybe for Gurugram and for Dharamshala, if I understand correctly, they were started in FY18 and it's been few years, So, I would have expected them to have reasonably good margin profile by now. So, is there a specific reason for each of those assets and why they've been delayed. You explained for SRCC, can you explain for these two assets also as to why they have been slow to ramp up to maturity?

Dr. Emmanuel Rupert: Yeah, one of the reasons is, Dharamshala is heavily focused on only the oncology specialties and we were trying to grow completely new specialties into that. The growth has happened in steps and stages in other things, but for more than 2 ½ decades they were known only as a cancer hospital. So, we are starting the other specialties as well and we are reaching there gradually and the growth we will see in all the specialties, including oncology, and that will bring us into the next phase of growth for Dharamshala.

As far as the Gurugram hospital is concerned, it's a smaller facility compared to many of the other major facilities which are present in Gurugram. And it's a little smaller facility, so we are picking and choosing the specialities that will fit into this profile and the kind of ALOSs and mix that needs to be there, and we're trying to sort that out.

Dheeresh Pathak: You said it's a smaller facility. How many bed hospital is Gurugram?

Dr. Emmanuel Rupert: Gurugram has 223 capacity beds and around 159 census beds.

Dheeresh Pathak: It's not that small, so there must be some other reason I'm sure, for the delay in the maturity profile.

Viren Shetty: Yeah. If we talk about all the reasons, one is, we are a completely new brand coming into Delhi, and Delhi is not exactly a virgin market. And, Gurugram especially is one of the most competitive markets for healthcare in the whole country. We came in cancer focused, yes, but a little bit niche in that. It was little smaller compared to, let's say, Medanta and Mr NRI. We had a very different set of clinical mix. We didn't have too many superstar doctors; we had 1 or 2, but we didn't have the best of breed of consultants that we were going up against. Our hospital was not very strategically located compared to the others. There are lot of lessons that we had to learn. Not to say we're justifying the performance in any way, they definitely did perform below our expectation of what we had, and we did learn our lessons and had to recalibrate our approach for the Delhi market. Having learnt that and understanding what we need to do going forward, that will be more on optimizing the infrastructure that we have, and looking at some capacity addition and some reconfiguration of the clinicians, looking at departments that do best fit well with things that perform well to our strengths. But yes, it's definitely something that we are far behind compared to all the other hospitals that are there, there's a lot more that we could've done, that we were not able to do. We have to consider Gurugram was started as the most important attraction to us even though it's not on the main road, the most important attraction to us because its closest JCI hospital to the Delhi International Airport. And, it started and one year later, we had COVID.

So, the international traffic had gone into a halt, and even before that, given the market for referral kickbacks is quite atrocious. We just had to get out of that business and rebuild the whole thing with a domestic focus. So that meant, whatever we built for, we have learnt from our experiences in that market, we'll keep expanding and based on the cash flow we generate, we'll be able to take it to a more sustainable future.

Dheeresh Pathak: This is very useful and thank you for that detailed explanation. While you were explaining the SRCC loss, if I understand correctly, in the western cluster, apart from the SRCC, there was the Ahmedabad as set. So, if I adjust for the SRCC loss, the Ahmedabad asset also doesn't seem to be making a mature kind of a profitability. Like you explained for Gurugram, if you could also explain... if I understand correctly... is that the right understanding? And, what is the reason for Ahmedabad also?

Viren Shetty: Ahmedabad also we came in with one set of assumptions. Ahmedabad shares a lot of similar dynamics as Mumbai, in that consultants don't get themselves empanelled by hospitals. They are visiting consultants and they take a percentage of the patient revenue. So when we came in, we didn't want to adopt that model revenue sharing on part time basis, so we brought consultants from outside who would work with us on a full time empanelment, only to learn later on that one of the worst things you can do for a market like Ahmedabad, is to bring outsiders. It was too late by then, we had the team, so we built it up. The other thing was that, coming from outside, we had no idea about the particular dynamics of the Ahmedabad market. There is a river that separates two parts of the city, and we coming as outsiders, had no idea of the inter-communal dynamics that exists between those two, and chose the one which we thought was a lot more densely populated and gave us a much larger land parcel and was more central, but it turned out to be not a place for at least people with money want to go.

So, it still does well. It attracts a large number of patients, but it's heavily dependent on poorer patients who are under a government scheme. Now, the government scheme pays on time, it doesn't pay well. And, to the extent that they keep paying, we're able to keep the show going, but, we have to take that hospital up to that next level. So, to reach that breakeven level it's got a decent number base, and now we can just build on that.

We are adding oncology over there, so that will change the mix of departments a little bit. We are adding a couple more beds that are little more high-end private rooms, semi-private rooms, and it will gradually build up from there. There again, lessons learnt on location, as well as coming in with the wrong consultant engagement mix. Again, we've passed that, and now we can just add to our strengths, but it won't deliver outsized returns to the kind that we had expected. So that's why, even in terms of the capex spend, the bulk of it we'll be spending on the hospitals that demonstrate the strongest numbers and the strongest potential for growth.

Dheeresh Pathak: I understand, thank you for that explanation as well. So now in Ahmedabad and Gurugram, instead of the fulltime consultant, do you have visiting consultants in those two assets?

Dr. Emmanuel Rupert: We have a combination of both. We do have in specialities, which has got larger footfalls and the potential we do have full -timers. But, we also have lot of part -timers who have the ability to bring their patients and provide services.

Debangshu Sarkar: Thanks Dheeresh. I think with that, we will conclude our session as I don't see anybody with the raised hand feature out here. Thanks everyone for your active participation, as always. Please do feel free to reach out to us in case of any follow -on queries that you might have. Thank you once again.

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Call: Feb 2023

Date of submission: 15th February 2023

To,

The Secretary

Listing Department

BSE Limited

Department of Corporate Services

Phiroze Jeejeebhoy Towers,

Dalal Street, Mumbai – 400 001

Scrip Code – 539551 To,

The Secretary

Listing Department

National Stock Exchange of India Limited

Exchange Plaza, Bandra Kurla Complex

Mumbai – 400 051

Scrip Code- NH

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter and nine months ended 31st December 2022

This is further to our earlier letter dated 11th February 2023 regarding audio recording of Earnings Call of the Company for the quarter and nine months ended 31st December 2022, held on 10th February 2023.

Please find enclosed herewith the transcript of the said Earnings Call. The same is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earning-call-transcripts>.

This is for your information and record.

Thanking you.

Yours faithfully

For Narayana Hrudayalaya Limited

Sridhar S.

Group Company Secretary, Legal & Compliance Officer

Encl: as above

“Narayana Hrudayalaya Limited

Q3 FY23 Earnings Conference Call”

February 10, 2023

MANAGEMENT : MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – CHIEF OPERATING OFFICER , EAST AND
SOUTH REGIONS

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS
SUBSIDIARY HCCI

MR. NISHANT SINGH – VICE PRESIDENT , FINANCE , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. DURGA PRASAD – SENIOR MANAGER , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

Nishant Singh: Good afternoon everyone. My name is Nishant Singh. I head the IR Function at Narayana Hrudayalaya. I welcome you all to the Q 3 FY23 earnings call of the company. To discuss our performance and address all your queries today, we also have with us Mr. Viren Shetty - our Vice Chairman, Dr. Emmanuel Rupert - our CEO and MD, Ms. Sandhya - our CFO, Mr. Venkatesh - COO of our Eastern and Southern domestic operations, Dr. Anesh Shetty - MD of our overseas subsidiary HCCI, better known as Cayman and Durga Prasad - Sr. Manager from the team. We hope you have gone through the investor collaterals which have been uploaded on the stock exchanges as well as on our website. As usual, before we proceed with this call, I would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website as well as on the stock exchange at a later date. I would also like to remind you that everything that is being said on this call that reflects any outlook for the future or which can be construed as a forward-looking statement, must be viewed in conjunction with the uncertainties and the risks that they face. Post the call, should you have any further query, please do not hesitate to get in touch with us. We would like to address it to the best of our ability.

With that now, I would like to hand over the call to Dr. Rupert.

Dr. Emmanuel Rupert: Good afternoon everyone. I warmly welcome you all to the Q3 FY23 Earnings call conference of Narayana Hrudayalaya Ltd(NHL) . The third quarter of fiscal year delivered steady performance supported by the growth in business across our flagship units,

newer hospitals and improvement in payor mix. Consolidated revenue for the quarter stood at INR 11,282 million reflecting a Year -on-Year growth of 17.5%, and a marginal decline of 1.2% compared to the previous quarter due to the seasonal nature of the business which affect patient footfalls. However, contribution from international patient footfalls remain unaffected.

Narayana Hrudayalaya generated consolidated EBITDA of INR 2,660 million in Q3 FY23 at a margin of 23.6% against 22.3% of Q2 FY23, when adjusted for the one -time other income. This margin improvement is attributed to improvement in payor mix and cost efficiency.

Our Cayman unit managed to contribute to the overall performance despite holiday season, and HCCI revenue marginally declined by 3.2% to USD 28.2 million. We remain optimistic on the robust growth prospects of overseas business in Cayman and continue to explore the investments in surrounding islands on opportunistic basis.

Our overall balance sheet and liquidity profile remains strong with group cash and liquid investments of over INR 5.6 billion against gross borrowings of INR 7.8 billion (Net debt of INR 2.1 billion) as of 31st December, 2022. Our debt -to-equity ratio has further improved to 0.11 against 0.14 in Q2 FY23, giving us sufficient room to fund our expansion through our mix of borrowings and internal accruals. We have incurred capital outlay of close to INR 6.8 billion (includes the Sparsh acquisition) for 9 months ending 31st December, 2022, and in the fourth quarter we expect to spend the balance amount of guided capex of INR 10 billion for FY23. We have also provided further disclosures in slide 12 of our investor presentations on capex spends for FY22 and expected spend for FY23 and FY24.

During the pandemic, we partnered with YouTube on digital awareness program to provide reliable healthcare information on a daily basis. We created more than 600 videos to provide evidence -based answers to questions of YouTube users across India in 6 different regional languages. This initiative which received 45 million consolidated views and more than 7,000 likes, allowed people to understand their affliction in a very difficult period. We are proud to share that this was featured in Google's

Impact Report

on 2021 which was released in December 2022. We're strengthening this partnership with channels like YouTube.

Our total investments in digital engagements channels grew online traffic by 97.5% to 5.4 million users in the current quarter as against Q 2 FY23.

We're also pleased to inform you about the soft launch of Athma SaaS version specifically designed for small healthcare facilities such as diagnostic labs, nursing homes and standalone hospitals. Athma SaaS offers a streamlined integrated solution for an underserved segment of the market scattered across tier 2 and tier 3 cities. The team is onboarding foundational customers to gather valuable usage data and feedback to further enhance and refine the platform.

Aligned with our vision to provide integrated healthcare to wide sections of the society, we have piloted a subscription program in Bangalore to cover individuals and family members. We have recently crossed a landmark milestone of 1,000 subscription plans and are witnessing continuous traction. These results are encouraging us and give us confidence to rollout a comprehensive personalized seamless healthcare service at the doorstep through digitally integrated patient touchpoints. We will keep providing more information in the coming quarters.

We continue to strengthen our current position and grow our brand through several various strategic initiatives that will drive our vision to offer a gamut of integrated quality healthcare services to people across geographies at an affordable cost.

Thank you.

Nishant Singh: I would request everyone to now use the 'raise hand' feature to start posing their questions.

Prithviraj: Sir, could you give us the revenue and EBITDA of the new hospitals for this quarter?

Sandhya J: Okay. So, we have been able to continue to improve our performance on the 3 new hospitals. We have, from a revenue growth point of view, the revenue was about INR 110 crores for this quarter for the new hospitals – SRCC , Gurugram and Dharamshila. This was a 17% growth compared to last year. And the EBITDA which we were able to deliver was 7.3%*. In this the Mumbai hospital has almost broken even, and Gurugram and Dharamshila are positive. So, that is the number for the new hospitals.

Prithviraj: Thanks for that. See, this 7.3%* EBITDA margin is actually quite a good improvement compared to what we have seen over the last few quarters. So, how long will it take for this to reach a sustainable EBITDA margin of say 15 -17% from now on?

Sandhya J: I think for Gurugram and Dharamshila we are almost there. So, I think as we see the next few quarters, we should be able to reach a reasonably sustainable margin for Gurugram and Dharamshila. Mumbai , we have broken even actually in January. So,

Mumbai will take us maybe more than a year, but we are in that direction. But, Gurugram and Dharamshila we are almost there.

Prithviraj: And, my next question is on the Cayman business. I guess in an interview you mentioned that the oncology department will commence from Q1. So, how much is the capex that went for this particular block and what kind of revenue potential are you looking at?

Anesh: Hi Prithviraj, thank you for the question. The last slide of the investor relation presentation talks about the capex that we've incurred, and we project to incur. I would *Inadvertently mentioned as 9.3%

say about 80 -90% of that is for the new oncology block which includes not just the oncology block, but the new hospital as well. The new oncology block, yes you are right, we are on track to treat our first patients from early Q1 of the next year. So, in April hopefully we'll be starting radiotherapy services. Since it is a department, essentially a service line for us, we won't be in a position to share specific revenue projections for a particular department.

Prithviraj: That's helpful, thank you.

Dhara: Thanks for the opportunity. I

had 2 sets of questions for Cayman. So, despite the declining number of discharges from last 4 quarters, our average revenue per patient has actually increased for Cayman. So, what is the driving factor for this?

Anesh Shetty: Hi Dhara, thank you for your question. So, there are few reasons for that. So, the first is essentially an increase in the complexity of patients that we continue to treat. As we commission more service lines which are catered towards our oncology offering and getting ready to ramp up the linear accelerator and the comprehensive oncology program, we are going to be doing more and more complex patients.

The second reason is the discharges, which is the denominator for the ratio you are referring to, as well as the discharge as an absolute number, we had a migration of our core EMR system from the legacy system to Athena a couple of quarters ago. So, Athena is more sophisticated in that. We are able to breakdown discharges into more granular details of daycare, emergency and regular IP discharges. So, we are working on reconciling the pre-Athena and post Athena discharge numbers for this maybe in a quarter or two.

Dhara: Sir, if we're saying the new oncology block will be operational from Q1 FY24, should we expect the number of discharges will also grow from here on?

Anesh Shetty: So, the oncology block which will be ready from April will be only offering radiotherapy. We continue to offer medical oncology. I mean, we have been offering medical oncology for a while and that will continue. Surgical oncology will build up as well. So, radiotherapy, the number of discharges, these are predominantly day-care patients, so they will not be in inpatient discharges. And, although the realization is high and we expect the margins to be commensurate with what we are earning, the number of patients will not be very significant to move the needle in the early days, that will take some time.

Nishant Singh: Thank you Dhara. The next in line is Arsh.

Arsh: The question is on EBITDA margin that was given in the previous investor presentations. From what I have from the previous quarter, Bangalore was around 35%, East was around 40%. So, could you possibly provide me the breakup for this quarter?

Sandhya J: Actually, we are kind of aligning to industry here. I think also, our units are getting matured. So, we are now stopping that. Earlier we were providing that distinction because different stages of maturity were there for different geographies. Now, more and more we want to align with how industry represents data. But what we will continue to do, like we did at the beginning of the call is, we will provide information on our new hospitals, revenue, and EBITDA, until they come to a state where they are steady and are able to make run rate returns. Until then we will provide that data in the investor call. But, we believe that the time has come that we don't need this distinction between regions. They are more or less consistent in terms of performance.

Arsh: Sure. And just one follow up question. So, the ALOS (Average Length of Stay) has been consistent over the last few quarters, but there was a lot of talk about robotic surgeries and other things. So, is this expected to reduce the ALOS going forward?

Dr. Emmanuel Rupert: The robotic surgeries, when you compared to the overall number of discharges across the group, the number is small to make a major difference. While however, we are continuously working on operational efficiencies and trying to reduce the pre and post procedure length of stays. And that's what, when we had given some kind of a guidance level, we said that over 6 quarters we'll try and bring it down to around 4.1 - 4.2 kind of a thing.

Nishant Singh: Thank you Arsh. The next question is from Mr. Ashish.

Ashish: Hi! Thank you and congrats for a good set of numbers. I had a question from a medium-term perspective. G

even the fact that we are spending 2,000 crores capex between this year and next year, which is basically doubling our current net block, how long do you think the new capex will need and lot of it is brownfield. So, how long do you think it will need to get to, let's say, a 20% plus ROC on the incremental capex? Would it be a couple of years or faster? What's your thought process around that?

Viren Shetty: Ashish, it will take couple of years. Construction itself, given that some of this is in running hospitals, means that the work goes half as quickly as it would in a greenfield setup. So 2 ½ years on construction. Breakeven time should be faster because you're leveraging... I mean, they may not be breakeven time because you're using it within the existing manpower and so on. But definitely in terms of returning positive EBITDA, that would take couple of more years after that.

Ashish: So FY26 would be fair to look at it?

Viren Shetty: It's a good ambition, but it could extend beyond that.

Ashish: Okay. When you said return, it means a 20% kind of ROC on incremental investment?

Viren Shetty: Yeah.

Ashish: Would Cayman be faster, because the fact that it's an oncology block and the construction is getting over for radiotherapy block very soon?

Anesh Shetty: Hi Ashish. So, the Cayman capex, like you said, is phased. So, oncology which is a small portion relatively, about 10 -15% of the total capex we'll be incurring. That will be ready from March onwards. It's the only radiotherapy facility in the country, so we don't expect any ramp-up phase for that; it will be active from day 0 and we expect that to be just like any other service line from the beginning. The larger hospital which will be ready, it's a separate campus, it's a separate location, we're not spending this within the same campus. Having said that, it is the same island and it's a small island. We don't have a specific number to share as to how many months it will take us to breakeven. But as I mentioned, it should be quicker than going to a new location and starting from scratch.

Ashish: Sure. And secondly Anesh, in terms of Cayman, in the past we've talked about business from new islands. What is happening to that now that we are post COVID, or do you think that's something which will still take some time?

Anesh Shetty: Oh no, definitely! Since the travel restrictions were removed post COVID, the other was the island opening up completely. So, as we discussed during the last few calls as well, there's no longer any restrictions and it's been that way for some time. So, we are seeing a lot of the Caribbean channels that we were working on for many years, open up. We are seeing a steady growth and return to normalcy in the overseas patients we're getting from the other Caribbean islands, and that's going to be very helpful as we ramp up the radiotherapy facility as well, because there aren't too many equivalent facilities in the region.

Having said that, as of now, the local patients constitute the bulk of our business, but we're seeing very positive signs with some of the overseas markets. These are patients that come to us, just to us soliciting them, whereas others are activities where we go to other islands for different levels of engagement to try and attract patients back to Cayman.

Ashish: And in terms of capacity utilization, where are we currently on the current facility, on the IP side?

Anesh Shetty: Sure. So the facility has 110 beds, and as we've said in the investor presentation, I think this quarter we're a little over 50 -55 beds in occupancy. We have adequate capacity to treat whoever needs to come. There are days when we have shortages in certain critical infrastructure, but on the whole, we are very comfortable with getting incremental business. And this will line up well with the new hospital which will be commissioned according to our previous guidance, which

will significantly increase our capacity across all classes.

Ashish: Sure. You're not expecting a step-up jump as the islands completely become open in terms of the other islands? 70% plus kind of a utilization on the existing capacity?

Anesh Shetty: It will happen, but it will not happen suddenly, because like we discussed a couple of calls ago, when Cayman had to shut down, essentially people in other markets didn't have anywhere to go. They had to diverge and go to other markets that were willing to take them. So, swinging those channels back to us is a gradual process. All islands are open completely right now, nobody has any restrictions. So, it's not going to be a sudden jump up. As you had guessed, it will be a gradual movement that will take time. And that works perfectly fine for us because it helps with phasing out towards the new building.

Ashish: Sure. Thanks, and wish you all the best.

Nishant Singh: Thank you Ashish. Do we have any further raise of hands? Yeah Gagan, please go ahead.

Gagan: Good afternoon. The first question is on the gross margins. From Q1, Q2 and Q3, gross margins have consistently improved. What is the sustainable number for gross margins

as we go ahead?

Sandhya J: I think the current number is sustainable, we believe. The reason you are seeing the improvement is also because we have been able to do more with the current capacity.

Venkatesh: I'll answer your question Gagan, this is Venkatesh here. Yes, you have seen correctly, the gross margins have been improving quarter on quarter. What we can say is, we can sustain these margins. Primarily what we have seen last year, we had COVID on the base lines, so the consumable cost was always high. But now that COVID is not there, we have been able to work our savings relating to that part of consumption, and plus we work very hard with the supply chain to bring out further improvements in the consumption area. More than that, we've also worked very hard in terms of improving our throughput so that we could utilize our existing capacity to the optimal. So, what we have done is, we have focused more on doing daycare cardiac procedures and robotic procedures which have better throughput, with a morning admission, evening discharge. We are also working on adding operation theatres, ICUs, diagnostic and lab setups so that we can process more and add more volumes within the existing capacity. We've also invested significantly in technologies to improve our throughput, say faster discharges, faster lab results and so on. And, it has actually shown results in terms of 17% reduction in the turnaround time for lab investigations so far, and even 36% of the top 5 investigations. If you see, 25% of all our consultations are all through appointment channels, and 80% of all our appointments are done through NH Care app and the portal. We also worked on increasing throughput through process transformations, improved on our discharge efficiencies, and are also working on reducing ALOS, which you will see the results in the next quarters to come. So, what has happened is, all these throughputs have helped us increase the revenues in the same capacity with the same cost in terms of overheads and manpower, and that's where we have been able to squeeze higher margins quarter on quarter. Unless there is any unforeseen headwind, this is reasonably sustainable.

Viren Shetty: But couple of the headwinds which we'll have to also keep in mind is that consumables could also come under price control, lot of the high-end implants, there is talk that the government may reduce the price of stent more than what it currently is, and add items to this. We're in a very competitive field. Doctors and nurses always have good job opportunities in other centres and in other countries. So, we consistently face a lot of challenges on the payouts. We're doing what we can, and we believe where we are is

sustainable, and we're working towards improving this. But that also gets tempered against the other massive challenges this industry faces.

Gagan: Thanks for that. My second question is around simply trying to understand, before all of the capex that you're doing, which is fruition, how much headroom do your existing Indian facilities give you to grow? I'm not looking at what growth you can get, I'm looking at the theoretical limit of what can be the potential maximum outcome of whatever you have basis tweaking and creating efficiencies.

Viren Shetty: For some of the larger format hospitals that we have, as long as we're able to put in smaller investments in just realigning the rooms, doing up the interiors, adding new machines, there will be no theoretical upper limit; you can keep growing at high single-digit revenue line, because there's enough space to tweak around. But, that is true only for 3, maybe 4 of our large hospitals. The rest of them which are smaller, they're not as well built, you may not have enough room to upgrade. So, they will reach a point of stagnation, beyond which you'll either have to invest or you'll have to just add or upgrade more capacity or go somewhere else. So, the largest flagship units have no theoretical upper limit for how much you can keep tweaking to improve the performance and that's true for most large hospitals across all the other groups.

Gagan: The reason I asked is given that there is adequate demand next year and thereafter, would your capacity suffice to grow at a double-digit rate?

Viren Shetty: With the exception of our RTIICS unit in Kolkata, which is extremely capacity constrained, double digit growth we don't want to get into the forecasting game. We will do our best to sustain this growth momentum and there are other avenues that we would try. But until the new capacity comes it will be a little challenging.

Gagan: Right, since you mentioned the two capacities, two questions there. One in terms of bed strength what does it add for India, and second in terms of funding, year to date, you managed to keep your gross debt very stable in spite of almost 700 crores capex. How should we think of the funding part of the capex as you go into the second tranche of that capex?

Viren Shetty: The bed capacity part I will answer quickly, and then hand it over to Sandhya for the capex spend. See, we haven't finalized the drawings for the new expansion that's there. And the bed capacity once we do it will come modestly, But like I said we can't keep measuring the performance and our utilization just purely the beds and occupancy.

There is a lot more of investment that we will be adding on procedure rooms, on OTs, on patient areas, patient amenities that will be a spend, can approve realization but not necessary count towards the total bed strength of Health City, Kolkata. Having said that but once we start finalizing these things and we have indicated the amount of spends over this year and next and as and when the drawings will get finalized, we will indicate that, okay, at this particular time these will get operational and how many beds will get added to the total capacity.

Sandhya J: On the net debt situation, currently our net debt is about INR 214 crores, gross debt by the end of the year we will be around INR 850-900 crores. Next year another 1,000 crores we are planning to invest. I think another 500 crores we will invest from internal accruals, 500 crores we will borrow. So, we will keep our liquidity profile constant between the years. So, our net debt will be around 500 crores by next year end, so that's the aspiration with which we are working. Of course, the situation depends on what choices we make and how it progresses. But this is the broad direction.

Gagan: Thanks, I will get back in the queue. Thank you for answering my questions.

Nishant Singh: Thanks. Next in line is Mr. Sameer.

Sameer: Hi, thanks for this, and good afternoon, everyone. The first question is on the pay or mix for the India business. The scheme has been coming down steadily now at 20%, so what's the outlook over here.

Sandhya J: So, we don't have a specific outlook per se on the scheme. We do want to keep a balance of scheme and other payors. The larger philosophy with which we have been working is that if there is an ability, if the patient does not have an alternative place to be served, we are serving those patients and we will not moderate schemes in those hospitals at all. I know where we are right now will be consistent with where we believe to be between 20 to 22%, that will be the range in which I think we will continue to be. It also depends on how the government looks at these schemes, we have been making continued representations both in terms of the collection issues that we constantly face, as well as in terms of the rates which have not been revised for a long time and they are not very economically very viable. So, we have been making a lot of representations, so if there are some outcomes that come out of those representations then we will be able to work better with these schemes.

Sameer: Okay, got it. But what you are saying is bottom line that as things stand you would maintain it at current levels 20-22%.

Sandhya J: Broadly, yes.

Sameer: Okay. That's what gets me to my next question that what are the drivers for ARPOB for India business going forward. I can see that it has been INR 12 to 12.8 million rupees for the last 4 quarters. So, where does it go and what are the key drivers for this?

Sandhya J: So, ARPOB like you are aware that price is not a significant lever that we exercise. We exercise cost levers to drive efficiency. To that extent pricing will not drive ARPOB, ARPOB will come because of mix of great going to higher quaternary procedures, as well as improvement on the pay or mix. So, you will only see, and throughput, like what Venkatesh explained that we are continuously working on the throughput. So, you will see a gradual improvement in our ARPOB, you will not see significant jump in the ARPOB. But I think we will be able to manage the overall P&L even with a gradual improvement in the ARPOB, that's how we look at it.

Sameer: Okay, that's very clear. And one final question from my side and that's for the Cayman Island, just thinking about it, is there a demand issue over there as in what would take your discharges and the in-patient volumes up with the current facility. Or it is also about the mix issue that what you can do, which specialties which you can do and which you can't and that will get addressed when you come up with the new block and we can talk about that.

Anesh Shetty: Hi, Sameer. For now, for the foreseeable future, the reason there is scope to improve our number of discharges and procedures by adding services we don't currently offer, or we offer in a very minimal capacity. So, as you are aware we recently announced an acquisition of a very niche ENT practice which is one of the highest volume practices in the Island. So, there are certain niche areas like this along with the broad specialty of oncology which we do not offer, that will definitely help us see some improvement in the volumes of discharges that you mentioned. Having said that, there are also certain specialties which we do offer which there are other players on Island where we do have a lot of market share to gain. A lot of this will be driven primarily by location. Our location is, we are disadvantaged by location although we have a massive campus, it is very far away from where most of the people live and work, whereas some of the other players have more central locations. So, this will be neutralized with the new hospital in Cayman Bay. So, that will lead to further, we expect that it will lead to a further volume gain in specialties.

ities we currently offer but where we do not have dominant market shares simply because there are other players. And of course, eventually it is not a very large market, we will reach a stage where which is your first guess where at a time there will be a demand issue. And our hope and our aim is by that time for the overseas patients to start kicking in a significant level to move the needle to drive the next level of growth. I hope that answers the question.

Sameer: Sure, Anesh, that's very clear and just on a new facility what would be the tax rate, I know I am jumping the gun but just to understand that. I think for the current business the tax rate is probably minimal, right. Will that continue for the new facility as well.

Anesh Shetty: Yes, Sameer, not just for us, for anybody, the Cayman Islands is a zero tax jurisdiction for business tax, so there is no applicable taxes. The tax concession is not time bound, that's for everyone operating there.

Nishant Singh: Thanks, Sameer. Any further raise of hands, any further questions. Yeah, Gagan, go ahead.

Gagan: Thanks for the follow up. The Bangalore new orthopedics facility that you acquired last quarter, if you could update us on what's the status there. I think last quarter you indicated that it gives you headroom to transfer the orthopedics cases from your current Bangalore facility out and therefore that creates that much space for you in terms of bed capacity. I mean are we going to see that sort of playing out in the coming year, or is that something that can happen later, and what's the status in evolution of that one.

Venkatesh: This Sparsh which we started in October, and we merged into our current operations of Health City in Bangalore. We have been able to have a smooth transition in terms of the operations in Sparsh in terms of least hiccups, most of the doctors working there have been taken into our rolls, and they are working with the same energy and vigor which they were doing when they were a part of Sparsh. To start with we are continuing to work with these doctors and also trying to aggressively source clinicians into the system.

We have already got in 3 surgeons and an anesthetist on board including the clinical lead for orthopedics trauma. As of now we have done reasonable numbers in terms of joints and other procedures in this quarter. And this quarter in fact has generated a very good EBITDA of around 30% in this quarter which is much higher than what we had forecasted. Of course, the first objective is to maintain the same performances in the next quarter and to make sure that we have got aggressive plans for growth of this particular unit in the subsequent year with higher topline volumes and more EBITDA.

Gagan: And on the margins that you used to give upto the prior quarter segmentally or at least city wise, I think cluster wise margins is standard reporting for most hospitals, be it Apollo, be it KIMS, everybody reports cluster wise operating margins. When you say when you want to align your practices to how the industry does it, the industry does report it in that way. are we discontinuing the practice? If you could give us some idea in terms of understanding the margin evolution by cluster.

Sandhya J: From how we have looked at competition and what disclosures, I think most of the focus is on the overall margins and the various levers that enable to judge performance better.

So, we are aligning to that. As we go into future quarters, we will try to publish more data points which gives, like this time we have put a separate slide on the capex across different types of capex that we will incur so we can give more visibility to how our future spends are. We will try and create more data points that helps you understand the underlying strength of the business. We will also share some of the new hospitals where the margin profiles are low, we will share that information

in investor calls so

that the clarity is there on the improvement plan. But otherwise, we believe that as a Group if we are able to share overall margins that is a reasonable well indication of where we are heading as a Group and this cluster-wise margin information is not enabling any better decision making than the data that we are already sharing. So, I think we would like to align to that model.

Gagan: Right. In between Sparsh, Mumbai and Dharamshila is there I mean you indicated there still is room to increase efficiencies with scale which could mean as you head into FY24 there is still room to improve operating margins. Any assessment of first of all the viability of this assumption into a ballpark scale of what could be possible margin improvements because of this.

Sandhya J: Sure. Sparsh I think is already around our ballpark margins like Venkatesh had shared. So, we were talking about Gurugram, Dharamshila and Mumbai which are the new hospitals. Like I had mentioned earlier, Gurugram and Dharamshila are almost there, I think they will get to reasonable margins, double digits. Dharamshila is already in double digits. So, we will get to double digit margins for Gurugram as well, in next couple quarters we should be able to get there. Mumbai will take a year. We will turn EBITDA breakeven positive also in the next quarter or so. But it will take time maybe more than a year to get to a double digit type margin profile.

Gagan: And on Cayman how should we think in terms of growth for the coming year with the addition of the radiology department. And also, when does the new hospital become operational, any timelines of when it becomes operational and when do you see a reasonable sort of occupancy in that one.

Anesh Shetty: Hi, Gagan. To your first question, I think for the past few years we have been able to demonstrate adequate double digit revenue growth from the existing facility and the existing service line. Even without the radio therapy facility being taken into consideration, we don't see that stopping at least for the next year. Having said that, adding the radio therapy facility does add a comprehensive oncology offering which will definitely help a lot. To your second question on the new hospital being commissioned, currently it is well under progress. Our timelines are dependent on a lot of externalities. We currently estimate and I think this was shared a while ago as well, it won't be commissioned in the coming financial year, I think the time we will be looking to complete the facility and get all the licenses and accreditations in place will be somewhere in the range of about 14 to 16 months from today, that's still given how far away we are from that, that is subject to change, and we will keep you posted every quarter but we expect that to happen in that timeframe. And adding onto even Sameer's question, we think that will be a big, big positive win for us simply because it neutralizes the location disadvantage that currently hampers all specialties that we offer.

Gagan: Thanks for that. Just one final question and this is more to get your opinion on observations around cataract surgeries, just a case in point to drive home a bigger point which is that if I look at the cataract surgeries done, the data that is put out for this year and if I look at what used to be pre-Covid trend, my assessment is that from pre-Covid it went down very sharply less than half maybe even worse and then this year it came to more than double of the pre-Covid levels which is basically a lot of pent-up cases getting done. And if you look at the monthly data it seems to be normalizing. If that is the case with a lot of elective procedures, as you get into the next year, and not necessarily only NH but for the entire industry per se, case mix would normalize which I would think put pressure on ARPOBs because this has been a year of exceptional case

mix, if that observation can be extrapolated to other elective surgeries. So, what's your thought on that?

Dr. Emmanuel Rupert: It might be very difficult to extrapolate from an ophthalmology practice because ophthalmology in general especially the cataract surgery is well covered and with Covid definitely it was considered as an elective non-emergency case, so most of them were postponed. And that is bound to jump up and then normalize over a period of time. But if you look at almost every other specialty in the country, we are still significantly underserved in terms of number of procedures that needs to be done vis-à-vis the population and the patient mix. And what Covid has done is, it has brought significant awareness of health to the public irrespective across the entire socio economic zones. We are quite positive that the awareness will keep pushing the relevant specialties to do certain numbers and even if you look at the kind of need that is required in the population for cardiac procedures or onco procedures we are way-way behind schedule what needs to be done. Looking at that I am sure we may not be able to draw parallels between ophthalmology and the rest of the specialties, but certain specialties may catch up at some point of time but we, from whatever 12 months data we are seeing, we have seen fairly good inflow of patients to most of these specialties.

Viren Shetty: I will put it in another way, of the people that have health problems in our country, most of them don't know they have a problem and will suffer from it for years and then eventually it will end them. For the people who know they have a health problem, very few of them will actually get help for it. Of all the people who try to get help for the problem, very few of them will go to the right place for the kind of help. And of all the people who go to the right place for help very few of them will come to us. So, in terms of the opportunity set, we are talking of many orders of magnitude between what startups like to call it a total addressable market versus the number of people that we are currently treating. So, Covid or no Covid the growth of patients with health problems into tertiary and quaternary care super specialty health care facilities like ours and comparative hospitals to ours has huge opportunity. Those generally form the first one or two decks that every hospital likes to put out. Now how we get there is through all the activities that we are doing on improving our efficiencies, improving our reach, going all over the place, increasing awareness, investing in digital branding to tell them that you know a heart problem needs to be taken seriously, get a checkup and so on. So, that's how you build the awareness and reach out to patients who otherwise are not consumers of health care which is not the case of ophthalmology because we are very well served, and most people understand.

Gagan: Thanks for the detailed answer. I will get back in the queue, thank you.

Nishant Singh: Thanks, Gagan. Now we have Nitin next in line, Nitin, please go ahead.

Nitin: Thanks for taking my question. I have some questions on the capex, two things one is for the current year how much capex has we done so far for the 9 months.

Sandhya J: 6.8 billion.

Nitin: And in terms of the depending capex about 300 odd crores which is there for Q4, what are the areas where we will be spending money ?

Sandhya J: Like we have indicated in the slide that we have shared, there are four chunks in which the capex is getting spent. There is the part on the Sparsh, etc., are done. There is certain, we had a lot of pent-up demand on our replacement of the biomedical equipment, etc. Also, there was a lot of civil upgrades that we are working on. We have spoken about it in our previous investor call. So, almost 35% of the investments that we have made and are going to make through the balance remaining part of

the year is

going into replacements and maintenance and upgrade. In addition, about 13% of the investments that we are making are going into new capabilities, setting up say new cath labs, new oncology blocks, those kinds of items. So, that's about 13% in the current year. And also, you are aware of the Cayman piece which is about 30% in the current year of our spends going into Cayman and about 20% into the Sparsh acquisition that we did. So, that's how the INR 1,000^ crores is split and this data is also there I think in the slide 12 of the investor deck. In that range only it will get spent, mostly towards the biomedical equipment and the work that we are doing on the civil side in the balance remaining 3 months.

Nitin: And 355 odd crores which is outlined for capex for the next year towards upgradation, are there any specific facilities where this investment is being done ?

^Inadvertently mentioned as INR 10,000 crore

Viren Shetty: It is very spread out across all our hospitals, the bulk of it will be spent towards Bangalore the large hospitals but the rest towards all of them, Jaipur, Ahmedabad, Jamshedpur, Delhi, those places.

Nitin: And where in Calcutta as you mentioned, how are we now looking at the capacity constraints there.

Viren Shetty: We have no choice but to look for land to buy something and set it up. The only other option otherwise was to demolish one of our old towers and rebuild it but that would cost too much disruption of the operation, so we don't want to consider that right now. We found a few land parcels and we are trying to acquire them but buying land in West Bengal is always difficult. So, it is taking time, but we are hopeful that it should happen sometime next year and we will start construction immediately.

Nitin: But there is still going to be another 2-3 years before this new facility comes up. How are we looking to handle our Kolkata capacity constraints in the meantime?

Viren Shetty: The Howrah Hospital construction is ongoing and that should get operational when that is complete it will take some of the load, RTICs will still have some challenges. We will do a little bit more realignment there and try to take over some buildings nearby on rent. We have taken over some of the apartments nearby to convert them into our purposes, move admin, redo the parking areas and so on, little things that we can do but won't change the needle too much.

Nitin: Okay. And one to Anesh sir, what is the total capex you are looking at in Cayman for this new expansion and, is it still sticking to the USD 90-100 million we talked about earlier or is it going to be lower or higher than the previous estimate ?

Anesh Shetty: Nitin, I wish it was lower. No, we are still on track for that. And the exact values have been broken in think in one of the last slides of the investor presentation converted USD into the INR, I think. So, that's the detail there, but we are still on track for the earlier guidance.

Nitin: Okay, thank you.

Anesh Shetty: There may be some slight upward revision but nothing significant.

Nitin: Thanks very much.

Nishant Singh: Thanks, Nitin. Next in line we have Mr. Prithviraj, please go ahead.

Prithviraj: Anesh, just one question on Cayman. See, given that the oncology department will be operational from Q1, do we expect margins to take a dip initially or can we sustain this 40% + EBITDA margin in Cayman?

Anesh Shetty: No, we don't expect any margin dilution because oncology so what we are adding incrementally is radio therapy which is capital intensive but from an EBITDA margin perspective very healthy and definitely accretive to what we are doing. So, we don't expect any margin dilution.

Prithviraj: Okay, thanks.

Viren Shetty: For this phase.

Anesh Shetty: Yeah, for this phase, I am talking about only the radio therapy which will be ready in a few weeks only for that phase. When a new h

ospital is commissioned that is essentially commissioning a new building , we will be duplicating a lot of our fixed costs unfortunately in completely new locations, etc. And as we have guided before there most definitely will be margin dilution at that time.

Prithviraj: Yeah, that's helpful.

Nishant Singh: Do you have any further questions, anyone? Alright, if we have no further questions, we would like to conclude our session. Thanks everyone for your active participation as always. Please do feel free to reach out to us in case of any further questions. Thank you, once again from all of us.

Call: May 2023

Date of submission: 29th May 2023

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code – 539551 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051
Scrip Code- NH

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter and year ended 31st March 2023

This is further to our earlier letter dated 23rd May 2023 regarding audio recording of Earnings Call of the Company for the quarter and year ended 31st March 2023, held on 22nd May 2023.

Please find enclosed herewith the transcript of the said Earnings Call. The same is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earning-call-transcripts>.

This is for your information and record.

Thanking you.

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q4 FY23 Earnings Conference Call"

May 22, 2023

MANAGEMENT : MR. VIREN SHETTY – EXECUTIVE VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MR. R. VENKATESH – GROUP CHIEF OPERATING OFFICER

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS
SUBSIDIARY HCCI

MR. NISHANT SINGH – VICE PRESIDENT , FINANCE , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. DURGA PRASAD – SENIOR MANAGER , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

2 Mr. Nishant Singh: Good afternoon, everyone. My name is Nishant Singh. I head the Investor Relations function at Narayana Hrudayalaya. I welcome you all to the Q4 FY23 earnings call of the company.

To discuss our performance and address all your queries today, we also have with us Mr. Viren Shetty - our Executive Vice Chairman, Dr. Emmanuel Rupert - our CEO and MD, Mr. Venkatesh – Group COO , Dr. Anesh Shetty - MD of our overseas subsidiary, HCCI, Cayman, and Durga Prasad - Senior Manager from the team. Our group CFO - Ms. Sandhya, is not available for this call due to some unavoidable reasons.

We hope you have gone through the collaterals, which have been upgraded on the stock exchanges as well as on our website. As usual, before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website as well as on the stock exchange at a later date. I would also like to remind you that everything that is being said on this call that reflects any outlook for the future or which can be construed as a forward-looking statement must be viewed in conjunction with uncertainties and the risks that they face. Post the call, should you have any further queries, please do not hesitate to get in touch with us. We would like to address them with the best of our ability.

With that now, I would like to hand over the call to Dr. Emmanuel Rupert - our CEO.

Dr. Emmanuel Rupert: Consolidated revenue for the current quarter stood at INR 12,216 million reflecting a year-on-year growth of 29.9% resulting in a consolidated fiscal revenue of INR 45,248 million at a year-on-year growth of 22.2%. NHL generated Consolidated EBITDA of INR 2,904 million in Q4 FY23 at a margin of 23.8% against 23.6% of Q3 FY23.

Our Cayman unit continues to contribute significantly to the overall performance. HCCI Q4 FY23 revenue increased to USD 29.3 million against Q3 FY23 revenue of USD 28.2 million. There are one-time income and expenses included in them and adjusted for the same. The Q4 underlying margin is 22.5% for the Group and 18.5% for India.

Our industry leading average ROCE of over 28% and our targeted CapEx strategy focusing on judicious allocation of capital continues to deliver sharp returns.

I am delighted to share with you that we recently operationalized our New Bone Marrow Transplant Wing in Health City Campus in Q4 FY23 adding to our existing capacity making it one of the largest such facilities in India and also the Asia Pacific region. This unit completed 63 Bone Marrow Transplants in Q4 taking the total Bone Marrow Transplants performed in the unit over the last 1.5 decades to more than 32,000 in India. This coupled with the advanced CAR-T clinical trials i.e. the Chimeric Antigen Receptor T-cell therapy trials in partnership with us, which is an advanced immunotherapy and precision medicine in partnership with Immuneel with their GMP facility located within our campus makes us one of the very few centers in Asia with end-to-end treatment for advanced oncology under one roof. The clinical trials which we participated in, we performed more than 20 CAR-T cell therapy out of the 24 and they were successful and 40% of them are nearing their one-year follow-up after their CAR-T cell therapy.

Our Leadership in Cardiac Care grew from strength to strength with Cardiac Hospital in Bangalore performing more than 2,2

50 Cardiac Surgeries and 5,500 Cath lab

Procedures and more than 150 minimal access cardiac surgery this quarter. Minimal access cardiac surgery is growing in numbers and they are able to perform very complex valve repair surgeries and coronary artery bypass surgeries using very small incisions of around an inch to 1.5 inches. This continues to be one of the largest cardiac centers in the world. Our Mazumdar Shaw Cancer Center performed 75 robotic Onco surgeries in Q4 and we have been able to extend the sphere of complex work that we do across all our centers. We successfully performed Gujarat's first-ever Mitra Clip Procedure for a very end-stage heart failure, repairing the Mitral Valve in very severe heart failure leading to severe Mitral Regurgitation.

The NH Ahmedabad unit also discharged Total Knee Replacement patient within 24hrs of the surgery. And this robust post-operative care is what we are trying to drive across our entire network converting many of the procedures into short stay procedures.

The NSH Howrah completed more than 200 Robotic Onco surgeries in a little over a

year.

Our newly acquired Department of Orthopedics, Spine & Trauma in Health City campus has successfully performed a complex Hand Reimplantation Surgery in the quarter. This young person had his forearm severed in an industrial accident and the team of plastic & reconstruction surgeons along with Orthopedicians painstakingly implanted the severed forearm. The unit has been progressing well with the clinical spectrum and does more than 200 procedures a month.

After implementing athmâ, our LIS – Lab Information System across our labs, we have experienced a noteworthy enhancement of 20% in the turnaround time and up to 36% in the Top 5 tests. This improvement in efficiency is across a substantial volume of over 24,000 samples per day. We can now provide our patients and doctors with precise results in a significantly shorter period.

Athmâ has successfully achieved the prestigious NABH -QCI certification. This recognition granted by the National Accreditation Board for Hospitals (NABH) in collaboration with the Quality Council of India (QCI) and the National Health Authority (NHA) recognizes athmâ as a trusted partner that upholds with the highest standards of interoperability, quality, and safety. It reinforces our commitment to providing our patients and doctors with a reliable and secure platform.

We are transforming our business to offer comprehensive healthcare services by becoming more patient oriented, digitally native and operationally efficient to offer high quality health care at an affordable cost. While continuing to consolidate our operations, we would simultaneously pursue growth opportunities both in India and overseas that derive synergies from our existing operations and maximize value for all our stakeholders.

I would like to hand over to Viren Shetty to share updates on some of our new ventures. Over to you, Viren.

Mr. Viren Shetty: Thanks, Doctor Rupert. When we started Narayana Hrudayalaya in 2000, we sought to transform health care by making high-quality care accessible to all. Nobody said it was possible at that time. But looking back, we are proud to have played a part in putting India on the path to disassociate healthcare access from affluence. At the same time, the medical research field is spending billions of dollars developing cutting edge treatments for diseases that were once considered inoperable. We shouldn't deny our people access to the latest and most expensive treatments but this

is only possible

when everyone's covered with Comprehensive Health Insurance. The IRD AI has been encouraging new entrants to develop innovative models of health insurance that cover the missing middle and NH would like to play a role in this new landscape.

Our existing health insurance system operates on a fee-for-service model. It will pay you when you get sick, does nothing for you when you're healthy and runs in the other direction if you have a preexisting disease. Large developing countries with low tax collection like ours cannot afford free universal health care on a fee for service model. So, as NH we would like to experiment with managed care where the interests of patients, payers and providers are fully aligned in an integrated manner. So, in this model care and coverage are seamlessly integrated and healthcare is delivered in a narrow network and the focus is on providing optimal levels of care and keeping your customers healthy.

This is a logical next step in our journey to create an affordable, globally benchmarked, quality driven healthcare services model. At NH, we've always believed in getting closer to patients and bringing the promise of quality, affordable healthcare to the doorstep of the country's millions.

So, to this effect, we've incorporated a new company Narayana Health Integrated Care (NHIC) and have undertaken a slump sale to transfer our existing clinic assets. The value may not be that significant, about Rs.10 crores, but we will slowly build scale, competency and domain expertise in order to be a market leader in this space.

NHIC has a strong leader in Ravi Vishwanath who joins us with 25 years of experience across HDFC Argo, Apollo Munich, Tata AIA Life and Reliance General.

These are the early days of our journey but our initial results have been very promising.

We strongly believe that an integrated care model based on whole person care, providing comprehensive coverage for all market segments is the need of the hour.

We will share updates on the progress over the coming quarters.

We now open up the floor to your questions. Thank you. I would request everyone to now use the raise hand feature to start posing the questions and we try to address them in this forum.

Mr. Nishant Singh: Thank you, Viren. I would request everyone to now use the 'raise hand feature' to start posing their questions and we will try to address them in this forum.

Yes, Nirali, you may go ahead.

Ms. Nirali Shah : Yeah . Firstly, congratulations on a good set of numbers. My first question is , could you provide an update on the EBITDA margins for Mumbai in Q 4 like in Q 3, to be precise , in January Mumbai had had come to a breakeven. So , I would like to know EBITDA margins for Mumbai in Q 4. Additionally, I'm also interested in understanding the current EBITDA margins for our relatively new hospitals in Gurugram and Dharam shila. And have we achieved a double digit margin in these two locations ? I understand that Mumbai being a new one and we have just come across breakeven , so double digit is not possible for Mumbai but the other two, have we reached there? Have we achieved it?

Mr. R. Venkatesh : Yeah, I will take this call . I am Venkatesh . So, your question on Mumbai.

Ms. Nirali Shah: Yes, Mumbai.

6 Mr. R. Venkatesh: Yeah, the consolidated Q 4 EBITDA for Mumbai is around INR 6 million which is around 2.3% in terms of EBITDA . This is the first time it has shown a positive EBITDA for the year at INR 6 million. If you look

at the previous quarters, it was on a negative but gradually what has happened is the negativity has come down gradually and this is the fourth quarter where we've shown a positive EBITDA which shows positive direction in which we are moving and we hope to maintain this momentum for FY24 as well. Now, in terms of Gurugram, the Q4 EBITDA margin is approximately at 2% on a base revenue of INR 35 crores . On the whole year, it has generated a revenue of around INR 133 crores with INR 3.7 crores EBITDA at around 3% margin . When it comes to Dharamshila , the performances have been robust over the year . Our EBITDA margin is at around 16% at INR 8.5 crores on a revenue basis of INR 54 crores and for the full year the EBITDA margin is approximately 14% on INR 29 crores at a revenue base of INR 209 crores .

This is the status of all our New Hospitals at this stage.

Ms. Nirali Shah: Okay . So, ideally currently we are at a double digit only in Dharamshila, right?

Mr. R. Venkatesh: That's right. See all the new hospitals are in positive. Now, the most positive thing if you look at in this quarter is all the new hospitals are in positive at 8.6% . Dharamshila is double digit margin and we are confident that the other two will also reach the following figures in the next 3 -5 years. But , of course, the stage is already set where we have started showing positive for all these new hospitals.

Ms. Nirali Shah: So, we can expect a healthy margin of 15 %-17% in the near term perspective of say five years and this will be sustainable, right? Okay and my next question is about the Cayman Islands . Is the new Oncology Block in the Cayman Islands on track that was to commence operation as planned in the first quarter of FY 24 ?

Dr. Anesh Shetty: Yeah. Hi, Nirali . Anesh here. Thank you for your question. So , yes, the initial guidance we had given was Q 1. We expected it to happen towards the end of April. However, we had some last minute glitches with the construction. We've actually started the operations about eight days ago and we've been treating the first batch of patients as we speak. In fact, just before the call started we were just playing a short video of the Radiotherapy Center for everyone's benefit.

Ms. Nirali Shah: Okay.

Mr. Nishant Singh: Thank you, Nirali. Next, we have Dhara.

7 Ms. Dhara Patwa: Yeah , thanks for the opportunity. So, I had two set of questions. One was for the ARPOB growth. So , this quarter we have seen ARPOB growth of 10% year over year and 5.5% sequentially. So, could you provide the breakup in terms of price hike and peer and specialty mix ?

Mr. Viren Shetty: Nishant, do you want to take this?

Mr. Nishant Singh: Yeah, Dhara. See, our ARPOB numbers have gone up to INR 1.35 crores per bed . This number in a time spread will continue to grow but we will not expect this to grow very fast because our focus is not to have this revenue per bed at a very high level , at a very fast speed because we want to retain that affordable for all policy and hence we will see this growth but will not be the best in the industry. We will go steadily up but we don't have a clear guidance on this number.

Ms. Dhara Patwa: Okay . So, this 10% YoY growth like what we have achieved in this quarter , so how much was the price hike we have taken in the quarter? Because I understand C GHS has also increased their price and since we have 20% mix from the skin patients, so I guess there could be some element of price hike in the quarter.

Mr. Viren Shetty: This is Viren here. This won't be due to price hike because price hike is only taken once a year. This will be mostly due to change in the patient profile, the case mix , improving the throughput, reducing the occupancy. Those are things that drive. So, it's essentially the performance improvement that drive it rather than

than the increasing prices.

Furthermore on the price, it's not that everyone pays the price what we put as a rack rate. A significant amount of discounts are offered to the patients. Similarly, institutional payors like government do not renegotiate their contracts and even private insurance companies negotiate only in a three year cycle.

Ms. Dhara Patwa: Oh, okay. That's it from my side.

Mr. Viren Shetty: Thank you.

Mr. Nishant Singh: Next we have Yash.

Mr. Yash Tanna: Good afternoon and congratulations team on good set of numbers. Can you highlight a little bit more on the new subsidiary that you opened and you mentioned in the initial remarks? What is this about and how will this grow for us?

Mr. Viren Shetty: So, as highlighted, we want to get into creating a model for integrated care. So, essentially this will be housing all our clinics right now. We will be offering primary care services in these clinics. We will be being closer to where the patients are staying and so in the periphery of our hospitals and close to the apartment societies, we will build up all these clinics. In the clinics, we will offer a combination of services both the services within the clinic as well as remote services such as homecare, online care and long term care plans.

Eventually, we will combine this with a comprehensive health insurance plan which would be able to cover not just the hospitalization in a narrow network but also the outpatient expenses within the clinic network that we have created.

Mr. Yash Tanna: Oh! All right, got it. So, the revenue generating model will be more on a subscription basis or something like that?

Mr. Viren Shetty: It will be a combination of both things. It will be a combination of subscriptions that we sell as well as the medicines, lab test, consulting fees, any diagnostics that happen in the clinic.

Mr. Yash Tanna: All right, got it. And how different will the unit economics be for this sort of a business versus setting up a hospital? Is it more a ROC affiliative or any calculations that we have done on that?

Mr. Viren Shetty: So, Capex wise these things are very light. They breakeven pretty fast. They may not be as high in EBITDA as the hospital business because the realizations are quite low and they can be spun up and spun down relatively quickly. So, it won't have too much of an impact on the overall consolidated ROC given that the quantum of investment at this stage as we are envisioning is not more than INR 50 crores. So, it won't have that much of an impact and we're doing it in a measured way. So, we won't plan on losing much money in running this clinic network but we want to test it out. We're trying it out in Bangalore. We'll eventually roll it out to Calcutta and see if people are more appreciative of this model of healthcare. So, where we have a thesis that a lot of healthcare should be delivered outside of hospital, we are yet to prove out that patients prefer this over coming to the hospital. Similarly, in anticipation of us offering these subscription plans whether they're fully compliant and whether there's something that has a lot of market acceptance. So, we're trying it out. We're very confident. The initial results are very good and we will start showing, you know, what we've been able to do from Q1 of this financial year onwards. But at this point, let's just say that it's not something you need to bake into your model. It's something we're also experimenting with and constantly changing it.

Mr. Yash Tanna: Right. This is very interesting, Viren, thanks for that. Just a question on the margins; India Business. So, as I see in the presentation, it's gone up from what 14% to 19% 9 year-on-year. So, now since our new hospitals are turning profitable and we are saying that we will maintain the momentum, what sort of peak margin potential does the India business have going forward?

Mr. Viren Shetty: Yeah. We won't really comment on this but, Venkatesh, if we can just give some color on generally how things stand in the short term.

Mr. R. Venkatesh: Yeah. Currently, when it comes to Q4 for India, also, we've done around 19.1%.

Having said that, we believe there are headwinds against these numbers and therefore it may taper a bit. One is that, you know, this year there has been windfall growth in revenue whereas in the coming years we can see more of an organic growth. Secondly, there are also significant cost headwinds which only partially we've been able to pass on to the patients; we are not going to pass on the entire thing. Of course, we will try and maintain the same margins of growth through the throughputs which we've always spoken about, efficiency improvement and cost rationalization which we have been doing since the last few years and we'll continue to do that and make sure that we maintain the margin.

Mr. Yash Tanna: All right, got it. Thank you and I'll get back in queue and best of luck.

Mr. Nishant Singh: Can we have the next question from Dheeresh?

Mr. Dheeresh Pathak: Yeah, hi. Thank you for the opportunity. For the India Hospital assets, I want to

understand the capacity that is available. So, I think the metric that you prefer is the IP discharges and not the occupancy. So, from that KPI point of view, if you can just help me understand how much capacity is there ; Indian Hospital Business in aggregate?

Mr. Viren Shetty: I'll start and then I'll ask Dr. Rupert to join in. The thing is , it's not something where you are constrained in terms . So, there is a space constraint for a lot of our hospitals but the throughput can be increased relative to how much more investment you're willing to put into these buildings. So , the discharges by moving to things like daycare, for example, we've been able to improve , by changing the bed configuration we've been able to increase the realization , by investing in certain things like certain processes that discharge patients faster in the day we can fill up beds much better. But in terms of, you know, what they ultimately will be able to stop at , this is more of a moving target.

Mr. Dheeresh Pathak: That's fine but, I mean, is there a quantification to it because if you've seen at FY20 you had about 286,000 discharges , this full year you had 229 ,000 which is still below what you did some years back . And obviously compared to the beds and all , the number looks very low. I know that's not the right metric. So , how do we sort of 10 get some sense in terms of where we are at least in the flagship assets, you know, Bangalore and the Calcutta one ? Because , see, if you see a lot of your revenue growth and a lot of the EBITDA , so 80% of the India hospital EBITDA is itself is clustered . So, at least from those cluster point of view, if you can just help me understand how much capacity is available ?

Mr. Viren Shetty: See, just off the top of my head, there is adequate capacity within all the hospitals we have, we just need to reconfigure to meet the requirements. As for us not reaching the pre-COVID numbers, there was a calculation error because back before when we were on the old system we were not making a distinction between the daycare discharges which don't end up as occupancy and the ones that do and the emergency visits also would not get counted whereas since we've reclassified all of that the absolute number of patients we're discharging has gone up and this is the highest it's ever been but can go higher.

If you would ask me exactly how much we can do , it depends on how much more of internal reconfiguration we are able to do for a lot of the hospitals that we have. We may not be able to double in the existing network that we have because that will require significant investment and a little bit of brownfi

eld expansion, which we are doing also, but these two things will go in parallel.

Mr. Dheeresh Pathak: Okay . On slide 14 which talks about CapEx, so this INR 150 crores as greenfield inorganic for FY24, apart from the clinic's part is there something specific to the hospital business that you've identified ? Any new land parcel? And new city ? This INR 150 crores represents what?

Mr. Viren Shetty: Venkatesh can take this up.

Mr. R. Venkatesh: Yes. So, when we were speaking about CapEx, of course, we are looking at replacement s, capacity building , capability building . So, INR 150 crores, of course , is on plans to require a land parcel in Calcutta ; for our plan in terms of Greenfield expansions . Calcutta; we've already spoken about expansions planned up for Calcutta and Bangalore. So, this is the first initiative or a step forward towards the Greenfield expansion to getting the land parcel in Calcutta has been factored in this INR 150 crores .

Mr. Dheeresh Pathak: Okay . Thank you for taking my questions.

Mr. Nishant Singh: Can we please have the next question from Prithvi?

Mr. Prithvi Earle: Viren, this question is on your CapEx . So, obviously this year the cash flow generation 11 has been very strong . Given this context for next year INR 1100 crores CapEx, can we assume that most of that can be funded by internal accrual and the debt number would be much lower than what we were guiding earlier?

Mr. Nishant Singh: Yeah. So , this is Nishant here. I'll take this question . See, we've added around INR 330 crores cash , net cash , in this year which has been very positive . And our plans for the next year in the CapEx of around INR 1,100 crores which we have highlighted , around INR 700 crores will be through the bank funding and the rest will be through our own internal accruals , which will take our Net Debt to around 0.6 2 of the overall EBITDA which is still very comfortable .

In terms of why we are not using the entire cash is because we have a lot of cash also at Cayman where we still continue to explore opportunities in the other islands and for India the cash balance is at around INR 200 crores plus. So , we'll have a judicious mix of both bank borrowing and the accruals in India to fund expansion in India. While the Cayman cash will stay there while we also add but we will also continue to look for opportunities at other islands around Cayman .

Mr. Viren Shetty : Provided we don't do any more expansion and acquisition and greenfield things. So ,

this is what we're giving the guidance for the next year and there is a lot . There's a huge opportunity that is presented to us in all the hospital that we run. So , it's something that we want to keep in mind. So , as it looks right now, there's a lot of work that can be done that we believe will really improve our realizations going forward and we'd like to continue that .

Mr. Prithvi Earle: And this question on Cayman 's new Oncology Department, so has there been an y markets or something of that sort you have done to just get a sense on number of people traveling to U.S. and, you know, what can be the potential from this block?

Dr. Anesh Shetty: Prithvi, s orry, could you repeat that question? This is Anesh here.

Mr. Prithvi Earle: On the new Oncology Block of the radiation , has there been any market survey that you people have done to get a sense on number of people traveling from Cayman to U.S. currently for this and then you know, who can now visit your blo ck in Ca yman ?

Dr. Anesh Shetty: Sure. Thank you for the question. So, because the service we are primarily offering is radiotherapy, which is in general about a 6 -8 week s treatment regimen. What was earlier happening before our facility was up and running was tha t patients had to fly 95% mostly to Miami to some of the Centr es over there and they would relocate there for a month to two months or

more with their family and they would not be working, not be earning , significant logistics and inconvenience costs as we ll as the, you know, 12 high prices in that market that the insurers had to bear. So , what is happening is because we are offering a similar comparable treatment modality with the same machine , you know, treatment protocols more or less in line with what they would experience in Miami but with the convenience of having it down the road in a very convenient location at home for them where we don't foresee much resistance to converting people who need the service to use us because , A, we are at home and , B, we are the only linear accelerator on island.

To your second question about the market size in terms of number of patients etcetera , we have, you know, gathered some of this information through some payers which share d this publicly but others is, you kno w, collating various other sources of information. So , it's still an uncertain number that we continue to work with but there is a significant margin of safety built in when we planned this facility. So , we continue to remain very comfortable with the inve stment and its potential returns.

Dr. Emmanuel Rupert: And also we have a fair idea. We've been treating patients on medical oncology and surgical oncology and if you can extrapolate the clinical numbers from them, we have some uh fair idea as to the numbe r of patients who will be requiring radiation from that.

Mr. Prithvi Earle: And can you give the Cayman EBITDA number for this quarter ; absolute EBITDA in million dollars ?

Mr. Nishant Singh : So, it can be derived fro m, you know, the statements, the differe nce between the consolidated and the standalone more or less but we generally don't disclose the specific numbers for that business unit.

Mr. Prithvi Earle: Okay . So, just final question. So , if that's the case even after the radiation Oncology Department can we maintain 40% plus EBITDA margin in Cayman ?

Mr. Nishant Singh : So, the Radiation On cology , we can confidently say will not be margin dilutive. So, this is a facility, you know, in any hospital , radiation, if you look from an EBITDA perspect ive is actually a very, very high EBITDA percentage business simply because the investment is significant in CapEx but not in operating expenses. So , definitely there will not be any margin dilution on a ccount of radiotherapy at this stage. Having said th at, as we've discussed several times before as well , in Q1 of next financial year when we commission the larger hospital, which is the Camana Bay Hospital, which will also have cancer treatment among other specialties , at that time there will be a margin d ilution simply because we will be bringing online a large chunk of fixed costs but for now with the radiotherapy Center there will be no margin 13 dilution.

Mr. Prithvi Earle: Thank you.

Mr. Nishant Singh: We have the next question from Mr. Dheeresh.

Mr. Dhee resh Pathak: Sorry, my question got answered. I 'll go at the queue.

Mr. Nishant Singh: Okay, thank you.

Mr. Viren Shetty: So, anyone else ?

Mr. Nishant Singh: Yes, Mr. Gagan. please go ahead with the question.

Mr. Gagan Thareja: Yeah, good afternoon. I hope I'm audible.

Mr. Nishant Singh: Yeah.

Mr. Gagan Thareja: Yeah . Sir, at the start of the call you mentioned , you know, there was a onetime contribution to the E BITDA for the quarter , if you could enumerate this contribution and also explain to what it pertains.

Mr. Nishant Singh : Yeah. So , we have also spelled it out in the deck. There's INR 28 crores EBITDA

contribution from Saint Lucia which is part of India unit but there's also a lot of onetime expenses as well because of our brownfield expansion /transformation projects across India units. So, if you net off this one time gain from Saint Lucia and the onetime expenses which we incurred in this quarter, t

he net effect is minimal .

Mr. Gagan Thareja: Okay. And the Oncology Center at Cayman, if you could enumerate the fixed cost associated ... Sorry, in terms of OpEx, the cost associated with that facility . I understand that, you know, in an aggregate over the year or thereafter, you know, it might reflect its optimal margins but as it scales up the margin profile would be different. Therefore, just to understand, you know, what could be the incremental fixed cost associated with this one.

Mr. Viren Shetty: Gagan, we run that Radiotherapy Center similar to how any hospital would or how we do it in India as well. You know, in terms of fixed costs, aside from the manpower cost to run the facility, which is not much because there are few people , expensive but few people , and the infrastructure cost which is the facility costs in terms of the utilities that fit out et cetera, the ongoing cost for those things . There isn't much of a fixed cost compared to the traditional, you know, your regular medicine, surgery, cardiology, orthopedic services lines. While we are not in a position to disclose specific numbers in terms of absolute value but it's not much as long as we stick to Radiotherapy.

14 Mr. Gagan Thareja: And in terms of utilization, you know, obviously this is not going to be like on a bed count basis but any idea you could give us as to what sort of average utilization you could see for the first year of operations and when could it reach a mature optimal sort of utilization?

Mr. Viren Shetty: Yeah, Gagan. So, you know, I'll start and Dr. Rupert can continue . When we look at mature optimal utilization for a linear accelerator of the kind that we have, you know, this is a true beam facility. Some of our other centers in India are treating in the range of, Dr. Rupert said, 70-80 patients .

Dr. Emmanuel Rupert: Yeah, 70 -80 patients .

Mr. Viren Shetty: 70-80 patients a day in this kind of a machine. Now, when we planned this Centre, we obviously knew that those kind of volumes will never happen but unfortunately, you know, we can't buy half a linear accelerator. So , we understand that the volumes will be significantly less . However, given the exponentially higher realization compared to what we see in our home market in India, as mentioned before, we're very comfortable with our returns from this investment, accounting for a very healthy margin of safety.

Mr. Gagan Thareja: And this could be achieved by when ? I mean, I understand, you know, these are estimates but just to understand given your assessment of the market and the fact that you are the only one on the island , at least so far , do you see this sort of hitting that mature utilization fairly soon ? Maybe a year or 18 months or is it too optimistic?

Mr. Viren Shetty: No, so there will be two phases to the utilization , Gagan. So, the first phase is the local business. So , local business, you know, like we elaborated in the previous question , simply because of the value proposition of convenience and logistics costs and the overall, you know, benefits of being treated at home , the local on island business will ramp up . We don't expect to have much of a delay beyond a couple of quarters to get all the local business that needs radiotherapy unless there are some patients who for a very, very specific reason will still travel to the U .S. and they'll do so no matter what. So, that's phase one, which will happen fairly soon. But what we're really counting on in the longer term, which is a little much more uncertain, is the demand for radiotherapy services from the broader Caribbean region, which is in line with our overall medical tourism play for all our services. So , if you look at the region as well although there are a few handful of Linux, you know, all over in a few markets, they're not really run well, they're not commissioned , in many cases they have maintenance problem

s, they don't have manpower shortages. So , in terms of having a modern true 15 beam line linear accelerator , which is 24/7 available online with comprehensive medical, surgical, oncology, and an entire hospital setting similar to what you get in Miami this is definitely a one -of-a-kind facility in the region, but selling this and you know to other markets to payers there will we'll take some time. You know we obviously won't be in a position to answer even we ourselves don't know how many quarters it will take, but I think that will be the second phase of volume expansion where after the local business is done, the overseas medical tourism will start kicking in.

Mr. Gagan Thareja: Is there a cost proposition for that because you know as you said people from Cayman go to Miami, so probably you know from the other islands they as of today go to Miami, is the travel cost or inconvenience and you know the treatment cost

differential substantial enough for people from the other islands to shift to this?

Mr. Viren Shetty: Yeah, that's actually a good question. So, when we thought about this and did some research into this, what we realized is since most patients who will be going to the US are coming - potentially coming to us and came in from other islands. There are basically two categories; there would be the insured patients and they would be the self-pay patients. So, if they're self-pay, there is definitely a cost advantage to coming to us, it does get slightly dampened by the although for the clinical service you would spend a lot less than you would spend in the US, but staying in Cayman, which is one of the most expensive jurisdictions in the world actually to live in you know it does dampen some of that compared to you know some of the East Coast - US East Coast markets, but there still is a value proposition, however, the larger market which is the insured patients you know the patients are insulated from the treatment costs largely. They feel that they've paid their premiums, so they're entitled to whatever the maximum benefits are offered and they feel that you know why wouldn't I go to you know a world class center in the US, why would I go to a new place in Cayman. So, there is that's selling to the payer, selling to the one who's paying in some many cases the governments as well that we'll have to work through, but we've been doing this for other specialties for some time now, so it's no different just because it's radiotherapy. So, there is a value proposition, there is a cost saving, but it's a question of communicating it to the person who is able to influence the direction the patient moves in that can take some time.

Mr. Gagan Thareja: In terms of realization you know you indicated that while volumes would be lower as compared to what you've seen in India, but realizations are substantially higher, any numbers we could work with in terms of realization per patient or ARPOB, if one could boil it down of that? So that was a question on this one if there's no data possible on that, I have two more questions. One you know the second unit that came in whenever it comes on stream you indicate that it'll come with substantial fixed cost and therefore initially you know have some sort of an impact on your margins, point well taken, but I presume that one of the reasons for doing that facility was that you know daycare patients I think you indicated in some of the previous calls don't want to come to the current facility because it's from a travel perspective not the most optimal location and therefore you want to you know to capture that bit you want to do something in the heart of the city. If that is the reason or the focus for that facility, then ideally the fixed cost there should be lower than you know a full-fledged inpatient sort of a facility, so just trying to understand how does the fixed cost on that facility compare with your

existing facility and also you know the proposed bed count there?

Mr. Viren Shetty: Yeah. So, the first question on the fixed cost and should it be lesser so, you know you are right in that you know the distance is a significant barrier to many patients even though it's just a 40 minute drive from the City Center to where we are, but in that market that's perceived to be a lot. So, the primary reason for this hospital was a location advantage and you know like we mentioned before when we decided to solve it, we decided to solve it with the best possible land. So, the land parcel we own, the location we have is the most premier land in the country and it's just at the roundabout by which half the country passes by every day. So, we've definitely neutralized that location disadvantage. Having said that because we've neutralized that we obviously would be for better more attractive to patients who require daycare surgeries, but also just you know outpatient visits, diagnostics, even some light inpatient work as well whatever the reason may be, so the way we are thinking of this is to make maximum utilization of our facility and our investment, we will not be restricting any services, so essentially whatever we perform in the main campus in East End will also be performed in Camana Bay because it doesn't make sense to not offer it when patients have a need for it especially since we have the most expensive fixed cost which is the human resources, the doctors already in the same island. So, to your question the difference in fixed cost, yes it will be lower than what we currently have, but that's only because we will not be hiring the second or third cardiac surgeon or the second or third neurosurgeon etc. You know a very large chunk of our costs are our clinical manpower and at the clinician level, at the doctor level they all have a good amount of redundancy or excess capacity to still be unlocked, so we will be using that so because of that you know the fixed cost of the facility will be lower. Added to that, the main campus is about 110,000 square foot facility whereas what we're building is about 64,000 square feet facility. So, to that extent you know the facilities cost will be proportionately lesser than that and I think your last question was on the bed count of that building. I think we've mentioned it before. We'll have inpatient beds, we'll have emergency, we'll have chemotherapy, we'll have a large obstetrics neonatal presence

as well. I think I don't want to give a number that's materially different from what we've earlier disclosed, but Nishant if we could follow up with that, I think what we said was in the 50 to 54 beds I think, so it's about 50 to 54 beds here.

Mr. Gagan Thareja: Right and just to follow up on that, when this facility come in would it mean that you know some of the revenue that you're currently generating in your existing facility would get sort of in a way cannibalized and shifted to the new one possibly to start with and then you know as the cases ramp up here that effect might go away and also final one you mentioned at the start of the call that you foresee some headwinds on the margins and there was some windfall gains if you could elaborate there as well? Thanks.

Mr. Viren Shetty: Yeah, absolutely. So, to your first question and I'll hand it back to Nishant for your second question because I think that was a comment on NH India, the margins there, yeah but to the first question, yes so there will absolutely be a sort of transfer, you know cannibalization is a harsh word, but yes a transfer of patients and revenue from one facility to another, but at the same time there will also be a transfer or sharing of costs you know like I mentioned we won't be hiring any incremental patient facing clinicians to do this which is a very significant fixed cost because they all have you know spare capacity and you k

now the same goes for other senior resources as well.

So, yes you are absolutely right that in the initial ramp up phase there will be a large amount of you know transfer of business revenue and cost until you know things settle in and then we will see you through incremental gain of revenue, hopefully without any incremental gain in costs and that's what we're really hoping for. So, back to – Gagan if that answers your question, can we move pass it along to Nishant.

Mr. Gagan Thareja: Yeah, sure. Thanks.

Mr. Viren Shetty: Thanks. This was a question, Nishant on the one time, the headwinds on the margin?

Mr. Nishant Singh: Yeah, I think this was already answered earlier?

Mr. Gagan Thareja: No, sorry to interrupt - this is not for the one time. I think there was a comment made that for FY24 you see some margin headwinds

Mr. Nishant Singh: Oh, sure, Venkatesh, yeah.

18 Mr. Gagan Thareja: Yeah, yeah, and

Mr. R. Venkatesh : I'll summarize that, so the margin headwinds mostly around the fact we're entering an election year and so generally what happens is if you're exposed to a lot of government business, you'll be a little cautious because of the growing receivables. The other is the usual global macro scenarios what we expect with the war and so on and the deteriorating currency situation in a lot of the Asian countries like Bangladesh, Nepal, lot of places that traditionally send patients around ; other than that there was a low base effect that led to a large gain between last year and this year, whereas now of a high base it may not be easy to replicate the same sort of revenue growth in this year and lastly a lot of the work we are still doing, a lot of the room reconfiguration, doing up the Cath labs, adding bone marrow transplant units or robots doing up the Cath lab OT all of that is still very much under progress and will be for the next three to four years. So, as and when these units are being built, it does lead to a lot of loss in productivity because of the areas you are shutting down. So, those are the headwinds that we expect over this year which is why we'd be a little muted in our optimism for being able to continue growing at the very high pace.

Mr. Gagan Thareja: Yeah one of your peers indicated that CGHS rates partially have been revised and expected to revise you know for the packages as well and by I think a reasonably substantial amount since it's happening after a gap would that not in some way help you, I mean I'm trying to understand your CGHS exposure here and secondly how has been the international patient flow for you and is the momentum there growing for you or you believe that in the coming year it might temper down?

Mr. Viren Shetty : I'll address the CHS and Venkatesh will address the Bangladesh. CGHS, they had announced a revision in the rates for a lot of outpatient consultation, so ₹100 they were paying a doctor, now they're going to pay ₹150. This is a doctor who charges ₹700 to ₹3,000 for a consultation. So, the enthusiasm that would have would be adjusted accordingly for this patient, similarly the daily room rate went from ₹1,000 to ₹3,000 on rooms that cost ₹15,000 for patients paying cash. So, similarly everyone's enthusiasm on the increase in the prices would be temporary. Now, they have said that some rate increase would happen over the CGHS rates we're to expect it in July. We don't know what that's going to be, we can speculate and obviously our fingers across that it will be substantial, but then again no one never knows with this thing, whether you're talking about 5% -10%, 50% -70% price increase. This is a rate that hasn't been revised since 2014 and if I'm a payer, why let a good thing you know why not let's let it continue going on. Venkatesh will address the Bangladesh

19 Mr. R. Venkatesh: What I was trying to say is that post the migration into from NHL to CGHS affiliated

claims have

also not been processed, there are significant outstandings. So, there has also been a delay to clear the bills, so we also need to keep a watch on how effectively the payments also happen along with trying to see what values the rate revisions will give us. So, that's what I wanted to say as an add on. Now, when we come into international patients, international patient volumes have been improving gradually from 6.5% in Q1 FY23 to about 8.5% in Q4. We're focusing on more market of Bangladesh, but with more of a direct outreach, but still what we feel is that we may not - we don't believe that we will still reach our pre-COVID numbers in near term because one is we have cut all the referral arrangements and we are getting directly into the market, we are also restricting our activities to pure key markets in direct engagement model and our strategy also would be to shift the marketing expense into digital and domestic activities which actually will be less impacted by travel disruption if anything happens in future, so it's a backup or a standby arrangement so that the flow of patients have no it's continues to be a seamless flow of patients irrespective whether we have an international sector opened or not, but the focus is Bangladesh patients keep coming, international patients keep flowing in, but we would of course as I say restrict the early key markets and focus more on digital and domestic activities.

Mr. Gagan Thareja: Sir, just one final comment I mean the three hospitals that you earlier talked about Gurugram, Bombay, and Dharamshila, they are on their way to relatively better margins as their utilizations improve your international patient flow you know maintains momentum, these are positive for you, and yet you're talking about net-net there being headwinds on margins, I am unable to reconcile if you could elaborate a little more?

Mr. R. Venkatesh: Headwinds is something which is a little different in terms of the cost aspect we're talking about and also these are key headwinds we were talking about is on the major units which we have already spoken about in terms of transmission activities, terms of the elections, and also certain cost increase which is there. We talk about the newer hospitals which is Gurugram, Dharamshila, and Mumbai, of course there is a - when we are looking at international marketing and getting into this on a direct engagement model there is a good scope for these units also, but at the same time the alternate in terms of digital and domestic and neighborhood activities is just as an add-on in terms of trying to see how we can add more volumes into the system and also as a standby just in case of any unforeseen circumstances come up in future just like what we've experienced in the earlier a couple of years. So, this is this is an alternate while we 20 keep focused on international marketing, we also need to make sure that we have alternate sources of patients for generation like this.

Mr. Gagan Thareja: Right. Thanks, and what would be the cost for debt for you, the 700 crores that you intend to raise?

Mr. R. Venkatesh: Sorry, can you please

Mr. Gagan Thareja: Cost to debt.

Mr. Nishant Singh: See it varies, now it's actually high for everyone so it should be in the range of 8% to 8.5%.

Mr. Gagan Thareja: Thank you.

Mr. R. Venkatesh: And we also have to include this tax on that so effectively should be around 6½%.

Mr. Nishant Singh: Sure. We can move on to Nitin.

Mr. Nitin Agarwal: Hi, thanks. I have two things, one is A - on the India business with all the CapEx that you've outlined can you give us a rough sense on a number of incremental beds you looking to add across various locations over the say next two to three years?

Mr. R. Venkatesh: The next one year, no incremental beds. Over the next two to three years, we'll start disclosing it when we put up the CapEx list for FY25, but right now a

lot of the things

that we're doing now is more on OPD Plazas and parking areas, internal refurbishment, and in fact some of our bed count may actually come down because a lot of that is going to existing hospitals. Some will be operational like Howrah for example we're adding more beds, but the net effect is not significant. FY25 onwards, there may be bed addition, but we'll start putting that out from the Q1 slides onwards.

Mr. Nitin Agarwal: And your Calcutta hospital was running to capacity the RTIICS Hospital, how do you sort of manage to work around that for now?

Mr. R. Venkatesh: As we have been always saying about throughput occupancy or capacity you can't view it in isolation especially in high throughput centers, even Calcutta is also one of the high throughput centers, we focus on very high throughputs perform cardiac surgeries or robotic procedures which has a morning-evening admission discharge. We're also working on efficiencies and improving discharge time especially in insurance and scheme discharges happening, we are trying to make sure that we get it done before

12, so that there is more admissions happening during the day, we're also trying to improve the throughput in the ICU's, trying to change the bed mix. We're also developing efficient communication tools between the doctors and nurses to coordinate care and discharge patients faster and we've said we are significantly investing in technology to improve our throughput, faster discharges, lab results, seamless appointments, and also increasing the throughput through process transformation, and also working on the ALOS coming down, we've got some targets and we're confident of achieving it in the next eight quarters. If the space we saved with all these measures, we try to focus on infrastructure enhancement like to address these patient bottlenecks like OTs, adding OTs or adding ICUs, Diagnostics, Lab Billing, which will you know enable us to increase revenues at the same costs and same capacity really come with Greenfield expansions.

Dr. Emmanuel Rupert: Yeah, Dr. Rupert here. We've been working with the clinical teams across the network and part of the thing is to change clinical pathways so that we are able to do the same procedure in a much more shorter period of time and get the same predictable outcomes as well and this is what we've been doing apart from all the things Venkatesh has mentioned. So, this is yielded very good you know results. If you see the cardiac hospital with less capacity in terms of actual beds and other things, we have been able to do more work, so somewhere around the pre-pandemic we used to do around 500 cardiac surgical procedures, we are able to do around 770 that was the number in the month of March. So, we have been able to do much more with the same number of beds and all because of many things which all work In Sync to enable us to do these things and this is being replicated across the entire network as far as the clinical work is, whether we have beds or not we are just working heavily on the process transformations as well as the clinical pathways to enable the procedures to become - for them the patients to use as less time as possible in the hospital.

Mr. Nitin Agarwal: And secondly on the Sparsh Hospital because it has been about six months since we integrated or merged at closer transaction, so any updates on our experience with that?

Dr. Emmanuel Rupert: Yeah, it's been going on track as per our expectations. We are continuously doing more than 200 procedures per month and we have a subspecialty work in orthopedics and spine in that hospital. So, we have specialists who do only joint replacements, some who do only sports medicine, some who do only spine, some do the pediatric orthopedics and things like that, but it's been doing very well and because of the neuroscience department which is very well established here, our work on

the traumatic brain injuries as also been excellent results and things have been moving in the right direction and we are also investing in the new age technologies as far as the robotic programs for both the joints as well as for the spine, our equipments are expected to land in sometime in the next quarter and we should be in a position to take this in a much in the right direction as what we want to do.

Mr. Nitin Agarwal: And when you just to sort of sum it up across the various measures you guys are taking to not really with the focus not being on adding specific amount of beds, but across increasing the you know the clinical complexity and the overall patient experience from an output perspective, I meant where is it reflecting, it reflects in higher EBITDA margins for us, higher ARFOBs or what are the outcomes that one that you really working towards out here?

Dr. Emmanuel Rupert: All of the above men, on the higher utilization, higher OC, higher ARFOB. See, the thing is we can run around the country blindly chasing beds, making expensive acquisitions provided we run out of pennies that are found under the couch and that's the easiest sort of you know gains that you can make because it's sitting right there you don't need to look too far, you don't need to run around the whole countryside to find it, and there is so much about our business that can be improved which is you know patients simply sitting around waiting hours and hours for a lab test. We implemented the Atma Lab System, we were able to get the lab results in the time it took for them to walk from the lab to get there - this one the bills collected, the results had already come. So, this kind of throughput is phenomenal. No other hospital in the country is able to achieve that and that it's not - it sounds easy to do, it's not just a question of putting money and put one software, everyone had to work together. We had to put one very expensive auto track system, we had to work with all the lab people to do auto certification and work with the companies to integrate their software into our Atma, we had to iron out all the kinks, we had to get the doctors all lined up to say that you know these are the tests that will be done with a very low tab, so it's a lot of hard work, but when it's done it makes you so much more efficient, improves the patient experience so much, reduces your cost, and allows us to be more competitive in a field which as you know there's it's not always that you can keep on

raising your prices year after year and significant challenges coming in the future where you have you know a lot of people and maybe rightly so saying that there are things that the healthcare cost should not keep rising up so much, there should be - there may be things coming to cap the price a lot of services which we're worried about and so a lot of things we're doing to address is the you know the fixed cost and we don't want to take for granted. Now, I'm not saying we're going to give up on adding beds, of course we want to add beds, why wouldn't we? We want to go all over the country, but going all over the country with the way in which we operate 23 makes us extremely inefficient, has a very low return on capital, and it's not the best way to deploy our capital. So, a lot of money in the way which we operate become the most efficient then we can run around the whole countryside buying up hospitals.

Mr. Nitin Agarwal: And the last one on the St. Lucia income that we booked for the year, what is the nature of this contract and how should we sort of look at this share of income on a going forward basis?

Dr. Anesh Shetty: Yeah, Nitin, Anesh here, I'll take that. So, essentially you know when we continue to explore opportunities in the Caribbean region outside Cayman, what we are very clear on is that we will not put our money or our boots firmly on the ground as a first step unless there

is a very, very compelling reason to do so. So, we will explore these markets in various ways. Now, given our almost decade of brand equity in the region and the fact that we've been able to demonstrate a very robust track record, our governments, private players, and you know everybody in between are actively keen and you know very actively pursue us to try and see if we can replicate what we've done in Cayman in those markets. We'd love to do that, the challenge is we need to understand the economics of the healthcare business on these islands, the certainty of rule of law, repatriation of profits, the general business environment etc. and of course the local medical conditions and the doctor's lobby which can always be a problem, so St. Lucia was one of the early market - the earliest markets outside came in which we explored and the way we explored it is essentially through a consultancy contract for the government of Saint Lucia to help them Commission their National Hospital and the intention was to you know help them do this and have an option to be the operator if we so desired. We did that, the contract is over, we've completed our work, and we've taken our call that you know the market is not favorable for us to make an investment. So, we no longer participate in that market, but the contract is terminated and the revenue has been booked and it was a onetime consultancy contract.

Mr. Nitin Agarwal: Oh. So, there is no more incremental cash flows from the St. Lucia business?

Dr. Anesh Shetty: Yeah not from St. Lucia.

Mr. Nitin Agarwal: And are there any other such contracts which could be meaningful I mean not meaningful, but which you're pursuing which can start accruing to our earnings on a going forward basis?

Dr. Anesh Shetty: Yes, there are a few that we are in discussions with. We will keep you updated as and when we are close to finalizing them.

24 Mr. Nitin Agarwal: Thank you.

Mr. Nishant Singh: Thanks, Nitin. Can you move to Dheeresh?

Mr. Dheeresh Pathak: Yeah, thank you again. So, just to understand this better, so the IP realization per patient in India last two years it's up only 9%, not sure if my numbers are correct and my understanding is that you are trying to minimize some of your assets by you know changing the ward threads into single room and other things, so I have expected slightly better you know although the ARPOB is better, but that is also largely driven by outpatient gets you know calculated in the ARPOB as well, but if I just look at the inpatient realization that has not increased that much despite the feminization effort that we would have undertaken, so please help me understand that little bit better?

Mr. R. Venkatesh: So, short answer is, it takes time. The long answer Dr. Rupert can talk about the challenges in increasing the per patient realization.

Dr. Emmanuel Rupert: Again, it depends upon the kind of procedures that we do and the complexities of the procedure so we do need to keep a work on the margins because some of this is extremely high catenary work do have a little bit on the higher side on the medicine consumables pattern, but that is something which when I mentioned upon the clinical pathways, these are some of the things that we look to standardize across the network to keep this under a very tight control and this is not something that from an administrative point of view we drive, but we have key clinicians and the mentors who drive the pathways to keep these things under complete control and the tech enables us to keep and seeing which are the outliers and from a clinical standpoint and see whether these things are justifiable or not so that we are able to keep this under close track.

Dr. Anesh Shetty: But price wise we always aimed to be the most competitive priced in the marketplace, so our you know realization for patient what they end up paying in our place will not gro

w in line with what ever yone else is able to do because we just want to maintain our mass market appeal.

Mr. Dheeresh Pathak: But is that a fair impression that I have that you wanted to premiumize by changing the bed configurations and all those things, so that mix effect that s hould show up right despite you not taking price increase on like to like basis let's say for the general ward patient.

Dr. Anesh Shetty: Correct, but that takes time and it's not that we converted all the hospitals into only private rooms and even in the hospitals it's just a couple of wards that were reconfigured to meet the existing patient demand where we had not much demand 25 for the general wards and more demand for the semi-private and private. So, we're not going too far ahead of what the market is t elling us they want to be treated and yes to very much a place where people struggle to meet their expenses where they sell assets to pay for healthcare and so yes we can pad up our numbers by going all private and refusing care to everyone else and you ca n get away with that for a short time, but you want to build something that lasts for a century, you have to give the customers what they're willing to pay for and make them very happy for it.

Mr. Dheeresh Pathak: So, because the number that I have not bee n restated for the change in the you know methodology that you would have done for ARPP and discharges, so can you just help us understand that over the kind of pre -COVID period let's say FY19 what kind of CAGR or how much higher are we in terms of IP disc harge because the number that I'm seeing they're still lower, but you're saying that you earlier to my question you said that there's a change in methodology in the way you calculate.

Dr. Anesh Shetty: Yeah, it's hard for us to reconcile the sets of number s because we were on an older system, we haven't moved to all the billing systems to our Atma yet. So, it's tough for us to do a pre -analysis to what it was post -COVID. Once we get caught up

Mr. Dheeresh Pathak: When did you change this methodology of m easuring this?

Dr. Anesh Shetty: Over 2019 to 2020. Now, Atma itself has been ongoing since 2018, but there are certain modules that were put in, but that's not an excuse. I would say that the larger issue is that every year there's a moderate increase i n the per patient realization, but the larger increase is due to volumes, thrupt, discharges, and looking at the case mix. So, it's not just the plain price increase that leads to the year -on-year revenue or margin increase for us.

Mr. Dheeresh Pathak: So, that I understood, but I'm seeing the throughput increase I'm not able to see it, because the numbers tell me that it is still lower?

Dr. Anesh Shetty: Yeah, that's the problem yeah because they're on different - the numbers are different in that the pr evious one was counting everything, so the baseline is there, so what will give you clear numbers is from 2021 onwards, but of course 2021 would have been a COVID year.

Mr. Dheeresh Pathak: But how much are the understated current IP discharges number just

Dr. Anesh Shetty: Yeah, it's not been understated or anything, it's just that it's a very different methodology of calculation.

Mr. Dheeresh Pathak: Yeah, yeah, okay. So, that we don't have, okay. So, next question is this Greenfield 26 CapEx which you' re saying land in Calcutta, this would be I think if I understand correctly after many years that you are buying land for a hospital, so probably the last one would have been in Gurgaon, I think if I understand correctly, so what have you like you know int ernally at what IRR, whatever payback, whatever measure that you have used how you under return this CapEx Greenfield land?

Dr. Anesh Shetty: Sorry, can you just say the last part of your question again?

Mr. Dheeresh Pathak: So, as per my understanding thi s will be after many years for a Gre enfield hospital, so

in internally when you're underwriting this CapEx what measures have you know what is the hurdle rate that you have taken?

Dr. Anesh Shetty: All our investments go between a 16% to 18% hurdle rate an d of course a lot of this is excel fiction, but this was after exhausting all options, so going for Greenfield either acquisition or doing construction wasn't our first port of call. We looked at adding something to the existing structure, but that meant demolishing an existing building, we looked at finding something nearby that was not available to us, so this is after exhausting all the other things. So, it definitely returns do come on the lower end given that on a 10 -year horizon, the returns won't m anifest themselves as well it will be more back ended from there, but we feel more confident about making this investment in Calcutta because it houses our flagship asset, we can leverage a lot of the existing clinical manpower and not have to double inves t in a lot of that and we

have a tremendous brand and patient flow that we can leverage for this. So, we're confident about making this investment, but it definitely will be just like in Cayman margin dilutive it will cause you know huge amount of upfront investments in the manpower and running expenses over there, which will dilute our numbers for a bit, but it will still take time. This construction is 2½ to 3 -year project, so we have time for that to just.

Mr. Dheeresh Pathak: One last question, is the FSI available in Bangalore and Calcutta understand is not, but is there FSI available apart from reconfiguration, is there actual FSI available for you our existing assets?

Dr. Anesh Shetty: Yeah, in the Health City in Bangalore, there is, I mean we can add as much as we want, we're on the absolute outskirts of the city, there's a lot of land available that we won't have a problem. In Bangalore, we're adding an expansion to our cardiac building, we're adding an OPD Plaza, we're adding a park near and we'll eventually add an inpatient area as well and that will come up in phases over the next five years.

Mr. Dheeresh Pathak: Okay, so in Bangalore when you expand you don't have to invest in land, it's only in 27 Calcutta that you have to invest in land?

Dr. Anesh Shetty: We may need to invest a little bit of land, but it doesn't come at a significant cost as it would be in Calcutta.

Mr. Dheeresh Pathak: Okay. Thank you.

Dr. Anesh Shetty: Any other questions?

Mr. Nishant Singh: Yeah, we have Madhu Gupta on line.

Ms. Madhu Gupta: Thanks for taking my question. I have a question regarding the Jammu facility which has turned around and has reported very good margins in FY23. So, what was the kind of I mean what is the growth trajectory going ahead do you foresee any growth potential over there or it's going to remain at the same level?

Dr. Anesh Shetty: The Jammu Hospital is more of a management contract that we run on behalf of the Mata Vaishno Devi Shrine Board, just for purposes of the way in which it's structured, it comes on our balance sheet, but we don't take any money, we don't make any money off it. It is something that's part of our seva to the government over there. So, it'll do well you know those numbers show up on our balance sheet, but it doesn't come at any margin and going forward we would also like to move away from the current system, which is as a limited company and move towards putting this into a trust because the Shrine Board has its own ambition, they want to create a Medical College, they want to take this to another level, and we're just there as management support to help them you know achieve that vision.

Mr. Nishant Singh: Dheeresh, do you still have any questions?

Mr. Dheeresh Pathak: No, sorry. Sorry, I don't have questions.

Mr. Nishant Singh: Yeah, so if we don't have any further questions, we would like to conclude this session.

Dr. Anesh Shetty: Any further questions?

Mr. Nishant Singh: No. So, thank you everyone for your active participation as always. Please do feel free to reach out to us in case of any follow-on queries that you might have and thank you all once again.

End of Transcript

Call: May 2023

(no extractable text — likely a scanned image PDF)

Call: Aug 2023

Date of submission: 11th August 2023

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code – 539551 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051
Scrip Code- NH

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter ended 30th June 2023

This is further to our earlier letter dated 7th August 2023 regarding audio recording of Earnings Call of the Company for the quarter ended 30th June 2023, held on 7th August 2023.

Please find enclosed herewith the transcript of the said Earnings Call. The same is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earning-call-transcripts>.

This is for your information and record.

Thanking you.

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q1 FY24 Earnings Conference Call"

August 7th, 2023

MANAGEMENT : MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – CHIEF OPERATING OFFICER , EAST AND SOUTH REGIONS

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS SUBSIDIARY HEALTH CITY CAYMAN ISLANDS LIMITED

MR. RAVI VISHWANATH – CHIEF EXECUTIVE OFFICER , NARAYANA HEALTH INTEGRATED CARE (P) LTD

MR. NISHANT SINGH – VICE PRESIDENT , FINANCE , MERGERS & ACQUISITIONS & INVESTOR RELATIONS

MR. DURGA PRASAD – SENIOR MANAGER , MERGERS & ACQUISITIONS & INVESTOR RELATIONS

2 Nishant Singh : My name is Nishant Singh. I head the investor relations function at Narayana Hrudayalaya Ltd. I welcome you all to the Q1 FY24 earnings call of the company . To discuss our performance and address all your queries to day, we also have with us Mr. Viren Shetty - our Vice Chairman, Dr. Emmanuel Rupert - our CEO and MD , Ms. Sandhya Jayaraman - our Group CFO, Mr. Venkatesh - our Group COO , Dr. Anesh Shetty - MD of our overseas subsidiary HCCI and Durga Prasad, Senior manager in the IR function. I would also like to take the opportunity to introduce Mr. Ravi Vishwanath who is the CEO of our newly incorporated company NHIC.

We hope you have gone through the investor collaterals which have been uploaded on the stock exchanges as well as on our website. As usual, before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website as well as on the stock exchanges at a later date. I would also like to remind you that everything that is being said on this call that reflects any outlook for the future or which can be construed as a forward looking statement, must be viewed in conjunction with the uncertainties and the risks that they face . Post the call, should you have any further queries, please do not hesitate to get in touch with us . With that, now I would like to hand over the call to Dr. Rupert.

Dr. Emmanuel Rupert : Good evening everyone. I warmly welcome you all to the Q1 FY24 earnings call conference of Narayana Hrudayalaya Limited. The first quarter of the fiscal year delivered steady performance supported by growth in the business across our flagship units and newer hospitals. Consolidated revenue for the current quarter stood at INR 12,334 million, reflecting a year -on-year growth of 19.4% and a marginal increase of 1% compared to the previous quarter. NHL generated consolidated EBITDA of INR 2,858 million in Q1 FY24 at a margin of 23.2% against 22.8% of Q4 FY23. This margin improvement is attributed to improvement in cost efficiencies and improved realizations across all our units including Cayman .

HCCI Cayman continues to contribute significantly to the overall performance with revenue marginally increasing by 2% to \$ 29.92 million. The newly commissioned radiation oncology block in the Camana Bay Hospital has seen good traction in its first quarter and is on track to meaningfully contribute to our growth in the future. We remain confident that our Caribbean business will continue to grow further through strategic initiatives and investments.

3 Our overall balance sheet and liquidity profile remains strong with a group cash and liquid investments of over INR 8.2 billion against gross borrowings of INR 8.4 billion , Net debt of INR 0.19 billion as of 30th June 2023. Our net-debt to equity ratio has further improved to 0.01 against 0.06 in Q4 FY23 giving us sufficient room to fund our expansion through a judicious mix of borrowing and internal accruals . Off the guided Capex of INR 11 billion for FY24 , we have incurred a capital outlay of close to INR 1.1 billion for Q1, and we expect to spend the balance amount in the remaining quarters of the fiscal year.

The first quarter of FY24 witnessed strong momentum in high -end Cardiac Sciences work in Congenital and Adult segments, Oncology, Gastro Sciences, Orthopedics, Image Guided Therapies, and other Quaternary work across all our centres.

- Our Leadership in Cardiac Care grew from strength to strength with NICS in Bangalore performing 2,263 Cardiac Surgeries in this quarter, thereby achieving the highest ever quarterly volume.
- Our Ahmedabad unit, in a first ever for Gujarat, successfully performed PDA Device Closure on a 30 -day old child weighing 900 grams, using 4/2 Amplatzer Occlude

r device.

- Mazumdar Shaw Cancer Center successfully conducted India's first Robotic Ampullectomy Surgery for malignancy of ampulla of Vater.
- RTIICS performed complex Robot Assisted Thoracic Surgery (RATS) for decortication and excision of posterior mediastinal mass.
- I am delighted to share with you that we have successfully started the Radiation Oncology services at our hospital in Jaipur, where over 70 patients have already been registered in this quarter. This hospital successfully performed Prostate Artery Embolization, which is the first ever in Rajasthan.

Our focus on digitization and business transformation has led to significant improvements throughout the NH system. With the implementation of athmâ across our labs, we have experienced a 36% improvement in TAT of our top 5 tests. We have been able to reduce our paper consumption 34% this quarter. Our AADI aap for doctors has led to 50% improvement in Discharge TAT over 2021 numbers.

Our recently incorporated company Narayan a Health Integrated Care (NHIC) has seen good traction with the retail health segment. With Bangalore as our focus, we have reached 6 points of presence as of June end. NHIC Revenue for the quarter has crossed 4 Rs 45 Mn, with the patient transaction numbers crossing 29,000. We are confident in growing this business and serving our customers with a clear focus on building India's leading Integrated Care program.

We continue to invest in our clinical and non-clinical operations across the Group, transform the patient service levels, increase our throughput capacity, invest in more digital patient outreach channels, and improve our operational efficiency. We are simultaneously pursuing organic and inorganic growth opportunities both in India and overseas that will derive synergies from our existing operations, maximize value for all our stakeholders and keep a close watch on return on capital.

Nishant Singh : Thank you, sir. I would request everyone to now use the 'raise hand' feature to start posing the questions and we would try to address them in this forum.

Yes Prithvi, please go ahead with your question.

Prithviraj: Anesh, this question is regarding Cayman business. So could you give us some numbers? From the new oncology block what has been the revenue contribution in this quarter? What kind of procedures are we offering here and how has been the patient flows?

Anesh Shetty: Hi Prithvi. Thank you for your question. So the radiotherapy block, as we've discussed earlier, is one part of the larger Camana Bay development for us. So there are block A and block B. Block A is the radiotherapy, block B is a much larger hospital that will be a multi-service similar to our hospital in East End that will cover everything else. We offer the entire spectrum of radiotherapy services up to stereotactic radiosurgery, which is a very high-end kind of radiotherapy. The machine is a true beam, which is a state-of-the-art latest generation machine and the most modern machine in the region. For your question, on the specific revenues, given that it really is a service line for us, like a specialty department, we won't be able to disclose service line level revenues. But we're confident and comfortable to say that it has met our expectations and we hope to continue on the growth trajectory we are on.

Prithviraj: Just a follow up on this. Despite the oncology block getting commissioned, the margins in Cayman remain high. So can we expect the same to sustain even for the coming quarters?

Anesh Shetty: Prithvi actually, the radiotherapy block is a very small block. While in terms of investment, it is high because radiotherapy is very capital intensive, from a P & L perspective, the costs were never a concern. So it was never planned to be margin dilutive. W

hereas the bigger hospital, which we will hope to commission in Q1 of FY25, will come with a lot of fixed costs because we'll be essentially doubling our capacity in the region. So there we will expect to see margin dilution, and it won't happen when the hospital commissions. As we build towards that date, we have to start accumulating a lot of costs in manpower, nurses, doctors etc. preparing for the commissioning of the larger building. So we will see some margin dilution as we have discussed before. But radiotherapy was always meant to be neutral, it was not expected to be margin dilutive.

Prithviraj : And on the India business, can you give me the breakup between the new hospital's revenue and profits in the quarter.

R Venkatesh : So, when it comes to the performance of India business, quarter-on-quarter we have actually shown a jump of around INR 5.5 crores from the previous quarter. Generally,

Q1 is weak for the hospitals as per historical data. But what is heartening to see is that we have shown a 14.4 % growth in revenues over Q 1 of FY23, which is actually considered to be a good performance. Though first quarter of the year is generally weak in the hospitals because of the summer vacations, family members delaying treatment, doctors not available at times, considering all these, our performance in Q 1 has been very strong. The business should pick up in the coming quarters as per the plans .

Prithviraj: My question is more on the actual numbers. So , what has been the revenue from the new hospital?

R Venkatesh: In terms of the newer hospitals, what we have seen is we have been more or less on track in terms of what we have done. Our new hospitals continue to perform in the expected lines. There is a little dip in Q 1 due to seasonality, but however , we will be able to recover and perform well in the succeeding quarters. Mumbai is also on track. We continue to target breakeven by the end of this year or even slightly EBITDA positive by the end of this year. What is more important for us to see is that we are currently prioritizing investments in our Bangalore, Kolkata and Chennai. Once we are exhausted with these, we will also look at bringing about more investments in these new hospitals, which will accelerate the growth in these hospitals. But overall, we are on track and the performances are on expected lines for the new hospitals.

Sandhya J: Just to add to that. From a specific number point of view, it's flat to last quarter, about slightly above last quarter. We came in at 115 crores for the cohort. That's near to 69.1% growth year-on-year. Our EBITDA margins came in at about upwards of 5% for the cohort. This is the new hospital cohort.

Prithviraj: Viren, the final question to you on the gross margins. See , there had been a sequential decline in the gross margins and are we seeing any further inflation headwinds on the gross margin side ?

Viren Shetty: What happens when you start a new year is that you incur a lot of manpower cost increases with the salary revisions. There is price revision, but that doesn't happen all at once. These are things that have to happen over the year as you renegotiate your contracts, and you are able to transmit any price increases. But these will all be moderated. The headwinds that we expect over the near term for the gross margin, these are all in the realm of probability . One would be further medicines being added to the NPPA, which as it is quite a lot of drugs have been added there. Some indication of trade margin capping on all the drugs and consumables. Whether it happens or not, we are not sure what time frame it will have, but these are headwinds that are there. Not all of it is instantly able to pass on to patients. It's not easy to do so, nor would you want to. So there'll be some hit that the business would take which we would recover not through just raising

sing prices, but through some process efficiency, increasing our volume and increasing the throughput, and that should be able to bring us back on track. But it will take this quarter for us to take the hit , that is Q 1 and Q 2, Q3 and Q 4 to recover.

Prithviraj: Understood. Thank you.

Nishant Singh: Thanks, Prithvi . Can we have the next question from Dhara?

Dhara: Just two questions on the India business. The employee cost in the quarter has somewhat increased to 14% year-over-year and 5% quarter-on-quarter. So is it only the element of price increase, annual increment, or is there any one-off item apart from that increment?

Sandhya J: Dhara, we normally have our annual increment cycles coming in from April. So that is causing the increase from an employee cost point of view. There is no significant one timer in that number.

Dhara: So ma'am, how much would be the annual increment in the quarter and will it normalize going ahead? Like, what could be the cost for employees going ahead?

Sandhya J: So, the baseline that we have set in Q 1, I think that will be the normalized baseline. Obviously, as we improve our revenue and improve our throughput on the same cost, we are able to deliver more output. So that will help us bring the cost as a percentage of revenue. But that will be over a period of time. We have to constantly work on the efficiencies like we have kept doing in the past. And as Viren explained earlier, over the quarters we'll neutralize the cost impact of the manpower cost increase that comes in the first quarter.

Dhara: One more question regarding the EBITDA. So , in Western Region , we have two hospitals , Ahmedabad and Mumbai. You said Mumbai has already been turned somewhat positive. So , should we assume the same for Ahmedabad also or is it negative EBITDA? Both these hospitals in this belt are negative EBITDA currently.

Sandhya J: So, Ahmedabad has never been negative EBITDA. It has not been a high number like the flagships. But Ahmedabad has always been positive EBITDA and positive cash flow .

It continues to be.

Dhara: So if we assume that Mumbai will be turned positive EBITDA in the Q 4 of this year, the overall margins of that entire group would come around 3 -5%. Should we assume that EBITDA margins for the western belt?

Viren Shetty: These are not significant drag on the EBITDA, nor a significant contributors. So the revenue base being a very small percentage of the overall, more of the EBITDA increase that would happen over there, which is more attributable to the performance of Bangalore and Kolkata hospitals and Delhi, which is much larger.

Dhara: That's it from my side.

Viren Shetty: Thank you.

Nishant Singh: Can we have the next question, please? Vijay, please go ahead.

Vijay: Thank you for the opportunity. So, my question is on a little bit broadside on the industry. So I was going through some reports, like where there was mentioned that 1.5 beds per thousand people in India. So in your opinion, like, what the number of beds should be there in ideal condition, or is there any guideline per thousand people? And let's say, if we have to grow from the current bed capacity at the industry level, so how many beds would we require let's say in one, two, three decades of the time to reach that ideal benchmark, it can be five beds per thousand people or something like that? So if you can highlight the broad industry landscape that will be quite helpful.

Viren Shetty: So, the statistic in India is 0.7 per thousand people. The WHO recommends anywhere from 1.7 to 2, as you highlighted. But that is a very fluid number. The truth is, in India, because we have so many people, you can't really expect that we will ever achieve the 8.2 per thousand norms that is there in Western Europe. Because what we're seeing with time, length of stay keeps coming down, the nature of the bed, what you needed

to get admitted for in one point of time, you don't really need to get admitted, a lot of daycare things are happening. So the nature of the business is changing. But I'll take your larger point in that the market size is enormous and not just in terms of the number of private hospitals you're seeing. All the listed PE backed, all the organized hospitals that you see, the ones which declare results and borrow money and grow and expand and run in organized fashion don't even account for 10% of the total bed capacity in the whole country. Another 40% is in the public sector, which means 50% of all the current bed capacity that is available in India is completely unorganized. And all of that will get organized slowly, either through them becoming more professional or market share, they are slowly seeding market share to more organized players. So there is tremendous headroom for growth. But there are a lot of challenges. The biggest one in India being availability of manpower, the second one being making the numbers work. Because the minute you start organizing a lot of facilities, you start to grow, then you have to comply with all kinds of regulatory norms, you have to start declaring the income, which a lot of single clinics don't necessarily have to do because the practice of an individual and the practice of a clinic is indistinguishable. So these things start to come in, and there is a cost, and those overheads that you have to be borne with which the patient won't always pay. But over time, with growing insurance penetration with all these things starting to come in, we see a huge scope for expansion for all players across the country.

Mr. Vijay Chauhan Right. So, is it safe to assume that even if we let's say triple the organized CapEx, so still we will have a far headroom to go ahead for maybe 2 -3-4 decades?

Viren Shetty: Easily, but there will be some concentration risk. If, for example, everyone concentrates on adding too many beds in just one. So, let's say you overbuild in Gurugram, then maybe for the Gurugram population you're a bit over served but then you're tapping into a much larger population growth, which is people from Haryana, people from Uzbekistan and all that will come to Gurugram for treatment. And, so, there is for the next easily three decades, there's enough growth that the sector will see but it will follow more closely to the underlying trends of where the doctors are most available, where the supply chains are most prevalent and more important than any of that where the ability to pay exists if we're talking about private healthcare.

Mr. Vijay Chauhan Right, Sir. And in your opinion, like if the organized sector has to capture the larger pie of the market, so what are the challenges that any organized player would face like even our business or the general mistakes we should avoid in terms of doubling the market share of the organized players? It will be quite helpful if you can highlight some like the risk parameters or some guidelines which can help the business to be successful.

Viren Shetty: One is, we have to change the cost structure of this business radically. What's happening right now is that the cost of construction is extremely high, the cost of setting up a new bed is very high, salary cost is extremely high. We can't change any of that. But there are a lot of these noncore costs that have to be lowered by a lot and a

lot of efficiencies in the way the hospitals work, in the sort of the waiting time patients have in the hospital, the number of people you need to take care of the nonmedical work all of that has to be addressed through some kind of process automation and digitization. So, unless you address that on a war footing, the more you expand the more the cost will also keep growing and then you'll face a situation where you will be completely outpriced for the services you're offering. So,

someone who's building a hospital in, let's say in Gurugram, you'll pay the top dollar for everything, and people will be able to pay for the services, but you try and replicate the same thing in Sonapat, it does not work. The market of people who can pay very high prices for surgery is much less. It doesn't mean there's no demand in Sonapat. 100% is demand there but at a very different price point. So, you have to build a very different kind of hospital and you have to still use the same equipment, still have to hire the same doctors still pay the same salary, still use the same construction cost but you have to be able to break even at a much lower realization. So, for that, all the companies that are trying to expand have to be very careful about what the return on capital looks like, how much they're spending, and whether there's enough of a market that can cater to at that price point.

Mr. Vijay Chauhan Right. And is there any internal capital efficiency guideline that we keep for any matured asset?

Viren Shetty: Sorry, could you repeat that? Internal what?

Mr. Vijay Chauhan ROC mark? Like this should be the ROC at the mature hospital, some guidelines if you are keeping or maintaining at an older hospital?

Sandhya J: So, currently, as you are aware, we are running at a high ROCE of over 26% but ROCE is a cycle. So, when a lot of capacity comes up then till the capacity catches the revenue in the EBITDA cycle, you will see some dilution in the ROCE and then we will catch back. So, we are targeting a healthy ROCE on a constant basis for each of our hospitals. 10 So, that's the target with which we are working. But it will be to some extent cyclical as and when we create capacity.

Mr. Vijay Chauhan Right. And in terms of like geography expansion, if we have to go by Tier 1, Tier 2 or Tier 3 cities so, again, my question is on the very long or long term point like maybe a decade kind of horizon. So, in your humble opinion are you focusing more on let's say metro cities or are you focusing on the Tier 2 or Tier 3 towns? And what will be your geographical expansion in terms of even the foreign land and also on the domestic landscape? If you can throw some light, it will be, again, very helpful. Yeah.

Viren Shetty: In India, the demand is pretty uniform wherever you go; small town, big town does not matter. There are still people needing heart surgery, Onco surgery but the ability to cater to that demand at the current set of price points, unfortunately, is best met in Tier 1 towns. Having said that, our priorities in the short run, it's not so much about Tier 1, Tier 2. For us, our priorities are the existing network. We have existing patient flow, we have incredible demand, we have a lot of need, and we have a lot of waitlist for people who want surgery and we don't have enough beds and OTs to cater to them. So, our immediate expansion priority is the existing network. Once we've exhausted the opportunity set in the existing network then we look at expanding in a little bit of a radius around those hospitals so that we build very strong overlapping capabilities where smaller hospitals in small towns refer to the larger hospitals in the same State. So, that's the strategy we would go. There is a much larger macro play where there's demand all over India but at this point, we can only think about what we currently have.

Mr. Vijay Chauhan Right. That was quite helpful. I will step out of the queue because other participants are also waiting, In case there is more follow up, so I would rejoin the queue.

Nishant Singh : Thank you, Vijay, for the questions. Can we please have Gagan to pose the questions?

Gagan Thareja: Yeah. Good evening. Sir, the first question is about the tax rate. For the first quarter it was fairly low at 10.6%, can you elaborate on that?

Sandhya J: Yeah. Hi, Gagan. Thank you for that question. Good, you asked. So

, we were in the earlier tax regime till last year because we had some brought forward losses and MAT credits that needed to be set off. So, we have completed that, and we have opted for the new tax regime. But what has also happened is that we had deferred tax asset which we were carrying, which came in as a deferred tax credit because the tax rate has gone down. So, effectively, our India tax has come down to around 18%. That's because around 26% -27% is the parent tax rate and then there is about a 9% deferred tax credit which has come, which is a one timer for the current year only. And that's why you've seen that you know, significantly low tax rate. We will still be lower than FY23 when we go into the next year FY 25, we will not have this about 9% deferred tax

credit, which we are enjoying in the current year.

Gagan Thareja: Okay. So, for the whole of this year you will have the benefit of the deferred tax credit and then starting FY25 it'll be effectively 26% tax rate for your India business. Is that correct?

Sandhya J: Correct. Some small point to keep in mind is, some subsidiaries will still have some brought forward losses to set off and we may not move to the lower rate of tax but it will not have a significant impact from an overall ETR point of view.

Gagan Thareja: Right. And if I recall correctly, last year you had some pending receivables from the government which came through because of which your cashflow was sort of saw some benefit and, I think, that would also have in some way helped to retain a healthy leverage on the Balance Sheet. This year, again, you have a sizable Cap ex, would the current gross rate need to rise further to fund this Cap ex and by how much, if so?

Sandhya J: Sure. So, I will just answer the receivable piece and then I will hand over to Nishant on the debt piece. So, two parts to the receivable. We had sovereign receivables from Saint Lucia, that came in in the last quarter of the last year. That, yes, gave us a cashflow as well as EBITDA benefit. And if you see, we normalized for that one timer impact in our disclosures also. Separately to that, as far as India receivables are concerned, I think it will follow a certain pattern this year being the election year. I think elections will be cyclical, but we will have some slowness towards the later part of the year as the elections start coming through. So, there will be some Working Capital impact that we are anticipating in the current year. It will not be as good as it was last year.

I'll just hand over to Nishant to explain the borrowing strategy.

Nishant Singh: Yeah. Thanks, Sandhya. See, as we have guided Capex for this year, if we add up both Cayman and India, it's going to be around 1130 crores -1140 crores. At this juncture, we are very comfortably placed, even better than the Q 4 of last year. Our Net Debt is only 19 crores. So, if we have this spend done for the entire year, furthermore we have mentioned we'll be boarding around say 60 %-70% of the entire Capex through debt, both India and Cayman put together, which would essentially mean that our Gross Debt and Net Debt both will go up, but the Net Debt amount will 12 go up by only say 550 -600 crores, which will still keep our Net Debt and EBITDA ratio at a very comfortable number of less than 0.60 or 0.65.

Gagan Thareja: Right. And for your Sparsh acquisition which you did last year, if you could give us some idea of where things stand as of today and, you know, for the whole year of FY24? As you exit it out, how should we think of this piece?

R. Venkatesh: Yeah, I'll take that. See, when it comes to Sparsh, for this quarter our revenues are very well in line with the plan and with a very healthy EBITDA margin of around 34%.

The unit when we took it up in Q 3 of the last year, the entire team of the erstwhile setup was taken up by us. We had actually worked a lot of

on cost controls and our

projected overheads were around 23% of the revenue. We could leverage a lot of benefits from the existing hospital operation because it's a part and parcel of the Health City and thereby we could work out better EBITDA margins much more than the projections. But what has happened is though it started very well with higher projections than the budgeted figures, it has grown in strength quarter-on-quarter and from a 30% to 31% EBITDA, we've grown up to 34% in Q 1. We expect the trend to continue in the coming quarters. But, of course, there are certain headwinds which we need to work around. But having said that, we are positive and confident that we'll be able to deliver a good percentages EBITDA percentages by the end of this financial year, consistent to what we have done in the last financial year for Sparsh.

Gagan Thareja: Okay. The final question, I mean, last year's Capex breakdown, if I look at your presentation, apart from Sparsh there are Daycare Beds and Radiology Equipment, Documentation, and Surgical Robots. So, these are equipment, you know, as and when they commission, they in a way give you additional capacity, you can increase your throughput, you've indicated it in the past as well. So, I would, you know, sort of infer that or surmise that this Capex gives you sort of a lower payback than simply adding bed capacity. You already have the demand but you are unable to meet it for the lack of equipment. Is that now taken care of? And if so, effectively how much additional capacity do you get by these increments on the Radiation Oncology, on the Surgical Robot and Radiology Equipment?

Viren Shetty: I'll take this one. The last priority for us for expanding is through adding new beds.

Why? Because for every bed you add, you need four walls to encompass it, you need a lot of people to hire to be able to treat the patients who occupy those beds. So, the fastest payback for us comes with incremental capacity expansion or things that reduce the throughput. So, lot of technologies such as faster MRIs, more Cath Labs, ICU beds increase the throughput of our existing infrastructure. So, through bed

13 reallocation or through changing the interiors, we're able to have patients get discharged faster with the same manpower or sometimes with even lesser manpower. So, those are the opportunities we pursue at the highest priority but in parallel also going with beds because beds deliver on topline and, ultimately, that's the one that we would most want to pursue.

Gagan Thareja: So, you know, if the bed count were to remain what it is, ceteris paribus, I mean the bed count doesn't change, with all of this additional investment how much can you move the revenue line or the profit line? I'm not asking for exact figures but if you could give us some quantification of what you get out of in terms of Return On Investment or Fixed Asset Turn from this sizable investment that you make? And over what timeframe does that happen? I presume it will happen quickly.

Viren Shetty: Yeah. So, the investment itself, some of it is to improve the throughput but some of it is inevitable. There are some things that are just about replacing aging MRI's and changing the interiors and so on. So, those are things you would have to incur but all of this is meant to sustain the current pace of revenue and EBITDA growth under the same current capacity of beds that we have until the new capacity comes in line, which will be 3.5 years from now. So, that comes in then the new capacity will lead to further topline growth. But, of course, certain margin dilution would be expected when that day comes. But right now a lot of this is to continue the current pace of growth what we're doing without necessarily having to add more hospitals to achieve that.

Sandhya J: So, I may just want to add that, you know, we had all

most 20% growth last year coming

in without adding any beds actually. So, a lot of it came out of the work that got done on the throughput side. So, there is a lot of juice in that to extract and that's a key focus area for us.

Viren Shetty: And that delivers the highest ROCE because it's just that one time investment that you make on this and it's immediately accretive.

R. Venkatesh: Just adding to that one more line. We continue to work on these efficiencies, we have discussed about this earlier also. So, this will actually create those additional space and that we will address the bottlenecks through adding more OTs, ICUs, the Lab, the Diagnostic, all these equipment which you've mentioned to make sure that we increase on the revenues at the same cost levels. So, with this, still we get into the capacity expansions. We'll be able to maintain the margins till such time where we are able to get the capacity expansion going in the next 2-3 years' time.

Gagan Thareja: Right. So, when you say that the current Capex '23 and '24 can risk 3.5 years of growth 14 requirements barring the greenfield at Cayman, are you sort of referring to the FY23 growth number as being a sustainable one?

Sandhya J: We really can't guide like that, Gagan. We are aspiring to maintain a healthy growth momentum and we believe that we have enough ability to expand our throughput and capacity may not become a bottleneck for us to grow. So, that is the that's the aspiration.

Gagan Thareja: I get it. So, just my final question, which is that it would seem to me that none of the Capex that you've so far commissioned is going to be EBITDA dilutive. And is it also a reasonable assumption that even at the PBT level this is not going to be dilutive even post the additional depreciation and interest cost?

Sandhya J: There will be a little bit cyclicality there because when we capitalize immediately, especially Cayman capitalization will happen back end of the year and early next year, the improvement in the EBITDA is not going to come, upfront. So, while Anesh spoke about some of the costs coming online, depreciation will also be a cost that will start coming online. So, that will cause some dilution but other than that, yes, a lot of the Capex that we are doing may not cause a significant EBITDA dilution. But one point we have to keep in mind is that, you know, there is a cost headwind that all hospitals are facing including us, both on the material side and on the other costs side.

Viren Shetty: Those are the macro things. It's not caused by adding these beds and Cath Labs and so on. The things outside of our control could have the potential dilute margins as and when they happen, mostly around the price control from the government.

Gagan Thareja: Right. Thanks. I'll get back in the queue. Thanks for taking my questions.

Nishant Singh: Thanks, Gagan, for your questions. Harith, can we please have the question from you?

Harith Ahamed: Hi. Thanks for the opportunity. So, can you provide an update on Narayana Health Integrated Care? So, you had announced this initiative last quarter and I see a ₹6 crores EBITDA loss for this quarter. So, you know, these are actually coming from new clinics that we have set up during the quarter or, you know, these were existing clinics? And any updates on your targets here and your strategy here?

Viren Shetty: Yeah. Some of the updates are on the slide towards the end but I'll ask Ravi to give the overview.

Ravi Vishwanath: Sure. Thanks, Viren. Thanks for the question. So, yeah, we've been piloting our

Integrated Care offering from six locations in Bangalore. It's still early days and, I think, we're kind of building that we've got pretty good traction at the moment as we've 15 noted in the tech as well, about close to 30,000 transactions and about 45 million in revenue so far. A

nd, as I said, we'll focus on Bangalore and then learn and adjust the model and expand subsequently.

Harith Ahamed: So, my question was do we have any targets in terms of number of centers or clinics that we aim to have in the next couple of years?

Ravi Vishwanath: So, at the moment it's you know it's in beta mode and our focus is really on building an integrated model and solving the customer's requirements and as we build that model and as we get more learnings. We will figure out the base and phase in which we want to expand, but at this point, we don't really have numbers that we are ready to share in terms of the number of clinics or expansion plans. We're very much in learning mode right now.

Viren Shetty: What if I can put it in another way. We want to provide coverage for our the current target market in Bangalore with as many clinics as is required to achieve that coverage, coverage meaning all the customers were going after where they were able to give them medicine in their house, whether we're able to you know take care of their kids when they're sick at 2:00 in the morning, those sorts of services we keep adding if that requires 10 clinics, 50 clinics, or 100 clinics is the iterative process we would take to get there, but this is entirely oriented around building capacity to serve the needs of the subscribers to our integrated plan not about adding a bunch of clinics and then running around and trying to get more people to walk in through the doors.

Harith Ahamed: Okay, got it. My second question is on the ARPO B growth. I see that ARPO B has grown at around 11% at the network level and this is in line with what we have seen in the last few years as well. So, you have guided a bit cautiously during the last conference call on ARPO B growth that we will not be able to sustain the rate that we've seen in the last few years, so how should we think about ARPO B growth going forward, not exactly looking for a number, but just qualitatively the levers we've had in the last few years are they still there and any color would be helpful?

Viren Shetty: You can just write down somewhere that the guidance from this company will always be cautious. See, ARPO B growth is essentially like we said a function of efficiencies as well as whatever realization increases that can happen, efficiencies are how much faster the discharges happen, realization through not much price increase, but in terms of the complexity of the procedure that we do, but more so increasing the number of discharges, daycare procedures, and that's what we're doing. So, the more these hospitals mature, more we invest in things like robotics and daycare procedures 16 and getting now more into orthopedics which has very quick discharges. This ARPO B number should improve marginally over, but yes we are cautious because we don't really use price increase as the leverage to try this.

Harith Ahamed: Thanks, Viren. That's all from my side.

Nishant Singh: Thanks, Harith. Asif, can you please have your question? Hi Asif? Okay. So, we can move to Hrishit.

Hrishit Jhaveri: Hi Sir. Can you throw some light on the insurance segment, where are we and where do we see ourselves in the next two years?

Ravi Vishwanath: Yeah. So, at the moment we have applied to the IRDAI for as a standalone health insurance company and we are working with them to get approval for setting up the company.

Hrishit Jhaveri: Okay and can we expect in this FY that the business can be up and running or the next year?

Ravi Vishwanath: I think you know the difficult to exactly commit to that because as you know it's a really long process, but we are engaging closely with the regulator to make sure that we have all the approvals in place, but it's not just the approvals, there are a number of other things that also need to follow in place in terms of our systems and other things that have to be re

ady so that we can start serving our customers well from day one, but the key thing of course is the regulation. So, I would say we would like to get going as soon as we can, but at this point in time, I don't want to give you know end of this year or early next year time frame either.

Hrishit Jhaveri: Okay and one more question regarding the Samyat Healthcare, can you throw some light there?

Sandhya J: It's just a more tax efficient distribution model because we have a lot of subsidiaries coming through, so to avoid GST leakages, we are finding a more efficient way of keeping our inventory and being able to dispatch them to our subsidiaries. So, at the moment there isn't too much to read into that.

Hrishit Jhaveri: Okay. So, it is right to assume that it will be restricted to our subsidiaries and not any outside hospitals or it is open for that too?

Sandhya J: Yeah, it will be restricted to our subsidiaries at the moment

Hrishit Jhaveri: Okay, can this improve a bit of margins because of cutting down the middle people?

Viren Shetty: So, what's happening is, because of the price control we faced a lot of margin pressure 17 on the medicine sales, so at best this would claw back a little bit of some of the losses, but it may not improve our consumer margin from where we are.

Hrishit Jhaveri: Okay Sir. Thank you, Sir and all the best for the coming year.

Nishant Singh: Thank you, Hrishit. Asif, if you are ready can we have the question from you please?

Asif Ali: Yeah, thank you for the opportunity. So, my question is regarding the Cayman Islands business. As we can see here the average revenue per patient is declining sequentially from USD 39.3 thousand in September 22 to currently it's USD 30.9 thousand, so could you please put some light on what's the reason behind this, what's happening?

Anesh Shetty: Hi Asif. Thank you for your question. So, just to clarify were you asking about the average realization per patient?

Asif Ali: Yeah, I'm asking about the average revenue per patient?

Anesh Shetty: Sure. So, if you look at the trend over a longer time horizon given that it's a relatively small asset in volume terms, we see these swings from time-to-time, but if you look at say the past six or seven quarters, there was an anomalous quarter where we were around close to \$40,000 to \$41,000 for patient, but more or less we have been hovering in the range of \$30,000 to the \$35,000 per patient and that's been pretty consistent for some time. The challenge is with looking at just a few quarters one or two quarters is it gets significantly skewed either which way sometimes up when we have one or two you know large very long stay patients who have a very high discharge value etc., that tends to skew it because the base is small. So, my suggestion would be to look at these trends over at least 6-7 quarters or so and they're more or less in a narrow range.

Asif Ali: Okay. Thank you very much.

Nishant Singh: Thanks, Asif. Can we have the question from Vallinayagam M please?

Vallinayagam M: Yeah. Thank you for the opportunity. Am I audible?

Nishant Singh: Yeah.

Vallinayagam M: Yeah. Sir, my question is about the trade payable days in our annual report 2023. So, as per the annual report, our trade payable turnover is 1.9 to 2 range and so it means we are paying our vendors at 172 or 180 days, but if you look over our sales, here a majority we are getting a cash in hand. So, in this context why do we need to pay the vendors at very long period like six months at all, whether my understanding is correct or not is all the first thing and the second one is whether it is an industry wide 18 phenomenon that is my second question and third one is suppose if you are able to release the payment bit early, is there any possibility to get discount from the vendors and the same may flow to our margins? So, this is what is my overall ques

tions. Thank you.

Sandhya J: Yeah. See, actually, Vallinayagam if you look at the trade payable days it encompasses various - it's a calculation, it has provisions and other non vendor line items there. As regards the underlying payable days, we pay between 60 to 90 days to all vendors and MSMEs much faster, that's the industry norm. Most of the companies pay between 60 to 90 days and I don't think paying faster than that has a significant cost benefit. We do assess from time-to-time if an early payment is more efficient and therefore whether it is able to give us any leverage in terms of our landed cost, then we do take those opportunities with the vendors. So, we are in line with the industry on this.

Viren Shetty: But I would add these conversations have been had with the companies, but since we have such a fragmented supplier base, no one vendor has such an outsized impact on the purchasing volumes. The early payment-based discounting, like what DMart and the rest of them do, does not apply to our business.

Vallinayagam M: Okay. Thank you so much.

Nishant Singh: Thank you. Can we have the question from Lohit please?

Lohit S: Hey, thank you for the opportunity. Just wanted to understand that strategically is there any thought process on adding complementary specialties you know other hospitals currently are also gearing up heavily on specialties like mother and child care, some are scaling up their diagnostic practices, so is there any thought process on that from the management?

Viren Shetty: We have all the specialties, but standalone businesses like mother and child care that is something that we'll be doing as part of the NHIC Program. So, Ravi's team is building out these different format of small clinics, medium sized clinics, polyclinics that will cater to a range of requirements. So, we won't be building like you know what Apollo Cradle or Nephro Plus have done where it's dedicated to just one

specialty. For us, these are called Narayana Clinic and the idea behind Narayana Clinic is that everything is available to you and the range of offerings will modify based on what the needs are. So, if we are in a location that let's say the demographic skew slightly older than there may be geriatric specialist over there. Whereas if the same Narayana Clinic is in a place where a lot of new apartments and young families, that may have a more robust Pediatrics, but complementary specialties that is we create a 19 referral pipeline in primary care through NHIC, which will cater to the entire primary needs or primary care needs of the patient, which is medicines, diagnostic, lab test, and consultations.

Lohit S: Understood and the plan will be to keep them co-located with the existing setup or go independently also?

Dr. Emmanuel Rupert : It will serve the larger hospital setup. So, like Viren explained, there's no point in children and antenatal mothers to come to the major hospitals for a routine checkup. So, they'll all be done in the clinics and give a better patient experience for that. You only need to come to the larger hospital setup for something which is more complicated and you need a higher imaging or a more complicated investigations to be done. Otherwise, everything else will be sufficient to be handled because many these things can be handled on an outpatient basis. So, these will be handled at the clinic level and when it is necessary, they will coordinate with the necessary subspecialist at the hospital for them to visit and take the necessary help.

Viren Shetty: So, in Bangalore for example, our health city is in Bommasandra, which is in the extreme southern tip of Bangalore. So, the places currently where we're building the clinics are in the areas in the southern part of Bangalore. So, in Electronic City, in HSR, in Sarjapur, Whitefield these places we'll slowly start adding cl

inics and then start to

cover the rest. So, it won't be the departments, we'll still be - there is a mother and child department in the Health City in Bangalore, but smaller OPD based clinics will be all over the place, so that you can see the doctor there, but when it comes time for the delivery that's when you come to the hospital.

Dr. Emmanuel Rupert : Even the chronic ailments like hypertension, diabetes, and things like that we can handle on a video consultation, but wherever they do need to come for a routine once in three months or kind of a checkout, they'll all go to the clinics where our doctors will also be available as and when required.

Lohit S: Understood. Sir, essentially more like primary, secondary care?

Ravi Vishwanath: Yes, yeah.

Lohit S: Understood. Thanks a lot for the time. Thank you so much.

Nishant Singh: Thank you, Lohit. Can you move to the Damayanti for her questions?

Damayanti Kerai: Hi, thanks for the opportunity. I have two questions. First wanted to understand your India operation. So, you recorded growth of around 14% year-on-year, so can you explain like of this growth how much might be contributed by volume or how much be

20 driven by mix improvement etc., so like broader understanding on some drivers here?

Viren Shetty : The drivers are the footfalls, occupancy, throughput. The business mix remains more or less the same. We did have some slight uplift in certain departments like

Orthopedics and Oncology which we've spent a lot of time focusing on, but a lot more of it has just been the overall volume of patients we've been able to serve in our hospitals has gone up, as well as our ability to discharge them faster and treat them and take them in the daycare mode has gone up. So, a lot of that is driven by the ability to service more patients. Some realization increase has also come up that is there as well both through pricing and efficiencies, but I would say volumes have increased. There's also been a slight change in the payer mix as well, the number of patients that we treat under the insurance programs have gone up as a percentage of the total slightly. International is more or less the same and cash segment has gone down a bit.

Damayanti Kerai: Okay. So, primarily volume-driven and then process efficiency then you think that the business mix broadly remains unchanged?

Viren Shetty : More or less, yeah.

Damayanti Kerai: Okay and in terms of annual price hike, what's the general range which you take, like I understand it's smaller part, but nonetheless by like what percentage you generally increase your tariffs while revising it?

Sandhya J: We take low single digit price hikes every year like we've always said, we don't pass on the entire burden of the cost increase to the price, we work through with our efficiencies and throughput improvement to offset some of the cost and only some of it we pass on price. So, that's the same philosophy we followed this year also. Our price hikes were in the range of low single digits.

Damayanti Kerai: And it's already taken for this fiscal year?

Sandhya J: Yes. Our price hikes are taken on the 1st of January every year. So, we make our price

list available on our website at that point in time, so that there is transparency to the patients. The impact of the price hikes are already flowing in, it flows in with a slight lag into the realization profile,

Damayanti Kerai: Okay. My second question is on your international business. So, what are your thoughts here like say over the next three to five years, how much do you want to scale it up from the current level?

R. Venkatesh: International patient volumes have actually grown quarter-on-quarter. Even in this 21 quarter it is grown much more than what it was in FY23, but that's not all I mean, we are basically focusing on the core market of Bangladesh. When it comes to the numbers, we are

not purely relying on these numbers and I don't think we'll be able to reach the numbers which we had pre-COVID because we have cut off all the arrangements with any type of an intermediary, we're also trying to restrict our activities to key markets on a direct engagement model plus what we are doing is, we're trying to shift more into the marketing spends on digital and domestic activities, which actually are much less impacted if there is any travel disruption due course of time. What happens is, the international medical travel will keep reducing as our neighboring countries keep developing in terms of the healthcare sector and eventually, they match us. So, a lot of new hospitals are also coming up in Dhaka, Chittagong, and other important cities. So, it's time that we try to focus more on our digital and domestic activities as an alternative. So, the core focus will be only on this, but the entire alliance will not be on international marketing though the trend shows positive fractions for the quarter on quarter even including this quarter.

Damayanti Kerai: Okay. Thanks. Thanks for your answers. All the best.

Nishant Singh: Thanks, Damayanti. Can we have this question from Alankar?

Alankar Garude: Hi, Thank you for the opportunity. The first question is, given that we are not going to expand bed capacity for the next three to three and half years and the focus is on sweating existing assets, would it be fair to assume a pretty decent margin expansion in the domestic business over the next few years?

Viren Shetty: Not that we're not expanding over the next three years. It just takes three years for all the significant capacity to come online. Meantime, certain smaller projects are happening in Ahmedabad, in Howrah Hospital, and here and there sometimes realignment in Bangalore and Kolkata. It's not that we're actively avoiding it, so that's one thing to note that we are investing, the construction projects take much longer than they used to. In terms of margin expansion, we'll do our best, but it won't be as much as when the new capacity becomes fully operational and break even and so on. We'll do the best that we can to counteract a lot of the margin pressures through salary increases, the trade margin capping, you know NPPA and those sorts of things so, whatever efficiencies that we get.

Alankar Garude: My second question is, is there any update on the second leg of C GHS price hikes, which was expected to come through in July?

Viren Shetty: We haven't heard anything yet so far. Whatever they announce before about changing 22 the OPD fees from ₹100 to ₹150, that has an impact of about ₹2 crores for the whole year.

Alankar Garude: Understood. Great. Thank you and all the best.

Nishant Singh: Thank you Alankar. Any further questions from anyone? So, as we don't see any further raise of hands, we would like to close this Q&A session. Thanks everyone.

Thanks for your active participation as always. Please do feel free to reach out to us in case of any following questions. Thank you once again.

End of Transcript

Call: Nov 2023

Date of submission: 21st November 2023

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code – 539551 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051
Scrip Code- NH

Dear Sir/Madam,

Sub: Transcript of Earnings Call for the quarter and half year ended 30th September 2023

This is further to our earlier letter dated 15th November 2023 regarding audio recording of Earnings Call of the Company for the quarter and half year ended 30th September 2023, held on 15th November 2023.

Please find enclosed herewith the transcript of the said Earnings Call. The same is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earning-call-transcripts>.

This is for your information and records.

Thanking you

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q2 FY24 Earnings Conference Call"

November 15th, 2023

MANAGEMENT : MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – CHIEF OPERATING OFFICER , EAST AND
SOUTH REGIONS

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS
SUBSIDIARY HEALTH CITY CAYMAN ISLANDS LIMITED

MR. RAVI VISHWANATH – CHIEF EXECUTIVE OFFICER ,
NARAYANA HEALTH INTEGRATED CARE (P) LTD

MR. NISHANT SINGH – VICE PRESIDENT , FINANCE , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. DURGA PRASAD – SENIOR MANAGER , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

2 Nishant Singh : Good morning everyone! My name is Nishant Singh. I head the Investor Relations function at Narayana Hrudayalaya Ltd. I welcome you all to the Q2 FY24 Earnings Call of the company. To discuss our performance and address all your queries today, we also have with us Mr. Viren Shetty, our Vice -Chairman, Dr. Emmanuel Rupert, our CEO & MD, Ms. Sandhya J, our CFO, Mr. Venkatesh, our Group COO, Dr. Anesh Shetty, MD of our overseas subsidiary HCCI, Mr. Ravi Vishwanath, C EO of NHIC and Durga Prasad, Senior manager at the IR Function.

We hope you have gone through the investor collaterals which have been uploaded on the stock exchanges as well as on our website. As usual, before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website as well as on the stock exchanges at a later date. I would also like to remind you that everything that is being said on this call that reflects any outlook for the future or which can be construed as a forward looking statement, must be viewed in conjunction with the uncertainties and the risks that they face . Post the call, should you have any further queries, please do not hesitate to get in touch with us . We would like to answer them to the best of our ability. With that now, I would like to hand over the call to Dr. Rupert.

Dr. Emmanuel Rupert: Good morning, everyone. I warmly welcome you all to the Q2 FY24 Earnings call conference of Narayana Hrudayalaya Ltd (NHL).

After a steady Q1, the second quarter of the fiscal year delivered strong performance supported by the growth in business across our units, leading to improvement in realisations in India and Cayman. Consolidated revenue for the current quarter stood at INR 13,052 mn reflecting a YoY growth of 14.3% and QoQ growth of 5.8%. NHL generated Consolidated EBITDA of INR 3,265 mn in Q2 FY24 at a margin of 25 .0% against 23.2% of Q1 FY24. This margin improvement is attributed to higher revenues, improvement in cost efficiencies and realisations.

Our Cayman units (HCCI and EICL) continue to deliver strong business performance with highest -ever quarterly revenue at USD 31.5 mn. The recently commissioned Radiation Oncology Block in Camana Bay hospital has seen good growth in this quarter contributing meaningfully to the overall growth in the region. We are confident that our Caribbean business will continue to grow through strategic initiatives and investments.

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The balance sheet and liquidity profile at the group level remain strong with group cash and liquid investments of over INR 8.74 billion against gross borrowings of INR 9.61 billion (Net debt of INR 0.86 billion) as of 30th September 2023. Our net debt -to-equity ratio remains steady at 0.03, giving us sufficient room to fund our expansion through a mix of borrowing and internal accruals. We have incurred capital outlay of close to INR 3.94 billion till the September quarter and are on track to spend the balance amount in the remaining quarters of the fiscal year.

We take immense pride to announce that Narayana Health is now the 1st healthcare group in India and the 6th Globally to be awarded Joint Commission International (JCI) Enterprise Accreditation. With this achievement, 8 of our hospitals were accredited with JCI accreditation for delivering world -class care at industry -highest standards of quality.

To Improve awareness towards Preventive Health Care, we conducted the highest number of electro cardiograms (ECGs) in a single day at a single place, which

has

earned us a coveted spot in Guinness World Records. This record -breaking attempt took place on September 21st at NICS, Bangalore.

On the clinical front, NICS Bangalore successfully performed 22 Cardiac Surgeries using a Robot, demonstrating our capabilities in adopting the latest technology. MSMC Bangalore successfully performed 5 cases of Limb re -attachments during the quarter.

RTIICS Kolkata performed some cutting -edge complex clinical procedures in the Oncology and Renal sciences segments such as Redo VATS Resection Surgery on a 3 - year -old for Pleuro -Pulmonary Blastoma Type -III, and Robotic Renal Transplant on an obese patient with a BMI of 36.2 where the patient was discharged within 9 days of surgery.

Our focus on digitization and business transformation has led to significant improvements throughout the NH system. We have been able to reduce the administrative workload by 36% through our Narayana Health app and Patient Kiosks. Our doctor app “aadi” has helped to reduce the doctor response time by 45% through real-time alerts and integrations. By enabling payments and discharge alerts on Whatsapp, our patients are now able to get discharged directly from the ward hassle - free. We have also launched a new app for nurses named Namah which is aimed at reducing paperwork and manhours spent on data entry.

4 Our new venture Narayana Health Integrated Care (NHIC) has shown healthy growth this quarter after starting on a positive note in Q1, seeing good traction with the retail segment. Revenue for the quarter has crossed Rs 52 Mn, with more than 45,000 patient transactions. We will continue to grow this business and serve our customers with clear focus for improving their health outcomes.

We continue to upgrade our clinical and non -clinical operations across the Group, transform the patient service levels, increase our throughput, build more capacity, invest in more digital patient outreach channels, and improve our operational efficiency.

We are reasonably on track on our ESG Commitments and continue to focus on creating meaningful social impact in addition to pursuing our environment goals and upholding highest standards of Governance.

We are simultaneously pursuing organic and inorganic growth opportunities both in India and overseas that will derive synergies from our existing operations, maximize value for all our stakeholders while keeping a close watch on return on capital.

Nishant Singh: Thank you, sir. I would request everyone to now use the ‘raise hand’ feature to start posing the questions. Ya Pradnya, please go ahead.

Pradnya Ganar: Good morning! So my question is, pertaining to your slide where you have mentioned the Capex break -up. So I see around 250 odd crores for some Brown Field Expansion. Could you please elaborate us where this Brown Field expansion is expected to come and by when we should be seeing that?

Sandhya J: Like we have explained earlier , we are looking at the brownfield ield expansion in our flagships which is mainly Bangalore and Kolkata . We have land in our Health City campus itself and we have already initiated the process to start building , and we are in advanced stages of the closure of land in Kolkata and we will start building next year. So that's largely the brownfield and greenfield that we have spoken about.

Pradnya Ganar: Ok, I thought the Kolkata land would be in the Green Field part of 150cr.

Sandhya J: Yes, that is correct. The land itself will be in the Green Field part, that is correct.

Pradnya Ganar: Ok. If I have to look at your bed capacity expansion from

here to 2 years or later, then

how should we look at the bed addition then?

Venkatesh R : See, what we do is, we keep continuing to enhance our ability to treat more patients 5 and bed addition is just one way in which we do it. What we have done is, recently we have added 2 more floors in our Howrah unit in Kolkata where we will operationalize 110 more beds by the end of Q4. We will also start construction work in Bangalore Health City. We have got all the permissions and we should start the groundbreaking from Q4 of this year. We plan to add 700 more plus beds for the next 3 -4 years plus Sandhya has already mentioned about the advanced stages of land acquisition in Kolkata with more updates in the next quarter. If things work out in the way we planned it, towards the end of 1st quarter or 2nd quarter of the next year , we will start construction there plus we are also looking at other projects in our flagships, both greenfield and brownfield in an opportunistic way. We believe that we have sufficient capacity to cater to our demand and growth aspirations in the interim till we have these beds completed in the next 3 years time.

Pradnya Ganar: Understood. Thank you. Lastly, from going forward, what levers do we have on margin expansions for our India business?

Venkatesh R: As we have said , we keep focusing on improving our high throughput all the time, like we have been doing. We are also significantly investing in technology to improve through put. We are working on faster discharges, faster lab results, and seamless appointments. We perform various cardiac surgical procedures and robotic procedures

which is a morning admission and evening discharge type process where we utilize our beds more effectively during the prime time. We are also working very effectively on those discharges that happen post 2pm and reducing shortcomings in morning admissions. There has been substantial penetration we have done on that front and things have improved a lot, improving throughput and increasing the no of ICUs. We are developing a communication tool for doctors and nurses to coordinate faster resulting in better care and faster discharge of patients. If you see the ALOS, our plans were to bring it down over a period of time to 4.1 and we have already got it down from 4.8 to 4.4 now, which means we are utilizing our beds more effectively. We are significantly investing in technology to improve our throughput and all these will help us achieve higher revenues. We are addressing the capacity bottlenecks by adding more ICUs, OTs, diagnostics and more labs which enable us to increase our revenues at the same infrastructure and the same cost levels in the next 2 to 3 years till we have our expanded capacity ready for utilization.

Sandhya J: I would only want to add one word of caution there, that there are also significant headwinds that we continue to face from inflation and government action point of view. So, we have to take the commentary in collaboration with that as well.

6 Pradnya Ganar: Sure! Thank you for answering my queries.

Nishant Singh: Thanks Pradnya. Can we have the question from Prithvi please?

Prithvi: Hi team! Congrats for executing so well over the last few years. So my 1st question, on the India business. Obviously see, now we are almost at INR 1000 cr. revenue and 18 to 19% operating margins. I mean obviously you have been talking about various throughput, efficiencies etc. but given that bed addition is going to take some time, how much more can we squeeze in the India business and can this revenue be maybe of 50-60% higher only from the existing beds?

Viren Shetty: With or without further investments?

Prithvi: Without any further investment.

Viren Shetty: Without any further investment, you can do maybe high single digit revenue and margin expansion but eventually doc

tors will leave and the building will start to look shabbier and shabbier. We will have to re-invest in the rooms every 5-6 years in changing the equipment every 10-12 years, all of those things are required to generate slightly above that as well as expansion to meet the needs of the fact that so many patients are coming and we are running waiting lists. So without investment, this business can still grow for about a decade but then you will be a 'has been' in the industry.

Prithvi: I think my question is more on next couple of years. As there wouldn't be any new bed addition, how much more you can squeeze from the existing assets?

Viren Shetty: There will be minor bed additions, it's not that nothing is happening. The things that are coming up are brand new buildings but there are still a lot of work that are going to happen in the existing buildings that won't necessarily lead to bed additions. The net beds may stay the same. Certain beds will come up, certain beds will close down. Certain ICUs will come up, OTs, procedure rooms, OPs there will be configured. What's more important to stress is, our ability to keep taking in new patients will go up. So throughput and total numbers served in the same infrastructure will keep increasing till the new infrastructure comes where we suddenly have a whole lot of new capacity that will take you much higher.

Prithvi: On the margin side, what do you think about sustainable margins for India business?

Viren Shetty: It is the guidance we always give. We will do the best that we can and keep retaining our focus on providing extreme value based care, be very fair on the pricing and treat as many patients as we can. What we can deliver, we will continue to deliver.

7 Prithvi: Can I have the break-up of new hospitals revenue and margins for this quarter?

Venkatesh R: Yes, I will give you that information. So when it comes to new hospitals, these 3 hospitals which are SRCC, Gurugram, Dharamshila combined is at around 119cr. for this quarter against 115cr. in Q1 FY24. So it has grown decently and EBITDA has improved meaningfully from 6cr. in Q1 to 8.7cr. in this quarter. In SRCC Mumbai, we have been able to reduce our EBITDA loss from 2.2cr. in Q1 to 0.6cr. in Q2. We remain on track to reach flat EBITDA by the end of this year. We are moving in the right direction on the new hospitals, and we are confident of the group continuing to grow and deliver improving margins in this coming quarter and years. In Mumbai and in Gurugram, we are looking at a combination of both special ties and inorganic and organic initiatives to improve our margins. Dharamshila is already on track and hitting 15-16% margin and the others will also follow in the next 3-5 years based on the plans which we have worked out.

Prithvi: Ok. Anesh, couple of questions on Cayman business. One, obviously this quarter, there seems to be a fall on the footfall numbers on Y-o-Y basis. Could you just explain what

led to that?

Anesh Shetty: Hi Prithvi! Thanks for the question. Which footfall are you talking about the outpatient?

Prithvi: Yes, it's on outpatients.

Anesh Shetty: Just to clarify that if you are looking at the investor presentation, outpatients are 9,615 in Q2 FY24 compared to 7,609 in Q2 FY23, So, I am not sure if you are looking at the right information.

Prithvi: Ok. I will check out this.

Prithvi: Could you give some timeline on the new capex. I mean the new hospital in Cayman?

Anesh Shetty: So as we spoke about the last time as well, we remain on track to commission that in Q1 of the coming financial year. So that will be in the April to June timeframe, hopefully in the earlier half of the quarter. We are making meaningful progress in the civil work in that way, starting the service and a lot of interior designing as we speak. Of course, there is some time that

it takes for approval and group inspection etc.

Prithvi: Thank you. That's all from my side.

Nishant Singh: Any other questions? May I have the next question, please? Anuj, please go ahead.

Anuj: Thank you for taking my question and congratulations on a very good set of numbers.

8 So I just wanted to get some understanding in terms of the occupancy and other basic metric like, I know you don't track ARPOBS. So from an ARPP perspective, while today morning, in the commentary, you guys have said that there was not a material impact on ARPPs, so I just wanted to understand if you could give a break-up of how Bangalore, Kolkata and other non-core geographies performed in terms of occupancies because you guys used to give that data earlier but now I think, you have stopped. It would be helpful if you could share that again. Thank you.

Venkatesh R: For occupancy, to re-iterate, we don't want to give occupancy in isolation. As we are a high throughput center, occupancy actually is not so important a metric to assess our performance. As we have invested in technology to improve the throughput, the patient in some cases is able to come in the morning and go in the evening. We are also developing communication tools to improve our ALOS. Our occupancy is higher at India level, upwards of 65% and these improvements can be seen across all our units. The midnight occupancy measured on census beds is not a good measure of hospital utilization. In fact, the revenues which have grown more than double digit for 2 consecutive years without any meaningful bed addition is actually a proof of that.

Anuj: Thank you. While I understand that, I just wanted to understand, if there is a break-up of core and non-core geographies that you can provide? It's more of an academic question, I understand that and that's why I said, it's not just occupancies, I wanted to understand the other metrics as well. While I understand that ALOS is down from 4.6 days to 4.4, I just wanted a view on how the other metrics are performing?

Sandhya J: Ya, let me comment a bit on ARPP which we like acknowledge is a better metric than ARPOB. As you are aware, for us pricing is not the primary lever for improvement, and we focus heavily on efficiency. So, we take very moderate price increases and if you see, there is a certain year on year, we have shown a reasonable price increase which is I think healthy and yet affordable for our patients. Over a short period of time, these numbers fluctuate a bit and therefore they are not very representative quarter on quarter basis. Our ARPP numbers are shared in our deck. From a utilization point of view, there is just one more small color I want to add is, while we have improved on utilization across all our units, as you can see from our revenue mix, flagships are continuing to perform very well and therefore they are also running at very high utilizations. As we keep the bottlenecking, we will keep being able to process more throughput through the same facility that will help us use our resources better till our capacity addition comes up.

Anuj: Got it mam, thanks a lot. Thank you.

9 Nishant Singh: Can we have the next question please? Yes Gagan, please go ahead.

Gagan: Good morning! I hope I am audible.

Nishant Singh: Yes.

Gagan: So the 1st question is on your ARPP, Cayman Islands, out-patient are from ~\$1000 to ~\$1300, if I read it correctly. Year on year, let's say 30% jump. If you could elaborate a little bit on the significant ARPP jump, I understand possibly incurred at the onco unit might have something to do with it but if you could explain that?

Anesh Shetty: Yes, actually Gagan your guess is spot on. We classify radiotherapy patients as outpatient because there is no length of stay and it is a relatively much higher reali

zation than your traditional outpatient services. You will see that spike over there.

We don't expect any further increases because you know, the full effect of the radio

therapy is already born out in the quarter. But ya, you are right.

Gagan: So one, it stabilizes at one point the overall 1300 is what your indication ?

Anesh Shetty: Yes, that's a very specific number. It's not something that we will look to aim for but yes, where it is now is more or less where we expected it to be. There will be some other changes when we add more, as Venkatesh and the others commented, we are constantly looking to convert more and more services to daycare . So we will be starting daycare joint replacements and so on. So once that happens, you know obviously, that number could move but for now this is where it should be.

Gagan: Till there is a good jump in your outpatients' volumes from 7,609 in Q2 FY23 to 9,615 in Q2 FY24 . Again, that would also have a very substantial contribution coming from the Onco unit or that is a wrong inference?

Anesh Shetty: No, of course, oncology would contribute to it but by volumes , it's not very substantial. This is the general improvement in all specially but a large chunk of it is, we are now reporting our ENT, the acquired business as well which is pre -dominantly an out -patient service. So it's a high volume out -patient service.

Gagan: So you are saying that erstwhile, it was not reported in the op volumes and now it's being reported?

Anesh Shetty: We recently completed the acquisition and we wanted to start it after finishing an entire quarter and this is now reported. So it's a combination of our organic growth, the oncology which you identified as well as a large percentage of it would be because of ENT. It's there in the footnote as well, in footnote 1 of the slide.

10 Gagan: Ok, alright. And the ARPP for your in -patients actually dropped year on year in Cayman from USD 39,000 odd to 34 ,000 odd. Any explanations there?

Anesh Shetty: Yes, I wouldn't look too closely , just that similar quarter last year was an unseasonably high number but if you look at the general trend, Q1 was around USD 30,918 whereas we are at 34 ,000 now. So in general I think, the trend is positive and where we are comfortable being, it's just once in a while you are going to be comparing it to our base quarter which was unseasonably high.

Gagan: I mean in this part of the call you indicated almost INR 394cr. of capex is to be done.

For the full year, the budget is around 1,137cr. Your cash position is very healthy and so is the prostrate position and not really moved up too much from last year. Is there reason to believe that 2nd half because the capex might spike upto 700 odd crores from the 394 crores that we have seen, there will be requirement to add further to the debt and if so, to what extent will you need to fund it via debt?

Nishant Singh : Yes Gagan, so out of the capex pending for the remaining of the quarters , we will be spending around another say 450 -500cr. of capex. Out of that, we can fund 50 % of that money from debt. So even if we do take that 250 odd crores as debt, our net debt to EBITDA ratio will still be very comfortable at 0.3 to 0.4.

Gagan: Correct, this is not really too much of a substantial data addition at all. Now you had 2 years running to 100 0cr.+ capex. You managed to do that without really leveraging your balance sheet.

Nishant Singh : Because we have got pretty healthy cash flows . If you look at EBITDA, we have seen crossing 1000cr. on an annualized basis. So, with that in mind, it shouldn't be 0.5 at any point in time for this financial year.

Gagan: So last year I think your indication that cash flow was very healthy because of certain one-offs. This year again, if you could sort of give some idea , has the cash flow been

equally strong or better and even adjusted for the one -off?

Sandhya J: I think the cash flow is a representation of the underlying performance of the business.

Given that our EBITDA has been healthy and we've been able to maintain the pay or mix with a reasonable balance. Therefore, it is reflecting in the cashflows.

Gagan Thareja: Right. So, one is was 587 versus 535 last year, so there's no change in the Working Capital here at all?

Sandhya J: No significant change in the Working Capital this year. We kind of operate at almost neutral Working Capital position. So, we've been able to hold on to that in the current 11 year also.

Gagan Thareja: Correct. Again, at the start of the call you indicated that without any investment at all you can still manage to grow high single digit and you obviously are investing and investing quite substantially. You're reconfiguring, you're adding OTs, you're adding equipment, your grading equipment. So, just to take a cue from that statement, if without investing you can add or incrementally to sales by a high single digit with this investment, you know, part of it which is going to increase throughput and part obviously comes later in in brick and mortar in bed terms. Even with the investment that goes in increasing throughput, ideally, the minimum growth limit should be a high single digit and possibly supplement to that with whatever capacity expansion you're doing in terms of equipment. Is that a reasonable surmise?

Sandhya J: That's a forward looking kind of statement which we don't want to make but what we

can definitely say is that indicators are all in the direction of being able to grow as well as we've grown in the past. Obviously, there are various factors like, for example, Q3 is a seasonally weak quarter and so, therefore, every Q3 volumes are slightly weaker. There are various factors that come at play, so it's very difficult to give a number like that.

Gagan Thareja: So, I understand seasonally. I'm not looking at quarter to quarter movements, I'm obviously comparing year-on-year which adjusts for seasonality and in any case I'm looking at a 2-3 year's time frame and I'm just trying to validate the logic not looking for numbers. The final one on tax rate, I think, it's been pretty low for the first half and you had indicated it last quarter that this year will be effectively a low tax rate number. How should we budget for tax rate this year and then going into next year?

Sandhya J : Tax rate, we will be around, what we have seen, around 10%. That is a good estimate to take because we have moved to the new tax regime in India, so we will be able to see that benefit.

Gagan Thareja: And next year how should we think of it?

Sandhya J: Next year there will be no significant change except how the mix moves, which we cannot comment on at this point in time.

Gagan Thareja: So, next year we should budget for an effective 25% tax rate from India. Is that a reasonable way to think?

Sandhya J: Yeah, around 25% for India, you can take that number.

Gagan Thareja: Right. The final one, again on Cayman. ALOS for Cayman tends to be 8.9 to 9.1 days, 12 any reason why that number is as high as that because in India you are at around 4.5?

Anesh Shetty: Yeah, Gagan. So, in Cayman, we are sort of the national hospital for the country and there isn't much healthcare capacity aside from much on the government hospital. So, there are certain chronic patients who are very, very sick and who perhaps need longer term care, some amount of nursing. In other countries, they would be assisted at home, or they would have some other sort of homecare or out of hospital care but the government's obligation is to care for these patients and they're universally insured. So, we, on behalf of the government, admit these patients for a very long time, you know, many months at a time. So, this just skews the ALO

S number.

Gagan Thareja: Alright, thanks. I'll get back in the queue. Thank you.

Nishant Singh: Thanks, Gagan. Can we have the next question from Vinay, please?

Vinay Nadkarni: Yeah, thank you. Just one thing on your numbers for the quarter, I'm comparing quarter to quarter. You have around 212 new doctors added in this quarter but if I see the expenses on doctors it has actually gone down. Can I have some explanation on that?

Sandhya J: Because of the revenue effect, revenue has come in much higher. So, therefore, as a percentage it is not reflecting.

Dr. Emmanuel Rupert : Yeah, from the number point of view most of them would be training positions from the postgraduate training positions who have joined in the Q2. But we do keep adding senior doctors as and when required from every unit. So, that is a constant ongoing process that we follow. But for this relatively larger number, it is from the students who have been joining us in this quarter.

Vinay Nadkarni: Yeah, because the total doctor expenses have actually gone down from 2,167 million to 2,113 million. And the other part, is the Other Admin Expenses and Other Employee Expenses have also gone up disproportionately high while the others have been very well controlled and I must give you pat for doing a good job there in controlling your costs but any particular reason why the Other Employee Expenses have really gone up by around 14% quarter-on-quarter whereas your sale has gone up by 5.8 %?

Sandhya J: We have taken some investments in repair and maintenance in Q2 because it was a good quarter and we wanted to get some of the R&M work done. So, because of which there is a onetime investment and increase that you are seeing. It will taper down going forward. There is a certain level of base that will continue but some of it will taper down.

13 Vinay Nadkarni: Okay. Lastly, I see a drop in depreciation, any particular reason; quarter-on-quarter drop in depreciation?

Sandhya J: We even capitalized our work in progress. Any new significant capitalization has not happened and some of the assets as their life cycle ends they get depreciated. So, you don't need to overread into that number because our depreciation will go up. We are capitalizing 1,000 crores this year, so that will go up only.

Vinay Nadkarni: Okay. And the last thing is on the NHIC working, you have seen a good, healthy addition of patients in this quarter compared to the last quarter but your average billing has gone down from 1,538 to 1,158 and the losses have also, I think, increased from 5.8 crores to 6.4 crores. Of course, it is early days and it's hardly a year since you put that up but how do you see that contributing in future?

Sandhya J: I will request Ravi to take the question.

Ravi Vishwanath: Sure, thanks. So, yeah, I mean, as you said, it's early days and we keep experimenting with various value propositions to the customers, so I would think that for a little while this might go a little bit up and down as we come up with new propositions and new products. And so I don't, again, read too much into that. Right now we're focusing on making sure we have transactions and learning from our customers, understanding exactly what they need, what is the best value and best service we can give them and as our learnings grow over time, of course, directionally we do want to take it up. And I think over time it will stabilize but I think for the next few quarters this might be a little bit up and down as we experiment.

Vinay Nadkarni: Yeah. And how do you see the contribution of NHIC ? Is this a feeder into your main hospital? So, what exactly is the role NHIC plays here?

Viren Shetty : I'll answer this one. At this point, NHIC is a standalone entity that will build out clinics and offer subscription plans for patients with the idea that we want to keep them healthy and do checkups and manage them in the clinic itself

f. Obviously, once they are familiar with our system and get to know the brand better should anything happen in the future then there was a possible referral to the main hospital but that's not the primary goal. The doctors over there are trying to keep them out of the hospital and with the mission that's oriented on keeping them healthy.

Vinay Nadkarni: Yeah. Fantastic! Thanks a lot. Thanks a lot. That's all from me.

Nishant Singh: Thanks, Vinay. Can we have the next question from Chinmay, please?

Chinmay Nema: Good morning. Thank you for taking my question. While responding to one of the 14 participants you talked about inflationary headwinds and government action, could you elaborate that point? What specific actions are we referring to?

Viren Shetty : Usual things that happen before elections. There are in the past certain populist things around price control, certain states will offer expanded Ayushman programs. A lot of government departments tend to run out of money just before an election, so there are little headwinds that one usually faces just before a national election. That we're just a little cautious about in addition to whatever seasonal impact that Q3 will have.

Chinmay Nema: Understood. But nothing material, right?

Viren Shetty: Nothing material yet but as and when these things happen then we'll all get to know at the same time.

Chinmay Nema: Understood.

Nishant Singh: Any more questions, please? We still have around 20 minutes to go, if anybody has any questions please ask. Yes, Vinay.

Vinay Nadkarni: Yeah, just one more thing on the Cayman Island business. We are looking currently working at around 50% occupancy of the 110 beds that you are working there with? Because when I see the Average Revenue Per Occupied Bed, you are saying it is 2.3 million for a 31.5 million sale. Am I reading those numbers right or there is some error there?

Anesh Shetty: Could you please clarify which document you're referencing?

Vinay Nadkarni: It is the Slide Number 11, Point 2. The small note to ARPOB, you're saying 2.3 million for this quarter FY24 against our operating revenue of 31.5 million. I couldn't relate to it. I mean, how do you read this figure?

Anesh Shetty: Yeah. No, I think we will clarify this. So, your question is there's an ARPOB of 2.3 million over there but we don't have the occupied bed. So, how are you relating that to the revenue?

Vinay Nadkarni: Yeah, I'll tell you what I'm doing. I'm just looking at the numbers of your discharges and multiplying them by the average length of stay to say how many bed days are occupied out of the total 9,900 bed days that you had in this quarter and then working out what is the average stay there. When I divide, I don't get this 2.3 million per occupied bed.

Anesh Shetty: Yeah, that's because the Average Revenue Per Occupied Bed will include all revenue, inpatient and outpatient whereas the discharge numbers will only represent the 15 inpatient revenue.

Vinay Nadkarni: Okay. That's the only thing which I couldn't reconcile here, so I thought I would just check on it.

Anesh Shetty: Yeah, sure. No problem. No, it won't add up because you're missing a component.

Vinay Nadkarni: Okay.

Anesh Shetty: Nishant, you can just clarify

Nishant Singh: Yeah. So, Vinay, if you have any further doubts, we can get on a one to one call and we can clear this.

Vinay Nadkarni: Yeah, sure.

Nishant Singh: As Anesh said that this is not the only number and we have even the OP numbers but we'll give you more clarity in case if you want after the call.

Vinay Nadkarni: Sure. Thank you very much.

Viren Shetty : Outpatient in the sense that they get admitted in the morning and leave by the evening. So, they don't contribute to the inpatient but they're still occupying the bed for the whole day and count to the OP numbers.

Vinay Nadkarni: Okay.

Anesh Shetty: This is also exactly why we don't pr

efer ARPOB as a metric. But, yeah, to your question that's the answer. Yeah.

Nishant Singh: Yes, Vineet, can we have your question, please?

Vineet Kishore: Yeah, thanks. Yeah, first of all, congratulations on the great set of numbers. I think my question is also on the Cayman Island. So, how do we see the future of Cayman Island?

And is there any competition coming from Indian hospitals? That's one. And is the U.S.

Insurance, can that be used in Cayman Island now?

Anesh Shetty: Hi, Vineet. Yeah, thank you for your question. So, in terms of competition there is the government hospital, which is a very large tertiary hospital, and there's also another private hospital. It's a very active healthcare market. There are over 400 registered medical practitioners and over 100 different practices of varying configurations . So, you know, it is a market that is quite competitive in certain areas. We're not aware of the plans of any other Indian operators, we can't comment on that. It is a small market, and most players know the other one but we have no information and we can't comment on that. So, that's your first question. Could you repeat the second 16 one, please?

Vineet Kishore: On the U.S. medical insurance, can that be used for having a treatment in Cayman Island?

Anesh Shetty: Yeah. So, most insurers will cover if you're here on a holiday or if you're traveling here for other reasons and you require hospitalization. Almost every U.S. insurer will cover you. Now, in terms of a patient electively going, that isn't usually done but when there is a process for patients to apply to the insurer to do that and in most cases , we have seen it being approved simply because the costs here are so much lesser than the U.S. But it's a very unusual, fringe use case aside from an emergency use of a travel insurance sort of benefit.

Vineet Kishore: Thanks a lot. Thank you. Clear.

Nishant Singh: Yes, Gagan, can we have your question now?

Gagan Thareja: Yeah, I mean, referencing the previous question on ARPOB, if I look at your operational slide on India, Slide 9, the ARPOB for India is also indicated at around 13.4 million for Q2 this year versus 12.1 million Q2 last year. I'm just trying to clarify, are you annualizing the ARPOB figure and then giving it out? because 13.4 million ARPOB, if I, then consider it as an annual number and then divide by 365, the ARPOB comes to 34,000 -36,000 odd which looks more sort of understandable or am I getting it wrong?

Nishant Singh : Yeah, we annualize the number, Gagan. Your reading is correct.

Gagan Thareja: Okay. So, the same would have been done with the Cayman number as well; your annualized number like USD 2 million odd?

Sandhya J: Yeah.

Nishant Singh : Yes.

Gagan Thareja: Alright, I get that. And on CapEx, while your slides indicate last year, I think, if I look at last year's presentation and even this year it indicated a CapEx of 1 ,000 odd crores but then if I sort of try and tally it up on the Balance Sheet, you know, by looking at how much gross block has moved up by and how much capital work in progress is moved up by and sort of take the difference between the end of FY22 and FY23, probably comes to 700 crores rather than 1000 crores, if I have got it correct. So, basically last year then has the CapEx been 700 odd crores or 1,000 crores ?

Sandhya J: It could also be because of some of the equipment in the pipeline. So, we would have raised orders, and the equipment would still be landing but from a commitment point 17 of view that money is spent. So, the amount we are reporting are all committed CapEx spends, which will be capitalized . You should give us six months' lead time for us to be able to capitalize and show those numbers in the books.

Gagan Thareja: Okay. But if you sort of committed, does it not come up in CWIP?

Viren Shetty : It won't line up exactly. The intent is that all things planned, we want

ed to spend

about 1 ,000 crores last year and this year but lot of orders got delayed, a lot of construction work got delayed and so it didn't exactly line up. So, there's been a bit of lag in that. We're trying to catch up but it won't be exactly 1 ,000 crores added to the Balance Sheet.

Sandhya J: But if you look over a six month time frame, it will start reflecting because the cash is spent. It's just getting capitalized or coming into CWIP at different points in time.

Gagan Thareja: Okay, fine. Last one on Cayman, you indicated that you'll start the new hospital, I think, in the first quarter of next financial possibly, if I got it correctly. How should we, I mean, because this is going to be different from the onco unit that you've put in which is ramped up fairly fast for you, how should we look at the financial implications of that new hospital phase by phase let's say for the first six months, next 12 months and then when it stabilizes what time will it take to stabilize and hit an optimal utilization? And at that level, what implications does it have financially for margins and growth? If you could, you know, give us some indication of the roadmap of how this will move, it will be great.

Anesh Shetty: Gagan, your question is very specific and unfortunately my answer has to be quite vague. But, you know, it is a new hospital, as we've discussed before, there will be a decent chunk of fixed cost that will come online when we commission the hospital. It is not a hospital in a new location, I mean, in a new country. It's in the same location, patients know us. So, the ramp up, we do expect it to be reasonably fast. But having said that it is obvious that there will be margin dilution until we truly see incremental revenues coming in. How long that will take and quantifying that is something, you know, we're not in a position to give out at this moment.

Gagan Thareja: Right. Will you be able to run it with the existing doctors that you have in your existing hospital or you'll incrementally add? And if you will add, can you give some indication how much staff - nurses, doctors, paramedics will you need to add? And how will you be, you know, sort of recruiting them? Will you be sourcing them from India and how you go about it?

Anesh Shetty: Yeah. So, to the first part of your question, So, definitely a lot of these services we 18 offer are similar to what we're already offering. So, it will be the existing doctors whose output will increase. A lot of departments will need, you know, the second or third line of doctors to come online to handle the increased volume, some don't.

Nurses, obviously, are directly correlated with the number of patients we treat.

There's not really any operating leverage there. The more patients we treat, which we will be, we'll be hiring more nurses.

To your question about sourcing, we'll follow the usual sourcing routes we've been following for our existing hospitals, which is a combination of our network in India and other relationships in the region closer to this part of the world as well.

Gagan Thareja: So, just curious to know, I mean, for your staff who goes from India to Cayman, do they sort of go for a specified tenure and then return and then you roll between staff and some new set of people going after a stipulated period or do they stay there till they serve you in there?

Anesh Shetty: Yeah. So, the senior people are more or less constant. They're here for, you know, it's a longer term relocation and this is more of a permanent or a longer term plan. Certain front line staff and junior people, they're here for certain times of, you know, in their career in their personal lives when this makes sense and after a couple of years they go back home or they cycle out but that's the junior staff. Most of the senior people would be more or less constant, at least that's our intention. Yeah.

Gagan Thareja: How does your sor

t of attrition at Cayman in your junior staff or in your nursing staff compared to your India?

Anesh Shetty: Yeah, definitely, nursing attrition isn't as much of a problem as it is in India because once you're earning, how much these people earn in Cayman, there aren't many places where you can earn more. Having said that, there is attrition mainly to the U.K. right now and the reason for that is not compensation. In fact, our compensation would be a little higher than the U.K. It's just an issue of post-tax compensation. It's just a concern around, you know, in the U.K. there are more opportunities for their partners and spouses to have, education, there's a more clear route to citizenship and a longer term security for them whereas here for a nurse those things are going to be difficult. So, we do have attrition but not as much as it is in India.

Gagan Thareja: I mean, do you sort of facilitate education for the children at Cayman to a certain degree? Is that something you can do and sort of help them retain or retain them longer?

Anesh Shetty: Yeah. It's not the cost of education, it's just that in Cayman for a nurse it's next to 19 impossible to get long term citizenship over here. They'll not fulfill the criteria whereas in the U.K. they will. And in Cayman if their spouse is not a nurse and if they're working there's very little employment opportunity outside. It's very difficult. Whereas in the U.K., those things are much easier. So, you know, all factors considered such as family, personal, et c.. for quite a few people, the U.K. is holistically sometimes a better option. So, they pursue that. But wherever we can to make this more attractive, we do that.

Gagan Thareja: Thanks, I will get back in the queue. Thank you.

Nishant Singh : Thanks, Gagan, for your questions. Can we have the next question from Alankar, please?

Alankar Garude: Hi, thank you for the opportunity. Just two questions. Firstly, would it be fair to assume that the sequentially higher margins in this quarter is also a function of higher margins at Cayman due to pick up in the Radiology block?

Sandhya J: It's a mix of both. Yes, Alankar, we have had seen good performance in India as well as in Cayman.

Alankar Garude: Understood. And the second question is, So, when you talk about improving throughput, are the efforts more skewed towards Bangalore and Kolkata given the relatively higher occupancies there or these are uniformly happening across our entire India network?

Venkatesh R: So, it's like we are just not concentrated only on these two flagships. All these initiatives in terms of technology is our core plan across the group, it happens irrespective across all the regions where we exist, which is not South, East or West but everywhere and there is nothing unique to only flagships in these initiatives.

Throughput technology efficiencies are driven uniformly across all our units in the group and not specifically only to Bangalore and Kolkata .

Alankar Garude: Fair enough. Yeah, sorry, go ahead.

Dr. Emmanuel Rupert : Yeah, the impact of the throughput changes is felt more in the larger units and the flagships. So, that is the stuff that we have noticed. In the Heart hospital, the same facilities, same infrastructure of ICUs and the operating rooms, earlier on like pre -covid we used to do around 500+ cardiac surgeries and now we are able to do around 870 - 900 cases with the same infrastructure.

Alankar Garude : That's the outsized impact ?

20 Dr. Emmanuel Rupert : It's just the impact becomes very big.

Alankar Garude: Understood. Yeah, that answers my question. Sir, thank you and all the best.

Nishant Singh: Thanks, Alankar. Do we have any more questions? Last five minutes. Yeah, Kapil.

Kapil Marwaha: Yeah, hello. Congratulations on a strong performance in the September quarter. I would just like to know if this traction of strong performance has continued so far in the

present quarter, October to December, of which we are already in the middle?

Sandhya J: Q3, Kapil, is a seasonally weak quarter as you are aware because there are a lot of festivals that come in Q3. We have Durga Puja, then we have Diwali and then we have Christmas coming on. So, for all hospitals , Q3 is relatively weaker. So, standing today we can only say that, anything else will be forward looking.

Viren Shetty : Lot of these things get pushed forward. These are mostly where we do a lot of elective cases. So, we've seen that patients tend to postpone their procedures to Q4.

Kapil Marwaha: Okay. Thanks.

Nishant Singh: So, if we have no further questions, we would like to conclude our session. Thanks everyone for your active participation as usual. Please do feel free to call us, reach out to us in case of any further queries. Thank you.

End of Transcript

Call: Feb 2024

Date of submission: February 22, 2024

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code – 539551 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051
Scrip Code- NH

Dear Sir / Madam,
Sub: Transcript of Earnings Call for the quarter ended December 31, 2023

Further to our earlier letter dated February 16, 2024 in relation to uploading the Audio Recording of the Earnings Call of the Company held on February 16, 2024 (Friday) for the quarter ended December 31, 2023, please find attached the transcript of the said Earnings Call.

We wish to inform you that the Earnings Call transcript is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earnings-call-audio-and-transcripts>

This is for your information and records .
Thanking you

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S .
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q3 FY24 Earnings Conference Call"

February 16th, 2024

MANAGEMENT : MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – GROUP CHIEF OPERATING OFFICER ,

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS
SUBSIDIARY HCCI

MR. RAVI VISHWANATH – CHIEF EXECUTIVE OFFICER , NHIC

MR. NISHANT SINGH – VICE PRESIDENT , FINANCE , MERGERS & ACQUISITIONS & INVESTOR RELATIONS

MR. VIVEK AGARWAL – SENIOR MANAGER , MERGERS & ACQUISITIONS & INVESTOR RELATIONS

2 Nishant Singh: Good afternoon, everyone. My name is Nishant Singh, I head the Investor Relations function at Narayana Hrudayalaya. I welcome you all to Quarter 3 FY24 earnings call of the company.

To discuss our performance and address all your queries today, we also have with us Mr. Viren Shetty, our Vice Chairman, Dr. Emmanuel Rupert, our CEO and MD, Ms. Sandhya Jayaraman our Group CFO, Mr. Venkatesh – our Group COO, Dr Anesh Shetty, MD of our overseas subsidiary HCCI (which is Cayman), Mr. Ravi Vishwanath, CEO NHIC, and Vivek Agarwal, senior manager in the IR function. We hope you have gone through the investor collaterals which have been uploaded on the stock exchanges as well as our website. As usual, before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website as well as on the stock exchange at a later date. I would also like to remind you that everything that is being said on this call that reflects any outlook for the future or which can be construed as forward-looking statement must be viewed in conjunction with the uncertainties and the risks that they face. Post the call, should you have any further queries, please do not hesitate to get in touch with us. We would like to answer them to the best of our ability. With that now, I would like to hand over the call to Dr. Rupert.

Dr. Emmanuel Rupert: Good evening, everyone. I warmly welcome you all to the Quarter 3 FY24 Earnings Call Conference of Narayana Hrudayalaya Limited. After a robust Q2, the third quarter of the fiscal year delivered steady performance across our units. Consolidated revenue for the current quarter stood at INR 12,036 million reflecting a growth of 6.7 % year-on-year and -7.8% quarter-on-quarter. NHL generated Consolidated EBITDA of INR 3,968 million in Q3 FY24 at a margin of 24.7% against 25% in Q2 FY24. This quarter-on-quarter growth was impacted on account of seasonal factors related to festivities in the current quarter. Our Cayman units, the HCCI as well as the EICL continue to deliver strong business performance with quarterly revenue at USD 30.6 million, a year-on-year growth of 8.5%. We are confident that our Caribbean business will continue to grow through strategic initiatives and investments.

The balance sheet and liquidity profile at the group level remain strong with group cash and liquid investments of over INR 10.39 billion against gross borrowings of INR 10.14 billion resulting in a net cash position of INR 0.25 billion as of 31st December 2023. Our net debt-to-equity ratio now stands at -0.01, giving us sufficient room to fund our expansion through a mix of borrowing and internal accruals. We have incurred capital outlay of close to INR 53 billion till the December quarter and are on track to spend the balance amount in the remaining quarter of the fiscal year.

On the clinical front, the Health City campus successfully performed 27 organ transplants, 44 bone marrow transplants, 90+ robotic procedures, 60+ robotic ortho procedures, and two very rare renal transplants in patients with nephrocalcinosis which is a genetic disorder. The Children's Hospital in Mumbai successfully treated a child with super-refractory status epilepticus that is convulsions continuously happening on this child, which had an expected disability of 80% or a mortality of 50%. The patient recovered to a combination of medical management and a procedure which resulted in a recovery without any disability. Our focus on digitization and business transformation continues to lead to significant benefits throughout the NH system. We have seen an 8% quarter-on-quarter increase in throughput and a 3.3% reduction in turnaround time across NH Labs because of the initiatives taken in our ATHMA Platform. Adoptive app called the AADI has helped saved 2500 surgeon hours

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annually with digital OT notes. Our new app for nurses called NAMA H has saved 83 nursing hours due to digital handover process in the first month at the HCCI campus. Its India launch will be in February. We just launched that in the Health City campus and in our other tech initiatives, our OPD paper reduction has reduced by 22% in this quarter and there has been 75% EMR adoption in OPD across the group. Our recent venture, the Narayana Health Integrated Care continues to perform well despite a seasonally weak quarter. The revenue for the quarter has crossed INR 53 million with more than 42,000 patient transactions. We will continue to grow this business and serve our customers with clear focus for improving their health outcomes. We continue to upgrade our clinical and nonclinical operations across the group, transform the patient service levels, increase our throughput, build more capacity, invest in more digital patient outreach channels, and improve our operational efficiency. We are reasonably on track on our ESG commitments and continue to focus on

creating meaningful social impact in addition to pursuing our environmental goals and upholding the highest standards of governance. We are simultaneously pursuing organic and inorganic growth opportunities both in India and overseas that will derive synergies from our existing operations, maximize value for all our stakeholders while keeping a close watch on the return on capital. Thank you and I hand it over to Nishant.

Nishant Singh: Thank you, Sir. I will request everyone to now use the Raise Hand feature to start posing their questions. Prithvi, please go ahead.

Prithvi: Sir, my first question is on domestic business. Compared to the previous quarter's growth in revenue for domestic has been quite low. So, just wanted to understand what extents this low growth in this quarter?

Venkatesh R: Yeah, see this quarter like the routine Q3s are always affected by seasonality impact, but our North and West regions had a higher seasonality impact than expected. There is some underperformance in Gurugram, which we are working on, but the major impact has come from Jaipur where due to change in the structure of reimbursements for medical management of the RGHS, which is the state scheme, it's become unviable for us to service that business, but the Association of Hospitals are approaching the government for renegotiation, should be sorted in three months. We have also taken a conscious effort to improve our payor mix, which is the cash and insurance, which has gone higher by 2%. Schemes have come down by 2%. It will take some more time for the same capacity to get filled up by higher paying customers. The improvement will show over a period of time, but the positive thing is this is reflecting in our improved margin profile despite our difficult quarter. Also, this payor mix rationalization is an effort to not carry large corpus of unpaid bills and to have a better handling of our cash flows as we get into this election year plus we have exited M S Ramaiah and Bellary contracts to improve our focus on our strategic focus. The impact of MSR also has some bearing on this, but not a major thing which we are confident overcoming in the coming quarters.

Prithvi: Could you quantify the impact from the Jaipur Hospital?

Sandhya J: We're not specifically sharing information relating to individual hospitals. You can see the overall North number. North has three units Gurugram, Jaipur, and Dharamshila, which are predominant. Now, if you see that Dharamshila has been reasonably okay in terms of performance. So, the underperformance in North has largely come from Gurugram and from Jaipur; more from Jaipur, less from Gurugram. So, that should give you a broad idea on the impact of the Jaipur unit.

Prithvi: And there is one question on the Capex, could you give the timeline for the new hospital i

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Cayman and then also you know the Greenfield and brownfield projects in Bangalore and Kolkata?

Sandhya J: The Capex for the unit in Cayman is almost complete. By the first half of next year, we will be able to Commission the hospital. Bangalore and Kolkata units will take two to three years cycle. We have managed to acquire the land in Kolkata. We already have land in Bangalore, so we have now applied for permissions to build in Bangalore. So, all of it put together, I think another two to three years, you will see your Capex build up on all of these units. We are also looking at additional capacity options in Bangalore and Kolkata and more growth options. So, therefore as and when we have those opportunities materializing, we will be able to share more detail with you.

Prithvi: Just a bookkeeping question. What is the revenue and EBITDA number for new hospitals in this quarter?

Venkatesh R: Yeah, our new hospitals as I said have also suffered due to seasonality impact more than our flagship. On a combined level, their EBITDA is around 4% against 7% in Q2 with Dharamshila performing the best. We're hoping that the last quarter will see a strong recovery and we'll be able to get closer to our target for the next two quarters.

Prithvi: Okay. Thank you. That's all from my side.

Arvind: Sir, Hello?

Nishant Singh: Yes Arvind, please go ahead.

Arvind: What about your insurance business?

Viren shetty: Ravi, do you want to answer about the insurance?

Arvind: Yeah, you got the license?

Ravi Vishwanath: I would request others just to be on mute please. Yeah, we received our license in early January, and we are now in the process of putting together finalizing the product, finalizing technology, the regulatory policies etc., and as soon as those are in place, we would go live.

Arvind: Okay. When you are going to start it, means tentatively when you are going to start it?

Ravi Vishwanath: We expect to go live sometime next year and as soon as these, as you would imagine there are many things that need to be put in place and as soon as they are in place, we will go live sometime next year.

Arvind: Okay. I meant to say you are going to start PAN India or at Bangalore only?

Ravi Vishwanath: We will initially start in Karnataka in Mysore to start with and over time as we build our

experience and our knowledge, we will expand into other geographies.

Arvind: All the best for the new business.

Ravi Vishwanath: Thank you very much.

6 Nishant Singh: Yeah, thanks, Arvind. Yeah, Rishi can we have your question please.

Rishi: Yeah. So, I had a few questions. So, the first one when I look at your Cayman IPD ARPOB which is basically, I'm just dividing Cayman ARPP by the average ALOS that you all have of around 9 days out there. I see that it's kind of dropped vs last year. So, could you explain what's happening there? Why is your Cayman ARPOB for IPD business going down?

Anesh Shetty : Yeah. Hi, Rishi. Thank you for the question. If you actually look at the investor presentation, we give you the exact number split up into OP and IP in slide 10 operational review Cayman Islands. So, I think the last quarter was an unseasonally high quarter. In Cayman, our recommendation always is to look at three or four quarters for any metric to get a better sense of the direction these things are heading in . Because of the low base effect in terms of volumes, a few large discharges, a few complicated cases can skew things either which way. So, we recommend looking at more of three or four quarters for any metric and if you go by that, there isn't really any significant change if you think about it.

Rishi: Okay. So, because basically what I'm looking at is Q1 , Q2, Q3, right. So, Q1 FY24 you all were better off versus Q1 FY23. Q2 , Q3 – you all have seen a 10% decline. So, is it purely seasonality and nothing structural change there?

ere?

Anesh Shetty : No, definitely nothing structural. In fact, even if you, a 10% decline or increase for that matter in this metric really wouldn't concern us too much quarter -on-quarter. More or less if you look at say the last eight quarters or so, yeah, that gives you a trend of a gradual uptick as an d when we are taking on more complicated patients and this is, it's not something unusual for us to see these minor variations up and down.

Rishi: Great. And just a bookkeeping question on Cayman. What's the ALOS for this quarter? I think you all have forgotten to report it for Cayman?

Anesh Shetty : Well, no that was a conscious call. I think in Cayman we've decided that, that's not a relevant metric to talk about. I mean one gets a sense of the direction we're heading in because of the number of discharges, our revenue per patient, OP and IP as well as our total revenues and any other information that can be there. We have sufficient capacity in the hospital that isn't a constraint for us to grow, especially with the new hospital being commissioned in a few months as well. So, we felt that, that was not needed, but we're happy to answer any question you have for which you need that information.

7 Rishi: Yeah. So, basically it was just to see how much bed utilization you all are at, because despite your daycare procedures not being counted in that, the bed is not free if the IPD patient is staying there for 9 days. So, just to get a hold of that understanding, that's why I was asking for ALOS.

Anesh Shetty : Sure. Yeah, I can answer that. So, I think we have moved more towards daycare -oriented procedures especially with an acquisition with it recently and certain change in the approach of the way we're performing certain procedures. So, we've not had any constraint on that, there will always be some busy times and certain busy days or weeks with certain things like ICU capacity etc., but at no point in time are we foregoing treating any patients because of IP patient, I mean the availability of inpatient beds. At the same time, we're happy with the utilization. We're heading in a good direction and it's a good build up towards commissioning the new hospital in a few months. So, we're pretty comfortable with the availability of capacity.

Rishi: Got it, got it. On the India business, right, this year, what I'm seeing is that your discharges have come down to single digit growth even if I remove Western and Northern piece from not this quarter, but for the full year, so just wanted to get an understanding , is it because are we hitting peak capacity utilization out here or what's happening or was last year an exceptionally good year and hence the base is normalizing this year?

Venkatesh R : No, if you see , one is the discharge and the other thing is the ARPOBS , which is the ARPOBS have grown from INR 13.4 million to INR 14.1 million , even though the discharge numbers have gone down. This is on account of our conscious efforts in terms of working on improvement in payor mix, which we have already said. So, there is a 2% increase in the cash and TPA volume as compared to previous quarters. See, our growth drivers as we have always said will be on higher throughput numbers , through efficiency improvement, digital initiatives, leveraging on operational efficiencies. Even at most of the hospitals are trying to convert general wards into semiprivate to meet the demand for insurance and cash payors . So, while we are working aggressively on the throughputs, daycare procedures, there are lot of robotic procedures which are morning/evening discharge, there are cardiac surgeries which are morning/evening discharges, which won't reflect much on the discharge numbers, but it will reflect more on the ARPOBS and with this system and capacity default making, we are in a position to do more. We are in a position to serve more patients within t

he same

capacity. So, that's the answer for this.

8 Rishi: Okay. Got it, fine. The third question I had at my end was the other expenses for this quarter. It has come down on an absolute basis on a 1% year-on-year. So, what, where is the cost savings coming from? Is there a one off in the base year or this year which you want to call out?

Sandhya J: There are some one timers that come especially in Q3, we had a higher, we took actually a higher impact of R&M given that it was a good quarter and we did use it up to work on our R&M expenses and Q3 are being a little bit of a leaner quarter, we went a little more slow on some of those discretionary spends. That's the call that we keep taking periodically. There is no underlying exception to read there. It is in line with what we were expecting to spend.

Rishi: Okay. So, basically the 20 crores that you all haven't spent, 14 crores that you haven't spent in Q3 FY24, it will basically bunch up and there will be a 30 -crore expense in Q4 is what you're saying, right on repairs and maintenance?

Sandhya J: It may or may not be because we've kind of accelerated a lot of the work that we wanted to do in Q2 itself. So, we will, depending on how our needs of each of the units arise, we will moderate the spends, but we have, I don't think you will need to assume that every money that you have saved is a postponement. Some of it got accelerated to Q2, some of it will get spent in Q4. So, some of it is postponement, some of it is already spent.

Rishi: Okay, got it. In your notes to accounts in other income, right, you all have called out that around INR 159 million out of that INR 179 million is on account of lease modification. Could you just share more color on that, what exactly is happening there?

Sandhya J: So, we have certain lease arrangements, which we have with our third -party partners and sometimes we do periodically renegotiate these lease arrangements depending on either party requirements, convenience, etc. So, some of those renegotiations have reflected because of the way IND AS is it has reflected in some accounting entries coming because of that.

Rishi: So, these rental renegotiations have happened on land or building rentals or it's on equipment rentals, where is it coming from, like which hospitals, where, how is it happening and because it's nonrecurring in nature, right, if I have to

Sandhya J: It is nonrecurring in nature, yes. This will come in our non -owned hospitals. This will come in terms of the share of compensation we paid to the partners and in some places some 9 equipments are also rented. So, largely this comes because of the compensation that we pay to them.

Rishi: Okay. Got it. Got it. My final question that I wanted to understand is that your manpower costs, right year-on-year have been increasing at a high double -digit growth. Now, in a situation where ARPOB s are growing, yes, you can offset these, but what are we doing to control these costs, if you can give some qualitative understanding on that front?

Sandhya J: So, there are two aspects to manpower cost. One is that because a lot of the manpower cost comes in from people who are either minimum wage earning or linked to minimum wage and therefore government actions in terms of minimum wages etc. have a direct bearing and that results in a higher manpower cost burden and that is not going to go away. We are going to see more and more - we have to understand that the underlying inflation in the economy is high, which means the cost of living adjustment that's happening in the minimum wages across states is also higher. So, therefore that part of the wage inflation coming on the baseline will not moderate. How we are trying to tackle this is through efficiency in operations. Now, Venkatesh had spoken about earlier about how we are looking at more digitized delivery, we had also spoken about

our NAMA H app which we have

just gone live in HCCI and now we are starting to go live in Health City, which is going to help us be able to organize our manpower better. We are also looking at many other digital initiatives across our organization, which helps us deploy the right people in the right place and therefore we are able to handle our manpower costs effectively. It will be a journey. I'm sure you would be seeing this across all hospitals, but you will see it more for us because our price actions are minimal and therefore our growth in ARPP comes mainly due to efficiency actions. So, that comes a little slowly and therefore you see the impact more profound for us, but this is the reality of the situation. We have to work this through and we will be able to improve this over a period of time.

Rishi: Okay, okay, and what has been this wage inflation across say the last 10 years on average, how much wage inflation do you all see on the employee, the non -doctor employee manpower cost and the doctor and nurses manpower cost, if you can?

Sandhya J: For the current year FY24, we have seen anywhere between 10% to 12% wage inflation. We are expecting a similar number in FY25 also.

Rishi: Okay and historically it's also been double digit inflation for you guys on this front?

10 Sandhya J: During COVID time, I think the inflation rates were fairly moderated. After COVID this has got picked up quite a bit, but it's been peaking now in the last year and going into the next

year.

Rishi: But these won't get revised downwards even if inflation comes down, right. So, I'm. Just trying to understand like the business model is exposed to this, so I was just getting a feel of how this line item will move for you also?

Sandhya J: Yes, business model is exposed to this. So, like I said, you have to factor some amount of increase that comes because manpower costs will keep going up, but you'll also have to factor that there is a lot of work and effort being done on the digital side and that will help us optimize. It is similar to any other manufacturing industry or any other labor-intensive industry where you would have a wage inflation coming through and the recovery partially happens through efficiency and partially happens through price. So, that's really how our industry will also progress.

Rishi: Right. Got it. Alright, fine. Those are the questions from my end. Thank you.

Nishant Singh: Thank you, Rishi, for the questions. Can we have the next set of questions please? Any other questions anyone? Yes, Gagan, please go ahead. You're on mute Gagan. We can't hear you.

Gagan: Yeah. Can you hear me now?

Nishant Singh: Yeah, yeah, we can hear you now.

Gagan: Yeah. so, sorry if the question is repetitive. I got disconnected from the call. What was the reason for the India revenue growth being relatively muted this quarter, specifically North and West, but overall, also I think it was a bit muted in volume terms?

Venkatesh R : Yeah. It was, as we said this quarter, always has a seasonality impact as compared to other quarters. But again, the North and West regions had a higher impact than before. There was some underperformance in Gurugram, which we are sorting out. There are a little bit of an issue in terms of medical management bills in Jaipur and the structure of reimbursement from the government there, which is practically unviable for us to service that business for the time being. There are discussions going on with the government through the Association of Hospitals. We would be in a position to get positive results maybe in a quarters time, also we've taken efforts to rationalize our payor mix. The result is very clearly showing in terms of the cash and TPA going up by 2% this quarter, but of 11 course it would take some time for this capacity to be filled in by the paying customers. The improvement will show over a peri

od of time and that is the reason why there is a little bit

of, I mean there is a bigger dip in top line as compared to the Q2, but the positive thing here is as a result of our conscious effort in terms of rationalizing payor mix, improving process improvement, working on process improvement, adopting technology, it has reflected in terms of our improved margin profile despite a low and difficult quarter. Also, we don't want to expose too much of payables and have a better cash flow coming into an election year.

Gagan: Yeah. So, given the fact that a part of the growth is seasonal, I understand that but part of it, that growth crimp in the growth is also coming from you trying to deliberately alter your payor mix. Are we in a transition phase where perhaps we take some time before growth normalizes back to a level which would be optimal?

Sandhya J: That's how we are looking at it. However, like we have always said, we are very cautious on how we take these steps and how we move. The desire is that we want to make a healthy transition so that our capacity is reallocated to better ARPP and cashflows. But we will be measured and careful about how we go about it. So, you can assume that it will take some time but we will also balance it off to the needs on the ground.

Gagan Yes, I get that. But is it possible for you to delineate, let's say, what part of the impact is coming from these deliberate steps and what part...? Seasonality year on year should even out, I can understand quarter on quarter seasonality impacts you but compared to the same quarter last year seasonality shouldn't impact you. Given that, how would you attribute the growth weakness to these various factors that you've been talking about?

Sandhya J: Our healthy payor mix has moved by about 2%, so that gives you some idea of how the payor mix has contributed to a little bit of stagnation in the revenue growth. But it does not take away the fact that best we did have underperformance in North, especially little bit in West. And the other aspect is that we did a rationalization during this year with the MS Ramaiah, which was a key contributor in terms of our South cluster revenue, which also contributed a little bit to the overall impact; maybe a couple of percent impact came from the rationalization. I mean, I don't have these numbers exactly calculated because we have not analyzed it in that fashion, but you can attribute 2-3% to our maybe poor performance in North sector, maybe 2-3% you can attribute to the payor mix calls we have taken. So, you know, you can assume that kind of mix in terms of our performance.

Venkatesh R : So, these changes won't play out as a one off effort, you know, and then you say it's just a one off effort and then you are completed with the exercise. It's actually a constant effort 12 to apply methodical changes to the way our hospitals operate. It will keep showing up improved earnings, improved volumes, higher Average Revenue Per Patient year after

year. You'll not see immediate results, but it will be over a period of time.

Gagan You're saying that the tradeoff is that you want better average , ARPOB perhaps, our Average Revenue Per Patient, better Gross Margins and therefore these steps, while they might impact the topline for some time, they'll result in better margins. Is that how we should think about it?

Sandhya J: Yeah. One is, definitely there's an aspiration for better margins but right now we are more focusing on better cashflows and we don't want to deal with significant cashflow uncertainty at this stage. So, a lot of moderation is also happening from that perspective.

Gagan So, how does that impact your cashflow or Working Capital so to speak, Receivables so to speak because these are aimed at better Receivables, I presume?

Sandhya J: Yes, that reflect in our June quarter and September quarter Receivables because the payment cycle for many of these

schemes is 6 -9 months. So, at the moment like every other hospital in the December quarter, collections are normally very poor from government customers because they have budget freezes. The budget release happens in the last week of March, early April and that's when the cashflow start coming in. So, all we are trying to do is not create a significant load on the cashflow going into the first half of next year. Having said that, it's a very small rationalization. We still have nearly 1/5th of our revenue still coming from these payors. So, that impact will not go away but to some extent whatever we can do we are trying to do in terms of improving our cashflow mix.

Gagan Right. And while for the full year your Capex is budgeted at north of INR 1000 crores, I think year to date what you've incurred is around INR 450 crores. Is it going to be a case where some amount of that Capex , you know, the actual cash outflows get incurred in the next financial year? Or are you going to be...?

Sandhya J: Yeah, a little bit flow over will be there in the next financial year but actually cashflows may largely happen. Capitalization may happen in the next financial year once the commissioning happens. A significant chunk of this Capex is being spent in Cayman and most of the funds are committed and there is a payment schedule to the vendors based on which it's being paid out. So, most of the cashflows are being spent. I think you may have a one quarter flow over maybe to some extent because of the timing of the payable dates.

Venkatesh R : Also, since we are in the last quarter and a lot of these Capex is also attributable to the Greenfield expansions, like acquisition of land in Kolkata , plans for Greenfield in Bangalore.

So, we are pretty confident of spending major part of the Capex by the end of this March 13 and, of course, there will be a small spillover, as Sandhya has said, in terms of the routine replacement and upgradation Capex , which may have a little bit of a spillover to the next year but most of it will be utilized in the next 45 -50 days.

Gagan Right. And coming on to Cayman, I mean in the quarter, as far as I can understand, Cayman ARPOB looks YoY down by about 4.5 -5% and OPD volumes are also down and down, I think, by a reasonable number, if I recall it correctly from your presentation. So, any thoughts there?

Anesh Shetty: Gagan, hi. Can I just get some clarity on the question? So, if we look at Slide 10 of the presentation, the box on the bottom right talks about the OP numbers. So, Q3FY23, which I think you are referring to, is at 7 ,647 and Q3FY24 is 9 ,558.

Gagan: Okay, I might be wrong there. Yeah.

Anesh Shetty: Sorry, yeah. Yeah, let me know if there's any question then.

Gagan: Okay. On ARPOB, in Cayman is there a drop? If I understood...?

Anesh Shetty: Yeah, similarly, if you look at the box on the top left in the same slide, Slide 10, we answered this question to the previous gentleman as well. So, if we look at Q3FY23 the inpatient revenue per patient is closer to USD 4 million whereas in the current quarter it's USD 3 million . Although that seems like a drastic drop, what we suggest is always in Cayman to look at 3 -4 quarters or perhaps longer to look at a trend. And when you do that, you will see that it's pretty in line with more or less the number we've traditionally been at. The reason was Q3 in FY23 was an unseasonably high quarter for this particular metric because of a few discharges that were very high in value because the underlying volumes are low, it's a low base effect of a few big discharges skewing this particular metric. But if you look at a longer time series, you will see that this is not something that concerns us.

Gagan: And from an occupancy standpoint, how's been Cayman doing year -on-year?

Anesh Shetty: Sure. So, we prefer to disclose discharges, outpatients and those kind of productivity metrics, but you know occupancy

will change and can be skewed basis on the percentage of patients we focus on Daycare versus non -Daycare. So, in terms of the activity metrics, like outpatient numbers, discharges, etc., number of surgeries, so we have seen a good growth that you can see even in the series of similar presentations every quarter. A lot of that has been driven by more of a high throughput procedures of, especially in the outpatient services that we are focusing on these days.

Gagan: Okay. And when does the new facility come on board? I presume it will be next year but is it possible to understand exactly when next year are you scheduled to bring it on board?

14 Anesh Shetty: Sure. The answer would be a range but we are aspiring to do it closer towards the end of Q1 of next year. We have more certainty around when the facility will be ready in terms of its infrastructure but then there is a series of regulatory requirements as well that does have a broad range and we have less control over. But our intent is to start in towards the end of Q1. More specifically, definitely, it will be around June; June -ish, I would say, yeah.

Gagan: Okay. And the Oncology Unit that you commissioned this year, year to date any possibility to understand the contribution to the Cayman revenue and profits from that specific unit?

Anesh Shetty: Sure. So, nothing that I can share in specifics but more qualitatively the unit is doing very, very well; better than what we expected. We're pleasantly surprised with it. Radiotherapy in any geography, whether India, Cayman or whatever, is always going to be a high margin service from a P&L perspective because our operating costs are quite low in Radiotherapy and that Oncology facility as of now is only Radiotherapy. So, in that sense, it's definitely margin accretive, and we are happy with the volumes and revenue we're raising so far.

Gagan: Right. Final one from my side on the India piece as we head into the next year, is there a reasonable case for an early double digit sort of growth? I mean, would you be in any way constrained to achieve that? I'm not talking from a demand perspective, more from, one, your ability to service that kind of demand and, two, from the fact that you are looking to do something about changing payor mix. So, is that a reasonable possibility given these constraints or are we looking at a different sort of aspiration there?

Viren Shetty: I will answer this one.

Sandhya J: Yeah, Viren, go ahead.

Viren Shetty: Nothing constrains us other than our ability to not disclose what next year's numbers are going to look like. Yeah, Sandhya, go ahead.

Sandhya J: Yeah, thanks, Viren. So, there are a few aspects we have to keep in mind. One, of course, like you rightly highlighted there is a payor mix work which is ongoing, and we will take it as the year progresses. It's an election year, so we are taking some actions towards that but we may take some different actions after we get more confidence on cashflows. So, that's one aspect.

The other aspect is that we are working continuously on our throughput and that's been our journey. If you see for the past several years, we've not added a single bed. In fact, we have rationalized beds, we have repurposed beds, we have converted some of our General Ward capacity into ICU, Private Ward capacity. So, we've been rationalizing our business and improving our throughput on an ongoing basis. So, that should help us service the 15 demand as well. Having said that, we can't make any forward looking statements on where we are headed. Our journey is a little more long-term journey because we are not taking very immediate bed addition decisions, we are not doing any significant acquisition. We are buying land and building, so which means our next point of inflection will come when all the capacities go online and until then we have to keep doing more of what we've been doing for the past

several years to be able to deliver results. So, I don't know if I answered your question, but I think that's really how we're looking at it.

Venkatesh R : We have seen the pickup happening to some extent after the Q3 but it is a task cut out for us and we are pretty confident along with our efficiency drive, as Sandhya mentioned, we would also be able to build up on our volumes.

Nishant Singh: Gagan, any more questions from your side?

Gagan: Just one follow up, which is that, on the throughput. Since a lot of emphasis is laid on improving the throughput and operational metrics, is it possible to understand what are your aspirations there? For example, if with the same bed capacity next year with all that you have invested in increasing throughput, how much can you improve the throughput?

Dr. Emmanuel Rupert : Yeah, we are constantly working on this, whether it is a procedure driven or whether it is any of our processes of diagnostics, by increasing the workstations in the diagnostic space for Ultrasound, ECHOs and seeing more throughput in the CTs/MRIs and all these things so that we'll be able to service more because we are able to see the need from our OP EMR which we have an adoption, which is very high, touching 80% plus and we know that there is an opportunity for us to see the throughput increase in these areas and that is one area where we are working on. And while the procedure areas, whether it is Infusion Centers with our Onco program or the program with the Dialysis or various other Daycare procedures, we are trying to see how much we can do within the very shortest possible time. So, we're trying to do most of our Angiograms like a short stay within 4-6 hours kind of a period so that we'll be able to do 2 and, in some cases, even 3 shifts in the same bed which we'll be able to utilize.

So, these are all some of the things which we are working on. It's a constant work with the clinical teams and hopefully we'll master it in 2-3 quarters down the line.

Viren Shetty: So, the way you track it is to check on the ALOS and the footfalls that we're achieving per

quarter.

Gagan: Thanks, Sir. I will get back in the queue. Thank you.

Nishant Singh: Thanks, Gagan. Robert, can we please have your question?

16 Robert: Hello. Could you just talk a little about your longer-term capital allocation? So, when you're looking at your key markets, how you're seeing supply demand, the way you see the opportunities? And so, looking beyond the kind of the next 12 months, how you're weighing up the different opportunities?

Viren Shetty: Yeah, I'll answer this one. Our key priorities are winning greater market share in Bangalore and Kolkata. So, for the next decade at least the bulk of our investment will focus on these two geographies. The remaining will just be strengthening the existing hospital set that we currently have. So, that means adding more beds or adding adjacent capacity to the existing network. So, most of this will be in some combination of Brownfield and Greenfield investment in hospitals, clinics, pharmacies in the network of cities that we're currently operating on and this will be the case for the next easily 10 years.

Robert: Thanks. And just with that in mind, so when you're assessing the kind of supply side of the equation what are you seeing in terms of your peers actions and so on? And do you think that gives you a kind of a clear enough runway in that regard because it sounds like you do?

Viren Shetty: Sorry, by peers action do you mean...? Could you just clarify that part?

Robert: In terms of general hospital and clinic supply across those markets. So, if you look, I presume, you can see what your peers are up to a reasonable degree.

Viren Shetty: Yes, it's fair to say that most of the mature cities in India have an adequate supply of hospital beds but they are short of very high quality beds and so there is a shift in th

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preference for patients to move from the very large unorganized sector, which constitutes essentially 90% of all the private bed capacity in the country, and for them moving away from unorganized hospitals and unlicensed nursing homes to larger, more accredited hospitals. So, in that sense, while there is a crowding effect of most large corporate healthcare groups being focused in the large cities, it does not represent the true demand that exists among the patients in those cities and the demographics are in our favor with rising incomes, rising old age. We do believe that the opportunity set is tremendous.

Robert: Sir, do you mind if I just ask one follow on onto that, just in terms of how you're anticipating, for example, the insured mix versus out of pocket, et cetera? How you're expecting that to evolve over the next 5 years?

Viren Shetty: For us or for the sector?

Robert: Well, for you in particular.

Viren Shetty: Sure. As of now, it's about 20% -22%, it hovers between that range for organized payors 17 whereas for a peer group it's sometimes close to 40% -50%. So, our aspiration is to reach those numbers. I'm not sure that we could achieve it in 5 years, it may take us a bit longer just because of our patient mix and our legacy of being a provider of very low cost, subsidized services to large numbers of people but the aspiration is to get to what the peer group is able to achieve.

Robert: That's great, Thank you very much.

Viren Shetty: Thank you.

Nishant Singh: Thanks Robert. Abhishek, please go ahead.

Abhishek Hello? Am I audible?

Viren Shetty: Yeah, we can hear you.

Abhishek Yeah. So, my question is, I think there is a decline in the Q3 versus Q2. I know we might have already spoken about the seasonal effect, is the seasonal effect more pronounced in the northern and the western region where we are seeing decline or is a seasonal phenomenon across the country? That's my question, Number 1. Yeah, then I'll ask another question.

Venkatesh R : Yeah, of course, the seasonality is a little bit more. The impact of seasonal fluctuations is a bit more in the northern and western side of the country. Of course, seasonality has affected the other parts also, but it has affected us more in the North and West. Typically, most of these hospitals have a higher volume of scheme patients and mix in such hospital. Added to the fact that we are consciously trying to take the effort to improve our payor mix and the replacement is going to take time for these paying customers, this benefit will also take some period of time and that's the reason why there was a little bit of further dip in this. But if you look at the way in which units in these regions are performing, Dharamshila generally generates reasonably double-digit margins quarter-on-quarter, Gurugram also expect few aberrations. This quarter is always in a single digit margin, and it's also shown good improvement. And Mumbai also is expected to be positive in the late Q4 and then maintain several margins from then on.

So, though there has been a bigger fluctuation because of this reason but, I guess, they are giving meaningful contribution to the overall scheme of things and should be able to

maintain this.

Abhishek Sure. I guess you also talked about that bulk of your focus over the next 10 years is going to be the Bangalore and the Kolkata region, right; the Greater Bangalore and Kolkata region if I may say so. Do we have, let's say, a huge untapped market in these two regions that we have a pretty long runway or is it because in the North and West we don't have a strong brand presence? Like what's the reason of like focusing on the Southern region and Eastern region?

Viren Shetty: So, two reasons for that. One is that the Bangalore and Kolkata hospitals that we run are all full and so you naturally build capacity where you're sure you're going to get all the patients to fill it up.

There is huge demand in North, huge demand in West, even more demand in other parts of Eastern Central India. It's just that it will take us longer to fill up those beds while the investment it's essentially the same; the construction cost in any part of India is roughly the same. So, the confidence that we have in filling our beds and generating quicker returns is much higher in Bangalore and Kolkata. That's one of the, let's say, the most non-emotional, spreadsheet driven reasons why we would invest in these markets. The other one is that these are the places where we have the highest brand recognition, highest Market Share but we're constrained in being in very few locations around the city. There was a time when a lot of patients would travel for days to come see us or they'd be willing to wait a very long time to get an appointment with some of our doctors. That's not the case anymore and so we are not able to hire more doctors, we're not able to accommodate more people in the same current setup. So, we have to add more capacity to accommodate all the patients who want to see us but are not willing to wait 4 - 5 days in order to do so.

Abhishek Understood -understood. And then from the capacity expansion standpoint, I guess, we have a very good Debt -to-Equity Ratio, could we not have expanded or done more Capex ?

Just a question on that because I see that we do have the requisite sort of comfort there.

Viren Shetty: Oh! We absolutely want, I mean, I'm the biggest proponent of pushing for more Capex but then Sandhya's job is to hold us back. So, we do what Sandhya says.

Abhishek Alright. Yeah, those were my questions. Thank you so much.

Nishant Singh: Vinay, please go ahead with the question.

Vinay : Yeah, can you hear me?

Nishant Singh: Yes, we can hear you.

Vinay : Yeah, just quickly, in terms of going forward areas of business like your Oncology and the Heart part, which other areas are you seeing expanding into or would you like to focus on only these two major ones? I'm in bulk of your business?

Dr. Emmanuel Rupert : Yeah. So, though these constitute a major portion of our clinical work . Orthopedic, Spine, Neurosciences is one area where we are constantly looking at opportunities for growth and 19 that is also more or less a community based growth because usually patients because of mobility reasons will come from a 50 kilometer radius for all these kinds of work. While GI Sciences and Acute Care, which constitutes Emergency Care and the Critical Care, forms a major portion of our work. So, these are the areas where we are constantly looking at growth on this.

Vinay : Yeah. My second question was, you had mentioned that you would like to be a lowest cost like not lowest but comparatively lower cost service provider, and you would want to match that with some specialty kind of procedures which would gain you more revenue with less time spent at the hospital. So, is there any kind of tradeoff between these two because I understand your service culture does prompt you to go for more value - added services?

Dr. Emmanuel Rupert : Yeah, if you look at it, Neurosciences is got a combination of more of OPD practice and most of the procedures now are becoming minimal access and endoscopic and different kind of procedures. It's not necessary when you say that all those days of neurosurgery being very large; ghastly surgeries are no longer there. A lot of things work on an interventional neurology platforms and things like that which has got a very minimal stay. So, we do look into all these aspects but when we give a service you can't give a patchy kind of a service because patients come for a variety of the spectrum of requirements across a specialty and we need to be there and we have the ability and have the clinical bandwidth to service them. But these are not something that will be in large volumes but will give a lift to the department so that even the routine work comes in large numbers for us.

So whether it is GI sciences or neurosciences these are all the same, these are all very short stay you know minimal access work that keeps happening. Critical care is a very crucial element because if you are going to look at emergency care and acute care , critical care becomes a crucial element then it is not necessary with all these advances and sciences that they will spend. There is a portion maybe 10 -12% or 20% of the patients will

spend time but a lot of them recover very quickly nowadays.

Vinay : And one last question, is there any geriatric care planned given the demographics moving?

Dr. Emmanuel Rupert : We do have in our flagships people who specialize in geriatrics so that they don't land up with 5 -6 cross consultations across multiple departments, but they will take into account specialists as and when required. But by nature of the way the spectrum of work happens geriatrics is a reasonable volume that we deal with in all our centers .

Vinay : Thank you very much.

Nishant Singh : Rishi, do you have any questions. Abhishek, any more questions? You have raised your 20 hands.

Rishi : Yeah, so I will just go ahead with my question . Viren, you just mentioned that you know the demand scenario has changed over the last few years for you guys where earlier patients used to come from far off, they used to wait for days or weeks for your doctors' appointments. Now that's not the case anymore. Doctor talent retention is becoming tougher. So, could you give more of an understanding here what's happening because from what I understand your value proposition is very high. So, a patient who cannot afford does he have more options now and that too closer to his city or what's happening here, if you could just give an understanding.

Viren Shetty : We are in a big city, the options for most normal kinds of surgery so gall bladder, caesarian section, any sort of compound fracture those are almost universally available within walking distance of your house if you are in a big Indian city meaning the top 10 cities of India. But that's not necessarily the area that most corporate hospitals play in. For them it is about being there for the heart surgery, cancer surgery, very advanced procedures. But it doesn't mean that those procedures don't happen, they absolutely do happen in large numbers. But then that tends to be driven by convenience. So , a lot of the investments we are making in increasing our foot print , in looking at the Greenfield expansions is to be able to cater to a large segment of diseases. We are building also a large number of clinics which we historically never did, so that their first point of entry for whether it is even for a cough or a cold has to be within our network. So, since we built an integrated plan around that then we see that there is a lot more value in sticking to one network provider for all your issues and we will have all information, we will be able to treat you much better and we will have a much better idea of the longitudinal view of what happened to you in your life . So, in terms of the demand, yes, it is currently being fulfilled in quite a large range of hospitals of different kinds of quality standards. The preference always is that people get it done at more accredited places and we want to be able to cater to that.

Rishi : Alright, so basically you are going closer to the customer but how does this play out in terms of ROCE, is it dilutive for you guys or accretive.

Viren Shetty : Short term, very dilutive, the thing is no patient came here and asked for my ROCE and I can't pay the doctors' salaries in ROCE. So, it is yes, it is something that has been definitely a point of concern for us which is why we don't do very large acquisitions and we take things in a much more moderated way. But it is the only determinant for how we decide to spend our

money. In a perfect world we will just stick with the network we have and just keep on flogging it till the day the building falls down. But that means that you are foregoing a lot of opportunities which we don't want to either. So, in terms of establishing a footprint, building out significant capex for the next couple of years since we have the borrowing ability to do so and it won't , you know, we have the cash service ability as well, we will do that till we are no longer able to expand any more in which case we operationalize all the things that we 've built and wait for things to improve again.

Rishi : Alright.

Venkatesh R : I mean, if you even categorize the way we service , one at the national level , one at the local level , and one at the hyper local level. We need to also have solutions across all the three categories. So, when it comes to quaternary care which we have in these flagships we look at catering to national level, even at international level and when it comes to tertiary care like neurosurgery, orthopedics, we have solutions at local levels. Also , when it comes to hyper local levels the obstetrics, gynecology, pediatrics, we have hospitals which cater to this level. So, you need to be available to provide solutions to each of these categories and that is where we are working towards.

Rishi : Got it, got it. Thank you.

Nishant Singh : We have a lot of questions which have been typed because there were some issues with the speakers of the participants, so I think we should take them now. There are questions from Utkarsh and Riddhi basically on the capex spending for this, yeah basically on the capex spending for this year and next year. So , I will take that question and maybe Sandhya can probably add. See, for capex for FY24 we had a projection of around INR 1100 crores and we have already spent close to INR 500 crores, and we have the POs also lined up, added to that we have this Rajarhat expansion for Kolkata where we have capex of roughly INR 180 crores lined up for this quarter. And we also are in the advanced stages of

procuring a land in Bangalore so if we add it up all I think we should be there at around INR 1000 crores and slightly above for FY24. For FY25 we don't have a finalized plan but given that we will be growing by adding hospitals and clinics at a lot of places, especially our flagships, you can take a rough estimate of around INR 1200 crores even for FY25. And as Viren has already mentioned very clearly that we will be mostly expanding in our flagships which is Bangalore and Kolkata in India. So, a lot of our capex specially for the brownfield and greenfield will be towards these two cities, the bulk of it and Raipur as well. For Cayman, as Anesh also mentioned we will be done with this new hospital construction by say first quarter of FY25. Beyond that we don't have any greenfield plans for Cayman, but we will keep exploring other inorganic expansions in the other nearby islands and other international locations. Anything else.

Sandhya J.: Yeah, maybe I will take the rest of the questions.

22 Nishant Singh : Yeah, sure, Sandhya.

Sandhya J: Thanks, Nishant. So, the question on pay or mix like we said this we do want optimize payor mix in the long term but what we are doing right now is rationalizing for cash flow and as well as trying to see if we are having the ability to correct the payor mix. We don't want to guide a target on this because there is a segment we want to serve as well. But we want to find the right balance, so we are not guiding on the payor mix.

In terms of our patient s' response to our services, I think one measure we could look at is say Google reviews. We have on an average the industry top tier players would be at an average of about 4.6 kind of number. In most of our hospitals, flagships and large hospitals it is about 4.8 and across a large number of review counts. We are b

elieving that whatever

actions we are taking in terms of improving the patient service outcomes it is being appreciated by our patients, but it is a constant journey and we will have to keep working on it. In terms of comparing our procedure cost versus our peers, I think a good way to compare this rather than we trying to put out some number other there, a good way is for you to be able to derive this, the costs are almost exactly the same for every player, any large player. I am not sure if we can say that we will be buying something cheaper than another large, listed player. But if you look at our realization profile it is relatively lower to many of the top 5 players. And at the same time our cost profile is at par with the top player so that's the reflection of the efficiency and scale at which we play. So, that should give you a broad comparison on how we are operating. In terms of how much it costs for building a hospital with land prices being premium and depending on where we choose to build it will cost anywhere between 1.5 to 2 crores. If the land cost is very, very high then it can even go up beyond that. But approximately including everything it is costing or going to cost 1.5 to 2 crores per bed. Our initial capital investment for health insurance we have invested INR 100 crores in the health insurance business, and we are going to start with that and this will run us into the next year and a little bit after, and after that we will see how the investment will build on. As far as the question on tech, maybe I will request Viren to take that question. The question is that you have developed a lot of tech internally.

Viren Shetty : We do have a patient facing app called NH Care, we are in the process of rebranding it. Essentially, we want this to be the one-stop shop for accessing all of your hospital-based services. Its capabilities currently include being able to book appointments, pay for any tests that you order, see the results of your tests, get any bio of all the doctors that we have and be able to check on the status of any of your people /your loved ones who are already into the hospital. The roadmap to this is eventually to roll this into our NH Integrated Care Plan which would then allow you to buy a Care Plan, be able to coordinate with your Care Coordinator on the app so that they would remind you of any upcoming whether it is checkup or appointments or any tests or medicines you want to order that can be done on the app as well. And you can use it to keep track of, ultimately when the insurance rolls out, you can keep track of your insurance, the sum insured and all those sorts of things. So, patient facing app has you know got a lot of good reviews, it is still got to be worked out but like all things a lot of work still needs to be done. We are also trying to roll in QR codes into the apps so then when you are in the hospital, this becomes sort of the fast track for you to go to the different parts of the hospital to pay for things, identify yourself at the entrance and act as a sort of passport for you within the hospital, that also is work in progress.

Nishant Singh : Even medical research wing.

Viren Shetty : We do have medical research wing, I will ask Dr. Rupert to talk about it.

Dr. Emmanuel Rupert : We have got a fairly robust medical research wing headed by a very good clinician who was a cardiac surgeon but he has worked in the Bristol University and has got a major interest in research. He works as our Chief Scientific Officer. And recently one of the professors from Vellore having major interest in immunology and research has joined us. They form part of the leadership team and are working on disciplinary research. And last calendar year we had in excess of 200 plus publications, with these publications are having an impact

factor of more than 1, and which have been published in the PUBMET acc

redited journals,

and these are not some journals which are having a lower impact journey. And we have various incentivizing plan for people who take time out for research and publications and presentations across the network. But we have a very good ecosystem in the H ealth City campus for basic science research in addition to clinical research. But as we are doing more and more electronic medical records and almost 100% of our OP EMRs, OP consultations are getting captured. The ecosystem for various kinds of trials , outcome analysis and survival scores after 2 years, 5 years after multiple kind of therapies is all going to be available in a couple of years ' time because we have a very good data capturing. The more data that gets refined the better the outcomes. And since we have distribution of patients coming from a vast segment across the country it is almost like doing multicenter trials within our own organization, you don't have to go outside the organization to do any kind of research. So, the ecosystem is for basic sciences research in addition to the clinical database , and we also have a tie-up what is called a redcap database for doing these clinical trials. And all our units have access to good journals from Elsevier's Clinical Keys and upto date and clinical decision support system which we have online and which is available to all of them even from their homes. So, they have an ecosystem for this, and we have 24 demonstrated for the last 3 years consistently we have very good publications in journals which have an impact factor of more than 1 , more than 200 plus this one. And that is something which we have been constantly working on . As of now industry funded research is in excess of 80 across the entire network. And main areas of research are in apart from cardiac sciences has been in oncology, neurosciences and critical care.

Nishant Singh : So, I think we have been able to answer all the questions, even the typed ones now. If anything is pending from the typed questions, can the participants again request. I think we are done with the questions.

Viren Shetty : Gagan, do you have a question?

Nishant Singh : Yes, Gagan.

Gagan : Thanks for the follow up. Just two questions, one on the tax rate as we get into the next year - we have had a low tax rate this year, how does it work out for the next year. And second, is the capex for the next year also budgeted at a INR 1000 crores if I heard it correctly?

Sandhya J : Yes, go ahead, Nishant, go ahead.

Nishant Singh : As we mentioned earlier capex for next year is not finalized but it should be roughly in the same number of around INR 1000 -1200 crores.

Sandhya J : Tax rate for next year will be similar, Gagan, except that this year we had some benefit accounting benefit in Q2 given that we shifted to the new rate and some of the deferred tax liability is reversed. Adjusting for that we should have a similar rate. So, essentially most of our units are now on the new tax regime except the NHSHPL which is we still have some brought forward some losses set off only after that it can move to the new regime. And depending on the mix between India and Cayman our effective tax rate will get moderated.

Gagan : Right, so as far as funding for next year's capex is concerned so far you know we managed to retain a net cash position with another INR 1200 crores for next year, how do you propose to fund it.

Sandhya J : So, we will do a mix of bank borrowings as well as we are planning to do an NCD raise. And some internal accruals also we will use. We don't expect to be in this very low debt EBITDA at least for the next 2 -3 years because we have stepped up the pedal on our capex , over the past two years we have spent close to INR 1000 crores and we want to continue to do that. So, which means that these will not come from organic cash accruals. So, you will see a higher ratio on debt EBITDA in the

medium term till you know all the investments that

we are making start returning money and given that we are prioritizing buy land and build 25 more greenfield so therefore the cycle is also going to be longer in terms from the time we make the investment to the time the cash flow starts coming in. So, you have to expect that a significant amount of the capex is going to be funded through debt.

Gagan : So, what's the debt to EBITDA levels you are sort of looking at as a ceiling or a peak which you don't want to breach .

Sandhya J : So, we historically wanted to stay at a ratio of 2 but then we also are aware that if there are meaningful opportunities which we have to go after and if there is in the short term we need to breach the ratio. We may not go after the opportunity because of the ratio. We will be balanced about it, but we have to breach it in the short term , we may take those calls depending on what opportunities come our way.

Gagan : Any further idea you can give as to where exactly is the INR 1200 crores being targeted, you've given the split for this and last year's INR 1000 crores each, any further details that can be offered on next year's INR 1200 crores .

Sandhya J : I didn't hear the question because I had a drop of connectivity.

Gagan : So, what I was asking is for the composition of that capex for this year and last year you have given a break down of the capex in Cayman and India and what exactly are you sort of doing with it. So, is it possible to understand for next year the INR 1200 crores of capex which you propose where exactly are you putting it.

Sandhya J : While we are still working on the details, maybe I will request Nishant to take this question.

Nishant Singh: Sure, Sandhya. See, Gagan, we have this normal capex of biomedical, civil for all the hospitals which is around INR 400 crores to 450 crores, so we can take a 10% jump on that so INR 450 crores for general capex. And anything apart from that within that gap of INR 1200 crores will be for the expansion which could be a mix of expansion in Bangalore, Kolkata and a bit of that in Raipur as well . So that is a broad split.

Nishant Singh: For Cayman also we will have regular capex and the last mild portion of the new hospital capex which will be around INR 150 crores. So, that is the broad split of the INR 1200 crores planned capex for next year.

Gagan: So, a final one from my side on the Sparsh acquisition that you did last year. What has been the ramp up like ? And if I recall correctly one of the reasons also was that it will help you create additional or you know sort of transition some capacity in the Bangalore facility to Sparsh and create some additional capacity at the Bangalore hospital. So, just wanted to understand how is that working out. And second one what's the status on your clinics and insurance program if you could update us there.

26 Venkatesh R : First one I will take which is the Sparsh, Sparsh has completed 5 quarters post -acquisition and has been moving the way we had projected it and doing very well in terms of margins it is working similar to the margins which we have in Health City. We have had a lot of benefits by acquiring Sparsh in terms of cost rationalization because since it is located in the same complex, we have been able to leverage the manpower of the existing campus into Sparsh to work on savings and manpower cost. We have been able to leverage on economies of scale and efficiency in consumption to have savings in consumption cost. And also in terms of the overheads. And as a part of capacity de-bottlenecking Sparsh has also helped create additional capacity in our main centers as an add on to the already executed program on improving operational efficiency throughputs and all. That has actually created basically added more virtual beds because since the inception of Sparsh over the last 5 quarters. So, this quarter of Spars

h has also been as per plan and as per the budget .

Gagan: The second one?

Ravi Vishwanath : Yeah, the second question was on the integrated care and insurance. So, these are still early days we continue to focus on, rather be the pilot in Bangalore, we may progress over the course of the quarter, we have added a clinic and would be adding more in the current quarter as well. And we are focused on building our product services and adding to mix. In terms of insurance, as mentioned earlier we expect to start operations sometime next year.

Gagan : A final one if I am allowed to, and this would be on Cayman, you will start the new hospital in June. How are you looking at you know the scale up of that one and the implications that has for the existing one specifically for FY 25 -26 if you could give a broad idea.

Anesh Shetty : So our intention is to hopefully start by June. I mean these things can get delayed a couple of weeks basis on factors beyond our control but that's the aim. In terms of how it will impact , so, as we discussed during the previous call as well we are going to be front loading a lot of costs because we are commissioning a new hospital. So, there will be some front loading of costs that hopefully will be mitigated fairly soon because it is not a new geography. So, although it is a new building, a new campus, it is still within the same island. The market knows us very well, it is a relatively small market we have been around for close to a decade. We don't foresee it taking very long to get people flowing through the facility in reasonable numbers. Having said that we do unfortunately have to frontload all our fixed costs. Cayman as a market in general there is a very high degree of operating leverage which explains the profitability but at the same time when we are ramping up it also explains the high fixed costs in the beginning. So , we hope to climb over those fairly soon.

27 Viren Shetty : I think we have one last question. After that this call automatically terminates. Rishabh?

Rishabh: Yeah, this is building on the previous response, you mentioned around INR 400 to 450 odd crores of routine capex that NH has. Is this only for India business or this includes the Cayman business as well.

Nishant Singh : This is largely towards the Cayman business - for India business, sorry, Cayman business we do not have too much of general running expenses . Most of the capex for Cayman is towards the new hospital construction. When we say INR 400 crores, majority of that is towards India business.

Viren Shetty : And this is a work that is already in progress across the different hospitals as part of their overall transformation and expansion activities.

Rishabh : Understood. One last question, what would be the overall impact of IND AS adjustment that we do for rent in India business as well as Cayman business?

Sandhya J : I think that is disclosed as part of our investor deck, we did give a footnote of adjustments.

Rishabh: So, INR 18 crores is what the overall impact, is there some way to bifurcate it into India, what would be the India aspect of it.

Sandhya J: You can assume all of it for India actually, you know, there is nothing material from IND AS point of view in Cayman.

Rishabh : Okay, understood. Thanks.

Nishant Singh: I think with this we would like to conclude our session. Thanks everyone, for your active participation as always. Please do feel free to reach out to us for any further questions you may have. Thank you.

End of Transcript

Call: May 2024

Date of submission: 31st May 2024

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code – 539551 To,
The Secretary
Listing Department
National Stock Exchange of India
Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051
Scrip Code- NH

Dear Sir / Madam,

Sub: Transcript of Earnings Call for the quarter and financial year ended 31st March, 2024

Further to our earlier letter dated 27th May 2024 in relation to uploading the Audio Recording of the Earnings Call of the Company held on Monday, 27th May 2024 for the quarter and financial year ended 31st March, 2024, please find attached the transcript of the said Earnings Call.

We wish to inform you that the Earnings Call transcript is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earnings-call-audio-and-transcripts>

This is for your information and records.

Thanking you

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

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"Narayana Hrudayalaya Limited
Q4 FY24 Earnings Conference Call"

May 27th, 2024

MANAGEMENT : M R. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – GROUP CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – GROUP CHIEF OPERATING OFFICER

DR. ANESH SHETTY – MANAGING DIRECTOR, OVERSEAS
SUBSIDIARY HCCI

MR. RAVI VISHWANATH – CHIEF EXECUTIVE OFFICER, NHIC

MR. NISHANT SINGH – VICE PRESIDENT, FINANCE, MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. VIVEK AGARWAL – SENIOR MANAGER, INVESTOR
RELATIONS

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Nishant Singh: Good afternoon, everyone! My name is Nishant Singh. I head the Investor relations function at Narayana Hrudayalaya. I welcome you all to the Q4 FY24 Earnings Call of the company. To discuss our performance and address all your queries today, we also have with us Mr. Viren Shetty, our Vice-Chairman, Dr. Emmanuel Rupert, our CEO & MD, Ms. Sandhya Jayaraman, Group CFO, Mr. Venkatesh, our Group COO, Dr. Anesh Shetty, MD of our overseas subsidiary Cayman, Mr. Ravi Vishwanath, CEO of NHIC and Vivek Agarwal, Senior manager in the IR Function. We hope you have gone through the investor collaterals which have been uploaded on the stock exchanges as well as on our website. As usual before we proceed with the call, we would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website as well as on the stock exchange at a later date. I would also like to remind you that everything that is being said on this call, that reflects any outlook for the future, or which can be construed as forward-looking statement must be viewed in conjunction with the uncertainties and the risks that they face. Post the call, should you have any further queries, please do not hesitate to get in touch with us. We would like to address them to the best of our ability. With that now, I would like to hand over the call to Dr. Rupert.

Dr. Emmanuel Rupert: Good evening, everyone. I warmly welcome you all to the Q4 FY 24 Earnings Call conference of Narayana Hrudayalaya Limited.

We are pleased to report the highest ever revenue and profitability margins for the financial year 2024. This was aided by improvements in realizations, payor mix, and increased patients' footfalls during the year. Consolidated revenue for the current quarter stood at INR 12,794 million, reflecting a growth of 4.7% year-on-year and 6.3% quarter-on-quarter. Narayana Hrudayalaya Limited generated a consolidated EBITDA of INR 3,184 million for Q4 FY 24, at a margin of 24.9%, against 23.8% in Q4 FY 23. Consolidated operating revenue for FY 24 stood at INR 50,183 million, reflecting a growth of 10.9% year-on-year, with a consolidated EBITDA of INR 12,275 million at a margin of 24.5%.

HCCI Cayman continues to deliver strong business performance, with a quarterly revenue of USD 30.5 million a year-on-year growth of 3.1% and a financial year revenue and EBITDA of USD 123.9 million and USD 58 million of EBITDA, respectively.

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We are confident that our Caribbean business will continue to grow through strategic initiatives and investments in the coming year.

The balance sheet and liquidity profile at the group level remains strong, with group cash and liquid investments of over INR 12.58 billion against gross borrowings of INR 14.41 billion, resulting in a net debt position of INR 1.84 billion as of 31st March 2024. Our net debt to equity ratio now stands at 0.06, giving us sufficient room to fund our expansion through a mix of borrowing and internal accruals. We have incurred capital outlay of upwards of INR 9 billion directed towards transformation of existing hospitals, repairs and maintenance, bio medical expenses and greenfield expansion projects.

On the clinical front, we continue to do cutting edge work and Health City, the Cardiac Hospital in Bangalore successfully performed 19 TAVI's, 19 robotic cardiac surgeries and 255 Minimal Invasive cardiac surgeries during the quarter, and more than 1000 Minimal Invasive cardiac surgeries during the year.

The RN Tagore Hospital in Kolkata performed a Trans-axillary Perceval Plus Suture-less aortic valve replacement using Central Cannulation technique, one of the first in the country. The Health City, Bangalore has successfully performed more than 16 solid organ transplants and 89 robotic surgeries in the quarter. It is also performed a bone

preservation surgery with a 3D pri

nted model. It also performed an ovary preservation surgery for a cancer of the ovaries.

Our focus on digitization and business transformation continues to lead to significant benefits throughout the Narayana Health System. We launched a comprehensive purchase model in our ATHMA platform which will streamline supply chain management operations across the group. Our nursing app, called 'Namah', was launched in India in February, resulting in savings of 5000 person hours till date. More than 95,000 discharge summaries were certified by doctors on the 'Aadi' mobile app, helping us discharge patients sooner. We have seen a 19.73% increase in lab throughput handled through our 'Athma' alias the Lab Information System. We have also introduced patient kiosks in three of our hospitals in order to streamline the patient transactions. The monthly active users count for our Narayana Health app has crossed 200,000, while the app getting a rating of 4.8 on the Google Play Store.

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Narayana Health Integrated Care continues to perform as per a growth plan. Revenue for the quarter crossed INR 68 million, the highest so far, with more than 43,250 patient transactions. We will continue to grow this business and serve our customers with a clear focus on improving their health outcomes. We continue to upgrade our clinical and non-clinical operations across the group, transform the patient service levels, increase our throughput, build more capacity, invest in more digital patient outreach channels, and improve our operational efficiencies.

We are reasonably on track on our ESG commitments and continue to focus on creating meaningful social impact in addition to pursuing our environmental goals and upholding the highest standards of governance. We are simultaneously pursuing organic and inorganic growth opportunities, both in India and overseas, that will derive synergies from our existing operations, maximize value for all our stakeholders, while keeping a close watch on the return on capital. Thank you.

Nishant Singh: Thank you, sir. I would request everyone to now use the 'raise hand' feature to start posing the questions. Ya Prithvi, please go ahead.

Prithvi: Viren, obviously, you know, if we look at the India business numbers for the last two quarters, ARPOB continues to increase at almost 10%, whereas the number of discharges has been on a declining trend. And it's not, I mean, looks like across all the hospitals of your network, there has been a declining trend of discharges. So, just wanted to understand, how do we see this and how exactly management is thinking about this change that's happening?

Viren Shetty: Thanks, Prithvi. I'll let Venkatesh start then I'll add to it.

R. Venkatesh: Yeah. So, if you look at Q4 of last year, FY 23, there was a significant upside due to post COVID rush, which you've seen across all the sectors. I mean, across the sector. So obviously when we're comparing with this quarter and the quarter before, we are obviously comparing against a very high base. Plus, there are also issues in terms of our infrastructural transformations, which we are working on changing the bed configurations into private, semi-private rooms, which has reduced bed availability at peak admission times, but things will normalize once we are ready with this. And of course, even when we focus on payor segments, it has helped us improve our IP average revenue per patient to our highest level of 1.31 lakh. But we have seen some

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volume rationalization even there. But this will get corrected. So, while we are focusing on these three major areas, including the payor segments, we have seen the highest ARPP. But resulted in some volume rationalization, which will definitely get corrected over a period of time.

Prithvi: So, if you look at the India business revenues, I mean, you know, earlier it used to be at 14-15% per annum. Obviously, that's a high number, that's not su

stainable. But the last

two quarters number of 4 to 5% growth how do we see this moving forward? I mean, will it touch a high single digit or 10% kind of number, or will it be lower at these levels?

R. Venkatesh: See I already told you when we were comparing this with this quarter, we are comparing it to the very high base of last year Q4, that's why this 4.7%. Because it was at a higher base. But in spite of this we have had good healthy bottom line. But of course, when it comes to the revenues, we would continue to work on our efficiencies and meaningfully utilize our existing capacities through a combination of throughput increase, payor mix, digital initiatives, and try and work on double digit revenues till our expansions kick in. We are pretty confident in terms of that.

Prithvi: Got it. Moving on to Cayman looks like even in this quarter, I mean, even Cayman had a

weak revenue growth in this quarter. So, just trying to understand, Anesh you know what's happening there?

Anesh Shetty: Hi. Hi, Prithvi. Thanks for the question. So, as you know, we are very, very close to commissioning the new hospital and this is perhaps the last quarter before we get started with the commissioning, the new hospital. So, we will see that slowdown in the number of new services we can add or the capacity or the number of new patients we can attract, given the existing challenges with the existing location, with capacity and location, more importantly but that's why we started the new expansion. And this is just, you know, a few months before commissioning the new hospital, which we are hopeful of seeing about 3 or 4 new service lines that we've never had before. And we will see the result in growth from that.

Prithvi: And when is the new hospital likely to get commissioned?

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Anesh Shetty: The inauguration will be in July. We'll be treating patients soon after, hopefully in early August, that's the goal. The inauguration is a set date because that's within our control. But when we can actually start seeing patients, there is a few weeks of uncertainty given certain inspections and compliance sign-offs that have to happen, but our aim is to start it in August.

Prithvi: So, one last question on Cayman, I mean, the average revenue per patient seems to have declined sharply in this quarter from 35.3 US thousand dollars to 29.1. Any change in the case mix that happened in the quarter?

Anesh Shetty: So, like we suggested before Prithvi, we prefer to look at longer term trends in Cayman and not quarter-on-quarter because a few large cases can skew that. But if you see the longer-term trend, there is a slight slowdown in the average realization per patient and this is because when we were at an annual revenue of around 70, 80, 90 million USD, we were predominantly restricted to few very high end, high complex special cases. But as we continue to grow, we will be getting into special cases and services where each patient yields lower, but there's decent volume and there's a viable business there. So, the per patient realization will gradually come down. As we do more daycare and as we open the new hospital and we go more into short stay quick turnaround procedures, this will continue, but this is not something we look at as a negative way. It's just a different case mix, so to say as we grow our revenue.

Prithvi: Okay. That's clear. Thanks.

Nishant Singh: Thanks, Prithvi. Vinu, can we have your question, please.

Vinu: Hi. Good afternoon I was looking for a little more detail on your bed expansion plans especially in the light from your PPT, I can see that the greenfield capex project for FY 25 is pretty high at thousand crores. So, where is this mostly getting invested? And if we could get a rough sense of how many beds are coming online over the next two, three years and when, that would be great?

R. Venkatesh: Yeah see, our expansion plans would be focused mainly on our centres in Bangalore and Kolkata, more

so in Bangalore going ahead than Kolkata. When it comes to Bangalore plan, it includes your expansion of the existing Health City campus on the land which is owned by us, where we will start construction of more than 3 lakh plus

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square feet and we will have a mix of rooms, ICU's, OT's multi-level car parks, state of the art setup. We've also acquired land in Rajarhat. We've taken the position we expect a 3 lakh plus square feet construction also to start up now by the mid of the 3rd quarter. We've also recently acquired one parcel of land in the upmarket Bangalore location, for which the construction should commence within the next 5 to 6 months. We are also in the process of shortlisting a few more land parcels for greenfield projects in areas where we don't have presence in our focus cities. So, all in all, we will have more than 8 to 10 lakh square foot of constructions coming up from the mid of or to the end of 3rd quarter. So, these will be the major expansions, which will happen in the due course of time. And apart from this, there are plans for capex of around INR 300 crores plus in routine biomedical and maintenance, plus an additional INR 250 crore for our new Cayman facility, which is in the end stage.

Vinu: So, when you say 8 to 10 lakh square feet do you mind translating into number of beds?

Sandhya J: We don't particularly want to do that equation yet.

Viren Shetty: The designs are still, at this point, very much being finalized right now in the split between the different departments. So, it's not a final number on the total number of beds yet.

Vinu: Understood. So just to get a context how many max square feet is your total of all facilities as of today?

Viren Shetty: I mean, we'd have to do a lot of math on that. No, we'd have to get back to you on

that. It's hard for me to tally all of that.

Vinu: Okay. I'm just trying to get you know, put the size of the expansion in context. Just to extend this question so you said the construction is set to start some time mid to Q3 this year, so can I assume most of these beds would be coming online say about 2 to 3 years from now?

Viren Shetty: Closer to 3. Yeah.

Vinu: Closer to 3. Okay. Understood. Perfect. Thank you. I'll join back.

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Viren Shetty: Yeah.

Nishant Singh: Thanks, Vinu. Yes Sukantha, can we have the question, please?

Sukantha: Hi, am I audible?

Nishant Singh: Yeah.

Sukantha: First of all, good afternoon, everyone. My first question is the company have guided that the company is doing investment of INR 850 crores. So, my question is, does it include the investment in subsidiaries?

Sandhya J: INR 850 crores, number you've taken from the capex sheet in the IR deck, is it?

Sukantha: No, not from the capex. It's the investment the company is doing.

Sandhya J: See, the capex that we have guided was about INR 1000 crores. We slightly underspent on that. So, that's probably the INR 850-crore number you're looking at, that does not include the investment in subsidiaries. The investment in subsidiaries is the cash burn that we have shown in the P&L side. Between NHIC and NHIL we have had a INR 12 crore of cash burn. So, that's pretty much the investment if you want to look at so far.

Nishant Singh: If you're talking about this, if there is a key balance sheet item of around INR 840 crores. Is that the number which you're talking about in the current investments?

Sukantha: Yeah, I think it was 840.

Nishant Singh: Yeah. So, that's an investment into, say, normal FDs and short-term liquid instruments, which is based out of both Cayman Islands and India. So, that's our liquid investments. It's cash.

Sukantha: Oh, okay.

Nishant Singh: Yeah.

Sukantha: My second question is, SHPL, specifically Samyat Healthcare Private Limited, a subsidiary of Narayana Healthcare, since it is supplying only to the Narayana group.

Please, can you help me understand, what are the added benefits the company is

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getting by forming

the subsidiary that the company is otherwise not getting by procuring from the market?

Sandhya J: This subsidiary helps us manage our supply chain better, because we are able to optimize stock holdings between different legal entities in an efficient way. So, it helps us with better stock holding, and it also helps us in better tax planning between the entities because this is a full GST entity. So, we're able to take credit and as and when units needing this stock, we can dispatch it on payment of tax. So, that way it also helps us manage the supply chain in a tax efficient way.

Sukantha: Okay. My next set of questions are regarding the greenfield project you are coming up with new town Kolkata. So, my first question is, land acquisition cost for the setup?

What is the land acquisition cost?

R. Venkatesh: Nishant, do you want to take it?

Nishant Singh: Yeah. So, for the Kolkata we paid INR 180 crores including registration cost for the land which we have purchased in Q4.

Sukantha: Okay. Thank you. And by what time are you expecting the break even for the setup?

Nishant Singh: See the construction will finish by say, 3 years, FY28 and as per the working, which we have done, we have an estimate of around between second and third year for the hospital to break even, EBITDA wise.

R. Venkatesh: These are set up in areas where you have a lot of confidence and brand presence. So, it won't take much time for a break even once we commence the facility.

Sukantha: Okay. And what is the capital employed for the project?

Viren Shetty: Sorry. Could you repeat that?

Sandhya J: Could you repeat the question, Sukantha? We didn't follow.

Sukantha: What is the capital employed for the project? The new town one.

Nishant Singh: Can we have your question again Sukantha, please? We were not able to hear it properly.

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R. Venkatesh: He's saying capital employed for this project?

Sukantha: Yes.

Nishant Singh: For Rajarhat?

Sukantha: Yes.

Nishant Singh: See, we have an estimate which is for 1000 beds, which is around maybe, say, INR 1000 crores for the next ten years. So, we don't have a split of how much will it cost in the first phase, I mean, we can't give it a phase wise, but that's the overall estimate of cost which we will incur for the next ten years.

R. Venkatesh: Overall, it will be a 1000 bedded project with a approx. capex of INR 1000 crore, which will be in phases. It may be in 2 to 3 phases at a max. But then, of course, we don't have the breakup of the total investments phase wise. We'll come back to you on that. But overall, it will be 1000 crore outlay for 1000 bedded project.

Sukantha: Okay. And what is your expected capacity utilization for the first year?

Viren Shetty: We won't be able to disclose that at this time.

Sukantha: Okay, okay, I understand. Okay. And can you please help me with your expected ARPU from this project? Will it be slightly better, or you are expecting it to be in line with the average ARPU?

Viren Shetty: It will be in line with our Kolkata business.

Sandhya J: ARPU is also driven by market factors, Sukantha. So, therefore this project is going to come like online after three years. So, it will also be driven by the market realities at that point in time.

Sukantha: Yes. That's why I asked for your expected ARPU, it's okay then.

Viren Shetty: Right. Thank you.

Sukantha: I have another question.

Viren Shetty: Sure.

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Sukantha: What time are you expecting it to be operational, the project?

Nishant Singh: Rajarhat will commence by FY28, the first phase.

Sukantha: Okay. Thank you. I have no more questions. Thank you.

Viren Shetty: We have some questions in the chat. We will address that first before we move on to the others. One is a question on Cayman seeing a significant increase in outpatient from 7,000 to 10,000. Does it translate into higher inpatient volume in the quarters to come? Anesh, if you could answer that.

Anesh Shetty: Yeah, sure. So, increase in outpatient

volumes will to an extent over the quarters, result in higher inpatient services. But what we really are focusing on is increasing the day care services, so, you know, endoscopy, colonoscopy, CTs, MRIs, pharma sales, all these day care activities for us, which the new hospital is also very focused on. And we currently classify all those kinds of services, even day care discharge surgeries under outpatient. So, the increase from 7,000 to 10,000 in outpatient is something very intentional and it is on track and as per our plans and we'll be seeing hopefully more of that with the new hospital.

Viren Shetty: Thanks. Nishant will answer the cost relating to the insurance business baked into the current quarter P&L.

Nishant Singh: Yeah. See we have mentioned in the footnote the expenses incurred for this NHIC and NHL businesses put together. So, that's a number of around INR 11.5 crores for the two businesses put together. Majority of that goes towards the clinics business.

Viren Shetty: The last question is about , we had some issues in Jaipur last quarter due to a change in the insurance procedures of the state. Venkatesh can answer. The question is, has it been resolved?

R. Venkatesh: So, this is a work in progress but what we have done is the unit has come a long way in replacing these medical management payments with a lot of procedure and surgical payments. The total surgical counts in this unit has gone up drastically in the last quarter.

Of course, we still have regular discussions happening with the government in terms of overcoming these issues. But when it comes to a drop of approximately one and a half to two crore on medical management revenues, the unit has come a long way in

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recovering at least one crore out of that. Over and above the surge in the surgical specialties, the unit has shown a very good growth in top line of more than 12% and more than 40% jump in the EBITDA in the last quarter as compared to Q3.

Viren Shetty: Okay. So, before we get to Parikshit, just one more question that's there on the chat. The basis for calculating ARPP.

Sandhya J: So, IP ARPP is calculated by the IP revenue divided by IP discharges. And for OP ARPP is similar, OP revenue divided by number of OPD patients seen. OP also includes daycare.

Viren Shetty: I see, a lot of people are in the chat. We'll come back to the chat questions if we can get Parikshit because he's had his hand up. If you could ask your question, please.

Parikshit: Hi, thank you. So, a few questions. You mentioned earlier in this conversation when the muted growth was brought up, among various reasons. One of the reasons was the base comparison from last year, same quarter. So, how long should we assume this high base impact should continue? How many more quarters?

Viren Shetty: See, the base impact. So, we are trying to ask if next year also the numbers would be the same? And the answer for that is, it would be hard for us to project. We did have quite a lot of pent-up demand in the post-COVID years and a lot of catch-up growth that's happening. Now, the next 2-3 years, we are in this little in-between period where a lot of the infrastructure transformation, a lot of the processes have been able to deliver some of their results. There's still quite a few more that need to remain. But we're also in a massive infra-ramp-up period. Which means that, some of our hospitals were adding more beds in the same campus, some hospitals were adding it to the same building and in some cities where we're fully built out, the infra will be coming in different parts of the city.

So, for that, we would be hiring people to match up to the same. We would be incurring a fair amount of expense in order to get those projects going. So, the sort of post-COVID numbers that I think a lot of people got used to seeing from the healthcare industry, at least in the case of our business, will probably kick in only post that. For

now, what we're trying to achieve the most is an adequate mix of payors, of getting adequate utilization for the existing infrastructure, and looking to see with the existing space that we have, which is quite constrained at this time, what best we can
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do with it. But we're always trying to prioritize sustainable profits over just plain revenue numbers, just given that all these beds will come online in three years. So, hard for us to say that this sort of performance will be the norm for the next 2-3 years. There's still a lot more that we need to do and things that we're working on, on our entire digital transformation program. But this is the best that we can give.

Parikshit: Understood. So, if I were to put it slightly more bluntly, we are basically...currently, our growth is getting bottled because of capacity constraints and not so much a demand constraint.

Viren Shetty: Well, it depends on what time period you're looking at. See, just generally as a country, Indians are the least medicated, least likely to seek treatment, least able to afford the cost of surgery, and we're coming in with offerings that are able to cater to all price points. So, in that, we don't see any shortfall of demand. The question is, the things that we need to do to attract patients, we have to do a lot more. And so hence a huge investment from our side in outreach, in building clinics, in putting an insurance program, in building multiple points of presence across the city, because while the previously, even currently, sorry, most of our revenue comes from acute cases; people coming with really serious conditions and that's what they end up in the hospital for. But as we start to broad base our revenue, just what we're doing in Cayman, where we're doing a lot more daycare, a lot more preventative, a lot more outpatient work, we will start reaching patients closer to where they are. So, the sorts of things we have to do are different.

So, I wouldn't say that there is any demand slowdown. I'd say there's a demand capacity bottleneck for the OTs that we have, for the cat lab that we have, for all the infra that's there. But there's still a lot more we can do by coming closer to patients.

Parikshit: Got it. So, just a sub-segment of that, you know, there is also a general feeling that there is a slowdown in the consumption sector in India. I know, of course, healthcare is a lot more required service rather than a discretionary spend. But does that impact our industry as well?

Viren Shetty: Maybe a quarter, maybe. We've seen. So, there is cyclicity during wedding season, during the rainy season, during a puja season, during any festivals. So, yes, people will push out certain healthcare procedures by a quarter, two quarters at most, but not to
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the extent that you imagined. And still, we are nowhere close to the saturation in demand for the population being exactly as it is. If everyone stayed the same age and the Indian population never grew, still there's sufficient demand that we're seeing for the healthcare services.

Now, given the fact that we are getting older, and the healthcare needs are constantly increasing and the population is growing, it's just three levers of growth that we have to risk. At the price point that we offer, I think that would be the important caveat that we're putting out there. We're trying to be a lot more cost-effective. We're trying to reach out to patients with price point they can afford.

Parikshit: Got it. Last question. I know that you have not been able to translate the future expansion into the number of beds. But the new Cayman Islands facility, can you give us in terms of... like currently 110 beds, how much will that become?

Anesh Shetty: Yeah, we'll add another 60 beds to that. But I would also like to mention that bed is not an appropriate indicator of growth or capacity or volume, given that the focus is more on daycare services and t

throughput. But to your question, it's about 60 beds, roundabout there.

Parikshit: Well, noted, Anesh. Thank you so much. Thank you.

Nishant Singh: Thanks, Parikshit. Damayanti, can we have your question, please?

Damayanti: Yeah, hi. Good afternoon and thank you for the opportunity. My first question is, can you update us on improving profitability at some of your lesser profitable units, SRCC, Gurugram, Dharamshila, etc. So, if you can talk about progress in each of these hospitals where they have reached in terms of margin improvement.

R. Venkatesh: Yeah, see, when we talk about Q4, Q4 has been really good for these hospitals. Gurugram has gone from a break-even to EBITDA positive. Mumbai has also shown positive in single digits, and Dharamshila has been very consistent quarter-on-quarter in double digits as usual. Combined EBITDA margin is around 9% with a revenue base of 115 crores for the quarter. When it comes to the NCR, we are very positive on NCR. This is one of our main focus markets. NCR has come to consistency in terms of steady performance and it will remain as one of our focus markets for expansions.

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Damayanti: And what are the main drivers for these improvements which you have seen? And, how confident you are about improving margins from this 9% to say corporate level?

R. Venkatesh: We will not be able to give you a very clear and indicative picture, but Dharamshila has been very consistent at 15% plus, and we are confident of growing more and more on that. Gurugram, there was a question of stabilizing and consistency in that unit, which has actually happened over the last 2-3 quarters. We have actually built in the clinical infrastructure, the clinical capabilities, enhanced on it with all the specialties across. So, it's actually grown in volumes, grown occupancy over the last 2-3 quarters and done very well on top line. And it was just a question of amortizing all the costs with the volumes coming in, and that's what has happened precisely when the volumes on the top line have built. So, we will continue to consolidate on this to make sure NCR remains positive and we keep focusing on it as one of the major areas for expansion.

Damayanti: Thank you. And my second question, if you can update us on your insurance business. I guess you launched pilot, right? So, what has been progress in that?

Ravi Vishwanath: Sure. Hi Damayanti, I can take that. So, I guess you're talking about the integrated care business, because the insurance business has not kicked in yet. We expect... we're just in the kind of final stages of putting... finalizing the product as well as the systems and everything else. And we expect to go live next quarter with the insurance business. In terms of the integrated care business, which is the one that we're piloting, that's going well. We've got seven clinics in Bangalore at the moment, they're growing as per our plan. We will add several more clinics in Bangalore this year, and then integrate the offerings with the clinic and the insurance once the insurance business goes live.

Damayanti: Sure. And my last question is, did you mention in your opening remarks that you are looking at some newer markets where you are not present? And if so, which markets or regions you are focusing apart from your existing markets?

Viren Shetty: No new markets as of this point. We are only in the Caribbean looking at anything that may come inorganically in certain markets. But at this point, it's still exploratory. In India, our focus is on our core geographies.

Damayanti: Okay, thank you. Thank you for your response.

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Nishant Singh: Thanks, Damayanti. Gagan, please go ahead.

Gagan: Yeah, good evening. So, the first question is around the funding of the capex. I think all put together, it's a figure north of INR 1,600 crores that you've budgeted for the coming year. How do you plan to fund it? I mean, the debt equity mix?

Sandhya J:

Around 20% will be internal accruals and around 80% we'll fund through debt.

Gagan: So, gross debt, where does it stand currently, and where do you see that going ahead in the next year?

Nishant Singh: The gross debt currently, India plus Cayman put together will be around INR 1,400 crores. With the borrowing plan which we have, out of that INR 1,600 crores, they will borrow, say, another INR 1,200 crores. So, the gross debt we see will be around INR 2,400 crores. If we do borrow, all we plan to borrow. And the cash in hand will also be roughly, say, half of it. So, the net debt, in the worst case, if we were to do all the expansion projects with all the debt we have planned for, will be around, say, INR 1,200 crores.

Gagan: I'm just wondering, in the last two years, you had operating cash flows of 1,000 crores each, right, if I go by the numbers reported in Q4 of this year for the balance sheet. And you have cash on book. So, I would have thought that it would have been possible to find a higher share of this capex through internal accruals than what you are indicating. So, just wanted your thoughts there.

Sandhya J: Conservative indication we have given, Gagan. Obviously, we will logically first use

cash accruals and then we will go to debt. But this is more a conservative estimate. Let's assume we find a good use for the cash we are able to deploy. So, therefore, we have just taken a view which is the max we could borrow.

Nishant Singh: Also, Gagan, a lot of the cash is parked in Cayman Islands. So, bringing that cash to India for Indian expansion is not a good strategy because you have to pay taxes. And we are still exploring opportunities to invest the Cayman money outside of India. So, that's a big reason why we are not able to spend that cash.

Gagan: But if I got it correctly from your presentation, a large part of the capex that you have for next year is for Cayman, is it not?

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Nishant Singh: Not for next year. If you look at the next year plans, we have only INR 270 crores for Cayman. Rest all of that is towards India expansion. But even that INR 270 crores is outside of... if we do come across some good opportunities outside of India, then we will probably spend more from the Cayman money.

Gagan: Yeah, because I mean, if I look at your presentation slide 16, FY 25 projected, perhaps I am confusing the colors. 10450, is it for Cayman or is it for Greenfield? Organic?

Nishant Singh: It is for Greenfield.

Gagan: Yeah. And in the last two years, you have done a fair amount of capex, INR 1,000 crores each in '23 and '24. And while this would not have been for a bed addition, you've been consistently pointing out that this is meant to increase throughput. So, I can understand that Rajarhat and Bangalore, they get commissioned a while away from now. But over the next three years, with whatever capex... it's almost INR 2,000 crores of capex that you've done in the last two years, what headroom for growth does that give you? It would help if you could enumerate in broadbands. I can understand that a specific number guidance is not possible. But if you could enumerate to some degree in broadbands, perhaps, what potential for growth is there over the next two, three years?

Viren Shetty: Venkatesh, go ahead.

R. Venkatesh: So, I'm just saying that, see, we will not be able to give you a clear-cut guidance on that. But we have been able to grow meaningfully within our existing capacity, and we will continue to do it till this expanded capacity kicks in. And it will be through a combination of all our throughput increase, improvement in payor mix, technology, guided through digital initiatives.

I n the meantime, we've also added our Howrah unit with five OTs, wards, ICU beds. We also added LINAC in Shimoga. We plan to add LINAC in Ahmedabad. We'll keep doing these small additions as and when, and as and where it's feasible till the major expansions kick in. This is

apart from our efficiency drive to create capacity. And with this, we will be confident of growth year on year till these expansions materialize. And we have actually demonstrated this in the last two years. And we are confident of

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repeating the same in the next two till we build up on the capacity and it starts materializing.

Gagan: So, is it possible to.

Viren Shetty: Yeah. Sorry, Gagan, just one second to clarify.

Sandhya J: Significant chunk of the capex over the last two years also went into Cayman. So, about that also, you'll have to adjust. 70-75 million dollars has gone into Cayman in that capex number.

Viren Shetty: That's greenfield capacity with the headroom for growth for filling out all the departments that we don't have in a grade A location to ensure that in Cayman we continue delivering strong performance. See, the reason we prioritize a lot of investment in throughput and capacity increase and things that are not necessarily adding beds, is because greenfield capacity in adding new beds, one, takes a very long time to break even and even longer time to generate a return on that capital. The most immediate and best use of the capital that we have is in the existing hospitals. And that's why FY22, FY21, FY20, all the money that we spent, was on improving the performance of our existing hospitals, which turned quite a lot of them around and made them into quite profitable entities. So, there is a diminishing returns aspect to that, which is why now you're seeing us deploy a fairly large amount of money towards greenfield, while keeping some amount of money still towards the existing hospitals and refurbishing the rooms and taking it up to do a balance between both. Now, we have a kind of a focus on ROC and trying to generate healthy returns, but that's not the only thing. The doctors, the patients, they don't care about ROC. They care about being seen by the doctor on time. So, for us, it wouldn't be one or the other. There are times in which we'd want to do greenfield expansion. That time is now. And there are times in which we take a step back and try and consolidate, which

was the story of the past 8 years.

Gagan: Fair point. I mean, I understand that. I'm just sort of trying to... perhaps, I'm thrashing around a bit blindly here, but are we looking at a phase of, perhaps, high single digit sort of growth, and then you kick into double digit when the new beds come in? Is that how one could broadly think of?

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Sandhya J: It's not fair for us to guide a number. Gagan.

Viren Shetty: You make us very sad, if that were the case. I mean, try not for that not to be the case.

Gagan: All right. And from a tax rate perspective, this was a year where the tax rate was subdued. Q4 was higher than the balanced three quarters of the year. Going ahead, what are we looking at in terms of tax rate?

Sandhya J: Yes. So actually, this year, if you see for NH India, not just about NHL, but on a portfolio of NH India, we finished the tax at about 15.5%. That's because we had the deferred tax credit which we got benefit, given that we moved from the old rate to the new rate. And therefore, there was a reversal that came on the tax line. So that one time of benefit is gone now. So next year, the India business, which is the main business plus the subsidiaries, the average tax rate will be about 26%. And Cayman business will come at zero tax and the average of that will be our tax rate for next year.

Gagan: Are we looking at perhaps higher growth in Cayman starting 2nd half of this year once you commission the new hospital at Cayman? And are we therefore, over the 2-3 years seeing an increased salience or share of Cayman in both revenue and profits?

Anesh Shetty: So definitely once we commissioned the new hospital, we will see an expected amount of growth in revenue. But as we have mentioned before, commissioning a new hospital brings online a very large amount of

fixed costs. So, there will be a very real amount of margin dilution until that hospital is able to break even over the next few quarters. And then we'll see where we end up on a long-term sustainable basis. So, there will be revenue growth, but there will certainly be margin dilution because a big chunk of fixed costs will be coming online. And in fact, it already has started as we speak right now because we are very close to commissioning those.

Gagan: For analysts, is it reasonable to assume that the bed capacity of the new hospital is proportionate in a certain way to the old hospital, so would the opex be? Or would it be a wrong surmise?

Anesh Shetty: No, that wouldn't be very accurate. So certain costs which are just related to the size of the building would be pretty identical on a per square foot basis. But the building is

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staffed very differently when the building is operationally very different. So that wouldn't be an accurate extrapolation.

Gagan: Right. Thanks. I'll get back in the queue. Thanks for taking questions.

Nishant Singh: Thanks Gagan, for your questions. Yes, Pravin, can we have you question, please?

Pravin: Hi team. So, I don't have any comments on your numbers, but I have feedback to provide to the management. Please tell me if I should go ahead and do that?

R. Venkatesh: Yeah, go ahead.

Pravin: Shall I go ahead?

R. Venkatesh: Yeah, go ahead.

Pravin: So, you know, the thing is that recently I had to reach out to Narayana Hrudayalaya Bangalore facility. Okay. And I had to reach out to this cardiology department. I somehow felt a little demotivated when I went for... when I took my parents for the appointments. Okay? I observed a couple of things which I want to bring into notice. Because look, I'm the owner of this business and I want my business to be growing and serving more and more people efficiently, in a peaceful way, at least from the patient perspective.

So what happened was, what happened was, I reached out to your cardiologist and he asked us to do Echo, ECG, you know, some of the normal things. And by the time I came back, it took me almost 3-4 hours. Okay. The person who manages the appointments to the doctor, she said, doctor is an OT. You can't meet him today. Then, I went to reception and then complained about this thing, saying, madam, we waited for this many hours, please at least give us some supporting doctor so that we can show our documents and we can leave the premises with a peace of mind, if there are no issues. And then I was connected to some junior doctor. And then junior doctor came into the picture and he started looking at our documents and then he was little confused there and he couldn't derive.

Viren Shetty: Pravin, sorry to interrupt. What you have, you have an email address and a contact details. I'm really sorry for the experience you've had, but just to respect everyone

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else's time, we can address the service complaints on a separate thing. Okay? But again, truly sorry for the experience you've had. Is there any shareholder related questions?

Pravin: Right. So I don't have anything. I'll roll back and I'll send out a mail then. Thank you so much.

Viren Shetty: Thank you.

Nishant Singh: Thanks Pravin. Dipankar, can we have your question please?

Dipankar: Hi. Sir, I just wanted to know if the surgical procedures that are offered in Cayman and the hospital services, are they reimbursable by insurance companies in the US?

Anesh Shetty: Yes. Hi, thank you for your question. So, most insurance companies in the US will reimburse for overseas care. So, there are two principal ways this happens. If you're a holder of a US insurance policy and you have an emergency while traveling, almost all of them will reimburse. Electively, most of them will reimburse. If somebody chooses to come, most of the large insurers will, because the rate that they would reimburse at is much lower than what they would pay.

But it depends on, you know, which insurance, etc.

Dipankar: Okay, thank you.

Viren Shetty: Sure. We'll wait for the other hands to come up. I'll answer some questions that came in the chat. One was about what is a better indicator of hospitals, ARPP or ARPOB?

Why? Just for those who are not familiar, ARPP is average revenue per patient.

Average revenue per occupied bed is ARPOB. Since we've focused a lot on looking at the throughput and the utilization of our existing beds, what's going to happen is that, we realize that ARPOB is an instrument that you can use to really manipulate numbers in a strong way. And so, it doesn't give you a true picture of what your realization per patient would look like, both in an outpatient and inpatient basis.

The other is that the medical field keeps changing. Things that require discharge over a couple of hours, things that require overnight admission, are all moving to daycare procedures. So, they are physically occupying the bed for most of the day, but they don't count as an inpatient and are not reimbursed by the insurance company as

inpatient stays done as daycare. So, in reflection of the way in which the entire field is moving, we decided to move towards a ARPP model, which gives a more moderated, less prone to manipulation way of thinking about what your realization per patient looks like. Now, not to say that one is better than the other, I think that gives a better reflection of the state of our business and gives you a more fair and honest assessment of how we are doing as a company when it comes to being able to price our services.

There are two more questions in the chat about how our services are differentiated from its peers and how do we price our surgeries to the customers. Dr. Rupert, respond to that.

Dr. Emmanuel Rupert: Yeah. So, the Narayana Services, how is it differentiated from the peers is in the geographies where we serve. We do give our patients and our customers the cutting-edge work irrespective of what is the paying capacity and what is the class. For example, we in the two centers where we have the Robotic Knee Replacement procedures, we have converted 100% of our procedures into robotics. And we are one of the highest use of robotic work for non-cancer surgeries as well; the numbers are very high. So, we do differentiate quite a bit on the peers and we do monitor the outcomes and other things in a much more stringent way and keep a very close watch on the outcomes and the way we go about with that. So, from that point of view, it is a value for money for the patient because we get the best of the class in the outcomes as well as the cutting-edge work and that is what we've been doing.

Yeah, as far as the pricing goes, wherever it is predictable clinical pathways, we prefer to have package the procedure so that the patient knows that there is a predictable amount that they know that they have to pay. But for the things where it's a little unpredictable, that's only where we go for the open billing kind of a system and that is where we work towards. Overall, from a price perspective, it is lower than what our competitors would be doing. And, mainly, because 80%-90% of the procedures are all very, very predictable. So, the patients also know exactly how much they need to pay as well.

Viren Shetty: Rajat, you have your hand up. We'll take your question and I'll go back to the chat questions.

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Rajat: Hi, good afternoon. I had a couple of questions on the chat as well. So, probably you could skip those. The first question is related, again, to the number of beds. While you shared the number of beds being added in Cayman, is it possible for you to share a

minimum number of beds that are getting operationalized in this financial year in India?

Viren Shetty: None this financial year.

Rajat: None this financial year? Alright, thank you. And the second question is related to the two su

bsidiaries, Medha and Samyat, are these being set up for captive purposes or is there a larger vision for them?

Sandhya J: For Medha, yes, we will be commercializing our analytics engine. Samyat is for captive purposes.

Rajat: Okay. And Medha will, I mean, any guidance or any thoughts on how will this ramp up in the next couple of years?

Viren Shetty: This is a healthcare analytics arm and they have quite a few interesting AI application that we are using. We have quite a few paying customers as well but the amounts are very small and nothing significant enough at this stage in time. But it is something we have an aspiration to build out because we do have very strong digital services and analytics capabilities and a lot of hospitals have expressed an interest in using these, both in India as well as overseas. When there's more to show in this, then we will start putting that out separately. But as of now, it was just something we wanted to highlight.

Rajat: Okay, thank you. Thanks a lot.

Viren Shetty: Thank you.

So, there's a question on INR 1000 crores Capex earmarked for 1000 beds in Calcutta only include the land cost of 180 crores that for building machine equipment? Don't go by the INR 1000 crore Capex or the 1000 beds in Calcutta. That was more of a, let's say, aspirational statement. The cost of the land was about 180 crores. That was the cost that we'll have to pay to West Bengal government. We're still finalizing the contractors and the price over there. Once we have a better sense of how much the

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final cost will be, we would put it out. But what we have projected is how much we know we will be spending in this year for a combination of buying land and putting up the foundation and so on. But it's not specifically broken up into how much for Kolkata, how much for Bangalore.

There's a question about the economies of Outpatient SBU and Inpatient SBU.

Sandhya J: Outpatient SBU, first of all, we don't break out our numbers into outpatient and inpatient. Within outpatient, you have Daycare and OPD. On OPD, a significant chunk of the fees goes to doctors. Daycare is similar to IPD depending on the cost we can recover and the inpatient is all our costs plus our margin. So, it works in that typical range. We are not giving that break up on what is the margin of each of these SBUs. But you can work that out, that's industry knowledge.

Viren Shetty: Next question on the chat, reason for the rise in other noncurrent assets going from 89 crore to 350 crore?

Sandhya J: That is because we are doing a lot of capital projects and there are various capital advances we have given for those projects. So, that get grouped as noncurrent assets. So, that's why you are seeing that.

Viren Shetty: Next question, last quarter we talked about wage inflation but they see that employee costs had dropped in the current quarter.

Sandhya J: See, actually, this quarter we have been able to manage some efficiencies in the employee cost because we've been able to add revenue without adding people. But that will not continue because our increments have kicked in from 1st April. And also we have caught up on the employee backlog in terms of hiring. So, therefore, you will see an increase in the Employee cost going into the next quarter.

Viren Shetty: So, another question on margin levers. To leverage, to improve the bottom line before the volume increase kicks in from a new hospital? That there are many but it is more of the same, which is discharging people faster, admitting them sooner, making sure they checkout rather they get discharged from the hospital at 12 o'clock and we get patients who get admitted at 1 o'clock. These are a lot of system driven and a lot of digitization involved. We've had a lot of savings from moving towards paper. We are doing a lot more things paperless and moving to kiosk for admission. So, that should

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improve the throughput a fair bit. It may

not have that much of a swing as compared to pure volume increase would but it does provide a very differentiated experience for patients given that we're dealing with large volumes of them and our pricing is not at a premium. And, so, if we are able to reduce their waiting time and improve the convenience most of the time, we believe that we'll be able to improve the bottom

line a little bit. Not a lot.

There was a question on the insurance related cost in the P&L.

Nishant Singh: That's already answered.

Viren Shetty: Okay, so that's been answered.

The current capex plan to be conducted versus current built up space? See, the current built up space, just the Health City in Bangalore will be around 9 lakh square feet, in Kolkata it will be around 5-6 lakhs square feet, Delhi another 6 lakhs square feet, Bombay 2 lakhs square feet, Raipur 2.5, Jaipur 2. So, it does add up to a fair bit. I would say that the number of square feet of space we plan to add eventually would be in the short term, about half of our current built up area. That's what we aim for. Now, we don't have all of them in the pipeline yet. That's what we're looking to add but it won't correspond necessarily to the same number of beds as before because the uses of space in these new setups is very different from what it was in the past. Prior, we never had much Daycare work. We had a lot of space dedicated for inpatient beds but now we're doing a lot more procedure rooms and so the actual bed count would not be significant.

There's a question on the occupancy numbers for Bangalore and Calcutta. If Venkatesh could share the rough occupancy percentages.

Anesh Shetty: Viren, somebody else also have their hand up.

Venkatesh R: Yeah, occupancy levels are stable at 60% plus at the group levels, Flagships have higher occupancies. These occupancies may obviously vary between hospitals but the flagships obviously have higher than this average occupancy.

But, again, as Viren said, we continue to reiterate that this is not necessarily the right measure given that daycare volume has gone up significantly and we are doing more

25 and more work in Daycare. So, obviously, the occupancy numbers may look very artificially low though so much of work is actually happening. We do largest number of cardiac procedures in the country, cardiology procedures like Angiogram, Angioplasties are like morning-evening discharges, robotic procedures are morning-evening discharges. So, this means that the bed is occupied during the day but at the midnight it shows vacant and that's why it looks artificially low. But then we are more working more on throughput through technology driven initiatives to create more capacity and do more with what we have.

Viren Shetty: The bottlenecks that we talk about and that's part of the question as well. We're bottlenecked in our OPD areas, we're bottlenecked in our billing counters, in the parking space, in the number of doctors chambers, in our Operating Theaters, in the Cath Labs, in the ICU and the emergency rooms. Those are places we have the most on a bottleneck. And, so, a lot of our infrastructure is built for a time when most work was long stay inpatient work. So, we probably have too much space dedicated for that and we're trying to change the layout so that we're able to cater to a lot more, very fast volume in and out. And, so, that's what you're seeing the bottlenecking show up. It won't necessarily translate to occupancy but it does translate to the service experience and our ability to see more patients in the same space.

Anyone else? Rajat, you still have your hand up.

Nishant Singh: Viren, there's a question on this. How do you read the Supreme Court observations on sanitizing the cost procedures?

Viren Shetty: Ah! Okay. So, this was a stray comment made by a judge that 'Look and see if the Clinical Establishments Act has actually been implemented in all the States. Otherwi

se,

we will force you to implement the CGHS rate or some rate. Now, if you actually read it, it's taken as more of a threat. What has happened since then is that the government has spoken to all the stakeholders, all the State Health Ministers and a lot of Industry Associations have gotten together and spoken about how it is impractical to come up to a common pricing standard as the petitioners, in this case, certain Public Health Activists had brought about. It does make sense for a country like India for the richest person in the country and the poorest person in the country to pay the same price. Then they're both getting... You know, the rich person should pay more for the service they're getting and the poor person should pay less. When you go to a

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standardized pricing that does not happen and so the observations haven't been made. We're trying to work with them to say that the spirit of what they want, which is being able to provide different price points, being able to provide more clear and transparent billing is something all organized hospitals do. Certain hospitals that are in unorganized sector are not able to do it as well but more transparency is required. So, we're overall concerned of outside of the larger concern that healthcare is this massive societal issue. There is a very large private sector that is catering to it and we need to

be very cognizant about being able to meet patient needs and expectations at all price points.

There's one more question in the chat, are Daycare procedure revenues captured in outpatient or inpatient revenues for domestic business?

Sandhya J: Outpatient at the moment.

Viren Shetty: But starting from next year, I think, we should start splitting up Daycare separately, which from FY25 Q1 we'll start doing it that way. So, it will give you a much clearer picture.

Any other questions?

Nishant Singh: If there are no more questions, Yeah. Gagan, yes.

Gagan: Yeah, Sir. Thanks for the follow up. You indicated occupancy with around 60%. And, obviously, it's a blend of higher occupancies in your mainstay hospitals and so on. But where is the theoretical limit of occupancy on the current capacity from 60%?

Viren Shetty: Anyway we've seen the highest occupancy in our hospitals hover around 75%-78%. But, again, it's pretty much a moving target. The real numbers that we're trying to go after is having more people, more inpatients, more outpatients, more Daycare procedures for all the square footage of space that we have built up. The occupancy itself assumes a stagnant book of business like a hotel. In a hotel, there's only so much you can do spending on a per night basis, whereas for us we have a range of services that can be provided in our infrastructure.

Gagan: Right. I understand what you're saying, you know. Well, it's sort of difficult for us to distill it down to some numerical level, I get what you're trying to say.

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On margins, I mean, what I can surmise is that in India, you know, as Delhi, Dharamshila and Mumbai improve and also you draw further cost efficiencies, there's perhaps some scope for margin improvement to a degree. It may be offset by, you know, the new hospital in Cayman coming onboard. Therefore, on a blended basis, are we looking at a future where you can maintain the exit margins of FY24? Is that reasonable to surmise?

Sandhya J: So, in the short term Cayman will be dilutive, Gagan, and I don't think India will be able to step up to offset the dilution that will come. But in the medium term, Cayman will catch up once the capacity start filling up. So, that is one aspect.

The second aspect is that, as you know, we don't pass on the complete cost inflation to price and, therefore, we absorb a lot of our cost inflation within our efficiencies. So, therefore, all our efficiency and scale doesn't flow through into margin. Some of it goes into offsetting the cost escalations that we see. And we have se

en a very high

impact of inflation across all the lines, whether it is manpower, whether it is consumption, capex. So, because of that it's difficult to say that. Even with the benefits that we are foreseeing from improved throughput and improved performance of some of our non-flagship hospitals, some of them can flow through to the bottom line but a lot of it will go to offset the cost increases that we are seeing. So, it will be a mixed bag for at least the near term. In the medium term, I think, we'll be able to bring it back.

Gagan: Right. When you say medium term, is it possible to enumerate that? Again, broadband.

Sandhya J: 18 months to 24 months' time period.

Gagan: Okay, alright. Yeah, thanks. That's all from my side. Thank you.

Nishant Singh: Yes, Parikshit.

Parikshit: Thanks for the follow up. You know, I'm really appreciating the management's effort to, you know, educate us how it is. Your strategy is about being outpatient and delivering more value through that. But my issue is that even with the outpatient numbers, if you look on year-on-year for this quarter, you have not grown by much, right. That has also been about 6% and 7%. So, question one is that, do you guys have

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any way of quantifying your outpatient capacity? If so, that would be lovely to hear.

And, two, is that why the outpatient hasn't grown more than 6% in that case?

Viren Shetty: Outpatient capacity, so all we can say is that in the space that is dedicated to them, they are bottlenecked and there are only so many hours in a day. So, our doctors are not able to put in more hours. Now, a simple fact is we need more doctors but we need more doctors and they're not all going to sit in the same place because there's no space, there's no parking, there's no waiting area, there are not enough billing counters there. And, so, we've created an entire new subsidiary around building Clinics and Outpatient Centers all over the city starting in Bangalore and then we move to Calcutta and all the other places.

So, those are the things we would have to do to increase the outpatient numbers.

Those are essentially the two things. So, in terms of what is the theoretical maximum

number of outpatients we can see in any hospital, that would just be a function of 20 minutes per consultation multiplied by the number of hours in a day patients will keep coming. But, again, it's a moving target because we are adding more outpatient chambers and more spaces in the existing hospital. So, we are, trying to address this both ways by adding Infra as well as doing more efficiencies.

Sandhya J: Just as our annualized revenue growth on outpatient is about 14% and a lot of it is Daycare. So, while there are two aspects to the volume, one is the count and another is the value of services that we are able to flow through. Because we are debottlenecking capacity across, so we are able to move through high value services through Daycare and that's also helping us. For example, a lot of procedures we are able to do morning-evening and discharge the patient before end of the day. So, that also helps us deliver better throughput.

Parikshit: I understand. So, did I hear correctly in terms of your plans to increase your outpatient and Daycare capacity was to set up smaller clinics around Bangalore and other regions? Did I understand that correctly?

Viren Shetty: Yeah.

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Parikshit: So, this is not part of your capex plans that we have discussed so far, the 8 lakhs and 10 lakhs where you're doing massive land acquisition and creating hospitals? This is a different plan?

Viren Shetty: This is the entity called as NHIC (Narayana Hrudayalaya Integrated Care). So, the capex isn't much. Each clinic is only between 1-2 crores and we're putting about 20-30 clinics per year. So, it's not a huge investment.

Parikshit: Got it, alright. Thank you.

Viren Shetty: Back to the questions in the chat. Is there any margin cap o

n the pharmacy? There

isn't a margin cap on the medicines. Certain payers, I believe, certain state governments, for example, will cap it at the price at which you are able to procure the medicine. Most of the medicines are actually capped on the MRP. So, either they come under DPCO, that's Drug Price Control or the payer themselves will decide that 'I will pay this fixed amount. It doesn't matter what sort of medicine you use'. So, a margin cap system of any consumers or medicine does not exist in India like it does in other geographies.

What is the impact we see on the revenue per patient? The DPCO and when every time the price capping happens on medicines, the impact is mostly on medical admissions. So, these are patients who come into the ICU and get medical care or for item like chemotherapy or so on. That definitely does impact on revenue per patient. The next question is, how do negotiations with the insurance companies change for medical packages and non-medical packages? So, insurance companies are obviously very hard negotiators. They will negotiate fixed rate packages for knee replacement, for a lot of these. But medical packages, it's very hard to quantify how much on the medicine side. So, those are usually paid on actuals whereas surgeries are paid on a fixed basis. So, Angioplasty or Knee Replacements are on a fixed rate.

There's a question on the cash accrual in Cayman. Are there plans to bring it back? And what is the status of opportunities to deploy cash? I think, we answered this a little earlier. Right now, we're trying to find overseas opportunities. In the absence of that, we would definitely bring it back to India. But to do so involves a 20% odd hit that we would take. So, that would be, I would say, the last possible opportunity. But,

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definitely, there are places to deploy in India. There's a lot of opportunity here as well as abroad. So, we want to build a diversified revenue base. And, so, that's why we're trying so hard to try and find something overseas.

There's a question on the insurance business. Can you give a roadmap on the insurance business in terms of breakeven long term plans of hiring it off as a separate arm listing, like etc? I'll just say, I mean, it's too early for us to declare on the breakeven timeframe. The long term plan is that, yes, it is a separate company by design; the insurance company. The IRDA norms say that it has to be listed as a separate entity and so on. There's no time limit per se but it is something that is on the eventual roadmap. So, the insurance is something we are very bullish on. We are building it to be a very differentiated sort of product and we'll have a lot more to tell you about this in the coming quarters.

Ravi, have we given any projection on how much we spent on the insurance company in this quarter?

Ravi Vishwanath: No, we've not. I mean, the main thing is, of course, the IRDA capital of 100 crores which has been infused into the company.

Viren Shetty: Yeah. So, you know, we won't be able to give guidance on the cashflow and so on

because we are yet to start the insurance operations. But it will not be as big as the amount that we'd be otherwise spending if we set up 400-500 bed hospitals. So, it's quite moderated since we're starting in just one city. Based on the response we get in Bangalore and Mysore, then we look to ramp it up. That is a call we will take at a future point.

Last question, I guess, on the chat. Any plans on the growth in your ancillary services? If my ancillary services, I would guess it means all the subsidiaries that we have. So, Samyat is an inhouse distribution now. So, that will mostly be catering to our inhouse programs. With our ATHMA and Medha, again, these were set up as inhouse units but they're offering services to other hospitals, clinics, nursing homes and so on. But the revenue potential there at the short term is very limited. So, it's nothing much t

o write

about at this stage. Other than that, NHIC and NHIL are still very nascent businesses that we incubating and over time they will start, you know, showing the numbers. But 31

whatever they've done so far has given us a lot of confidence that this is a business worth investing in.

Nishant Singh: Yeah. So, I think, we don't have any more questions neither in the chat nor in this forum. So, with that, we would like to conclude our session. Thanks everyone for the active participation as usual. In case if you have any further questions, please feel free to reach out to us. Thank you.

End of Transcript

Call: Aug 2024

Date of submission: August 13, 2024

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code – 539551 To,
The Secretary
Listing Department
National Stock Exchange of India
Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051
Scrip Code- NH

Dear Sir / Madam,

Sub: Transcript of Earnings Call for the quarter ended June 30, 2024

Further to our earlier letter dated August 06, 2024 in relation to uploading the Audio Recording of the Earnings Call of the Company held on Tuesday, August 06, 2024 for the quarter June 30, 2024, please find attached the transcript of the said Earnings Call.

We wish to inform you that the Earnings Call transcript is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earnings-call-audio-and-transcripts>

This is for your information and records.

Thanking you

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q1 FY25 Earnings Conference Call"

August 6th, 2024

MANAGEMENT : MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – GROUP CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – GROUP CHIEF OPERATING OFFICER

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS
SUBSIDIARY HCCI

MR. RAVI VISHWANATH – CHIEF EXECUTIVE OFFICER , NHIC

MR. NISHANT SINGH – VICE PRESIDENT , FINANCE , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. VIVEK AGARWAL , SENIOR MANAGER , INVESTOR RELATIONS

2 Nishant Singh: Hello everyone. My name is Nishant Singh. I head the investor relations functions at Narayana Hrudayalaya. I welcome you all to the Q1 FY25 earnings call of the company . To discuss our performance , as usual, we have Mr. Viren Shetty, our Vice Chairman , Dr. Emmanuel Rupert, our CEO and MD, Mrs. Sandhya Jayaraman our Group CFO, Mr. Venkatesh, our Group COO, Dr. An esh Shetty, MD of our overseas subsidiary, HCCI, Mr. Ravi Vishwanath, CEO of NHIC , and Vivek Agarwal, Senior Manager in the IR Function. As usual, before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the of the same shall be made available on a website as well as on the Stock Exchange at a later date . I would also like to remind you that everything that is being said on this call, that reflects any outlook for the future or which can be construed as a forward -looking statement , must be viewed in conjunction with the uncertainties and the risks that they face. Post the call should you have any further queries, please get in touch with us , we would like to address them to the best of our ability . With that now, I would like to hand over the call to Dr. Rupert.

Dr. Emmanuel Rupert: Good evening , everyone. I warmly welcome you to the quarter one FY25 Earnings Call Conference of Narayana Hrudayalaya Limited . We are pleased to report the highest ever revenue at a quarterly basis at the group level with a sustainable profitability margins. Despite being a seasonally weak quarter due to summer holidays , Eid, and the impact of general elections, we are able to drive improvements in realizations and increased patient footfalls. Consolidated revenue for the quarters to that INR 13,4 10 million reflecting a growth of 8.7% year -on-year and 4.8% quarter -on-quarter . Narayana Hrudayalaya generated a consolidated EBITDA of INR 3 ,274 million at a marg in of 24.4% against 23.2% in Q1 FY24. H CCI continues to deliver strong business performance with quarterly revenues of USD 32 million a year -on-year growth of 4.8% and a quarter -on-quarter growth of 5.8%. We're delighted to share that our new hospital in Caman a Bay was inaugurated in July, with patient inflow expected to start before the end of the second quarter. We are confident that our Caribbean business will continue to grow through synergies between the two hospitals further strategic initiatives and investments in this fiscal year. The balance sheet and liquidity profile at the group level remains strong with group cash and liquid investments of over INR 13.2 billion against the gross borrowing of INR 14.75 billion, r esulting in the net debt position of INR 1.55 billion as of 30th June 2024 . A net debt to equity ratio at 0.05 gives us sufficient room to fund expansion through a mix of borrowings and internal accruals. We have incurred a capital outlay of INR 2.8 bill ion directed towards purchase of land in Bangalore for Greenfield expansion, regular maintenance, biomedical upgrades , and interior 3 transformation work within existing hospitals. We are confident of realizing the projected Capex for the fiscal year as we take on more Greenfield projects during the rest of the year. On the clinical front, we have had many success stories and increase in the complexities of the work . The Health City, Bangalore successfully performed 3D printed patient specific implant for a total knee replacement in a dwarf patient and a navigation guided pelvic bone tumor resection , which is a state -of-the-art technique for bone cancer surgeries which only very few hospitals are capable of performing it globally . The R N Tagore Hospital, Kolkata successfully performed what is called as a paintbrush technique for calling of a patient who had an aneurysm in the brain , which is the left middle cerebral artery bifurcation aneurysm and they also did a robot -assis

ted kidney transplantation as well. During this quarter, across the group hospitals, we performed 40 TAVIs and 389 robotic surgeries across the entire Group.

Our focus on digitization and business transformation continues to lead to significant benefits throughout the system. We launched an AI system which auto creates discharge summaries of patients stay as an initial step , so that the doctors can screen them before giving the final report. The AI alert feature within the NAMA H app, which is a nursing app, triggers early warning signals to the nurses, who can then reach out to the doctors for the

timely attention. We also recently launched Virtual Tumor Board for Oncology where the various special list doctors can come from across all the centers on the same virtual platform and discuss the diagnostics and treatment on a real time basis with all the history of the patient in one place. Our digital appointment management system has been revamped and now includes the N H app, the website, queue management and Kiosks for OPD consultations. In the units where all these are deployed, we're witnessing more than 60% of the consultations being booked digitally, leading to reduced waiting time for patients, and freeing up manpower and valuable space. Deployment of Athma in Emergency Department has reduced the ALOS by 50% in the emergency room by digitizing all the time-consuming documentation process.

Narayana Health Integrated Care continues to perform as per our growth plan. Revenue for the quarter has crossed INR 80 million, which is the highest so far with more than 51,000 patient transaction across 8 clinics. With the confidence start in its first year, we will continue to grow this business and serve our customers with a clear focus on improving the health outcomes.

4 We continue to upgrade our clinical and non-clinical operations across the group, transform the patient service levels, increase the throughput, build more capacity, invest in more digital patient outreach channels, and improve our operational efficiency.

We are reasonably on track on our ESG commitments and continue to focus on creating meaningful social impact in addition to pursuing our environmental goals and upholding the highest standards of governance. We are simultaneously pursuing organic and inorganic growth opportunities growth both in India and overseas that will derive synergies from our existing operations, maximize value for all our stakeholders while keeping a close watch on return on capital. Thank you.

Nishant Singh: Thank you, Sir. I would now request everyone to now use the raise hand feature to start posing their questions. If anyone wants to ask a question through the chat, please use the NH investors relations chat room. Thank you. Yes, Prince can we have your question please?

Prince: Yes, good evening, everybody. Sir, my first question would be regarding the new hospital in Camana Bay. So, as the Cayman being a suitable geography for NH, what will be the expected occupancy for the new hospital?

Dr. Anesh Shetty: Yeah. Hi, Prince. Thank you for your question. So, the hospital will start in a couple of months. We've already inaugurated it. We have about 50 odd beds, that's our official bed count. You know, we hope to be occupied from the early quarters, but at the end of the day, this is something we'll have to wait for a few months to see. We won't be able to project what the occupancy levels could be, but this is in a good location. We have all the inputs to suggest that the hospital will be occupied as per our expectation.

Prince: Can I get a range?

Dr. Anesh Shetty: It's hard to say. Having said that, it's also important to consider that you know the occupancy as a metric for revenue growth is less and less important these days, especially because it's a daycare focused hospital, but let's see in a few months.

Prin

ce: Sir, my second question is

Viren Shetty: Prince, you're disconnected. Could you repeat the question?

Prince: Yes Sir. So, I'm audible now? Hello?

Nishant Singh: Yeah, go ahead, go ahead.

5 Nishant Singh: Yes, you are audible.

Prince: Yes. Can I get the CapEx break up for the Bengaluru expansion and Kolkata expansion?

Viren Shetty: This CapEx breakup of Q1?

Prince: No. Going forward, we are planning to for the Brownfield expansion in Bangalore and Greenfield in Kolkata. So, can I get what CapEx we will be keeping for both the hospitals?

Nishant Singh: Yeah. For the Kolkata Greenfield one, the first phase project estimate is almost around INR 1000 crores, which includes the building of around 350 beds in the first phase and there will be a lot of infrastructure development for the entire for the 1000 beds as well, but the first phase will cost around INR 950 crores. For the Bangalore land, cost will be around INR 500 crores.

Prince: And what would be the expected capacity of beds, Kolkata and Bangalore?

Viren Shetty: This is yet to be finalized.

Prince: Okay. Is there any expansion in future, we could be doing with the leased land?

Viren Shetty: Which land? Yes, we would explore different options and models. Right now, these are the projects that are in hand, the one in Bangalore and Kolkata, but we are talking with various projects and developers to look at leasing options as well.

Prince: Okay. Thank you. I'm done with my questions.

Viren Shetty: Thank you.

Nishant Singh: Yes, Damayanti, can we have your question please?

Damayanti: Hello. Good afternoon. So, my first question is again on the new facility in Cayman. So, just want to understand it better. You mentioned it's mostly outpatient set up where you're

focusing on preventive care etc. So, my question is how do you plan to ramp up your operations, focusing on which key segments and what kind of initial losses you expect to incur from this facility till it scale up to a reasonable level? So, that's my first question.

Dr. Anesh Shetty : Sure. So, just a slight correction there. Yes, it is predominantly focused on daycare procedures, but these are not necessarily preventive, they could be diagnostic preventive, but these are mainly short-stay surgeries, quicker surgeries, robotic surgeries, these kinds of things. The primary goal of that facility is to offer the services we currently do not offer, the major gaps being Emergency and Trauma Care, Obstetrics and Women's Health, Neonatal Intensive Care, etc. So, you know when we start there, these would be the services we will look to lead with the services. Your question on the second part of your question around you know, operational losses, as we've said before, it's a new building in Cayman. Any hospital we have in Cayman including the existing one has very high fixed cost. So, initially there will be obviously, certainly margin dilution and operational losses. We are not in a position to, you know to give out a particular number or a hard value to quantify that, but they're actually, you can obviously expect operation losses and margin dilution for some time.

Damayanti : Sure and if you can also update us on the existing facility about any improvement in IP or any volume pickup etc., what are you seeing on the existing Cayman unit?

Dr. Anesh Shetty : Sure. So, in the existing Cayman Unit, we continue to see growth on a revenue basis and some growth in good quarters on volumes, but as you can imagine, we are reaching a sort of saturation point in the existing facility and that's why the timing for the new facility is good and very much needed. We have undergone some operational improvements in the existing facility that's why you can see the good improvement in margin, but you will see a slowdown in volume and revenue growth until we commission a new

facility.

Damayanti : Sure. That's helpful. My second question is on India operations. So, on the IP volume part, we are seeing a bit of gradual pickup, which you earlier attributed to some reconfiguration of beds or facility upgrade, etc. So, my question is, are you broadly done with your initiative and how should we see IP volumes moving up from here on?

Viren Shetty : Can I ask Venkatesh to take this one.

R Venkatesh : Yeah. When it comes to the reconfiguration, basically in terms of infrastructural upgrade, transformation, these are something which is an ongoing exercise and we still have a little bit more to go before we come to a conclusion on that. Of course, when it comes to the IP numbers, we all know that we are in an expansion phase and these capacity expansions obviously will kick in, say, in the next three years' time. Till such time, it is important for us to make sure that through these transformations, these efficiency improvements, we keep improving on the numbers. So, we've got a long way to go before we run out of these benefits or efficiency improvement and we've actually started reaping these benefits only now in terms of more OPD visits, procedures, quicker discharges, lab reports and all. So, what I would say is that while we look into the volumes, we are focused more on quality of revenue. Our margins actually because of the quality of revenue has shown more positive trajectory in this Q1 FY25 as compared to Q1 FY24 by more than 1.1% and we are actually taking a balanced approach between cash flow, profitability, and growth. We've done a lot of work, as I've said, on changing our bed mix, the digital journey throughput, and we are also not very aggressive on the pricing. Therefore, our buildup has always been very slow or has been slow, but very steady. So, the current growth will be normal growth and we will get the next point of inflection when our new capacities kick in, in few years from now.

Damayanti : Sure. That's helpful. My third Point is actually a request. So, in your presentation you have now started reporting ARP for clusters, so that's helpful, but request you to also share volume number because that I guess will help us to understand your business improvement in a better way? So, earlier like you used to give ARPOB, but now we have OP and IP, ARPP, but maybe volume numbers will be also helpful? Thank you.

Sandhya J: We can share. Actually, there's one more question on the chat and I'll take both of it together. We've been asked whether ARPOB is not being shared and can it be shared? So, we will share the volume numbers and we'll also work with you offline in terms of how to derive the ARPOB straightforward number, but we will share that volume and ARPOB calculation.

Damayanti : Okay. Thank you. I'll get back in the queue.

Nishant Singh: Thanks Damayanti. Nitant, can we have your question please?

Nitant : Congratulations to the management on the great set of numbers. My first question was regarding the insurance business, which was supposed to commence in Mysore. So, what are the updates on the insurance business commencement?

Viren Shetty : Ravi, if you can comment on the insurance?

Ravi Vishwanath : Hi Nitant, thanks for your question. This is Ravi Vishwanath. So, on insurance, I'm pleased to let you know that we launched Narayana Health Insurance in Mysore as we have said in late June with our first product it's called Aditi, the headline feature which is providing 1

crore cover for surgeries for a price of about ₹10,000 for a family of four, where the oldest member is 45. You're also asked about regulations and things like this. So, yeah, I mean, our focus has been on making sure that our systems, processes, data flows, etc. are compliant and in line with the regulations, including the most recent changes, which have been several by the IRDA

and we're happy to report that, you know, all those things are in place and working as expected, which now gives us a good base from which to focus on making sure more and more customers are able to take advantage of our products.

Nitant : Alright. Thank you. Another question was that you mentioned that you were venturing into new specialty whether ortho spine, neurosciences, GI Sciences, so has that impacted your revenues in this quarter or will this affect in the quarters to come?

Dr. Emanuel Rupert: Yeah, these are not new departments per se, but we're just increasing the spectrum of work so that we can do a little more complex spectrum. We've just recently installed the robotic spine program and the equipment and we've done around 5 cases with that very complex cases, but over the next few quarters it will start, we will see some impact on the complexities of this patients as well as spine is concern. So, all those things is a routine stuff, which we are doing and it will, but it's not something that dramatically, it's not a new department, it just adds to a different spectrum within the same department.

Nitant : Alright. Just one last question regarding the Kolkata facility. What are the updates regarding the commissioning of the facilities?

R Venkatesh : Yeah. This is the Rajarhat facility in Newtown, Kolkata that you are talking about. So, we are currently seeking approvals for plans from the authorities. The work is going up in full flow. We expect the approvals to come by October and we hope to start the construction towards the end of Q3 this year. So, we will try and speed it up, but mostly what it looks like by the end of Q3 this year, we'll be in a position to do the groundbreaking for the Kolkata projects.

Nitant : Thank you. That was helpful.

Nishant Singh: Yeah. Thanks, Nitant. Parikshit, can we have your question, please?

Parikshit: Hi, good afternoon. Thank you for the opportunity. I was trying to understand the volume number, the footfall number, and we can break it down between India and the Cayman. So, to begin with, in India our inpatient numbers have not been growing for the last 3 quarters. Is there any particular reason for that?

R. Venkatesh: What I would say first point is, if you look at this quarter, in our health city, in the major facility there, there is a little bit of a dip. Q1 generally is known to be a low quarter because of the summer holidays, Ramzan period but this year, due to the Indian elections, there was a phase wise election throughout the months of April, May and some part of June. And since our health city sees a lot of these patient inflows happening not only from Bangalore, but 9 from other parts of Karnataka and also from other states, these numbers got tempered a bit due to the election, which will obviously get corrected in the coming quarters. But of course, what I would want to reiterate is, along with volumes more than that, I have said I'm just repeating it again, that we are focusing more on quality of revenues. So as a result of that, if you see, even if the volumes are not growing substantially, there are good quality of revenue, which is resulting in our margins showing a good growth from 18.3% last year, Q1 to 19.4% this year. Obviously, we have started working a lot in terms of changing our bed mix, upgrading it to single rooms, twin sharing rooms. So, within the same volumes, we are able to recover more quality revenue. And of course, it is also a constant endeavor for us to make sure we work on increasing volumes, and we are confident that eventually in the course of the next few quarters we'll be in a position to get more traction from the volumes. But, the current growth, what we are having will be very normal growth, of course, double digit growth, year-on-year, mostly. But the major inflection point will happen when these capacities come in the next 2 to

3 years' time. Till such a time, we will see very normal growth, but we will work more on efficiencies to ensure much healthy margins and bottom line.

Sandhya J : Just adding to the data point, actually, we saw about 26 million patients in FY23, now we've seen about 28 million in FY24 and based on the Q1 run rate, if you extrapolate, we'll see upward of 30 million patients in terms of the number of people who are visiting us. And so, there is that footfall in volume that is coming through. But everything else, I think Venkatesh nicely explained.

Parikshit : Thank you. So let me just follow up. So, I appreciate that there is growth and that's why I'm actually breaking down the numbers between outpatient and inpatient and my question was particularly towards the inpatient, because that's the one that hasn't been growing for the last 3 quarters. It was not just this quarter which might be election dependent, right. I appreciate the point you're making that you're trying to change your pay mix, and hence that is impacting the volumes to some degree. But that also would imply that your margins

are improving. So even though you're saying that you will see a normal double digit but a normal revenue growth, you should see a slightly more faster EPS growth. Is that a fair statement?

Sandhya J: See, just one maybe data point I want to clarify, though, the general trend, I think Venkatesh spoke about Q3 and Q4, we saw about 56,000 patients in our inpatient, and this quarter we saw about 59,000 patients in our inpatient. So even though Q1 is a weak quarter for us in terms of inpatient volume, we were okay. So just that's a little bit of clarification on the data 10 point. But in general, I think there is a little bit of investment that we're making in terms of balancing between quality of revenue, between healthy cash flows and in terms of growth and that balance is reflecting in the numbers that you are seeing.

Parikshit: Got it. And so, while I appreciate that we are tempering our volumes with the pay or mix in India, the Cayman number also in terms of footfall, has not been a great quarter for us, right. So, the inpatient has year-on-year degrown by -7% and the outpatient has also just grown by 6.5% year-on-year as compared to some of the previous quarters we were growing very strong double digits. So, what's the pattern, the trend we're seeing in the Caymans in terms of volume?

Dr. Anesh Shetty: Yeah. Parikshit, I think you are largely right. The fact remains that, you know, as I said in the earlier question as well, we have sort of reached a point of where we will not see double digit growth with just the existing facility and that's why about two and a half, three years ago, we started planning the new facility, which should be commissioned anytime now. We have inaugurated it. It should be ready. We should be treating patients in a couple of months or hopefully sooner in a few weeks. And at that point in time, we will see the next wave of growth. But in the existing facility, in the existing location with some other market dynamics at play, we will just see this small incremental mid-single digit growth and some quarters up or down, like for example, this quarter. But the next leap forward will only come with the new facility.

Parikshit: Sorry, Anesh, and I appreciate you saying that I've obviously heard you say this. The reason I'm being so stubborn about pushing this point is that in the previous quarters our same facility was capable of handling a much larger volume. So, it doesn't seem like an infrastructure bottleneck, right. Because in the previous 3 quarters we've done higher numbers?

Dr. Anesh Shetty: Sure, sure. It's not an infrastructure. It's more of a market dynamics situation. So, the first point is, you know, as in Cayman, my request would be to always look at a trend over 2 or 3 quarters because the base is so low. There are silly reasons

that could skew a quarter up or down, if a couple of people around leave or if there was a bad weather, like we had a hurricane that came pretty close, etc. So don't read too much into 1 quarter. I would say 2 or 3 quarters to get a general trend is true and if you see 2 or 3 quarters, you will see a sort of stagnation in the IP volumes and a moderate growth in the outpatient volumes. So that is the picture over a couple of quarters.

11 Parikshit: Alright. So, am I reading it wrong because I think the outpatient growth has been fantastic over the last few quarters, right?

Dr. Anesh Shetty: See, there are several factors to it. Yes, the outpatient growth has been there. I wouldn't say, too fantastic. The reason being we have started changing a lot of the procedures that would traditionally be inpatient into outpatient. This is in line with our efficiency initiatives, etc. So, some of it is a reclassification from IP to OP. The other thing also is we have commissioned new service lines, like we did an ENT acquisition, which is outpatient heavy about a year ago. So, this is what we expect, outpatient growth is good. Yes. It can be better. Inpatient growth is what it is given the constraints we have until the new facilities is up and running.

Parikshit: Got it. And if you don't mind me following up, you mentioned that besides the new facility opening up, you are also seeing, there was hinting on some market dynamic. Is there a new competitor there that has opened up or what market dynamics?

Dr. Anesh Shetty: No, no, no, I'm saying whatever the existing market dynamic is with the location we are in and with the other players who are already there, there's no new change to it.

Parikshit: Okay. Alright. Thank you so much.

Dr. Anesh Shetty: Thank you.

Nishant Singh: Thanks, Parikshit. Uday, can we have your question, please?

Uday: Hello. Thank you for the opportunity. Just wanted to get some color on the insurance product that you have launched. Can you highlight some differentiator in terms of underwriting that we are doing in this product? Because it's a very different product compared to a normal health insurance that we see in the market?

Ravi Vishwanath: Sure. Let me answer that as well. So, I think, our philosophy is that we want to make the claims process as simple as possible and as I guess based on your question, you understand that, largely the way the market operates today is, that underwriting effectively happens in the time of claim which can lead to unhappy experiences for the customer. We want to do

it differently. We want to be sure that we underwrite at the time of acceptance, and make sure that the customer has got absolutely no issues when it comes to claim. So, every customer for our product undergoes a full physical examination. It happens at our facility through our doctors. It's a very smooth process that we put in place. It is provided completely free of charge to the customer whether we accept the customer or not. So even if we are not able to accept the customer for some unusual reasons, our goal is to accept as many people as possible. You know, it is still provided to the customer at no charge. And it's a pretty comprehensive physical checkup that is done. The results of that are immediately available to him on the app. And, in cases where they need further treatment or further attention, we work with them to make sure that they get that attention as well.

Uday: Sure, sir. And just another question is, are there any plans to expand coverage to other hospitals as well in the future or it will be restricted to Narayana Health only?

Ravi Vishwanath: So, at the moment the focus is on Narayana across the country. There are situations where the customer can go to any other hospital, for example, if there's an emergency or something like that. I don't think that we will ever have a wide network like, you see with a typical carrier. We will always have a narrow network. It's something that we are always

listening to our customers and trying to understand what their needs are, and we kind of respond accordingly. So, at the moment it is across the country. You know, you can go to any Narayana facility. We may look to expand that over time depending on what our customers tell us, but I expect that it'll always be a narrow network as opposed to a broad network that allows us to provide a much superior experience to our customers, which is the main goal of what we're trying to do is to make sure that we are their partners in helping them stay healthy.

Uday: Okay. And lastly, if you can squeeze in one last, how is the traction that you are seeing? I know it is a very short period after launch, but any color on the traction that you are getting on?

Ravi Vishwanath: Yeah. As you said, it's very early days, right. So, I think, I won't be able to give you any numbers in terms of traction, what I would say is that we are happy to have put in place the processes and systems and all the other things that are required for an insurance company to be able to successfully serve its customers, which is a gargantuan exercise. And all the testing that one does before you go live is fine. What really matters is whether that works in the real world. That's been really our focus. And am happy to tell you that the team has done a great job, and all the systems are working really well and the customers that have purchased the product are really happy with it. They are happy with the process. They're happy with the transparency with which we are explaining the benefits and the limitations in the product to our customers. And I think that overall, I would say that they feel comfortable that they are with a trusted brand and should they ever require complex surgery they will be well taken care of, not just financially, but also, surgically by a really high-quality team of doctors at a high-quality facility.

Uday: Sure, sir. Thank you.

Nishant Singh: Thanks, Uday. Adrith, can we maybe have your question, please?

Adrith: Yeah. Hello. Good evening. Am I audible?

Viren Shetty: Yeah, you are audible. Go ahead.

Adrith: Yeah. Thank you. Thank you for being so open about your volume issues. If I could just push the envelope a little bit here. So, the overall IP volumes are flat and Bangalore is the only cluster that is seeing a drop in your Y-O-Y discharges. So, I believe it's being driven there. And when I view that, in the line of your specialty mix, the cardiac mix has actually come down, over the last year. So, if I compare it to Q1 FY24, so what comes to my mind and please clarify if I'm incorrect, is that the Health city hospital, which is essentially cardiac, have you seen any kind of drop particularly regarding surgical demand there when it comes to heart surgeries?

Viren Shetty: Never a drop when you're selling heart surgery in India. The numbers that you're talking about, maybe on a quarter-to-quarter basis, would not be growing the way it grew in a post-COVID catch up growth scenario. But we are focused on growing our overall top line and re-engineering the interior bed allocation and focused on certain payer classes. Where it may demonstrate that volume wise, it's not growing the way it did, it's still delivering revenue and margin growth and that's been our focus.

Adrith: Yeah. So, the share of cardiac, could you help me out, why it's going down, like it's down, 2% from 35% to 33% on the overall IP?

Viren Shetty: That is a conscious effort. See the cardiac, it's a disproportionate component of our business compared to all the other listed hospital groups, predominantly because we have this very, very large, the largest cardiac hospital in this part of the world, which does only one specialty. Most of the groups, it'll be between 10-12%. Now, it's not that there's a lack of demand for

or cardiac services, it's just that the ortho, oncology, other businesses have been growing much stronger and more robustly across all the other hospitals. Cardiac still remains our flagship anchor business and will still contribute anywhere from a third to maybe it can go down to a quarter of our business 10 years from now, but it is nearly half the reasons 14 why people end up in a hospital in India. So, it will always remain a very large growth engine for us.

Adrith : Got it. So, you could confirm that you have seen an increase in discharges regarding cardiac in your Bangalore cluster?

Viren Shetty : Over a year -on-year basis. Yes.

Adrith : Yeah, on a year -on-year because obviously this seasonality. So, I'm not looking sequentially.

So yeah, okay, thanks for that. Secondly on the payor mix. So, the scheme business is again flat, like at 21%. Is there any kind of guidance or direction as to you would, it's decreasing proportion in the coming quarters or at least in the next few years? Because as it was reiterated before multiple times that, the quality of revenues are important and even though realizations are only up like 6%, I'm just wondering if a lot of the growth levers would have to come from the payor mix because specialty also is not margin accretive at times. So, I guess the improvement in margins will have to come from a scaling down of the scheme business share. So, is there any kind of guidance on that?

Viren Shetty : No. So, two things. We're not scaling down the scheme business. We are taking the calls that make sense for the hospital that's over there. And guidance on the revenue mix, I mean, I'm sure, you know, we don't give forward looking guidance, but Sandhya can comment on the things we're working on.

Sandhya J : So, we are making a judicious choice between balancing cash flows because some of the schemes do not pay on time margins as well as capacity utilization, because if you have spare capacity, you would want to utilize it judiciously. Another aspect on schemes is that there are certain geographies where requires a little bit of scheme support, because there is a certain affordability and a demographic reality of those geographies where we will have to support the geography with certain scheme predominance. So, keeping all those balances in mind, we are taking our scheme choices. So, I don't think we'll be able to guide a number, but I can definitely say that the choices we are making are reflecting in the expansion you've seen in the margins and in our cash flow positions also. So therefore, we will continue to keep playing this balance. But at the same time, we will also be socially focused and relevant. So therefore, we are not aspiring to say that I want to bring it down by 5%, 10%, not like that, because we want to be socially relevant also. So, we will find the right balance as we have done in the past.

Adrith : Okay, fine. Thanks a lot.

15 Nishant Singh : Thanks, Adrith. Yes, Mohit, please ask your question.

Mohit : Hi, sir. Can you hear me?

Viren Shetty : Yeah.

Mohit : Yeah. Thanks for taking my question. So, my question is more on the industry part. So, my question was on the free medical services that government obliges us to provide to EWS section of the society. So firstly, do we come under this? And secondly, what is the total percentage of the beds that needs to be given to the EWS section?

Viren Shetty : Yeah. So that requirement only applies in some variation or the other to three of our hospitals. One is our Dharamshila hospital in East Delhi, where 10% of the beds need to be reserved for EWS category of patient. The other one is SRCC Children's Hospital in Mumbai, where 10% is under the Charity Commission, and Guwahati Hospital where under an arrangement with the government, some proportion of the beds need to be treated under the Assam scheme. So, these are very small by total number of beds and are not that much

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of, they don't contribute that much to the overall proportion of the patient services that we offer.

Mohit : Alright, sir, thank you. Thank you very much.

Viren Shetty : Thank you.

Nishant Singh : Thanks, Mohit. Nancy, can we have your question, please?

Nancy : Hi, sir. Thank you for taking my question. Could you please throw some light on the IndAS 116's impact on our EBITDA and PAT numbers?

Viren Shetty : Give us a minute.

Sandhya J : Yeah. Our EBITDA will increase by, because of IndAS our EBITDA has increased by INR 148.7 million, and our PAT has decreased by INR 19 million for Q1 FY25 on a like to like basis.

Nancy : Sure. Thank you so much.

Nishant Singh : Thanks, Nancy. Prince, can we have your question, please?

16 Prince : Yes, Sir. So, my first question would be, Sir, regarding the debtor days, do we see any improvement in debtor days as elections have gone and we are also surgically planning to reduce the patient scheme?

Sandhya J : So, in Q1 normally debtor days will be slightly adverse for few reasons. One is that most of the government payers, they have their cash flows released in Q4. So, most of the collection

happens in Q4 and Q1 their budgets are not fully approved and, therefore, we don't get the payouts. This year, we also had the added impact of the elections. So, like every Q1, this Q1 also had a little bit of a slowness in terms of collection. Between Q1 of last year to Q1 of this year, we are holding the same levels but between Q4 of last year to Q1 of this year, we have deteriorated by about 7 -8 days, which we will catch up in the next quarter.

Prince: Okay, thank you so much. And regarding the payer days, you have been telling that you have negotiated with the payers and we have seen that the payer days have increased significantly. Can you give some guidance regarding the future that how it would be moving in future? Will it be maintained or will it increase?

Sandhya J: You're referring to days payable/vendor payments, right?

Prince: Yes, Ma'am.

Sandhya J: Okay. I think we will bring it down to levels of 90 days. Right now it is slightly on the higher side. We've gone through a certain ERP migration and because of which there has been a little bit of lag which has got built up there. Between 60 -90 days is what we have negotiated with the payer. So, we will go back to those levels.

Prince: Yes, that will be helpful. And my next question would be regarding the India business. For the India business, revenue growth in the last year has been 10%. And as per your guidance, you are not focusing on aggressive revenue growth but quality revenue growth. So, should we consider in next few years, till the new facilities of Kolkata and Bangalore will be inaugurated, the revenue growth will be around 9% -10% only?

Sandhya J: Can't give a forward looking guidance like that. I think Venkatesh already explained a couple of times the outlook on growth and how we are looking at it. The growth will be organic in the short term. The next big inflection point will come when our capacities come online. Until then, we will grow in line with the market and in line with our own efforts that we are putting in in terms of throughput maximization as well as quality of revenue. So, I think we already answered that.

17 Prince: Yes, Ma'am, thank you. And my last question would be regarding the lease. In FY24 we have seen a slight increase in lease liability, can you please guide is it regarding some clinic facility or something else?

Sandhya J: Yeah. See, to some extent it is because this clinic facility but actually if you see our overall lease costs have come down by almost 0.6%. Now, that is because a lot of the lease contracts which we had taken 5 -8 years back, we have completed those lease obligations and we have unbound those leases. So, our overall lease cost has actually gone down and our overall lease liability has also gone down because

of that. So, I need to see where you are seeing this increase from but there is definitely some amount of lease obligations coming on board because of the clinic but they are not material in the way you're seeing. Maybe our team can offline reconcile with you.

Prince: Sure, Ma'am. Thank you for the answers. I am done with all my questions.

Nishant Singh : Thank you, Prince. Kunal, may we have your question, please?

Kunal: Hi, thanks for the opportunity. First one on our international business, can you share what is our proportion of revenue coming from Bangladesh?

Sandhya J: So, our Q1 FY25 was about 7.6%, which was our international patient. A large part of it, a substantial part of it comes from Bangladesh which will we'll have to see how the situation pans out.

Kunal: But have you seen any reduction in flows in recent months or for you the businesses as usual, at least in July?

Viren Shetty : No, it's not. It has been disrupted during the election time, during the festival season Ramzan and so on. See, but one thing is, consciously as a company we're moving away from a reliance on international medical tourism and more towards domestic patients. So, this is the number over five years, we expect to go down to zero. What will come will come. We're not going above and beyond to maintain these numbers.

Kunal: Then would it be fair to say that for you the profitability of international business is not so much different because all other peers are saying that international business profitability is higher, right?

Viren Shetty : Yeah.

18 Kunal: So, to that extent, why would we don't let it go?

Viren Shetty : We don't distinguish between domestic patients, international patients. So, a patient is a patient. The only distinction pricing comes from the room category and the sort of procedures that they take. So, we've never in the recent, I mean, ever since a few years before COVID we have normalized the prices between domestic and international patients.

Kunal: And Cayman, we do not understand the unwinding impact. So, is it that now that you have paid the lease expense you own certain assets and hence your lease expense is going down?

Because a large part of your EBITDA improvement on a year -on-year basis, EBITDA improvement is driven by the rental expense going down.

Sandhya J: Yes, correct. We have paid those. We have completed the lease tenure and completed the

payouts. We own these assets now and that is why the unwinding of the lease charges have happened. But we've seen improvement across lines, not just on the lease charge line but across lines we've seen improvement in terms of cost. Lease is also one of the contributing factors.

Kunal: So, which asset this is where we have now completed the lease...?

Viren Shetty : This is lease of equipment and not land.

Sandhya J: This is equipment finance. Maybe I could categorize that.

Kunal: Lease equipment. Okay.

Sandhya J: The biomedical equipment which we purchased.

Viren Shetty : This is not asset leased buildings.

Kunal: Sure, sure. Okay -okay. So, currently INR 195 crores of rental expense in this quarter, will we see similar decline? I think last year same quarter would have been INR 235 crores, right.

So, will there be any meaningful declines in the coming quarters due to this?

Sandhya J: Most of the unwinding is done. Most of it is done, Kunal. There is little bit left but most of it is done. So, maybe slight decrease you will see in the next couple of quarters and then this will normalize.

Viren Shetty : But we will be taking up new equipment on lease. So, it will go back up again as and when the new capacity comes on then.

19 Kunal: Sir, are these related to the robotic surgery, if I may ask?

Viren Shetty : It's all the major equipment. The equipment companies give you the option to either buy it upfront or lease it from them. And based on the cashflows, based on the volum

es, we take

a measured call on trying to match that. So, we take lease cum sale options from a lot of these vendors.

Kunal: Sure, Sir. Thank you very much.

Nishant Singh : Thank you, Kunal. Mohit, can we please have your question?

Mohit: No, my question is answered. I'm sorry.

Nishant Singh : Okay, sure. Kunal, is your answer also...because I think you only asked the last few questions, right?

Kunal: Yes, yes. Yes, it is answered. Thank you.

Nishant Singh : Yes, Prashant. Can we have your question, please?

Prashant: Yeah. So, two questions. Firstly, can you give us the profitability EBITDA number at your Delhi, Gurugram and Mumbai hospitals?

R. Venkatesh: So, when you talk about the revenues from... I'll take all the new hospitals together with Delhi and Mumbai, revenue of these new hospitals have been reported around ₹123 crores with a year-on-year and a quarter-on-quarter growth of more than 7% and with the EBITDA margin of 7%. Dharamshila will and continues to give reasonable returns. Gurugram is also mildly positive for this quarter. Mumbai is in a single digit loss, which we expect to stabilize to EBITDA breakeven in Q2 because the opening in Q2 has been pretty good. So, overall, this is the summary of our new hospitals.

Prashant: Okay, thanks. And the second question is regarding your Integrated Care and Health Insurance businesses. What is the outlay in terms of cost that we can see on these two businesses put together, say over the next couple of years? And at what point would they break even? Your assessment?

Sandhya J: So, for now we have invested about ₹100 crores as a capital commitment into the Insurance business and as and when the business grows we will make further commitments and investments into the business depending on the scale and the growth trajectory. As far as 20 the Integrated Care business is concerned, there is a cash burn which is given as part of the analyst deck already and there will be a cash burn because the Integrated Care business is a long term venture and we have to build out this business over a period of time. So, there will be cash burn in this. However, we've kind of kept our cashbook to be as judicious as possible.

May we request everyone else to go on mute because there is a lot of background noise for us here. Okay, thank you.

So, we are being very judicious about the cash burn and we're also keeping ourselves very tightly accountable to breaking even at a singular clinic level at a reasonable timeframe. So, there we are tracking, okay, but because we are opening new clinics, therefore, there is a little bit of a cash burn which is happening. We will continue to build on that. We are not guiding a full number for Integrated Care and Insurance because we are responding to what our customers are asking and the way the demand patterns are coming through. Therefore, we are not putting an outlay out there but we will do whatever it takes to make these businesses successful.

Prashant: Alright, thank you. That's it from me.

Nishant Singh : Thanks, Prashant. Vinay, can we have your question, please?

Vinay: Hello. Just wanted to check out, on your Slide 7 for two years given the ARPOB numbers, is that the right ARPOB number for Q1?

Nishant Singh : Yeah, that's the number for India.

Vinay: So, that is an annualized number or a quarterly number?

Nishant Singh : That's the annualized number.

Vinay: Okay. So, Q1 here it is 15 Mn and last year it was 13.6 Mn. The second part is, when I look at your performance of the hospitals in Western region, they seem to have shown a jump in the overall revenues. Now, you're saying still Mumbai is a loss making proposition.

Sandhya J: Yeah.

Vinay: Okay. So, today we have only two hospitals which are loss making, is it, out of the total?

21 Sandhya J: Actually, if you look at Q1, Gurugram also was not loss making. So, at the moment only Mumbai is loss making. E

ven Mumbai Q4 broke even. Q1 because of the little bit of the seasonality impact, we kind of slide it back into a little bit of loss. Other than that, all our other hospitals have been profitable in Q1.

Vinay: Okay. And lastly, just 7.9% of patients you said who came from Bangladesh, these where in Kolkata region only, right? Or Kolkata and this East?

Sandhya J: 7.6% of our revenue comes from international patients largely from Bangladesh. Most of them come into our Health City campus as well as our Kolkata campus.

Vinay: Okay. So, this is 7.6 of the overall region?

Sandhya J: Yes.

Vinay: Okay, thank you very much.

Nishant Singh : Thanks, Vinay. We have got a few questions from the chat. We'll just like to read out a few questions. Yeah, sorry. Parikshit, you can have your question, please, and then we can move to the chat.

Parikshit: Okay, fine. Okay, great. Thank you. Thanks for the follow up again. And my previous question when I was asking about the volumes, we discussed about how we are changing our payor profile and that is also going to impact our volumes but in return, you know, benefit our margins and I just quickly went back to pull the data. I think our EBITDA margin, and I'm talking about the India business, started moving to about 19% all the way from FY23 itself, right. Q2 of FY23 we hit 19%. And the payor profile also, I'm just looking at the percentage of customers that are from the schemes, in the scheme section of it, that has been hovering around 20% -22% throughout this period. So, are we still changing it or has the transition happened and it's done already?

Viren Shetty : Not done. It's a very dynamic process and it's not just who you're getting in. It is what we are doing for them. And, so, a lot of the work that gets moved towards daycare, a lot of the emergency services, a lot of things that don't count towards occupancy and the pay up and these are cash and insurance patients will contribute towards performance. There are other efficiencies we're getting from throughput and being able to manage a lot more tasks with the same manpower which do contribute towards performance.

22 So, the payor mix, sometimes the hospital we have are locked into a certain payor mix. So, Ahmedabad, for example, we run a hospital that is in a poorer part of town and that is a State that has a very high contribution from the government for doing the surgery. So, that will always have a disproportionately high proportion of patients coming from scheme. Whereas our hospital in Gurugram, for example, it's in Cybercity Phase -II. It's built just as a private, semi-private sort of hospital. So, it barely has any contribution coming from the government program. So, it's not something we are consciously looking at. What we're trying to do is improving the cashflows of each hospital looking at the patient base that makes sense for that place.

Parikshit: Okay, alright. Thank you.

Nishant Singh : Thanks, Parikshit. Now we can take up the questions from the chatroom. The first question is that, Is ROCE significantly different in case of capital intensive SBUs like Oncology and Cardiac treatments compared to less capital intensive areas like Neurology?

Sandhya J: See, actually ROCE is not just about the biomedical equipment. The cost of a bed when we say is say between ₹1.5 - ₹2 crores, it is the cost of the infrastructure, cost of the bed, cost of the biomedical equipment, cost of the diagnostics that go into it. So, it's a large investment that goes into getting a hospital or a bed up and commissioned. And ROCE is a factor of that. So, if you just look at the biomedical equipment of a particular specialty versus another, that is not a big player in the calculation of the ROCE.

Nishant Singh : Okay. There is another question, are you looking for a finance partner in the Health Insurance business or do you plan to invest your own money into this Insurance business?

ture?

What are the reasons why you are choosing the option that you have chosen?

Viren Shetty : So, at this point we're not looking for a partner for Health Insurance. We're still building this up and since we're doing it in a very moderated way, phased out across our different hospitals it's something we believe at least for the near future until it proves itself like we're able to build an insurance plan that's sustainable, that provides a lot of value for patients

and improve their outcomes, we won't want to look at scaling it nationwide.

Why we are choosing this option is because the market has enough providers who will just sell you a policy and not ask any questions and that does lead to a lot of dissatisfaction with premium holders. So, we want to provide something that is a policy that has no questions asked. Once you're in, we shouldn't have to doubt the intentions of the patient coming in for any sort of procedure. And we take a more proactive role in managing it. That's why it's positioned very differently and a lot of financial investors may not be fully aligned with that way of running the business.

Nishant Singh : There is another question on Insurance which Ravi can take. How long would the Insurance business take to breakeven on this INR 100 crores initial investment as per your business model to get positive ROI?

Ravi Vishwanath: Right. I mean, I think it's might be a bit premature to comment on that because as Viren just said, you know, we are kind of positioning ourselves very differently. And the important thing to keep in mind is that we are starting in a very modest way in Mysore and four districts around that and we will expand gradually and kind of see how the market reacts in response. And I'll just repeat what Sandhya has said earlier, which is that we're doing this very prudently and looking to make sure that we extend our capital as much as we can. So, a little bit premature to say that. But, hopefully, we'll be able to share more information with you in coming quarters.

Viren Shetty : The question is from Riddhi about how do you assess the performance of your administrators? We give them targets and they operate in an operating budget every year and there are multiple parameters for assessing them, not just on hitting the financial parameters but also on keeping the cost within line, improving the digitization, reducing the length of stay and improving the throughput of the hospital. So, at the senior levels, they have a 80 -20 fixed to variable pay structure which incentivizes them towards that.

Question on what type of business intelligence, AI tools are used for measuring performance? Those are 2 different things. The business intelligence, we were on Microsoft Power Bi. We moved to an in-grown application developed by our analytics team called Medha. The name of the, at least what they have called their BI tools so far is AERO. This is something that we use extensively for our internal performance, benchmarking measurement, not just for business operations but also for clinical operations and supply chain and so on. And it is a tool that is deployed across few other hospitals as well. People pay to use these services.

For the healthcare professionals, other than using the analytics, they also use our in-grown hospital information system. That's called AT HMA. And AT HMA has multiple applications on it which include an app for nurses called NAMAHA, an app for doctors called AADI. It has its own analytics program called ATMI TRICKS. It also is deployed across all our hospitals and clinics as well as this is being sold to other hospitals and clinics outside of our network.

Other questions – the ARPOB for Gurgaon region.....

Sandhya J: Yes. So we aren't specifically sharing out particular hospital but in the North region, the ARPOB is about 16.6 Mn for Q1 FY25. Actually North, when I take north, it's slightly higher for the Delhi a

nd Gurgaon units and slightly lower for the Jaipur units but in that range.

Viren Shetty : The last one is occupancy percentage in India and Cayman.

Sandhya J: For India, it's not a number that we are sharing, like as a separate number because we have always said that occupancy, the way it is measured, midnight occupancy is not a representative of evolution of our business. Having said that, we always operate in the range of between 60 -65pc on midnight occupancy basis. Daytime occupancy and day care occupancy will be much better, so in Q1 and Q3 quarters, we kind of end up in the lower end of the range and in Q2 and Q4 quarters, we normally hit the upper end of the range and that's how we have come in this quarter also. In some of our mature hospitals, we will be at a much higher number than that and then in some of our smaller hospitals, the number will be lower.

Viren Shetty : There is a question earlier on from someone who is new to the conference call. When you say inflection point, when you see better growth, what will that be? This we have mentioned earlier and we will mention it again, this is when all the new hospitals come online, so that will take about 3 years from now.

De-bottlenecking in adding infrastructure, so debottlenecking, that refers to the fact that a lot of our patients wait, they spend a lot of time waiting in our hospitals, waiting for the doctor, waiting for the tests, waiting to get a bed, waiting for certain services and so a lot of that is addressed through digitization and process improvement initiatives and what it does is, it increases the throughput. We can see more patients per given day as well as reduce their length of stay. So we are trying to achieve best in class, length of stay of 3.5. We have some way to go and lot of it we are doing helps improve the flow.

What do you consider as quality revenues, is a question? So, quality comes in various kinds but essentially you can simplify it as payers that pay on time and you are able to cover most

of the cost incurred in providing the services. Now in a perfect world, you could run a hospital just with patients paying cash up front and paying it at a 30 -40pc margin to 25 whatever services you are providing. But that doesn't happen in the real world. Running in a country like India, we have to cater to multiple payer classes. And so within those payer classes, we try to rationalize certain payers who may have a very poor track record of paying us on time or have a huge track record of disallowing certain things and we would try to mix and match, the patient to the services that make more sense for that.

Nishant Singh : The question on the ARPOB number for north. We mentioned 16.6 Mn, that is the 1.6 6 crores for the year , that is the annual ARPOB. There is a question that it is much lesser than the north where it is 50,000. So if you make it a daily ARPOB, that number comes to around 45000 that we have. So it's not that much lower as compared to what others are making in the north.

Viren Shetty : But having said that, we do peg our services to be at a slight discount to what is available everywhere else and we try not to be at the top of market in terms of our pricing for patients on a like to like basis.

Nishant Singh : Our next question is, how many total new hospitals are planned for the next 3 yrs.?

Sandhya J: It is still work in progress. As and when we keep identifying opportunities, I think we will keep sharing. We have indicated a certain capex for the current year and we are expecting that we will spend a similar amount as capex over the next 2 -3 years and therefore, there will be significant investment that will happen. We will share more details as and when we will make those choices.

Viren Shetty : As and when they come up, we announce like we announced in Kolkata , we have announced in Bangalore and certain capacities being added in Raipur. So those are

what....so far we are giving clarity on.

Sandhya J: There is a suggestion on bonus shares issue. So we thank you for that suggestion. We have tried to stay as consistent as possible in terms of our return to shareholders in dividends etc. but we will take your suggestion on Board and we will evaluate them on its own merit. Thank you for that. There are a lot of questions on chat, but they have been answered through questions which have already been asked. So, if we have missed to answer any question, we request you to please put your hands up and we will be happy to answer.

Nishant Singh : I think Gagan has questions. Gagan, can you please raise your question.

26 Gagan: So, the 1st question is on the tax rate. I think the indication at the close of last financial year was that the tax rate will go up. It will be around 13pc, year on year, it is slightly up but still low. So, any explanations there.

Sandhya J: We had some one -timers in the last year because there was a deferred tax credit we got because we shifted from the old regime to the new regime. That came last year. So that benefit is not there but otherwise, what tax rate you are seeing right now is the steady state tax rate, that we are estimating.

Gagan: So if I got it correctly, the 1st quarter tax rate is at 13pc. is that what you are saying can be extrapolated for the whole year?

Sandhya J: Yes, 13pc is correct. If you see, last year in the 1st quarter was 10.6pc. 2nd and 3rd quarters were 8.7 and 9.8. For the full year, last year we were at 11. 13pc. That was the effect of the deferred tax credit that came in in last year. So that is not repeating. Therefore we have kind of come on to that 13pc kind of range. One thing you will have to keep in mind is that it will vary on the mix effect because the India revenue is at a higher tax rate and Cayman profit is at a no tax rate and therefore this is the average. So if the mix changes between India's profitability and Cayman's profitability, then accordingly the tax rate will also get moderated.

Gagan: Ya, ok. the 2nd one pertains to your existing facility in Caymans. If I understand it correctly, the occupancy there is still fairly low. Right? It's a 100 bed facility and if I remember it correctly and I think last 2 indicated the occupancy tends to be 50 to 55 odd something out there. And you indicated in your comments in the call that there is not much headroom for growth there. So is the constraint there more demand related constraint or location related constraint or am I understanding the capacity itself incorrectly?

Dr. Anesh Shetty : No Gagan, you are right on the capacity. The constraint is that the number of specialty service lines we can offer, in that facility we reached the limit. The new service lines we are going to start in the new hospital. We cannot offer it in the existing hospitals. The 2nd constraint is obviously the location. As we have discussed before, the existing hospital is located very far away from centre of the city where most patients live and work, most people live and work. So by moving to a very very prime location, we become a lot more accessible to patients who otherwise would not consider driving so far for smaller services or entry services. But the primary constraint is just the services we can offer, the new specialties that we are going to offer cannot be offered in the existing one.

27 Gagan: How should I put it? A very layman like but my question is, if best case utilization at this facility cannot exceed what it is, does it make sense or is it even a possibility

Dr. Anesh Shetty : Gagan, sorry, could you repeat that please? I am not able to hear you.

Gagan: My question to you is that, if you don't see occupancies in your existing hospital in Cayman improving, is it then a possibility to maybe reduce the bed count and perhaps utilize the additional space that you get in some other way in that hospital?

pital?

Dr. Anesh Shetty : Ok, understood. No, it won't work like that because when the new hospital comes, the existing facility will also see an increase in occupancy. They are not 2 distinct hospitals, they are just 2 campuses, think of them as just 2 locations for the same unit. It's the many of the doctors overlap but there are some new specialists also but when we have the new hospital, we absolutely expect the occupancy of the existing hospital to increase meaningfully. So it just wouldn't make sense to reduce the number of beds in the existing hospital. Having said that, it's more an issue of, it's the opportunity cost, unless we start the new services, there is nothing that's going to happen just by removing beds. We will have to add either equipment or re-equip the facility etc. But none of that is needed because once the new hospital is there, once we start the new services, we will see an increase in occupancy in the existing hospital. The new hospital will be primarily for patients who are short stay. The existing hospital will be for patients who have a long or more complicated stay.

Gagan: So you are saying that the new hospital will act like a catchment to the

Dr. Anesh Shetty : Yes absolutely. It will also render services of a particular nature in that facility but it will also significantly feed into the existing hospital.

Gagan: Right! Final one, again on the facility one. While one understands that your 1st venture when came in took a reasonable amount of time to mature. Today, you have credentials, you are not an upstart there and therefore, is it reasonable to then assume that breaking even in the new facility will take a lot of time frame compared to the 1st one.

Dr. Anesh Shetty : Yes, you are absolutely right. That is the expectation we have.

Gagan: Is it possible for you to perhaps give us some understanding of, at least from an intent standpoint or an anticipation standpoint, how much duration the break-even period or the pay back period potentially be on the new one?

Dr. Anesh Shetty : It's not something that we disclose or like to but having said that, I think you just have to wait for the next quarter and then we will have a much idea of where things stand. Even I am far away from knowing the actual answer.

Gagan: Thank you and finally on your comments, you intend to keep the capex at 1500 -1600 crore run rate for the next 3 years. One, the debt hasn't really moved too much in the 1st quarter.

How should we think of leveraging towards the end or closure of this year ? Does it continuously keep increasing from hereon over the next 3 years and to what levels?

Sandhya J: Yes, that is the right assumption to make Gagan. The reason the debt hasn't moved is because the capex hasn't moved. A lot of our capex commitments is in Greenfield which means that we have to identify the land and then go through the plan approval process which is a long process and then start to build and therefore the consumption of that capex is little slow and that is also reflecting in the debt that we are picking on though. We will for whatever capex we have indicated, significant part of that will be through debt only, so you can assume that at least 80pc of the capex we are indicating will come through debt and to that extent, the leverage will be, it will increase.

Gagan: Have you considered equity raise as an option or that's completely off the books and you intend to completely use debt in internal accruals?

Viren Shetty : It's not something that we need to look at right now, given how under levered we are and how much cash we still have on the books. Nature of this sort of capex is that it operates at a low intensity at our choosing and it goes in a very staggered manner over many years. So if we are able to match our internal cash flows plus the traditional ones, from banks and from NCDs to match the capex and the green field and land construction and all as and when

it comes. Should we get to a point where we have the ability to service our ambition is no longer matched through our cash flow and borrowings, then we would look at an equity route. But at this point, it's not really required.

Gagan: Of the total cash flows that you accrue annually, is the ratio of cash flow accrual from India and Cayman be very much in line with the EBITDA that you generate in India and Cayman? Or is that also sort of different from that?

Sandhya J: Cayman has just gone through an intensive capex cycle and therefore there is some amount of cash flow that's got deployed in that capex and obviously, there is a lot of borrowing that has been taken on board, so the repayment of that will kick-in in Cayman. That's from a Cayman point of view. As far as India is concerned, the cash flow generation is in line with the EBITDA but like how we choose to invest in terms of capex etc. accordingly the cash flows will get moderated.

Gagan: What will be the repayment schedule at your Cayman, I mean over what time frame do you pay off the loans?

Nishant Singh : In Cayman, we have a mix of the new loans and the old loans. The new loans are almost at the expiry stage than the old loans. The new loans will expire in the next 3 -5yrs. The average maturity is around 3 .5yrs., the large maturity is around 5.5yrs.

Gagan: Thanks, I will get back in the queue. Thank you.

Viren Shetty : Riddhi , there is a question from your side.

Riddhi : Hi! Can you repeat on the capex expansion for Bengaluru and Kolkata?

Viren Shetty : In Bangalore, we are adding an annex to our cardiac building. It's a piece of land we bought next door. This will focus mostly on Aneurysm surgery and very advanced valve repair surgery to expand the capabilities of our flagship cardiac hospital. We also bought a 1.5 acre piece of land in HSR layout, this is a suburb of Bangalore where we have an existing hospital whose capacity is quite filled up, so this will operate as an extension of that hospital as well. This will be a green field hospital that will accommodate about 200 beds or so. this is for Bangalore. Anything else you wanted to know about?

Riddhi : Kolkata as well.

Virent Shetty : For Kolkata , we have got land in Raja rhat. This is a new suburb of the city and there we bought a very large piece of land. We will make a very large hospital over there in faces. Indicatively, this could go up to about 600 beds but we will do it in 2 phases and plan the action accordingly.

Riddhi : Ok, and the capex amount is 1000cr. Right? For Kolkata?

Viren Shetty : Yes. That is the amount we expect it should cost us, yes.

Nishant Singh : The total cost of the project for the next 10yrs, is going to be more but for the 1st phase, it will cost around 900cr,

Riddhi : Ok, thank you.

30 Nishant Singh : There is a last question from the chart – the Bangalore and Kolkata hospitals will get fully operational in the next 3yrs. That's the question. So, we will start the construction by the end of this year in Kolkata and Bangalore will be another couple of months after that. So from the start to end, it will be around 3.5 -4yrs.

Nishant Singh : As there are no more questions, we would like to end our session. We thank you all for the active participation as usual. Thank you.

END OF TRANSCRIPT

Call: Aug 2024

"Narayana Hrudayalaya Limited
Q1 FY25 Earnings Conference Call"

August 6th, 2024

MANAGEMENT : MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – GROUP CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – GROUP CHIEF OPERATING OFFICER

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS
SUBSIDIARY HCCI

MR. RAVI VISHWANATH – CHIEF EXECUTIVE OFFICER , NHIC

MR. NISHANT SINGH – VICE PRESIDENT , FINANCE , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. VIVEK AGARWAL , SENIOR MANAGER , INVESTOR RELATIONS

2 Nishant Singh: Hello everyone. My name is Nishant Singh. I head the investor relations functions at Narayana Hrudayalaya. I welcome you all to the Q1 FY25 earnings call of the company . To discuss our performance , as usual, we have Mr. Viren Shetty, our Vice Chairman , Dr. Emmanuel Rupert, our CEO and MD, Mrs. Sandhya Jayaraman our Group CFO, Mr. Venkatesh, our Group COO, Dr. An esh Shetty, MD of our overseas subsidiary, HCCI, Mr. Ravi Vishwanath, CEO of NHIC , and Vivek Agarwal, Senior Manager in the IR Function. As usual, before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the of the same shall be made available on a website as well as on the Stock Exchange at a later date . I would also like to remind you that everything that is being said on this call, that reflects any outlook for the future or which can be construed as a forward -looking statement , must be viewed in conjunction with the uncertainties and the risks that they face. Post the call should you have any further queries, please get in touch with us , we would like to address them to the best of our ability . With that now, I would like to hand over the call to Dr. Rupert.

Dr. Emmanuel Rupert: Good evening , everyone. I warmly welcome you to the quarter one FY25 Earnings Call Conference of Narayana Hrudayalaya Limited . We are pleased to report the highest ever revenue at a quarterly basis at the group level with a sustainable profitability margins. Despite being a seasonally weak quarter due to summer holidays , Eid, and the impact of general elections, we are able to drive improvements in realizations and increased patient footfalls. Consolidated revenue for the quarters to that INR 13,410 million reflecting a growth of 8.7% year -on-year and 4.8% quarter -on-quarter . Narayana Hrudayalaya generated a consolidated EBITDA of INR 3,274 million at a margin of 24.4% against 23.2% in Q1 FY24. HCCI continues to deliver strong business performance with quarterly revenues of USD 32 million a year -on-year growth of 4.8% and a quarter -on-quarter growth of 5.8%. We're delighted to share that our new hospital in Caman a Bay was inaugurated in July, with patient inflow expected to start before the end of the second quarter. We are confident that our Caribbean business will continue to grow through synergies between the two hospitals further strategic initiatives and investments in this fiscal year. The balance sheet and liquidity profile at the group level remains strong with group cash and liquid investments of over INR 13.2 billion against the gross borrowing of INR 14.75 billion, resulting in the net debt position of INR 1.55 billion as of 30th June 2024 . A net debt to equity ratio at 0.05 gives us sufficient room to fund expansion through a mix of borrowings and internal accruals. We have incurred a capital outlay of INR 2.8 billion directed towards purchase of land in Bangalore for Greenfield expansion, regular maintenance, biomedical upgrades , and interior 3 transformation work within existing hospitals. We are confident of realizing the projected Capex for the fiscal year as we take on more Greenfield projects during the rest of the year. On the clinical front, we have had many success stories and increase in the complexities of

the work . The Health City, Bangalore successfully performed 3D printed patient specific implant for a total knee replacement in a dwarf patient and a navigation guided pelvic bone tumor resection , which is a state -of-the-art technique for bone cancer surgeries which only very few hospitals are capable of performing it globally . The R N Tagore Hospital, Kolkata successfully performed what is called as a paintbrush technique for calling of a patient who had an aneurysm in the brain , which is the left middle cerebral artery bifurcation aneurysm and they also did a robot -assis

ted kidney transplantation as well. During this quarter, across the group hospitals, we performed 40 TAVIs and 389 robotic surgeries across the entire Group.

Our focus on digitization and business transformation continues to lead to significant benefits throughout the system. We launched an AI system which auto creates discharge summaries of patients stay as an initial step , so that the doctors can screen them before giving the final report. The AI alert feature within the NAMA H app, which is a nursing app, triggers early warning signals to the nurses, who can then reach out to the doctors for the timely attention. We also recently launched Virtual Tumor Board for Oncology where the various special list doctors can come from across all the centers on the same virtual platform and discuss the diagnostics and treatment on a real time basis with all the history of the patient in one place . Our digital appointment management system has been revamped and now includes the N H app, the website, queue management and Kiosks for OPD consultations . In the units where all these are deployed , we're witnessing more than 60% of the consultations being booked digitally, leading to reduced waiting time for patients , and freeing up manpower and valuable space . Deployment of Athma in Emergency Department has reduced the ALOS by 50% in the emergency room by digitizing all the time - consuming documentation process.

Narayana Health Integrated Care continues to perform as per our growth plan. Revenue for the quarter has crossed INR 80 million , which is the highest so far with more than 51,000 patient transaction across 8 clinics . With the confidence start in its first year, we will continue to grow this business and serve our customers with a clear focus on improving the health outcomes.

4 We continue to upgrade our clinical and non -clinical operations across the group, transform the patient service levels, increase the throughput, build more capacity, invest in more digital patient outreach channels , and improve our operational efficiency.

We are reasonably on track on our ESG commitments and continue to focus on creating meaningful social impact in addition to pursuing our environmental goals and upholding the highest standards of governance . We are simultaneously pursuing organic and inorganic growth opportunities growth both in India and overseas that will derive synergies from our existing operations, maximize value for all our stakeholders while keeping a close watch on return on capital . Thank you.

Nishant Singh: Thank you, Sir. I would now request everyone to now use the raise hand feature to start posing their questions. If anyone wants to ask a question through the chat, please use the NH investors relations chat room. Thank you. Yes, Prince can we have your question please?

Prince : Yes, good evening, everybody. Sir, my first question would be regarding the new hospital in Camana Bay . So , as the Cayman being a suitable geography for NH, what will be the expected occupancy for the new hospital?

Dr. Anesh Shetty : Yeah. Hi, Prince. Thank you for your question. So , the hospital will start in a couple of months. We've already inaugurated it. We have about 50 odd beds , that's our official bed count. You know, we hope to be occupied from the early quarters, but at the end of the day, this is something we'll have to wait for a few months to see. We won't be able to project what the occupancy levels could be, but this is in a good location. We have all the inputs to suggest that the hospital will be occupied as per our expectation.

Prince : Can I get a range?

Dr. Anesh Shetty : It's hard to say. Having said that, it's also important to consider that you know the occupancy as a metric for revenue growth is less and less important these days, especially because it's a daycare focused hospital , but let's see in a few months.

Prin

ce : Sir, my second question is

Viren Shetty : Prince , you're disconnected. Could you repeat the question?

Prince : Yes Sir. So , I'm audible now ? Hello?

Nishant Singh: Yeah, go ahead, go ahead .

5 Nishant Singh : Yes, you are audible .

Prince : Yes. Can I get the CapEx break up for the Bengaluru expansion and Kolkata expansion?

Viren Shetty : This CapEx breakup of Q1 ?

Prince : No. Going forward, we are planning to for the Brownfield expansion in Bangalore and Greenfield in Kolkata. So , can I get what CapEx we will be keeping for both the hospitals?

Nishant Singh : Yeah. For the Kolkata Greenfield one, the first phase project estimate is almost around INR 1000 crores, which includes the building of around 350 beds in the first phase and there will be a lot of infrastructure development for the entire for the 1000 beds as well, but the first phase will cost around INR 950 crores. For the Bangalore land, cost will be around INR 500 crores.

Prince : And what would be the expected capacity of beds, Kolkata and Bangalore ?

Viren Shetty : This is yet to be finalized.

Prince : Okay. Is there any expansion in future , we could be doing with the leased land?

Viren Shetty : Which land? Yes, we would explore different options and models . Right now , these are the projects that are in hand , the one in Bangalore and Kolkata , but we are talking with various projects and developers to look at leasing options as well.

Prince: Okay. Thank you. I'm done with my questions.

Viren Shetty : Thank you.

Nishant Singh: Yes, Damayanti, can we have your question please ?

Damayanti : Hello . Good afternoon. So , my first question is again on the new facility in Cayman. So , just want to understand it better. You mentioned it's mostly outpatient set up where you're focusing on preventive care etc. So, my question is how do you plan to ramp up your operations , focusing on which key segments and what kind of initial losses you expect to incur from this facility till it scale up to a reasonable level ? So, that's my first question.

Dr. Anesh Shetty : Sure. So , just a slight correction there. Yes, it is predominantly focused on daycare procedures, but these are not necessarily preventive, they could be diagnostic preventive , but these are mainly short -stay surgeries, quicker surgeries, robotic surgeries, these kinds of things. The primary goal of that facility is to offer the services we currently do not offer , the major gaps being Emergency and Trauma Care , Obstetrics and Women's Health, Neonatal Intensive Care , etc. So , you know when we start there, these would be the services we will look to lead with the services. Your question on the second part of your question around you know, operational losses , as we've said before, it's a new building in Cayman. Any hospital we have in Cayman including the existing one has very high fixed cost. So , initially there will be obviously, certainly margin dilution and operational losses . We are not in a position to, you know to give out a particular number or a hard value to quantify that , but they're actually, you can obviously expect operation losses and margin dilution for some time .

Damayanti : Sure and if you can also update us on the existing facility about any improvement in IP or any volume pickup etc., what are you seeing on the existing Cayman unit?

Dr. Anesh Shetty : Sure. So , in the existing Cayman Unit , we continue to see growth on a revenue basis and some growth in good quarters on volumes , but as you can imagine, we are reaching a sort of saturation point in the existing facility and that's why the timing for the new facility is good and very much needed. We have undergone some operational improvements in the existing facility that's why you can see the good improvement in margin, but you will see a slowdown in volume and revenue growth until we commission a new

facility.

Damayanti : Sure. That's helpful. My second question is on India operations. So , on the IP volume part, we are seeing a bit of gradual pickup , which you earlier attributed to some reconfiguration of beds or facility upgrade, etc. So , my question is, are you broadly done with your initiative and how should we see IP volumes moving up from here on?

Viren Shetty : Can I ask Venkatesh to take this one.

R Venkatesh : Yeah. When it comes to the reconfiguration, basically in terms of infrastructural upgrade , transformation, these are something which is an ongoing exercise and we still have a little bit more to go before we come to a conclusion on that. Of course, when it comes to the IP numbers , we all know that we are in an expansion phase and these capacity expansions obviously will kick in, say, in the next three years' time . Till such time , it is important for us to make sure that through these transformations, these efficiency improvements, we keep improving on the numbers. So , we've got a long way to go before we run out of these benefits or efficiency improvement and we've actually started reaping these benefits only now in terms of more OPD visits, procedures, quicker discharges, lab reports and all. So , what I would say is that while we look into the volumes , we are focused more on quality of revenue . Our margins actually because of the quality of revenue has shown more positive trajectory in this Q1 FY25 as compared to Q1 FY24 by more than 1.1% and we are actually taking a balanced approach between cash flow , profitability , and growth. We've done a lot of work, as I've said, on changing our bed mix, the digital journey throughput, and we are also not very aggressive on the pricing. Therefore, our buildup has always been very slow or has been slow, but very steady . So , the current growth will be normal growth and we will get the next point of inflection when our new capacities kick in , in few years from now.

Damayanti : Sure. That's helpful. My third Point is actually a request. So , in your presentation you have now started reporting ARP P for clusters, so that's helpful, but request you to also share volume number because that I guess will help us to understand your business improvement

in a better way ? So, earlier like you used to give ARPO B, but now we have OP and IP , ARPP, but maybe volume numbers will be also helpful ? Thank you.

Sandhya J: We can share. Actually , there's one more question on the chat and I'll take both of it together. We've been asked whether ARPOB is not being shared and can it be shared ? So, we will share the volume numbers and we'll also work with you offline in terms of how to derive the ARPOB straight forward number , but we will share that volume and ARPO B calculation.

Damayanti : Okay . Thank you. I'll get back in the queue.

Nishant Singh: Thanks Damayanti . Nitant, can we have your question please?

Nitant : Congratulations to the management on the great set of numbers . My first question was regarding the insurance business, which was supposed to commence in Mysore. So , what are the updates on the insurance business commencement?

Viren Shetty : Ravi, if you can comment on the insurance ?

Ravi Vishwanath : Hi N itant, thanks for your question. This is Ravi Vishwanath. So , on insurance, I'm please d let you know that we launched Na raya na Health Insurance in Mysore ass we have said in late June with our first product it's called Aditi, the headline feature which is providing 1 crore cover for surgeries for a price of about ■10,000 for a family of four , where the oldest member is 45. You're also asked about on regulations and things like this. So , yeah, I mean, our focus has been on making sure that our systems, processes, data flows, etc. are compliant and in line with the regulations, including the most recent changes , which have been several by the IRDA

and we're ha ppy to report that, you know, all those things are in 8 place and working as expected, which now gives us a good base from which to focus on making sure more and more customers are able to take advantage of our products.

Nitant : Alright . Thank you. Another question was that you mentioned that you were venturing into new specialty whether ortho spine, neurosciences , GI Sciences , so has that impacted your revenues in this quarter or will this affect in the quarters to come?

Dr. Emanuel Rupert: Yeah, t hese are not new departments per se, but we're just increasing the spectrum of work so that we can do a little more complex spectrum. We've just recently installed the robotic spine program and the equipment and we've done around 5 cases with that very complex cases, but over the next few quarters it will start, we will see some impact on the complexities of this patients as well as spine is concern. So , all those things is a routine stuff, which we are doing and it will, but it's not someth ing that dramatically, it's not a new department, it just adds to a different spectrum within the same department.

Nitant : Alright. Just one last question regarding the Kolkata facility. What is the updates regarding the commissioning of the facilities?

R Venkatesh : Yeah. This is the Raja rhat facility in Newtown, Kolkata that you are talking about. So , we are currently seeking approvals for plans from the authorities. The work is going up in full flow. We expect the approvals to come by October and we hope to start the construction towards the end of Q3 this year . So, we will try and speed it up, but mostly what it looks like by the end of Q3 this year, we'll be in a position to do the groundbreaking in for the Kolkata projects.

Nitant : Thank you. That was helpful .

Nishant Singh: Yeah. Thanks, N itant. Parikshit, can we have your question , please ?

Parikshit: Hi, good afternoon. Thank you for the opportunity. I was trying to understand the volume number, the footfall number, and we can break it down between India and the Cayman. So, to begin with, in India our inpatient numbers have not been growing for the las t 3 quarters. Is there any particular reason for that?

R. Venkatesh: What I would say first point is, if you look at this quarter, in our health city, in the major facility there, there is a little bit of a dip. Q1 generally is known to be a low quarter because of the summer holidays, Ramzan period but this year, due to the Indian elections, there was a phase wise election throughout the months of April, May and some part of June. And since our health city sees a lot of these patient inflows happening not only from Bangalore, but 9 from other parts of Karnataka and also from other states, these numbers got tempered a bit due to the election, which will obviously get corrected in the coming quarters. But of course, what I would want to reiterate is, along with volumes more than that, I have said I'm just repeating it again, that we are focusing more on quality of revenues. So as a result of that, if you see, even if the volumes are not growing substantially, there are good quality of revenue, which is resulting in our margins showing a good growth from 18.3% last year, Q1 to 19.4% this year. Obviously, we have started working a lot in terms of changing our bed mix, upgrading it to single rooms, twin sharing rooms. So, within the same volumes, we are able to recover more quality revenue. And of course, it is also a constant endeavor for us to make sure we work on increasing volumes, and we are confident that eventually in the course of the next few quarters we'll be in a position to get more traction from the volumes. But, the current gro wth, what we are having will be very normal growth, of course, double digit growth, year -on-year, mostly. But the major inflection point will happen when these

capacities come in the next 2 t

o 3 years' time. Till such a time, we will see very normal growth, but we will work more on efficiencies to ensure much healthy margins and bottom line.

Sandhya J : Just adding to the data point, actually, we saw about 26 million patients in FY23, now we've seen about 28 million in FY24 and based on the Q1 run rate, if you extrapolate, we'll see upward of 30 million patients in terms of the number of people who are visiting us. And so, there is that footfall in volume that is coming through. But everything else, I think Venkatesh nicely explained.

Parikshit : Thank you. So let me just follow up. So, I appreciate that there is growth and that's why I'm actually breaking down the numbers between outpatient and inpatient and my question was particularly towards the inpatient, because that's the one that hasn't been growing for the last 3 quarters. It was not just this quarter which might be election dependent, right. I appreciate the point you're making that you're trying to change your pay or mix, and hence that is impacting the volumes to some degree. But that also would imply that your margins are improving. So even though you're saying that you will see a normal double digit but a normal revenue growth, you should see a slightly more faster EPS growth. Is that a fair statement?

Sandhya J: See, just one maybe data point I want to clarify, though, the general trend, I think Venkatesh spoke about Q3 and Q4, we saw about 56,000 patients in our inpatient, and this quarter we saw about 59,000 patients in our inpatient. So even though Q1 is a weak quarter for us in terms of inpatient volume, we were okay. So just that's a little bit of clarification on the data point. But in general, I think there is a little bit of investment that we're making in terms of balancing between quality of revenue, between healthy cash flows and in terms of growth and that balance is reflecting in the numbers that you are seeing.

Parikshit: Got it. And so, while I appreciate that we are tempering our volumes with the pay or mix in India, the Cayman number also in terms of footfall, has not been a great quarter for us, right. So, the inpatient has year -on-year degrown by -7% and the outpatient has also just grown by 6.5% year -on-year as compared to some of the previous quarters we were growing very strong double digits. So, what's the pattern, the trend we're seeing in the Caymans in terms of volume?

Dr. Anesh Shetty: Yeah. Parikshit, I think you are largely right. The fact remains that, you know, as I said in the earlier question as well, we have sort of reached a point of where we will not see double digit growth with just the existing facility and that's why about two and a half, three years ago, we started planning the new facility, which should be commissioned anytime now. We have inaugurated it. It should be ready. We should be treating patients in a couple of months or hopefully sooner in a few weeks. And at that point in time, we will see the next wave of growth. But in the existing facility, in the existing location with some other market dynamics at play, we will just see this small incremental mid -single digit growth and some quarters up or down, like for example, this quarter. But the next leap forward will only come with the new facility.

Parikshit: Sorry, Anesh, and I appreciate you saying that I've obviously heard you say this. The reason I'm being so stubborn about pushing this point is that in the previous quarters our same facility was capable of handling a much larger volume. So, it doesn't seem like an infrastructure bottleneck, right. Because in the previous 3 quarters we've done higher numbers?

Dr. Anesh Shetty: Sure, sure. It's not an infrastructure. It's more of a market dynamics situation. So, the first point is, you know, as in Cayman, my request would be to always look at a trend over 2 or 3 quarters because the base is so low. There are silly reasons

that could skew a quarter up or

down, if a couple of people around leave or if there was a bad weather, like we had a hurricane that came pretty close, etc. So don't read too much into 1 quarter. I would say 2 or 3 quarters to get a general trend is true and if you see 2 or 3 quarters, you will see a sort of stagnation in the IP volumes and a moderate growth in the outpatient volumes. So that is the picture over a couple of quarters.

11 Parikshit: Alright. So, am I reading it wrong because I think the outpatient growth has been fantastic over the last few quarters, right?

Dr. Anesh Shetty: See, there are several factors to it. Yes, the outpatient growth has been there. I wouldn't say, too fantastic. The reason being we have started changing a lot of the procedures that would traditionally be inpatient into outpatient. This is in line with our efficiency initiatives, etc. So, some of it is a reclassification from IP to OP. The other thing also is we have commissioned new service lines, like we did an ENT acquisition, which is outpatient heavy about a year ago. So, this is what we expect, outpatient growth is good. Yes. It can be better. Inpatient growth is what it is given the constraints we have until the new facilities is up and running.

Parikshit: Got it. And if you don't mind me following up, you mentioned that besides the new facility opening up, you are also seeing, there was hinting on some market dynamic. Is there a new competitor there that has opened up or what market dynamics?

Dr. Anesh Shetty: No, no, no, I'm saying whatever the existing market dynamic is with the location we are in and with the other players who are already there, there's no new change to it.

Parikshit: Okay. Alright. Thank you so much.

Dr. Anesh Shetty: Thank you.

Nishant Singh: Thanks, Parikshit. Uday, can we have your question, please?

Uday: Hello. Thank you for the opportunity. Just wanted to get some color on the insurance product that you have launched. Can you highlight some differentiator in terms of underwriting that we are doing in this product? Because it's a very different product compared to a normal health insurance that we see in the market?

Ravi Vishwanath: Sure. Let me answer that as well. So, I think, our philosophy is that we want to make the claims process as simple as possible and as I guess based on your question, you understand that, largely the way the market operates today is, that underwriting effectively happens in the time of claim which can lead to unhappy experiences for the customer. We want to do it differently. We want to be sure that we underwrite at the time of acceptance, and make sure that the customer has got absolutely no issues when it comes to claim. So, every customer for our product undergoes a full physical examination. It happens at our facility through our doctors. It's a very smooth process that we put in place. It is provided completely free of charge to the customer whether we accept the customer or not. So even if we are not able to accept the customer for some unusual reasons, our goal is to accept as many people as possible. You know, it is still provided to the customer at no charge. And it's a pretty comprehensive physical checkup that is done. The results of that are immediately available to him on the app. And, in cases where they need further treatment or further attention, we work with them to make sure that they get that attention as well.

Uday: Sure, sir. And just another question is, are there any plans to expand coverage to other hospitals as well in the future or it will be restricted to Narayana Health only?

Ravi Vishwanath: So, at the moment the focus is on Narayana across the country. There are situations where the customer can go to any other hospital, for example, if there's an emergency or something like that. I don't think that we will ever have a wide network like, you see with a typical carrier. We will always have a narrow network. It's something that we are always

listening to our customers and trying to understand what their needs are, and we kind of respond accordingly. So, at the moment it is across the country. You know, you can go to any Narayana facility. We may look to expand that over time depending on what our customers tell us, but I expect that it'll always be a narrow network as opposed to a broad network that allows us to provide a much superior experience to our customers, which is the main goal of what we're trying to do is to make sure that we are their partners in helping them stay healthy.

Uday: Okay. And lastly, if you can squeeze in one last, how is the traction that you are seeing? I know it is a very short period after launch, but any color on the traction that you are getting on?

Ravi Vishwanath: Yeah. As you said, it's very early days, right. So, I think, I won't be able to give you any numbers in terms of traction, what I would say is that we are happy to have put in place the processes and systems and all the other things that are required for an insurance company to be able to successfully serve its customers, which is a gargantuan exercise. And all the testing that one does before you go live is fine. What really matters is whether that works in the real world. That's been really our focus. And I'm happy to tell you that the team has done a great job, and all the systems are working really well and the customers that have purchased the product are really happy with it. They are happy with the process. They're happy with the transparency with which we are explaining the benefits and the limitations in the product to our customers. And I think that overall, I would say that they feel comfortable that they are with a trusted brand and should they ever require complex surgery they will be well taken care of, not just financially, but also, surgically by a really high-quality team of doctors at a high-quality facility.

Uday: Sure, sir. Thank you.

Nishant Singh: Thanks, Uday. Adrith, can we maybe have your question, please?

Adrith: Yeah. Hello. Good evening. Am I audible?

Viren Shetty: Yeah, you are audible. Go ahead.

Adrith: Yeah. Thank you for being so open about your volume issues. If I could just push the envelope a little bit here. So, the overall IP volumes are flat and Bangalore is the only cluster that is seeing a drop in your Y-O-Y discharges. So, I believe it's being driven there. And when I view that, in the line of your specialty mix, the cardiac mix has actually come down, over the last year. So, if I compare it to Q1 FY24, so what comes to my mind and please clarify if I'm incorrect, is that the Health city hospital, which is essentially cardiac,

have you seen any kind of drop particularly regarding surgical demand there when it comes to heart surgeries?

Viren Shetty : Never a drop when you're selling heart surgery in India. The numbers that you're talking about, maybe on a quarter -to-quarter basis, would not be growing the way it grew in a post - COVID catch up growth scenario. But we are focused on growing our overall top line and re-engineering the interior bed allocation and focused on certain payer classes. Where it may demonstrate that volume wise, it's not growing the way it did, it's still delivering revenue and margin growth and that's been our focus.

Adrith : Yeah. So, the share of cardiac, could you help me out, why it's going down, like it's down, 2% from 35 % to 33 % on the overall IP?

Virent Shetty : That is a conscious effort. See the cardiac, it's a disproportionate component of our business compared to all the other listed hospital groups, predominantly because we have this very, very large, the largest cardiac hospital in this part of the world, which does only one specialty. Most of the groups, it'll be between 10 -12%. Now, it's not that there's a lack of demand f

or cardiac services, it's just that the ortho, oncology, other businesses have been growing much stronger and more robustly across all the other hospitals. Cardiac still remains our flagship anchor business and will still contribute anywhere from a third to maybe it can go down to a quarter of our business 10 years from now, but it is nearly half the reasons 14 why people end up in a hospital in India. So, it will always remain a very large growth engine for us.

Adrith : Got it. So, you could confirm that you have seen an increase in discharges regarding cardiac in your Bangalore cluster?

Viren Shetty : Over a year -on-year basis. Yes.

Adrith : Yeah, on a year -on-year because obviously this seasonality. So, I'm not looking sequentially. So yeah, okay, thanks for that. Secondly on the payor mix. So, the scheme business is again flat, like at 21%. Is there any kind of guidance or direction as to you would , it's decreasing proportion in the coming quarters or at least in the next few years? Because as it was reiterated before multiple times that, the quality of revenues are important and even though realizations are only up like 6%, I'm just wondering if a lot of the growth levers would have to come from the payor mix because specialty also is not margin accretive at times. So, I guess the improvement in margins will have to come from a scaling down of the scheme business share. So, is there any kind of guidance on that?

Viren Shetty : No. So, two things. We're not scaling down the scheme business. We are taking the calls that make sense for the hospital that's over there. And guidance on the revenue mix, I mean, I'm sure, you know, we don't give forward looking guidance, but Sandhya can comment on the things we're working on.

Sandhya J : So, we are making a judicious choice between balancing cash flows because some of the schemes do not pay on time margins as well as capacity utilization, because if you have spare capacity, you would want to utilize it judiciously. Another aspect on schemes is that there are certain geographies where requires a little bit of scheme support, because there is a certain affordability and a demographic reality of those geographies where we will have to support the geography with certain scheme predominance. So, keeping all those balances in mind, we are taking our scheme choices. So, I don't think we'll be able to guide a number, but I can definitely say that the choices we are making are reflecting in the expansion you've seen in the margins and in our cash flow positions also. So therefore, we will continue to keep playing this balance. But at the same time, we will also be socially focused and relevant. So therefore, we are not aspiring to say that I want to bring it down by 5%, 10%, not like that, because we want to be socially relevant also. So, we will find the right balance as we have done in the past.

Adrith : Okay, fine. Thanks a lot.

15 Nishant Singh : Thanks, Adrith. Yes, Mohit, please ask your question.

Mohit : Hi, sir. Can you hear me?

Viren Shetty : Yeah.

Mohit : Yeah. Thanks for taking my question. So, my question is more on the industry part. So, my question was on the free medical services that government obliges us to provide to EWS section of the society. So firstly, do we come under this? And secondly, what is the total percentage of the beds that needs to be given to the EWS section?

Viren Shetty : Yeah. So that requirement only applies in some variation or the other to three of our hospitals. One is our Dharamshila hospital in East Delhi, where 10% of the beds need to be reserved for EWS category of patient. The other one is SRCC Children's Hospital in Mumbai, where 10% is under the Charity Commission, and Guwahati Hospital where under an arrangement with the government, some proportion of the beds need to be treated under the Assam scheme. So, these are very small by total number of beds and are not that m

uch

of, they don't contribute that much to the overall proportion of the patient services that we offer.

Mohit : Alright, sir, thank you. Thank you very much.

Viren Shetty : Thank you.

Nishant singh : Thanks, Mohit. Nancy, can we have your question, please?

Nancy : Hi, sir. Thank you for taking my question. Could you please throw some light on the IndAS 116's impact on our EBITDA and PAT numbers?

Viren Shetty : Give us a minute.

Sandhya J : Yeah. Our EBITDA will increase by, because of IndAS our EBITDA has increased by INR 148.7 million, and our PAT has decreased by INR 19 million for Q1 FY25 on a like to like basis.

Nancy : Sure. Thank you so much.

Nishant Singh : Thanks, Nancy. Prince, can we have your question, please?

16 Prince : Yes, Sir. So, my first question would be, Sir, regarding the debtor days, do we see any improvement in debtor days as elections have gone and we are also surgically planning to reduce the patient scheme?

Sandhya J : So, in Q1 normally debtor days will be slightly adverse for few reasons. One is that most of the government payers, they have their cash flows released in Q4. So, most of the collection happens in Q4 and Q1 their budgets are not fully approved and, there fore, we don't get the payouts. This year, we also had the added impact of the elections. So, like every Q1, this Q1 also had a little bit of a slowness in terms of collection. Between Q1 of last year to Q1 of this year, we are holding the same levels but between Q4 of last year to Q1 of this year, we have deteriorated by about 7 -8 days, which we will catch up in the next quarter.

Prince: Okay, thank you so much. And regarding the payer days, you have been telling that you have negotiated with the payers and we have seen that the payer days have increased significantly. Can you give some guidance regarding the future that how it would be moving in future? Will it be maintained or will it increase?

Sandhya J: You're referring to days payable/vendor payments, right?

Prince: Yes, Ma'am.

Sandhya J: Okay. I think we will bring it down to levels of 90 days. Right now it is slightly on the higher side. We've gone through a certain ERP migration and because of which there has been a little bit of lag which has got built up there. Between 60 -90 days is what we have negotiated with the payer. So, we will go back to those levels.

Prince: Yes, that will be helpful. And my next question would be regarding the India business. For the India business, revenue growth in the last year has been 10%. And as per your guidance, you are not focusing on aggressive revenue growth but quality revenue growth. So, should we consider in next few years, till the new facilities of Kolkata and Bangalore will be inaugurated, the revenue growth will be around 9% -10% only?

Sandhya J: Can't give a forward looking guidance like that. I think Venkatesh already explained a couple of times the outlook on growth and how we are looking at it. The growth will be organic in the short term. The next big inflection point will come when our capacities come online. Until then, we will grow in line with the market and in line with our own efforts that we are putting in in terms of throughput maximization as well as quality of revenue. So, I think we already answered that.

17 Prince: Yes, Ma'am, thank you. And my last question would be regarding the lease. In FY24 we have seen a slight increase in lease liability, can you please guide is it regarding some clinic facility or something else?

Sandhya J: Yeah. See, to some extent it is because this clinic facility but actually if you see our overall lease costs have come down by almost 0.6%. Now, that is because a lot of the lease contracts which we had taken 5 -8 years back, we have completed those lease obligations and we have unbound those leases. So, our overall lease cost has actually gone down and our overall lease liability has also gone down because

of that. So, I need to see where you are seeing

this increase from but there is definitely some amount of lease obligations coming on board because of the clinic but they are not material in the way you're seeing. Maybe our team can offline reconcile with you.

Prince: Sure, Ma'am. Thank you for the answers. I am done with all my questions.

Nishant Singh : Thank you, Prince. Kunal, may we have your question, please?

Kunal: Hi, thanks for the opportunity. First one on our international business, can you share what is our proportion of revenue coming from Bangladesh?

Sandhya J: So, our Q1 FY25 was about 7.6%, which was our international patient. A large part of it, a substantial part of it comes from Bangladesh which will we'll have to see how the situation pans out.

Kunal: But have you seen any reduction in flows in recent months or for you the businesses as usual, at least in July?

Viren Shetty : No, it's not. It has been disrupted during the election time, during the festival season Ramzan and so on. See, but one thing is, consciously as a company we're moving away from a

reliance on international medical tourism and more towards domestic patients. So, this is the number over five years, we expect to go down to zero. What will come will come. We're not going above and beyond to maintain these numbers.

Kunal: Then would it be fair to say that for you the profitability of international business is not so much different because all other peers are saying that international business profitability is higher, right?

Viren Shetty : Yeah.

18 Kunal: So, to that extent, why would we don't let it go?

Viren Shetty : We don't distinguish between domestic patients, international patients. So, a patient is a patient. The only distinction pricing comes from the room category and the sort of procedures that they take. So, we've never in the recent, I mean, ever since a few years before COVID we have normalized the prices between domestic and international patients.

Kunal: And Cayman, we do not understand the unwinding impact. So, is it that now that you have paid the lease expense you own certain assets and hence your lease expense is going down?

Because a large part of your EBITDA improvement on a year-on-year basis, EBITDA improvement is driven by the rental expense going down.

Sandhya J: Yes, correct. We have paid those. We have completed the lease tenure and completed the payouts. We own these assets now and that is why the unwinding of the lease charges have happened. But we've seen improvement across lines, not just on the lease charge line but across lines we've seen improvement in terms of cost. Lease is also one of the contributing factors.

Kunal: So, which asset this is where we have now completed the lease...?

Viren Shetty : This is lease of equipment and not land.

Sandhya J: This is equipment finance. Maybe I could categorize that.

Kunal: Lease equipment. Okay.

Sandhya J: The biomedical equipment which we purchased.

Viren Shetty : This is not asset leased buildings.

Kunal: Sure, sure. Okay-okay. So, currently INR 195 crores of rental expense in this quarter, will we see similar decline? I think last year same quarter would have been INR 235 crores, right.

So, will there be any meaningful declines in the coming quarters due to this?

Sandhya J: Most of the unwinding is done. Most of it is done, Kunal. There is little bit left but most of it is done. So, maybe slight decrease you will see in the next couple of quarters and then this will normalize.

Viren Shetty : But we will be taking up new equipment on lease. So, it will go back up again as and when the new capacity comes on then.

19 Kunal: Sir, are these related to the robotic surgery, if I may ask?

Viren Shetty : It's all the major equipment. The equipment companies give you the option to either buy it upfront or lease it from them. And based on the cashflows, based on the volume

es, we take

a measured call on trying to match that. So, we take lease cum sale options from a lot of these vendors.

Kunal: Sure, Sir. Thank you very much.

Nishant Singh : Thank you, Kunal. Mohit, can we please have your question?

Mohit: No, my question is answered. I'm sorry.

Nishant Singh : Okay, sure. Kunal, is your answer also...because I think you only asked the last few questions, right?

Kunal: Yes, yes. Yes, it is answered. Thank you.

Nishant Singh : Yes, Prashant. Can we have your question, please?

Prashant: Yeah. So, two questions. Firstly, can you give us the profitability EBITDA number at your Delhi, Gurugram and Mumbai hospitals?

R. Venkatesh: So, when you talk about the revenues from... I'll take all the new hospitals together with Delhi and Mumbai, revenue of these new hospitals have been reported around ₹123 crores with a year-on-year and a quarter-on-quarter growth of more than 7% and with the EBITDA margin of 7%. Dharamshila will and continues to give reasonable returns. Gurugram is also mildly positive for this quarter. Mumbai is in a single digit loss, which we expect to stabilize to EBITDA breakeven in Q2 because the opening in Q2 has been pretty good. So, overall, this is the summary of our new hospitals.

Prashant: Okay, thanks. And the second question is regarding your Integrated Care and Health Insurance businesses. What is the outlay in terms of cost that we can see on these two businesses put together, say over the next couple of years? And at what point would they break even? Your assessment?

Sandhya J: So, for now we have invested about ₹100 crores as a capital commitment into the Insurance business and as and when the business grows we will make further commitments and investments into the business depending on the scale and the growth trajectory. As far as the Integrated Care business is concerned, there is a cash burn which is given as part of the analyst deck already and there will be a cash burn because the Integrated Care business is a

long term venture and we have to build out this business over a period of time. So, there will be cash burn in this. However, we've kind of kept our cashbook to be as judicious as possible.

May we request everyone else to go on mute because there is a lot of background noise for us here. Okay, thank you.

So, we are being very judicious about the cash burn and we're also keeping ourselves very tightly accountable to breaking even at a singular clinic level at a reasonable timeframe. So, there we are tracking, okay, but because we are opening new clinics, therefore, there is a little bit of a cash burn which is happening. We will continue to build on that. We are not guiding a full number for Integrated Care and Insurance because we are responding to what our customers are asking and the way the demand patterns are coming through. Therefore, we are not putting an outlay out there but we will do whatever it takes to make these businesses successful.

Prashant: Alright, thank you. That's it from me.

Nishant Singh : Thanks, Prashant. Vinay, can we have your question, please?

Vinay: Hello. Just wanted to check out, on your Slide 7 for two years given the ARPOB numbers, is that the right ARPOB number for Q1?

Nishant Singh : Yeah, that's the number for India.

Vinay: So, that is an annualized number or a quarterly number?

Nishant Singh : That's the annualized number.

Vinay: Okay. So, Q1 here it is 15 Mn and last year it was 13.6 Mn. The second part is, when I look at your performance of the hospitals in Western region, they seem to have shown a jump in the overall revenues. Now, you're saying still Mumbai is a loss making proposition.

Sandhya J: Yeah.

Vinay: Okay. So, today we have only two hospitals which are loss making, is it, out of the total?

21 Sandhya J: Actually, if you look at Q1, Gurugram also was not loss making. So, at the moment only Mumbai is loss making. E

ven Mumbai Q4 broke even. Q1 because of the little bit of the seasonality impact, we kind of slide it back into a little bit of loss. Other than that, all our other hospitals have been profitable in Q1.

Vinay: Okay. And lastly, just 7.9% of patients you said who came from Bangladesh, these where in Kolkata region only, right? Or Kolkata and this East?

Sandhya J: 7.6% of our revenue comes from international patients largely from Bangladesh. Most of them come into our Health City campus as well as our Kolkata campus.

Vinay: Okay. So, this is 7.6 of the overall region?

Sandhya J: Yes.

Vinay: Okay, thank you very much.

Nishant Singh : Thanks, Vinay. We have got a few questions from the chat. We'll just like to read out a few questions. Yeah, sorry. Parikshit, you can have your question, please, and then we can move to the chat.

Parikshit: Okay, fine. Okay, great. Thank you. Thanks for the follow up again. And my previous question when I was asking about the volumes, we discussed about how we are changing our payor profile and that is also going to impact our volumes but in return, you know, benefit our margins and I just quickly went back to pull the data. I think our EBITDA margin, and I'm talking about the India business, started moving to about 19% all the way from FY23 itself, right. Q2 of FY23 we hit 19%. And the payor profile also, I'm just looking at the percentage of customers that are from the schemes, in the scheme section of it, that has been hovering around 20% -22% throughout this period. So, are we still changing it or has the transition happened and it's done already?

Viren Shetty : Not done. It's a very dynamic process and it's not just who you're getting in. It is what we are doing for them. And, so, a lot of the work that gets moved towards daycare, a lot of the emergency services, a lot of things that don't count towards occupancy and the pay up and these are cash and insurance patients will contribute towards performance. There are other efficiencies we're getting from throughput and being able to manage a lot more tasks with the same manpower which do contribute towards performance.

22 So, the payor mix, sometimes the hospital we have are locked into a certain payor mix. So, Ahmedabad, for example, we run a hospital that is in a poorer part of town and that is a State that has a very high contribution from the government for doing the surgery. So, that will always have a disproportionately high proportion of patients coming from scheme. Whereas our hospital in Gurugram, for example, it's in Cybercity Phase -II. It's built just as a private, semi-private sort of hospital. So, it barely has any contribution coming from the government program. So, it's not something we are consciously looking at. What we're trying to do is improving the cashflows of each hospital looking at the patient base that makes sense for that place.

Parikshit: Okay, alright. Thank you.

Nishant Singh : Thanks, Parikshit. Now we can take up the questions from the chatroom. The first question

is that, Is ROCE significantly different in case of capital intensive SBUs like Oncology and Cardiac treatments compared to less capital intensive areas like Neurology?

Sandhya J: See, actually ROCE is not just about the biomedical equipment. The cost of a bed when we say is say between ₹1.5 - ₹2 crores, it is the cost of the infrastructure, cost of the bed, cost of the biomedical equipment, cost of the diagnostics that go into it. So, it's a large investment that goes into getting a hospital or a bed up and commissioned. And ROCE is a factor of that. So, if you just look at the biomedical equipment of a particular specialty versus another, that is not a big player in the calculation of the ROCE.

Nishant Singh : Okay. There is another question, are you looking for a finance partner in the Health Insurance business or do you plan to invest your own money into this Insurance ven

ture?

What are the reasons why you are choosing the option that you have chosen?

Viren Shetty : So, at this point we're not looking for a partner for Health Insurance. We're still building this up and since we're doing it in a very moderated way, phased out across our different hospitals it's something we believe at least for the near future until it proves itself like we're able to build an insurance plan that's sustainable, that provides a lot of value for patients and improve their outcomes, we won't want to look at scaling it nationwide.

Why we are choosing this option is because the market has enough providers who will just sell you a policy and not ask any questions and that does lead to a lot of dissatisfaction with premium holders. So, we want to provide something that is a policy that has no questions asked. Once you're in, we shouldn't have to doubt the intentions of the patient coming in for any sort of procedure. And we take a more proactive role in managing it. That's why it's positioned very differently and a lot of financial investors may not be fully aligned with that way of running the business.

Nishant Singh : There is another question on Insurance which Ravi can take. How long would the Insurance business take to breakeven on this INR 100 crores initial investment as per your business model to get positive ROI?

Ravi Vishwanath: Right. I mean, I think it's might be a bit premature to comment on that because as Viren just said, you know, we are kind of positioning ourselves very differently. And the important thing to keep in mind is that we are starting in a very modest way in Mysore and four districts around that and we will expand gradually and kind of see how the market reacts in response. And I'll just repeat what Sandhya has said earlier, which is that we're doing this very prudently and looking to make sure that we extend our capital as much as we can. So, a little bit premature to say that. But, hopefully, we'll be able to share more information with you in coming quarters.

Viren Shetty : The question is from Riddhi about how do you assess the performance of your administrators? We give them targets and they operate in an operating budget every year and there are multiple parameters for assessing them, not just on hitting the financial parameters but also on keeping the cost within line, improving the digitization, reducing the length of stay and improving the throughput of the hospital. So, at the senior levels, they have a 80 -20 fixed to variable pay structure which incentivizes them towards that.

Question on what type of business intelligence, AI tools are used for measuring performance? Those are 2 different things. The business intelligence, we were on Microsoft Power Bi. We moved to an in-house application developed by our analytics team called Medha. The name of the, at least what they have called their BI tools so far is AERO. This is something that we use extensively for our internal performance, benchmarking measurement, not just for business operations but also for clinical operations and supply chain and so on. And it is a tool that is deployed across few other hospitals as well. People pay to use these services.

For the healthcare professionals, other than using the analytics, they also use our in-house hospital information system. That's called AT HMA. And AT HMA has multiple applications on it which include an app for nurses called NAMAHA, an app for doctors called AADI. It has its own analytics program called ATMI TRICKS. It also is deployed across all our hospitals and clinics as well as this is being sold to other hospitals and clinics outside of our network.

Other questions – the ARPOB for Gurgaon region.....

Sandhya J: Yes. So we aren't specifically sharing out particular hospital but in the North region, the ARPOB is about 16.6 Mn for Q1 FY25. Actually North, when I take north, it's slightly higher for the Delhi a

nd Gurgaon units and slightly lower for the Jaipur units but in that range.

Viren Shetty : The last one is occupancy percentage in India and Cayman.

Sandhya J: For India, it's not a number that we are sharing, like as a separate number because we have always said that occupancy, the way it is measured, midnight occupancy is not a representative of evolution of our business. Having said that, we always operate in the range of between 60 -65pc on midnight occupancy basis. Daytime occupancy and day care occupancy will be much better, so in Q1 and Q3 quarters, we kind of end up in the lower

end of the range and in Q2 and Q4 quarters, we normally hit the upper end of the range and that's how we have come in this quarter also. In some of our mature hospitals, we will be at a much higher number than that and then in some of our smaller hospitals, the number will be lower.

Viren Shetty : There is a question earlier on from someone who is new to the conference call. When you say inflection point, when you see better growth, what will that be? This we have mentioned earlier and we will mention it again, this is when all the new hospitals come online, so that will take about 3 years from now.

De-bottlenecking in adding infrastructure, so debottlenecking, that refers to the fact that a lot of our patients wait, they spend a lot of time waiting in our hospitals, waiting for the doctor, waiting for the tests, waiting to get a bed, waiting for certain services and so a lot of that is addressed through digitization and process improvement initiatives and what it does is, it increases the through puts. We can see more patients per given day as well as reduce their length of stay. So we are trying to achieve best in class, length of stay of 3.5. We have some way to go and lot of it we are doing helps improve the flow.

What do you consider as quality revenues, is a question? So, quality comes in various kinds but essentially you can simplify it as payers that pay on time and you are able to cover most of the cost incurred in providing the services. Now in a perfect world, you could run a hospital just with patients paying cash up front and paying it at a 30 -40pc margin to 25 whatever services you are providing. But that doesn't happen in the real world. Running in a country like India, we have to cater to multiple payer classes. And so within those payer classes, we try to rationalize certain payers who may have a very poor track record of paying us on time or have a huge track record of disallowing certain things and we would try to mix and match, the patient to the services that make more sense for that.

Nishant Singh : The question on the ARPOB number for north. We mentioned 16.6 Mn, that is the 1.66 crores for the year, that is the annual ARPOB. There is a question that it is much lesser than the north where it is 50,000. So if you make it a daily ARPOB, that number comes to around 45000 that we have. So it's not that much lower as compared to what others are making in the north.

Viren Shetty : But having said that, we do peg our services to be at a slight discount to what is available everywhere else and we try not to be at the top of market in terms of our pricing for patients on a like to like basis.

Nishant Singh : Our next question is, how many total new hospitals are planned for the next 3 yrs.?

Sandhya J: It is still work in progress. As and when we keep identifying opportunities, I think we will keep sharing. We have indicated a certain capex for the current year and we are expecting that we will spend a similar amount as capex over the next 2 -3 years and therefore, there will be significant investment that will happen. We will share more details as and when we will make those choices.

Viren Shetty : As and when they come up, we announce like we announced in Kolkata, we have announced in Bangalore and certain capacities being added in Raipur. So those are

what....so far we are giving clarity on.

Sandhya J: There is a suggestion on bonus shares issue. So we thank you for that suggestion. We have tried to stay as consistent as possible in terms of our return to shareholders in dividends etc. but we will take your suggestion on Board and we will evaluate them on its own merit. Thank you for that. There are a lot of questions on chat, but they have been answered through questions which have already been asked. So, if we have missed to answer any question, we request you to please put your hands up and we will be happy to answer.

Nishant Singh : I think Gagan has questions. Gagan, can you please raise your question.

26 Gagan: So, the 1st question is on the tax rate. I think the indication at the close of last financial year was that the tax rate will go up. It will be around 13pc, year on year, it is slightly up but still low. So, any explanations there.

Sandhya J: We had some one -timers in the last year because there was a deferred tax credit we got because we shifted from the old regime to the new regime. That came last year. So that benefit is not there but otherwise, what tax rate you are seeing right now is the steady state tax rate, that we are estimating.

Gagan: So if I got it correctly, the 1st quarter tax rate is at 13pc. is that what you are saying can be extrapolated for the whole year?

Sandhya J: Yes, 13pc is correct. If you see, last year in the 1st quarter was 10.6pc. 2nd and 3rd quarters were 8.7 and 9.8. For the full year, last year we were at 11. 13pc. That was the effect of the deferred tax credit that came in in last year. So that is not repeating. Therefore we have kind of come on to that 13pc kind of range. One thing you will have to keep in mind is that it will vary on the mix effect because the India revenue is at a higher tax rate and Cayman profit is at a no tax rate and therefore this is the average. So if the mix changes between India's profitability and Cayman's profitability, then accordingly the tax rate will also get moderated.

Gagan: Ya, ok. the 2nd one pertains to your existing facility in Caymans. If I understand it correctly, the occupancy there is still fairly low. Right? It's a 100 bed facility and if I remember it correctly and I think last 2 indicated the occupancy tends to be 50 to 55 odd something out there. And you indicated in your comments in the call that there is not much headroom for growth there. So is the constraint there more demand related constraint or location related constraint or am I understanding the capacity itself incorrectly?

Dr. Anesh Shetty : No Gagan, you are right on the capacity. The constraint is that the number of specialty service lines we can offer, in that facility we reached the limit. The new service lines we are going to start in the new hospital. We cannot offer it in the existing hospitals. The 2nd constraint is obviously the location. As we have discussed before, the existing hospital is located very far away from centre of the city where most patients live and work, most people live and work. So by moving to a very very prime location, we become a lot more accessible to patients who otherwise would not consider driving so far for smaller services or entry services. But the primary constraint is just the services we can offer, the new specialties that we are going to offer cannot be offered in the existing one.

27 Gagan: How should I put it? A very layman like but my question is, if best case utilization at this facility cannot exceed what it is, does it make sense or is it even a possibility

Dr. Anesh Shetty : Gagan, sorry, could you repeat that please? I am not able to hear you.

Gagan: My question to you is that, if you don't see occupancies in your existing hospital in Cayman improving, is it then a possibility to maybe reduce the bed count and perhaps utilize the additional space that you get in some other way in that hospital?

Dr. Anesh Shetty : Ok, understood. No, it won't work like that because when the new hospital comes, the existing facility will also see an increase in occupancy. They are not 2 distinct hospitals, they are just 2 campuses, think of them as just 2 locations for the same unit. It's the many of the doctors overlap but there are some new specialists also but when we have the new hospital, we absolutely expect the occupancy of the existing hospital to increase meaningfully. So it just wouldn't make sense to reduce the number of beds in the existing hospital. Having said that, it's more an issue of, it's the opportunity cost, unless we start the new services, there is nothing that's going to happen just by removing beds. We will have to add either equipment or re-equip the facility etc. But none of that is needed because once the new hospital is there, once we start the new services, we will see an increase in occupancy in the existing hospital. The new hospital will be primarily for patients who are short stay. The existing hospital will be for patients who have a long or more complicated stay.

Gagan: So you are saying that the new hospital will act like a catchment to the

Dr. Anesh Shetty : Yes absolutely. It will also render services of a particular nature in that facility but it will also significantly feed into the existing hospital.

Gagan: Right! Final one, again on the facility one. While one understands that your 1st venture when came in took a reasonable amount of time to mature. Today, you have credentials, you are not an upstart there and therefore, is it reasonable to then assume that breaking even in the new facility will take a lot of time frame compared to the 1st one.

Dr. Anesh Shetty : Yes, you are absolutely right. That is the expectation we have.

Gagan: Is it possible for you to perhaps give us some understanding of, at least from an intent standpoint or an anticipation standpoint, how much duration the break-even period or the pay back period potentially be on the new one?

28 Dr. Anesh Shetty : It's not something that we disclose or like to but having said that, I think you just have to wait for the next quarter and then we will have a much idea of where things stand. Even I am far away from knowing the actual answer.

Gagan: Thank you and finally on your comments, you intend to keep the capex at 1500 -1600 crore run rate for the next 3 years. One, the debt hasn't really moved too much in the 1st quarter. How should we think of leveraging towards the end or closure of this year ? Does it continuously keep increasing from hereon over the next 3 years and to what levels?

Sandhya J: Yes, that is the right assumption to make Gagan. The reason the debt hasn't moved is because the capex hasn't moved. A lot of our capex commitments is in Greenfield which means that we have to identify the land and then go through the plan approval process which is a long process and then start to build and therefore the consumption of that capex is little slow and that is also reflecting in the debt that we are picking up though. We will for whatever capex we have indicated, significant part of that will be through debt only, so you can assume that at least 80pc of the capex we are indicating will come through debt and to that extent, the leverage will be, it will increase.

Gagan: Have you considered equity raise as an option or that's completely off the books and you intend to completely use debt in internal accruals?

Viren Shetty : It's not something that we need to look at right now, given how under levered we are and how much cash we still have on the books. Nature of this sort of capex is that it operates at a low intensity at our choosing and it goes in a very staggered manner over many years. So if we are able to match our internal cash flows plus the traditional ones, from banks and

from NCDs to match the capex and the green field and land constr

uction and all as and when

it comes. Should we get to a point where we have the ability to service our ambition is no longer matched through our cash flow and borrowings, then we would look at an equity route. But at this point, it's not really required.

Gagan: Of the total cash flows that you accrue annually, is the ratio of cash flow accrual from India and Cayman be very much in line with the EBITDA that you generate in India and Cayman? Or is that also sort of different from that?

Sandhya J: Cayman has just gone through an intensive capex cycle and therefore there is some amount of cash flow that's got deployed in that capex and obviously, there is a lot of borrowing that has been taken on board, so the repayment of that will kick -in in Cayman. That's from a Cayman point of view. As far as India is concerned, the cash flow generation is in line with the EBITDA but like how we choose to invest in terms of capex etc. accordingly the cash flows will get moderated.

Gagan: What will be the repayment schedule at your Cayman, I mean over what time frame do you pay off the loans?

Nishant Singh : In Cayman, we have a mix of the new loans and the old loans. The new loans are almost at the expiry stage than the old loans. The new loans will expire in the next 3 -5yrs. The average maturity is around 3 .5yrs., the large maturity is around 5.5yrs.

Gagan: Thanks, I will get back in the queue. Thank you.

Viren Shetty : Riddhi , there is a question from your side.

Riddhi : Hi! Can you repeat on the capex expansion for Bengaluru and Kolkata?

Viren Shetty : In Bangalore, we are adding an annex to our cardiac building. It's a piece of land we bought next door. This will focus mostly on Aneurysm surgery and very advanced valve repair surgery to expand the capabilities of our flagship cardiac hospital. We also bought a 1.5 acre piece of land in HSR layout, this is a suburb of Bangalore where we have an existing hospital whose capacity is quite filled up, so this will operate as an extension of that hospital as well. This will be a green field hospital that will accommodate about 200 beds or so. this is for Bangalore. Anything else you wanted to know about?

Riddhi : Kolkata as well.

Viren Shetty : For Kolkata , we have got land in Rajarhat. This is a new suburb of the city and there we bought a very large piece of land. We will make a very large hospital over there in phases. Indicatively, this could go up to about 600 beds but we will do it in 2 phases and plan the action accordingly.

Riddhi : Ok, and the capex amount is 1000cr. Right? For Kolkata?

Viren Shetty : Yes. That is the amount we expect it should cost us, yes.

Nishant Singh : The total cost of the project for the next 10yrs, is going to be more but for the 1st phase, it will cost around 900cr,

Riddhi : Ok, thank you.

30 Nishant Singh : There is a last question from the chart – the Bangalore and Kolkata hospitals will get fully operational in the next 3yrs. That's the question. So, we will start the construction by the end of this year in Kolkata and Bangalore will be another couple of months after that. So from the start to end, it will be around 3.5 -4yrs.

Nishant Singh : As there are no more questions, we would like to end our session. We thank you all for the active participation as usual. Thank you.

END OF TRANSCRIPT

Call: Nov 2024

Date of submission: November 11, 2024

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code: 539551 (EQ), 975516 To,
The Secretary
Listing Department
National Stock Exchange of India
Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051
Scrip Code- NH

Dear Sir / Madam,

Sub: Transcript of Earnings Call for the quarter and half year ended September 30, 2024.

Further to our earlier letter dated Tuesday, November 05, 2024 in relation to uploading the Audio Recording of the Earnings Call of the Company held on Tuesday, November 05, 2024 for the quarter and half year ended September 30, 2024, please find attached the transcript of the said Earnings Call.

We wish to inform you that the Earnings Call transcript is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earnings-call-audio-and-transcripts>

This is for your information and records.

Thanking you

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q2 FY25 Earnings Conference Call"

November 5, 2024

MANAGEMENT : MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER & MANAGING DIRECTOR

MS. SANDHYA J – GROUP CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – GROUP CHIEF OPERATING OFFICER

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS SUBSIDIARY HCCI

MR. RAVI VISHWANATH – CHIEF EXECUTIVE OFFICER , NHIC

MR. NISHANT SINGH – VICE PRESIDENT - FINANCE , MERGERS & ACQUISITIONS & INVESTOR RELATIONS

MR. VIVEK AGARWAL - SENIOR MANAGER , INVESTOR RELATIONS

2 Nishant Singh: Hello everyone. My name is Nishant Singh. I head the IR function at Narayana Hrudayalaya. I welcome you all to the Q2 FY25 Earnings Call for the company . To discuss our performance and address all your queries today, we also have with us Mr. Viren Shetty - Vice Chairman , Dr. Emmanuel Rupert - CEO and MD, Mrs. Sandhya Jayaraman - Group CFO, Mr. Venkatesh - Group C OO, Dr. Anesh Shetty - MD of our Cayman Business, Mr. Ravi Vishwanath - CEO of NHIC , and Vivek Agarwal - Senior Manager in the IR Function.

As usual, before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the of the same shall be made available on a website as well as on the Stock Exchange later . I would also like to remind you that everything that is being said on this call, that reflects any outlook for the future, or which can be construed as a forward -looking statement and must be viewed in conjunction with the uncertainties and the risks that they face. With that, now we would like to start the Q&A Session straight away. I request everyone to now use the 'Raise Hand' feature to start posing their questions. Yes, Damyanti, please go ahead.

Damyanti : Yeah, hi. Good afternoon, everyone. My first question is if you can talk about how the new unit in Cayman is ramping up? Whether it was a soft launch or it is a full-scale launch for you? And how do you see cost normalizing for this facility?

Dr. Anesh Shetty : Hello. Hi, Damyanti. Thank you for your question. This is Anesh here. So, the outpatient facility of the hospital has been commissioned about a week ago. So, the quarter which results you are seeing, that is Q2, the facility was not commissioned. We've started outpatient services that is doctor consultations and limited diagnostics now. We hope to get approval to start the entire hospital in about next four weeks or so. And that's when we'll start.

Having said that, to the cost section of your question, in the last month of Q2, we have incurred approximately about 65% -70% of the total cost for that new hospital. And in October '24, which is the month just gone by, that's closer to about 75% -80%. So, we are currently incurring most of the costs and the revenue ramp up has gradually started and we'll see that more aggressively once the entire hospital is commissioned.

Damyanti : Okay. So, in the next 4 weeks, you mentioned, you will be seeing full scale offering of services and then the footfalls, etc. should be much better compared...

3 Dr. Anesh Shetty : Yeah, in the next four weeks we expect the whole hospital to be commissioned and ready. It's a new building, so it will take a couple of weeks for people to start flowing in. But, yes, you are right. We expect to be all good to go in approximately 4 weeks is our hope.

Damyanti : Okay. And in terms of cost, as you mentioned, already 75% -80% of cost are already in the books, so going ahead we assume most of the cost will be in line with like what kind of pick up you see on the business side .

Dr. Anesh Shetty : You are right. you are right. Yeah, that's correct.

Damyanti : Okay. My second question is on your Capex projection for FY 25. So, I understand you have earmarked around 9 billion of Capex towards greenfield and inorganic growth opportunities.

Nishant Singh: Damyanti, sorry, we are not able to listen to you. We can't hear you.

Viren Shetty: Damyanti, can you hear us?

Dr. Anesh Shetty : Maybe we can move on to the next.

Viren Shetty: We'll move on to the next one, Damyanti can join back in the queue.

Nishant Singh : Yeah, Prithvi, please go ahead.

Viren Shetty: Prithvi , can you go ahead?

Prithvi : Yeah. The first question is again on Cayman. Given that the hospital is now almost ready to commission, do you have some kind of guidance , over next few quarters, how do you expect the ramp up to happen? And how do we see mar

gin shaping up?

Dr. Anesh Shetty : Yeah. Hi, Prithvi. So, if you recall, in the last quarter's call what we had said is that once the hospital is fully started, we expect margin dilution to last for approximately 6 -8 weeks.

Having said that, now that we are very close to the hospital being fully commissioned and we've commissioned the outpatient services, we're happy to see that our cost control is more on the positive trajectory. So, we'd like to change our estimates from the 6 -8 quarters and change it to about 4 -5 quarters is when we expect some margin dilution. So, we're more optimistic on the cost front. Now that we have commissioned the outpatient services, we are seeing a lot of interest. Everybody is very, very impressed with the hospital and the reviews have been fantastic. But it's just the outpatient, which is a limited service. We hope 4 to continue to see this positive response in about 4 weeks or so when we commission the whole hospital.

Prithvi : Sir, when you say margin dilution for 4 -5 quarters, you mean the margins will be less than 40% for four quarters. Is that the point?

Dr. Anesh Shetty : No. No -no, so if you recollect, just prior to commissioning the hospital when we did not have any of the costs on the books, our margin was higher than the number you mentioned. And at that time, we had said that once the hospital is commissioned and once all costs are in line we expect to take 6 -8 quarters for the margin to come back to a steady base. We are now more comfortable changing that 6 -8 quarters to about 4 -5 quarters.

So, the worst will be in the first quarter and then it will hopefully taper away as we move closer to the end of the year.

Prithvi : Fine. That's clear, Anesh. Second on the India business, can you give me the breakup of the new hospital's revenue and margins in this quarter?

R. Venkatesh : Yeah, the new hospital cluster has done pretty well in this quarter. It's given a very strong revenue growth around 13% year -on-year , with the revenue crossing around ₹130 crore s for this quarter , with an EBITDA of around 11%. The Mumbai has also shown positive EBITDA of around 1.5% and Gurugram at a high single digit EBIT DA and expected to grow more by the Q 4. So, overall , the new hospital cluster has done well in this quarter .

Prithvi : Sir, what explains this structure in the new hospitals? Is it more of any seasonal factor or you're saying it's structurally going higher?

Viren Shetty: This would have happened, it's just that it took a long time for it to occur. We are on track to achieving normalized performance for all the hospitals. The Mumbai hospital being a children's hospital , that will take a much longer given its very specific focus. But the Gurgaon and Dharamshila Hospitals will soon go on the normal trajectory.

Prithvi : Final question on the Capex , did you finalize on the number of beds that will be added in Bangalore and Kolkata?

Viren Shetty : Yeah, we are still working with the architects .

R. Venkatesh : So, I mean, we know that the majority of our investments are now planned around Kolkata and Bangalore and some other geographies where we have our strong presence. We already 5 have committed projects which include greenfield in Kolkata and Bangalore totaling to around 700 beds and the brownfield in Raipur with around 150 beds. This will come up in the next 3 -3.5 years' time. We are also working to close 1-2 more greenfield projects. But all in all, it would be adding around close to 1500 beds in the next four years from now provided there are no material delays on the projects. And, obviously, the large portion of it will come in the third and the fourth year.

Prithvi : Thanks. That's all from my side.

Nishant Singh : Thanks, Prithvi. Nitant, can we have your question, please?

Nitant : Congratulations to the management on a good set of numbers. My first question would be regarding the Rajarhat

facility in Kolkata. It was supposed to get approvals by October 2025

and the constructions were supposed to commence in Q3 FY25. So, for any updates on it?

R. Venkatesh : So, the applications have all been done. The process approvals are in progress. We expect all the approvals to be sought by the next 3 -4 months ' time before we start the groundbreaking there.

Nitant : And the second question is regarding the Cayman facility, it's going to be commissioning 50 beds, I suppose, right? The new facility which is the outpatient daycare facility?

Dr. Anesh Shetty : Yeah, that is correct. But, again, given the way the facility is configured and the focus on daycare and emergency and services that would not constitute a bed as the unit of revenue, just be cautious about extrapolating a bed driven revenue number with the existing facility.

Nitant : All right. And one last question is regarding Cayman as well. So, the number of outpatients have been declined on year -on-year basis and as well as ARPP has been declined 6.6% quarter -on-quarter, as well as discharges and OP volumes are also a bit down on a YoY basis.

So, is there any particular reason of seasonality , can you share some light on it?

Dr. Anesh Shetty : No, not particularly but on an overall basis we weren't too happy with the quarter. There were few factors that led to the slowdown, you know, all across its uniform. It's not any particular specialty, a particular category. You know, everybody, the whole team has been focused for the past few weeks on commissioning the new hospital. It's a lot of time to fulfill

all the compliance related obligations. We had a couple of hurricanes that led to about 7 -10 days of unproductive work. Just a general slow quarter but we're happy and comfortable that in the month of October things are back to a very positive trajectory and we think that that's behind us now.

6 Nitant : Alright, that answers all my questions. Thank you for it.

Nishant Singh: Thanks, Nitant. Abhishek, can we have your question, please?

Abhishek: Hi, am I audible?

Nishant Singh: Yeah.

Abhishek : Yeah, thank you for taking my question. So, my question is regarding your liquidity levels.

Right now if we look at your Balance Sheet, you have a lot of liquidity tied up in Cayman.

That is after considering the outlay that has happened for expansion in Cayman. So, any plan to put that liquidity to work because I understand getting that money to India there might be taxes which we'll have to pay? So, right now we are using it to pay dividends but how do we get that money to work because it's a large sum of money? And I'm assuming that you would not want to do further of Capex in Cayman, right?

Dr. Anesh Shetty : Yeah. So, Abhishek, you raised two good points. The first is you are right that the majority of our capital outlay in Cayman is mostly complete. I mean, we have a last few, little bit of expenditure for the new hospital post commissioning. But with this, with the new Camana Bay hospital we're done with any significant capital outlay in that market. So, that is correct. The liquidity, I would just, you know, although there is a cash balance, it is also significantly netted off against our outstanding debt, which we essentially borrowed to finance the new hospital. So, just looking at only the cash reserves in that market, it should be offset against the outstanding debt there to see on a net cash basis how much we have. There is some surplus even on a net cash basis and we are obviously using that whenever we have a suitable opportunity. As we have previously said, we continue to be on the lookout for opportunities in the region and elsewhere as well. And all options are on the table, whether it's to repatriate it to India, if there's a need and a suitable opportunity or in some other market. We continue to consider all of them from time to time.

Abhishek : Understood, thank you so

much. My second question is regarding your growth. So, now I understand that it's a long gestation period projects for the hospital industry where you know bulk of your Capex is frontloaded, and it takes three years to make the hospital and another year and a half to breakeven. So, why is it that we are not looking at acquisition as an opportunity because you already get a built-up hospital and potentially broke even? So, we save a lot of time know, so why this, I mean, decision to go greenfield?

7 Viren Shetty : Abhishek, it's not that we aren't looking. Nishant leads a fairly busy M&A practice looking at all the acquisitions. It's just that, you understand as well as we do, on the outside valuations of some of the private equity backed assets that are out there. So, Group hospitals do not make a lot of sense for us. One is that it's fully, fully priced in and there isn't too much delta available for us to be able to make a lot of sense of the acquisition.

The last thing that we would look at then would be single hospitals, individual hospitals.

Those are not as easy to find. The ones that are for sale are very badly built or they're not in strategic areas of focus for us or they may have a lot of issues with the promoter or they'll be really, really old and without assets. So, we've spent since '2016 trying to pursue multiple opportunities for Asset Light acquisitions, all of that. It's just that out of absolute compulsion in the absence of finding anything else to do, we have gone for the greenfield. But still we are pursuing few acquisitions, things that are still in discussion both here as well as overseas. We are looking at certain O&Ms and Asset Light opportunities. It's just that these are the ones that we were able to finalize in a very long timeframe absent anything else happened. We haven't turned our back on the availability of the opportunities out there and constantly evaluating. But when these things come, we do take it up. For example, Spars had one hospital that we acquired. Similarly, in Cayman there was an ENT practice that we acquired. So, whenever these things come up, we do.

But you're right. It does seem like all the things we're presenting to you are greenfield and these have a very long gestation period and what are we going to do for growth in the meantime. There remains sufficient headroom in the existing business to deliver on moderate levels of growth and these Greenfields will deliver a much larger footprint for us three years from now. In the meantime, we'll look for things. Those haven't materialized in the price band that we wanted.

Abhishek : Thank you for such a great explanation, that was very helpful. One last question, anything we are looking at Tier -II, Tier -III cities? Because I'm assuming considering...

Viren Shetty : Yes-yes. But with all the same problems, which is, asset price almost the same as in a Tier -I city with a much lower earning profile. So, there are a few distress acquisitions we found in our co-geographies, which is in West Bengal and Karnataka. We'll be pursuing those. But so far the ones that we've been able to finalize and announced, happen to be the ones that we had to buy the land and get going. But there are opportunities, and we are in active pursuit

of all of them.

Abhishek : Okay, thank you so much. I'll get back to the queue.

8 Viren Shetty : Thanks, Abhishek.

Nishant Singh : Thanks, Abhishek . Yeah, Riddhi, can you please pose your question?

Riddhi : Yes. So, my first question was despite the growing demand for healthcare services in India and the relatively low debt to equity ratios of established hospitals like Narayana, why are there not more new hospitals being established across the country? And I'm talking from an industry point of view and not just related to Narayana.

Viren Shetty : That would be a combination of things. If you're just asking for broad level industry reasons

why aren't more hospitals coming up, for the same reason that Abhishek had brought up. It's very expensive, it takes a long time, the Return on Capital oftentimes is less than the cost of capital for a lot of these new hospitals coming, the huge reliance on payers aren't very dependable on paying on time, very poor patients in most parts of the country where there is greatest demand. So, greatest demand exists in the very small towns but over there , there are almost no organized pay pocket or the government is a payer there, lack of doctors, extreme lack of super specialists, them wanting to work in large Metro towns and not so much in the small town and so there's this oversupply in large towns and very undersupply in the smaller towns.

Dr. Anesh Shetty : The organized players have all announced significant bed additions over the next three years.

Viren Shetty : So, all of us are, I would say, expanding to the best of our capabilities, keeping our debt to equity ratios in mind, making sure we don't overstretch and overleverage ourselves. And all the players are in a pretty, I would say, by historic standards aggressive Capex expansion. But, yes, we are nowhere close to say any of the quick commerce companies or the cement companies or road and railway companies in terms of how much Capex they are doing and how much projects they have announced. Our business is not one that has any barriers and any sort of inbuilt protections. It is highly competitive, highly consumer oriented and it's one in which it's very, very difficult. We have the same upfront investments and long payback periods as infrastructure with none of the benefits that infrastructure projects get, which is extremely long -term lending as well as a guarantee that when you put up a highway no one builds a highway right next to you.

Riddhi : Yeah. So, I also wanted clarity on the Capex that you are currently coming up with, with respect to Kolkata as well as with respect to Bangalore.

9 Sandhya J : We have spent about ₹530 crores in H1 of which ₹320 crores is in India and balance is in Cayman. Out of this ₹320 crores, ₹160 crores was for the land we bought in Bangalore and the rest was regular Capex , which we spent for biomedical et cetera and some amount of small brownfield Capex we have incurred in Kolkata. So, this is what we have spent so far . Once we get the approvals for a building then the Bangalore and the Kolkata build will start. And, therefore, while we have put in some money there in terms of how much we will spend , some of it may get pushed out by a quarter or so. In addition, we're also looking at a few more greenfield land opportunities. So, depending on how we time the deeds and conversion as well, some of those costs will also get incurred.

Riddhi : Okay. So, earlier in one of the concalls, Narayana had mentioned that 1000 odd beds were going to be built in Kolkata and then in last quarter you all said that the it will be around 600 beds in Rajarhat. So, I'm just a little confused whether it's 1000 or 600 odd beds which are going to come up in Kolkata ?

Sandhya J : That is correct. The intent is to add 1000 beds in Kolkata but it will go live in two phases. So, the first phase which we are building, is what we have factored right now. Then once those get completely occupied, then we'll go for the second phase. The 1000 beds is coming from our longer term demand view as well as the ability of the infrastructure to be able to accommodate that capacity. So , from that perspective, that outlook has been given.

Riddhi : So, in first phase it will be 350 odd beds and then later on in the remaining which you plan to add, right.

Sandhya J : Yes. Yes. Yes.

Riddhi : Okay. And, secondly, you already have a land as an annex to the hospital in Bangalore, right.

Viren Shetty : Yes.

Sandhya J : Yes.

Riddhi : Correct? So, there also in last concall it was said that the Capex that you

plan to do is around

₹500 crores. So, that is only with respect to now constructing the building, right? There is no land cause that will be there?

Sandhya J : I think the ₹500 crores estimate was given for the greenfield opportunity that we are doing in Bangalore. And there is a land available next to our current hospital and we will be building in that. But we will be prioritizing on the cashflows and the opportunities that we

10 are attempting from time to time. So, the confirmed builds which have commenced are the ones that Venkatesh called out in his part of the answer.

Riddhi : Okay. So, any idea on the number of beds that you plan to build for the annex that you already have, the land which you already have?

Viren Shetty : We won't be building it at this point in time. The things that we've announced, the 1500 beds between Bangalore, Kolkata, Raipur will be all the new projects. The annex building we had announced earlier but then we decided to push it in favor of pursuing something much closer to the city because there are sufficient beds in our flagship health city. We have a desperate need to fill that right now.

Riddhi : Okay. So, this 500 is for the new greenfield project that you're saying; ₹500 crores Capex ?

Sandhya J : In Bangalore, yes. The one that we've already announced.

Riddhi : This is both land and beds costing?

Sandhya J : Correct, correct.

Riddhi : Okay -okay, thank you.

Nishant Singh : Thanks, Riddhi. Parikshit, please go ahead.

Parikshit : Hi, thank you for the opportunity. So, first part I want to understand is that the India business, it seems like the bulk of the growth that we have gotten is because of the IP ARPP growth, which seems to be about 14.5% nearly this quarter. I'm just wondering why such a high number in the ARPP for inpatients?

Sandhya J : Sure. See, actually, if you see the India growth, there is a mix which is playing out and we have called this out. Our international mix has significantly declined and we have made it up with domestic business. Historically, the domestic ARPP has always been higher when compared to international ARPP. So, some of that has played in terms of the average ARPP that has come through.

Second also is, some of the advanced procedures that we do, there was a healthy growth in these complex procedures which come at higher realization. So, that has also helped in the growth of the Revenue Per Patient. So, in all, the ARPP has shown a healthy year-on-year and quarter-on-quarter growth.

11 R. Venkatesh : Yeah, adding to that, even the peripherals including the eastern and the western part of the country, there has been a substantial moderation of certain poor pay or profiles. Obviously, because of that shifting them into more of preferred payers, the benefit is also shown in ARPP and margins. That is also an added contributor to increase in the ARPP for this quarter.

Parikshit : Thank you. But can I just request you to repeat the first point again, Sandhya, because I didn't understand that? We usually have better ARPPs in India versus abroad and then I couldn't follow why that would increase the ARPP in India.

Viren Shetty : I'll explain that. Our international patients primarily come from Bangladesh. We had taken a call that we will not charge people coming from abroad a different rate than what we charge domestic patients, the people who come from Bangladesh to our hospital tend to be poorer than the average patients that we get from patients in Bangalore. The ones who can afford more end up going to the hospitals that charge more. And so, the average patient realization goes up when we refocus on domestic patients.

Parikshit: Got it. So that means that the footfall that we were getting from Bangladesh is impacted and continues to be impacted because of whatever is happening there. Is that a fair statement? Correct.

Viren Shetty: And there are multiple volatilities as

sociated with a business in India depending too much

on foreign patient travel. In the past it was the war in Afghanistan or some visa issues or you may have trouble with some countries in the neighborhood. So, it's not something that we want to be too reliant on going forward. And so, we have given guidance and I'll continue to do so to say that the international business is one number that I expect to go to zero over the next 10 years. So we're not spending too much time and energy focused on attracting more patients from international geographies, and focused instead entirely within a domestic radius, highest priority within the 15 kilometers of all our hospitals and larger circles around central India and eastern India.

Parikshit: Got it. So the flip side of this coin is that for the fourth quarter now, our inpatient footfall has either been flat or slightly negative on a year-on-year basis. So is the story here the same, that because we have to replace the footfall you're getting from Bangladesh or from abroad and we are replacing them with domestic customers and that is taking us some time? Is that the story?

Sandhya J: I think we are replacing with higher paying customers, whether it is... For example, one and a half scheme patients equal to one cash patient, that is how the mix works. So, when we 12 are able to attract more cash paying customers, it may look like you are seeing a lesser than regular growth on the footfall. So, I think that's why it is coming out like that. We are replacing reasonably well.

R. Venkatesh : So value wise, the replacements are far superior to what it was earlier. But the degrowth through these international patients have been to the extent of 19%, but the domestic has

grown by around 13.5 to 14%. But, that 14% has contributed to a higher value than the degrowth on the international patients to the extent of 19%. So that's how this equation is worked out.

Parikshit: Perfect, perfect. The other thing you mentioned is that the mix of the complex procedure is growing. Is this a one time or a slightly more arbitrary change in mix, or is this a mix that we should continue to expect going forward?

Viren Shetty: Dr. Rupert can answer that.

Dr. Emmanuel Rupert: Yeah, this mix, we should continue to expect that. There will be some month-on-month changes but considering the way we are focusing and some of our flagships are into very high -end tertiary and quaternary work, we will continue to see this trend.

Viren Shetty: For example, our flagship cardiac centre did a record number of TAVI procedures. TAVI is a very advanced cardiac implant and same on the cost.

Dr. Emmanuel Rupert: Yeah. And also, the kind of procedures that we are doing, we are able to very high -end cardiac work. Not only that, but oncology, robotic works have been going up. We did the highest number of robotic cardiac surgery in one quarter. We did around 113 robotic cardiac surgeries in one quarter, and there are some hospitals which have done 500 -600 over 7 -8 years period. So, this is the volume of work that happens here, and we will continue to see this kind of high -end work going up in this one. And the good thing is, most of our centres are also picking up on the complexities of the work, and we will see the uniformity of the complex work across most of our centres.

Parikshit: Got it. So, there is a pretty large difference between the ARPP growth in the IP segment versus the OP segment. Would I then be correct to assume that these complex procedures are mostly inpatient procedures and not outpatient ones?

Dr. Emmanuel Rupert: Yeah, most of the complex procedures are inpatient, but we are also reducing the length of stays of this. As the team is getting better and better, we are constantly working on that to reduce the length of stays of these procedures.

13 Parikshit: Got it. So, this 14.5% in ARPP growth for inpatients, would it be directionally

possible to split

this between normal inflationary increase versus complexity increase versus better pay masters or better paying customers increase? Would that be possible to directionally split that growth?

Sandhya J: See, our typical price increase is low single digits, 4 -5% every year. So, anything beyond that is all efficiency driven. Efficiency realization, higher order procedures, better payor mix, so everything is driven by that. Our price increase typically compensates for inflation partially, and partially we offset inflation through efficiency.

Parikshit: Got it. Coming to Caymans, I think you gave an explanation that the team has been focused on setting up the new hospital, they have been slightly more distracted. But this is now the second quarter where there has been negative footfall in terms of IP, right? So again, I would ask, is there anything structurally happening in the Cayman's market, which is leading to a fall in market share in that space, or is this the hurricane stuff and the distraction and basically that's it, nothing else?

Anesh Shetty: No, there's no structural change. In fact, we continue to be positive about the early traction with the new hospital and all the new services we are commissioning. I think it gives a more holistic picture in a small unit with relatively low transaction volumes to focus on not one metric, but multiple metrics to triangulate on a picture to get a better sense of the performance of the business. So, while just focusing on one metric can lead to alarm, I would just request to look at more of a holistic picture and the directionality over several quarters at a time.

Parikshit: All right. All right. Thank you. Thanks a lot. Thanks.

Nishant Singh: Yes, Bino. Please shoot your question.

Bino: Hi. Good afternoon to all. Most questions answered, but just carrying on from the previous question, what is the latest situation about these Bangladesh patients? Has the travel and other issues sorted out or is it still impacted?

R. Venkatesh : The situation in Bangladesh, it continues to suffer. The visa restrictions are still in its minimal. Only the critical patients allowed entry into the country. It would definitely take a long period of time for the situation to normalize. We don't even know and have no clarity in terms of the timeline by when it is going to get normalized. But as of now, the situation still remains grim in terms of footfalls from Bangladesh to our hospitals here.

14 Bino: Got it. And just follow up one question on Cayman. So, this quarter, for whatever reason, the inpatient numbers and the revenue per patient was lower. You just mentioned that it's just a quarterly phenomenon. So, are we expecting growth to pick up and continue next quarter onwards?

Anesh Shetty: Yeah, absolutely. In fact, we've already seen that in the first month of Q3, which is October. And there will be two reasons for that. One is just a return to normalcy. And also, at least by December of this quarter, the last month of the quarter, we hope to have the early signs of the new hospital with outpatient work and inpatient work kicking in. So, we have every

reason to believe that that will be back to normal and a little better.

Bino: Yeah, but the new hospital, of course, it is a new hospital. But just the patients, inpatient volumes and revenue from the old hospital, would that separately continue to grow or get back to growth back in this quarter?

Anesh Shetty: Yeah, as I said, so in October, we did not commission. We had no activities in the new hospital. It started only in November. And even in November, only the outpatient work. And in October, we've already seen a return back to our old trajectory for the old hospital as well.

Bino: Got it. Thank you.

Nishant Singh: Yes, Rishit. Can we please have your question?

Rishit: Yes, sir. Congratulations team for the decent set of numbers. Sir, I wanted an update on the

insurance plan, Aditi. We did our pilot in Mysore. Now we are expecting expansion in Bangalore. So, what's the ground attraction and what's the response at the pilot level?

Ravi Vishwanath: Hi, Rishit. This is Ravi Vishwanath. Yeah, I mean, you rightly said, we started with our pilot in Mysore. And the focus, of course, is making sure that our process and everything was good. We were sufficiently encouraged by the results that we actually pre-announced our entry into Bangalore and launched in Bangalore as well towards the end of the quarter. And right now, our focus is on building our distribution. We've done about Rs. 1.2 million of premium GWP since we started. But obviously, we're building a retail business and a direct retail business and that takes time to scale up. And so, our focus right now is on scaling it up and building various distribution support and channels for this. But the results have been encouraging and we're looking to accelerate growth in the coming quarters.

15 Rishit: Okay, that's great, sir. My second question would be on the overall business with Cayman coming in from this quarter and a lot of Capex coming in already. What type of growth number do we see for this financial year as well as the next financial year? What's the internal growth target for us?

Viren Shetty: I mean, we don't normally give out guidance on the growth numbers. We're just trying to maintain a healthy pace of growth, keeping in mind that we're not adding any new beds and we're doing a lot of internal re-optimization, and with a focus on a very healthy EBITDA and focus on improving the realizations per patient and reducing the ALOS. So, the trajectory that we've been able to keep so far, we'll continue to try and maintain.

R. Venkatesh : We have a long way to go for utilizing our efficiencies, transformation projects, using technology for improving the efficiencies. We've always demonstrated this growth in the last few years, and we'll continue to do that.

Rishit: Understood, sir. And sir, one accounting question that we have been taking advantage of at the consolidated level. At the consolidated level, we are getting a tax advantage due to the Cayman facility. Now the new facility coming in, do we see any further tax advantage coming in at the consolidated level or will stay in that bracket of 14 to 17%?

Sandhya J: See, Cayman comes at zero tax and India comes at India taxation rates. So if the share of profit of Cayman in the overall mix goes up, the ETR goes down. So that's the logic. But what we are seeing is that India also has been doing quite well in terms of India's performance itself, and Cayman will come back up after the new facilities commence. So, we would like to maintain the outlook on the tax rate as it is going currently.

Rishit: Okay, all the best team for the coming year. Thank you.

Nishant Singh: Yeah. Avneesh, can we have your question please?

Avneesh: I just have two questions. Means first one is what's the ROE number of the old established hospital and what's for the new hospitals?

Dr. Anesh Shetty: Can you repeat that please?

Avneesh: What's the ROE number for the old hospital which were established earlier and what's the ROE for the new ones which we recently established?

16 Sandhya J: It is not a number that we are separating out and reporting. Our ROE overall as a group, if you would see, as of 30th September 2024, we would be around 26.3%. So that's the number that we've reported out. If you split it between India and Cayman, India is slightly on the higher side and Cayman is slightly on the lower side, but still in that range only, mainly because of the Capex that has gone online in Cayman. But we've not reported out separately ROE for mature assets and new assets. It may also not be representative to split it out like that because some of the newer assets are asset light and the mature assets, some of them are

aged out. So, there are different stages of investment. So, it may not really make sense if we split it out like that also.

Avneesh: Okay. And I have a second question. How many years in which our investments come back entirely from when we done a Capex ? Means suppose we've done a Capex of ₹100 crores and established a hospital, then in how many years will that ₹100 crores return?

Sandhya J : See, hospitals are a very long -term investment business. So, it takes 8 to 10 years, but you

must also appreciate that a lot of the investments outside of biomedical, like the big chunk of the investment goes in land and building. So, your ability to sweat that asset exists almost perennially. So, to that extent, while you may take 8 to 10 years to recover the money, but on an ongoing basis, when you have such investments, they deliver more positive return on investments.

Avneesh: Okay. That's it. Thank you.

Nishant Singh: Yes, Kapil. Please go ahead.

Kapil: Yeah. Hi. Thanks for taking my question. My question is on the expenses which are classified under other expenses in the P&L. Now, the quantum of such expenses shown for the preceding quarter and in the quarter of the corresponding year were at similar levels. But for this particular quarter, there is a fairly significant increase. So, can you kindly elaborate on the reasons for the same?

Sandhya J: I think we would need to understand which line you're specifically looking at.

Kapil: Other Expenses in your consolidated P&L or your standalone P&L item under expenses shown as 'other expenses'. The terminology used is 'other expenses'.

Sandhya J: The Other Expenses, essentially overheads. If I look at the numbers, we are more efficient in terms of our overheads versus last year's same quarter. So are you asking what has contributed to that efficiency?

17 Kapil: No, let me clarify. What I saw was that the expenses incurred in the preceding quarter of this year as well as the quarter of the prior year are at a similar same level. However, in this particular quarter, there is a fairly significant increase. I was just curious what led to that increase.

Sandhya J: Can we get back on this to you?

Kapil: Yes, no problem. Yeah, that's it from my side.

Nishant Singh: Thanks Kapil. Nitant, you still have your hand up?

Nitant: Yeah, so there are certain other things I want to ask. So, on the insurance platform, what's the guidance regarding the gross written premium growth? And any reasons that we're going to expand in the coming years?

Viren Shetty: Ravi, the question is on GWP growth and just a rough estimate how we're thinking about the first year.

Ravi Vishwanath: Yeah, so I think generally we're not giving specific guidance on numbers, some of the things that we're looking at the moment which should lead to accelerated growth is, building out a lot of our distribution. We're also looking at sell a new product that we'll be launching later on in this financial year. For this financial year, we expect to be focused on the markets that we're in in Karnataka, and look to expand the market as we build momentum and build experience and understand the product market. There's a little bit of disturbance. I hope you were able to hear me.

Nitant: Yeah, I got you. Thanks, that was helpful. And one last question is that on the ATHMA platform, that's a SaaS provider. So what's the growth trajectory that the application is on?

Any new hospitals, any new healthcare institutions or processes?

Viren Shetty: ATHMA, for those who are not aware, that is an in-house hospital operating system. Our entire hospital runs on that. All our clinics, doctors, labs, radiology, EMR, patient apps, doctor apps, all of that essentially works off of that. We have created a SaaSified version that is available to outside partners. And we've enrolled on a trial basis Ida Scudder Memorial Hospital, which is a spin-off of the

CMC Vellore Group. We've partnered with a

few non-profit trust hospitals. There is a very large diagnostic chain in Karnataka that uses this. There are a lot of other smaller clinics that take this up. But we've not been aggressive at all in pricing the services. For us, at this point, we're just driving adoption. So, the revenue from our external clients is minimal.

18 But what we want to do is to build up a habit and understand what are the modules that they value the most, so we get a better understanding of their utilization. And over time, once we increase adoption, then you can see any significant contribution both from ATHMA as well as from Medha. Medha is our analytics and AI product, which again is used both internally as well as a lot of other hospital groups are using that. So as a revenue contributor, it's not contributing anything much now, nor we foresee it will be anything significant in the next 2-3 years. For now, we're just looking to really increase adoption and improve the product.

Nitant: That was a great explanation. Thank you.

Viren Shetty: Thank you. Nitesh, you have a hand up?

Nitesh: Yeah, thanks for the opportunity. Can you share the revenue and the burn in integrated care and insurance business for first half?

Viren Shetty: Yeah, give us a second.

Nishant Singh: So that number is there in the IR deck that EBITDA burn for this NHIC and NHL put together is around ₹13.5 crores for this quarter. And this number was ₹11 crores for the 1st Quarter. So collectively for the first H1, first half of the year, we have spent around... EBITDA losses

around ₹25 crores.

Viren Shetty: And the remaining year may not be significantly different from this.

Nishant Singh: You can expect a bit more, but the total number should not exceed ₹55 crores.

Nitesh: Okay, sure. I am a bit new to the company. Can you explain two things? One is that what is the business model in integrated care? And how do you see this scaling up? And secondly, what is the reason for entering Cayman Islands? And do you see further foreign entry in future?

Viren Shetty: How much time do we have? These are two of my favorite topics. I'll give you the quick summary. Integrated care, the way we think about it is that the growth of the healthcare business in India is closely tied with the growth of the payment systems. No country in the world you'll see at this level where so many people are paying out of pocket. In our system, 60% pay out of pocket. And even for the best healthcare group in the country, easily half their patients are paying out of pocket. That's simply not the case in most parts of the world where you either have the government or your insurance company paying.

19 Now, with insurance, of course, the health insurance industry in India is growing very fast, but it's still not growing fast enough as it should because of various reasons. So, we wanted to create an alternative product that is sort of like health insurance, but is more in line with bundled healthcare. So bundled healthcare means, where you pay us insurance premium and you pay us a care management plan. Both of these together will take care of the entire healthcare needs of your family within our network, as well as a narrow network of providers we enroll. And the goal with this is that you pay us and we've screened you. We will keep you healthy. We will keep you out of the hospital. We'll make sure you live a long, productive life. But should anything happen, we will take care of you, no questions asked, no exceptions, no room rent disallowance. None of the issues that normally plague the traditional insurance company, because we have 100% trust and alignment between the hospital and the insurance company. So, we created these as two separate entities, NHIC, which will run clinics and care management plans and house a lot of the doctors and clinicians. NHIL, which will be the regulated, IRDAI regulated

insurance entity, which will sell

the policies, do the underwriting, collect the premiums and disperse to the things in the network.

The question you asked on Cayman, Anesh will answer.

Dr. Anesh Shetty: To your question, why we started Cayman, essentially in India, we have a structural cost advantage. So, if you look at us versus peers, we realize on a per patient or per transaction basis, a phenomenally lesser amount, almost 35-40%. But on a profitability metric, there isn't that much of a difference. So, the goal and aim was to see if we could take the structural cost advantage or this more efficient way of operating in India and translate it to a very, very different market, where reimbursements are exponentially higher and see if that would work. So, Cayman essentially was an experiment to see if we can take our low cost, high outcome, high clinical outcome model and export it to another country. We started that about 10 ½ years ago. And so far, it's been very, very successful.

Nitesh: Sure. Thank you. That's it for my side.

Nishant Singh: Yes, Santosh. Can we please have your question?

Santosh: Yeah. Hi. So, I have a question regarding our revenue contribution from our specialty. Are we concentrating on any specific specialty apart from the top three, which we have for the next 3 to 4-5 years? And what is the reason behind it?

20 Dr. Emmanuel Rupert: Yeah. Apart from the cardiac sciences and the oncology, I think a lot of focus is going into GI sciences, orthopedics. Nephro Urology has been a very strong point in most of our centres.

But we are also focusing on the acute care aspect of it. So, the ICUs and the emergency cares, some of these areas we are working on. So, we have close to 1,800 beds in the ICUs.

So that is one area where we are working significantly on the care of that. As the number of geriatric patients are going up and comorbidities are going up in the country, there is a significant need for clinics and that is where we are looking towards.

Viren Shetty: Outside of specialty, we will be looking at investing in the latest modularity for treating patients. So, a lot of our hospitals have robotic surgery. Eventually all our hospitals will have robotic surgery. Similarly, in cancer, we will be getting into immunotherapy and offering that for our patients. So, we also want to be at the very advanced edge of what new therapies are available in the market.

Santosh: Okay. So, the Greenfield expansion and all this Capex, is it aligned with this kind of shift which you are looking into?

Viren Shetty: Yes. So, all the Capex that we are putting forward are almost adjacent or within a short driving distance of the hospital that we have. There will be tremendous complementarities between the doctors who work in our system and the doctors who work in the other hospital that we run. These hospitals will be built to the absolute cutting edge of spec. The thing about our flagship hospitals that even though they are the largest revenue contributors, their infrastructure dates back to 25+ years and so they are quite dated in their design,

layout and spacing. The new hospitals will have very very different way of thinking how patients come in and they are organized. We have invested a lot with Atma on digitizing the entire patient journey. So the new hospitals will not have as much waiting rooms, as many registration desks and reception. It will be a fully paperless reception deskless system where with your phone and with kiosk, you are able to navigate yourself through the hospital. Similarly, all the rooms will be able to turn into ICUs because our focus on intensive care means we want to be able to offer the full modality of care. The rooms will be single and double rooms and they will... all the operating theatres will have the ability to convert into robotic operating theatres. So these facilities will be built in going forward in our

network

other than the ones we have acquired but whichever hospital we would build from scratch will be absolutely future proof.

Dr. Emmanuel Rupert : They will also have significant number of day care beds, so that will take care of the short stay and the ambulatory care. So even though you look at the overall bed strength, a 21 significant portion of them would be allocated for the ambulatory and day care allocation.

Very similar to what the new unit in HCCB is doing.

Santosh : Ok, ok. My second question is regarding the insurance part. If you see, in the presentation, you showed the pay or profile, 29pc comes from the insured patients and all. In the call, you also explained about the insurance part where in developed countries, most of the payment is done through insurance and in India also, we are slowly moving towards health insurance.

So in the next 5 -10yrs., what is the difference we can see in our revenues or the working capital when the percentage of insurance increases in our profile?

Viren Shetty : Definitely cash does wonders for your working capital but it is also very deleterious to your realization because people who paying out of pocket are much more price sensitive whereas people who end up paying insurance while the insurance company will negotiate very hard with the companies on the Rack Rate and discounts on the Rack Rate. What ends up happening is that, your customers are a lot less price sensitive and they will opt for private rooms, they will opt for robotic surgery over the normal surgery. So, one is definitely much better than the other. If given a choice, we definitely want a situation where bulk of operations are insured so they are not fully exposed to the actual cost. So working capital part, yes, you will be stressed but we are also investing in an insurance company of our own and so we are hoping that 10 years hence, a large number of patients in our hospital come from our in-house insurance units. So it can be netted off.

Participant: Ok, understood, thank you so much.

Nishant Singh: Yes Raman, please have your question.

Raman: Ya, am I audible?

Nishant Singh: Yes.

Raman: Good afternoon! Most of my questions have been answered. I just wanted to understand the overall current mix of rates coming in from the out of pocket expenses and the ones which are covered by the insurance? So what is the bifurcation of the topline?

Sandhya J: Topline?

Raman: Ya.

Sandhya J: How much is cash and how much is insurance? You are asking about pay or mix, right?

22 Raman: Payor mix, right.

Sandhya J: It is part of our in-depth deck itself. 45pc of our patients pay in cash and about 29pc of our patients, I am talking about India, are insured patients.

Raman: How do you see these numbers going forward? What has been the industry like?

Viren Shetty : Industry wise insurance is 40 -45pc on average. It will be higher for some cities like Delhi, if hospital group is only focused on NCR or only focused on Hyderabad or Bangalore, it will be higher, maybe 50 -55pc. But on an average, if you do hospital groups, that are a mix of tier 2 and tier 1 cities, around 40 -45pc insurance patients is about average.

Raman: Ok thank you. That will be ok.

Sandhya J: Before we go to the next question, I want to answer that overhead question that was asked earlier. Sorry, I couldn't catch the question properly at that point in time. See, the reason the other costs are going up is because the overall number that you have seen includes the cash burn that we are seeing in the integrated care business. So, a large part of that is bunched in other expenses. If I eliminate the impact of the cash burn then the absolute overhead costs which are being incurred in the hospital segment has come down only as a percentage of revenue, though we have seen some stress in terms of fuel and power costs which have gone up across all locations. However, we have been

able to offset it through

other efficiencies. So as an overhead of the hospital business we are ok. But some of the cash burns, when you are seeing overall numbers, they are getting bunched up in other expenses and that's why it is looking disproportionate to you.

Viren Shetty : Rohan, can we get your question.

Rohan: Hello? Thank you for the opportunity. Am I audible?

Nishant Singh: Ya.

Rohan: So my first question was largely about the occupancy in the India business as well as the Cayman business for the quarter compared to last year? Thank you.

R. Venkatesh : Occupancies have been good for this quarter as far as the India business is concerned. Other than the flagship, the other units have nearly accounted for nearly 70pc which includes units like Raipur and units like Mysore, Shimoga. When it comes to flagship s, the occupancies have been at 64 -65pc and had not this issue of Bangladesh and the issue which had 23 happened in Kolkata in this last quarter that happened, the flagship would have also registered occupancies to the level in which the other hospitals have registered, which is nearly 70pc.

Viren Shetty : Cayman occupancy was a little muted in Q2, on the count of a lot of patients postponing their long stay procedures because of the hurricane . Generally, occupancy is not a number we track to, because of a huge amount of focus in Cayman for daycare and a lot of the out patients also but Cayman occupancy is about 50 to 60pc on an average.

Rohan: Ok. On my 2nd question, it was regarding the Cayman facility. As we see that Q2 is generally a stronger quarter than Q3 and we have seen a decline of 7pc in the topline Y oY, how do we see that going ahead in the next few quarters? Are we expecting the same numbers ahead or are we seeing the business is going up? Thank you.

Dr. Anesh Shetty: As I answered previously, the quarter was a little slower than we expected. There were quite a few small things adding up to lead to a disappointing quarter but then in the month that's have gone by, that's October, the first month of quarter 3, we are already back to our previous growth trajectory and a little better as well. So it was just a quarter that is likely to be, I mean certainly a onetime occur.

Rohan: Ok, alright, thank you.

Nishant Singh: Mr. Alankar, please go ahead.

Alankar: Hi, thank you for the opportunity. Sir, you mentioned that 65 -70pc cost of the Camana Bay Hospital were incurred in September. Possible to share that number for the full quarter, 2nd quarter?

Dr. Anesh Shetty: So in the 2nd quarter, we did not.....it would be very difficult and I am not sure it will be helpful also but what would help answer your question is that, by September this cost would ramp up. It's not onetime, they are ongoing costs, so the cost would be lesser and lesser in the early months of the quarter but we estimate about 60 -65pc would be incurred by the end of the quarter and now we are closer to early November, we are closer to about 70pc or 75pc of the cost. And in Q3, you will see, by the end of Q3, you will see a 100pc of the cost having been incurred. Having said that, what I can share is that, we are more confident that our measures taken to control the start -up costs have been successful. We initially anticipated about 6 -8 quarters to return to pre -commissioning margins. We now think that will be closer to 4 -5 quarters as I had shared earlier as well.

24 Viren Shetty : And cost in this case mostly refers to salary of the manpower we have hired to take care of the facility?

Dr. Anesh Shetty: Big fixed cost for the facility itself, that is just one shot.

Alankar: Actually that was my 2nd question because the hospital is yet to be fully operational and even the OPD you mentioned is operational for just a week. So is it just the cost factor which is giving us confidence to advance the timelines?

Dr. Anesh Shetty: Ya, so definitely the revenue

is ramping up quicker than we anticipated will help but we don't know if that's going to be quicker than we have anticipated because it's just been a few days but looking at the way the costs are shaping up, that did give us the confidence to give a slightly better guidance on that.

Viren Shetty : Sorry, but the confidence comes from the outpatient footfalls and the expression of interest from patients who are saying that they will come and bring their business to the new place.

That is giving us confidence that it will ramp up much faster and we won't be bearing this huge fixed cost without any commensurate revenue for as long as we had initially thought.

Alankar: Understood, ok. The 2nd question is, I mean if you look at the margins for Cayman, based on our understanding, seems to have reduced by more than 500 -600 basis points in a sequential basis in this quarter. Now possible to broadly break this decline due to higher cost which you mentioned for the new hospital as well as on the 2nd side, lower sales in the existing hospital?

Dr. Anesh Shetty: Yes, I mean an exact break -up won't be possible but you can sort of.....for example there are USD 29 Mn for Q2, the previous quarter was at USD 32 Mn and the one before was at USD 30.5 Mn or so, between USD 30 Mn & USD 31 Mn. You know our margins over there. So that gives you a rough sense of what can be attributed to the revenue decline and what will be on account of the new facility cost, a rough sense that should help.

Alankar: Fair enough! And 2 final book keeping questions. Broadly, what are our annual corporate overheads today? I mean what's the current run rate?

Sandhya J: 4pc of revenue is what we normally clock in as corporate overheads but that also includes a significant investment in technology because we are investing significantly in ATMA and MEDA and including that, we do about 4pc of revenue.

25 Alankar: Understood and the 2nd one is, what was the contribution from Bangladesh to overall sales before the disruption?

Sandhya J: Bangladesh was typically around 8 -9pc of revenue. We have come down to 6.5pc of revenue.

Viren Shetty: Prior to pandemic, it used to be 11pc, then it settled down post pandemic at 9pc, now it has come down to 6 -7 but as I indicated earlier, in a decade's time, this number will go down to 0.

Alankar: Understood! That's it from my side, thank you.

Nishant Singh: Thanks Alankar. Rohan, you have any questions to ask or we move to Deven? Deven, we can have your question.

Deven: In the integrated business, the realization per transaction is dropped sharply in Q2, almost at 25 -30pc, so any reason for that?

Ravi Vishwanath: No particular reason. Part of it is also to do with seasonality and people coming in for current lower ticket items is one reason in terms of the transaction.

Deven: Ok, got it. And finally for the next few years, what kind of EBITDA, cash burn rate do we expect for the NHIC and NHIL business?

Sandhya J: Similar burn rate we can assume at least for the next 2 quarters. The logic with which NHIC works is cohort by cohort which means that we start a particular code which is a group of clinics and that cohort breaks even in about 18 months. In parallel, we move on to the next cohort which will take another 18 months to break -even. So it is a cyclical thing. So we will continue to burn similar amounts of cash in the near future as we will be adding cohort .

Deven: Ok, ok, got it.

Nishant Singh: We will take a question from Prithvi and then we will go on to the chat questions.

Prithvi: I just have one question, you know. It is for Cayman business. As you have mentioned, 65pc of the cost from the new facilities are already part of P&L. So can we assume that for the entire Q2, it is zero revenue with a 65pc of cost from the new hospital? Is that right?

Dr. Anesh Shetty: Prithvi, just to clarify, I said, as of now, that is.....sorry, as of the

last month of the quarter,

so last month of Q2, we estimate that about 60 -65pc of the cost were online and zero revenue, that is correct.

26 Prithvi: But for June and August?

Dr. Anesh Shetty: It will be a number much less than 60 -65pc but it's tough to accurately estimate because there are some fixed costs, some people who are shared between both facilities. We are not maintaining 2 distinct P &Ls as we discussed before because it's a very, it's sort of like 2 campuses of the same facility, lot of costs are shared between the two but it is a rough estimate.

Prithvi: If that's the case, can we assume that Q2 margins are like bottom and from next quarter as revenue scales up, the margins should gradually improve on sequential basis?

Dr. Anesh Shetty: Because in Q3, we will see all of October again, zero revenue, in November minimal revenue because of just outpatient facility is being commissioned. If all goes according to plan, December is when we will start seeing the revenue but for October, November, we would have incurred very significant costs, November especially and December will obviously be 100pc of cost.

Prithvi: Ok, got it. Thanks a lot.

Viren Shetty : So I will get to the chat questions in sequence. The 1st question was, is the QIP on the company's mind? Not at this point for 2 reasons – one is, we have sufficient leverage available within the India balance sheet to be able to fund a lot of the expansion that we have planned through borrowing. We have sufficient internal cash flows also to take care of a lot of that. We are planning to do a lot more than that, post this phase of expansion, which point we consider. The other reason why the QIP is not something that, we are too happy about right now is because we do believe that the value that the market expresses, our company is not reflective of the underlying potential that exists. It's something that we think maybe we have to do a better job at communicating with the shareholders and we will work harder towards achieving that. So for those two reasons at this point, we are neither raising money at what we believe, at unfair valuation nor the need exists for it to be done right now but it's something we are open to doing in a year or two time.

The next question was asked about sharing current occupancy at Cayman. It was asked and we had shared earlier.

Question after that, on MED HA AI which was incorporated in December, any updates? We had shared that earlier but I will give a quick update again. MED HA is our AI and Analytics Division. They run a lot of the programs that we use internally, both for the AI tools that our 27 doctors and our managers use as well as the dashboards that help us run the finance and supply chain function. This is also a service that we offer to other hospitals and quite a few hospitals run our analytics program and they are also in discussion with a couple of more

hospitals but we are not too aggressive on the pricing as well as on the go-to-market strategy, it's something that we are focused a lot more on product building at this point and having it organically. So the revenue impact of this would be muted for the next year or two. The next question was, NHIC revenue per patient dropped Q on Q. Ravi answered that, it's just seasonality. Anyone who was in Bangalore in this past quarter and had to deal with the rain and the traffic, will understand why no one wants to get out of their house and walk to a clinic.

The next question is a pretty long one. So this is a question that, it's asked about the AT HMA, which is our hospital operating system. Are we targeting any particular hospital and some clarity on what the plan is for AT HMA? So the plan for AT HMA is, even though we have at least for this part of the world, the most comprehensive hospital operating platform, that includes nearly every module that you require for running a modern hospital, it is still

s

omething that we believe has years' worth of work left into it, both to make sure its comprehensive and end to end and two, to achieve our business objectives. The business objectives being able to either eliminate or automate or extremely simplify a lot of the manual wrote paper based, excel file based, system that exists in hospitals throughout the world. Unless we do this, we don't believe we can bring a transformational reduction in the cost of deploying every subsequent hospital we are looking at. So our headline quote for ATHMA and MED HA is that, the hospitals that we put up in future should not have a single person working there who is not touching the patient. Now the things we have to do to achieve that is a huge amount of digitization and reorganization and that is the stuff we are working on but in terms of the value that it currently provides to us is recognized by few hospitals and they are happy to have the software. It is just not matched entirely by their enthusiasm to pay a lot of money for that software and we are not pushing them too hard either because at this point, we just want to drive this option.

There is a question from Vinay. Can a Mumbai based person get covered under insurance? Not at this point. Our insurance is restricted to Bangalore and Mysore at this time and the way we will be rolling out insurance will be, rolling it out in all the areas where our network provides a huge amount of coverage. In Mumbai, we only have a Children's Hospital at this point. We are actively looking at getting an adults' program out there as well. When we do, we will set up a large number of clinics and along with the clinics, we will roll out insurance. 28 But for right now, our priority markets are Bangalore and Mysore, then eventually we will get into Calcutta and then Delhi.

The next question, give the break-up of EBITDA margins between India and Cayman. Sandhya will answer.

Sandhya J: So our India margin is, our India hospital margin is there in our investor deck. We have also given the cash burn that we incur in NHIC and NHIL. So the group margin minus the cash burn and the margin that India has made is the Cayman margins. So I think that's something you can calculate for yourself.

Viren Shetty : Next question is, does Bangladesh footfalls affect only the Kolkata cluster? Actually no. So Bangladesh, our Bangalore hospitals specifically the cardiac hospital, Bangalore has a much greater exposure to patients coming from Bangladesh because of the legacy impact of Dr. Shetty's active clinical practice. So Bangalore tends to be a lot more affected by reduction in Bangladesh footfalls. But as to why the discharge growth has reduced 8pc? It's a combination I think. As Venkatesh has said earlier, one is obviously the reduction of people coming from Bangladesh but also, the general movement away from very low payor segments towards those who are much more better placed. So that means you don't end up having a large amount of footfalls in low yielding patients. We can focus on fewer higher paying patients, taking higher end procedures that are much more viable for running this business.

This is a good question that Avinash has asked. What does the management think before establishing a new hospital? Any new initiative? The most important thing before establishing a new hospital is at this point, is it beneficial to the thing we have? So a new hospital can be there in Bhopal, there are opportunities in Agra, Meerut, there are opportunities all over the place but if it is not immediately value accretive to the current network which is not in the vicinity of any of the hospitals we have right now, it gets assigned a kind of relatively lower priority. The highest priority we assign to our flagship centres – Bangalore and Kolkata . Next in priority would be Delhi and Mumbai and then the other hospitals we will look at in reducing the order of priority. What do

we think before any new initiative? Again there are many initiatives that exist in healthcare industry. There is single specialty hospital, that is all the rage, there are people doing eye care, there is a lot ofthere are companies that have spun off their diagnostics, their lab business, their radiology, pathology business, there are hospitals that have gone into home care. For that, again we think, what makes sense for the whole? We have retail pharmacies but we have retail pharmacies only in retail clinics. We

have retail clinics but we don't have retail clinics all over the country. We have retail clinics 29 that are highly beneficial in terms of getting patients to our main hospitals. Home care is something that is offered as a service bundled with the managed care plans but it is not something that you can independently pursue. Similarly eye care is something we haven't done because we believe that most parts of the country are adequately served with eye care. Other single specialty – we had experimented with dental in the past. We had thought about single specialty, but it is not something we think is, something that customers expect from Narayana. I think people come to us for an overall value proposition, someone who is able to offer really high levels of care across the specialties at a very affordable price. So it's not the best fit for us. So in the end, it's generally the strategic intent of how we think about it. Does mental healthcare feature in your future plans? This is a good question. So mental health is a huge and growing need in the country. It is something that is offered as a service in all our hospitals but as to whether we can offer it as a standalone service, we don't believe so. In our NHIC clinic program, as part of the bundled healthcare services that are being offered, we would eventually offer physiotherapy, mental healthcare, lifestyle coaching, wellness and those sorts of things but it wouldn't be something that you would specifically come to a Narayana run mental health clinic. I don't think we would be very good at that. I think we are good at offering a large bundle of services and not hypo-focused on just one thing.

Priyank asked a good question. How do we add new operational better existing hospitals....

Viren Shetty : Priyank asked a very good question – how do add new operational beds in the existing networks without incurring much capex? Easy! You don't add more beds. You take the bed that you have and do more with it, which is you turn it into a higher yielding bed or you can look at things that can reduce the length of stay or improve the number of discharges. So adding more beds is the last thing we would absolutely think of. And if you absolutely have to add a bed, you would try and add it within the same building, either next to an existing bed or on top of the existing building or adjacent to the existing building or if you are not able to do that, then you have to buy land and go somewhere else. So when O&M models are a way of paying for it but the problem is, you don't always get to choose when you go for an asset lite building. How it is built? Where it is built for us? The location and its proximity to our existing hospitals matter the most. If we are able to find a hospital which is absolutely synergistic on all fronts, it is built the way we want, built where we want and if it comes at a price we want, then we would add that. If not, then we will have to make the investment ourselves.

30 There is a question about issuing bonus shares. That's a good point. I will sit with the IR team and discuss with them.

That is all the questions that I think is there on the chat. If you have any other question in the call....

Sandhya J: Damyanti couldn't finish a question earlier... I think she has dropped.

Nishant Singh: If there are no other questions, we would like to conclude our session. Thank you everyone for th

e active participation as usual. Please feel free to reach out to us for any follow-up queries that you may have. Thank you.

END OF TRANSCRIPTION

Call: Feb 2025

Date of submission: February 24, 2025

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code: 539551 (EQ), 975516 To,
The Secretary
Listing Department
National Stock Exchange of India
Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051
Scrip Code- NH

Dear Sir / Madam,

Sub: Transcript of Earnings Call for the quarter ended December 31, 2024.

Further to our earlier letter dated Tuesday, February 18, 2025 in relation to uploading the Audio Recording of the Earnings Call of the Company held on Tuesday, February 18, 2025 for the quarter ended December 31, 2024, please find attached the transcript of the said Earnings Call.

We wish to inform you that the Earnings Call transcript is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earnings-call-audio-and-transcripts>

This is for your information and records.

Thanking you

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q3 FY25 Earnings Conference Call"

February 18, 2025

MANAGEMENT : M R. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &

MANAGING DIRECTOR

MS. SANDHYA J – GROUP CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – GROUP CHIEF OPERATING OFFICER

DR. ANESH SHETTY – MANAGING DIRECTOR, OVERSEAS
SUBSIDIARY HCCI

MR. NISHANT SINGH – VICE PRESIDENT - FINANCE, MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. VIVEK AGARWAL - SENIOR MANAGER, INVESTOR
RELATIONS

2Nishant Singh: Good afternoon, everyone. My name is Nishant Singh, and I welcome you all to the Q3 FY25 Earnings Call for the company. To discuss our performance and address all your queries today, we also have with us Mr. Viren Shetty - Vice Chairman, Dr. Emmanuel Rupert - CEO and MD, Mrs. Sandhya Jayaraman - Group CFO, Mr. Venkatesh - Group COO, Dr. Anesh Shetty - MD of our Overseas Business HCCI, and Vivek - Senior Manager in the IR function. Before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the call shall be made available on our website as well as on the stock exchange later. We would also like to remind you that everything that is being said on this call that reflects any outlook for the future, or which can be construed as a forward-looking statement, must be viewed in conjunction with the uncertainties and the risks that they face.

With that now, we would like to start the Q&A session. I request everyone to now raise the 'raise hand' feature to start posing their questions. Yes, Prithvi, please go ahead.

Prithvi: Yeah. Obviously, the first couple of questions to Anesh on Cayman Business. Anesh, is it possible for you to provide some color on the scale-up of the new facility? Which departments have been commissioned, how is the response, etc.?

Anesh Shetty: Yeah, hi, Prithvi. Sure, happy to take that. So, we started the outpatient facility in the new hospital in the beginning of December. So, the results that you're seeing in Q3 only include the outpatient services, and that is also only from December, which is the last month of Q3. Prior to that, we were only bearing, I would say, a lot of the costs of the new facility, but we didn't have any revenue, incremental revenue, coming in. And the first few weeks of the new hospital have been fantastic. We've been very, very happy to see the positive response, and it really proves our investment thesis is in the right direction.

From the end of Q3 i.e. Jan 1st onwards, although those are not disclosed in the financials here, but just to give you some additional information, we commissioned a lot of other services such as the emergency room, inpatient surgeries, etc., as of January. The plan is in February i.e. mid-February onwards, to start obstetrics and neonatal care. So, hopefully by March or so, we'll commission all service lines. But the results you see only pertain to outpatient services being commissioned for the month of December. I hope that was helpful.

3Prithvi: Yeah. Just a follow-up on this. If you look at Cayman margins this quarter, they have been quite exceptional with the kind of sequential improvement that we saw. I mean, I'm not asking for the specific guidance, but is it fair to assume that margins, going forward, will be sequentially better from now on?

Anesh Shetty: So, sequentially better is a tough bar to cross. But let's look at Q2, where we had about, I would say, 5% to 7% margin dilution. And that's because we were starting to bear a lot of costs for the new facility, but no revenue coming in. Whereas if you see now, you will see a good amount of recovery, almost very close to our usual run rate on an EBITDA basis. And that's only with the outpatient services being commissioned. But for Q3, in our estimate, we've been bearing about 85% of the costs, if not slightly more, for the new hospital. So, we're very happy to see that a lot of that was covered because of the revenue growth. And it's a fair assumption... no hard numbers to guide on, but it's a fair assumption to say that I think the worst is behind us, that was Q2, and let's see how things play out next quarter onwards.

Prithvi: Got it. Just on the India business bookkeeping question, can you give it a breakup of the new hospital's revenue and margins for the India business?

R. Venkatesh: I mean, if you take the new hospitals combined, the EBITDA, if you take G

urugram,

Dharamshala and SRCC together, is around % for this quarter - Q3, which was around 3% in the Q3 FY24. So, there is a marked improvement. SRCC was marginally negative, but obviously Q3 is a weak quarter, but it will improve in Q4. Gurugram is marginally positive, and we

consolidate in Q4, while Dharamshala, it continues to do well at higher double-digit EBITDA. Going into Q4, we expect all these units to perform better, with Gurugram likely to lead in the improvement metrics.

Prithvi: Can you give the revenue number for these new hospitals?

Viren Shetty: Sorry, we will be breaking out the revenue by cohort, I think, going forward. Going hospital by hospital for the new one, would not be a material thing.

Nishant Singh: Yes, the three put together, is roughly around INR 130 crores.

Prithvi: Okay. Thanks. That is all from my side.

Nishant Singh: Thank you. Thanks, Prithvi. May we have the next question, please?

4Viren Shetty: Nishant, there is a question on the chat on occupancy of India business this year versus last year, and also EBITDA for Cayman.

Nishant Singh: See, the occupancy is, we do not give the exact numbers, but it is roughly around the same between both the quarters of the two financial years. It is slightly below 60%.

Viren Shetty: For the Cayman, do you want to help them derive it?

Anesh Shetty: Yes, I think, I mean, you have the console and India. The difference between the two is more or less close. There will be some other minor things in between.

Nishant Singh: Yes, so if you just add up the two, the difference is not exactly HCCI, there is a bit of ICO, inter-company elimination. But the number which you get to see is INR 293 crores as per the press release, which is what we should consider the revenue for HCCI. And EBITDA also we have given the console and India. So, from that, you can deduct Cayman EBITDA.

Viren Shetty: Hi Yash, please go ahead.

Yash: Yeah, hi. Thanks for giving me the opportunity. So, what I wanted to understand is, all the cash proceeds that you get from your Cayman Islands business, what is going to be the utilization of the same? And do you see any sort of prospects in any other regions nearby or probably you want to enter the United States?

Anesh Shetty: Yeah, so we've recently just disclosed a small investment in the Bahamas. That's another Caribbean Island, which is a high priority market for us. We continue to be focused on finding an opportunity to grow in that market. We get a lot of medical tourism patients from there. So, that's a high priority market for us. Nothing in the US that we are looking at the moment or in the meaningful past.

Having said that, of course, as a console, as an organization, India is home and the highest priority market for us. But we'll continue to explore opportunities where we think we can make a significant value addition in other markets. And it's just good timing you asked the question because I think on Friday, we disclosed a small investment in the Bahamas, which is a market in the Caribbean. So, it's very close to where we are in Cayman, and a lot of synergies between what we do there and in Cayman.

Viren Shetty: With regards to the question on cash proceeds, the India dividends are funded out of Cayman.

5Yash: Okay, fair enough. And another question that I had was on your Cayman Island business itself.

So, if you could just give a split of all the patients that come to a Cayman Islands unit. So, how many of them would be residents of Cayman Islands versus how many of them are international tourists from other countries?

Anesh Shetty: Yeah, we don't give that split. It's a small market. And for competitive reasons, we prefer not to get into those details.

Yash: All right, fair. Thank you so much. I'll jump back in the queue.

Nishant Singh: Thanks, Yash. Yes, Damyanti, can we have a question, please? Damyanti, please go ahead.

Damyanti: Hello. Thanks for th

e opportunity. I want to understand your gross margin for the quarter.

So, again, I guess very strong number, and this is even better than I think the Q2 numbers, which is sequentially better quarter for India business. So, can you explain this, what has led to such strong performance at the gross margin level?

Sadhya J: Yes. So, from, I think, gross margin, there is only consumption that kind of applies on gross margin. So, consumption has two factors that contribute. One is the mix effect. Depending on the type of procedure, the benefit in terms of the consumption mix. And second is the effect of efficiencies. Being a very lean quarter, so we've been very tight in terms of our execution and efficiency, and that is also reflected in the gross margin numbers.

Damyanti: Okay. So, although like India is seasonally weak, but because of efficiency, and as you mentioned, very tight control on consumables, etc., that actually led to better margins.

Sandhya J: Correct. Correct.

Damyanti: Okay. So, Sandhya, I think if you can also help me understand the other operating expense?

As Anesh mentioned, we are incurring almost 85% of cost upfront for the new facility in Cayman. So, is Q3 mostly reflective of the base structure for operating cost, and we should be looking at this cost base going ahead?

Sandhya J: Yes. From a Cayman point of view, that is correct. Most of the costs that need to be baked in into the operating setup, has already been baked in. And therefore, this can be considered as a good extension.

As far as India is concerned, Q4 operating cost structures will be similar to Q3, except that

the variable cost structures are variable to revenue and Q4 will be a higher revenue quarter. 6As we hit Q1, we will obviously see the impact of increments that will come across the line, and therefore, the costs will see an escalation to that effect.

Damyanti: Understood. And one question for Anesh. Anesh, you mentioned by March, all the parts of new facility will be like... all services will be started by March. So, just want to understand, are you done with all the hirings in terms of doctors, nurses, etc., or that also will happen in a more phased manner?

Anesh Shetty: No, no. So, generally, the manpower and the other costs we have to incur, and there's about a one-month lag between when they are here and when... I mean, when they're on island and when we can actually start the services. So, as of December, like I said, about 85% of the manpower hiring will be and the costs would be complete. 100% of the fixed costs of the facility, about 85% of the manpower costs. By Jan, that's going to be almost 90% and by Feb it will be 100%. It is 100% as of now.

Damyanti: Okay, got it. That's helpful. Thank you.

Nishant Singh: Anesh, there's a question on the message.

Anesh Shetty: Let me just go ahead with it.

Viren Shetty: The question is, are we looking for a controlling stake in Spire Healthcare Group? We had already put out a disclosure. We are not looking to make acquisition of controlling stake in Spire Healthcare.

Anesh Shetty: Yeah, that's correct. So, essentially, there's a no-bid statement, which is a formal declaration to the takeover code under Rule 2.8, where we're restricted from making anything. And we said that was not our intention and we don't intend to do so.

Nishant Singh: Yeah. Thanks, Anesh. Can we have the next question, please? Yeah, Prithvi. Please go ahead.

Prithvi: I just have one follow-up question, Anesh, on the Bahamas stake acquisition. What is the rationale for 4% acquisition? I mean, what kind of value will we be in a position to add if we have only 4% stake? Is company looking to acquire majority stake going forward, or what exactly is the thinking here?

Anesh Shetty: Sure. So, Prithvi, in the Caribbean, there are only two private healthcare assets, which are 100 million plus scale. Anything below that doesn't make much sense for us to wor

k with.

That's one is us, one is a doctor's hospital in Bahamas. It's also a publicly listed health system.

7Now, the intention is, this is very much an initial step. It gives us the optionality to increase that, or to learn more about the market, or to just see whether there is a potential to expand into the Bahamas. Similar to what we've done in Cayman, which is a significant investment or it's something where we'll only get medical tourists coming in and grow our international business. It's an entry in the market that gives us a lot of optionality to explore various strategies.

And having said that, with the management and with the existing promoters, we have a very good relationship, where on an arm's length basis, we are providing them with various synergistic services around procurement, planning, strategy, etc. But yes, you are right. At a 4% level, we don't have the complete operational control, obviously. But this is an initial step and let's see how things proceed.

Prithvi: So, just one follow-up question on this. Cayman government has been quite business friendly with respect to the way approvals, etc. have been given. How do you rate Bahamas in that?

Anesh Shetty: I think we have a good working relationship with most governments. We do have to find a fit between the priorities of the government, the other doctors and the medical community in general and what our needs are. In Cayman, we've made a large upfront investment, and then we worked to make it possible. But over here, this is an initial step in the direction. We continue to interact with the government stakeholders. They're generally very, very supportive of what we've been doing. In fact, one of the largest payers for us from an international perspective is the government in Bahamas. So, we have a good relationship with them, and even with the existing promoters of the hospital and the management. And we're optimistic about the future.

Prithvi: Thanks, Anesh. Thanks a lot.

Anesh Shetty: I'd also add that the investment was made at a fairly attractive valuation, if you look at the performance of the asset, but that's just a side bonus.

Nishant Singh: Prashant, can we please have your question?

Prashant: Yeah, thanks. My question is about the expansion plan that you have highlighted in the presentation. So, this includes a lot of greenfield projects, and most of the projects are... I mean, all the projects are either fiscal '28 or beyond. So, how do we read this? Is there any additional, say, brownfield bed addition that you would be doing over the next couple of 8years? Or are we to assume that the current bed capacity is what you will have for the next two years before these come on?

Viren Shetty: I'll answer this one. We are scouting for brownfield opportunities for buildings that already are there or hospitals that can be acquired or inorganics. It's just that those, while you can start the operations fast, are terribly hard to negotiate and the price has never been attractive

for us. So, the ones where we either build the whole thing by the land, put up the construction, or partner with the developer, he puts up the building, we put up the equipment, those have long lead times, but those are the only deals that end up finally closing.

It doesn't mean that over the next 2-3 years, we won't try and get some half-billed building or a hospital for acquisition; those are out there. And one day, maybe God will be kind to us and we will be able to acquire one of those at a price that makes sense. But until that happens, it's one of those 'bird in the hand is worth two in the bush' situation. So, we will try.

And actually, that ties into one of the questions that's asked on the chat, which is, owning land versus leasing, which is the preferred strategy going forward? The preferred strategy is whatever is available to us at the right price. If in a certain part of Bangalore, the only opportunity that

exists is to own it, then obviously, we would. That is the only choice there.

Whereas if someone else is willing to build and lease to us, that's the one that we would prefer. Most priority is for acquisition at the right price, but those are very few and far between.

There's another quick question on pre-engineered buildings to build hospitals, given it saves time and cost. Yes, but so my background is in civil engineering, although I'm no means the expert on this. Pre-engineered buildings are better specked for housing developments. These are for large townships, apartments; things that are the same size, same dimension, same plate, that repeats again and again and again. Whereas hospitals are unique, because one is, the spec of everything is different and the way the way the land works and the way the specialties are developed, pre-engineered doesn't offer any great advantages in India, plus all that additional cost. That's one why it hasn't been so successful. Two, the only companies that got into this in a way, went bankrupt. So now there are a few companies still left doing pre-engineered prefab structures, but those are quite not as reputed.

So yes, there are prefabricated elements inside the hospital that we will try to put in, like precast labs and so on, but mostly it will be this long in-site pour that we will be doing.

9Prashant: Just one more question from my side. So, acquisitions I understand are difficult to predict. Do you have scope to add beds in any of your existing hospitals?

Viren Shetty: Yeah. We have tremendous scope. But the question is whether the need is there. So in the Health City, for example, there's a lot of land and the scope for adding another 4,000 beds, but that part of Bangalore does not need 4,000 beds. In fact, we don't because we've been focused so much on reducing length of stay, improving efficiency, it's just easier for us to generate more revenue out of the same space than blindly go chasing after beds. But where we are losing out is that we don't have a presence in different parts of Bangalore. So we're spending a lot of money to be closer to where the patients are to improve the overall funnel of patients who will come to the Health City.

So yes, same for Jaipur, same for Ahmedabad, same for our Mumbai hospital, same for our Delhi hospital. All of them have the capacity to add more beds, but not the business case that requires it.

Prashant: Got it. Thanks. That's it from me. Thank you.

Viren Shetty: Yash, you still have a question?

Yash: Yeah. I just had another question. So, over the last 5-10 odd years, Narayana was little conservative in terms of adding more beds, doing some sort of brownfield or greenfield Capex in relation to other hospital chains. So, given that the cost of capital is a little lower than before, do you think this is the right time to get a little aggressive? I mean, this is beyond the addition of the beds that you have already planned.

Viren Shetty: It is evident from those who are paying attention to the Capex slides that, that has stepped up to another level. You can't time this in a perfect world. We should have gone in a more distributed fashion where there's, let's say, 1-2 hospitals coming up every year, spread out over 10 years. But now it is more of a lumpy distribution where a lot of hospitals are coming up at the same time. Historically, we were conservative. We will continue to be conservative. We will stay within our borrowing limits, and we'll try to fund with borrowing to the extent of our serviceability on repayments, as well as keeping in our dry powder for special situations as and when they occur. Rates coming down isn't the only determinant of whether we go complete gangbusters and build hospitals. It's a helpful determinant, but not the only one. Most is determined by what the business requirements are and the changing customer need.

10Previously, people would travel from all ov

er the country to come visit our places. Now, nobody does. I mean, very few people do that. So, we have to go to where the customers are. And so, that's why our hospitals need to be more spread out and across the city. And because we're committed to being an integrated care provider, we need to build clinics, we need to be much closer to customers so that they see value in buying our insurance plans. So, that is more the driver for the enhanced Capex allocation in our core cities i.e. Bangalore, Kolkata.

Anesh Shetty: Question on the chat Viren, on cash and bank balance available in Cayman. So, just a quick clarification. We disclose the consol cash and debt and we also disclose what percentage of the debt is foreign currency denominated, almost all of which is for Cayman. But we don't disclose the split between... investments between the two markets. Sandhya, you want to add something here.

Sandhya J: Yes, Anesh, that's what I was going to respond also. That part is not disclosed at the moment, yes. Obviously, in our financial results, which get declared at the end of the year, the breakup is readily available, because the standalone balance sheet, as well as the console balance sheet is available. So, I think that number can be computed.

Viren Shetty: Sorry for that. But we hope you'll understand why some things need to be calculated rather than put out there.

Nishant Singh: Okay. Deekshant, can we have your question, please?

Deekshant: Hi, management. Thank you so much for taking the question. The first one is that, I understand that it is expensive for us to buy a brownfield opportunity. But could you help us understand what kind of opportunity are we looking for? If you could just paint a word picture for us, how this opportunity would look like?

Viren Shetty: The number one determinant of the opportunity is it has to be in our core market. As I said earlier in the call, it could be a great asset in Nagpur, but we're not looking to expand in Nagpur right now. So, number one, it has to be in Bangalore or Kolkata, or second priority, Delhi or Raipur. If it meets that criteria, then is it in a good location? Is it the right size? Does it meet our business requirements? Meaning, is it in a place where we don't have a presence rather than being one more in a place where we already have a hospital? And the last determinant is the price, right? Do I generate a decent return on capital if I do acquire it? Or if there are some issues with how it has to be rebuilt to our purposes, because some buildings are very badly built, some will not be able to fit as many beds, some will not be compliant with all the licenses and fire and occupancy certificate, all of it. If it meets all of that, then we would take it up.

Deekshant: Okay, makes sense. What kind of IRR are we targeting for a brownfield expansion? Just a ballpark figure.

Viren Shetty: Nishant, tell him our threshold.

Nishant Singh: So, the IRR we expect for many of these projects is upwards of 15%. Yeah, so that's that ballpark number which we target.

Deekshant: Okay, so that's like for us, that would be the bottom line, if it makes sense at 15%. Makes sense. Sir, if we are to do a brownfield expansion in the core market that we are looking at, what would be the general timeline it takes for us to make it operational? So, if the deal is signed today, how long would it take for us to make it fully operational as per our standards?

Viren Shetty: Three years.

Deekshant: Three years. And the sort of greenfield expansions that we are doing two years down the line, sir, that would be much more of an accelerant for us as a fundamental driver?

Viren Shetty: No, whatever it is, it's three years. It takes 2 to 2 ½ years for construction, half a year for all the license; just buffer zone for commissioning and whatever extra regulatory things we have to deal with.

Deekshant: Okay, so the announcements you have made right

now, that would be first for construction

and then another 6-9 months for all the other fitments and licenses. So, that would make three years. Is that the right way?

Viren Shetty: Yeah.

Deekshant: Sir, last question is on the sort of work that we are doing in Caymans. Sir, I know that stem cell is a very big part of the business when we are looking in places like Cayman. Is Narayana Hrudayalaya really looking at that one function of business?

Viren Shetty: No.

Anesh Shetty: No, I mean, there are legitimate uses of stem cell therapy and there's a lot of non-approved, non-FDA approved, non-CEA approved indications. So, we do not engage in any services that aren't FDA approved.

Deekshant: Sir, if I can just expand on a question a little bit here, there is a lot of great work that is being done by legitimate doctors in US part of the world on stem cells, which is in Panama's, which is for basically shoulder reconstruction, healing, hips and all that. But there is no such opportunity in India as well. And Narayana Hrudayalaya, having a facility in Cayman is an opportunity structure for us. Is that something that we have thought about internally, or is it something we are staying away from right now?

Anesh Shetty: Dr. Rupert, do you want to?

Viren Shetty: Yeah, Dr. Rupert is the best person to talk about this.

Dr. Emmanuel Rupert: Yeah. So, like Anesh mentioned, there are a lot of indications which are there, but it has not been having a great proven value for all these things. So, we would be slightly conservative before we embark on something like that. Yeah. So, the availability of also those things are also not easy, but we keep evaluating all these new age therapies. But we have a core clinical

governance team, only when we think that it is the right approach for us to do, only then we will enter into something like that.

Deekshant: Makes sense. Sir, when you talk about value, is it a financial value or the treatment value.

Dr. Emmanuel Rupert: The treatment. It should make purely make sense for the patient. We would not like to do something which is not... It is there, but it gets constantly evaluated. From a stem cell perspective, the ones which we are doing is the CAR T therapy, which we have started. We have an in-house partnership with Immuneel to manufacture those things. So, we are constantly doing the CAR T therapy. But the new age... like what you mentioned indications for various other things for shoulders and various orthopedic applications, we are evaluating. And once we are fully convinced, only then we will be going.

Deekshant: Makes sense. Sir, just one last question if I am allowed to. Sir, we really like your Mumbai kids' clinic. Is there any sort of scope for a full-fledged Mumbai expansion as well?

Viren Shetty: In the existing hospital, we are in touch with the Trust to expand into the multi-specialty adult program. We are in discussion. We should hear positively from them over the next couple of months.

Deekshant: Okay, sir. Okay. Thank you so much for the clarity and transparency. Wish you the best.

Nishant Singh: Thank you. Can we have the next question from Nitin please?

13Nitin: Hi, two questions. Viren, if you can, a) just give us an update on the primary health clinic business and on the insurance venture, in terms of how it has fared for the year? And what are milestones for the next few quarters for the business?

Viren Shetty: Yeah, we've taken a pretty aggressive target for next year. This year and the previous one, was the year when we were just building it out, getting the operating team in place and focusing just on Bangalore. Next year, we will move into Kolkata as well. I've given them a target of 50 clinics in a year. It doesn't seem like we'll be able to hit that, but at least optimistically, that's something that we want to go after.

The burn on clinics will start to increase. It was about INR 14.5 crores in Q3. It may b

e a little

bit more in the year after i.e. the quarterly burn. We also launched Arya, which is the insurance plan that we actually wanted to build, which is full outpatient, full inpatient. And this is the one that's going to be our flagship product. It's just been out for a week or so, so it's not really showing much numbers and progress. And so, we're building out the sales team as well. Over the next year, you'll start to see both of them start to show. Still, it will pale in comparison to the hospital business, but this is something that we see really as an engine of growth for the integrated care business.

Nitin: On the clinics, what is the expansion that you have in mind? Where do you see the sort of peak losses really scaling up to?

Viren Shetty: We will not do what other competition has done, which is scale too fast. We will control the amount of burn to keep it manageable within the overall performance levels of the hospital.

So, when I gave the team a target of 50 crores, if they come back and show me that I'm going to lose about INR 40-50 crores every quarter, then I'll say, "No way". We'll try and keep it. Fortunately for us, the break-even period for these clinic cohorts is not that much. So if you're looking at 18 months of break-even for a lot of these clinics, then the profitable clinics will start subsidizing the newer loss-making ones. So, we've totally assumed about INR 400-500 crores worth of investment in this entire business. Now, that's roughly about how much we'd spend if I'm putting up a 250-bed hospital. So, it's one hospital's worth of spend to build a new business vertical. Now they have to prove that these guys can contribute, they can send patients, and they can sell policies. If they can, great. Then we'll look to scale up much more than that, but that will still be about 2-3 years away.

Nitin: And this INR 400-500 crore investment is over what time horizon?

14Viren Shetty: Total. I mean, this is the amount that I mentally set aside that it's worth building a new business out of it.

Anesh Shetty: That also includes the insurance related...

Viren Shetty: Yeah, all of that. That includes whatever I need to put in for the insurance. The integrated care means clinics plus insurance. Sorry.

Nitin: Right. And in the initial experience that you've seen on these clinics so far, what are some of the benefits that you've seen coming to the network? If you can probably share some?

Viren Shetty: For now, we're not able to capitalize on it so much. So, the normal thing what people... even for us, when we used to build clinics before we took it more seriously, we thought, oh, a lot of patients will come to clinic and we can send them to the hospital. That's not been the experience so far. These are high frequency, low acuity, low value transactions. For us, we want to have a much closer relationship with our patients. It won't lead so much to referrals now because clinics are seeing relatively healthier patients. But we want Narayana to be top of their mind, so that tomorrow they will be more enticed into buying the plan, the insurance plan, the Arya plan, what we're offering. Or, even if they don't buy the insurance, if they ask, "Oh, you know, I have this serious problem or something's been diagnosed, who should I go to?" Our name will be top of mind. So, it'll take a while for those clearly defined referral

pathways to emerge. But still, they benefit a lot from having a common earmark, from us being able to work with our specialists to come there and do evening OPDs or to come on weekends. So, it just gives us a larger surface area to expose patients to the kind of services we offer, rather than keep everything just stuck inside the hospital. But yeah, for now, the referral revenue may not be material.

Nitin: And just last one on this, so far in your experience a well-run clinic which sort of takes off well, establish itself well, what kind of profitab

ility it can do?

Viren Shetty: Retail have as such the only comparables are, I think, AHLL, which is out there. It is low to mid-single digit EBITDA margins on a consol level. Clinic to clinic level, see, we are running 3 different business verticals inside the clinic. One is the Pharmacy; one is the Test and Diagnostics and then the consulting revenue but also we're adding the insurance and the subscription plan revenue in that. So, it does look to be quite healthy on the balance sheet at least. We will have a much better sense of what the EBITDA should look like, like I said, 2-3 years from now once everything is mature.

15And we won't need to expand beyond a certain number of clinics because we're only going after the cities where we have a presence. We will not expand across the country; we will not build clinics in place we don't have a hospital and we'll only build it in our focus area. So, this is a build out that has a stop date. Post which, their only job then is to sell policies.

Nitin: Got it. Thanks. If I can just take the last one with Anesh. Anesh, on the overseas business, how do you overall think about this piece or what's the next 2-year perspective? I think in the Cayman bulk of the Capex is done. I mean, when do you see you hitting sort of optimal levels of revenues on Cayman? And what beyond Cayman from here on?

Anesh Shetty : Yeah, the next 2 years are going to be busy because we've just started the new hospital that gives us a lot to do at least for the next 12 months. From Jan 1st we've also started selling our Integrated Care, that is our insurance policies in Cayman. That's a completely tangential, I mean, it's a related business and very much feeds into what we're doing but it is a separate effort, separate build out as well. So, we have no shortage of opportunities to continue the trajectory we're on for the next 2 years. Now, what beyond that, obviously, we cannot time these things perfectly. We've been spending the better half of the past 5 years actively looking for opportunities in the region and beyond. And so far the only thing we have is the small investment in the Bahamas that we just talked about.

So, we continue to explore markets both in the Caribbean and elsewhere. But like we've always said, our focus is in India and which is at the moment the world's most attractive healthcare market for deployment of capital. So, whatever we look for has a very, very high barrier and high threshold to cross. That makes us very picky. But let's see, we'll keep trying.

Nitin: Okay, thank you so much.

Nishant Singh: Thanks, Nitin. Deven, hi.

Anesh Shetty: Sorry, before that, Viren, do you want to take the question on the chats.

Viren Shetty: Let's finish the ones on the call. Then we will go to the chats after that.

Anesh Shetty: Sure, sure. Yeah.

Deven: Okay, so should I ask?

Viren Shetty: Yes, Deven, please go ahead.

16Deven: Yeah. Anesh, so for this quarter can you give, share the revenue from the new Cayman unit?

And now that the hospital has started, can you share any sort of operating metrics for the new unit like IP, ARPP or OPARPP? What sort of ARPP should we expect?

Anesh Shetty: Sure, Deven. So, we briefly touched upon this the last quarter as well, but we'll repeat. I think it won't be possible for us, even internally, to differentiate revenue between the two units.

We're running them as two campuses of the same hospital. So, almost all the doctors are common, we don't have separate clinical staff. Only maybe a few frontline executive staff are dedicated. But it's next to impossible for us even internally to differentiate revenue and cost between the two. So, we'd only be reporting it and even for our own reviews and our own analysis as well we look at the hospital businesses as one block. And that's just because of the way we are operationalizing it.

And it's a good thing we're doing it this year because we've not added significant co

sts to get

the second hospital going but it also means that you lose that ability to have separate metrics and separate standalone analysis of these two. It's very overlapping, almost everything is overlapping.

Deven: Okay-okay, got it. And, secondly, coming to the Bahamas acquisition, the 4% stake, in the disclosure you have mentioned that it will also help you in referral access for tertiary care.

So, is that going to be something material for us?

Anesh Shetty: So, the material part of our existing business comes from international markets because Cayman is a relatively small market. So, we do have a good portion of patients coming from elsewhere. In the international markets, Bahamas is in the top tier of its potential for us as

well as its contribution potential more so. So, it already is part of the financials and the performance that you see, and we hope it will be a bigger part of it and this investment help to drive that.

Deven: Okay, understood. Thank you.

Anesh Shetty: Nitin, you want to go next?

Nitin: No, sorry, I'm done. Thank you so much.

Viren Shetty: You can take questions from the chat.

Nishant Singh: The first question is the long-term strategy for Gurgaon Hospital. Context being the location and the approach limiting the catchment area and in the context of Apollo and Max coming 17up, that will probably capture the extended catchment area. So, long term strategy for Gurgaon market?

Viren Shetty: Venkatesh, Rupert, you want to talk about the domestic focus?

Dr. Emmanuel Rupert: Yeah, so we are focusing significantly on the domestic market with the radius of around 30 kilometers and, like Viren was mentioning, the clinics are also coming up. We are looking at coming up with a couple of clinics there in that region. And the specialties will be focused quite a bit on the secondary and the basic tertiary specialties as well in addition to all the other things which we've been doing. Together, it will become a comprehensive kind of an approach which we have been doing in many other locations and we feel that going forward we will see reasonable traction in the units.

Nishant Singh: Okay, thank you. The next question is, are there any change in the expansion plans of the insurance subsidiary for the next 2 years, next two quarters?

Viren Shetty: Any change in the?

Nishant Singh: Expansion plans of the insurance subsidiary for the next 2 quarters.

Viren Shetty: Insurance. Okay, if it's just insurance, no. I mean, they are scaling up moderately, so I think it's adequately capitalized. We don't need to infuse too much capital just for the insurance part. The only ramp up in spend would be mostly on the clinic side, which is a separate vertical called NHIC.

Nishant Singh: Okay. There's another question on the expansion plans on the west side.

Viren Shetty: Yes. I assume they mean the western part of India.

Nishant Singh: Yeah.

Viren Shetty: So, the hospital we have in Western India are Mumbai and Ahmedabad. At this point, the only thing we're looking at is if the SRCC Hospital in Mumbai, if we can turn that into an adult program as well. So, it'll run as a children's hospital plus also have the adult program. Outside of that, it is not an immediate thing that we're looking at. It's something once we've finished the buildout for Bangalore, Kolkata, we may look at that.

Nishant Singh: Okay. There's a question on the same U.K. thing, which you already clarified. The next is on the expected ARPP growth and revenue growth which we're targeting.

18Sandhya J: Let me take that. From ARPP growth point of view, as you are aware that our price strategy is not a very aggressive price strategy. So, the growth will be similar to what you've been seeing so far in the past. It'll grow reasonably but over a period of time. As far as revenue growth is concerned, I think our organic growth trajectory is already established and, therefore, we will be able to sustain t

his kind of organic growth trajectory. As and when we

have some opportunities to inorganically augment this, we will see the benefit of that. We will obviously see the benefit of Cayman growth in the near quarters because at the moment not much of the revenue of the new hospital is on the baseline. So, that uplift we will see in the coming quarters. Similarly, we will see some amount of uplift coming in from the growth of the capacity that we've added in our MMRHL hospital and a little bit of capacities that we will add in across different hospitals. But broadly, it will be in line with the trends you have seen so far from our organic growth point of view.

Nishant Singh: Yeah, so we'll go back to Deekshant.

Deekshant : Hi. Just a question on the insurance category. So, I'll just ask some ballpark numbers. We have talked about having the product in such a manner which aids in regular treatment plus the recovery of individual if something major has to happen, do we also cover senior citizens in this category as a floater policy?

Viren Shetty: Yes, but there will be some loading on senior citizens because of the comorbidities and the things that work with that. There is a thing, we're still in the early stages of formulating, which is a senior citizen specific policy, but that would be quite expensive but also very comprehensive. We don't know how well that would sell but it's something we think the market does require. But for this one, yes, with some amount of loading, we can add senior citizens but not in all cases.

Deekshant: Would that senior citizen policy would also have if somebody has existential diseases because most senior citizens in India do.

Viren Shetty: Yeah, it depends on that. So, I mean, if you're looking for your parents, I'll share my number and after this we can sit and talk to you. But, yeah, it is something that comes as a frequent request. There are some things that we know are quantifiable and manageable because we're

in the healthcare business, we know there's certain risks that you face and at the various ages where most insurance companies will just say, 'No, I don't want the hassle, forget about it'. But for us, we can make very customizable solutions to offer to our customers. But a senior citizen specific one, that's something definitely we are working on and would like to have a rollout sometime next year.

19Deekshant: Thank you so much, Sir. I mean, I'm also looking at it from actually a business standpoint for us. Of course, personal standpoint would be helpful for all of us. I'm just assuming here, if we are looking for a plan where it's wife and husband plus family, is that some sort of product that is being created or being sold right now? And what is the revenue leader product for us right now in the insurance category?

Viren Shetty : Right now it's still very early. So, we have two products, which is family of 4 or family of 5. There is the inpatient only product, that one is called ADITI, then there's the inpatient plus outpatient product called Arya, that we believe will be the flagship product because that is the fully integrated thing that we're offering. Sandhya, any extra things you want to add on that, on pricing and so on?

Sandhya J : Yeah. So, ADITI is a missing middle product and, therefore, it is priced in a very affordable way because ADITI is like an entry level for people who are not able to access insurance today. And Arya is for people who are actually having insurance or are able to afford insurance but are not happy with the experience of insurance. So, Arya is an end-to-end seamless experience that is available right from outpatient to management of clinical services that are needed as well as if there is an inpatient event that happens, then management of that as well.

Like Viren was saying, we're still working with these products in a very controlled way so that we are able to manage the overall thing. So, it will take

some time for us to be able to share numbers on these products. Though from a customer feedback and people who have experienced the product point of view, we are getting very, very positive traction. The biggest advantage that you have in both these products is a complete walk-in, walk-out experience. So, when a patient is in our network and they walk in, there is no pre-approval and pre-auth and clearance and then waiting for insurance clearance and no deductions, no co-pays. So, it's a very clean walk-in, walk-out, 100% trustable kind of product and that's what we see the people who are now buying the product or understanding the product like about them. But like we've said, these are still in very early stages and as these products build up we'll be able to share more information.

Deekshant : Just two small follow ups here is on the Arya product that you have mentioned, Ma'am. Ma'am, this would, I assume, only be viable at Narayana Hrudayalaya.

Sandhya J : For elective, yes. For emergency, we are covering in all hospitals.

20Deekshant: Oh! Okay. Got it, got it. So, if it's a pre-planned surgery...Okay, got it, got it. Another part is if we are doing so end-to-end of a product, which is amazing for the customer and really revolutionary product in the industry as well, does it not lead us to more risk in terms of our net ROI of the product?

Sandhya J: So, we have a very strong underwriting process that we are following as well as a mathematical model by which we are managing the risk. Obviously, only with scale the risk balancing will happen and as we build scale, we'll be able to balance out the risks. But whatever the norms are available and whatever our judgment on those are, we have considered the risk in the pricing of the product. Viren, you want to add to this?

Viren Shetty : Yeah. See, the biggest risk we face are the thousands of crores of Capex we're putting up for the hospitals and people are complaining a lot in the market that healthcare is becoming very unaffordable, and I don't blame them. The sort of price increases what you're seeing is ridiculous and the sort of price increase in insurance also what you're seeing is ridiculous. So, for us, all this capacity what we're making, we want people to utilize healthcare more because that is what the natural order of things is. Your great grandparents never went to a hospital, your grandparents with some reluctance went, your parents would have gone once or twice, you will go quite often, your children, for every cough and cold you're taking them to ICU. That's just how it is worldwide. You can't stop that and insurance is the one that removes the barriers.

So, the ROI, it's something we'll dynamically adjust. We are not insurance experts, we don't know it but we know healthcare and we know you need to access more healthcare and we know that once you make an offering that works with the patient and does not look to extract as much value as you can, your patients pay over long periods of time and they are loyal and you can lose money upfront but eventually you make it back.

Dikshant: Agreed, agreed, agreed. Thank you so much, Sir. Thank you so much.

Viren Shetty : Thank you, Deekshant, for allowing us to advertise our insurance products for the 15 minutes of this call.

Nishant Singh Yes, Deven, please go ahead.

Deven: Yeah. So, in your slide, on the Capex slide, at the start of the year we had budgeted a

Brownfield Capex of INR 1350 Mn but we have ended up spending only INR 240 odd Mn, so there is a significant deviation from what we had planned at the start of the year. So, if you can just explain this difference? What happened throughout the year that we could not spend or did not want to spend the budgeted amount?

Viren: For one, these things take longer. Sandhya, please go ahead.

Sandhya J : No, go ahead. Go ahead, Viren. Sorry.

Viren Shetty: It just took us much longer, the negotiating time, it took for

us to expand in India. The time it

takes to negotiate contracts to work on the licensing, just generally we haven't been able to execute fast. It's not for want. We want to spend; we want to add capacity.

Deven: Okay. No, but this will be largely in our existing hospitals, right, the Brownfield capacity addition budget that we give in the slides?

Sandhya J : Yeah, I'll just add to what Viren was answering for the Greenfield part where we budgeted quite a bit and we have not completely caught up to it. But there are projects in pipeline and we do believe that we will bridge some of it in the current year. Otherwise, we'll just flow over by a quarter. As far as the Brownfield and capacity addition part of it is concerned, roughly half of it is just the CWIP in different stages which we need to capitalize because as we keep doing these projects, a lot of the bills, etc. come for settlement and then we have to take them through capitalization and that process has taken some time for us. We will bridge most of that Brownfield capacity addition; we will bridge. Some of it will flow over to next quarter.

Brownfield; there was one more aspect in Brownfield, which kind of we had anticipated.

Sorry, one more aspect in Brownfield which we anticipated, which was the entire Health City expansion Capex. If you remember, at the beginning of the year we were looking at commissioning an extra tower inside Health City because we were looking at that as a growth of beds but then we pivoted to more capacity creation across the city and that is why we have projects now - the HSR project, the Central Bangalore Project, the South Bangalore Project. And, therefore, we have for the time being reprioritized a little bit on the Health City expansion plan and we have to the expansion across the city. So, that also is some part that will not get bridged on that line, but it will get bridged on the Greenfield line.

Deven: Okay-okay, understood. And whatever Brownfield Capex that, let's say, we will do in this year, where are we adding the beds? In which regions are we adding these beds?

Sandhya J : So, one is, we have done the expansion in Howrah. That is one place. We are also doing a little bit more expansion in our Barasat unit. In addition, we have 5 beds, 10 beds, expansion across different, different units where different types of capacity bottlenecking initiatives are there. Within Health City campus itself we are completing some amount of remodeling and restructuring. We did a lot of it last year, this year there is still some flow over and capacity re-shifting. So, moving more beds from General Ward beds to Critical Care beds where are seeing greater traction. So, those kind of work also is currently happening. So, that's where the Brownfield and capacity addition is happening.

Deven: Okay, understood.

Viren Shetty: It won't be a material addition of beds, most of this is just a reallocation to target and improve the yield.

Deven: Okay, understood. Thank you.

Viren Shetty: There was a question and that's related to what he asked about the new HSR hospital. There's a question on the chat about why we're spending so much more than peers. So, the peer spend is 1.5 crores per bed, ours works out to little less than 2.5 crores per bed. For those who are familiar with Bangalore, HSR is where all the startups are. It is nearly impossible to get large parcels of land and the fact that we found this at all, we had to pay through the nose to get the land. And, so, the increased cost is a reflection of the cost of running the only tertiary care hospital in HSR Layout, which otherwise does not have any.

I can maybe do the other chat questions. We're looking at Group Insurance policy for faster rollout of the insurance business. Group, yes, you get faster rollout, you also get to lose money faster. We're targeting small employers, SME employers, smaller companies because they tend to find these things a lot

more attractive. Large employers will not find a restricted narrow network plan very attractive, whereas smaller companies in the vicinity of our hospital network will say that the value is much worth to them. So, those are the targets we go after. The discussions have started and that also will be an important channel, but we don't want to lose too much money chasing volumes.

Anesh, can you answer this question on the doctor attrition in Cayman? And how do you replace them in case anyone goes back home?

Anesh Shetty: Sure. So, attrition for doctors in Cayman is very, very low. They've made a big commitment personally and career-wise to relocate there. So, attrition is not the problem. But it is difficult to hire new people and the rare occasion that they leave to replace because not just

professionally but a lot of things from a personal standpoint have to fall into place with 23 children, parents, family, spouse, etc. So, it is tough. But that's how it is in many markets, I guess.

Viren Shetty: There's one more question on the chat. Last time someone brought up the bonus shares and we had a big philosophical argument on our side on what does that really do on a per share price because essentially nothing is changing on the equity side, you're just issuing more shares and so you're dividing it by a larger number. Philosophy aside, we don't think now is the right time to look at bonus shares. I think there are much more higher priority things we're working on in our company and much better performance to be expected from a lot of the investments we made in technology in our clinics in Cayman that will drive the performance of this company.

Sandhya J: Also, to add to what Viren said, we will continue to be consistent in our dividend strategy. What we've done for the past few years, we'll continue to maintain that and that way we will also be ensuring that we are giving a reasonable cash return to the shareholders subject to approve approval of the shareholders, of course.

Nishant Singh: Are there any more questions, please, because we're done with the questions on the chat? Yeah, there's one more question now.

Viren Shetty: Yeah, Nishant, this is your question. How do you find Capex?

Nishant Singh: Yeah. So, funding for the Capex will happen through a mix of debt and internal accrual. So, for all the projects we have closed so far, we are closing the project finance with the banks for long term loans, say, between 10 years and 20 years. 80% odd is going to be funded by the bank and the rest is going to be funded by the internal accruals.

Any more raise of hands for asking any questions? So, if there are no more questions we would like to end this session.

Nishant Singh: Yeah, sorry.

Viren Shetty: This one is for Venkatesh or Dr. Rupert. How do you measure throughput in OPD? What are the benchmarks we look at? Maybe you want to talk about all the kiosk work and the TAT and the lab test results.

R. Venkatesh: Yeah, I mean, we are working very aggressively on the apps which we have where you have clear cut appointments worked out through the app before the patient comes into the OPD.

So, from the time you book the appointments and the time you visit a doctor, the benchmark 24 is, if you look at 6 months back the benchmark was around less than 30 minutes, now the benchmark is less than 15 minutes; how fast you can process the patient from the time he enters the hospital.

And, also, what we're working on is the kiosks across all the major hospitals where we've installed all the kiosks over the last two quarters. Here, what happens is, the entire queuing systems are going to be discontinued. The patients do all the registrations through the kiosks and there'll be only one counter there in every hospital where all the cashless or the credit systems will be addressed but that will also disappear over a period of time. So, it's all going to be automated where the patient

just comes in, gets the consultation and the investigation billing done through the kiosks and then gets a doctor consultation, gets the investigation again done through the kiosk and then gets the report on the app and moves ahead without any files.

So, it's all going to be digital, it's all going to be paperless, and the benchmarking obviously is that the consultation time should be within 15-20 minutes from the time he reaches the hospital and the routine laboratory and the investigation reports should come to him within 30-40 minutes from the time the samples are taken from the patient. That's a benchmark which we're following and we're also working towards gradually trying to improve even those benchmarks going forward in the quarters to come.

Dr. Emmanuel Rupert: Broadly if you look, from the time they enter into the consultation area they should be out with the labs and diagnostics and other things, with the reports, go back to the doctor, get a clear clinical plan and move out of the hospital within a 2-hour period in most cases.

Viren Shetty: See, why that is important is we are investing in a significant amount of money in a footprint in the city and in the cities we don't have the luxury of having very large spaces and thus we're not able to accommodate too many people waiting for test reports, waiting for the doctor, just waiting for information that can be conveyed. So, we want to be able to build a 200-250 bed hospital but essentially have it function like a 500-bed hospital.

So, the limitation for us being the physical infra and the way we address that and increase the throughput by investing in all of this is to ensure that a lot of it happens on the app and doesn't require any sort of counter skewing of that. And so, this will really, really be useful for us when the next lot of expansion comes in.

Nishant Singh: There's another question on the chat. Why Cayman Islands as a location is important for Narayana?

25 Viren Shetty: Anesh, your favorite question.

Anesh Shetty: Yeah, Viren, go ahead. You want to take it?

Viren Shetty: Okay. See, Cayman, as we've gone through in the past, it has a history behind it. This was an area we chose because we wanted to make a very large scale medical tourism. We wanted to go after the largest medical tourist market in the world, which is the United States, the largest healthcare market in the world, but we didn't want to build in the U.S. itself. We thought we'll come to Cayman, we'll be able to attract large numbers of medical tourists from the U.S., we tie up with the U.S. healthcare hospital operator and they will send their patients over there, large employers in the US who have very high expenses will send their patients over there. Large employers in the U.S. who have very high expenses will send their patient. It made sense on spreadsheet; reality was the medical tourism from U.S. never materialized but there's much more of a Caribbean and a local Cayman business that was there. And, so, why it is important for us is to show that our model, which delivers good performance in India, but it's very Capex heavy, it takes a long time to breakeven, it's operationally very complex. You apply the same model in the Western environment where the reimbursements are much better, the hospital also performs much better. So, why it is important for us is because it does well. We've run a pretty good business, we'd like to say, and we want to see if we can replicate. So, it is for us to use that as a place to show that our model is far superior to a lot of the Western healthcare operators and to use that as a place to see if we can try any other things overseas in the Caribbean or other, you know, U.K. jurisdiction type countries.

And there's a reverse flow as well because the things that we learned in Cayman, because it's a small country, manpower is very expensive we learnt how to be much more efficient and a lot of th

ose lessons also flow back to India as much as India provides a template for Cayman.

Nishant, there's a question for you on Debt to EBITDA ratio. How are we looking at it?

Nishant Singh: Yeah. See, it is true that there will be a lot of debt for this project finance. We are comfortable with the Debt-EBITDA levels up to a level of around 3, so we will be conscious of that number and we will probably reach there if you do all the projects we have planned for, say, by the end of the 4th year. But then what will happen is that as the new hospitals start to accrue cash and be positive on the EBITDA, those numbers will rapidly come down. So, when those numbers start to come back down then that's the time when we start to have the next phase 26of expansion. But around 3 is the number which where we have with this internal discipline, which we'll try not to breach.

Viren Shetty: Anesh, what are the reasons for Americans not leaving their country to go overseas for treatment?

Anesh Shetty: We can have an hours long discussion on this but a quick 2-minute version. It's very complicated but patients in the U.S., as far as we've learnt, they don't even cross State borders for healthcare unless it's to a center of excellence like the Mayo Clinic, Cleveland Clinic, etc. And patients are in the U.S. are not making consumer choices. There are a lot of intermediaries, principally the insurance company and others. So, unlike India where patients make a consumer choice, they're really customers and not patients. In the U.S. there are a lot of channels, preset pathways, discount arrangements, networks and so patients are not fungible from one location to one another and especially this is within the U.S. from State to State or from hospital to hospital. And if you're talking international borders, it's several orders of complexity more challenging.

Viren Shetty: Having said that we are seeing that in India as well. A lot of the bets and assumptions we made on medical tourists coming to India from. We used to get a lot of patients from Indonesia, we've got patients from Malaysia, we used to get from Iraq; those don't last very long. There's certain habits and given a choice people want to just get treated in their own city, forget about their own country. And even within India, there's a lot of cross-State, across the city travel that has nearly stopped.

In Bangalore we're located in one corner of the city all the way in the South and we know that we don't get patients from North Bangalore there. So, we want to attract those patients, we have to be there physically. So, this is a behavior that we see in the U.S. and is very fast catching up in India as well. And so, we can't just accept that things will change and if we make it more attractive, people will travel in India. We have to go and make the investment and go to where the customers are.

Next question is, how do you look at your organic, inorganic expansion plans in medium term, 5-7 years? 5 to 7 years, with confidence I can see that it will still only be what we're currently doing. Stick to our focus markets, just look at building smaller sized hospitals in Bangalore, Calcutta and a little bit of expansion in Raipur. Have only one large Health City per city, the rest Health City means a flagship hospital that's 1000 beds plus. Every other hospital, 200-250 beds is sufficient. Just make sure that you are located dotted around the city so that patients will not need to travel more than half an hour to get to one of your setups and all 27the gaps you fill up with clinics and have an insurance plan to tie it all together. So, 5-7 years, this is more than enough to work for us to be able to achieve within these 2-3 cities. We prove it works then we'll see if we can roll it out to the other major cities.

Nishant, anything on the chat on your side?

Nishant Singh: No, there are no new questions. We'll just wait for a minute to see if the

re are more questions

on the chat.

Viren Shetty: Okay. Yeah, I guess we are done.

Nishant Singh: Yeah. Yeah, so I think as there are no more questions we would like to conclude this session.

Thanks everyone for the active participation as usual. If you have any more questions, please feel free to reach out to us. Thank you.

END OF TRANSCRIPTION

Call: Jun 2025

Date of submission: June 02, 2025

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code –539551(EQ), 975516 & 976418 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051

Scrip Code - NH

Dear Sir / Madam,

Sub: Transcript of Earnings Call for the quarter and financial year ended March 31, 2025

Further to our earlier letter dated Tuesday, May 27, 2025 in relation to uploading the Audio Recording of the Earnings Call of the Company held on Tuesday, May 27, 2025 for the quarter and financial year ended March 31, 2025, please find attached the transcript of the said Earnings Call.

We wish to inform you that the Earnings Call transcript is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earnings-call-audio-and-transcripts>.

This is for your information and records.

Thanking you

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q4 FY25 Earnings Conference Call"

May 27, 2025

NH MANAGEMENT TEAM:

M R. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – GROUP CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – GROUP CHIEF OPERATING OFFICER

DR. ANESH SHETTY – MANAGING DIRECTOR, OVERSEAS
SUBSIDIARY HCCI

MR. RAVI VISHWANATH – CHIEF EXECUTIVE OFFICER, NHIC

MR. NISHANT SINGH – VICE PRESIDENT, FINANCE, MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. VIVEK AGARWAL, SENIOR MANAGER, FINANCE &
INVESTOR RELATIONS

2TRANSCRIPT

Nishant Singh: Good afternoon, everyone. My name is Nishant Singh and I welcome you all to the Q4 FY25 Earnings Call for the company. To discuss our performance and address all your queries today, we also have with us Mr. Viren Shetty - Vice Chairman, Dr. Emmanuel Rupert - CEO and MD, Mrs. Sandhya Jayaraman - Group CFO, Mr. Venkatesh - Group COO, Dr. Anesh Shetty - MD of our Overseas Subsidiary, HCCI, Mr. Ravi Vishwanath – CEO, NHIC and Vivek - Senior Manager in the IR function.

Before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website as well as on the stock exchange later. We would also like to remind you that everything that is being said on this call that reflects any outlook for the future or which can be construed as a forward-looking statement, must be viewed in conjunction with the uncertainties and the risks that they face.

With that now, we would like to start the Q&A session. I request everyone to now use the 'raise hand' feature to start posing their questions. Yes, Prithvi, please go ahead.

Prithvi: Thanks, Nishant. The first couple of questions are on Cayman business. If you look at the extent of growth, that has suppressed quite positively in this quarter. So Anesh, could you provide some color on which are the departments that are doing quite well? You know, how is the reception from the residents there? Also, just a follow-up on this, is it fair to assume that \$45 million will now be the new base for Cayman business going forward?

Anesh Shetty: Yeah, thanks, Prithvi, for the question. So, to the first part of your question, the hospital has been received very, very well. We've been very surprised with the response from patients. As you can see in the results, what we expected to take much, much longer, has happened in the first quarter itself. In terms of the new departments that you asked about, we have the urgent care and emergency, which is the trauma centre. We started obstetrics and gynecology, essentially women's health, women and children, because we have pediatrics and neonatal care as well. So, those started towards the end of the quarter. These are the few new departments. But more importantly, for all our departments, we're available now in a more premium and a more convenient location. So, that helps as well.

3To the second part of your question about the revenue run rate being a base, you know, there will always be some fluctuations here and there, but I think that this is a good assumption to make, that this will be here in terms of a sustainable revenue. There will always be some quarters where things may fluctuate up and down, which is the case in Cayman, given the low volumes. But this is a good, good starting point. Yeah, we don't see it going below this significantly for a sustained period of time for any reason.

Prithvi: Got it. Moving on to the Cayman margins, again, you have done exceptionally well with respect to margins, again, coming back to 45%. Given that there is still scope for utilization or occupancies to further ramp up in Cayman, can we expect margins to slightly inch higher, or do you want to retain margins at these levels and focus on volumes?

Anesh Shetty: So, definitely beyond the point where we are, it doesn't make much sense to focus on improving margins at the expense of not tapping into a bigger revenue base. So, we're happy with where we are. It's a good place to be. The goal now would be revenue growth. And, it would be very hard to cross this level in terms of margins, and it wouldn't make sense, long term sense, quite frankly.

Prithvi: Moving on to the India business, Viren, this question is on the insurance and the clinic losses. I think for the last few quarters, the incremental margins that the hospitals are making in India, that's getting offset by the higher losses in clinic

and insurance business. I just wanted

to get a colour from you, how are we looking at this from a long term? I mean, are we at the peak losses, or do you think the losses can further extend going forward?

Sandhya J: Hi Prithvi, I will take this. For FY25, like you said, INR 65 crores is the loss which we have, cash burn that we have taken in Integrated care. This is a business that has... every time we are adding a new clinic or we are adding a new city, we will have initial costs that we will have to incur, and therefore, we will have a cash burn scenario. And as we complete a certain time frame, like for clinics, it's 18 to 24 months, we break even and then we move on to the next cohort. So given that we have a certain expansion plan in terms of the clinic portfolio in the current year, there will be some amount of growth that you will see in the losses. So, it won't stagnate at this level, the losses will grow. But on an overall basis, over a time frame of 3 to 4 years, we have a certain number with which we are working as an investment in this business, and we will be very careful that we are able to stay within that broad investment horizon we have set for ourselves.

Prithvi: Is it possible to share that number? I mean the investments with respect to clinics and insurance?

Sandhya J: Around 400 crores, Prithvi.

Prithvi: Both put together, right?

Sandhya J: Both put together, yes.

Prithvi: Okay. Thanks, that's all from my side.

Nishant Singh: Can we please have the next question? Anybody with any questions? Yes, Deekshant, please go ahead.

Deekshant: Hi, congratulations on good numbers here. So, the first question is really on, till the time that we are seeing our greenfield expansion starting place and kicking in, what kind of growth can we expect before that?

Sandhya J: Yeah, hi, Deekshant. We will sustain growth through throughput. Like you've seen in the past several years, we have not added any capacity, but we've been able to deliver a sustained growth momentum driven by our throughput initiatives. So, we will continue to do that without giving any forward guidance. I think we aspire to be the way we've been in the past few years.

Deekshant: Yes, ma'am. So last few years we have been growing around 10%. Are we going to grow higher than 10% till FY27 or are we going to grow at 10%?

Sandhya J: That would tantamount to a forward guidance, Deekshant, so we would refrain from that. But we are definitely aspiring to grow in line with market on an organic basis, and also in line with what we have done in the past.

Deekshant: Sure, ma'am. Second question is on our insurance business. I see it in the presentation giving some basic numbers, but can you share how our patients are reacting to our product portfolio right now? And what kind of throughput are we seeing in our hospitals right now?

Nishant Singh: I request Mr. Ravi to take this question.

Ravi Vishwanath: Sure. Thanks for the question. So yeah, I mean, I think customers have been responding very positively to what we've been doing with insurance. We've had one product earlier, and then in the last quarter we also launched another product, which is called Arya. And we are currently providing insurance in, or as of last quarter, we were providing insurance in Bangalore and in Mysore. We've got about 4,000 lives that have been covered by insurance so far. And as you may recall, we are taking a very differentiated strategy when it comes to insurance. So very high-quality underwriting and risk management, which is quite different from the way the market approaches this.

Our focus is on providing exceptional experience at time of claim, which we're able to do, one, because of the underwriting and also because we have a narrow network. The claims experience that we've had, we've got feedback from customers. You can check our website, there are videos there of customer feedback, which has been ve

ry, very positive. We do have

a direct distribution model. And so, we are building on that and our focus now... We've been in market for about six months and our focus now is on building our distribution going forward.

So very happy with the processes that we put in, the systems we put in, the early response from the customers. And our focus now is to build distribution and take the message of Aditi and Arya to more and more people.

Deekshant: Thank you, sir, for the explanation. So, at 4,000, I think we have reached enough of trial and error for ourselves to have a go-to-risk model route. The question really is, what are we waiting for before we push the accelerator?

Ravi Vishwanath: I think the things that we needed to put in place, we have put in place. And as I said, we're now in the process of building distribution capability and capacity and accelerate our growth.

Deekshant: So you're saying that we would start pushing the accelerator now?

Ravi Vishwanath: That's our aspiration. In this quarter, we have expanded to Kolkata. We will also be expanding to Raipur. So we'll be opening more markets as well. We're also building our direct distribution capabilities. We're building out our data-driven marketing capabilities. We're also looking at other opportunities and areas for growth. And so that's been our focus. As you know, getting the processes and systems right is absolutely critical. I think we've done that,

we put those things in place. So now our focus is on growth.

Deekshant: Sir, one last question on the insurance front. At what number of lives can we say that we are approaching breakeven?

Ravi Vishwanath: That really depends on a number of things. I think it'll be maybe a little bit early to answer that, primarily because our approach to market is so radically different from what everybody else is doing, right? So the way that our book will develop, I think is going to be very different from the way a normal insurance book develops, because of the underwriting that we're doing, because of the broader terms and conditions that we have in our policy compared to others, because of the network strategy that we have and the distribution strategy that we have. So I think it's a little bit early to kind of give a number on that. The point though is that, we are focused on building a book where the quality of risk management is very high, and there is superlative focus on the customer experience, especially at time of claim. I think those are two big pain points, one from the customer side and second from the industry side, that industry generally is looking at right now in terms of risk and customer experience. And those are the things that we are kind of totally focused on.

Dikshant: Got it, sir. Thank you so much. I'll join back in the queue.

Nishant Singh: Yes, Ridhi, please go ahead.

Ridhi: Hi. So, I had two questions. One is regarding bed capacity. If we see from FY23 till this financial year, the bed capacity has been decreasing in India. It decreased by around 398 beds in FY23, in FY24 by 112 beds, and this year also 160 beds. What is exactly the reason for that?

Nishant Singh: See, this quarter the reason is, we have changed our contract in the Jammu region, Katra region, so that's why the number of beds has come down by 370 odd beds. And for the other quarters, we have discussed that we did not continue few of our partnerships, Bellary and other locations before. But this quarter the reason is that the large number in overall reduction is coming from the Jammu beds.

Ridhi: Okay, so part of the reason even the hospital, when you say owned hospital, it has decreased by one hospital.

Sandhya J: Some reorganization we have done in terms of general ward capacity being upgraded to private and semi-private. We have been doing it over a period of time across our large hospitals. So, it's not a big number, but there is some amount of reduction that has c

ome

because of the reorganization also.

Ridhi: Okay, so this reorganization, restructuring will happen... like any particular year till when this restructuring is going to happen?

Sandhya J: It's an ongoing process, but I think it will be a very small impact on the overall numbers. And if at all, it will only give a positive benefit to the ARPP, as well as to the financial position. It will be a small impact.

Ridhi: Okay, I think that's about it. Thank you.

Nishant Singh: Rajit, can we please have your question?

Rajit: Yeah, good afternoon, sir and congratulations on a good set of results. Just following up from previous participant's question on insurance, do you have any number for the year as to on the number of cities that you plan to launch insurance in, or the number of lives to be covered for FY26?

Nishant Singh: Ravi, if you can please respond to that question.

Ravi Vishwanath: Sure, I mean, I'll answer in terms of the cities. As I said earlier, we are expanding to Kolkata, Raipur, and also Shimoga. That's our immediate plan for now. And, there are obviously a number of other markets that we're very interested in, where NH also has a strong presence, and we're exploring those markets as well.

In terms of a number while, of course, we have internal targets, we think it's a bit early to share those publicly at the moment. So, it's too soon to do that. But we are looking at expansion, certainly in the locations that I mentioned, and we're constantly looking at other markets where it would make sense and where the market dynamics are appropriate for us.

Rajit: All right. Thank you, sir. And the question is on an announcement recently regarding opening up of chemotherapy centers. Is it possible to share further details on the same? As in on the typical size of a center, what kind of revenues we're expecting? Something on those lines.

Viren Shetty: Yeah, just request the rest of the participants to keep their microphones on mute. Thank you.

So, the announcement was relating to an investment we've made along with the venture funded startup called 2070 Health. The idea is for us to create a series of retail chemotherapy centers across the country. So, we will be contributing half the capital, investors will be contributing the other half. Our idea is to create chemotherapy centers outside the hospital in retail locations in the areas of the city where there's great demand, by partnering with doctors and partnering with real estate people who provide the space. The size we're not yet finalized our first centers coming up in Gurgaon. This is about a 5,000 sq. ft. space. We're not yet sure if this is too big or too small. But based on the first model, we will try and see if we can come up with a scalable model around this.

Rajit: Right. Thank you, sir. And when will this first center be coming up?

Viren Shetty: By this week, we'll start seeing patients and there will be a soft launch. It will officially start in a big way in about a month's time.

8Rajit: Okay. Thank you, sir. Thanks a lot.

Nishant Singh: Thanks, Rajit. Deekshant and Ridhi, you have more questions to ask? You're still showing the raise hand.

Deekshant: So, sir, you have mentioned that we are looking at organic ways of growing. And without giving any forward guidance, would you just say that, what is the input that we are looking at? What are the growth drivers for us for this organic growth? And also, how are things progressing in the Mumbai facility right now?

Sandhya J: So, from an organic growth point of view, I guess it is the same that we have explained earlier, which is, improved throughput, higher order procedures, better payer mix, better bed mix, being able to continue to keep providing higher order care, and also integrated care is helping. As far as Mumbai is concerned, we are continuing to be in that near break-even kind of situation. We aren't losing money, but we aren't really a

dding financially. We've already explained this journey earlier. It is a long journey for us in terms of recovery, and we are working with the Trust in terms of levers that can enable us to expedite that. But those discussions are in various stages. And so, when we have greater detail to update, we will share with you.

Deekshant: Have we started our operations, we were just pediatric care in Mumbai, and we were waiting at serving also the adults. Have we started with those operations yet?

Sandhya J: No, we don't have approval for adult yet. And we are working with the Trust in terms of the same. That was the thing I was referring to as we are having some conversations. It will take some time. It is a work in progress.

Deekshant: Ma'am, last question is on Bangalore, Kolkata and the Southern peripheral still are a good percentage of our overall revenue. Of course, North is as well. But out of these three, where are we seeing the most amount of satisfied customer who are giving word of mouth? What kind of... where are we really concentrated right now where word of mouth is improving for our brand?

Sandhya J: If you look at our overall Google rating across the country, it is one of the highest, hovering between 4.8 and 4.9 across all our facilities. So, from a customer satisfaction point of view, both in terms of value for money as well as the quality of care, I think we are able to provide that impact to our customers across the country, not necessarily in the flagships. Of course, because of the high throughput in the flagships and the level of sophistication and complex work that happens, therefore, the word of mouth for higher order work that we do in the flagships is also more prominent.

For example, we do the largest number of robotic cardiac procedures in the country and it is done largely in our flagship location in Bangalore. We again do the largest number of pediatric bone marrow transplants in the country; a lot of it is performed from our flagship locations. So, in terms of word of mouth, we have a very high recall, both in terms of the quality of service we provide for basic care, but also in terms of the high-end capabilities that we have in our large units. Does that answer your question, Deekshant?

Deekshant: Yes, ma'am, it does. Just what kind of pricing differentiation are we offering with the higher quality of service that, of course, we have been offering, that's how we are getting such good reviews. But ma'am, what kind of pricing differential is really driving sustained growth for us? Are we higher than the market?

Nishant Singh: I am requesting Mr. Venkatesh to take up this question, please.

R. Venkatesh: Obviously, when we are looking at... if you have seen the performance across, we have been doing well, we have been growing reasonably well and more on our domestic throughputs, domestic volumes.

When it comes to pricing, pure price increase is very low single-digit numbers, just to cover up the inflations. But when it comes to our differentiation between a ward to a semi-private and private, what we have done is, because of the recent transformation programs which we have done in the major units i.e. the flagship units, we have tried to do more of private, semi-private beds from the general ward beds, and we've realized that the patients are actually preferring more of these private beds than general ward beds, which has actually been a contributor to our margins.

But when it comes to pricing as compared to market, we would be at least 5 to 10% lower. I won't be able to indicate exact numbers, but in terms of numbers, will be slightly lower than the market in terms of pricing. But we focus more on the service and the outcome and the quality of outcomes to improve on our end product and reputation in the market.

Deekshant: Do I have permission to ask one last question? Sir, while we are now going on this Capex spend, and of course we are go

ing phase-wise, of course, we will need the doctors, we will need the nurses, we will need the support staff. So, how do we expect that once we have hit

10 this sort of new openings, what kind of way can we integrate our staff overall, so that we are up and running on time and also providing the superior care at affordable prices?

Nishant Singh: I request Dr. Rupert to take this question, please.

Dr. Emmanuel Rupert: Yeah. So, our expansion is mainly in Bangalore cluster and the Kolkata cluster. And so, if we look into that, we already have a large base of clinical staff. As far as nursing is concerned, we do run two large nursing schools, and we constantly incorporate them once they finish the training into our own system. So, as the number of seats keep going up year-by-year, we will have a large volume of people whom we can deploy into our new centers, knowing our cultures and knowing the way of functioning and the clinical governance and other structures which we have put into place.

Similarly, we have the largest doctor training programs in the private sector in the country. We have almost 800 postgraduates in training across the network, predominantly in Bangalore and Kolkata cluster, and we have cross movements of these trained manpower across our entire structures. So, we are already well on our way to have a clinical manpower planning for all the centers right away. We will be identifying key staff, not only for the middle and the senior level, but also for the other support structures across all the hospitals that we are planning to start. So, we have a very clear plan in place, and we will be able to execute it to the best of our ability.

Deekshant: Thank you so much. I really appreciate your detailed answers.

Nishant Singh: Yeah, Rajit, you have any question? We'll please move to Pratik. Sorry. Yes, Rajit.

Rajit: Okay, sorry. I'll just go ahead with a follow up question regarding the chemotherapy centers.

In terms of services to be offered, will there be pure chemotherapy centers as of now, or do you plan to extend surgery or any other diagnostic services, etc as well?

Viren Shetty: For now, it will just be providing daycare chemotherapy in a retail setting. But going forward, if we find a good opportunity to get into the entire spectrum of onco services, onco surgery later and radiation therapy last. But for right now, the focus is on chemotherapy.

Rajit: Okay, thank you. Thanks.

Nishant Singh: We'll take one question from the chat. This is for Viren. The question is, Viren, can we say the success of Camana Bay in Cayman serves as an indicative benchmark for NH's footprint at Africa?

11 Viren Shetty: No, we're not looking at Africa at this point. Camana Bay is a development in the Cayman Islands. The fundamentals are very different. Cayman Islands is a first world country, serving a primarily Western clientele. We did try to set up a joint venture in Africa with IFC and Abraaj Capital a long time back, but we were not able to conclude that successfully. The operating challenges of working in other developing countries are very unique, and even different parts of India offer different challenges.

For our overseas expansion, we would focus a lot more on Caribbean-like geographies, which are smaller overseas territories with very stable currencies, developed economies, very large insurance penetration, a stable rule of law, and an ability for us to run something that operates with a great amount of benefit for our low-cost operating model.

Nishant Singh: Pratik, please go ahead with the question.

Pratik: Sir, can you tell me just the reason behind the increase in working capital days, which in March '24, it was in negative, -9. And in March 2025, it is 66 days. So can you just tell me the reason behind that?

Sandhya J: Yeah, Pratik, it wasn't negative in March '24, but yes, the working capital days has gone up for us. The reason being that we have

a certain exposure to government payors, as you are aware, 19 to 20% of our revenue comes from government payors. And in the March quarter, typically, we get settlement from government payers. But there was a slowness in the payout, mainly payers like ECHS and RGHS, and that kind of impacted the DSO for the quarter. And hence, we ended up at a slightly higher number than what we usually finish at.

Pratik: Yeah, ma'am, any future guideline for FY26, for the top line?

Sandhya J: From a payor mix point of view?

Pratik: No, no, an overall business point of view, the sales side, revenue side.

Sandhya J: Okay, we answered that question earlier, given that we are not having any capacity addition.

Pratik: I wasn't there in the meeting then.

Sandhya J: Yes, Pratik, given that we are not having any capacity addition coming up, so our growth will be organic. And we aspire to be able to deliver the kind of growth we've delivered in the past, without giving any forward-looking statement.

12 Pratik: Ma'am, any guidance on Capex?

Sandhya J: Yes, we have put out our Capex number as part of our Investor Presentation that we have uploaded. For the FY26, there are two parts to the Capex, one is the regular capex that we incur, which is about INR 300 crores, which is on replacement, maintenance and new capacities that we are creating inside our existing facility. And of course, we have also indicated investment in greenfield and brownfield capacity. So, that is approximately INR 450 crores that we will spend in these new capacities. That also depends on the progress of the

construction. So, there will be little variability to that. But the money we will spend; it may move a quarter or so.

Pratik: So ma'am, the fund which you are getting for then capex, it's all coming from debt or we are getting also from the cash flow of our own... from the market... from the capital? Or, we are getting from all from debt?

Sandhya J: Yes, so some part of the capex is funded by own capital because banks have a certain own contribution threshold. Beyond that, we are raising debt, and we are funding capex through debt only.

Pratik: So, in future, we are still having a mindset of getting more debt, or we are into the phase that we are reducing the debt in the coming future?

Sandhya J: At the moment, our debt to equity is 0.15. So we have a huge headspace in terms of our ability to borrow. So, we will leverage that headspace at least as of now for the growth.

Pratik: Okay, thank you, ma'am.

Nishant Singh: There's one question on the chat, which is for Anesh. Are you looking at any other overseas expansion now since your model is working in Cayman?

Anesh Shetty: We have been looking at several markets for a while. In fact, we made an initial small investment in the Bahamas a few quarters ago. Nothing significant, nothing to report at this moment, but we have been looking for some time.

Nishant Singh: Pratik, if you are done with the questions, can we move to Vinay? Vinay, please go ahead.

Vinay: Yeah, thank you. Just a couple of things. In your ICU occupied bed days, it has moved from 346,000 in FY23 to 368,000 to 372,000 in FY25. Can I know what is the occupancy? What are the total number of available bed days?

13Sandhya J: Our occupancy normally ranges between 60 to 65%. You're asking for ICU occupancy?

Vinay: Yes.

Viren Shetty: No ICU bed days. ICU occupancy is not a useful measure.

Vinay: The reason I'm asking you this is if I look at your patient footfalls in out-patients, rather in-patients, it has moved from 229 to 216 to 220 in three years. So it's hardly any great movement in terms of number of thousands. Your average length of stay is also stagnating at 4.5. So, the growth is coming primarily from what do you call, new methods or whatever, whichever new ideas that you're bringing in. How do you increase your footfalls to drive those sales further? Otherwise, it will be stagnating

at this 10% growth rate, right? Is there any problem in getting higher footfalls?

Viren Shetty: So, there are capacity constraints. A lot of the hospitals are fairly mature. And we are also going through a lot of the retrofitting of the existing infrastructure to move out certain beds and add in different quality of beds and adding new ICUs. So, you are right, until we add new greenfield capacity, you are not seeing any dramatic jumps in the volumes. The existing hospitals can take more patients. It's that we are prioritizing certain payor classes that allow the hospital to have a healthy sort of revenue growth without overburdening the system with receivable days and working capital issues. So, what we are doing is taking a judicious mix between some amount of growth, some amount of payer changes, as well as renovating the existing hospital to reduce the beds and add in private, semi-private rooms, ICUs and OTs.

Vinay: Yeah, I just wanted to check on, how do I read this 'ICU occupied bed days' number? I mean, how does it relate?

Dr. Emmanuel Rupert: See, the bed days is just a calculation of the occupancy of that particular number of patients they spend in the ICUs. And because we do... you know, as science is progressing, the lesser you keep the patients in the ICUs, the better it is from capturing... I mean, from the patient acquiring some hospital-born infections and few other things. So, we constantly keep working on that as well. So, it may not be exactly an ideal way for you to look at it, but we can connect with you outside this call to explain if you need further explanation.

Vinay: Thank you very much. Thanks a lot.

Nishant Singh: Yeah, Abhishek, can we have a question, please?

14Abhishek: Yeah, thank you for the opportunity and congratulations on another quarter of excellent execution. My question is around the capex guidance on the India business. Now, for the next three years, we are going to be adding a significant amount of capacity, and the total project cost that you have given to us is roughly around INR 2,800 to 3,000 crores. For FY26, the projected spend is around INR 425 crores. And if we look at in conjunction with the fact that even last year, our actual expenditure was INR 400 crore lesser than what we had initially planned, this capex number for FY26 seems to be a bit on the lower side. So just wanted to know what the reasons for this could be, and if those capex spends are going to be back-ended towards FY27 and FY28.

Nishant Singh: So, these capex numbers, it's very difficult to match with exact projections because these projects, they take a lot of time. And even the selection of projects is also subject to our diligence process, which also takes a lot of time. So, because of this, some of the numbers may get pushed to the next year. So, whatever we had indicated previously for FY25, we may be a bit short, but all of that may come up in this FY26.

So, the capex number which you see here, is the actual cost which will incur on the committed projects. So, these are just the numbers for the ones which are already in the pipeline and which have been committed and signed up. So, these numbers may vary depending on that. If we get some more opportunities, which we think is going to add value to us, we will take up those projects. And we are adequately capitalized to take up anything which interests us in the future.

So, there'll be a bit of variance. There may be a bit of a delay in terms of closing a project because it involves a lot of diligence processes. But we will probably stick to this number, especially for the committed ones. And whatever new comes, we adequately have enough cash in the bank balance to take up new projects as and when they comes.

Abhishek: Understood. Thanks for the clarification. Just if I could ask one more follow up to that. So, this year we generated around INR 1,000 crores from our operations. So, if there's any

instruction delay or there are delays to this project, that gives more time for our existing business to generate more cash. So, do you think in that sense, our target net debt to equity or net debt to EBITDA levels, the target levels might come a bit down if our existing business gets more time to generate more cash for these projects?

Viren Shetty: It will, unless we find something else to build.

Abhishek: Sure. Sure. Makes sense. Thank you and all the best.

15Nishant Singh: Thanks, Abhishek. Rishit, can we have your question, please?

Rishit: Yeah, hi. Congratulations for the great set. Two questions of mine. First on the capex, again, continuing what the participant asked. This year, we have a guidance of around INR 400 to 450 crores, excluding the replacement. Out of which, how much are we planning to raise the debt? Because currently on the long-term side, we are already at a INR 2,000 crore mark on a gross level. So how much do we plan to scale it up in this year?

Nishant Singh: See, the gross debt is obviously above INR 2,000 crores. But if you look at the cash, we also have a very large cash corpus sitting in the balance sheet of around INR 1,600 crores. So, our net debt levels compared to EBITDA is still very small.

So, your other question of how much capex borrowing will take from the planned capex for next year? It should be around 60% of the overall numbers.

Rishit: 60% includes the replacement cost as well? So total?

Nishant Singh: Yeah. So, see, what we actually do is that for the project finance for the new projects, we are committed in terms of the project finance. So that would be 80%. But for the regular maintenance capex, we do maybe around 50%. So, the net average would be around 60 to 65% of the entire spend.

Rishit: Understood. And sir, we have a lot of cash parked in our investment as well as in current books. So, on a consistent basis, we are getting a yield of around INR 24 to 25 crores of other income. If possible, can you give a split of the other income quarterly?

Viren Shetty: Just give us a second. Yeah, we will just....

Sandhya J: Mostly our other income comes from the interest that we earn on the cash that we are holding. In addition to that, some accounting entries get passed by auditors, certain items get reclassified as 'other income'. But broadly, it is coming from the interest line item.

Rishit: Okay, just last question on the taxability.

Sandhya J: Pardon me, Rishit, can you repeat your question?

Rishit: Yeah, the last question on the taxability part. So, Cayman giving us the advantage and now scaling up of Cayman, can we assume this 16% to be a sustainable tax rate for next two years or we might go higher with the new India business coming up?

16Sandhya J: Yes, for FY26, yes, 15 to 16% looks like a reasonable assumption to me on tax. FY27 is a little difficult to commit at the moment. We'll have to see how the business context evolves at that time. But you can work in this range.

Rishit: Okay, understood, ma'am. Thank you and all the best.

Nishant Singh: Thanks. Gagan, can we have your question, please?

Gagan: Yes, good afternoon. So, the first question is for the tax rate effective for the 4th Quarter. Year on year, there is a substantial increase in the tax rate for Q4. Any explanations on that?

Sandhya J: Yeah, Gagan, actually, we received dividend from Cayman. The dividend gets a tax shield when it gets paid out to the Indian shareholders, for which we need the approval of the shareholders. So that will come only in Q2 of FY26. So therefore, we had that timing difference where we had to do the deferred tax hit for the dividend we have received, which we will reverse in Q2.

Gagan: Okay. And if I look at your India business EBITDA margins, Q3 to Q4 is a good jump. Can you explain what went into that improvement? And also, is the Q4 number sustainable?

Sandhya J: So for maybe what went into the improvement of the

EBITDA margins, I will request

Venkatesh to comment.

R. Venkatesh: You are talking about the current quarter vis-a-vis the previous quarter?

Gagan: Yeah, the improvement in India business EBITDA margins from 3rd Quarter to 4th Quarter.

R. Venkatesh: So, one is, if you look at the quarter-on-quarter, we have had improvements on the manpower costs by around 2% and also improvement in the other expenses by 1% on an overall. But more importantly, over and above the cost discipline, if you also look at the realization, there are increased realizations if you look from the previous quarter to the current quarter, which I've already said earlier. These come from patients preferring higher bed categories in semi-private and private, which also results in higher realization on a similar cost structure, which as a combination with cost discipline, has helped us for Q4 as compared to Q3.

Gagan: My confusion is that, on the one hand, you were pointing out that you're basically upgrading general wards to semi-private and private wards, and which obviously means that your average realization will push up because of that. But then at the same time, it would favorably impact your working capital. But your working capital is deteriorated. So how does one reconcile these two?

Sandhya J: So, the working capital deterioration is a temporary phenomenon because of certain payors who excessively delayed the payouts. Also, if you see from a payor mix point of view, we are making more conscious choices even within the scheme business, to be able to pick up volumes which are more accretive in terms of realizations. So that is the reflection of that you are able to see.

So overall percentage of schemes have not significantly changed. They have hovered between the 19 to 20% range, but we have been able to improve the overall ARPP because of that. So, working capital impact on schemes will be there. It depends on the cash flow for these payers and the ups and downs will be there.

The positive part of this is that the money will come. The only impact is that sometimes it gets delayed.

R. Venkatesh: Yeah, and just add one more to this, which is if you look at the schemes, of course, it has gone down a bit from 19.5% to 19%. And that has primarily happened only because of a slight increase there in northern region and some part of the eastern peripherals. But when it comes to flagships, because of the transformation programs, there is not much of intake of scheme at all.

And as Sandhya said, I am just adding more to it. Even though the numbers have gone a bit high in the northern and certain part of the eastern peripherals, our choice in schemes is reflecting more on effective realization and good payor capabilities in the scheme. So, even though there was a little bit of an increase in the working capital, we are sure that this will remain, but we'll be in a position to regulate this in the quarters to come.

Gagan: I mean, if I simply try to reason this out, given that it is a continuous process of reducing general wards and increasing more share of the semi-private and private wards, there is a natural increase in ASP. Three hospitals, I think Mumbai, Gurugram are on an improving trajectory. Perhaps you could elaborate further on that. It would seem that the India margin should further improve. Is that a reasonable surmise or am I wrong there?

Viren Shetty: No, no, you're right. And, it should.

Gagan: Yeah. Okay. And if I look further into the breakdown of your India sales growth, while overall reported is what, 10-11. But if I knock out the international patients, I think your presentation indicates a 14-15% growth for the India business on a standalone basis, without the international patients. And the international patient flow's revenue contribution in Q4 sort of bottomed out to INR 40 crores, if I got it correctly. Are we therefore, looking at a 15-16% India growth ex of interna

tional patient flow to maintain, going ahead? And for the international, are we looking at stabilizing at Q4 level, or do you see further deterioration there?

Viren Shetty: We hit 14% India growth, but then Venkatesh will have to just move into the hospital. We never get to see his wife and children again.

Gagan: No, I am simply taking a cue from your statement, that you can maintain the past growth rates and therefore, the question.

Sandhya J: Gagan, we don't want to give a forward-looking guidance, but we aspire to maintain our past growth rates. So, we'll try to do our best. As far as international is concerned, it has not bottomed out yet. I think we will go down to zero eventually. It's just a matter of time.

Gagan: Okay. And does that have any bearing on your margins?

Sandhya J: It has a slightly positive impact on the margins because it is a low realization book for us. So, some of the improvement you have seen in the ARPP, that's also been because of the reduction in the international revenue and increase in the domestic revenue.

Gagan: Right. And if I look at the India OP and IP volume, very marginal growth, and yet your revenue growth is double digit, right? Which basically means that almost entirely it's coming from an increase in realization per patient or per bed, whichever way you look at it. Is it possible to sustain this kind of realization growth further? Because, I am presuming you have a capacity constraint, and therefore, OP and IP volumes may not really substantially move.

Sandhya J: Actually, just one second.

Dr. Emmanuel Rupert: Yeah. So constantly, the idea is to keep coming up with new procedures, like we have mentioned in some of the data we gave you. A lot of our procedures are moving to minimal access cardiac surgery and robotic cardiac surgery, wherever it's indicated. And we have seen that a lot of people are taking that up. This is just an example of some of the things which we are moving towards - more and more of minimal invasive and shorter and enhanced recovery program for many of the procedures and things like that. So, by doing these things, we feel that the complexities of the work will constantly keep moving up the chain and we will be able to sustain what we have been able to do.

19Viren Shetty: It's harder to do. It takes longer than simply just raising the prices year on year. But that's the path that we choose.

Sandhya J: One more point I want to add, Gagan, here is that we have had a huge volume dip in Bangladesh, and we have compensated it with domestic volumes. But the domestic realization is higher than the Bangladesh realization. So that mixed effect is playing out.

Gagan: How much of an impact would that have had on the average realization per patient?

Sandhya J: We have not quantified it at that level, Gagan. But you have the revenue dip numbers available with you and you can take an assumption on a lower realization, closer to schemes kind of realization. So, you will be able to calculate.

Gagan: Okay. I have a few more questions. Should I get back in the queue or would it be okay if I go ahead with it?

Viren Shetty: Yeah, finish it off.

Gagan: Okay. And for the year closing FY25, your discharges in Bangalore and eastern periphery have come down, actually. Any particular reason there?

R. Venkatesh: For eastern peripheries, it's not going to be a major number. Certain flows in this region is impacted due to local dynamics and local development. So, this is going to be kind of a very temporary phase, which will recover soon. And of course, payor mix optimization is something which we are always looking at. That's also a contributor. But then this is a temporary phenomenon in eastern periphery. It gets recovered.

When it comes to our flagships, and majorly in Bangalore and Calcutta, we have seen that more than 60% dip is there in international patient volume. But in spite of that, we've been able to sustain

a growth of 11%, only because the domestic has gone up by 15% to 16%. You can see the numbers from the last year, Q4, to what we have done in this year, Q3. There have been an increase in numbers. But on a broader scale, we are not only trying to drive volumes, as we discussed, but also focused on controlling the realization, which is apparent in our strong domestic revenue growth.

And when it comes to schemes, we are also targeting schemes, as I'm saying, which are more selective in terms of higher realization and better payment history. So in spite of this dip, because of a good performance on the domestic front, we have been able to sustain to a 20% growth of 11% on the whole when it comes to a quarter-on-quarter or on a year-on-year basis.

Gagan: Okay. One question on Cayman. I mean, you talked of margins in Cayman have sort of reached a peak level. Is it possible to understand, one, the occupancy levels in your new hospital?

Because in principle, I would have thought there would be a ramp up in occupancies, which would create operating leverage of some sort. And in the past, when you were on the verge of commissioning the hospital, you indicated that it might actually divert some patient flow from your existing Cayman facility to the new one. So, has that been the case, or has the response been strong enough for the existing facility also to sort of hold on to its business?

Anesh Shetty: Yeah, Gagan, we don't run the two hospitals as a separate units. So, it's the same doctors, it's the same services with a few exceptions. It's just a different campus of the same hospital. So, for example, the same doctor would see an outpatient on Monday in one facility, do the procedure on Tuesday in the old hospital, and back on Wednesday for something else, you know. So, it's not really... you can't think of it as moving volume of business from one hospital to another. It's just one unit with two campuses or two buildings.

To the second part of your question around operating leverage. It is a new building, a new infrastructure. So essentially, the operating leverage we were getting in the old hospital, now that we have a new building, it essentially gets reset pretty much to zero, because we have to cover all our fixed costs of the new hospital, which we have done now, and been able to restore margins to where they are. So, the reason... if I can guess, your question was as to why we didn't go higher than where we were before? It's because we have to cover an entire new building's fixed costs, which we've done now. I hope that answers the question. Yeah, please go ahead.

Gagan: Just a clarification there. Are you therefore indicating that the occupancy levels in the new hospital are optimized?

Anesh Shetty: What do you mean by optimized? Sorry.

Gagan: I'm basically saying that, if the sustainable occupancy, whatever be the number, you have

already attained it?

Anesh Shetty: No, between the old hospital and the new hospital, we believe we have room to grow and we have capacity to grow into it as well. So, we're not capacity constrained and there is an opportunity to grow into it. It will take some time. Our first big leap happened when we opened the hospital, because it's a new campus, a new pool of patients and new services. And going forward, we will gradually cover up a few gaps that we have.

Gagan: And, while you can plan the itinerary of a doctor for a certain surgical procedure and for his OPD, but on a regular OPD volume basis, you will need to have certain set of doctors dedicatedly operating in each of these hospitals, right? You can't have all your doctors circulating between the two hospitals, which are quite a distance apart, from what I understand.

Anesh Shetty: That is correct. So, all appointment, I mean, all OPD consultations are by appointment unless it's a semi-emergency, which is called urgent care or actual emergency. So those are walk-in, but everything

else is by appointment. So, every doctor has their schedule decided as to which day they'll be operating out of which campus and where their patients are going to be seeing them. And for most specialties where we have more than one doctor, we will be covering both facilities.

Gagan: Right. And on funding of the capex, you generated INR 1,000 crores of operating cash. You have cash on the books. You will perhaps be generating another INR 1,000 crores, if not more next year. While I understand part of it is in Cayman and you can't repatriate it, but there's still a substantial portion coming from India. In principle, it would seem that INR 700 - 750 crores of capital requirements for your capex can, for substantial degree, be funded by your own cash flows. If at all, there's a timing mismatch, you could use perhaps recourse to short term debt and repay it as soon as the cash flows in. Is that not a reasonable understanding of this? I mean, why would you require 60% or 70% of your capex to be debt funded?

Nishant Singh: So, you're right in one sense that we are generating a lot of cash as well. See, but our cash generation is also a bit staggered. As Sandhya mentioned in one of the answers before that, we get a lot of cash in the month of March, because the receivables which we get from the government payors is not very uniform.

Gagan: Yeah. So, March is the start of the new year. You will get your cash now, right?

Nishant Singh: Yeah. So that's what we are seeing now. See, it is a temporary phenomenon. We will not have this kind of cash at all points in the year, and it's more like a war chest. We are continuing in the expansion mode, so we want to retain some cash for any sizeable, good opportunity which we see during the year. But we understand, that if we don't find any suitable purpose for this cash, we would like to use it for some other purposes, like maybe even look at repayment of some other, some very high cost interest loans. Yeah. But for the time being, as we continue to expand, we would like to retain this cash, so that whenever good opportunity comes, we're able to seize it up.

Sandhya J: But just to summarize, Gagan, I think we will be prudent with the negative leverage. We will not put too much negative leverage on the books, unless where we have definitive reasons on why we want to carry that.

Gagan: Okay. So, you are saying 60% is perhaps the max, if at all required. I mean, you may not necessarily even require it. Is that correct?

Nishant Singh: Project finance loans are for a bit long term. So, we can't take decisions of not taking those loans basis our temporary cash distribution. Because these are going to be 15-25 years loan, which we cannot say that we will not take just because we have got a very large cash corpus now. So those loans, we will continue with that 80% debt. But the other loans, as Sandhya mentioned, that we will always calibrate in terms of what is better.

Gagan: Okay. So final one from my side. A few months ago, there were articles in the media that you are interested in Spire Health in UK. I don't know. I haven't followed it thereafter. But any comments from you on that side? Do you have the appetite for doing deals in the European market or the British market?

Viren Shetty: Anesh, would you address the disclosure that we made?

Anesh Shetty: Yeah. So, Gagan, as soon as there was that information in the media, we issued a clarification to the regulators in both markets that we had no intention of pursuing or making any offer or investment in Spire Healthcare. So that was the formal clarification we did, I think the next day itself.

Gagan: Okay, thanks. Thanks for taking my questions. I will get back in the queue. Thank you.

Nishant Singh: Thanks, Gagan for all your questions. Can we move to Ravi, please?

Ravi: My first question, although I know that the guidance are like not given in the call, but still like if I talk abo

ut a 3 years perspective, then are we expecting a 10-15% kind of sales and profit growth over the next 3 years every year?

Sandhya J: Ravi, we have answered this question actually. We are aspiring to improve profitability every

year. We are aspiring to grow like we've grown so far, and we do believe that we have put in the right measures in place. But we can't really give a forward guidance.

23Viren Shetty: But to the latter end of the three years, it's when the hospitals will start coming online, at which point then these become comfortable targets to achieve.

Ravi Okay, and the FY25 numbers, like those will be the base numbers. We are not having any risks to those numbers, right, on top line and bottom line that we may see any slowdown kind of thing? Are we expecting any negative on prudent side, or it will be positive only?

Viren Shetty: A more slowdown from Bangladesh, whatever little revenue we've been getting there may drop even further. This we had anticipated. Above and beyond that, if you're asking about macro slowdown, those are very hard to predict, and healthcare is one of those things that are really not discretionary. It may shift from quarter-to-quarter. Say a festival quarter, you may postpone something but get it done in the quarter after that. But the long run trend, we don't anticipate any sort of slowdown in the growth.

Ravi: Okay, second part, I want to know about the current utilization percentage. For the company as a whole console level, how much utilization we are at, considering FY25 numbers?

Sandhya J: Venkatesh, you want to take that question?

R. Venkatesh: No, I mean, when we talk about the group as a whole, when it comes to utilization, we are working more on throughputs. So, when it comes to effective utilization, we keep striving hard year-on-year to improve on throughputs. We work mostly around the daycare, improving the daycare numbers. Even a lot of these robotic surgeries, cardiac surgeries, where you have these morning-evening discharges, admission and discharges, the throughputs keep increasing, your utilizations are also effectively done. Wherever there are bottlenecks in terms of utilization of Cath lab or operation theaters, we try and improve on those bottlenecks to improve further utilization.

So, though we hover around... our reasonable occupancy between 60 to 65%, those are the numbers within which we do it. But those are just numbers. But since we are mostly a high throughput center, we focus more on efficiencies, and that's how we've been growing in the last 2-3 years. And also, we will be working towards such growth, even in the next 3 years, to keep aspiring for those numbers, which Sandhya indicated, till our greenfield capacities keep coming up. So that's how we will focus on improving our utilization and working towards this.

Ravi: Okay, throughput and efficiency part I understood. So, we can safely assume that another 20 to 25% capacity can help us to increase the revenue over the next 2-3 years, apart from the 24expansion we are having. Like, is my understanding correct since you mentioned about 60-65%.

Viren Shetty: So that's been the case for nearly the past 10 years. The total occupancy number never goes down, because we just are able to do much more with the space, discharge people faster and move people away from spending the night here, to getting discharged in the morning. So it's not a hotel business where it means that I have, in 100 bed hospital 40 beds empty. Those beds are used, it's just that it cannot stay occupied overnight. But it means that we do have capacity, we can keep further increasing the number of people who come in and out of the hospital. But that will show up in different numbers, not just the occupancy.

Ravi: Okay, understood. One more thing, because I invest a decent amount in the healthcare sector as a whole so when I read about other hospital institutions, I find that their expansi

on is

relatively higher side on number of beds compared to Narayana. But I feel that the business which in which Narayana is catering, that is a very high growing business in India. So is my understanding correct, or is it is this a misconception that the growth number which we are targeting in the Investor Presentation is relatively slower than what it should be considering the growth opportunities in India?

Viren Shetty: So, you're right in that if you consider the overall India growth opportunities, yes, ours is a very conservative company. We are not expanding to fully capture the entire India market, nor is the pace of our expansion in India, matching up to what the peer set is investing.

Now, why that is, is for many reasons. Chief among them is that it really... the return on capital for any new capacity, greenfield, brownfield, whatever you want to call it, is quite poor; has been for a very long time. Ours is a company that really focuses a lot on the return on capital and return on equity. So, our expansion is a lot more measured and focused on the cities where we have the strongest presence and where we're greater able to justify investments we make.

Furthermore, our investments are not just in hospital beds; we're investing in the insurance, investing in the clinics, we invest a lot in technology as well as overseas. We are trying to build a much stronger balance sheet, and a company that's able to perform similar to peers at a realization that is half of the peer set. So it means we can't do the same things.

Ravi: Okay. And regarding the long-term vision, since we primarily invest for very long term, like 10-20 years. Just to mention that I'm invested in Apollo Hospital since last 10-15 years. So can we expect that maybe in another 10 years, Narayana will have self-owned hospitals nearly to 2540 or 50 from today around 20. Is that assumption correct, keeping the long-term vision in

mind?

Viren Shetty: 40 hospitals in 10 years seems reasonable. Now, if we have as much of the footprint as Apollo, that's hard to tell because Apollo is also not going to sit still.

Ravi: Okay, I will get back into the queue. And lastly, I want to thank you as well, to the fantastic management. We have been invested since last 2-3 years and the growth and returns are very fabulous. So, thank you so much for all the hard work and delivering the shareholders' growth value.

Nishant Singh: Thank you. Ajay, can we have your question please?

Ajay: Yes. So basically, my question is on revenue mix. So, as I see, cardiac segment share as a percentage of revenue is coming down, while segments like oncology and orthopedics are growing significantly. So, what kind of margin these segments offer? And can we expect the same kind of growth in the segments which are growing faster as the compared to the revenue? So, what does this impact in the overall revenue it has?

Viren Shetty: Sorry, which department you're saying is the impact on the revenue?

Ajay: Onco and orthopedics.

Dr. Emmanuel Rupert: Yeah, so I mean, we still have a very large share of the cardiac sciences giving to the revenue. Still, around 30-34% of the overall revenue comes from cardiac sciences. Some of this marginal dip which has come is only because of the Bangladesh patients in the Health City hospital, which has got the work coming down. But we've seen a reasonable upward trend in the last few months. And, we have been focusing on orthopedics and the other specialties, and especially oncology we have been doing a lot of good work in the last few years, and that is beginning to show. And I'm sure going forward, we would like to have a healthy mix of all centers of excellence and subspecialties so that we have a much more meaningful growth in all specialties.

Viren Shetty: The specialty mix is more reflective of the demographics. Since the incidence of oncology has gone up, so hospitals have naturally had to add o

ncology as a department. So, the fact that it is growing, is a reflection of the sheer amount of cancer that exists in society. Cardiac will still be a very large contributor, but orthopedics and oncology will keep increasing as well.

26Ajay: Okay, so I can consider that what the current pace of the growth is, can sustain in the future, right? Around 20-25% is what oncology is growing and 20% around is what orthopedics is growing, right?

Viren Shetty: No, it may not be a stable state. These are temporary things because of our addition of orthopedics as one single unit in the Health City and oncology. We made some investments in Shimoga, a few other cities. I would say, just take the 3-year run rate and look at where the trend is, and I think that will be easier to project. But I don't see the cardiac business ever going below 25-30%.

Ajay: Yes, you have stated in the past con call, correct.

Nishant Singh: Yeah, Deekshant.

Deekshant: Yeah, thank you so much for taking my question. Sir basically, I want to understand that, we have been working also on Cayman for some time now, and I'm sure that the management has discovered a 'secret sauce'. I am sorry if I am not able to use the right words here, but it seems like the average revenue per user is much better, population is much better than Cayman's, and going ahead with an international expansion would make sense. But what is it that is our secret that is helping us be better than even the people who are currently serving the market in Cayman, or maybe in other expansionary geographies? What is our edge over the others that is helping us to be more competitive in the market?

Anesh Shetty: Viren, do you want to take this?

Viren Shetty: If we knew what the secret was, we'd apply it everywhere and we would. I would say that it's just been a very long slog. So, we spent 10 years in Cayman with the wrong business model, which is catering for medical tourism for the US. And I think a lot of it came from experience and the humility to understand what we are good at and what we are not. And so, by refocusing on building a very strong relationship with the patients and reflecting more what the on-ground domestic customer demand looks like, rather than wishing what it could be, I think is what made us successful.

Anesh Shetty: It's a very detailed focus around costs and controlling our administrative costs, especially manpower costs. A lot of it is made possible by the investments we have made over the years in our software platform. So, if we had to identify a few key differentiators between us and the rest, it's our fantastic technology platform that really allows us to perform the same tasks with much fewer resources and much more efficiently.

27Deekshant: See, why I ask this question is that, it takes us to know what not to do to understand what to do. And since you have been in the market for some time, you know what does not work very well and what is starting to work. This gives us, let's say, progression towards a recipe of success, which can be further replicated in other geographies. Tier-1 sort of a place where people have higher spend is obviously one of them, but the way that we are taking care of our people, our staff, how important is that playing a role if we are not in the home country?

Anesh Shetty: It's always going to be important because we have to convince people to relocate and to move to a very different geography. They obviously get paid better, but there are the trade-offs as well. That's always going to be difficult.

Having said that, we also hire a significant number of people locally and experts from other countries as well. It's a very global workforce, and it's as important as in any other place when we're trying to convince people to move to a different country.

Deekshant: Anesh, just a last question here. See, the software thing is really giving dividends, as you have said in the last

couple of con calls. And what is it that we are looking out for? If you could paint us a word picture, "these are the sort of things if I look at, I'll take the opportunity in an international market". What are those things that you're looking for that will enable you to take that opportunity?

Anesh Shetty: Sure. I think Viren touched upon that at the beginning of the call, but I think it has to be a stable market with stable rule of law, an established insurance penetration and a mechanism to pay. It has to have a high purchasing power, essentially high GDP per capita. We are not looking to go into a frontier country. And there has to be an existing track record of successfully managed private healthcare assets. Outside India and outside Cayman, we are not trying to be very innovative over here. We are just trying to find a place where there is an established track record, and maybe we can do it better than the others.

Deekshant: The management has said in the past that we are also giving our software product to other hospitals in order to test it out; maybe to make it better. Are we sort of thinking of monetizing this, or is this still a work in progress and we want to improve on it right now?

Anesh Shetty: Maybe we can... if you could get back in the queue, Nishant, I don't know if that's how you want to manage it, so that we can go to the other question.

Nishant Singh: We're running out of time here.

Dikshan: No problem. I'm so sorry for taking so much of your time. Thank you so much.

28Nishant Singh: So, Sahil, I think you've already put questions on the chat. Do you have any more questions, or we should take the chat questions first?

Sahil: It's the same questions.

Nishant Singh: Yeah. So, we'll just quickly go to the chat questions. What would you do differently if NH were a private company again with less investor pressure? Are you open to new land parcels if you get, which is not a part of the current capex plan?

Viren Shetty: Yeah. So, there's also a couple more questions on chat about why we're doing organic and land parcels and so on. We are doing a mix of both. It's that there are parts of Bangalore where we want to be present, but there are no inorganic opportunities available for us. So, we have to make our own. And thus, in HSR and in Bannerghatta, we have to buy land. Whereas other places like Banashankari and Chamaraipet, where we are able to tie up with developers, then we went to the inorganic route. Being private or public really does not make that much of a difference because most of the money the company raises is debt funded. So, in the end, we just get to have more people to give us feedback on the business that we're doing, and at some point, participate in the success.

Nishant Singh: Yeah. There is a question from Sahil again on that. I think, tech we have already covered. So, you asked about this, there is another listed company, an insurer, that is aiming to build a hospital chain effectively. They are aiming to do what NH is already doing. Do we see this as a threat or a market validation?

Viren Shetty: Yeah, not threat. We don't have a significant presence in NCR. Proof of concept and validation, we would say it remains to be seen. And we haven't validated for ourselves yet, that running insurance, hospital, clinic all under one entity serves the patient need. In most parts of the world, these operate independently. In very few parts of the world there are companies like Bupa, Kaiser, Hapida that run all of them, but even they are not the largest providers in their home markets. We just want to be different. We always want to keep trying to provide much more value to the customer, and we thought that being an insurer as well, will allow customers a completely seamless experience. But our hospitals will cater to all payer classes, and the insurance will allow customers who buy the insurance, to have some, there's advantages of course

coming to a Narayana network, but you can get treated any way you wish.

Nishant Singh: Yeah, there's a question from Sevant on the chat. Why are we giving dividend and raising debt at the same time?

29Viren Shetty: So, the dividend is coming from Cayman. So, our dividend policy is linked entirely around being able to monetize the Cayman cash reserve and do it in a more tax efficient manner.

Raising debt is for expansion.

Nishant Singh: Question on expected interest cost going forward and interest rate.

So, this I'll take. You can assume my interest cost of around 8%, which is actually coming down because of the movements in the T-bill on our overall borrowing for India and for the Cayman,

you can take a rate of around 6 to 6.25%.

Viren Shetty: Okay, the last question was while we are not successful in medical tourism, are we looking to reconsider it because of the sheer size of the American market?

No, we will never make that mistake again. The American market is huge and overwhelming.

The only way you can get American patients is to run hospitals in the US, which we're not looking at right now. There are other overseas geographies we're looking at, and the Cayman market... the Caribbean market is much more attractive for us. Some US medical tourism does come, but that is incidental to the overall business. There's a much larger opportunity in the Caribbean.

Nishant Singh: There's a question, despite having a large cash reserve we are using debt. But that we've already covered as a response to Gagan's question.

So, I think with this we are done with Sahil's and Ravi's questions. Am I right?

Ravi: I have two small questions if comfortable.

Viren Shetty: Yeah, go ahead.

Ravi: Firstly, are we planning to have any preferential issue in the next like one year to raise the funds? Any equity preferential issue?

Viren Shetty: There's no need for it right now. But, should the need come, we would look at it, but that's much in the future.

Ravi: Okay, and another thing is, I know it must have been already covered, but what is expected tax rate on company as a whole for FY26?

Sandhya J: 15 to 16%, Ravi.

30Ravi: Okay, thank you.

Nishant Singh: Yes, Sukanta, can we have a question please?

Sukanta: Thanks for taking my question. First of all, I want to congratulate the management for doing the great job in the healthcare sector that is pretty much competitive in India right now. So, my questions are on more on a qualitative side. First, my question is, as we are not being as aggressive as other peers in terms of pricing or improving our output, and we focus more on efficiency improvement and the throughput improvement. So, what can be the long-term income margin or sustainable income and operating profit margin that we are looking at?

Viren Shetty: We are not giving guidance on margins. We will just look to improve the margins from where we are and try to see if we can sustainably grow it.

Sukanta: Will it be what we have right now, what we are operating at, or whether our new facilities will come up in the next 3 to 4 years, then obviously margin will get diluted. So, will it sustain at this level or it will be what, going forward?

Sandhya J: There are three parts to the margin, Sukanta. The India hospital margin, we are aspiring to grow every year. So, we will see an improvement there. There will be cash burn on the integrated care side, so that will dilute the margins. And Cayman, as Anesh indicated, will be more or less stable.

Sukanta: So, we can say that okay, it will be around 20% or 24%. Am I correct in my assessment?

Viren Shetty: Console level, yes.

Sukanta: Okay. So, my second question is, as our new facility is coming in Rajarhat, Kolkata, so what will be the efficiency that we can bring? Because as we see the entire Kolkata, if we see the Kolkata, then there is a high possibility of the entire Kolkata shifting to Rajarhat right now.

The s

hift is going on, and our new facility is coming at the heart of Rajarhat. So, how fast we are aiming to utilize the beds that we are coming up with? How fast we can ramp it up?

Viren Shetty: Yeah. Construction started. Venkatesh, do you want to give some colour on when it will be ready?

R. Venkatesh: So, we've just started the construction in Rajarhat after all the approval processes have gone through. It happens in stages from the piling and then into the construction. So, as we talk about phase 1, we've already given an indication that it will take us at least 30 months as we speak, from now, for this commissioning to happen. So, mostly it will be functional somewhere near FY28 when it comes to Rajarhat at the first phase of around 350 beds. '28, I think.

Sukanta: No. Finish what you were saying.

R. Venkatesh: Yes. You wanted to ask something else? What I said is at least two and a half years from the start of the construction, which we've just done now. That's the timeline for starting of phase 1.

Sukanta: Phase 1 will be 350 beds, right?

R. Venkatesh: That's right.

Sukanta: And how long it will take to... if I can recall correct, what we have projected is 1,100 beds in total, right?

R. Venkatesh: Yeah, that's right.

Viren Shetty: That's been projected. But for now, we're just focused on getting the first 350 beds off the ground. The remainder we'll take up as and when that capacity becomes full.

Sukanta: Okay. I have 2-3 more questions. So, should I ask it right now?

Viren Shetty: Yeah, yeah. Please, please. Go ahead.

Sukanta: So, in one of the previous calls, management said that retaining doctors and nurses is becoming very difficult day-after-day because of the competition and all the facilities and everything coming up in India. Although India is a growing market, there are other hospitals that are funded by the big players and the venture capitalists. So, as our model is, we keep doctors on payroll. So, is this the model that we are going to follow in future also, or are we looking at in a different way?

Dr. Emmanuel Rupert: All our clinicians are on a professional contract and they work predominantly with us. Because the volumes of work is so much, they hardly have any time to go and work in any other place. But they work mainly with us.

And as far as the manpower is concerned, I think we did mention about this in the previous question. But we do have in both the Bangalore and Kolkata and as well as in the Raipur clusters, nursing schools and paramedical training programs and the postgraduate training 32 programs, which is the largest in the private sector. And we have a very careful manpower planning right from beginning itself, so that we know exactly whom all to identify and recruit well in advance to the projects being materializing. So, we are fairly confident that we will be able to handle these expansions and not have any major issues with the manpower.

Sukanta: Actually, my question was exactly, how we are planning to retain our high-quality group first.

Anesh Shetty: Sorry, a clarification. I don't think we said that we had challenges retaining staff or nurses in any previous call. If you could please let us know where you heard that, so we can clarify.

Sukanta: Actually, not exactly challenges, but what we are seeing in the industry that more and more hospitals and large hospitalizations are coming up. So, if few of our nurses and doctors, if they get a chance to get an offer to go to some other hospital on a higher package or there's something like that, will there be any challenge to retain them, or we are pretty much sorted there? There will be no challenge in retaining our talent.

Dr. Emmanuel Rupert: We are fairly very good with our doctor engagements and especially our senior and middle level doctor engagements. We hardly have any attritions at that level. And we are very confident going forward. In spite of all the competi

ons and new opportunities that keep

coming up in every cities, we will be able to retain our doctor talents. It's got lots to do with doctor engagements and we have a unique way of doing it. And we are confident of continuously engaging with them.

Sukanta: Thank you for your answer. And another question is, whenever we talk about international patients, we mainly talk about the patients from Bangladesh that we cater mainly from our hospitals that we have in Kolkata. So, my question is, why you are not looking at international patients from some other geographies, from other countries? Because as we can see, and through various reports on medical tourism in India and all sorts of things, India is a very competitive market, and India is at an advantageous position in terms of cost. The procedures that we do in India, it is far superior in terms of quality, and also it is very less expensive.

Viren Shetty: Sukanta, in the interest of time, I'll answer that very quickly. Pre-COVID in 2018, we took a call to slowly ramp down all our international medical tourism patients. All the things what you said are correct, and all the hospitals are investing a lot of money in trying to attract international patients. We do not want to be one of them. We see significant domestic opportunity. We see a significant opportunity for Indian customers and for people who live within the close vicinity of the hospital that we have. So, our services will cater mostly to the domestic patients. Those that come from abroad, will come, but we will not actively pursue 33 this. If there is international opportunity to be had, it will only come from us building facilities in those international geographies which will follow the criteria that Anesh and I laid out earlier.

Sukanta: Thank you for that clarification. The last question. We talk a lot about AI incorporation and the technology that we put in in our procedures and operations. How is that going to change and improve our efficiency in the long run, in the next 5-10 years? How can AI impact our business and how can it improve our efficiency? Incorporating AI and ML, how can it impact our profitability?

Viren Shetty: Not AI specifically, but digitization in particular. See, no modern business can survive without digitization. The healthcare industry has been uniquely resilient towards adding electronic medical records, towards using the computers in all the basic transactions, and patients' most common experience is going from one doctor to the other, carrying big files of paper. When you go from clinic to hospital, hospital to lab, lab to blood bank, again file-file-paper everywhere.

So one of the most obvious things that we said is, we want to be the most digitally efficient healthcare institution in the world. Not just the country, in the world. And so, we invested a huge amount of money in building a large team in Bangalore, who are building all the technologies we need to make the patient's journey inside the hospital seamless. We have eliminated nearly all paper from all the departments. We've made it so that our doctors can get data instantly and they can do the rounds of the ICU in their house.

Now, why is this important? This is important because we constantly look to lower costs and reduce inefficiencies. So, if you ask what the impact is? The impact may not be directly attributable to what is the spend on the software, but the very fact that as the only healthcare group that has not added a single bed since 2016, but has in fact reduced beds, yet continue to grow revenues in line with the industry, is just testament to how much we were able to do by looking at our efficiencies and looking at all the manual processes making it as digital. So, this is what any modern corporation should look like. We have to invest, because no one is going to invest and give these products for us. AI, that's just the new flavour of whatever digital tools that w

e use. When it becomes stable, we will start using that as well.

Sukanta: Okay, thank you for your answer. Thanks for all the clarification. Good luck to the management and to the entire team for the journey ahead, because we have a huge opportunity in India in terms of the medical field. So, best of luck.

34Viren Shetty: Thank you. Just to very quickly answer one of the questions that Sahil asked in the chat. Which is, what is our attrition for the Technology team, given that they sit at HSR layout, which is the startup capital of India? Our attrition so far has been 7% for the Technology team, which is, I have been told, highly impressive.

Nishant Singh: Thanks, Viren. So, as there are no more questions, we would like to conclude our session. Thanks everyone for your active participation as always. Thank you.

END OF TRANSCRIPT

Call: Aug 2025

Date of submission: August 11, 2025

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code –539551(EQ), 975516 & 976418 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051

Scrip Code - NH

Dear Sir / Madam,

Sub: Transcript of Earnings Call for the quarter ended June 30, 2025

In relation to earnings call of the Company held on Monday, August 04, 2025 for the quarter ended June 30, 2025, please find attached the transcript of the said Earnings Call.

We wish to inform you that the Earnings Call transcript is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earnings-call-audio-and-transcripts>.

This is for your information and records.

Thanking you,

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q1 FY26 Earnings Conference Call"

August 4, 2025

NH MANAGEMENT TEAM:

MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – GROUP CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – GROUP CHIEF OPERATING OFFICER

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS
SUBSIDIARY HCCI

MR. RAVI VISHWANATH – CHIEF EXECUTIVE OFFICER , NHIC

MR. NISHANT SINGH – VICE PRESIDENT , FINANCE , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. VIVEK AGARWAL , SENIOR MANAGER , IR FUNCTION

2 TRANSCRIPT

Nishant Singh: Hello , everyone. My name is Nishant Singh, and I welcome you all to the Q 1 FY26 Earnings Call for the company. To discuss our performance and address all your queries today, we also have with us Mr. Viren Shetty - Vice Chairman, Dr. Emmanuel Rupert - CEO and MD, Mrs. Sandhya Jayaraman - Group CFO, Mr. Venkatesh - Group COO, Dr. Anesh Shetty - MD of our Overseas Subsidiary, HCCI, Mr. Ravi Vishwanath – CEO, NHIC and Vivek - Senior Manager in the IR function.

Before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website as well as on the stock exchange later. We would also like to remind you that everything that is being said on this call that reflects any outlook for the future or which can be construed as a forward -looking statement, must be viewed in conjunction with the uncertainties and the risks that they face.

With that now, we would like to start the Q&A session. I request everyone to now use the 'raise hand' feature to start posing their questions. Thank you. Yes, Prithvi, please go ahead.

Prithvi: Thanks, Nishant. Anesh I just have a few questions on Cayman . If you have to look at the discharges and OP patients ' number, it has come down on a sequential basis and the decline seems to be quite high. We just wanted to understand this is something we saw in the past , there is some quarterly volatility in Cayman numbers. Is it the same and you expect patient numbers to return back in this quarter , or has there been anything structural that happened in the last quarter?

Anesh Shetty: Yeah, I think you're right. As you recall, since we commissioned the new hospital ... you had the Q4 of last year and Q1 of this year being the first two quarters of the new hospital. And if you recollect, even in the early days of Cayman when the hospital was new, when we're starting a new building with low base, low volumes, you're going to see this volatility up and down. But these are just standard blips quarter -to-quarter because the structure is new . And the next quarter as we proceed, maybe two or three quarters , things should stabilize. But your intuition is right. These are things that happen in the early days and they will settle in as we progress. On the ground, we have no reason to have any concerns around the growth trajectory with the new building.

3 Prithvi: Got it. And if I have to look at the absolute EBITDA number for Cayman business, on a sequential basis, the decline seems to be around INR 20-25 crores. I understand there is a INR 9 crore number coming from the Integrated Care. What explains the remaining difference in the EBITDA numbers on a sequential basis?

Anesh Shetty: So if you account for the slight decrease quarter -on-quarter in the Hospital business, aside from that, maybe I'll just say that the Hospital business as such, the economics have not changed, whether it's comparing Q4 to Q1 or a few quarters later. It's just that right now we have broken up and we are starting to see the Integrated Care business ramp up significantly in revenue terms, which is obviously at a very different margin profile, A) given it's a new business and B) it is Integrated Care insurance, which is very different on a margin profile from the Hospital business . So , the Hospital business itself, the economics remain unchanged. We did have a slight revenue decrease quarter -on-quarter. You are, again, going to see some early volatility in case mix. In the previous quarter, we had few in volume, but high value cases, which you generally tend to have higher margins. This time, that was not the case ; you saw that reflected in the volumes as well. But over the next few quarters, we think on the core Hospital business , things will continue to be similar to what we've seen previously.

Prithvi: And on the Hospital s

ide, earlier, before commissioning the new hospital, you were making \$120 million revenue and a 45% margin. I understand, a new hospital is focused more towards OP, where it can be a bit of margin dilute too. So, can we expect, or can we look at the Cayman Hospital business as \$200 million revenue under 40 to 42% of EBITDA margin business on a sustainable basis?

Anesh Shetty: Yeah, I think it is difficult to give exact guidance to that extent. Having said that, we think that the margin profile of the Hospital business, you will not see too much variation from what we've been used to. Obviously, your first comment is partly right that in the new hospital, you are more focused towards quicker short-stay daycare sort of procedures, which are obviously lower margin than the long, complicated cases. So, there will be to that extent that change. But the underlying profitability of the Hospital business itself will not see any big swings.

Having said that, like we've said before as well, for the entire bucket of the geography, whether it's the Hospital alone, the Hospital combined with the Integrated Care, whichever way we want to look at it, the intention very much, is in absolute terms, to continue to see 4 healthy growth in earnings because of the new Integrated Care on a much larger revenue base. But absolute growth in earnings is definitely the intention.

Prithvi: One final question on the Integrated Care. Can you throw some light on how exactly you're looking at this business? What kind of opportunity? And how does it transfer to investments and the breakeven number?

Anesh Shetty: So, this is the first time we're sort of adding small detail to break it up because it's starting to get significant. It was always subsumed in the financials that everybody's used to seeing. So, it's not a new outflow of any kind. Having said that, we started formally selling to the external market on the 1st of January. Obviously, it takes time to ramp up, but in just a quick 5 to 6 months, we've been very, very happy with the response we've seen. We have some of the most prominent and largest employers and organizations on board. So, the trajectory has obviously been very, very positive for us. It's going much better than we planned.

Having said that, this business is not going to have, even at steady state, and it was never intended to have the same margin profile as the services, the Hospital business, which we all understand and know. Right now, yes, it is loss-making in the 1st Quarter, first one to two quarters of the business. Longer term, we expect this to be a breakeven to plus or minus, slightly positive, hopefully by the end of the year or 1st Quarter next year. But this is going to be a very different margin profile from the services business. The reason we're doing it is to aid in our entire ecosystem of services to have a very safe lock-in with all our customers, to create this multi-touchpoint experience for patients so that longer term, we have a very safe, non-volatile earning stream from this geography.

Prithvi: But do you see any sizable revenue coming from this particular segment?

Anesh Shetty: Yes, it's starting to happen, and we do think that it is on that trajectory. It is something that even on a standalone basis is attractive. Having said that, its real goal is being part of the ecosystem. In fact, I would say the reason why we find it difficult to give an exact prediction on EBITDA as a percentage terms, because it's going to be challenging to predict the relative revenue between the Integrated Care business and the Hospital going forward, especially because the Integrated Care is going to see good revenue growth since it's starting from a low base.

Prithvi: Sir, just to squeeze in one final question. So, we can assume this quarter losses are the peak for the Integrated Care, and it will start coming down in the next couple of quarters?

5 Anesh Shetty: I think that's hard to say. As a percentage of revenue, that maybe it's going to be a narrow band; you're not going to see wild swings. But in absolute terms, it would depend on where the revenue lands up the next quarter. And this is just the first quarter we are disclosing it, so maybe in a couple of quarters, we'll get more clarity on what that could look like.

Having said that, we don't expect to see wild swings, unlike the Hospital business. I would also point that maybe quarter-on-quarter is perhaps not the best way to look at it, because it depends on when large claims come in, etc., and these kinds of factors. So, let's give it a couple of quarters to play out, and we'll have much more clarity there.

Prithvi: Thanks. That's all from my side.

Anesh Shetty: Thanks, Prithvi.

Nishant Singh: Thanks, Prithvi. Dam ayanti, can we please have your question?

Dam ayanti: Yeah, hi. Thank you for the opportunity. My question is again on Cayman operation. So just want to understand, in the newer unit, the services which you intend to offer, are all those now in place, or you have some more to offer, which can help the volumes to pick up from here on? So just want to understand on that part.

Anesh Shetty: Well, So, all the new services have been activated, to your question. Having said that, we don't see the entire potential of them kick in immediately. It does take some time for these, depending on the service line, for them to grow to their potential. But yes, as of the quarter we are talking about, which is Q1, all services were commissioned, albeit just recently commissioned. But yes, all have been commissioned.

Dam ayanti: Okay. So how long do you expect the things to reach steady state in terms of operations being in place and volumes, etc., also reaching up to some steady state?

Anesh Shetty: I mean, it's difficult. We don't know how these things take to ramp up with an exact quarterly number or prediction of that sort. It depends, some will obviously be quicker than others.

Some are dependent on some other changes with payer relationships, et c., happening. That would be very difficult to answer. What could help is that you did see from say Q4 to Q1, a very big step up in revenue. And that is almost exclusively because of the Hospital, almost all of it, because the Integrated Care had just started or was negligible. So, you did see that big jump, which is bigger than we expected, but sort of what one would predict because you've started a new facility with new services.

6 Going forward, the increased quarter -on-quarter will obviously not take that big leap up. It will be more linear as the existing services grow in and mature and there's more awareness and marketing and sales about them. So, we're not going to see that kind of jump that you saw from Q3 to Q4, but we do expect to see a linear growth going forward.

Dam ayanti: Okay. And for the combined operations, you mentioned 40% to 45% margin looks sustainable.

Anesh Shetty: No, we didn't. Yeah, we didn't say a number, but sorry, please go ahead.

Dam ayanti: No, I'm saying, say compared to the ... because on Q1 numbers, we saw IP, OP volume seeing some decline quarter -on-quarter, but you grew in the top line, right? So, I assume the mix would have been much better. So, I just wanted to understand, compared to last quarter's EBITDA profile for the Cayman operations, the Q1 EBITDA margin profile, were like those very different, or how should we think?

Anesh Shetty: Yeah. So maybe I'll try and help with that. The Hospital business will not see any significant change in the profitability or the margin profile. You are going to see certain variations quarter -on-quarter, but on the whole, that remains unchanged, the core economics of that business.

The challenge to give you a percentage prediction or guidance for the console picture, is because it depends on t

he relative revenue contribution of the Integrated Care business, which as of now is yet to break even. And even when it does, it'll be a significantly lower margin business than the Hospital business. So, it's too early to tell how significant in the total revenue console that will be, to give you a guidance on what we expect the console Cayman EBITDA percentage to be. So, predicting a percentage is tricky, but predicting what it would be in absolute terms or the trend in absolute terms is easier. I hope that was helpful.

Dam ayanti: Yeah, that's helpful. Thank you. My another question is on your India operations. So just want to understand the kind of scale -up or progress you have seen in your insurance offering. So, I see in the presentation that it has been rolled out to other segments as well. But how do you see this piece picking up?

Ravi Vishwanath: Sure. Hi. This is Ravi. So, I think the Insurance business overall has picked up quite well. As you saw in the presentation also, we've added some new markets and new products as well. The A rya scheme has started encouraging ly well. We have great feedback from our customers. We've got about 6,000 lives at the moment, and we are now seeing acceleration in that pace as well.

7 Over the quarter, we started in Kolkata, Shimoga and Raipur. We've also launched an upgraded version of our first plan is Aditi, called Aditi Plus for the Kolkata market. We've also developed and taken to market a group omnibus product for non -employer -employee plans, which is something we're actually quite excited about, because it opens up new opportunities for us to partner with external parties, potentially, that have got pools of customers. We are building the business steadily. The focus is on risk management and underwriting, and making sure that we are building a business that will be sustainable over the long term.

Dam ayanti: Sure. So, the current quarter loss from new initiative s, can we assume mostly it's coming from your spend, which is happening on the Insurance part?

Ravi Vishwanath: Sandhya, do you want to answer that?

Sandhya J: It is a combined number between both insurance and the NHIC, which is the clinic business.

Dam ayanti: Okay. That's helpful. Thank you. I'll get back in the queue.

Nishant Singh: Thanks, Dam ayanti. Ronak hi, can we please have your question?

Ronak : Yeah. Thank you for the opportunity, sir. So, as we know that we are going to start the new hospitals in 2028, 2027. So, I mean, what would be the break -even cycle we are expecting? Because these hospitals are in the existing region. So, first of all, are we expecting the break -even to happen or the ramp -up to happen very fast? And second, like, are we or would we be able to charge a similar revenue per bed comparable to the existing revenue per bed in the same region?

Viren Shetty: Hi Ronak, the break -even characteristics of the hospital we're starting up will be in line with market. The realizations that are possible in these hospitals will be at a discount to the current market prices in the geographies where we are setting up hospitals.

Ronak : Okay. Got your point, sir. And, coming on the specialty profile, in the past few years, we have seen th at oncology has increased very much. So, can we expect oncology in the coming five years to become 20 -25% as a percentage of revenue?

Dr. Emmanuel Rupert: The oncology has been growing very well for the last couple of years. And that's the same trajectory which we have been observing and working towards with the clinical team and the kind of investments that we are making. And the aim is to get towards that number which

you've mentioned, and we will be seeing a good healthy growth year -on-year on that.

8 Ronak : Thank you, Sir. I'm done with my questions.

Nishant Singh: Thank you. Bhavi, can we have your question, please?

Bhavi: Hello. Thanks for the opportunity

. So, I would like to ask that there are 400 beds currently in the pipeline. When will this be operationalized, and what Capex is expected for these 400 beds?

Nishant Singh: So, at all possible times, we keep exploring for the new opportunities. So, the moment we close something, we'll be able to announce them. But right now, we are looking at multiple opportunities, but that pipeline is not confirmed yet.

Viren Shetty: These are M&A sort of transactions that we're in discussion with various bankers with, and we're not exactly sure when they can conclude.

Bhavi: Okay, thank you for that. And another question. The hospitals are getting commissioned in FY27 and FY28. So, will the margins be impacted due to the commissioning of new hospitals?

And if they are impacted, will the impact be similar to the dilution in the past, or it will be less due to the brownfield?

Viren Shetty: It will be a margin impact. We will do our best to minimize the impact. Whether it will be less than the past, that's hard to say. Just because in the past, the cost structure was very different. The manpower cost and a lot of the startup costs are much higher in these days than it was when we had done in the past. But again, we will try and keep margin dilution, breakeven period more in line with what the peers have been able to achieve.

Bhavi: Okay, and another question. I have seen that ALOS fell from 4.5 to 4.3. So, can we expect this to fall for the year, or it is just for the quarter basis?

Dr. Emmanuel Rupert: We are working towards this, I mean, we want to bring it down to closer towards 4 and we are working towards this. Hopefully, this will keep coming down in the future as well.

Viren Shetty: But quarterly variances will show this slight fluctuation because of a lot of seasonality, but the long-term trend is towards lesser ALOS.

Bhavi: Okay, thank you. I am done with my questions.

Nishant Singh: Thank you. Rehan, can we have your question, please?

9 Rehan: Yeah, good evening to our team and thank you for giving me the opportunity. So, I want just more clarification regarding the Integrated Care and Insurance business. So, with the Integrated Care program is now scaled to multiple cities. So, what's the next frontier? Are you considering disease management or wellness-led models under NHIL? Or on the other insurance side, how is Aditi Plus being received in Kolkata and what are your plans to build distribution or partnership for this vertical?

Ravi Vishwanath: Sure, I will try and answer most of those questions. So, in terms of the Clinic business, just to clarify, the Clinic business itself is focused on Bangalore at the moment. And in fact, we are increasing our footprint in Bangalore. There's a new clinic that's opening tomorrow. There's another one that's under construction. Over the coming quarters, we'll be focusing on offering a wide range of services to our customers. Now, these will include, I wouldn't call it wellness, but these are basically services that we'll be offering to help our customers get well and stay healthy. And that's kind of our focus. In terms of geographical expansion, we continue to evaluate opportunities to grow the Clinic footprint, both in Bangalore and beyond, and we'll try and share updates on that as and when it's appropriate.

In terms of Aditi and Kolkata, I think that's gone well. And in fact, we also did launch the new product, which is Aditi Plus in that market. But we've only been selling in Kolkata for about six weeks, but the initial response to this has been very good. And we're very excited about how we think the Kolkata market will react to Aditi.

Rehan: Okay, Sir, thank you for clarifications. And my second and last question is on the digital and technology investment side. You have mentioned digitalization of 85% patient documents.

How are these initiatives transferring into measurable operational

efficiencies like reduced

turnaround time or improved clinical outcomes? And just a follow up on this, can you elaborate on the roadmap of your doctor app and ICU master suite initiatives? Are there early signs of improved compliance or productivity? If you could just throw some light on this.

Dr. Emmanuel Rupert: Yeah, so we have rolled out our ... just like Aditi, which is a doctor application, we have rolled out our nurse application called NAMA. And also, the digital for the inpatient services, the medication card and various other modules. It's getting stabilized, and we hope that it will get stabilized somewhere by October. All these things will speed up the discharges and other things, because we are working along with our MED HA, which is the analytical division, to work towards multiple improvements in the operational efficiencies, so that we'll be able to streamline the processes and make documentation, both clinical and operational documentation, much more streamlined and enable things to happen in a much quicker way.

10 So, these are some of the things which we have been doing. This will keep continuously happening and improve the overall efficiencies, and it will make the manpower, clinical as

well as operational manpower to be very effective in what we are doing.

Viren Shetty: It's hard for us to accurately gauge out how much of the improvements that we've seen, both in realization, discharge time, and the patient experience can be attributed solely to the digital investments, just because the nature of technology today is that it is so diffused, and we have parallel projects running at the nursing team, at the clinician team, with our technicians, with our senior doctors, with the administrators, supply chain people, who are embedding this into everything that they do. So, our hospitals are fully paperless. Our patient experience can be done in such a way that no patient has to visit a counter for anything. Most of the transactions in our hospitals can be done either on their phone, with the app, or going to a kiosk. Now it is far from perfect, there's still a lot of bugs that come from time-to-time, but this is the experience that we're investing in, so that it is a fully seamless experience in all our hospitals.

Rehan: Okay. Sure, sir. Thank you for the clarification, and I'll jump back in the queue.

Nishant Singh: Thanks, Rehan. Yes, Bino, can we have a question, please?

Bino: Hi, hope you can hear me.

Nishant Singh: Yes, we can hear you.

Bino: Yeah, thanks. I've got a little bit of confusion around this Integrated Care. So, you have these two entities, NHIC and NHIL. May I know what each of them does?

Viren Shetty: NHIC is a company that runs clinics. So, these are brick & mortar clinics in Bangalore, and they run care plans where people can buy subscription packages for loyalty access to the clinics, for doctor follow-ups, medicine, home delivery, and home collection of lab samples. That is part of NHIC, NH Integrated Care.

NHIL is the IRDA I-regulated health insurance company. This is the entity that has the license for being a standalone health insurer, and this is the entity that issues policies called Aditi, Arya, Arya Plus, and so on. So, this is our insurance entity. The insurance entity sells through NHIC as a channel as well as through multiple other arrangements.

Bino: And those who have policy from NHIL, are they as of today free to go to other facilities as well or do they have to come to NH facility?

11 Viren Shetty: Ravi, you want to answer that one?

Ravi Vishwanath: Sure. So, for the moment, the way that we look at this is that we can provide the best experience to our customers when they come to our hospital. One of the biggest pain points is the entire pre-authorization, discharge, something not paid, something else covered, etc., confusions. This we want to avoid. So, we would prefer our

customers to come to our hospital

for treatment. Having said that, there are situations where if somebody needs to go outside, they are able to go to any hospital in the country. There are four specific situations –

- If somebody has an emergency, they can go to any place they want.
- If the treatment is not available in our network, they can go any place they want.
- If they happen to be traveling and require assistance, they can go any place they want.
- And if they buy the policy here and then they move somewhere else within India, they can go wherever they want.

So, in those situations, people can go to any accredited hospital in the country. Otherwise, we do prefer that they come to our hospital because the whole point of this is so that we can give them the best possible experience at the time of discharge and remove the trust deficit that typically exists between an insurance company and a hospital. In this case it's not there and that allows us to give a much superior experience to our customers.

Bino: Understood. So, this was all India, now is there any Integrated Care model like this in Cayman as well?

Anesh Shetty: Yeah, it is. The local dynamics are such that you can use it anywhere, in any hospital in the US, anywhere in the world, any other provider in Cayman. We don't restrict anybody because the local market conditions and dynamics are different.

Bino: I'm sorry, I haven't quite understood that. The reason I asked was earlier when there was a discussion about the margins in Cayman Island, it came up that there was some Integrated Care model which has come up there. I was not sure if I heard it right or sure, that's why I asked.

Anesh Shetty: Yeah, you are correct. So, we do have that. It's just that to your question on whether the holder of this plan or policy is restricted as to where they can use it, it's not like in India. In India, you predominantly have to use it in NH with certain exceptions that Ravi mentioned. In Cayman, with these plans that we sell, you can use it anywhere you want.

Bino: Understood. But that is a completely different operation, right? Or does it come under NH?

12 Anesh Shetty: Separate entity. Completely separate. Yeah, completely differently regulated. Everything is different, yeah.

Bino: Got it. So, I was wondering, your Cayman operation is pretty big now. In your PPT, it would be great if you give the Cayman EBITDA separately as a number. That would be great. You give the revenue, but I don't think you give the EBITDA number.

Anesh Shetty: So, there are ways to get a rough sense of what that is with the information that's there.

Maybe, Nishant, we can connect with Bino offline to help walk him through that.

Nishant Singh: Sure, sure.

Bino: Yeah -yeah , sure. Thank you very much.

Nishant Singh: Thanks, Bino. Rajit, can we have your question, please?

Rajit: Yeah, hi. The Southwest Bangalore expansion, given that the civil and structural works are in final stage, do we have a sense of exactly when this will be operational, whether it will be Q1 or Q2 or FY27 or later?

Viren Shetty: Not that much accuracy. It will take about a year. The building shell has already come up , we are doing on the interiors and ordering the medical equipment.

Rajit: So, a year from now ?

Viren Shetty: Yeah , at least, right.

Rajit: Okay , all right. So, in the last call there was a discussion on chemotherapy centers coming up in Gurgaon. You had mentioned there will be a soft launch and then a formal launch , any update on that and what kind of footfalls you might be seeing there? And if you can share some kind of numbers on it ? Revenue per patient or something like that so that gives us a better sense of where that is going to.

Viren Shetty: This is an investment we've made in a startup, which will be doing this. It's called Everhope Oncology . Their first infusion center is in Gurgaon, in Emaar Business Park . The center is up

and running. They've had the soft launch already , they've not gone for the official launch yet. And they are seeing patients.

As far as it comes to reporting out the numbers, it will, for the near future, be a very small contributor. And being a startup, have significant setup costs and losses and so on. And our 13 involvement in this is as an investor. We are providing a lot of the backend services like referrals for radiation oncology and surgery as well as providing them supply chain and software help but we're not managing these units. And it won't count as any of our subsidiaries.

Rajit: Okay , understood. And they're obviously free to see patients referred from other hospitals also?

Viren Shetty: Yes.

Rajit: Alright. One last question. The gross written premium of the insurance segment has marginally come down during this quarter compared to the last quarter. So, I mean, given it's not even one year since we launched it and we are actually being very bullish on it and it's growing, what could be the reason for that?

Ravi Vishwanath: It was more or less flat. And, as you know, the bulk of the buying behavior in the country is that the large majority of sales actually happen in the Jan -Feb-March quarter for tax and other reasons. And , so, there is a bit of a seasonality effect. We are seeing much improved momentum in the latter half of the last quarter and into this quarter as well. So, we continue to be bullish about that. But , largely, if you look at insurance anywhere, typically, the first quarter there's always a large drop off from the JFM quarter across industry.

Rajit: But these would be annual policies, right?

Ravi Vishwanath: It doesn't matter because what's booked is the GWP, the Gross Written Premium, which is a full year's premium. So , when somebody buys, if 100 people buy the policies in JFM, the premium is counted in that quarter and it's not split out over the year. The GWPs are counted in the same quarter.

Sandhya J: Just to add to what Ravi mentioned, also the numbers that you are comparing is YTD . Because last year was a small year for insurance , so we were reporting YTD numbers and now we are reporting quarter on quarter. So , they are not exactly comparable also.

Rajit: But it says YTD Q1 FY26, right? So , it's only for this financial year?

Sandhya J: Yes.

Rajit: Oh, these are not cumulative?

14 Ravi Vishwanath: Yeah, only for this. No-no, it's not cumulative. On a cumulative basis we've had.

Viren Shetty: But your larger point is taken , Ravi needs to work harder and make it grow more.

Rajit: Alright. Thank you, Sir. Good luck with that.

Ravi Vishwanath: Thanks.

Nishant Singh: Thank you, Raj it. Nancy, can we have your question, please?

Nancy: Hi, team. Thank you for the opportunity. I also wanted to discuss the Cayman numbers. It seems that the EBITDA has fallen. So, like, firstly, if possible, I wanted to confirm the number.

And if it has fallen according to what I'm able to calculate , then I wanted to know the reason.

And if possible, I also wanted to get the Ind AS adjustment number for Cayman.

Anesh Shetty : Yeah. Hi, Nancy. Thank you for your question. So, as we explained in the beginning of the call, we're now seeing a significant ramp up in our Integrated Care business. So , the economics and the margin profile of the hospital business remain largely unchanged from what you would have seen previously. But on a consol picture, you will see a diluted EBITDA percentage. And that is because of the new Integrated Care business which was not a significant contributor to the consol earlier and now is starting to become so.

And, secondly, it is yet to break even given that it's just the first or second quarter that we've started it. So, you are going to see that drag effect of that. We expect this to settle down over the next few quarters.

ters.

On your second question, I'll pass it on to Sandhya on the Ind AS verification.

Sandhya J: USD 658K is the impact for the quarter.

Nancy : Alright, sure. And also I just wanted to confirm, so basically is it possible for the team to tell the drag number from the Integrated Care business on the Cayman?

Anesh Shetty: That will be the slide. Sorry, go ahead, Sandhya. Yeah, go ahead.

Nancy : That's INR 9.3 crore negative number, right?

Sandhya J: Yes. Actually, there is a revenue as well as EBITDA number that has been given there. So, for you to derive, you have to adjust both the numbers.

15 Nancy : Understood. Understood. Thank you so much, team. Super helpful.

Nishant Singh: Thanks, Nancy. Archit, can we have your question, please?

Archit : Sure. Thank you for the opportunity, Sir. My question is regarding the Oncology division. In addition to the previously asked question regarding the Everhope chain which we established in the last quarter, you mentioned that we are yet to start on the chains. Do we have any expectation on the number of centers which are currently in operations and the pipeline for this year?

Viren Shetty: As mentioned earlier, the investment we make is in a startup that is still in the ramp up phase. They are not a publicly listed company, and they will be making their investment as per their own strategic business plan. Our investment we've disclosed earlier, which they will use to build a business with. They have some very aggressive projections and as and when these things come up and are clear for us to disclose publicly, we will do so.

Archit : Understood.

Nishant Singh: Thanks, Archit. Vedant, can we have your question, please?

Vedant: Thank you for the opportunity. I think this question has been asked earlier but I was not able to hear it properly. On the slide 13 of the PPT where the operational review of Cayman Islands has been given, in the bottom left corner the revenue and EBITDA of CIHL for Q1 FY26 is there. Could you please elaborate on that?

Anesh Shetty : Yeah, that's the Integrated Care we were talking about. It stands for Cayman Integrated Healthcare Limited or something like that.

Vedant : So, does that include only the insurance business?

Anesh Shetty: Yeah, Integrated Care. That's the only activity of that.

Viren Shetty: Just for Cayman, just to clarify.

Anesh Shetty: Just for Cayman. Yeah, sorry. Just for Cayman. I think there's a confusion because the currency is in rupees. Sorry about that mistake. Yeah, it's just for Cayman.

Vedant: So, the amount is in rupees only, there is no mistake there, right? INR 9.3 crores.

16 Anesh Shetty: No, the amount is correct, but we should have represented it in dollars. I think we represented it in INR but whichever way you convert it the absolute number is accurate.

Vedant: And if we adjust this with the overall EBITDA, I think then we'll be able to find out only the hospital business EBITDA, right?

Anesh Shetty: More or less. Yeah, more or less. Sandhya, any comments there?

Sandhya J: Yeah, you have to adjust both on the revenue and on the EBITDA side.

Vedant: Right, right, correct. Okay, thank you.

Nishant Singh: Rehan, you have any other questions because you've been raising hands for a while now?

Rehan? Yeah, so we'll go to Prithvi.

Prithvi : Thanks for taking the question again. Anish, again on Cayman, given that now you have two hospitals and you're also coming to insurance, so how do we look at the opportunity size here? I mean, you're already at \$45 million revenue in Cayman and the population rate is quite less. So, could you throw some light on, you know, I mean, for how many years you think you can grow in Cayman before reaching a growth of population growth number?

Anesh Shetty: Yeah. So, you know, Prithvi, as you'd recollect from previous discussions, yes, the population is small, but our market

is a broader Caribbean. Having said that, both the new businesses, which is the new hospital as well as the new Integrated Care. We are still only in quarter two and especially the new hospital is a very substantial investment we've made. So, we continue to absolutely expect and be confident of, you know, growth for several, you know, at least for the foreseeable future given we're still just in Q2 of where we've started. The only caveat I would add is, you know, it's not possible to predict. There'll be some lumpiness in this growth but on the whole, we are confident for a healthy runway still to follow given both the untapped demand domestically as well as the larger market we cater to internationally in the region.

Prithvi: If I have to just understand this a bit better, you know, how much of growth is coming from Cayman and how much is from other Caribbean islands? I mean, I'm not talking about exact

numbers but incrementally can we assume half of the growth is coming from other Caribbean islands? And this is where there is still significant scope for you to grow?

Anesh Shetty: Before that, Bino, sorry, could you just go on mute? Thank you so much.

17 Yeah, Prithvi, back to your question. You know, it is difficult to give a split between domestic and international. I will say that for both, whether it's domestic as well as international, we still have, you know, an untapped opportunity. Of course, much, much more in international but more challenges to access it. Domestically is much more known and, you know, we understand what we need to do. But we continue to see good growth both from domestic and international. We do predict eventually, not now but eventually, the bulk of our growth will come from non-Cayman markets. Obviously, we will saturate this market, but we are not yet there.

Prithvi: Understood. So, one final question on India business. In the last con call it was mentioned that this year will be the peak losses for insurance and the clinic business, are we still retaining it? And can we expect losses to come down from next year onwards?

Sandhya J: No, we haven't given a peak loss kind of guidance but more so we said that we are investing, and we have set aside a certain amount of money and we are thinking that a lot of the investment will happen in the initial period. So, from that perspective, I think you have inferred that. So, our first cohort of clinics have broken even. So, we are able to see that model that we built saying each clinic will breakeven in 18-month time, we are able to see that. There are corporate costs to recover and there are growth ambitions. So, as new clinics come online, we will have losses in the initial period. We are still sticking to our overall cash burn number that we have indicated for both the businesses put together. And as you can see, the absolute cash burn in the business in the current quarter is lower than what it was in the previous quarter. So, we are projecting in the right trajectory.

Prithvi: If I remember the number, it is almost INR 450-500 crores of investments is something that you have guided, right?

Sandhya J: 450. Yeah, 450 crores we have.

Prithvi: May I know how much of that has been invested till now?

Sandhya J: So, INR 100 crores we have invested as capital and INR 150 crores have been invested for the clinics and the cash burn relating to that, including some Capex

Prithvi: That's it. Thanks. Thanks a lot.

Nishant Singh: Thank you, Prithvi. Aryan, can we please have your question?

18 Aryan: First of all, thanks for the opportunity, Sir. I am sorry if it is already answered, I just wanted to know that what growth we are expecting in Indian business till new facility commission because as we all know that there are capacity constraints in Bangalore and Kolkata cluster, right? So, for time being, how we are going to deal with that?

Viren Shetty: Y

Yeah, we will try and maintain the growth trajectory we have had for the past 2-3 years. What we are doing is still more of the same - more digitization, more automation, more work on the patient mix, more work on improving our efficiencies over here, more investment in robotics, in oncology, looking at the case mix. There are enough and more things that can be done in a hospital to generate, you know, the hard-fought and hard-won revenue and margin growth while the new hospitals take time to come. But there is plenty of work for us to manage. But it will not be in the high double-digit growth numbers that would otherwise come from adding new beds in new parts of the cities.

Aryan: Thanks, Sir. I am done.

Nishant Singh: Thanks, Aryan. Yes, Ajay.

Ajay: So, yes, thank you very much for the opportunity. So, I have a couple of questions. So, first of all, I can see that there is a good growth in every peripheral for ARPP. So, what were the factors that led to the growth and is this trend going to continue?

Sandhya J: So, there were a few factors.

- Like we always do, we have taken a small price increase on 1st of January, low single digits. Some of the effect of that has come through also in the current quarter.

- The second is that there is a pay or mix change. As you can see, from last year to this year we have had almost a percentage improvement in the pay or mix. So, that is also helped.

- And, thirdly, we have had almost a 50% drop in the Bangladesh revenue from last year to this year same quarter. And Bangladesh was coming in at a lower realization. So, that having been substituted by domestic revenue, domestic revenue has grown by 12% this quarter. So, that is also contributing to the ARPP.

- Finally, we are also doing a lot of high-end procedures. For example, we have done the largest number of cardiac robotic surgeries in the country in the last quarter and so on and so forth. So, there are a lot of high-end work that is also coming through, which is also improving our ARPP.

Ajay: Okay. So, is this trend going to continue forward, the growth in ARPP?

19 Sandhya J: To the extent of the one-time benefit we have got from the Bangladesh mix change, I think that kind of increase will not come. But the rest of it, yes, we can look at it as a ...And, again,

the price increase will next happen only on 1st January. So, that benefit will also come next year. But the regular improvement in pay or mix that we drive as well as the higher value procedures, that we will continue to push.

Ajay: So, second question was regarding the expansion. So, what I have seen in the presentation was the expansion generally in the higher ARPP regions, right, in Bangalore, I think, and Kolkata, third is in Raipur. So, going forward in the longer term, say after when the expansion is completed, so can we see a margin increase from the current levels?

Viren Shetty: Absolute basis, yes. Because once they have broken even, on absolute basis the EBITDA numbers will be higher. On a margin basis, that will depend a lot more on the cost structure as it looks because as much as Bangalore, Kolkata, Delhi, Raipur places are high realization, they are also very high -cost markets. And the cost drivers of this industry are relentless. Meanwhile the pricing pressures what we face from various payors, both insurers and the government, are also equally relentless. So, question of, you know, will things look better, one will balance out the other.

Ajay: Okay. So, last question was, I can see that the owned hospital revenue mix has increased to 73% and the management part has little bit come down. So, has this any impact on the margins, that mix of owned and managed hospitals?

Viren Shetty: I think this may have been the removal of Jammu, which is a managed hospital that we were managing on behalf of the Shrine Board. We have moved to handover of the hospital to the Shrine

Board Trust, and we are just there to manage the operation. They have no fee. So, I think that may be explaining the difference over there.

Ajay: So, going forward, it is going to look the same, right, owned and managed hospital?

Viren Shetty: Yeah. Right -right.

Ajay: Okay. Thank you very much.

Nishant Singh: Thanks, Ajay. Niraj, can we have your question, please?

Niraj: Yeah, thank you for taking my question. Sir, I have a question regarding this insurance. Sir, I was going through this Aditi insurance plan, it is very competitive. So, I have a question 20 regarding, do we add any loading on the insurance premium if the customer has any pre-existing ailment or any surgeries undergone in the past before underwriting the claim?

Ravi Vishwanath: So, Neeraj, our goal is that we want to give people a policy with no waiting periods and either /or pre-existing conditions, etc. But obviously, as you know we are also in the business of taking risks, so what we ask is we ask our customers to undergo a health checkup. It is a comprehensive health checkup; it is part of the service we offer. It is a comprehensive health checkup that they can do actually every year. It is built into the plan, there is no fee for it. It is completely free checkup that they do. The majority of customers who complete that health checkup, they go through the plan on a standard basis. In some cases, we may ask for some slight addition loading in premium or depending on, again, on the underwriting results we may ask for a waiting period. But that is not the rule, those are the exceptions.

And, of course, in some cases, we may not be able to offer cover as well. For example, if somebody is, you know, immediately scheduled for surgery, then it is not really insurance. But the point is that we ask every customer to go through a health checkup and, you know, we decide on the basis of that.

Niraj: And, Sir, second question is, for growing the GWP, are we looking to tie-up with some national distributors pan India or it is just through NH network?

Ravi Vishwanath: So, we are always looking at the balance of this. As you know, some of those national distributors come at a very high cost and our focus, especially with Aditi, is to make sure that we are offering something that is affordable for the people who otherwise cannot afford insurance. So, it is always a balance between the cost of distribution, which can be very high, and, you know, passing on those benefits to the customer.

What we are focusing on at the moment is offering our plans directly to the customers and giving them the benefit of both lower premium and better service by going direct. But, again, this is something that we do evaluate all the time and, you know, we are looking for partners that are kind of like -minded where we can offer this through certain partners. But the focus has always been on the customer and making sure the customers get the product at a good price; great value and we are able to provide good service.

Niraj: Correct. Sir, one last question. Sir, do we have any internal guideline in terms of underwriting that since we have just started this insurance business because in terms of age, say for example if we underwrite 100 claims, so if the ratio of people who are above 60 where the claims can be higher in future, so say only 40% or 30% of the customer should be above 21 certain age and then if it crosses above certain age then we slow down on the underwriting part.

Ravi Vishwanath: No, it's not that. There's no automatic loading if somebody is above a certain age or anything like that. And by the way, this is one of the great strengths that you get when a hospital that really deeply understands healthcare and health risks, you know, does insurance and looks at, you know, evaluating these risks. And that's where the health checkup comes in. So, it's a

detailed health checkup and

we have a very, very extensive underwriting process in terms of guidelines and rules. And it's completely based on that. There's no automatic, you know, if somebody is above 60, they get a loading of this or somebody's got diabetes get a loading of that. It's not that way at all. It's a very holistic review and really takes advantage of our understanding of healthcare. And I think it's one of the key differentiators to help us manage the risk in the book over a long-term basis.

Niraj: Thank you. Thank you so much.

Ravi Vishwanath: Sure.

Nishant Singh: Thanks, Niraj. Rajit, please, can we have your question?

Rajit: Yeah. Given the current projects in hand, what is the peak debt that you are aiming for? And if you can share the Q1 numbers of borrowings and lease as well?

Nishant Singh: We'll answer this question in terms of the leverage ratios. For us, we are looking at the maximum leverage ratio at the consol level of around 2.5 -3 and same for the respective entities as well. Maybe India also at the same number. So, the leverage is on the Net Debt EBITDA, which will be maximum around 2.5 -3.

Viren Shetty: It may fluctuate up to plus or minus depending on in case any acquisition were to come temporarily but on a sustainable basis that's the range we would look, the upper bound of the range we would look at.

Rajit: Right. Much appreciated, sir. Actually, where I am coming from is because of the ongoing expansions, while your EBITDA has in absolute terms gone up Y-o-Y, the earning per share has actually gone down, and that is because of higher depreciation and interest cost.

So, all I'm trying to do is forecast the earnings growth over the next few quarters or couple of years. And if I could get a sense of the interest cost, then that helps.

22 Sandhya J: We've announced projects so far for about INR 3,000 crores. Roughly, 80% of that we will borrow. So, and we will repay over 10-to-15-year time period. So that can help you forecast the interest costs.

Rajit: Right. So, we are already at INR 2,100 crores if I do not count the lease liabilities. Right? I mean, that is as of March '25, which is a combination of long term and short-term debt. So, you're saying over the next three years, it'll only go up by 300 crores. And I'm assuming you said INR 2,400 crores with excluding lease liabilities. I mean, still I'm a little unclear on this, if you can help me.

Sandhya J: It is incremental. If you go to the Capex slide, which we have given our expansion number. So, we have announced almost INR 3,000 crores of incremental Capex.

Rajit: Right. So, INR 2,400 crores will be over and above the number I have as on March '25.

Sandhya J: Yes, but we will also be repaying some of the existing loans.

Rajit: Exactly. So that's why I asked if you have a sense of whether ...

Nishant Singh: The INR 2,400 crores will come on the books in the next three years, not eight years. Plus, a bit of regular capex of around INR 300-350 crores every year. So, there'll be addition of around INR 800 to 1000 crores of gross debt every year, minus the repayment on the existing loans.

Rajit: Understood. Thank you, sir.

Nishant Singh: Shreyans, can we have your question, please?

Shreyans: Yeah. Hi. So, I see on the Capex slide that, our FY26 projection for greenfield projects is about INR 420 crores, and we have only done about 5 crores for that in this quarter. So, are we on track for this year for that projection? I mean, because the difference is so huge, so that's why I'm asking.

Viren Shetty: Yeah. We are running a little behind because of the rainy season. A lot of the contractors and a lot of the work has not been able to start on time, but a lot of them have geared up, and they should catch up over the next three quarters.

23

Shreyans: Okay. So, still seems on track for the INR 420 crore s target, right.

Viren Shetty: There may be a slight overrun as usually is the case with construction projects. Our expertise is in the patient care. Construction is, very, very difficult for us to understand. But we're hopeful that these things work out .

Shreyans: Understand. Thank you very much.

Nishant: Ravi, can we have your question, please? Ravikiran, can we have your question, please? Okay. So, we'll move to Deekshant.

Deekshant : Hello, sir. Congratulations on the results. First question is, is this margin a slight contraction margin that we have seen . Is this seasonality only, or is there any other sort of impact that has given us these margins?

Sandhya : You are talking about India or you're talking about consol?

Deekshant : Consol, ma'am.

Sandhya J: Okay. So, consol, I think, was explained. The dilution is coming from Cayman. And, as far as India is concerned, actually we have expanded margins. And even in Cayman the hospital business is stable on margins. The dilution has come from integrated care, which Anesh explained a couple of times during the call.

Deekshant : Thank you. Secondly is, we have started to expand our insurance business right now. So , I know that we have talked about our direct distribution thought process. But, since we're ramping up right now, could you and, also, there has been now word -of-mouth going around with people. So, what's the further expansion idea on distribution strategy that we are working with right now?

Ravi Vishwanath: So, look, I mean, I think, our focus right now is still pretty early days , right. And as you know, direct distribution takes time to build. So that's our focus. And there are multiple avenues to distribute directly. So , our focus right now is on doing that. Having said that, we would be , we are open and, you know, always talking to and considering various other options that can help us in our growth. But as I said to an earlier question, our focus is on doing this in a way that, you know, the value remains with the customer in the form of lower prices and better service. And, also, a key part of our entire program is that we want to be actively involved with the customer and in helping the customer manage their health, etcetera. So, making sure we have that direct relationship with our customer is very, very important. So, you know, as long as we're able to do those things, we would be open to partner with people. But our focus is on building our direct distribution through multiple avenues.

Deekshant: So, historically, insurance I'm sure you have been asked this question a lot of times. But, historically, insurance is a business of gathering enough pool in order to manage the risk correctly. And, of course, our expertise being a hospital provider, we are able to manage risk better. But at what number or is there any metric that you can share - at this metric we would be breaking even and we would be doing much better so that we can push the accelerator a little harder?

Ravi Vishwanath: Yeah. I mean, surely , I mean, obviously, internally, we have those metrics that we track very regularly, but I think it's a little bit early to start talking about that, you know, you know, publicly. One thing that is worth noting is that our business model is substantially different from the traditional players. And what that means also is that while we can give customers better value and, hopefully, better service and partner with them on their health care, it also does mean that we need fewer lives to break even than a traditional, very, distributor-led type of model , right? So , it's really about balancing those things back, you know, balancing both things and, you know, making sure that where we do partner with , that distribution cost is acceptable to us. And because I said, our goal is to save it, pass it back to the customer in the form of lower premiums, better service,

and also be really proactively managing their health. So, that's our focus. And our model is very, very different from the traditional models. You can't really compare the two.

Deekshant : Can you share the number of policyholders as of quarter end?

Ravi Vishwanath: About 6,000.

Deekshant : About 6,000? Okay. Perfect.

Lastly, on Cayman Islands, so we already are now operational, and we are getting better sort of demand input that we had expected as was told on the call. So, what's the next hospital 25 that we're looking in the Mediterranean Islands? What's the sort of goal of expansion? Because cash flow will start pouring in pretty soon now in a couple of quarters, right?

Anesh Shetty : Yeah. So, we're in the Caribbeans, so we've, we've made a yeah. You know, we did make a small investment in a large health system in The Bahamas. That was a couple of quarters ago. We continue to evaluate that opportunity. You know, nothing to announce as of as of now. It's tough to find, you know, similar geographies. We continue to do so and explore all options both in the Caribbean and elsewhere, but there's nothing to talk about now.

Deekshant : If I can just expand a little bit here on the question, sir. A lot of opportunity there is people flying from nearby countries to get better health care, and we have built that reputation in Cayman over time. So, do you think that the opportunity with the cash flow that we would be getting, we are at that right moment right now that we can start pushing for new facilities this year, next year, or is it too open ended right now?

Anesh Shetty: Oh, we definitely want to. It's just a question of when a suitable opportunity arises because for us, you know, the first port of call is, essentially our home is the world's most attractive health care market, which is India. So, but we already have plans there. They're already on their trajectory. So, for us to allocate capital anywhere else, it has to, you know, pass a very, very high bar, which is the opportunities in India. So, it's not easy. We say no to, we've said no to all of them. There have been many potential opportunities. But let's see what happens. We definitely would like to do something, but let's see.

Deekshant: Okay. Just one last follow-up. How are things looking in our Mumbai facility? We were looking to ramp it up from a pediatric care to a full, fledged hospital.

Viren Shetty: We're in discussion with the trustees. There's been a lot of positive momentum, and we're confident it should happen soon, but it's not in a position where we can publicly state that it'll definitely happen in Q2, Q3, Q4. But it is something we know should happen sometime this year.

Deekshant: So, since the last two quarters this has been the conversation. That's why I was asking you again because of that.

Viren Shetty: That's a fair point. And even once we do get permission, it will take time to ramp up as well. We need to hire a lot more doctors and build up the marketing plan to run this as adult multi - 26 specialty. So, we are quite disappointed with the progress that has happened, but it's something we're working very hard to try and rectify.

Deekshant: Yes, sir. I'm sure you'd be doing amazing at it. It's a great market, and you are great people to run it.

Viren Shetty: Thank you, Deekshant.

Viren Shetty : I think Ravi Kiran, figured out his hand raise, I think. Could you, Ravi, if you can ask your question?

Ravi Kiran: Oh, yeah. Hi. So, this is regarding the oncology initiative. Just thinking about it from a patient perspective. What does it mean coming to, Naraya na Hrudayalaya, as against let's say, approaching a dedicated specialized treatment provider such as, you know, Health Care Global or whatever. I mean, I imagine lower cost would be one of the factors. But apart from that, in terms of the treatment, especially,

what does it mean for them? How do we compare?

Viren Shetty: So, if you are comparing us to HCG, HCG is a dedicated oncology provider versus us. For a pure oncology patient, there would be no difference in terms of, maybe our facilities will be larger, maybe our infrastructure may be slightly better than some hospitals. But other than that in terms of the treatment being offered, we have, because we are multi-specialty and we offer the entire range, we are able to afford much more advanced equipment. As well as cancer being multisite, it would be useful in the early detection of a lot of cancerous tumors. So, we also have huge amounts of investment in bone marrow treatment, in CAR T cell therapy, and lot of advanced genetic screening. So, a dedicated cancer setup can do all of these things. It's just that our DNA is in being multi-specialty. And so for patients, they get to have a much more comprehensive view of their health.

Ravi Kiran: Okay. But in terms of the level of treatment, etc, you would say it's on par or better than even a specialized cancer.

Dr. Emmanuel Rupert : Yeah. It's I mean, we are closely working with them on the protocols, but they're on par with what we are doing.

Ravi Kiran: Okay. Thank you. That's helpful.

Viren Shetty: Thanks, Ravi.

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Nishant: Vedant, your question, please.

Vedant: Yeah. Hi. Thank you so much for taking my question. So , there's been news around private hospitals not receiving Ayushman Bharat's payment on time. How are things looking for us? It also seems that our contribution from schemes has been going down. It's around 18%. So , will it be same going forward, or is it just a quarter -on- quarter change?

Sandhya J: So there are challenges with respect to A yushman Bharat payments. For some hospitals some quarters are a challenge. In some other hospitals, other states, maybe different quarters. So , it's not across the country at the same time, but at different points in time, there are cash crunches with the government, and therefore, we do have payment challenges. Our aspiration is definitely to continuously improve the pay or mix, but it has to be set in the context that we also want to be accessible available to our patients. So we will try to continue to improve pay or mix, at the same time not compromising on the fact that we want to make high quality care accessible to everyone.

Vedant: Okay. Got it. And also, if it is possible that how much percentage Ayushman Bharat contributes to our scheme patient mix .

Sandhya J: Overall, our scheme patient mix is 19%.

Vedant: No, no. How much how much does Ayushman Bharat contribute ?

Sandhya J: Roughly less than half.

Vedant: Okay , it's not major.

Sandhya J: It is major because it is still a big number for us. But, yes, roughly less than half, we can say.

Vedant: Of the scheme patients - of the 19%?

Sandhya J: Yes. Yes.

Vedant: Okay. Got it. Got it. Thank you so much.

28 Nishant: Ravi and Deekshan t, do you have still any further questions or should we move to the chat questions in the chat?

Ravi Kiran: Yeah. Just last one here. You have mentioned the 6000 number for our insurance, and you have mentioned this in passing on a couple of things about insurance. But still do you think that there is a number where we can see breakeven? Because I don't think so we'd be breakeven right now at all.

Viren Shetty: No, we won't be. 6,000 of the current number of policyholders, but we wouldn't get into the forward projections of how much it would take. Suffice to say, we still have a couple of years ' worth of work to figure out the right model. You raised a fairly good point about the distributor led model. There are pluses and minuses on that, as well as the slow ramp up because we're getting a lot of our systems in place. But we do intend to reach t hat operational breakeven soon. The m

odel will evolve to be able to achieve that.

Ravi Kiran: Sir, there is a huge market of people who are over the 50 -60 age limit. Is that the market that we are focusing on? Because they are a very high -risk market, but we want to cover all people. So, is that the market that we are looking at , at all?

Viren Shetty: We are looking at it. It's just that the pricing would not be able to meet the requirements because the older cohorts, as you're aware, will have very high health care utilization. And to be able to price products just for them means that they would find it very unaffordable. So, we'd have to create a mix of products that cater to both younger cohorts as well as older ones.

Ravi Kiran: So, there would be mixed policies to adjust our risk premiums.

Viren Shetty: Yeah.

Ravi Kiran: Okay. Got it. Thank you, sir.

Viren Shetty: Thanks. I'll just talk through the chat questions, and I'll ask the people to answer them. So, the first one, how much burn we can expect from health insurance business? We haven't called out any specifics, but we've indicated earlier about how much we intend to invest in both the clinic and the insurance business.

29 There's a question of any new branch name which has been started last quarter? We haven't started any new branches. I think they may have been referring to the Cayman New Hospital, which is Health City in Camana Bay, as well as the Cayman Integrated Care which is our which is our insurance plan.

There's a question from Suruchi about price capping from the government similar to the drug price control. What are our thoughts on the possibility of price capping for services in the hospitals? To this, I think it's come up several times in the past as well. The probability of such a thing happening is not zero, but it is not 100% either. This is a highly regulated industry. In a lot of advanced western countries, the realizations, depend a lot on the large payers. So Ayushman prices, for example, have been capped from the day of launch. The CGHS, ESI, ECHS have, also the price has not changed in the past nearly fifteen years. And the large private insurers also have a body that allows them to negotiate as a group. So, body by body, the large payers are working out ways to control the prices and hospitals are at the receiving end of that. But as far as it comes to one uniform price for heart surgery across the country for everyone, depending on how much they can pay, doesn't seem likely. But, again, the chance of that happening are not zero.

Next question is asked about the maintenance Capex there in Cayman. Sandhya could answer.

Sandhya J: It is given in our expansion slide, Slide 14 of our investor deck. We have budgeted, for INR 457 million for Cayman maintenance Capex, and so far, we have incurred INR 158 million.

Viren Shetty: The next one is Capex plan and bed additions for the next five years. We've projected out the Capex budget that's in the investor deck.

Viren Shetty: There's a question asked two times about other administrative expenses increasing. Sandhya can give the explanation on that.

Sandhya J: Yes. Roughly half of that increase is coming, because of our insurance business. Because the way consolidation happens for insurance businesses, all the costs and the claims that are paid out from the insurance business, it gets consolidated as other expenses. So, roughly half of that INR 100 crore increase that you're seeing is the insurance business P & L, essentially, all the costs of the insurance business. The other half are genuine increases in costs that we have seen. A big chunk is because of the minimum wages' revision across states. Now that has an impact on our cost lines, like housekeeping, security, etc. So that's one, chunk of costs.

The second chunk of cost is that we've had increase in our hospital, equipment R&M and also, the costs relating to the robotic investments we have made. There is an increased cost of the robotic investment, so that also is coming in as the second big chunk.

The third chunk is a more reversible chunk which is coming from PDD, which is provision for doubtful debts. That's an accounting entry that the auditors take based on the receivables. Q1 is generally a slow quarter in terms of collection. So, therefore, our PDD provisions go up, but then they reverse. We've hardly taken any write off in our books for doubtful debt. So, we will recover this money, but there is a timing issue there, and that's adding another about INR 10 crore on the provisions.

Other than that, there are small, small increases across different expense lines, including CSR, because CSR runs on a three -year average profit basis. And we had the effect of the COVID year on the base till last year, so that's gone. So, therefore, our three -year average profit has gone up, and therefore, our CSR commitments have also gone up. So that's another item which is increasing.

Viren Shetty: Last two questions on the chat are on Cayman. First question is on the CIHL. Did the impact come from Q1 FY26 ? And, can you share the previous quarter numbers if applicable? Anesh, I don't believe we break that up.

Anesh Shetty: Yeah. It's not material. Yeah , we don't break it. It wasn't material of any kind.

Viren Shetty: And, the last question has been asked in several forms, multiple times in the past. How much would the cannibalization happen between the new block in Camana Bay versus the old hospital?

Anesh Shetty: Yeah. There's no cannibalization because it's a single P&L. The teams are common. The cost structure s are same. Patients freely move between one to the other. So , they are not two separate businesses.

Viren Shetty: So, that answers all the chat questions.

31 Nishant: I think we still have some questions from Ravi Kiran, Di kshant, and Ravindra.

Viren Shetty: Do y ou'll have questions?

Nishant: Yeah. I think Ravindra has not asked any question before.

Ravindra: Yes, yes, I want to ask one question, sir.

Viren Shetty: Go ahead, please.

Ravindra: Am I audible?

Nishant Singh : Yeah. Yeah.

Ravindra: I'm looking at the slide of the presentation, slide number nine. So, I just want to get some clarification or insight. The inpatient average revenue in particular to Bangalore is INR 231K. And, rest all I think it's more or less an average, around INR 120K around. So, if I average of that out, it's around INR 120K. So, what should I infer from this slide ? Are we doing some kind of a specialized treatment particularly in Bangalore so that the revenue is INR 231K only in Bangalore? That's one question.

Viren Shetty: I can answer that.

Ravindra: Because you're showing us an average. So , how there can be a huge divergence in the average revenue per patient? That's my question.

Viren Shetty: Specifically , about Bangalore, Bangalore consists of the Health City which is our actual centre.

Ravindra: I'm from Bangalore, I'm from Bangalore, so that that's fine. But I want to know the huge divergence because INR 231K , the average when you're taking average, so rest all you see there's southern peripheral , Calcutta, East ern, Western , there is, like, some kind of an evenness over there. So , only in Bangalore when you're averaging out, how we can get in such a huge divergence, which is almost 100% divergence.

Viren Shetty: When you take averages into consideration, then things start to happen. The other hospital we have across the country so in eastern India, the average realizations are less. The GDP per 32 capita is less. Most of the hospital s we offer are in tier two towns with the exception of our hospitals in Delhi. So even if you compare Delhi against Bangalore, Bangalore is a 25 year old cent re of excel

lence with multiple surgical robots, very advanced onco -therapies with a massive bone marrow transplant unit that does very cutting -edge work, which the other hospitals don't have. So , the case mix is disproportionately in favor of long stay, very high realization procedures.

Ravindra: So, what I can infer from this that we are doing some kind of a specialized treatment particularly in Bangalore. Correct?

Viren Shetty: To a much larger extent than we are able to do simply because of the size. The Bangalore Hospital is also close to 1,800 beds.

Ravindra: See, if I compare Calcutta, the capacity of which is 1453 . Bangalore is 1503 . There's not much difference.

Viren Shetty: Calcutta as I said, is a city where the realizations are lower. The patient payor mix is also lower, and the patient profile and payor base in Calcutta is much different from Bangalore.

Ravindra: But, I don't know. I'm not that satisfied with the answer that the average can be increased in other hospitals. That's what if I go with Bangalore as a case study, there's a huge scope to increase the revenue in other, you know, places as such if we can implement the same kind of treatment available in other places as well.

Viren Shetty: That's a fair point, and that's something we will work on over the coming years.

Ravindra: We have the capacity in Calcutta, for example. The capacity is there. So only maybe we are not doing a particular advanced treatment over there maybe. So , it may require some of a fixed, you know, equipment as such. So, there's a huge scope because we are already having the inventory of beds there, but we are not utilizing in a better way if I compare to Bangalore.

Viren Shetty: Yes.

Ravindra: So, rather than expanding in a newer hospital, why can't we use the hospital which are available there where there is a capacity? Why can't we do the same advanced treatments in other hospitals?

33

Viren Shetty: Buildings aren't fit for purpose, but it is a fair point and something we'll take into consideration.

Ravindra: Thanks. And, I have the other question on other expenses as well. So , I think it's more or less answered, but not to my satisfaction.

Viren Shetty: Okay, t hank you, Ravindra .

Nishant : Yes, Niraj. Please go ahead.

Niraj: Aneshji, just your last clarification what you did. So , I have stayed both in Bangalore and Calcutta. So , I very well agree with your point that the customer profile in Bangalore and Calcutta is totally different. So just that and because I stayed in both the places. So , you are right, actually.

Viren Shetty: Thanks, Niraj.

Nishant: So, Ravindra , do you still have any more questions?

Ravindra: That's it. Thanks.

Viren Shetty: Thank you.

Nishant: So, I think with that, we've got no more questions. Thank you, everyone. With that, we will conclude our session. Thanks, everyone, for your active participation as always .
End of transcript

Call: Nov 2025

Date of submission: November 24, 2025

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code –539551(EQ), 975516 & 976418 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051

Scrip Code - NH

Dear Sir / Madam,

Sub: Transcript of Earnings Call for the quarter and half year ended September 30, 2025

In relation to earnings call of the Company held on Monday, November 17, 2025 for the quarter and half year ended September 30, 2025, please find attached the transcript of the said Earnings Call.

We wish to inform you that the Earnings Call transcript is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earnings-call-audio-and-transcripts>.

This is for your information and records.

Thanking you,

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Q2 FY26 Earnings Conference Call"

November 17, 2025

NH MANAGEMENT TEAM:

MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – CHIEF FINANCIAL OFFICER

MR. R. VENKATESH – CHIEF OPERATING OFFICER , EAST AND

SOUTH REGIONS

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS
SUBSIDIARY HCCI

MR. RAVI VISHWANATH – CHIEF EXECUTIVE OFFICER , NHIC

MR. NISHANT SINGH – VICE PRESIDENT , FINANCE , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. VIVEK AGARWAL , SENIOR MANAGER , IR FUNCTION

2 TRANSCRIPT

Nishant Singh: Good afternoon , everyone. Go I am Nishant Singh and I welcome you all to the Q2 of F Y26 Earnings Call of Narayana Hrudayalaya Limited. To discuss our performance and address all your queries today, we have with us Mr. Viren Shetty, our Vice Chairman, Dr. Emmanuel Rupert, our CEO and MD, Mr. Sandhya Jayaraman, our Group CFO, Mr. Venkatesh, our Group COO, Dr. Anesh Shetty, our MD of our overseas business, Mr. Ravi Vishwanath, CEO of NHIC and Vivek Agarwal, Senior Manager IR Function. Before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website as well as on the stock exchange at a later date. We would also like to remind you that everything that is being said on this call that reflects any outlook for the future or which can be construed as a forward-looking statement must be viewed in conjunction with the uncertainties and risks that they face.

We would also want to highlight that for UK, we will not be in a position to answer any of the questions for this quarter. We will consolidate the numbers by Q3 and then we can have a call on the UK asset as well. For the acquisition matters, we have already had a call in the recent past. So, we would just request to skip any questions on UK for now.

With that now, we would start the Q&A session. I request everyone to please use the raise - hand features to start posing their questions. Thank you.

Prithvi: Congrats on the good set of numbers. Let me begin with Cayman first. Anesh, this seems to be doing exceptionally well with 70% kind of revenue growth. So, if I have to look at insurance, the revenue has doubled compared to the previous quarter. Could you explain how the product is being received? What is the market size ? And what are we aspiring for with the market share in the medium term?

Anesh Shetty: Yeah, Prithvi. So, the response to the insurance product has been very, very good. We have most of the large employers who have already moved to us or are actively considering moving. That's why we're seeing the very aggressive revenue growth. It will take some time for us to decide on what is the market share and where we want to stabilize. We may take a calibrated call depending on how the insurance product is integrating with the hospital. Maybe in about 2-4 quarters, we'll have a view on what the steady state will look like. But for now, it's progressing very well.

Prithvi: Is it possible to quantify the market size in Cayman for the insurance?

Anesh Shetty: So, that information is available on the Monetary Authority website. It will be approximately , including the government and private sector , it will be closer to about USD 300-350 million or so. But that does include the government insurance as well. So, an adjustment needs to be made for that, which can be done.

Prithvi: Understood. Now, if I have to look at the insurance losses, losses have come down significantly in this quarter. Given the way the scale -up is happening, should we expect the insurance business to breakeven in Q3 and move to profits from Q4?

Anesh Shetty: Bit tough to say. So, as the size of the book grows larger, definitely the underwriting performance will stabilize. Having said that, quarter -on-quarter movements in loss ratio is a little unreliable in the insurance business. So, let's look at a rolling 3-4 quarter performance and then take a call. But yes, as the size of the book is growing, it is stabilizing, definitely. But too early to say whether, that we will break even and then profits from Q4, given the variation quarter -on-quarter for underwriting performance.

Prithvi: And on the hospital side, after commissioning of the new hospital, the revenue went up from \$25 million to \$40 million at this point of time. Are we largely done with a complete scale -up and then should we expect only high single -digit growth rate on this base? Or do you think there is some more room for higher growth in the next few quarters before getting moderated to high single -digit?

Anesh Shetty: So, between pre - and post -Camana Bay Hospital, just if you look at the volume terms, we're looking at 50% increase in discharges, little more than that in outpatients and daycare, etc. So, we have seen a significant jump post the commissioning. I think there is still a couple of quarters of growth left before it starts to moderate and we focus on restoring the profitability

to where it was. But let's see how it plays out. We have started a few more services recently. They're still playing out. Let's re-evaluate maybe in a quarter or two. And then, as we've always said, it will go into a sort of a high single-digit. But then, earnings growth will be different because we have some levers to improve our cost structure still. But from a revenue side, let's take a call in one or two quarters.

Prithvi: Okay. So, just a follow-up on this. When you say some more levers to increase the cost structure, I think the hospital margins are roughly around 40% to 43%. Do you see a further upside to the margin number?

Anesh Shetty: It will depend on where we stabilize from a revenue standpoint, because right now we have certain costs to grow business and to add new services, etc., which we are incurring the cost and the revenue is not yet playing out. Maybe in two quarters or so, we'll be able to have a better view on this. Once we've sort of commissioned all the services, we know the trajectory they're heading, they're in sort of steady state with just incremental growth, then we can consider that.

Prithvi: Moving on to the India business, even here we're seeing a decent scale-up in the clinics and insurance revenue. And for the last couple of quarters, losses have been declining. Can we expect this trend to continue going forward, and hopefully FY27 will be a much lower loss number for the clinics and insurance?

Ravi Vishawanath: Sure. So, yeah, thanks, Prithvi. Yeah, we've had good momentum on both the clinics and the insurance business. I think on the insurance business, while our retail business is growing, we've also had a lot of incoming interest on our SME offering, which integrates our clinic and our insurance offerings. And so, we expect that to grow. And obviously, with insurance, as we grow, we would be seeing better overall results and so on and so forth.

In terms of clinic business as well, yeah, the clinics themselves are performing well. And I think it'll be a balance between the profitability of the existing clinics versus future growth and investments in those growth because clinics do take a little bit of time to break even. So, I think you'll see a balance of those two things in the coming years and quarters. But yeah, I do expect that. I mean, luckily touch wood, the momentum is heading very much in the right direction. So, we're hoping that that trend will continue.

Prithvi: Okay. And just one last question on the Mumbai hospital. Are we still just breaking even or did we start making profits? And any update on the conversion of the hospital?

Viren Shetty: I'll answer the update on the conversion. Venkatesh can talk about the performance. We are in discussions with trust. We do get, there are few children who have grown up, and they had congenital conditions. But when we operate them, they are adults. So those few cases have been done here and there. And we will slowly start opening up the birthing program. But as of now, it is something we're working slowly towards. On breakeven, Venkatesh?

R. Venkatesh: See, for Mumbai, it has been a little bit of up and down in terms of its performances. It was somewhere in the early single digit negative EBITDA for this quarter. But what we've seen is in October, the trend has

improved and the performance has been very good in October.

Hopefully, with the trend continuing as it is indicated in November, we should have a much better and a positive Q3 in Mumbai.

Prithvi: Thank you. That's all from me.

Viren Shetty: Nitin, can you go ahead?

Nitin Agarwal: Thanks for the question and congratulations to the team for a pretty solid set of numbers.

Viren, on the India business, I think we've had a 20% EBITDA growth this quarter, which is quite remarkable. And in a broader sense, I think our growth is coming through despite we not really adding too many beds, literally, over the last few quarters. So two things, one is, what really is driving this, our ability to grow profitability without adding capacity? And how much scope do we have? Is there enough scope still left in the current network to extract more juice out of it from a profitability perspective?

Viren Shetty: Nitin, just to clarify, we've reduced the total number of beds we run in India from the time we listed till today. But on the scope for capacity, I'll ask Venkatesh and Dr. Rupert to answer.

R. Venkatesh: We are continuing to work on our optimization. From the last six to eight quarters, we've been working hard on that and things are working out well for us and we continue to do that.

But what we have seen here in this particular quarter is that, in our flagships, we've seen the impact of our transformation initiatives, which we've been working on in the last two years.

It's kicking in positively. We've witnessed patients opting for higher configuration beds, resulting in increasing in the realization, keeping the occupancy intact. Plus, coupled with technologies, we've also worked on case-mix, high-end work, robotic cardiac surgeries to keep us apart and define new benchmarks, which has also led to a higher realization in revenue achievements. So this, even with the volume being the same or stagnated at this point of time, realizations have actually helped in substantially increasing on the EBITDAs.

And even for the rest of the regions, rest of the units across the Group, we have been focusing constantly on the payer-mix optimization. With this payer-mix optimization, we've seen improved realization across hospitals. While we're still working on catching up on these

numbers to improve payer s, the substantial incremental realization that you've got from these payer s have boosted up your realization and that has improved the EBITDAs by a big amount. Our endeavor will be to maintain these margins and even try to improve, obviously subject to cyclical issues and adventures that may come up, but we remain positive on this. Nitin Agarwal: And, from a capacity which we have in the current network, do we have enough to continue to grow in the business, both through the optimization measures that you have for the next couple of years , before the newer beds begin to come through?

6 Viren Shetty: Yeah, Dr. Rupert will answer this.

Dr. Rupert: Yes, so just like what Venkatesh had mentioned, we have been slowly ramping up the niche work and mainly in the subspecialty. Venkatesh did mention about the robotic cardiac surgery. In the month of September, we did 97 robotic cardiac surgeries and in the whole quarter we did around close to 200. This is a very large number, and all this pushes the niche work that keeps going up. This is just one example of a subspecialty work that is constantly growing and increasing in number , and we are slowly ramping this up across the entire network. And we are fairly confident , this ability for us to do more and more niche work will always be there within the existing bed capacity and the existing infrastructure that we have built.

Nitin Agarwal: Okay. Thank you so much.

Viren Shetty: Thanks, Nitin. Sunaina, if we can have your question.

Sunaina C: Yeah, thank you so much. So, I'm speaking from behalf of Chola Securit

ies. I just have one

question. This is relating to the Cayman Islands operations that are there. So, like it's been reported for the India business that the margin is about 23.8% for this quarter. Would you be able to talk a bit about the margins that are seen in the Cayman Islands business?

Anesh Shetty: So, Sunaina, that information you should be able to derive by looking at the console and adjusting for the India margin, which is separately reported. But just to give you some broader guidance, the hospital does about 43 -44% , here and there , in that range every quarter. That's the hospital business. The insurance company is yet to break even or just slightly broken even this quarter. We've separately disclosed that in a footnote on the same slide , and you'd be able to derive a console figure for that.

It's a little tricky to derive, I mean, to talk about a console because they're two very different businesses ; insurance and the hospital. But if you add it up, you should get to what you're looking for.

Sunaina C: Got it. Thank you so much.

Nishant Singh : Vinay , can we have your question, please?

Vinay N: Yeah. Sorry. Just one thought. When I'm looking at your Slide No. 8 , your ICU occupied days have gone up, ICU bed occupied days. And your patient footfalls have also gone up , while the ALOS is the same. But , yet average revenue per patient is also more or less the same as Q1.

7 Is that the efficiency kicking in? Is it more high -ticket surgeries getting done in the same time?

What exactly is driving this?

Dr. Rupert: Yeah. I mean, all of the points we mentioned are all kicking up here because some of the niche work obviously has less length of stays, but also there are certain very complicated work which increases the overall length of stay. Though we have marginally come down on the length of stay, but a major impact will be there. But we are pushing more and more for short stay and the day care surgeries. You would have seen that a significant number of our ... even our coronary angiograms are done as a short stay. That means they get moved out of the hospital within four to six hours. So, we are constantly working towards , wherever it is possible for us to keep the patient safe , at the same time, reduce the length of stay to constantly move towards in that direction. But there are certain subset of patients, especially the elderly and those who need medical management who do require a long length of stay. So when you start averaging it out, the movement of length of stays, the A -losses will not be moving in a very substantial manner like that. But our hope is that we should be coming down to somewhere around 3.9 or something like that.

Vinay N: Yeah, but I'm a little intrigued by the average revenue per patient remaining the same with such a great increase in your revenues.

Viren Shetty: Between the quarter, we don't take much by way of price increases. And every patient who comes for a high value procedure has to be balanced out against those that are very very affordable. We cater to nearly 18-20% percent of our revenues to government patients. The reimbursement for that haven't changed in more than 11 years. So, when you average it out, it does have a bit of a flattening effect.

Vinay N: Okay, just last question . I can see some international business returning. Where exactly is it coming from ... the operation?

R. Venkatesh: On the international, our focus will continue to be there on the domestic only. We've done very well in the last eight quarters on the domestic business. And we're very confident because we've seen a lot of tractions. International business is not our main area of focus at this stage. There has been a lot of fluctuations. The issue started last year Q3. And as we see,

we should look at somehow stabilizing in terms of the volumes in Q3 of this year. From which we can see and set a base for that . But

having said that, we will continue to focus on our domestic numbers going forward for growth.

8 Viren Shetty: The international business is primarily Bangladesh. But this is a business we're looking to wind down over the next couple of years.

Vinay N: Yeah, that's why I was a little surprised by the 27% Q oQ increase in international business.

Viren Shetty: That's purely on Bangladesh.

Vinay N: Okay, thank you very much. All the best for your next quarter.

Viren Shetty: Thanks, Vinay. Om prakash, can we get your question?

Omprakash D: Hi, thanks for the opportunity. So, I have mainly a question around this Cayman area, right?

So, this quarter, we have seen excellent growth in terms of revenue and EBITDA margin as well. So, if it is possible for you to give some guidance on how this is going to pan out, maybe not just in the short term, medium term as well. The question is also on a comparative basis of the population . As given in India domestic business , you have a lot of population where that would be a bit of limited in that region. So, I just want to understand that how this growth is going to be sustainable in medium to long term basis.

Anesh Shetty: Yeah, Om Prakash. So, the first part is related to the hospitals . As we've said before as well , we've commissioned the new hospitals , so we've seen this tremendous revenue growth. After a few quarters, we will have a better view of where that will settle down and slow down to a single digit sort of steady state growth.

The insurance business is still in the early days. The market is just getting started. There is substantial room to grow. Having said that, we will take a call , after a few quarters , whether we want to keep growing that business, the insurance business, or we want to maintain it at a particular level. That will depend on the strategic fit with our consolidated integrated care plan in that market. But there is room to grow in the insurance business. Like I said, the market size is about USD 350 million or so , including the government insurance. And we're still in the first three quarters.

Omprakash D: Got it. Thanks, An esh. All the best.

Viren Shetty: Thanks. Shivam, can we have your question?

Shivam Singh: I have a question regarding the acquisition we are doing. It is more related to finance rather than operations. Are you okay answering?

9 Viren Shetty: Yeah. I'll tell you what. Let's finish all the Q2 performance -related questions. Then we can come back to this if we have time. Would that suit?

Shivam Singh: Yeah. Sure. Thank you.

Viren Shetty: Thank you. Raj it, your question?

Rajit Aggarwal: Yeah. Thank you. Just a small question on the digital transformation initiatives. So, there is a mention of service rendering management rolled out in Kenya and Saudi compliant patient portal enabled. So, I mean, are we looking at doing something more in these regions? How did these two regions happen? Can you throw some light on this?

Viren Shetty: Yeah. We created two SPVs called Med ha AI and At hma Health Tech. These are for commercializing the investments that we've made in our significant software expenditures.

So as part of the work, a lot of hospitals come to us and ask if we can roll out the systems that we have in their clinics as well. So, the Kenya and Saudi are these small contracts that we've taken. The revenue is not that material. This is a business that we like to encourage and grow because we do have very good technology. And those that want to adopt it, we would not say no to that. But I wouldn't say that this would be a massive and significant contributor to the overall top line. It's something we will pursue. But running the hospital is our main business , and building extremely good technology for our doctors and nurses is the purpose of our ventures.

Rajit Aggarwal: Right. So, it was more like an inward query rather than any sales per

sonnel reaching out to

these hospitals in these regions. I'm just trying to figure out how did these two regions happened . And if we are looking to expand within Africa or within Middle East or something like that, even if it just means these two products.

Viren Shetty: Yeah. We are looking to expand, obviously, any business, good business. We've developed quite a reputation for having extremely good and affordable technology that scales quickly , built on the latest platform. And a lot of people who have worked for us have moved into those regions and they know how good our software systems are. So, they inform the CIOs and CEOs of those hospitals and they come and we do POCs for them. There is a slight amount of effort that we take in doing the POCs and expl aining the products. But it will grow very slowly.

Rajit Aggarwal: All right. Thank you. And just one more request. Even I had one question on the acquisition of Practice Plus. So, at the end of it, if you would allow that ? Thank you.

10 Viren Shetty: Yeah. Sure. Thanks. Deekshant , any questions relating to Q2?

Deekshant B: Yeah. So, firstly is on Caymans. Is it right that we have actually seen a million dollars of dip in our revenues? 42.6 versus 41.6. What would be the reason for this dip in the revenue?

Anesh Shetty: Deekshant, are you referring to the hospital revenue quarter-on-quarter?

Deekshant B: Yes, sir. Yeah.

Anesh Shetty: No. So, I mean, quarter-on-quarter, as if you've been following Cayman for some time, there will always be variations like that. We recommend looking at a rolling three or four quarter average to get a sense of the trend. But specifically, Q2 is the season for school holidays, summer vacations, etc. Even historically, every year, if you see, that tends to be a slower quarter. You know, USD 1-2 million up or down is we don't think would be indicative of any long term trend. Please consider a rolling few quarters.

Deekshant B: Sir, I completely understand. But the reason for asking this is last quarter, we have just ramped up our second hospital in Cayman. And ideally, you should now start seeing that top line kicking in just because of some sort of utilization being better. Is that a fair assumption for us? We are still doing very well. But what is the indicative outlook that we should have for the coming, let's say, 2-3 quarters?

Anesh Shetty: Yeah. So, the new hospital was operational from January 2025. So Q4 actually is when you would have seen the first big jump in hospital revenue. And then Q1 and Q2 is that continuing. We've commissioned 90% of the departments in the new hospital, or little more than that. There are a few that have just been commissioned recently that will start ramping up. So obviously, the biggest jump you would have seen is from December of last year to Jan, Feb, March of this year. And after that, the quarter-on-quarter trend that we are seeing will continue until these new services start growing into a maturity phase.

Deekshant B: So, is it fair to say that we are now being operationally more efficient and now it's directly falling into our net profits? So, from now on, whatever we see, our larger operating margin will inevitably be an outcome. Does that make sense?

Anesh Shetty: Yeah, we'll try to refrain from giving that kind of definitive statement. But we still do have room to grow in the hospital revenue front. It just won't happen in the 30%, 25% quarter-on-increase, obviously, that we saw the first time we commissioned the hospital. But there is still room to grow on the revenue front in the core hospital business.

11 Operational efficiencies relating to margin, obviously, will continue. It is a profitable asset, a highly profitable asset that we'll continue to do. Once the revenue starts sort of getting into a steady state, then we will focus on maintaining or trying to increase those margins with further

optimization. But we're not yet there. We're still a while away from tapping out on growth.

Deekshant B: So, second is on the insurance revenues and total business. You have mentioned that we are now reaching that ...

Anesh Shetty: Sorry, Deekshant, there's quite a bit of disturbance at your end. I'm not able to hear you very clearly.

Deekshant B: Is this better, sir?

Anesh Shetty: Okay, go ahead.

Deekshant B: Sorry. So, on the insurance revenues front, this \$9 million, is this the breakeven that we should consider on quarterly? And, what has been the sort of acceleration that we are seeing? Because we have mentioned that we have started to push. So, what sort of outcomes are we seeing on our insurance business now that we are ramping up on sales?

Anesh Shetty: To the first question on the breakeven, please don't consider any one quarter performance in insurance as indicative of a trend. There's a lot of volatility in high value claims or in loss ratios quarter-on-quarter. We highly recommend looking at only a rolling three or four quarter period to get a sense of the sustainable loss ratio of the business. That is tricky to do now because every quarter we are seeing significant growth in the size of the book. So it's still very early quarters. I don't think it will be reliable for some time. It's still a very dynamic growing business. We'll take a call in a few quarters once we have a better idea of where things settle down, and then we'll be able to answer that question.

Deekshant B: So, sir, but on our sales front, now that we have started to accelerate our sales, what kind of response are we seeing and what is the total policies out in force right now?

Anesh Shetty: Yeah. So, I mean, I can only give you broad subjective guidance that we have a lot of interest from the large employers. We've moved many marquee accounts and we continue to have a good, strong sales pipeline. You know, we'll stay away from commenting on any GWP number quarter-by-quarter because that's not helpful. Every quarter we can discuss the further outlook. But there's still room to grow, considerable room to grow, and we'll take a call every two quarters about when we decide to slow that down if we want to.

12 Deekshant B: Sir, sorry to harp on this again, but we have been giving out the number of total policy in force up till the last few quarters.

Anesh Shetty: No, I don't think we say that we talk about the GWP. That maybe you're mixing the India Insurance Integrated Care Business with Cayman. I can hand over the question to Ravi.

Viren Shetty: No, I'll answer this. Cayman is entirely employer sponsored insurance. So, the companies take

it as a large book. India, it's mostly retail. That's why the numbers may be confused.

Anesh Shetty: We definitely don't disclose the number of lives covered or the number of policies for Cayman. What we do disclose is the number of subscribers and number of policies, I think, in India.

Deekshant B: Okay, got it. So, can I ask one more question for Q2?

Anesh Shetty: Yeah, please.

Deekshant B: So, the hundred beds that we are seeing now in Bangalore that might be coming up, what's our progress on that? Because it seems like we are near completion. Can we see it in FY26 itself?

Dr. E. Rupert: Yes, progressing as planned. And I think we should be able to commission by Q1 of FY26.

Deekshant B: That's it, sir. Thank you so much. Best of luck.

Viren Shetty: Thanks, Deekshant. Nitin, you had a follow up?

Nitin Agarwal: Yeah, Viren, I just quickly want to check on, since you mentioned the government scheme part of it. Have you guys done some work on what is the possible impact of the CGH S rate hikes and which likely would be followed by other central agencies on our business?

Viren Shetty: Yeah. Venkatesh, can you answer that? The CGH S rate hike impact?

R. Venkatesh: Yes. The CGH S and the few of the other

corporates which follow the CGH S rate, I mean, the pure CGH S out of those accounts are around 60%. The rate revisions have come into force. What we've seen is that, we expect around 30% benefit on the packages because of this revision. But on the non-packaged part, we would not see any benefit beyond 12% because most of the component on the non-packaged part is the medicines consumables, which is around 55% of the numbers. We won't see any impact because it's on cost-to-cost.

13 All in all, we could see, because of this improvement, around 1% benefit on the monthly revenues and maybe 70 to 80% of it flowing through into the EBITDA. But of course, we also expect revisions of these corporates, which also follow CGH S, eventually in the course of time.

Nitin Agarwal: So, Venkatesh, what would be a ballpark, absolute revenue impact that you can look through on the central government schemes that we have right now?

Viren Shetty: Nishant?

Nishant Singh: See, the ballpark number from pure CGH S will be roughly in the range of INR 2 to 2.5 crore per month. If we have others also to follow up with this, the other payers like PSUs, then probably that number may increase a bit. But for us, for now, the ballpark number is INR 2.5 crore per month increase in pure revenues.

Nitin Agarwal: So about like a INR 30 to 50 crore impact on the revenues or including the other schemes, some similar ballpark.

Nishant Singh: Yeah, that's more or less right. But that is when all of them come in line, and that may happen by say, Q2 of next year.

R. Venkatesh: Yeah, around about that. But there are many moving parts in this overall things. We will have to wait and see how things flow in. But as of now, this is what it looks like.

Nitin Agarwal: Okay, thank you so much.

Viren Shetty: Thanks. Naveen, you have a question?

Naveen R.: First of all, I want to start off by saying how deeply fulfilling it is to be invested in a company that's trying to help people. I want to double-click on the footfall number, and specifically the patient footfalls that is stabilizing in India, right? I see that the revenue per footfall is increasing. But how do you see the footfalls stabilizing?

Viren Shetty: Venkatesh can take this one.

R. Venkatesh: See, what we have done over the last few quarters is, we have been continuing our efforts on focusing more on improving payor-mix and case-mix. As we've already spoken about, when it comes to case-mix, trying to see how we do the high-end surgeries and take us apart from the others and set high benchmarks. At the same time, we are also trying to improve our payor-mix to get better payers into the system, against few where we have concerns in 14 terms of the receivables, the recovery part. It's an ongoing exercise where we are still working hard in terms of catching up with those where we've rationalized on the schemes or the poor payers. It's going to continue. And in a very short period of time, we'll catch up on those numbers and stabilize on this growth. And eventually, from the next two quarters onwards, we will see positive growth in the volumes, alongside the incremental realizations which we have already seen due to this payer-mix. So, as we speak, we are more or less stabilizing on these numbers, and on which we will continue to grow going forward.

Naveen R.: Thank you. Also, do you see an upside in terms of ... like a nationwide branding? Because still, the hospitals are known by their legacy names. And do you think a good recollection in terms of, like, a nationwide brand will help?

Viren Shetty: It will help, definitely. But the thing is, we have taken a lot of hospitals under O&M arrangements that were operating under previous names. For example, in Delhi, we've taken over Dharamshila Cancer Hospital. Everyone knows we run it, but it's still called that. Similarly, the Calcutta Hospital, Rabindra Nath T

agore was a name that predated the creation of the brand. So, that's what it is known as. As long as they know that it's affiliated with us, and we've invested a fair amount in doing up the websites and creating a common identity, but these things do take time.

Naveen R. : Okay, thank you.

Viren Shetty: Thanks, Naveen. Can we have Narayan for the next question?

Narayan D: Hello, sir. And I don't know if Mr. Viren Shetty is there or not. My high regards to you, the way you guys have been managing the hospital and taking care of the health of a lot of people.

Commendable. And I still remember an interview from you on a nationwide news channel, where the anchor asked you that, "Sir, how much do you look for profits?" And you said, "Profits is not the agenda. We want to help people. And that creates an organization with a purpose." I'm happy to be invested here for the organization with the purpose. So, great to see these numbers after a few quarters.

Now, the only question that I have is, why do we come a lot late in terms of results, published results? I think we are the last few. Is it the offshore business for us? That means, the Cayman's accounting that takes time or what happens? Why are we not early in terms of reporting quarterly?

15 Viren Shetty: Narayan, thank you so much for your kind words on why our results are not published earlier, I'll pass it on to Sandhya.

Sandhya J: Yeah, thank you, Narayan, for that feedback. We are also trying to accelerate the speed at which we can publish results. We have ... across countries, also we have to consolidate, and also we have to align all the stakeholders before we publish the results. So, that takes a little bit of time. But this is a very, very meaningful feedback. And we are also internally striving that we should start pulling the timeline of our results forward. We will definitely work on this.

Narayan D: Sure. Thank you for taking the feedback positively. But any specific reasons that we are not able to collate the balance sheets or prepare the consolidated ... what are the problems that you come across when you are trying to do this? Do you have any problems, or it's just that we want to be at this time of the quarter disclosing our results?

Viren Shetty: No, no, it's that we as an organization, we do follow the highest standards of compliance, both in terms of the accuracy of the numbers, as well as the proper accounting of all our receivables and the government accounting that comes in. We have top tier auditors who also scrutinize everything we have line-by-line and we have a large amount of data to sift through. We are in the process of moving towards a more automated system to help out with a lot of these systems there. But as to why it should take so long when other people are able to do it in a much quicker way, I can only say that maybe we're doing a little too much work in presenting the numbers. But a lot of that can be automated and that's something we're going to invest in going forward.

Narayan D: Thank you. That helps. No more questions. Congratulations on the good numbers.

Viren Shetty: Sure. I guess if there are no other questions on the Q2 results, I think we can take that up.

Nishant Singh: We have a question from Rajit now.

Viren Shetty: Sorry Rajit, please go ahead.

Rajit Aggarwal: Sorry, I just raised my hand for the question on Practice Plus.

Viren Shetty: Okay, I guess we can take that up. And, actually the one ... the question that came before you was on from Shivam. I believe it was on the debt financing. Sorry Shivam, could you clarify?

16 Shivam Singh: Yeah. So actually, we financed £150 million of it. So the remaining shortfall, are we planning to fund it via India, or is that entity going to support itself, stretch it out or something like that? Sure.

Sandhya J: Shivam, the GBP 190 million is approximately the cost of the transaction. About G

BP 40

million has gone in as equity and GBP 150 million has gone in as debt. The GBP 40 million equity has been funded from Cayman. Cayman has reasonable cash balances and we've been able to fund it from the accruals of Cayman itself. The GBP 150 million debt will be serviced by the target.

Shivam Singh: But ma'am, we have acquired it at 9.2 EV to EBITDA. So, the net profits will not be enough to service it.

Sandhya J: So we have clarified that in the presentation that we have given. There is a one-time loss impact which is in the current results because of our new centre. If you adjust for that, there is adequate cash flow in the business to service. There is another part that we have done a structured debt with the lenders to synchronize it to the cash flow of the target.

Shivam Singh: Okay. Thank you, ma'am.

Viren Shetty: Thanks. Rajit, your question?

Rajit Aggarwal: Yes, sir. Sir, I went through the transcript of that call as well. Now, you had mentioned that there will be operational efficiencies brought in and your learnings from Cayman in India will be implemented. The question is on the demand side. So, one of the rationales is that the private payor-mix will go up in that region. So, what are the first one or two steps that you intend to take to increase the footfall from such patients?

Viren Shetty: Anesh, will you be able to take this up?

Anesh Shetty: Yeah. So, I think we've just completed the acquisition a few weeks ago. We're still understanding the post-closure formalities and getting settled in there. The team already had put in place the resources, the certain initiatives to increase their private payor-mix. This was not something that we had exclusively initiated. They were already aware of this and they're work-in-progress on a few initiatives. We're just going to accelerate those. We're not in a position to tell you when we will start seeing results because we need to get ourselves acclimatized here and understand more about the market. But this is just something that the company and the management, which is in place before and now, was already on track to do. 17 And we will ... in a few quarters, we will have more information about what that outlook is looking like.

Rajit Aggarwal: Right. Sir, may I just have a follow up on this?

Anesh Shetty: Yeah, sure, sure.

Rajit Aggarwal: The reason I'm asking is, I believe now I could be wrong, that UK territory will behave in a somewhat different manner, because the way hospitals could probably differentiate in India or in Cayman, those levers are different in UK. And the competition obviously is huge. So that's why I was just trying to understand how do we build that customer base. And I'm not asking for outlook or such. What are your initial thoughts as to how this will progress in, let's say, years to come?

Anesh Shetty: Sure. So, let's look at the absolute base case where the bulk of the revenue, which is 90-93% is contracted long-term secure revenue by the NHS. So, these are long-term evergreen contracts, as they call them here. There's no volatility we expected in that revenue base. The cost structure is what is already been priced in in the deal; we know those financials. Now, with our software platform, with our technology, with our group synergies like what we've seen in Cayman, we have a high degree of confidence that we'll be able to make a meaningful movement in the cost structure of this organization here, predominantly driven on the back of our software and technology. So, it's not dependent on taking any people from India or anything that is not fungible. This is just with our overall technology, digital capabilities. And we've already proven this out in Cayman. So that's the base case for that. Now, any movement we see on the pay-or-mix is extremely incremental because on a like-for-like basis, private patients, whether cash or PMI, pay anywhere from 20 to

30%, sometimes

35% more than the NHS. What gives us comfort that we can make progress there? That if you look at peers, we are at about 7-8% private mix. The next lowest private mix is as high as 30-32%, which is Ramsey. And then the others best in class are 60-70% or higher, 75% private. So even in the market compared to the others, we're starting from a very, very, very conservative low base for the private patient mix.

Rajit Aggarwal: Right, sir. Thank you. And just one quick question. Do the insurance companies there have tie-ups with all the hospitals in the same fashion, or will you have separate insurance companies, or you will have to have with some different companies and the top guy will have a tie up with some other companies?

18 Anesh Shetty: No, it's somewhat similar to India, actually quite similar to India. There are very few insurance companies, health insurance companies of scale. They generally tie up with ... they're incentivized to give their members choice across the nation. So, they generally are willing to reimburse with most providers. It's a question of the patient choosing to come to us versus going somewhere else.

Rajit Aggarwal: Right, sir. Thanks. Thanks a lot.

Viren Shetty: Thanks, Rajit. Deekshant, you have a question?

Deekshant B: So, sir, two questions. One is on our UK business and one on India. So for India, there seems to be a lag on our Mumbai operations. What is the update on our Mumbai operations and how do they plan to scale them up now? And then I can move to the UK operations.

Viren Shetty: Yeah, the Mumbai one we answered already. The Mumbai hospital started as a children's only hospital and it operates at the very highest standards of excellence and treating patients from across the spectrum. However, we found that being able to attract sufficiently large volume of patients, was not able to be achieved and the inbound cost structures are very high. So, we are in talks to slowly start transitioning this hospital from pediatric only towards a heavy pediatric focus, along with adult programs. Those that transition will take a few quarters to get through. But this is something we keep rolling out to improve the performance of Mumbai.

Deekshant B: So, the reason I ask this is because Mumbai is a very good market for the insurance business. But if there is an outage, then why would a person want to insure? That's a critical problem statement right now.

Viren Shetty: Sorry, what's the question in there?

Deekshant B: The question is that what would be the timeline here? Because the timeline has been going forward for some time. We have been pulling out again and again on this.

Viren Shetty: Agree. Our performance in Mumbai compared to the performance of most other Mumbai

operators has not been good. It has been very difficult for us with our way of operating a hospital to be able to run it in a breakeven manner. The fault lies with us entirely, and this is something we are looking to address. But it will be very hard for us to give a timeline for when exactly the hospital turns around.

19 Deekshant B: Got it, sir. It comes from a place of love for the brand. So please don't take it otherwise. We really love the brand and we want to see you expand and grow and give great healthcare.

Viren Shetty: We understand Deekshant, thanks.

Deekshant B: On UK particularly, we see that there is a major lag on the weeks that it takes for a person to get operated. How long do you think it will take us to integrate our systems and our people into this new acquisition? And when can we start seeing the first results for us? When is it that we will see the first result?

Viren Shetty: Anesh, go ahead.

Anesh Shetty: Deekshant, sorry, I missed. There was an interference.

Viren Shetty: I'll answer that. The question was, how long will it take to integrate? How long will it take to turn it around? And

how long will it take for it to start contributing meaningfully? This is very hard for us to see at this point. We have a tremendous amount of confidence in the UK market as a whole. We have tremendous confidence in the growth of private healthcare in UK. We have tremendous confidence in the management of Practice Plus, and we know that this is the right-sized acquisition for us to make a significant difference to the performance of this hospital. And us coming in with a highly efficient operating model with world-class technology and a world-class management team, we know that if applied to a well-running, highly efficient business that operates almost 93% on government patients and slowly uses that to change the payer profile, as well as improve the cost structure and grow and expand, that we can turn this into a significant lever of growth for our business overseas. Timeline, guidance, all of that, the usual caveats apply. It's not something that we do, and it'd be very hard for us to see at this point, given that we just finished the transaction a few weeks ago.

Deekshant B: Sir, usually you have said that it takes us 6-9 months to sort of put in our business once the structure is in place, the hospital is in place. I'm not asking for guidance, but I'm asking for, is that the right sort of thought process that it will take us two to three quarters to start giving out guidance and seeing that, okay, where we are right now. Is that the right mindset for us?

Viren Shetty: No, it wouldn't be. Yeah, I'll answer that then Anesh can add on. It wouldn't be because we know how to run hospital in Cayman now, we know how to run hospital in India. We apply that to UK, it'll be a disaster because the circumstances are very different, payors are very different, the way in which they operate is very different. So, it is a collaborative learning process in which we work with the management to see where we add value. And a lot of 20 failed experiments along the way. We may find that certain tech implementations that take three weeks in India may take three months over there, whereas certain things that take three months in India may take three seconds in the UK. So, those are part of the discovery process we'll have to go through.

Deekshant B: Thank you for the clarity, sir. Wish you the best.

Viren Shetty: Thanks Deekshant. Hanisha, you have a question?

Hanisha G: Hello, yeah, thank you for the opportunity. So, my only question was that in Q1 you had mentioned that you have deployed INR 250 crores out of the INR 450 crores of investment guidance that you had given. So, in this quarter, have you deployed any more of the funds?

Nishant Singh: See, this quarter there is less outflow of cash, but all of that is going to come in the next quarter. So, the set projects will always have a spillover. So, whatever we had projected, we will end up spending that and maybe a bit more. But there may be spillover that the March end may flow to the April beginning. But we are on track in terms of our cost and the time estimates.

Hanisha G: Okay, thank you.

Viren Shetty: Thanks. Do we have any other questions on the Group? Otherwise, I can move on to the chat. Yeah, Rajit.

Rajit Aggarwal: Yeah, regarding the transaction cost of the acquisition. So, will it be in the range of some INR 70 to 80 crores, and will it be debited to the Indian books or amortized in the UK books? How do you plan to treat that?

Sandhya J: Yes, this will be, the costs will be in the UK books. We have a parent entity in the UK which has made the acquisition and which is incurring the transaction cost. The typical transaction cost range is about 5%. We are in that range, though we are still finalizing all the costs and therefore, we will be able to give the correct number when we report out at the end of the quarter.

Rajit Aggarwal: Thank you, ma'am.

Viren Shetty: Thanks, Rajat. Chat?

First question is on whether we'll rebrand the UK assets. Although we do have a strong brand in India and Cayman, it's not as strong in the UK. So, we will continue

with the existing brand.

21 The question that was asked about NH as a unified brand that I think we had answered. We have created Narayana Health as a unifying brand ; Narayana Health Insurance, Narayana Clinics, Narayana One Health. But it will take time for it to see the public consciousness.

Anesh, the next two questions are on the ARPP for Cayman hospitals.

Anesh Shetty: Yeah, ARPP always for Cayman was not really a relevant metric and we wanted to add information breaking out the insurance and the hospital separately. We felt that was much more relevant and useful now. So, we've dropped the ARPP and we've replaced it with the insurance revenue quarter-on-quarter.

Viren Shetty: Can you please provide revenue mix for this quarter and going forward . By this quarter, I'll assume he means Q2.

Sandhya J: What is the revenue mix of ... sorry, we didn't follow that question, Aniket. Can you please help us understand?

Viren Shetty: Would you be able to get on the mic and explain to us the question, please?

Aniket M: Hello. Yeah. So, I was just asking for the revenue mix from the core hospital business and the insurance.

Sandhya J: Its given as a part of our Investor Deck itself, the revenue of the core hospital segment is given and the insurance and clinic revenue is given separately.

Aniket M: Okay. And what are the margins for both?

Sandhya J: For core hospital margin has been given again as part of the Investor Deck , Slide No. 6. As far as Integrated Care is concerned, it is still not making margins. So, the cash burn also is given in the Integrated Care slide on our Investor Deck .

Viren Shetty : The question over there is, can you share the console CapEx guidance for FY26 and FY27? FY27 we give once we're closer to the end of this year. But we had guided that over the next three years, we have a plan to spend about 3000 crores in India in a mix of Greenfield, Brownfield, O&M and acquisitions. The exact mix will come as and when we start disclosing different transactions.

The breakup of the individual hospitals are there in the investor slide where we've broken it up project -by-project and told you about the status and when they will be coming online.

22 Nishant Singh: Question is from Mandar Pendse. It says, I believe NH is one of the best set of promoters, management and doctors. It is evident that management is driving a lot of efficiencies in operations, taking out juice. As an investor in NH, while I believe these driving efficiencies will not be at the cost of patient treatment, even one unfortunate incidence will put all the juice to waste. Just need a confirmation that the management is on the same page.

Viren Shetty: Dr. Rupert will clarify. No point do we ever compromise patients.

Dr. Rupert: Yeah, so we don't ... So we have one of the strongest governance structure here, both clinical and non -clinical. And from the clinical standpoint, we also recently got the award for the Best Clinical Governance from the International Hospital Federation and the Association of Hospital Executives of US. So , we do have a very strong audit and oversight on all of our units.

We monitor more than 500 metrics for clinical outcomes. And we have a very strong department of quality . And quality is a way of life. It's not just a Quality Department, but the entire clinical team and the non -clinical team participate in all the aspects of it.

So, we have a very strong governance on not only clinical outcomes, but also all operations that affect the clinical outcomes. So, we take a very strong view of that and we keep a very strong oversight on that, both from the corporate and from the leadership on the regional side.

Viren Shetty: Thanks, Dr. Rupert. I think for the last question,

Narayan, did you have anything?

Narayan D: Yes, sir. So, on previous con-calls, I had heard that we are trying to make best use of patient data that we have, and that would help us in our insurance cum health treatment endeavor also. And we are trying to use software systems and include artificial intelligence there. Just wanted to understand, have we made any breakthroughs on creating any AI models that would help us there , or even clinical diagnosis, early clinical diagnosis? Where are we on those innovation fronts?

Dr. Rupert: Yeah. So, from a clinical analytical point of view, we are constantly working with the clinical teams as principal investigators to work on different areas of interest for the different clinicians. As far as from a public health perspective is concerned, you would have seen our numbers of the health checkups and various other data which we can analyze. And that is something which is prevalent in the community, and we know exactly what is the prevalence. And that is the data which you were mentioning , that can be used for our clinics program or for the other... insurance and other things. So these are some of the things that we do work together from a public health as well as from a strong clinical analytical point of view as to how to work together with clinicians to see the AI use in actual clinical work.

From an in -patient journey, we are able to do many of the tasks through a coordinated clinical workflow so that there is a strong guidance for the clinical team as to the next course of action to be done. Early warning systems, not only from a risk for morbidity and mortality, but also

from a suggestive plan for the clinical pathways that needs to be followed, which is vetted by the Clinical Working Group across the network. So, these are all incorporated within the apps, whether it is a doctor app called A ADI or the nursing app called NAMAH . So, we are able to integrate that . And it's a work in progress and we are constantly working on that to make it better and better.

Narayan D: Happy to have that answer. Just wanted to say that , that's the place where as a Narayana, as a proactive management, we can really make a difference. So I'm happy that you guys are already doing it . Maybe with some more rigor , that would help all of us, the part owners also.

Thank you.

Viren Shetty: Thanks Narayana. Two minutes left, I'll answer the last few questions that are in the chat. The question is, which of the Indian entities have started contributing to margins? We wouldn't attribute it to anything specifically. We've seen a sharp increase in performance across all our hospitals, especially the hospitals in East India. And a lot of these are volume driven and changing the case -mix and pay or-mix. So these do have a contribution to margins . But obviously the flagships, given their huge size relative to the overall hospital group, any movement in the flagship hospitals in Bangalore and Calcutta will have an outsized impact on the margins.

The next question is on the financial and HR implications of robotic surgeries with respect to Opex and CapEx . So robotic surgeries, while they are the future, they are extremely expensive. They have a huge front -loaded cost as well as a significant operating cost. For the first couple of years, as a hospital, we will have to bear the cost of training the doctors , as well as the patients will not be able to absorb the full cost of doing robotic surgeries. So, we do those nearly at break -even or sometimes at a loss. But over time, once we become better at doing these procedures, the patients are able to absorb some of the cost of doing the robotic surgeries, both in terms of general surgery, onco surgery, cardiac surgeries, and so on. But this is definitely the future, and an investment in this is something that will separate us from all the other hospitals that are out there.

The next question i

s on professional fees to doctors? That has come to 15.5%.

24 And, do we see it stabilizing? So essentially, the hospital is new, and the patient flow has not yet been established. But for th at individual hospital, it tends to be on the higher side, expecting very large minimum guarantees. But over time, as the volume fees, then the professional fees as a percentage of the overall revenues, starts to moderate and go down. This will be more or less in line with the peer set, maybe some advantages to a certain hospitals over others. But in the end, it averages it out.

Sandhya J: Also, the number that you're looking at is the console number. So that factor is also there. You should look at the standalone number.

The last question is, is the growth in EBITDA coming from pricing? It's not coming from pricing per se, because our pricing change happens only in lower single digits at the beginning of the year. The volume pick up is not being seen because of the payor -mix correction. As you can see, we have done a lot of improvement in our payor -mix, and our incremental revenue is coming through better paying payors . So, that is contributing to top line, in addition to all the factors we spoke about, efficiency, throughput, as well as higher order procedures.

Nishant Singh: Since there are no more questions, we would like to conclude our session. Thanks everyone for your active participation always. Thank you.

Viren Shetty: Thank you, everyone.

END OF TRANSCRIPT

Call: Nov 2025

Date of submission: November 07, 2025

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code –539551(EQ), 975516 & 976418 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051

Scrip Code - NH

Dear Sir / Madam,

Sub: Transcript of Investor's Call -Acquisition of UK-based Practice Plus Group Hospitals Limited

In relation to the investor's call of the Company held on Monday, November 03, 2025 on the acquisition of UK-based Practice Plus Group Hospitals Limited, please find attached the transcript of the said call.

We wish to inform you that the investor's call transcript is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/company-announcements> .

This is for your information and records.

Thanking you,

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

"Narayana Hrudayalaya Limited
Conference Call on Acquisition "

November 3, 2025

NH MANAGEMENT TEAM:

MR. VIREN SHETTY – VICE CHAIRMAN

DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER &
MANAGING DIRECTOR

MS. SANDHYA J – CHIEF FINANCIAL OFFICER

DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS
SUBSIDIARY HCCI

MR. NISHANT SINGH – VICE PRESIDENT , FINANCE , MERGERS &
ACQUISITIONS & INVESTOR RELATIONS

MR. VIVEK AGARWAL , SENIOR MANAGER , IR FUNCTION

2 TRANSCRIPT

Nishant Singh: Hi. Good Morning , everyone . My name is Nishant Singh . I Head the IR & Strategy for the Group. I welcome you all to this special call organized to discuss our latest acquisition PPG Hospitals , UK. In this call, we also have with us Dr. Emmanuel Rupert , our MD and CEO, Mr. Viren Shetty , our Vice -Chairman, Dr. Anesh Shetty , MD of our International Operations, Mrs. Sandhya Jayaraman , our Group CFO , and Vivek Agarwal, Senior Manager , IR & Strategy. With this brief introduction, I will hand over the forum to Anesh to take you through a brief introduction of the target post which we can start the Q&A.

Anesh Shetty: Thank you , Nishant . Thank you everyone for joining us. We thought we'll begin the call by spending the first five minutes just giving an overview about why we decided to enter the UK market and within the market why we decided on Practice Plus Group Hospitals.

So, as most of you , who have been following the company for a while , know it's been about 12.5-13 years since we first entered the Caribbean in the Cayman Islands. The assets have been operational for about 11 years now. And once we overcame the initial 3-4 years of settling in into that market and things started looking very positive, we've always been on the lookout for our next international operation, our next international opportunity.

Through that process over the past, I would say 5-6 years, we have been to almost every developed country which has an opportunity for private healthcare. We've considered definitely every market in the vicinity of Cayman, which is in the Caribbean, as well as most other markets elsewhere as well. And it is challenging for us to settle on something simply because home is in India, a market we are most familiar with, which many would agree is currently one of the world's , if not the world's , most attractive investment market for healthcare globally. So, whatever else we found, it has a very, very high bar to compete against. It has to be meaningfully more attractive than us deploying capital in India, which makes us very, very selective. So , even though we came across several opportunities over the past few years that were good across all sizes and shapes, we never did anything meaningful until today. We're very happy to report that after a long search we believe we've arrived on not just a country that is very, very interesting and attractive but also an asset within that market that we believe will be very, very accretive to our larger picture.

Coming to the UK market, what makes this attractive for us aside from the obvious aspects of being a stable country, a developed country, clear rule of law, business certainty, policy certainty and things that are more common across various industries . For us, particularly in the business of private healthcare, it is very reassuring to know that across political cycles, irrespective of which party or which orientation was in power, the role of the private sector in being a key pillar of addressing the healthcare needs of the country has been widely and universally acknowledged and is now seen more as a matter of fact and one that will take on an increasingly important role in the years to come. Within that industry as well, we are also comforted by knowing that there have been meaningful success stories with the other large operators, many of which, I would say most of the top large players , are owned by international multinational corporations or international investors . So, we find an interesting market, which not only has growth potential but which is stable in its broader conditions, which has a track record of other operators who are doing wonderful work and who have seen good success as well as a long term future where we can build essentially a multi -decade business scaling and operation.

Within that market, within the UK, we considered several assets. Some

of you may know,

over the years we've spoken or had fairly early discussions with a whole range of providers, some of whom are very small, some of whom are very, very large, and some in between.

We believe Practice Plus fits into the sweet spot in terms of its size. If the asset is too small, then there's not much we as NH can do to add value. At the same time, if it's very, very large, there are certain other complications where it becomes unwieldy , there are other complexities in running that. In terms of size, this is a good , sweet spot where we believe it is large enough for us to not only own 100% of it but large enough for it to have the critical size for us to make a meaningful difference with what we bring to the table, which I'll come to shortly.

Yes, we also note that this asset has certain aspects of the business in terms of its pay or

profile that are meaningfully different than other peers. We see this as an interesting opportunity. There were certain assets we considered that had a very, very privately oriented pay or mix, which is great from a profitability standpoint, but that also leaves less room to improve and less value for us to add. More importantly, it is very, very important for us that we buy a business which has a management with a proven track record that is safe and secured with the new ownership. And we're happy to report that the fantastic management, the entire senior leadership of this organization is going to be staying with us, even though we are buying only one division out of the three. It's a company with three divisions, we're just buying the Hospital s division. But we've secured and aligned the key people in the senior management. And we're very confident in our ability to work with them and they're very bought into the larger vision of what we bring to the table as well.

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I'm just going to request Mr. Rakesh to go on mute, please.

So, to continue, so we're very confident and eager to get started with the management team under new ownership. And this is the capable group of people who've been running the business so far and we feel that there is all the ingredients in place for us to essentially make a good attempt at replicating what we've done in Cayman but on a much larger scale and in a much larger market.

So, I think that addresses the two largest questions we got, which is - Why the UK? and Why this particular asset? But also happy to get into the Question & Answer session and answer any specific questions or take this any other direction that you guys want. Nishant, maybe we can start the Q&A.

Sandhya J : Before we get into the Q&A...

Anesh Shetty: Yeah. Sorry, Sandhya, go ahead.

Sandhya J : ...I just wanted to clarify one more financial point, which wasn't there in our earlier deck. So, the numbers which we had shared in our deck earlier was pre -IFRS. So, when we reported GBP 20 million as the EBITDA, it was pre -IFRS, which means after adjusting the lease costs. So, if we take the like -to-like post -IFRS EBITDA, which is before the lease costs, it is GBP 29 million for FY25. So, we have since added this in the deck and we will be uploading the revised deck shortly.

Anesh Shetty: Nishant, how do you want to do this?

Nishant Singh: Yeah, so we can start the Q&A session now. We request everyone to now use the 'Raise hand' feature to start posing the questions. I think we already have a raise of hand from Ravindra. Ravindra, can you please go ahead?

Ravindra : Hello, am I audible? Hello?

Nishant Singh: Yes, you're audible.

Ravindra : Yeah, this is Ravindra from Bangalore. So, I'm a retail investor. Actually, see, over the weekend I got hold of the Annual Report of the acquired company. So, I was going through that. I have a few questions around that. So, firstly, I want to clarify that we are going to acquire only the Hospital division, that's the Secondary Car

e division, am I right?

Anesh Shetty: That is correct.

Ravindra : Right. Okay. So, I'm looking at the revenue for the financial year ending '24, okay. So, as per that, the revenue in terms of Indian rupees is around 2,660, it comes. I'm taking an exchange rate of 116, okay. So, in million terms, it is 229. That has been reported in the Annual Report last year. So, in your deck, I think the numbers are not matching as such, that's the one thing. So, am I correct on that?

Anesh Shetty: Yeah, we're happy to take that question offline. Nishant, and the team will reach out to you to reconcile that currency conversion.

Ravindra : Not just currency conversion, it's 229 and also the EBITDA numbers, everything looks very, very different. Because I'm looking at the Annual Report of the reported company.

Anesh Shetty: Sure.

Ravindra : Okay. So, based on that I have a few questions, right. Because I think it would have been better if the deck had been prepared with updated numbers so that it would reflect real value of the company as such. Because when I'm calculating all the numbers, for example, the Secondary Care margins, it's coming around 11.8% for me based on the Annual Report reported numbers of September '2024. So, when we look at the deck, it doesn't give me that bullish stance as such.

Viren Shetty : Ravindra, if I may, the thing that was announced was of the overall holding company, yes, the one with three business verticals.

Ravindra : Right, right. I'm just taking the Secondary Care.

Viren Shetty : We understand, yes. The thing about disentangling a business that has three verticals into one is that there will be cross charges. There are certain things that reflect across multiple balance sheets. So, the company is in the process of disentangling, which may lead to slight adjustments here and there. As we close down this process over the next quarter and so, we'll be able to give you a much better picture of the Hospital s-only business.

6 But right now, the businesses within themselves also render services to the subsidiaries and so that may be part of the reason why some few million here and there may be a discrepancy. But as far as the audited numbers that we have, this is what is there. But the next quarter will give us a much better picture.

Sandhya J: What we have given you, Ravindra, in our deck what we've given is the carved out financials that have been diligenced by our diligence partners and extrapolated for the full period. So , therefore, this is the number that we will be acquiring and integrating with our balance sheet. What you see in the public domain has, like Viren said, other than Hospital data also mixed up and , therefore, you will not be able to reconcile it. You should take the number we are presenting as the correct number.

Ravindra : Because they have strictly bifurcated everything , so that's why I'm having this question because it gives a lot more bullish stance than what has been reported. I don't know but it should have been better that if you had...

Viren Shetty: Any other questions, Ravindra

Ravindra : Yeah. So, again going on, I have few questions, say, for example, what happens to the liabilities on books? So , how much liability? Because as you say, you are taking only certain, only one division.

Viren Shetty: Yeah, that's fine. We can answer this now.

Sandhya J: Yeah. So, Ravindra, other than the liability for the leases and the regular liabilities for creditors, we are not taking on any liability on our books. So , we are acquiring the company on a debt -free basis. So , whatever liabilities you are seeing in the balance sheet, which is uploaded in the company house, those will be left behind. Only the regular creditors and lease liabilities, we are taking over.

This also answers Rishabh Doshi's question on the chat.

Ravindra : Okay . That means the re will be no term loan as such, we wil

I be repaying or will not be taking over the term loan as such . That's what you're saying ?

Sandhya J: The seller will take care of the repayment. We are acquiring the company on a debt -free basis.

Ravindra : Okay. Okay. So, again, if I take that point, you are giving a very bullish stance as such. So , okay.

7 Viren Shetty: Any other questions, Ravindra?

Ravindra : I think, what about the dividend distribution as such, because company has been paying regular dividends. Is there any stance on that, like how that will be accounted for, whether we will get some part of the dividends as such?

Viren Shetty: We can answer that.

Sandhya J: So, our current plan, we do intend to continue with our current dividend policy, which we have .

Ravindra : No-no, I am not talking about the NH , because this company has been paying a dividend regularly. Last year they paid around GBP 34 million as a part of an entire company. So, we are taking a part of the company , that means there is an embedded dividend within that. So , has there been any arrangement?

Viren Shetty: Yeah, we haven't a call yet on what the inter -company dividend would be between the subsidiaries and the main. We are still in the process of merging and acquiring these companies and consolidating the account sheets and taking over the Finance department. We will have a much better sense of this over the coming quarters. But as of now, it will be hard for us to comment.

Ravindra : Okay. Okay. Fine. Apart from that, yeah, actually I had a lot of numbers related since you...

Viren Shetty: Fine. Maybe we can move on to the next person. And if the queue exhausts, you can join back again.

Ravindra : Sure. Sure. Yeah , thanks.

Viren Shetty: Thanks, Ravindra.

Nishant Singh: Prithvi , can we have the question, please?

Prithvi : Yeah. A nesh, just, you know, a couple of questions. Obviously, the first one, you know, you mentioned about UK saying stable government or, you know, stable policies, growth, a track record of international investors, etc. But this might be the case with even other European nations, right . So, why particularly UK? You know, how is UK market different or, you know, what kind of growth levers are you looking in the UK market?

8 Anesh Shetty: Sure. So , we did consider other opportunities in various other markets. So , just with regards to the country itself, you know, we see a track record of every larger, if you take the Top 5 healthcare providers, barring 1, which is a not -for-profit , the others have demonstrated consistent growth in revenue terms as well as in earnings with some volatility. And they're all owned by international investors as well for the most part, barring 1 or 2.

The second thing is that in terms of the total health spend , the percentage contributed by the private sector is still in its infancy. There is, we believe, a long journey ahead and meaningful change that could happen in the decades to come. So , if you combine the entire spend by the private sector, it's approximately 16 %-17% of the grand total. So , it is still relatively the

benefit of a low base.

Having said that, it's not just the market that is the country, it's also the asset. There are certain other, you know, markets that also have certain similar favorable dynamics or certain other favorable aspects but it's also a question of identifying the country as well as the asset within that country that fits our criteria. And for the most part, Practice Plus within the UK was a good intersection of many things that we were looking for.

Prithvi : Okay, got it. So , you're mentioning that the country might incrementally move from NHS to private and private sector will benefit from that ?

Anesh Shetty: It's not, I mean, it's not something that, you know, one would obviously be able to predict in the short term. There are periods where it's taken the other way as well. If you look at histo

rically, there are periods when the dependency on the private sector increases, there are periods where it decreases. But for the large part, we think that this is a relatively low base.

So, there is certain safety that we're not going to see a drastic reduction in the levels where it is right now because we don't want to be fighting against, you know, the headwinds of a decreasing , shrinking market. That's always hard to do. So , from that perspective, this is an option we narrowed down on.

Prithvi : Got it. And , you know, one of the key reasons why Cayman has done exceptionally well for you people is that you were able to replicate more or less Indian cost structure for the international realizations. So , how should we look at UK? I mean, can you take Indian doctors, Indian medical staff to UK? How about the law state? Can you import equipment from India? So, how does the entire cost structure work?

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Also, because the hospital is currently at 8% EBITDA margin, how do we look at margins, you know, in a 3-5 years down the line?

Anesh Shetty: Sure. So, just addressing the comment on Cayman, you know, we do have clinical as well as non-clinical staff from every other country as well. But even to the extent that we are able to take people from India, we can't pay them an Indian salary, obviously. They earn just as much as they would in any other equivalent opportunity in that market . So, we're not looking at essentially an on-site labor arbitrage opportunity . In healthcare services , that's very difficult to do. So , for example, for decades now India and other South Asian countries have been sending thousands, tens of thousands, in fact, of nurses to the UK and, obviously , they earn just as much as any other nurse.

So, the reason we were able to drastically alter the cost structure in Cayman and have a lasting competitive differentiation in cost compared to peers in the region is because of the entire ecosystem of operational changes we were able to begin, which is the most part underpinned by our technology platform, which does scale across markets. So , we run, as you know, in Cayman, we operate on an entire technology platform that's built and owned by us . That gives us the opportunity to perform every single transaction with far fewer human touch points or far fewer people involved and far less steps than competitors. So, whether it's a revenue cycle process of processing an invoice, whether it's processing payroll, whether it's admitting or discharging patient, every individual process is done with fewer touch points. And when you add it all together, we just get a very different, simpler, leaner operation. We think a lot of this is replicable in the UK. Obviously , the scale is different , the market is larger. You know, we'd have to go through that learning journey . In Cayman , it took us a while to get there. That was also the first time we were doing that. We're confident we won't take as many years as we took in Cayman. But you are right in the broader sentiment that with what we've done in Cayman, which is meaningfully alter the cost structure on the backbone of our technology and other broader capabilities, that's also what we hope to do in the UK. And through the process of our diligence on this asset, we've developed a good degree of conviction working with the management as well, that this is possible. So , we had senior members of the ir team come to Cayman, see what we've done and really pressure test the hypothesis that 'Can we replicate this or largely replicate this in the UK ?'. And there was, you know, unanimous agreement that many elements of this, what we've done in Cayman from 10 an efficiency standpoint , can be replicated here. Of course, they have to be adapted but directionally, I think you are right.

Prithvi : If I have to ask you for a number, right, you know, some broad -based number, say 5 years down the line , once

you implement all these things or should we look at EBITDA margin as well as revenue growth?

Anesh Shetty: Yeah, I think it's a little, you know, very, very early to say that but what would help is you do have the historical growth rates of the company in, both revenue and margin. You know, it's early days for us to comment on that. We hope to develop a view that we'd be happy to share in the years, I mean, in the quarters to come when we meet next time. But I think for now

the direction that we have is we are internally quite confident that there are advantages we bring to the table, both from a revenue trajectory as well as earnings. The management is confident in the path they are on. They've started many things, even pre-dating us. So, even if we did not close this transaction, they are already at an inflection point with certain investments and changes they are making to improve their revenue trajectory. And we're lucky to be entering at the right opportune time.

Prithvi : So, let me put it this way, you know, if you have to look at ROCE, right, eventually, you know, everything has to translate to returns. Given that there is such a large investment and you have India as an alternative market, so what kind of ROCE are you looking at?

Anesh Shetty: Yeah, so just, I mean, our conservative, you know, base case is by FY29 -30 or so we want to be, you know, in the range of 20 %-22% from a ROCE perspective is where we see. Now, a lot of that is driven by, you know, the entry price, which we know, certain assumptions around the trajectory the company is on, as well as certain assumptions around the difference we can bring to the table. But, you know, we wouldn't have entered this if that wasn't a high conviction thesis that we had that by FY29 -30, approximately in that range, we'll be within the 20%-22% ROCE level.

For the reason you said, because of the alternative opportunities for us, yeah.

Prithvi : So, one final question before I get back into queue again. Will this be, I mean, after assuming the interest cost for the entire GBP 150 million debt, will it be EPS neutral in the first year and then it will turn positive or will it be earning dilutive in year one?

Anesh Shetty: Yeah, there's a little bit of nuance to that question. I'll hand it over to Sandhya to walk you through that thinking there.

11 Sandhya J: So, as far as EPS is concerned, it will be neutral, maybe mildly favorable also in the first year. How it picks up from there, we'll have to see based on the performance of the business.

Prithvi : OK, that answers. I'll join back in the queue.

Nishant Singh: Thanks, Prithvi. Harith, can we have your question, please?

Harith : Hi, good morning. Thanks for the opportunity. So, in the presentation you've talked about attracting more private pay or patients, so currently the mix is around 93 % coming from NHS.

So, if you can talk about how this mix has trended over the last few years and do you expect this to change or improve materially in the next few years? And what are some of the steps that you're looking at in terms of attracting more private pay or patients?

Anesh Shetty : Yeah. No, thank you. So, if you just look at the competitive landscape, so if you look at the large, the only publicly listed asset that's available with robust data is Spire. So, Spire currently has about, as of the recent half year, about 35 % of their pay or mix being NHS, the rest combination of self-pay and PMI, which is their private medical insurance. The other assets have, you know, the other equivalent assets, Circle, which is ADQ owned, around the similar range. Ramsay Health, which is, I would say closer to PPG than the others, has between 60 %-70% NHS. So, Practice Plus is definitely an outlier being at 93 %. But this is a factor of where they've started off the journey in the origins of the company and where they're trended. So, to y

our question on, you know, they started out being 100 % NHS. This movement to, you know, 93 % is a little more, 90%-93% is a little more recent. And it is our intention as well as the management's intention, Bridgepoint, their erstwhile owner as well, even if this transaction did not happen they are on the trajectory to increase the percentage of their payor mix from private sources. And we are definitely going to enable that journey.

It's a little hard to give you, you know, a hard number as to where we'll be in 6 months and a year, et cetera. It's still early days for us. But that is definitely the intention. And they've taken some very concrete steps and decisions, not just on individuals but also on asset selection and certain assets they've recently acquired as well to enable that.

Harith : Okay. And within the NHS part of the business, you know, given pricing is, you know, set by the government, what are the steps that we can undertake from a margin improvement standpoint?

Anesh Shetty: Yeah, so it's a very fair system to be, to be honest. Broadly, in a nutshell, NHS England would pay you, that is a private hospital, the same as they would pay themselves, which is their own 12 hospital. And this is also largely inflation linked accounts for changes in wages. So, you know, aside from HCA, which is, I would say, next to Zero NHS, all the other players that are privately owned as well as publicly listed do vocally state that, you know, there is a role, a meaningful role, for an NHS in a broader pay or mix because it is stable, it is secured, it is sustainable and it is largely inflation linked.

So, it's not something that, you know, we would individually do to influence reimbursement, etc., because we're just a very small cog in a very large wheel. But in terms of improving margins within an NHS framework, you know, you have all the levers of cost, around cost structure and operational efficiency, because in some way the price is set to be largely sustainable or close to sustainable for the government themselves. So, as long as we can have, execute on a strategy that improves our day-to-day operations and our cost structure and differentiates ourselves compared to peers, both public and private, then the

profitability, even being a predominantly NHS pay or mix, starts looking very, very different from what it is now.

Harith : Okay, one last one , more of an observation . When I look at the specialty mix, it's skewed towards Orthopedics. And I understand that, you know, NHS backlog could be a reason here and the high number of elective procedures in the Ortho specialty. Are there any other specialties where you think a similar dynamic exists, where, you know, there's a high proportion of elective procedures, which you can target and you see as an opportunity?

Anesh Shetty: Yeah, that's the right way to think about it. So , the existing specialty mix, not just for us but other private providers as well, is largely reflective of what the NHS chooses to outsource as well as their backlogs and the other factors as well. So , this is a lot of heavily skewed towards Orthopedics, Ophthalmology , you know, General Surgery ; these kinds of things. There are certain other services and specialties where other providers have a good chunk of their revenue mix coming from which Practice Plus doesn't. So , that's an easy opportunity for revenue enhancement without any additional investment in infrastructure.

So, to your question, you know, there are several services that we've already identified where it is possible to start in the existing infrastructure without adding, you know, hard infrastructure, just some operational execution and some people and teams and clinical orientation. We definitely will be assisting the company to start these services and grow within the same property footprint.

Harith: Got it. Thanks , Anesh. I'll get back in the queue.

13 Anesh

Shetty: Sure.

Nishant Singh: Thanks , Harith h. Raman, can we have your question, please?

Raman : Yes. Thank you, Sir. I just have 2-3 questions. First is , I just want to understand, it's more or less like a follow up on the previous participant's question. So , majority of our revenue comes from NHS , I just want to understand the payment cycle. And are we shifting towards like is there any plan to shift more towards private players? Because my understanding is the margin mix between NHS and private player, private players have better margins. So , if you can help me clarify that.

Anesh Shetty: Sure. Sure .

Sandhya J: Yeah, I'll take that question.

Anesh Shetty: Yeah, go ahead, Sandhya.

Sandhya J: Yeah . So, in terms of payment cycle, NHS pays within 15 days on average . So, the company almost operates on virtually zero working capital. As far as the private pay piece is concerned, I think Anesh just explained to the previous speaker the dynamics around that. So , I think that question is answered.

Raman : Okay .

Anesh Shetty: Yeah, but happy to essentially just answer quickly . Essentially, yes, you are right. The private payors do pay a rate like for like basis , that is more than the NHS. But, you know, each has its own attributes that would make it attractive and a good mix between all would be the ideal outcome.

And to your question on whether our intention is to increase the non -NHS sources, that is PMI and self -pay. Yes, that is our intention and we hope to make good progress on that.

Raman : My second question is our total bed capacity is 330 beds with respect to the acquiring entity. What's the operational bed out of this?

Anesh Shetty: So, they're all operational. In fact, the way the market is oriented there because the procedures are more shorter stay, daycare, lower in acuity . Unlike in India, which is the market we're all familiar with, the bed isn't the fundamental unit of measuring capacity, etc. , 14 there are other units that are more reflective of capacity. So, yeah, in our parlance, when we say capacity and operational beds, all these beds are operational. Yeah.

Raman: So, are we having any Capex to increase the number of beds?

Anesh Shetty: No, we don't need to increase the number of beds because the bed wouldn't necessarily be the bottleneck or the driver of throughput or capacity. But in the current footprint, we don't anticipate any Capex being spent on a bed addition.

Viren Shetty: But to the larger question of can these existing infrastructures do more, the answer is yes. There's scope for significant throughput enhancement, both through the existing specialties, adding more volumes and adding more bolt -on specialties.

Anesh Shetty: Yeah.

Raman : And my final question is with respect to the Birmingham Centre, when will it achieve breakeven?

Anesh Shetty: Yeah, Raman , so that's a centre that was recently acquired by one of the other peers as part of a merger and disinvestment. So, it's in a great location. The centre is our new centre, I would say the newest centre , it's still in a sort of a pre -commissioning phase and that's why you have operating losses. Some aspects are commissioned, some haven't. We hope to update you and the rest of the followers maybe in a quarter or two once we get a better sense of where we are in that. Yeah.

Raman: And, Sir, I just have one technical question. What's the difference between centre EBITDA and adjusted EBITDA?

Anesh Shetty: Yeah. So, essentially, centre EBITDA would just be the profitability at the business unit in aggregate, combining all the business units. But then between the centre EBITDA and what we would finally land up with is our corporate costs, you know, things like our IT contract, central staffing, payroll, everything that forms a central shared service. These hospitals, unlike what we see in NH in India, in isolation are small units. They'

re relatively small. So, a

lot of their costs are shared across the entire organization. So, that would be the difference.

Sandhya, you want to add anything there?

Sandhya J: Yes. So, there are divisional overheads and corporate overheads that come between the centre and the adjusted EBITDA. The only item on the adjusted EBITDA is the adjustment for 15 the Birmingham losses which we have called up. Other than that, the adjusted EBITDA is equal to the EBITDA as we measure pre-IFRS in NH parlance.

Raman: Sure.

Nishant Singh: Yeah, Kaustabh, may we have your question, please.

Kaustabh: Sure. Firstly, congratulations, guys. I think it's very heartwarming to see an Indian entity make global presence. So, kudos on that. I have a couple of questions.

The first question is NH in India is known for its focus on throughput, the DNAs of throughput, which is very counterintuitive to the industry versus what the other peers track, right. What will be the, would it be essentially the same kind of DNA and focus that you guys will be able to implement in the UK entity? And what will be the benefits that you guys can derive from integrating Athma, which is your tech stack, in the UK acquired entity? Is there any quantitative versus qualitative target or some projection you guys have made internally to determine the benefits that come from integrating Athma?

Anesh Shetty: Thank you, Kaustabh, not just for your initially kind words but for asking our favorite question.

But, yeah, so, you know, one reason we like this company is because there is a very good philosophical alignment and match in terms of the importance of operational efficiency and throughput. So, I will call out an important fact about this company that we really, really admire. There is no private provider at this scale and above, so they may be smaller but at this scale and above, that we know of, that can operate at a 90%-93% NHS pay or mix and have the margins that this asset has. So, even before we come in, this is a management team and a company that is efficient, that is focused on throughput, that understands the value of simplifying processes and automating either with software or with other interventions. And that's very much how we like to think of it. So, it is very, very heartwarming to see that and that really attracted us to this company because we're not trying to bring in a very large culture change, we're just enabling them with our platform, with our tools, with what we bring to the table to just turbocharge what they're already doing.

So, to your second question around the Athma technology platform, yes, absolutely, it's very similar to what we've been able to do in Cayman. We believe that in time, hopefully sooner than later, we are able to infuse our technology and digital capabilities to automate away things that don't need to be done, to simplify the way a lot of processes happen. In markets like Cayman, like the UK, like Europe there are many, many, many steps to take a patient from admission to discharge that have nothing to do with the clinical care, that have nothing to do with their disease but it's just administrative steps and functions to either record the care, document the care and get paid for it. And this is ideal fertile feeding ground for us to come in with our broader capabilities and what we've already built to identify opportunities for improvement and efficiency.

Kaustabh: Splendid! Splendid! Thanks for that.

My next question is, in India, Narayana is known to be one of the most efficient and most capable hospitals for robotic surgeries and robotic capabilities. Do we have any plans or if you can comment on the same robotic surgery capabilities of the UK entity?

Anesh Shetty: So, it's slightly different market dynamics. So, in India, private hospitals, especially the larger private hospitals, generally tend to do tertiary care and comp

lex care, organ transplants, open

heart surgery, etc. In the UK, for the most part, there are exceptions in some of the London hospitals, but for the most part the private providers are doing work that isn't that high in acuity but they're doing it at scale at throughput, largely helping the NHS with their backlog. So, your robotic work, your organ transplants these kinds of things would still tend to happen in the NHS, not in the private sector. So, it's a meaningfully different level of acuity that we would see either compared to Cayman and India.

And there's nothing wrong about it. It's just the nature of the market and we're happy to identify where we can add value in terms of enhancing the scope of services they have with our clinical background. If it leads in a direction where it leads to more complex work and someday, you know, robotic and advanced work, so be it. But I don't think that will be the

initial focus.

Kaustabh: Sure, got you. My next question is, is price discrimination subsidizing the lower end of the pyramid? Is it even relevant in UK given it's a developed market with essentially a wealth distribution that is not as skewed as India? So, is price discrimination subsidizing the lower end of the pyramid in terms of customers? Is it possible? Is it on cards for you?

Anesh Shetty: No, that's not really how it works because the NHS has essentially, you know, a tariff that you get paid. You don't decide that, that's what you get paid. You can obviously increase volume and quality, etc., but that's essentially decided tariff. There are self-pay patients, essentially like in India, they're paying cash. You know, they're shopping around because they don't want to wait on a waiting list, etc. So, they're paying cash. So, over there, yes, there is more of a free rein in pricing, which within reasonable limits.

17 Kaustabh: Got it.

Anesh Shetty: Then on the other end, which is privately insured, again, it's like in other markets where you have a negotiated tariff with the insurance company and the patient is not responsible for that. So, in the non-NHS, there are two buckets, self-pay and PMI. In the self-pay bucket, there is some element of what you're saying but it's not as prevalent. But in PMI, it's also a fee schedule.

Kaustabh: Understood. Fair enough. My last question is, since we already have been operating in Cayman, which is a UK administered territory, and now we're in UK, do we have any benefits in terms of our learnings from our operations in Cayman, dealing with the UK regulator? Do we at all have any benefits in that direction?

Anesh Shetty: I don't think so because they're differently regulated. I mean, Cayman is a British overseas territory but that doesn't flow down in any meaningful way in terms of clinical regulation. Of course, all providers in the UK are wonderfully regulated by the CQC and related bodies. And it's a very robust, time-tested, fair system in our experience. And another good attribute about this company is, it's in a good group, a select group where every asset we operate, every unit is rated good or excellent by the CQC, which are the Top 2 ratings. So, every asset that this company operates is good or excellent. So, they're doing a phenomenal job with regards to their regulatory requirements and compliance. And we definitely look to continue that.

Kaustabh: Amazing! That's all of my questions. Thanks for answering and congratulations once again.

Nishant Singh: Yeah, Ramesh, please go ahead.

Sandhya J: Ramesh, I think, already asked.

Nishant Singh: Ramesh has already asked? So, we'll move to Shivam. Shivam, can we have your question, please?

Anesh Shetty: Sorry, if we could just request, if anybody has asked a question and wants to get back in the queue, if you could lower your hand, please, and then raise it again so we can keep track of who's next. Thank you.

S. Ramesh:

S. Ramesh: I haven't asked a question yet.

Viren Shetty: Yeah, Ramesh, go ahead, please.

18 S. Ramesh: So, just to understand the transaction.

Nishant Singh: We're not able to hear you, Ramesh. Can you please be a bit louder?

S. Ramesh: Just hold on. Can you hear me now?

Viren Shetty: Not well, Ramesh.

S. Ramesh: Just hold on.

Viren Shetty: Yeah, that's fine. Go ahead.

S. Ramesh: Yeah. So, if you look at your overall transaction cost, is it all in cost or is there any other adjustment you expect? And in terms of other charges and asset valuation, everything is included in the valuation?

Anesh Shetty: Sandhya, do you want to take that?

Sandhya J: The cost is all in cost. However, typically, as in every deal, there is a 4% - 5% deal cost, which comes for diligence, for stamp duties, for lawyers, etc. So, that will come on top of the costs that we have indicated.

S. Ramesh: So, this cash cost is already paid or it will be done in the second quarter?

Sandhya J: In Q3, yes, we will pay. We have signed, we have not closed. Closing will happen by end of week. At that point in time, we will make the payment.

S. Ramesh: Okay, fair enough. So, that means your Q3 results will show the consolidated impact of this acquisition, right?

Sandhya J: Yes, from the date of closing.

S. Ramesh: Okay. So, if you look at the value of this acquisition, now obviously, you know, you possibly have done the homework or you would have possibly assessed similar acquisitions elsewhere in the world or in India. So, have you also considered similar acquisition opportunities in India? Because that is your home market, that is the reason why I am asking this question. And why is the private equity firm, which has owned this asset, is selling now? Is it just because they are catching in on their value over the years or is there any other structural

reason why they are getting? Because the NHS itself has been going through a lot of political debate in the UK, so in terms of your own value proposition, what is it that you are adding to that business? And why is the existing investor selling, if I may ask?

Viren Shetty: So, the second question, first, Ramesh. The investor is a bridge point to the private equity fund, they reached the end of their lifecycle. So, they have to return money to investors and they need to offload this asset as well as the other two that are part of Practice Plus Group. So, that is why they are selling and that is why we are taking up.

To the question that you asked first, are we looking at opportunities in India? Yes, of course, we look at opportunities constantly in our core markets. We have identified quite a few. We have tied up with real estate developers, we bought land, we are building in Bangalore, Calcutta, Raipur in the projects that we disclosed. And here and there, M&A opportunities do come up. But a lot of the time private equity backed M&A opportunities in India tend to be priced very, very aggressively. And in all the calculation that we make, the payback tends to get quite stretched. And, so, that is why it is still something we do consider but either they are too small and not that we are able to do much with it or the few opportunities that get written about in the newspapers, the ones in India, are beyond our ability to afford.

S. Ramesh: Okay. So, if you look at the manpower cost for this UK asset, it is about 39%, doctors plus the others. So, what are the controllable levers you have in terms of pension liability and given that there is also a currency risk involved? So, when you look at your overall ROCE target, have you built in any increase in this manpower cost, assuming that, you know, consumables and the other operating expenses is something which possibly is within your control? And to the extent to which, you know, you are talking about the NHS patients, accounting to more

more than 90%, so is all the increase in the cost built into this overall revenue mix and to what extent can you make up any increase in margins required to the self-paying patients?

Viren Shetty: Yeah. Anesh, if we can talk about the inflation linked tariff, please.

Anesh Shetty: Yeah. So, to Ramesh's first question, the composition of the P&L to various cost heads is not very different from what we see in India or in other larger markets. And as Viren mentioned, you know, the NHS sets its tariff the same for all providers largely. And it does reflect, a tariff increase does reflect the cost of inflation of medical goods, wages and other services. So, there is that element of being insulated from that. That's also passed down to the private insurers as well with some delay.

20 S. Ramesh: So, is it fair to assume that your UK asset is going to be something of an annuity business over time?

Anesh Shetty: It's a bit early to say that but let's address that maybe in a couple of ...once we are settled in there, yeah.

S. Ramesh: Okay. Thank you very much and wish you all the best. Appreciate it.

Anesh Shetty: Thank you. Thank you very much.

Nishant Singh: Shivam, can we have your question, please?

Shivam: Yeah. Am I audible?

Nishant Singh: Yeah.

Shivam: So, actually, I was asking that there's a shortage of healthcare workers in UK and given that the NHS isn't very keen on letting people come from outside, what stance do we have on that?

Anesh Shetty: As a private provider, you know, you're slightly more insulated from the workforce shortages compared to the public employers who have a lot of other factors to consider and wage constraints. So, for the most part, because of the acuity of services not being tertiary and above, and because of the attractive workplace that private workplaces offer compared to public workplaces, the shortage, the larger shortage, is not that much of a concern for the private sector and particularly for PPG.

Shivam: Okay. And given the NHS is very tight on the budget, do we have a process in which we can get away from the NHS pay or mix?

Anesh Shetty: Yeah. So, you know, we answered that previously as well. The intention is to move away with certain changes that have been made on the ground with regards to people as well as asset selection and marketing efforts. We'll continue to do that. And the intention will be to increasingly attract, you know, more private and self-pay patients.

Shivam: The way we have started the insurance, providing insurance in India, will we be doing that business as well there?

Anesh Shetty: No, we're just entering as a provider now. And it's still early days.

21 Shivam: Okay, sure. Thank you.

Anesh Shetty: Thank you.

Nishant Singh: Thanks, Shivam. Shreyansh, can we have your question?

Shreyansh: Yeah, hi. I had a couple of questions. So, the first one is, if you could give, of the 330 beds what's the kind of utilization currently for the beds?

Anesh Shetty: Yeah. So, as earlier mentioned, you know, bed isn't the unit of utilization. There are other capacity levers about, you know, theatre sessions and theatre utilization. But if you take beds

as well, as well as theatre, you know, other metrics of utilization, it'll approximate be closer to 50 %-55%. So, as Viren mentioned, there is adequate room to accommodate additional volume in the same properties.

Shreyansh : Got it , got it. So, since it's 50 %-55%, so trying to understand what stops, as given the backlog that NHS has, you know, from increasing this utilization ? And why is it, you know, at that level?

Anesh Shetty: So, backlog doesn't translate into NHS willing to pay for it. They need to have funds to pay for it because, you know, the backlog largely exists because they do not have the funds to, you know, render the services within their own property or to pay for it in the private sector. That

is one constraint. And also, the intention would be to attract non -NHS patients as well. And that, like in any market, whether India or otherwise, has its own learning curve about understanding the patient acquisition journey. It's more consumer choice because they can go wherever they want and addressing those, you know, those levers.

Shreyansh : Got it , got it. And my last question is, so by when do we expect, you know, the tech systems to be integrated into the UK, into the hospital?

Anesh Shetty: It's a journey. So, we'll start immediately with low -hanging fruit, you know, certain things that are easier to do but certain core systems will take more time, a couple of quarters, maybe longer than others. So, it's a journey and it won't be one date when systems transition . It will be module by module, application by application. And, you know, the benefits will, we hope, correspondingly flow incrementally over the years as well.

Shreyansh : Got it , got it. That's all from my end. Thank you so much and all the best.

Nishant Singh: Thanks, Shreya nsh. Nidhi, can we have your question, please?

22 Nidhi : Yeah, sure. Hi, thank you so much for taking my question and congratulations on the strategic partnership. Just wanted to understand how is the ecosystem for private hospitals versus NHS ? And who would be our targeted audience, our targeted customers?

Anesh Shetty: So, sorry, Nidhi, could you be a little more specific? What do you mean by ecosystem for private hospitals? Sorry about that.

Nidhi: So, when we talk about all the major services being catered by NHS , what type of, you know, services we would be catering to?

Anesh Shetty: Yeah, okay. So, largely, you know, the biggest success story in private outsourcing is Ophthalmology and Cataracts. That's well served. The others would be what you'd expect, Orthopedics, a lot of Joint replacement, Arthroscopy, Gastroenterology procedures, General Surgery, etc. You know, these would be the more, think of anything that's elective, short stay and is not life -threatening or critical , it's not, you know, emergency trauma and things like that.

Nidhi: Right. And, fundamentally, what we would be catering to over a longer period of time given that we are going to reduce our dependency on NHS to catering to non -NHS segments? Like, who would be our, you know, audience with regards to the cash payment patients or is it going to be more on insured size where corporates do not have coverage on those insurances where they are going to cater the services?

Anesh Shetty: If I understood your question right, the non -NHS typical patient profile is somebody who has private insurance. This is in the country largely , for many people, employer provided for some individually purchased as well. I think that's fairly similar to what we see in India as well. So, that kind of person would be the target demographic.

Nidhi: Right. And just the last one , On margin side like our consolidated margins would be diluted with this acquisition , so even on a longer term, let's say , we are able to achieve 8 %-9% or maybe 10% kind of margins which the industry is catering at the moment , how do you see this planning on a longer term? Where are we going to find a pace on our consolidated books?

Anesh Shetty: Yeah. Sandhya, you want to take that?

Sandhya J: Yeah. It's a forward -looking view we don't want to give but we have given a certain idea on how we look at the next 12 months margin in the deck that we have shared. We are hoping that we can build on that and improve on the margins. Anesh spoke about all the cost and efficiency variables that we have , so over a period of time we will have to build on these 23 levers. But we have shared a kind of an indicative number in the deck we have put up for the next 12 months.

Nidhi: Right. Got it. Thank you. I'll get back in the queue.

Nishant Singh: So, now we'll go to Nira

njan and Kapil because Ravindra, Prithvi , we've already answered and we'll get back to them again. But , Niranjan, can we have your question, please?

Viren Shetty: Niranjan, can you hear us?

Anesh Shetty: Maybe we move to Kapil and come back to Niranjan.

Viren Shetty: Yeah, fine. Kapil, can we get your question, please?

Kapil: Yes, please. I would just like to get some more clarity on the answer relating to the impact of the acquisition on consolidated EPS. Now, just considering the present profitability rate of Practice Plus Group and after considering the interest to be borne on funding costs, would

this acquisition to the EPS be mild or moderate or more? If you can please give some indication.

Sandhya J: Broadly flat . If at all , mildly positive.

Kapil: I see. All right.

Viren Shetty : That's it, Kapil?

Kapil: Yes, please. Yes.

Viren Shetty : Thank you. Damayanti?

Damayanti: Yeah , hi. Thank you for the opportunity. Just want to understand, you are getting already established doctor's team there and the incremental update which will come from this transaction is that you will bring in more efficiency. So , two points. So , you don't need to add on any doctors, even you don't need any infra in near term . And just with the existing capacity which they have, you try to bring in more technology to drive the growth. Is my understanding correct? At least , this should be the setup in medium term?

Anesh Shetty: Largely. But, you know, once we get in and understand more about the market, if there are interesting opportunities, you know, straight of f the bat we will look at starting other services, new services within the same properties, which would require new doctors. But if there's 24 anything, you know, the company has been exploring certain other opportunities as well. And, you know, we'll evaluate them as and when we get settled in.

Damayanti: Okay. So, there is like a good headroom to add on more services and for that you might need to expand your team a bit.

Anesh Shetty: Yes-yes.

Damayanti: Okay. And just from the segments or categories which is currently served, you mentioned about few segments. So, from NHS outsource work will that be the key opportunity which will continue? Or you think since the backlogs continue to increase at the NHS, you can see more businesses coming in other categories as well?

Anesh Shetty: No, I think the services that usually form the bulk of what private providers do is fairly known and constant. Of course, certain things may change here and there but not in the short term.

Damayanti: Okay. And my last question is, what will be the key offering to you when you target more of these private or self -funded patients, apart from, you know, they don't have to wait for the treatment, which is a case with NHS ? But from your perspective, what will the key pull factor, if you can help us understand?

Anesh Shetty: Sure. So , for the private patients, you know, the competition isn't the NHS, it's other private providers and, you know, the standard levers such as quality, brand, location, etc. But , more importantly, we believe that, you know, cost, either to a private insurance, to a PMI provider or to a self -paid patient cost of treatment is something they're very, very sensitive to. So , if we are able to meaningfully lower the cost and offer an equivalent quality option at a meaningfully different price then that becomes very interesting. Even in markets where Practice Plus is successful in attracting private patients, they're doing so at a price advantage to the others and we hope to continue to build upon that.

Damayanti: Yep. And my last point is , the other players who are in the private space, I understand they might be also investing a lot in technology, right . So, what will be the, you know, key offer from you? Because in markets like Cayman, I understand , you are the only player, right

, and

that's why your focus on technology, etc. , could lead to great results. But maybe in the UK market, things are very different on the competition part.

Anesh Shetty: Yeah . So, Cayman, so we're not the only player . We wish we were the only player in Cayman but, no, there are others . But it's a small market. So, there are a few other players , you are right . UK is a much larger market. So , in different locations, obviously, they're going to be 25 more players. Yes, lik e any business, they continue to invest in technology and some of them have very good systems. And, you know, we believe that the efficiencies that we are able to derive, we hope will be, you know, better than the competition . But let's see.

Damayanti: Okay, this is helpful. Thank you, An esh.

Anesh Shetty: Thank you so much.

Nishant Singh: Okay, can we move to Niranjana now.

Viren Shetty: Yeah, Niranjana, can you hear us?

Niranjana: Yeah. On the previous question you mentioned we are planning to increase the...

Anesh Shetty: Niranjana, we're not able to hear you.

Niranjana: Hello?

Viren Shetty: Maybe you can type your question in the box, Niranjana. We'll move on to Alankar.

Anesh Shetty: Viren , what do you want to do about the questions on the chat? Should we address them offline?

Viren Shetty: Let's finish up. No, w e'll finish up the queue and then we will address.

Anesh Shetty: Okay.

Viren Shetty: Alankar.

Alankar: Yeah, hi . Good afternoon, everyone , and congrats on the acquisition. Sir, firstly, you spoke about increasing the private mix gradually over NHS , you also spoke about margins. Can you

comment a bit about the growth aspect for PPG? Asking this question because if you look at the 5-year CAGR it was about 12% , if you look at the last 2-year CAGR it's about 9%. So , did PPG see any COVID linked bump up in the earlier part of the 5-year cycle? And is the 9% growth , which you've seen over the last couple of years, is it more indicative of what the asset can deliver without any significant increase in the private contribution?

Anesh Shetty: Yeah . So, thanks , Alankar , for the question. So, you know, COVID did not play a meaningfully different role compared to usual for this asset. Having said that, they are on a growth trajectory, like you said, which was in the longer term 12 %-14%, nearer term 9%, with their 26 existing, you know, pay or mix. A lot of that depends on ...By the way, none of this is with adding sites except the Birmingham , which is not yet operational. They've been largely static with the number of sites. So , this you can consider the organic growth year -on-year . You know, we see no reason why that should slow down. In fact, there are certain interesting opportunities with adding more services now that we are involved within the same properties that would further build upon that. But it's still early days , we'll get a better grip of the growth that we can expect to see on a shorter term year -on-year basis, maybe in a couple of quarters.

Alankar: Understood. The second question is more on the transaction, actually two questions there. Firstly, how should we look at the purchase price allocation for this acquisition? Possible to share any broad split between goodwill and intangibles?

Anesh Shetty: Sandhya .

Sandhya J: It is too early actually , we are still working through with the auditors. We have just signed the transaction , after closing the auditors will go through the details and then we'll have a better view of this. So , we are not in a position to answer this question at the moment but we will answer as soon as we have better visibility on this.

Alankar: Got it. And the final one, apart from the 150 million pounds debt, will the rest of the acquisition be funded entirely by Cayman's balance sheet?

Sandhya J: The entire acquisition is be en funded by Cayman balance sheet. 40 million is going as equity , th

e rest of it is a leveraged buyout and , therefore , it is a debt on the books of the target.

Alankar: So, no India balance sheet cash will be used to fund the acquisition ?

Sandhya J: No.

Alankar: Okay, understood. Thank you . That's it from my side.

Viren Shetty: Thanks , Alankar. Niranjan, would you like to speak?

Niranjan: Yeah, am I audible to you?

Viren Shetty: Yeah, go ahead.

Niranjan: Yeah, for the previous question you mentioned we are planning to increase the non -NHS part, right . What are the strategies and plans for increasing the non -NHS part?

27 Viren Shetty: Sorry, strategies for increasing?

Anesh Shetty: The non -NHS , the patient mix .

Niranjan: Yeah, yeah . Yes.

Viren Shetty: That has been answered already. I think just in the interest of time, maybe we'll move on to the other questions. Any other question , Niranjan ?

Niranjan: Yeah . And also one more question is, the PPT I saw , it is the 4th largest player in NHS and for the other 3 players what is the percentage share of non -NHS part?

Anesh Shetty: So, that also we answered previously. Yeah.

Nishant Singh: Thanks Niranjan.

Viren Shetty: If we can go to Ravindra, please.

Ravindra: Yeah, hello ? Am I audible?

Anesh Shetty: Yeah.

Ravindra: Yeah, t hanks for the opportunity once again. So , just one kind of a broader question , as per last year's Annual Report, there is a cash on books in the company around ■600 crores, sorry, ■400 odd crores. Is there any arrangement between the parties that how this cash is to be used ? Or do we get any benefit out of this deal to NH?

Sandhya J: We are acquiring the company on a zero debt basis. There is working capital of about GBP 5 million that is being left behind in the company. Other than that, the rest of the cash and the debt will be cleared by the seller.

Anesh Shetty: Thank you.

Ravindra: Okay, that's fine. Just have one last question. What is the interest cost that we are going to operate for this acquisition ? Like GBP 150 million , as you said, what would be the interest cost?

Sandhya J: Yeah. We have borrowed at about SONIA plus 200 bps on average.

Ravindra: Okay . Currently I think they are having 6.25 %. I think it's a floating interest rate, that's what they are operating.

28 Sandhya J: So, their interest rate is not relevant for us because we are acquiring on a debt -free basis. So , we are acquiring at a SONIA plus 200 bps kind of range.

Ravindra: So, what would be the ballpark figure , just for the calculation?

Viren Shetty: SONIA plus 200 bps , Ravindra.

Sandhya J: 6+. 6 odd.

Ravindra: Okay . Okay, more or less the same range.

Sandhya J: Yeah.

Ravindra: Okay, thanks.

Anesh Shetty: In the interest of time, if we can move on to Prithvi , Viren.

Viren Shetty: Yeah, yeah . Prithvi.

Prithvi: Thanks. I just have a couple of questions. One, going forward if you have to move cash from Cayman to UK, for Capex , etc., is the cash taxable ? And what will be the tax rate?

Sandhya J: So, equity is not taxable. At the moment , we do not intend to move cash from Cayman . The target is sufficiently cashflow positive to take care of its own Capex requirements , Prithvi.

Prithvi: Okay, got it. Second, Viren, I mean, obviously it took 7 -8 years for you to get into another geography , can we assume that for the next few years the focus will be on UK before looking for something else?

Viren Shetty: If you're worried about our inability to stick to one geography, please rest assured the UK is a very large market. There's a lot of learning that we have to make and there is tremendous scope for growth. We are pursuing still in the Caribbean opportunities there because it is highly synergistic with the Cayman hospital that we run as well as in India, which is our core target market. Whatever we're spending in the UK is small compared to how much we're planning to invest in India in expan

ding our core market here. This came about because we

see a good opportunity to grow in a developed country where there's a very large and growing need for private healthcare.

So, we can't give a commitment on 7-8 years but just that we've made a commitment here and we will continue to invest in this.

29 Prithvi: That's clear. One final question. An esh, you mentioned Spire Hospital, that has 35 % payor mix from NHS. If I look at margins, they make close to 18 % EBIDA margin. Is it fair to assume that's something that you will look forward for this acquisition, maybe in 5-7 years down the line if the pay or mix changes?

Anesh Shetty: Yeah. So, the more fair comparison would be to say best -in-class like say Circle, which has the same capital structure, which is all leased assets. Spire has half their assets , approximately half owned and only half leased . So, one would have to account for a lease effect , which I think you've not done to arrive at that figure. But , yes, to the broader question, I think there are other competitors in the market operating at a meaningfully higher level of profitability accompanied with a differ ent pay or mix and we see no reason why we shouldn't get there sooner or just as much. Yeah.

Prithvi: So, j ust squeezing one more question. What will be the realization difference between NHS and the private insurance and the self -pay in UK?

Anesh Shetty: Yeah, i t's a good question. It depends on location, who the hospital is, who the insurer is . But, broadly , for many procedures PMI would pay anywhere from 20 %-30% more than NHS. But it just depends . These are individual contracts , it's not publicly available but it's just a broad industry guidance, this is a consensus number.

Prithvi: Okay .

Viren Shetty: Thanks, Prithvi.

Prithvi: Thanks. All the best.

Viren Shetty: Nitin, you get the last word.

Nitin: Thank you so much and congratulations , team , for a very interesting acquisition. Just a couple of housekeeping questions to start off. One is, from a Capex perspective, with the plans that you have right now, did you envisage any meaningful Capex for the business over the next, say, 3-4 years?

Sandhya J: Yes, the business has regular routine maintenance and refurbishment Capex, which the business will continue to incur . Will be in the range of GBP 10-20 million every year but that's baked into our financial plan.

30 Viren Shetty: These businesses are fully asset light. All the buildings are leased from private lessors as well as the NHS and a lot of the equipment is leased as well. So , the growth will happen in a very Capex light manner.

Nitin: Obviously, very early days for you but do we, for example, Birmingham Center is there , are we on the lookout for adding centers over the next short term or is it going to happen over a period of time?

Viren Shetty: It's a bit of both. An esh can talk about the consolidation and expansion.

Anesh Shetty: Yeah. So, you know, t he Birmingham Center is something that is yet to be fully commissioned.

We're eagerly working towards that. The management has looked at certain centers but nothing that is imminent or that can happen right away. These are things we'll look at. There are one or two oppor tunities. But as Viren mentioned, these are structured , from a capital structure perspective very different from what we're used to in India, where they're very, very light and loaded towards the end just p re-commissioning when you're bringing in medical equipment and they're largely leased and that appears to be how the industry

broadly operates there.

Nitin: And, Sandhya, what kind of cash breakeven period are we looking at for this transaction?

How many years do we start to cash breakeven, from an investment perspective?

Sandhya J: So, it's a leveraged buyout. So, we are positive that we will be able to pay for the loan that we have taken on the books of the target

from the cashflows of the target itself. From an IRR

point of view, it is crossing our threshold IRR; high double digits. So, with that I think you can understand the payback because it's not a number we are officially disclosing.

Nitin: Sure, sure.

Sandhya J: But this I think gives you an idea.

Nitin: Perfect. And from a PBT perspective, when do you think this transaction on its own starts to make a meaningful impact to our consol numbers?

Sandhya J: From the first quarter of full integration it will be significantly material in our consolidated number. But from a PBT point of view, it will take maybe 2 years because of the margin profile of the rest of the businesses.

Nitin: Something like FY28 or thereabouts.

31 Sandhya J: Yes, yes.

Nitin: Okay, thank you so much.

Viren Shetty: There are a couple of questions on the chat. A lot has been answered, I will skip those. In terms of how does this align with NHS global expansion delivery priorities? Anesh had answered that already because this is something that's highly in sync with our value system and it is a growing market and it is a place where it is stable, where it has a very strong and secure market.

Question about are we planning to do Cardiac Care or Oncology? We are not ruling it out. As of this point, the Practice Plus does not offer Cardiac Care nor Oncology, it offers more General Surgery. It's something we'll definitely be in discussions with the NHS as well as with existing service plate but the buildings are simply not configured for this sort of work. So, on the existing asset base, it may not. As we get more comfortable, both in terms of the leverage this has and what expansion would actually look like, we would consider other service lines. EBITDA margin guidance.

Sandhya J: So, we have given a next 12 months broad guidance in our deck itself for the pre-IFRS EBITDA.

The lease costs, you can assume, similar to what is there in the balance sheet today because there aren't any additional lease costs that are coming onboard. So, that will give you a post-IFRS indication. But those are indicative numbers, we cannot give forward-looking guidance.

Viren Shetty: This is a good question. How much of the NHS contract influences or constrains the growth of operations? The answer is it doesn't constrain. This provides the base of every hospital, which assures you a level of breakeven. Everything we do above that is either through volume growth in more NHS contract revenue, adding more departments or increasing the private pay business. So, it is a very useful thing to have for the entire hospital group, and it is something we won't materially alter. In fact, we'll try and grow as much as we can but private pay and other service lines add on top of that.

Nidhi's question, what percentage can afford private hospital services? As of our understanding, in London it is slightly higher but across the rest of the UK it is much lower.

Anesh, do we have a percentage figure that's publicly available?

Anesh Shetty: I mean, not in that exact sense but the amount of people who privately access care has been steadily growing over time. But what we'll be really looking to do is lower the cost of self-pay care and PMI care and, hopefully, that improves this number.

32 Viren Shetty: So, the next question is, what is the component of absolute depreciation for the acquired entity?

Sandhya J: It's about 8 million pounds.

Viren Shetty: And the net assets that we'd be acquiring?

Sandhya J: About 30 million pounds. The rest are Right to Use assets.

Viren Shetty: Yeah. The next question is on impact on near-term EPS. We said it's slightly positive to neutral.

Vinay's question, is there a correlation between NHS waiting lists and hospital capacity? I think Anesh answered that. The waiting list is more in terms of ability to pay. The NHS waiting lists are because they are

challenged in the current way the whole thing is structured and their ability to pass through. It does mean there is an assured business, yes, because as a more efficient low-cost operator, the NHS would be very happy to partner with hospital groups like us to be able to clear the waiting lists and it's something that we will continue to grow on.

The debt level on consol level.

Sandhya J: Sure. So, we have given the debt that we are taking on the books for this acquisition, we've also given the long-term debt plan for India and Cayman. We are in silent period and we cannot disclose a forward number at the moment but I think you can calculate with the data

that we've made available.

Viren Shetty: As for the question of other QIP plans, as we mentioned earlier, there is no real need for it now but it's not something we will rule out at some point in the future because we do not want the debt level to constrain our expansion, provided it's done in a measured and conservative manner.

Nishant Singh: So, as there are no more questions, we would like to conclude our session. There's one more question on the chat.

Viren Shetty: Whether Tamil Nadu is in expand ...

Sandhya J: We cannot comment right now on the NHS business strategy because we are in silent period.

If you have any specific questions, please dial into our Investor Call in 2 weeks from now and we will answer questions relating to NHS strategy.

33 Viren Shetty: But if you have a hospital in Tamil Nadu that you'd like to sell us then please get in touch.

Nishant Singh: Okay, thank you everyone. Please feel free to reach out to us in case you have any further follow-on questions. Thank you.

END OF TRANSCRIPT

Call: Feb 2026

Date of submission: February 24, 2026

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code –539551(EQ), 975516 & 976418 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051

Scrip Code - NH

Dear Sir / Madam,

Sub: Transcript of Earnings Call for the quarter ended December 31, 2025

In relation to earnings call of the Company held on Tuesday, February 17, 2026 for the quarter ended December 31, 2025, please find attached the transcript of the said Earnings Call.

We wish to inform you that the Earnings Call transcript is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earnings-call-audio-and-transcripts>.

This is for your information and records.

Thanking you,

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

“Narayana Hrudayalaya Limited
Q3 FY26 Earnings Conference Call”
February 17, 2026

NH MANAGEMENT TEAM:
MR. VIREN SHETTY – VICE CHAIRMAN
DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER & MANAGING DIRECTOR
MS. SANDHYA J – GROUP CHIEF FINANCIAL OFFICER
MR. R. VENKATESH – GROUP CHIEF OPERATING OFFICER
DR. ANESH SHETTY – MANAGING DIRECTOR , OVERSEAS BUSINESS ES
MR. RAVI VISHWANATH – CHIEF EXECUTIVE OFFICER , NHIC
MR. NISHANT SINGH – VICE PRESIDENT , FINANCE , M&A & INVESTOR RELATIONS
MR. VIVEK AGARWAL , SENIOR MANAGER , IR FUNCTION

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Nishant Singh: Good afternoon , everyone, my name is Nishant Singh and I welcome you all to the Q3FY26 earnings call of Narayana Hr udayala ya Limited. To discuss our performance and address all your queries today, we also have with us Mr. Viren Shetty, our Vice Chairman,

Dr. Emm anuel Rupert, our CEO and MD, Mrs. Sandhya Jayaraman, our Group CFO, Mr. Venkatesh, our Group COO, Dr. An esh Shetty, MD of our overseas businesses, Mr. Ravi Vishwanath, CEO of NHIC and Mr. Vivek Agarwal, Senior Manager in the IR function.

Before we proceed with this call, we would like to remind everyone that the call is being recorded and the transcript of the same shall be made available on our website as well as on the stock exchange at a later date. We would also like to remind you that everything that is being said on this call that reflects any outlook for the future or which can be construed as a forwarding statement must be viewed in conjunction with the uncertainties and risks that we face.

As a special request for this time, as we now have multiple business streams across the globe, we suggest we spend the first 30 minutes Q&A on India and the rest 30 minutes on the UK and Cayman piece .

With that, now we would like to start the Q&A. I would request everyone to now use the 'raise hand ' feature to start posing their questions.

Yes, Prithvi , please go ahead.

Prithvi: Hi, you know, congrats for a good set of numbers.

Let me start with India first because, you know, you mentioned that.

This is the second consecutive quarter where we have seen a very high profit growth for India business. I mean, it looks like finally we're benefiting from the initiatives that we have been taking over the last few years. The margin expansion that we saw in India business over the last two quarters, which is almost 150 -200 basis points on YOY basis.

Do we expect the same trend to continue for a few more quarters? Do you think still there are levers for margin expansion in I ndia business?

Venkatesh R: I'll take this. See, we've been putting a lot of effort over the last couple of years o n our transformation programs, our p ayor mix optimization initiatives. So , the effect of our 3 transformation program has seen results now where patients are opting for higher rate configuration keeping our volumes and occupancy intact. Also, with a lot of technology infusions and increased volume of robotic cardiac surgeries and other procedures, the realizations have impr oved meaningfully , resulting in higher revenue and better margin. And as I said, payor mix optimization initiatives consistently helping in building up on the margin and increase in realization. Though , we can't have a specific indication or guidance, our efforts will always be to maintain these margins we've realized in the last couple of quarters, except for unknown short -term impacts.

Prithvi: Got it. Yeah. A follow up on this. The losses that the company has been making on insurance and clinics has been coming down in the last few quarters. So , when should we expect breakeven for this particular business segment?

Viren Shetty : I'll ask Ravi to answer and then I'll follow up at the end.

Ravi Vishwanath: Yeah. So, you know, we are still in building stage in these businesses , Prithvi . So, right now our focus is on making sure that we are attracting customers and taking care of them and building out the various propositions for them. So, that's our focus right now. I think it's a little bit early for us to talk about breakeven on this. But I'll request Viren to add any further comments.

Viren Shetty: We are balancing out the scope of expansion of our clinic program across the country and merging it into NHL. So , we're better able to manage the synergies and costs between two entities. So, the diluted impact should minimize over the coming quarters.

Having said that, this is a business that sti

ll we want to invest in and build out across all our core geographies. So , there will be some amount of margin dilution going forward. We will call it out in the investor deck and you'll get a sense of how much we are spending on this. But too early at this point to tell when the breakeven will be achieved.

Prithvi: And so , it looks like, you know, at least we are behind the peak losses , okay . I have a couple of questions on Cayman and UK, but, you know, I'll join back in the queue.

Viren Shetty : Thanks.

Just to repeat, first 30 minutes will be for India questions. Those with India questions, please raise hand.

4 Nishant Singh : We have Raj it with his questions. Rajit, can we have your question, please?

Rajit: Yeah. Just a small request. And if it's possible to present the financials of each of the three entities in a proforma way, the way you, you know, file your financials with the exchange.

And you can have, I mean, they could be unreviewed or unaudited as well. Will that be possible going forward?

Sandhya J: We are presenting relevant information in different segments. I think this is the model that we will continue with. However, if you have any specific questions on how to understand the numbers from our investor deck, you can set up time with our IR team and they'll be very happy to help you construct your entity wise P&L.

Rajit: All right. Thanks a lot. And just a quick, quick question on an announcement that was made some time back on setting up a subsidiary to look after some initiatives in North. I

think you announced a subsidiary being set up for this specific purpose. Am I right?

Viren Shetty : Yeah.

Rajit: Could you elaborate on that? What are we looking at? You've been present in North for quite a long time. Is there anything specific that you're looking at?

Viren Shetty : Nothing that we can disclose as of now. But the North is an area of interest for us. And it's something that we're looking to see what we can do there.

Rajit: Okay. Thank you.

Nishant Singh : Nitin, can we have your question, please?

Nitin: Hi. Thanks for taking my question. Wanted to know, this quarter we've had a pretty strong growth in the Bangalore cluster. So, anything which sort of stands out in terms of what has gone differently in Bangalore this quarter?

Venkatesh R. : Yeah. So, I have already mentioned in the previous question about how the transformation has given the results for us in mainly our flagships, where higher realizations have come out from the high level of beds. Of course, and that again, I'm repeating on the payor mix optimization which has consistently helped our flagships and including Bangalore cluster to work constantly on increasing realization. Plus, the most important thing is the high-end robotic work aided with technology across all these specialties, including cardiac surgeries, which have improved our margins and also the 5 volumes. Plus, a lot of emphasis have been put in around Bangalore – urban /rural and also in the northern parts of Karnataka to have more footfalls coming in from the domestic segment

As we specifically mentioned, our whole emphasis going forward will be to consolidate on the domestic volumes and revenues. And that's exactly what we've been doing over the last 6 quarters. And all these together have improved our volumes, margins, realizations and revenues in this quarter, if you compare on a year-on-year basis for the Bangalore cluster.

Nitin: And, Venkatesh, does it become a template for the other clusters or this is more of a Bangalore phenomenon that we've seen, you know, some of these initiatives you're talking about?

Venkatesh R : So, this is the same template we're going to follow for all our clusters, including the eastern cluster. They are also following suit in terms of how the margins and the realizations are working, because these are the two major clusters where

our flagships

are there. And we will continue to work towards the same type of objective in the north cluster as well. There is a little bit of a gap which we have to cover up there, but with the way things are set up for the north, this is going to be the template for all our regions going forward.

Nitin: And on that point, you know, on the northern cluster, you know, there has been a little bit of, again, it's sort of quite contrary to the way Bangalore played out. Anything that you want to call out on what sort of kept the growth a little soft on the northern cluster this quarter?

Venkatesh R : Yeah, we have been a bit cognizant on the receivable problems in some of the scheme payors and also on the capping on reimbursement of certain drugs, which has resulted in a conscious call for controlling volumes on schemes. Plus, a little bit, I mean, constant efforts towards optimizing the payor mix has resulted in volume reduction in schemes. We have yet to catch up on the preferred payor, but of course, the volumes will catch up soon. But having said that, this optimization actually has led to an increased realization and revenue in spite of slight dip in volumes. Plus, increased competition from newer hospitals in the region around north has also contributed to a bit of a shortfall. We are confident of overcoming this through our active marketing and optimization strategies over the short period of time, because it's just time bound and I don't think this problem will persist beyond a quarter or a couple of quarters.

Nitin: Okay, thank you so much.

Nishant Singh: Alankar, yes, please go ahead with the question.

Alankar: Hi, good afternoon, everyone. One question on Bangalore and contrasting it with some of the other clusters. So firstly, you spoke about following the same template in the other clusters. If I look at the ARPP in Bangalore, it's significantly higher than other clusters, including Kolkata, as well as the two hospitals in Delhi and Guwahati. I just wanted to understand, even once you try and bridge that gap and follow the same template in, say, Kolkata, the east cluster, as well as Delhi, NCR. Structurally, is there anything different which is happening in Bangalore on case mix or payer mix, which is likely to keep the realizations in Bangalore significantly higher than these two other clusters going ahead, assuming those changes which you mentioned are incorporated over the next few years in these other clusters.

Viren Shetty : Yeah, Dr. Rupert will answer this.

Dr. Emmanuel Rupert : Yes, Bangalore compared to the Kolkata cluster, so you are going to see these kinds of

numbers . Especially in robotic cardiac surgery, bone marrow transplants , all these are very large numbers here.

Sandhya J : In the last quarter, and in fact, past few quarters, we have done the largest robotic cardiac surgery in the country, largely from our Bangalore unit. Similarly, we continue to do the largest volumes in terms of bone marrow transplant, in terms of several advanced procedures. So that comes in at a higher realization .

Alankar: Hello. Can you hear me?

Nishant Singh: Yes.

Alankar: Okay, so, my question was not specifically for the third quarter. But yeah, I mean, structurally also, I think some of the points which you mentioned are fine.

The second question was, if I just look at Bombay, the Mumbai hospital, you had spoken about trying multi -specialty there or adult multi -specialty there earlier. Any update on those plans?

Viren Shetty : Yeah, we're still working with the trustees and the charity commission on getting the licensing shifted .

7 Alankar: By broadly, when can we expect any progress there, Viren?

Viren Shetty: We don't have a timeline on this.

Alankar: Okay. Fair enough. That's it from my side. Thank you.

Nishant Singh: Can we have the next question, please?

Viren Shetty : There was a chat question, whi

ch is, does the OP consultation doctor revenue count as part of the overall OPD revenue? How do we track it?

The question is we track it internally, but OP consultations are a very small part of the overall OPD revenue. And all of that payout goes towards the doctor.

If we have no other questions on India, we'll probably move to Cayman. So , could you raise, anyone raise their hands for questions on Cayman for Q3?

Nishant: We have a question from Damayanti. Damayanti, please go ahead.

Damayanti: Hi. Thank you for the opportunity. I just have one question on your India business regarding the competition scenario in Bangalore market. So , we are seeing a couple of competitors expanding their presence there. So , from your perspective, how do you see these dynamics to play out for your business? Thank you.

Viren Shetty : There is enhanced competition. A lot of new hospitals are coming out in Sarjapur area and in North Bangalore. We currently don't have hospitals there, so it's not easy for us to comment on the impact it has. But just broadly, if you are seeing that Bangalore is a large market, it is well served. More hospitals would serve the community even more. There may be a lag between any new hospital that comes up and the time it would take to break even and the business practices that have to be followed to fill up those beds. We would say like all competition has definite short -term impact in terms of enhanced cost and time to break even. But long term, it evens out because still all the organized corporate hospitals put together are barely able to service the true demand that exists. But the lag exists because not everyone gets treated for the procedure that they require. Not everyone is aware that they may be suffering from underlying chronic or any sort of life - threatening condition.

Damayanti: Sure. And in your flagship hospital, the majority of volume will be the local population volume or you see mostly the out -stationed patients for high -end procedures, etc.?

8 Viren Shetty : Most of the business we get comes from within a 15 -kilometer radius.

Damayanti: Even the high -end transplant, bone marrow surgeries, etc. that is within this 15 -kilometer catchment ?

Viren Shetty : That is two different questions. So, bone marrow transplant, yes, that comes from across the country. So, that will have a very high representation from eastern India. But for very high -end cardiac procedures, they are more represented by people traveling locally.

Damayanti: Okay. Thank you.

Nishant Singh: We have some questions in the chat.

Viren Shetty : All right.

How do you see the oncology payor and revenue mix going forward? The oncology started from a very, very low base to becoming our second highest specialty. It is the fastest growing department. We believe going forward, oncology and cardiac will account for more than half of our revenue going forward. But as to what percentage share it will constitute going forward, it will be hard for us because with the newer hospitals, the case mix may skew slightly differently. So, cardiac at a third will continue to remain our largest department. Our oncology could go up possibly another 20% depending on the years going forward.

Nishant Singh: Question on the vision objective, Viren, what do you want to be in the next five years?

Viren Shetty: We will take that last. ARPOB in oncology, we do not break out department -wise ARPOB .

Nishant Singh: Yeah. Vinay, do you have any questions India specific?

Vinay: Yeah. Just wanted to know, your gross return premium has really gone up quite

significantly this quarter. Is there any, I mean, how many new policies have we done or what exactly has led to this expansion?

Ravi Vishwanath: It is a combination of things. I mean, so, our retail business where we started, that is the productivity there and the acceptance of that in the market has been increas

ing. As we

told you last time, we also started offering business outside of Bangalore. So, we have got Kolkata and Raipur and Mysore also available. We have also entered the SME market where we are looking at small and medium enterprises and providing them with an integrated approach for not just hospitalization but also comprehensive care which also includes outpatient care, consultation, medicines, et c. And that has been appreciated quite well by our customers. And those are some of the things that have been driving our performance this quarter. And yeah, we continue to work hard to keep that trajectory going.

Vinay: Any numbers that you could share if you have plans for FY27 in insurance?

Ravi Vishwanath: No, we are working through those things now. But we continue to be optimistic about the pace of growth in insurance. And we think there is quite a large market for it, especially for an integrated approach which combines hospitalization and primary care at our clinics as well as at our hospitals. We think that is a proposition that is, [a] unique and, [b] that is relevant and resonating with the market. So, we are quite excited about the future growth. We do not wish to comment right now on next year's numbers.

Vinay: Okay. You are now looking at delinking it from the NHIC. So, therefore, going forward, NHL will be reported as independent of the business of the care, correct?

Sandhya J : Yes, correct. The insurance business even otherwise we are reporting out separately only in our investor deck. And integrated care we are reporting separately. Integrated care will merge into NHL. The insurance business we will continue to report out separately.

Vinay: Would we get some color on the profitability of that business, the insurance business, or is it too early to comment on that?

Sandhya J : Yeah, we have given the integrated care losses. At the moment, we are giving that out as part of our investor deck. Once the merger happens, we will report out the profitability of the insurance business separately.

Vinay: Okay. Thank you very much.

Sandhya J. : It is not very substantial right now.

Vinay: I understand. Yeah, that is understood. Yeah. Thanks a lot.

Viren Shetty: While we wait for analysts to populate for the India questions :

Any plans of diluting stakes to offset debt? No. Our view on Gurgaon and Delhi Hospital profitably aspects an ability to fill the beds from a competition viewpoint, giving multiple 10 large players are expanding already and have an existing presence. That has been our biggest challenge. Gurgaon, there are much larger hospitals than our existing Gurgaon Hospital . It has been quite challenging for us, as no doubt all of you have been aware. We have done a lot of things to improve profitability. We have done a lot of cost optimization and we run a lot of efficiencies within the overall network to make the hospital break even and run in a sustainable manner that delivers very high quality of clinical care. Its path going forward could not get more challenging if more hospitals come in. It would continue on its current path. But yes, this is something that is a challenge faced even by the largest hospital in Gurgaon, which is every incremental bed does have a short-term dilutive impact. But over the long enough time frame, there is still sufficient demand to fill up these beds.

There was a question on a reason for such a high increase in salaries and doctor fees. I'm guessing the person who brought this question up would have been looking at the consolidated numbers, which adds the UK to that. But from India mix, we've actually improved the doctor cost as a percentage of the overall payouts.

The question on sharing occupancy rate for the current quarter. This is a number we are moving away from. We are not in the hotel business and occupancy matters less to us as the overall patient volumes that come in.

Nishant Singh: One more on the insurance. Just wanted to know whether we opted only in Bangalore and Mysore markets for insurance segment. Which other markets do we look to tap for insurance segment?

Viren Shetty : We have expanded to Kolkata and we will be slowly expanding to Raipur as well. Over time, we want to operate our insurance plan in all the markets where we have a significant physical presence. But we will be opening it up phase wise.

Question is, are we looking at growing the pharmacy business? The pharmacy is an integral part of the NHIC clinics. So , pharmacy as a proportion of business within NHIC is quite high and that's how we will be growing it. Would not be running a standalone pharmacy business in a big way.

Nishant Singh: I think we should come back on the online questions. Prithvi, can we please have your question?

11 Prithvi: Yeah, thanks. Before getting into Cayman, I just have one question on India business.

Given that you mentioned you will implement the similar template even in Kolkata cluster, how many years it will take for Kolkata cluster ARPP to reach closer to Bangalore? Just to get a sense, you know, how many years it will take for you to implement all these measures?

Viren Shetty : Yeah, Prithvi, I can answer that very quickly. The hospitals in Kolkata , given the payor mix and sort of patient -led after, will always be at a discount to the Bangalore hospitals.

Prithvi: I mean, yeah, I understand there will be a discount, but I'm just trying to understand the extent of discount because the way the Bangalore ARPP has risen in the last few years. We expect similar trend to happen even in Kolkata .

Viren Shetty : Not in the near term. The Rajarhat hospital, which we are planning as a flagship health city, built along the same lines as the health city in Bangalore with modern construction and the best equipment and getting very good infrastructure, should serve to fill up that gap a little bit, but it will still be diluted a lot by the impact of our older hospitals there.

Prithvi: Understood. Viren , just one final question on India business. You think before the new hospitals get commissioned, can you sustain this double -digit revenue growth momentum, or you think the growth rates might moderate by FY29 before you commission a new hospital?

Viren Shetty: The like-to-like hospital growth , we believe definitely should be able to sustain. There will be quarterly variations, barring any kind of major adverse events. Say, for example, should a hospital poach an entire clinical department or anything of that nature, there's no reason that the same hospital growth should not be sustainable till the new hospitals come online.

Prithvi: Okay, thanks. Can I move to Cayman now ?

Nishant Singh: Just a couple of more questions on the chat, Prithvi, and then we can start with Cayman .

Viren Shetty: Yeah, so there's a very quick question on INR 1000 crore capex to be funded. It will be internal accruals and debt. The number is actually closer to INR 3000 crore , but the answer is still the same. Any other chat questions?

Nishant Singh: One more on the expansion plan , the question was there.

12 Viren Shetty : All right , Prithvi, we move on to Cayman .

Prithvi: Anesh, on the Cayman revenue, I mean, especially for the hospitals right now, we are at \$45 million. I know occupancy is not a right metric to look at it, but can you give some data or some number that will help us to understand how are we with respect to the percentage of full potential for Cayman hospital business?

Anesh Shetty: There are two aspects to that, Prithvi. One is the local market; one is the international market. The international market, obviously, we have no way of quantifying how big it is. We just know the progress we are making. Locally in Cayman , we know that the government hospital is still larger than us in terms of revenue.

So, of course, there are certain structural reasons for that. They have an exclusive right over the entire payor class that we don't have. There is another private hospital that also does well. So, we know that there is room to grow. A bit tricky to put an exact number to it, but there is still market share to be had.

Prithvi: Got it. And on the insurance side, I mean, despite having higher revenue for Cayman insurance this quarter, we saw even losses widening on sequential basis. I mean, what explains that? And also, I think last quarter or a quarter back, you mentioned by Q4 or Q1, you might reach break -even for the Cayman insurance. Can you update on that?

Anesh Shetty: Sure. I think even when we spoke last quarter, like we said, it is quite challenging to have a quarter -on-quarter predictability in insurance loss ratio. There will be large claims and things like that. There'll be quite a bit of volatility. If we take a rolling couple of quarters that should give a better picture.

Having said that, up until now, our focus has been on aggressively expanding the size of the book, which we have been successfully able to do. From the coming quarter onwards, the focus will be more on improving our underwriting performance and improving the underlying processes, as well as the clinical decision -making better. But we achieved where we wanted to get fairly quickly ahead of schedule in terms of the size of the book that we have. We have most of the marquee clients. Now the focus will be on optimizing the book that we do have.

Prithvi: Is it possible to give market share number for the insurance business?

Anesh Shetty: It's actually publicly available on the Monetary Authority website. One can derive it with a lag of a few quarters because it's not up -to-date. So even we wouldn't have the most

13 up-to-date figures, but with a couple of quarters ' lag, one can understand the size of the

market.

Prithvi: Okay, fine. I'll take from that. I have one more question on UK. I'll join back in the queue.

Viren Shetty: Okay, so there was a question on, this is more Group level. There are some previous questions in the chat. Are there targets for net debt to equity? Nishant, if you can take that.

Nishant Singh: We track the ratio of net debt to EBITDA on the consolidated basis. And our endeavor is to maintain a number below 2.5.

Viren Shetty: Another point that came up is I mentioned poaching of entire departments. This is anecdotal. It has not happened to us. There are doctors who leave for various reasons, such as relocating to cities where they would like to move to be with their families. I was just using this to illustrate. But our doctor attrition at the senior level is high single digits. It's quite low.

Okay, this question has come up. Vision, objective, goal in Narayana. What do you want to be in the next five years? And where you'd like to be?

The vision is, as Dr. Shetty had always defined for us, which is building a world-class healthcare institution that provides accessible, affordable care for everyone who comes in. The objectives are to build a healthcare institution that's able to deliver on that. The goals are how we achieve those objectives. The goals used to be bed driven, which is chasing after having the largest presence and the largest number of beds all over the country. We found out that using that as a route to getting to our objective was diluting it a lot because we entered into markets where we had very little presence and recognition and we were not able to execute well. As of today, what we are working on is consolidating our presence in our core markets, starting with Bangalore and Delhi. And from there, the other markets where we have success, such as Raipur, Ahmedabad, Jaipur, Delhi, Mumbai, etc. We are growing there with a combination of hospitals, clinics, and insurers. And we will also be offering our

integrated care offerings to patients so that we can offer health care services to them throughout the year rather than them coming in for cancer and cardiac services.

What we would like to be in 5 years in our core markets? As a significant operator with a presence so that wherever you are at least in Bangalore or Calcutta you are never more than 25 minutes away from an NH Centre, be it a hospital or a clinic. With those points of 14 presence we would then work towards earning the trust of our patients, increasing our market share and total overall health spend which is money spent in clinics, pharmacy at procedure level and health insurance. The steps we will take to do it would be the combination of all the offerings that we have invested in.

Nishant Singh: I think Damayanti has a question. Damayanti, please go ahead.

Damayanti: Hi, I have a question on the UK operations. Shall I go ahead?

Nishant Singh: Yes, please.

Damayanti: So, we have some data available in the presentation for the UK operations. And, when we look at the profitability, that obviously is significantly below your India or Cayman operations; and we understand the market is different there. But from your perspective or strategies, what are the key points which you will focus on to improve margins from here on, and reducing the gap between what UK operation has in terms of margins versus the console numbers?

Anesh Shetty: Yeah, thanks Damayanti. So, as you identified in the beginning, you know, every market will have its potential, we don't think that the profitability of what the operation in the UK will ever reach where we are in Cayman because they're very different markets, very different risk profile.

Secondly, in terms of what are we going to do? So it's been about a little over a few months since we acquired the company and there are quite a few opportunities to implement essentially our entire technology platform and what we've done with Cayman from India, which is a lot of operational process level efficiencies related to both clinical and non-clinical functions, as well as the company has a very, very small revenue composition from non-NHS sources i.e. private insurance and self-pay. Those tend to yield higher realizations from a like-to-like basis compared to NHS. There are some initiatives related to growing that market share, the private market share. Those will also help meaningfully contribute to the margins along with revenue growth.

But in a summary, the broad idea would be a much larger scaled version of what we've been able to do in Cayman, which is essentially implement our technology platform and other operational efficiencies, but at a larger scale.

15 Damayanti: Sure. And these majors will take, say, how much time before we start seeing some notable changes happening in the UK numbers? You do have avenues, but in general, shall we assume 2-3 years or even higher timeline to see these initiatives to bring fruits?

Anesh Shetty: Yeah, I don't think we'll have to wait two to three years to start seeing results, but obviously, to do a lot of what we can do will take some time. But we should start seeing

early results trickle in. No guidance on exactly how long that will take, but I don't think we will be waiting 2-3 years to see benefits start flowing in.

Damayanti: Okay, and my last question is , in these UK setups , it's all local teams, right? In terms of doctors as well as non -medical teams, it's the local?

Anesh Shetty: Yeah, absolutely.

Damayanti: Yeah. Okay. Thank you.

Nishant Singh: Yes, Vinay. Please go ahead.

Vinay Nadkarni: Yeah, I just wanted to check out, you have been mentioning this Birmingham unit of the UK operations. How big is it and how long will it take to come out of the losses there?

How long will it take to get completed and completely operational?

Anesh Shetty: Sure. So, the hospital is operational, but recently so.

It is a hospital that the erstwhile

owners had acquired from another health system as part of a divestment. So , it has been a hospital for decades, but it was largely neglected for a long time. So , under our ownership ... sorry, under practice plus ownership, it's been about a year, year and a half , and NH for the past few months. So, the hospital is fully operational.

To your question about how long it will take to come out of our losses, we've always hoped that such an operation would take about four quarters or one year. It's been half that time. We will continue to monitor it. There are some positive changes on the ground, but it is still a new market for us and a new asset for the company that we are still getting our hands around.

Vinay Nadkarni: In size, is it bigger than the average PPG hospital?

Anesh Shetty: No. No, all the hospitals are more or less the same in terms of template. There are minor variations here and there, but in terms of number of square feet or number of operation theatres or beds, etc., they are very little variation between them. Yeah.

16 Vinay Nadkarni: Okay. And lastly, would you be required to put in some money on CapEx in Birmingham or is that all done already?

Anesh Shetty: No, that's done. There are some minor equipment that will be coming online in the next few weeks, but the bulk of the investment was made before. Nothing, nothing major.

There are some sterilization units, etc., but nothing that was left for us.

Vinay Nadkarni: Okay. Just one last question on the total. It is net 183 million GBP, right? How much of it is equity and how much of it debt? You may have mentioned that in the past. Maybe if you can just repeat it.

Sandhya J: I'll take this. We have taken debt GBP 150 million on this. I also want to take another question here, which is on the repayment of the debt. We have a 2 + 5 years repayment schedule over the period of which we aspire to repay this debt.

Vinay Nadkarni: So, it is GBP 33 million equity and GBP 150 million debt, is it?

Sandhya J: We had put in GBP 45 million equity because there were also deal costs which we had to spend on. So, GBP 150 million debt and GBP 45 million equity is what we have put in. But what we paid was GBP 183 million net, after the netting of the cash, which was there in the entity.

Vinay Nadkarni: Yeah. Thank you. Thank you.

Nishant Singh: Prithvi, do you have any follow -on questions on UK?

Prithvi Raj Earle: Yeah, I just have one question. Anesh, this is again on UK. Setting it would have been a couple of months for you taking over the business. Are there any shocks that you're facing because it's a new geography, etc., or is it fairly , I mean, or relatively easy for you to implement whatever you wanted to implement it?

Anesh Shetty: It's still too early to say, Prithvi. So, fortunately, no bad shocks. But it's been about three months. We have a good idea of ... essentially, we've scoped out a lot of the process changes we're going to be making ; a lot of the digital applications and the rollout of certain transformations that we're going to be doing. In terms of how hard it is to roll these out, we'll know in a few quarters. But so far, we're fairly optimistic. I don't think there's any negative surprise, thankfully, yet.

17 Prithvi Raj Earle: And you think there are many low -hanging fruits for you to implement in the first few quarters?

Anesh Shetty: We definitely will get started. There are obviously some initiatives that are easier than others, some that will be quicker, some that will take a longer time. But I think, in a few quarters, we will get a better sense of the timelines as well as a better quantification of these things. We have a broad sense of where we're going. And internally, obviously, we do have a roadmap for what we'll do when and when we expect these synergies to start kicking in. But nothing to share as of now.

Prithvi Raj Earl

e: Okay. Thanks, Anesh. All the best.

Anesh Shetty: Thank you. Vinay, I think you have your hand up.

Vinay Nadkarni: Yeah. Just one more question on UK. You mentioned about there being 4 to 6 weeks

waiting time for surgeries in UK. Was that because of operational constraints , or is that just the sheer number of people and the capacity to occupy them? Is there a chance of reducing this backlog?

Anesh Shetty: Vinay, when you say 4 to 6 weeks, I assume, are you referring to our waiting time within our hospital or in the NHS?

Vinay Nadkarni: I mean, I'm looking at your deck that you had circulated in November, where it says latent demand 4 to 6 weeks waiting time for surgeries. So, I was just trying to see how quickly can we increase our EBITDA there. So, is that one of the options to go about? Is it a problem or is it an opportunity?

Anesh Shetty: Sure. I'll try and answer the question because I'm not very sure. I'll look back at the slide you're referring to later.

Viren Shetty: Sorry. Anesh , that was the background information. 4 to 6 weeks is NHS waiting list.

Vinay Nadkarni: Yes.

Anesh Shetty: Yeah. So that's much larger. So, it's not 4 to 6 . It's actually ... I mean, the national waiting time for ... depending on which elective procedures, more than 18 weeks to 20 weeks.

And there are some that are quicker. But essentially, the concept that we shared was that there is a waiting time for elective surgeries, which is more than ideal in the public health system. That's the opportunity that exists for all private operators. So, the motivation for 18 patients to pay out of pocket rather than get good healthcare free is the waiting time and the quicker access in the private sector. So, this is something that all private operators are looking to capitalize on. And this is particularly related to certain procedures such as joint replacements, cataract, you know, other orthopedic procedures, general surgery, etc.

Vinay Nadkarni: Okay. So, it makes sense to keep with that long waiting list.

Anesh Shetty: No, it's not up to us. That's the restriction that the government ... the public NHS hospitals have with regards to their resources available. And that's been a multi-decade problem and it doesn't appear that it's going to go away anytime, anytime soon.

Vinay Nadkarni: Okay. Thanks a lot.

Nishant Singh: Rajit, do you have any questions?

Rajit Aggarwal: Yes. On the UK financials, just wanted a few clarifications on the numbers. So, the depreciation for UK as per the Slide 14 , comes to around INR 40 crores. Now, the balance sheet of the Annual Report of Practice Plus gives a very different number. So how do we understand this? And is this the number which we should take going forward as well , INR 40 crores for two months kind of a number?

Sandhya J: Yes, you should take this number going forward. So , the Practice Plus balance sheet was three legal entities which were there , and this is now after the carve out. There is also the... most of the depreciation is also coming from the leases. And as we consolidated, there was a re-accounting that we did with the statutory auditors in terms of some of the lease charges. So that's why you're seeing a slight ... it's not a very material deviation from the number. So , this number you can take going forward.

I would just recommend that you wait for Q4, where we get the full effect of all the numbers in our P&L. I think that's a good ... Q4 or Q2 of Practice Plus. That will be a good representative of a full quarter number for us.

Rajit Aggarwal: Okay. Okay. Understood. Okay. So similar would be the case for interest costs as well, I guess.

Sandhya J: Yes, interest cost has gone up because we have borrowed. So, that entire borrowing has come on the ... And I think we've given a small schedule on that for clarity.

19 Rajit Aggarwal: Yeah, yeah. Yeah, that's fine. And just a subjective question on the

doctor's expenses and

other employee expenses compared to the rest of you r... I mean, ex -UK. UK obviously has these expenses, much higher expenses, as percentage of sales. So , is there anything which can be done or which you think can be done to bring them lower by any margin?

Sandhya J: You would see our doctor costs, doctor and employee costs, whether you take it year -on-year versus last quarter, or you take it quarter -on-quarter. Quarter -on-quarter is almost flat. Slight increase is there mainly because of the lower revenue in Q3 , and it has improved year -on-year .

Rajit Aggarwal: No, what I meant is as a percentage of sales , it's much higher compared to ex -UK.

Sandhya J: Including UK will be higher, yes, because UK or doctor cost profile is very different. I think for India, you could look at the India slide s where we call out the doctor costs separately .

There is a table that we give.

Rajit Aggarwal: Which is fine. So, my question is, do you think that these expenses in UK can be brought down to a certain extent?

Sandhya J: Oh, in UK, okay.

Anesh Shetty: No, Rajit . So, I mean, essentially, anything we do around improving the payor profile , will lead to a reduction in the doctor's cost as a percentage of revenue. And of course, any other savings we have with regards to clinical efficiency , would also help. That is definitely

in the bucket of what we are targeting, but it's more a mid to long term ambition.

Rajit Aggarwal: Okay. And other employee expenses as well will be similar.

Anesh Shetty: Other employees, there is definitely much more scope. To put it in perspective, compared to peers, the doctor cost as a percentage of revenue is by far the lowest compared to peers. But in the non -doctor bucket, there are a lot of operational efficiencies as our software is implemented , that we hope to realize.

Rajit Aggarwal: Okay, thank you. Thanks a lot.

Anesh Shetty: Should we take some questions from the chat, Nishant?

Viren Shetty : Yeah, Anesh . On UK hospitals, are there expected timelines around payor mix improvements away from NHS?

20 Anesh Shetty: Yeah, I'll read these through and answer them as we go. Again, you know, that's an ongoing ... I mean, directionally, we obviously want to improve the private pay or mix. No expected timelines to quantify that , but hopefully in one direction, which is upwards.

The next question is, in over five years, would NH significantly scale up international presence, blah, blah, blah? Are you open for another international acquisition opportunity? Definitely not for the foreseeable future. I think we have our hands full with this large operation in the UK and what we already have happening in Cayman. And as we've said several times before, the right of first refusal, so to say, for our capital will always be at home country in India, where we are most familiar and where we have the most opportunities to grow and where we are deploying the bulk of our capital presently and over the next five years as well.

The next question is, from an ROCE perspective, why UK? Isn't this ROCE dilutive move in case there is a cap on profitability compared to your Indian operation as NHS share can't reduce substantially? The entire private sector compared to the NHS is a very, very, very tiny percentage of the market, far lower than it is in surrounding European countries or other first world countries as well. Our thesis wasn't counting on the NHS share reducing materially. There is far more than enough to go around for the size of where Practice Plus fits in the private market hierarchy as well. And even a tiny, tiny shift from the massive elephant that is the NHS , has very, very significant positive ramifications for all private players. So, we aren't counting on any drastic moves in the NHS market share.

The next question is for adjusted EBITDA numbers for UK, should we look at post -IFRS or pre-IF

RS as the one that gets into the console EBITDA in NH books?

Sandhya J: Anesh , I can take that.

Anesh Shetty: Yeah, yeah.

Sandhya J: Yeah, correct Anesh . It will be post -IFRS only. The reason we are calling out pre -IFRS, at least for some time we will call out is because it's a substantial number, the lease charges.

So just to give that transparency, we are calling it out separately.

Nishant Singh: There's a question Anesh on revenue seasonality of UK hospitals across each quarter.

Anesh Shetty: Yeah, no, not much seasonality because ours is elective work. There is seasonality depending on contracting with the NHS Trust, but that's subjective for each location and each hospital because the work we do is different from India. We are not a full spectrum 21 hospitals; we only do elective secondary care surgeries. So, there isn't much seasonality here. The variation quarter -on-quarter would depend on contracting relationships with the Trust.

Okay, the next one. Could you... early observations on how the disease burden there differs from India across key specialties, evolving UK demographics, age, migration, etc. No, I don't think this is the correct way to think about it because our UK hospitals and all private hospitals in the UK are not general tertiary or secondary care hospitals like you see in India. The NHS is the primary place where people would go to for what this question seems to be asking about. Private sector providers only do a very narrow spectrum of elective cold surgeries. So, we don't have any specific insights about the question that the gentleman is asking.

The next one. We mentioned last quarter that the UK acquisition is expected to be EPS neutral to slightly positive, even in the near term. Based on the disclosed pro forma financials after the interest and amortization costs, it seems that we will have losses for full year. There are elements of one -timers, but Sandhya, you want to take that question?

Sandhya J: Yeah, so this quarter has been slightly distorted because we had the one -timer of the deal cost also coming in into the UK P&L. We do expect that the path will be flat or mildly positive , like we had indicated earlier. So therefore, we do continue to hold our position that this acquisition will be EPS neutral for the group. Because we're just two months into the business, we are still getting our hands around it. And you have to give us some time to be able to give a more confirmed view on this.

Anesh Shetty: I think, Vinay, your hand is up.

Vinay Nadkarni: No, no. You have answered my question. Thank you.

Anesh Shetty: Okay. Thank you.

Viren Shetty: So, the other question from the chat , on the direction of the doctor related costs over the next couple of years. Dr. Rupert, if you could just address.

Dr. Emmanuel Rupert: It is on track. We don't see a major change in what is happening. And even with the new hospitals, I think there will be some minor fluctuations here and there , but we have it covered as far as that is concerned.

22 Viren Shetty: Your question is, what is the core competency of NH compared to its peers? I think that we do a lot of work on improving the in -hospital efficiencies both, by using operational expertise, by doing streamlining, cost cutting and using digitization to be able to provide a like -for-like experience and world class clinical service at a price that few institutions can match without compromising on the clinical quality. It's not core competency, every hospital is supposed to do that, but we'd like to believe that we do it far better than most. And how it manifests itself is in the almost close to what the industry is able to get on the India levels of EBITDA at a realization that ... average realization that is far, far lower. The expansion plans for India , that's another question in the chat , are, as we have mentioned earlier, it's in the slide in the

investor presentation. The core focus is in Bangalore and Kolkata . That's where the bulk of our spend is going to be. There is some expansion happening in Raipur as well with an expansion to the existing hospital. And we will be adding a lot of medical equipment next year. We plan for four Davinci robots so that all our hospitals become robotic surgery equipped. We'll be adding a lot more oncology services in all the hospitals. So, these are minor investments.

Anesh , how does PPG compare on average revenue for a patient compared with peers in the UK?

Anesh Shetty: Yeah, so private peers in the UK have anywhere from 30 to 50 / 60% of NHS work, whereas we are almost 90% or more NHS work. So, an average revenue per patient, those numbers would be quite, quite different given the pay of exchange.

The next question is around what kind of PAT growth should we expect in next financial year? Sandhya can take it, but we usually don't give guidance.

Sandhya J: Yeah, I think we have given a reasonable view of where we are looking at India, Cayman. India will grow, Cayman will sustain , and UK we are looking to grow. So that gives you a direction of where our EBITDA is headed. PAT will follow the same direction. We will have interest costs coming on and we have given our CapEx plan for India. There is no significant CapEx in Cayman that we anticipate. And UK, it will be largely the borrowing costs that we will service for the acquisition. So this will give you a fair idea of how you could calculate our PAT for the next financial year.

Anesh Shetty: Gaurav has his hands up. Sorry, go ahead Nishant.

23 Nishant Singh: No, no, we can come back on the chat questions. We'll take Gaurav's question first. Yeah, Gaurav, please go ahead.

Gaurav Tinani: Yeah, hi. Thank you and good evening. So firstly , on Practice Plus's margins, if I recall correctly, this business was at a EBITDA margin of 12% ex of the Birmingham asset. And this quarter we've done close to 10%. So, anything that's changed in the business post - acquisition, where costs have gone up ? And do we expect this 10% to stay here or improve again back to 12% going forward?

Sandhya J: The business was always in that 8.5 to 9% range and it continues to be in that range. Over a period of time, Birmingham losses will come down. It has come down also. It will come down further as well. And as far as the base core business is concerned, I think it's just too early for us. We're still getting a handle of the business. So, we'll need some time to comment on it. But broadly, we have not seen any dilution in the performance , in the two months that we have seen or we have taken over the business.

Viren Shetty: If the number was from the Practice Plus disclosures, just know that it accounts for three separate businesses with corporate costs allocated across three different business units. So, there would be a distortion. Once you set up the hospital, then we are fully responsible for that.

Gaurav Tinani: Yeah, we have done that. But you're saying that 8.5 to 9% is the normalized EBITDA ex of Birmingham for now , that this business is ...?

Anesh Shetty: That was always what it was, Gaurav. I'm actually not sure where you got that.

Gaurav Tinani: Got it. Got it. And, you know, you've taken £150 million of debt. And if you've spelt out the interest costs, if I back calculated the cost of debt, is it 4.5% ? Is that assumption, correct? Is the cost of debt for us at 4.5% ?

Sandhya J: It's not a number that we've kind of made available public. But broadly, we have taken SOFR plus 200 bps is the broad range we have taken. Obviously, there are a lot of plus - minus in that number and that's the reason you're not able to see it clearly.

Gaurav Tinani: But it's 200 bps plus. Okay, got it.

Sandhya J: SONIA plus 200.

24 Gaurav Tinani: Yeah. And what was the 2 + 5? If you can just help me understand that

2 + 5 a little better
in terms of tenure?

Sandhya J: We have a two year of moratorium in which we are only servicing the interest for the debt. And then we have a principal and interest servicing for the next five years.

Gaurav Tinani: And is that equally over the next five years or is it, you know, again skewed towards the end of the five years?

Sandhya J: It is equally over the next five years, after the first two years is finished.

Gaurav Tinani: Got it. Got it. Separately on your joint venture where you're looking at health care centers for the treatment of cancer patients and specifically provide chemo services, which geographies would that be, and how many centers you plan to come through with this JV? Any color on this?

Viren Shetty: This is an investment we made in Everhope Oncology, where focus areas are creating chemo centers in Delhi. The first center has come up in Gurgaon. They're scouting for more partners to open up more centers with. The next investment they made is in SSO Oncology, Surgical Service Oncology in Mumbai. And they have three centers and they're looking to expand more.

Gaurav Tinani: So, any investment that we've earmarked for this particular venture over the next three years?

Viren Shetty: No, we've just made the investment. The rest will take a call once we see the trajectory of the existing business and how they're able to scale.

Gaurav Tinani: Perfect. Thank you. All the best.

Viren Shetty: Thanks, Gaurav. Srinath, we can go to your question, please.

Srinath Saravanan: Yeah. Sir, in Bangalore market, as we've seen in the presentation, for the next four years, you're doing an additional bid of around 900 beds. Also, if you see other peers, they're also doing aggressive taps towards the Bangalore market. Do you think there would be enough room for growth in this market?

Viren Shetty: Yes.

Srinath Saravanan: Any color on that, sir?

Viren Shetty: There is room for growth in these markets.

Srinath Saravanan: Okay.

Sandhya J: There was a question in the chat on the ROCE dilution impact because of UK. What we'd like to say is that initially, because of the size and scale, I think we are seeing that dilution.

But a), it's a leveraged buyout. b) it's an asset light model. So, we do believe that the UK acquisition will deliver reasonably strong ROCEs for us.

Our current ROCE is very high because of the assets in India coming up long back, and therefore, the cost of acquisition is lower. There is a normalization that is happening on ROCE at the excluding UK also at a group level. We will still be healthy. We won't be at that very high level we were till last year. And UK will, in the medium term, not be dilutive to the group ROCE.

Nishant Singh: There is a question on the expansion plans for Cayman and UK. Over five years, would NH would significantly scale up international presence based on Cayman UK experience?

Viren Shetty: We have our hands full right now. Until we are able to improve the performance of the UK, there is no scope for a span internationally. We've already spelled out what our expansion plans are.

The question is, when you look at the pay or profile, our government schemes are the highest in the industry. This has always been the case for NH. We cater to the mass market and the government payers are a very large portion of that. We try to balance out our commitments to society, maintaining a healthy mix of different patient base against our cash flow requirements. So the government numbers will be effective.

The question is, are there plans to raise equity capital? Not right now. We don't see a need for it.

Anesh, there's a question on sourcing elective treatment from the NHS board. Would you be able to answer that in the chat?

Anesh Shetty: When you refer to sourcing elective treatments, could you clarify the types of disease and procedure involved?

For the most part, orthopedics would be joint replacement, arthroscopy, some amount of general surgery, gastroenterology and ophthalmology as well. So this is the bulk of what we source from the NHS pool, which is elective secondary care surgeries. That's also the answer to the treatment mix.

The last part of the question, are you primarily focusing on building cardiology in the UK or are you opening to scaling other specialties as well? We will not be building ... starting cardiology services immediately. There are very, very few private cardiology services in the country and especially outside of London. So this isn't a first step forward. There are diversification and enhancements of the existing specialties that we will be doing, such as getting into back and spine surgery, more complex orthopedics, etc. So those would be the first topics we'll be taking up ... the first new services we'll be starting with.

Sandhya, I think the next question is, why does NH come out with its results towards the

end of the period? Is there any particular reason?

Sandhya J: Yeah, so I think this is something that we'll have to work on. This quarter especially was because we had to go through the consolidation with UK and we are still getting the systems in place. But even otherwise in general, I think we come out a little late in terms of how we are able to release our results. This is work-in-progress for us and we may take your feedback and we'll work on this.

Viren Shetty: The next question on the chat was on India business. What is the impact of the increase in CGHS rates? I think we had given this number last quarter. Nishant, do you recall what the impact of enhanced CGHS rates would be? It was a non-material amount for us given our limited presence in Delhi and that we have limited export to CGHS. But the exact number, we can come back to you on later.

All right. If there are no other questions ...

Sandhya J: There was just one small clarification I wanted to give. See, I'm not sure who it was. You pointed out this 12 % versus 8.5 to 9%. I think one small thing is, we were tracking the pre-IFRS number, which is 8.5 to 9%, you're right. Post-IFRS is 12 %. So that has, you've seen a slight moderation in Q3 because Q3 is also like a partial quarter for us and we are still getting complete handle of how the numbers are rolling up. We do aspire to be at that 8.5 to 9% pre-IFRS. Next time onwards, we will start giving the same ambition post-IFRS. I think that will clear the confusion which got created in that answer.

Gaurav Tinani: Sure. Thank you.

Nishant Singh: There are no raise of hands. So, with this, we'll like to conclude the session. And, thank you everyone for the active participation as usual. Thank you.

27 END OF TRANSCRIPT

Call: Jun 2026

Date of submission: June 02, 2026

To,
The Secretary
Listing Department
BSE Limited
Department of Corporate Services
Phiroze Jeejeebhoy Towers,
Dalal Street, Mumbai – 400 001
Scrip Code –539551(EQ), 975516 & 976418 To,
The Secretary
Listing Department
National Stock Exchange of India Limited
Exchange Plaza, Bandra Kurla Complex
Mumbai – 400 051

Scrip Code - NH

Dear Sir / Madam,

Sub: Transcript of Earnings Call for the quarter and financial year ended March 31, 2026

Further to our earlier letter dated Tuesday, May 26, 2026 in relation to uploading the Audio Recording of the Earnings Call of the Company held on Tuesday, May 26, 2026 for the quarter and financial year ended March 31, 2026, please find attached the transcript of the said Earnings Call.

We wish to inform you that the Earnings Call transcript is also available on the website of the Company at <https://www.narayanahealth.org/stakeholder-relations/earnings-call-audio-and-transcripts>.

This is for your information and records.

Thanking you

Yours faithfully
For Narayana Hrudayalaya Limited

Sridhar S.
Group Company Secretary, Legal & Compliance Officer

Encl: as above

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"Narayana Hrudayalaya Limited
Q4 FY '26 Earnings Conference Call"
May 26, 2026

MANAGEMENT : MR. VIREN SHETTY – VICE-CHAIRMAN
DR. EMMANUEL RUPERT – CHIEF EXECUTIVE OFFICER
AND MANAGING DIRECTOR
MS. SANDHYA J – GROUP CHIEF FINANCIAL OFFICER
MR. R. VENKATESH – GROUP CHIEF OPERATING OFFICER
DR. ANESH SHETTY – MD, INTERNATIONAL BUSINESSES
MR. NISHANT SINGH – VICE PRESIDENT, FINANCE AND
INVESTOR RELATIONS
MR. VIVEK AGARWAL – DEPUTY GENERAL MANAGER,
FINANCE AND INVESTOR RELATIONS

Moderator: Good afternoon, everyone, and welcome to the Quarter 4 FY '26 Earnings Call of Narayana Hrudayalaya Limited. Thank you all for joining us today. On the call with us from the management team are Mr. Viren Shetty, Vice Chairman; Dr. Emmanuel Rupert, CEO and MD; Ms. Sandhya Jayaraman, Group CFO; Mr. Venkatesh, Group COO; Dr. Anesh Shetty, MD of the International Businesses; Mr. Nishant Singh, Vice President, Finance and Investor Relations; and Mr. Vivek Agarwal, Deputy General Manager, Finance and Investor Relations.

The results presentation and financial statements have already been uploaded on the stock exchanges and are also available on the company's website. Before we proceed with this call, we would like to remind everyone that everything that is being said on this call that reflects any outlook for the future or which can be construed as a forward-looking statement must be viewed in conjunction with the uncertainties and the risks that they face. Please note that this call is for a duration of 1 hour. We will address questions pertaining to the India business in the first 30 minutes, followed by the international business.

Given the limited time available, participants are requested to ask maximum two questions at a time and join the queue for any follow-up questions. With that, we would like to start the Q & A. Anyone who wishes to ask a question. Please use the Ask a Question tab on the webcast and then there is an option available on Zoom. You may also post your text questions. We will wait for a moment while the question queue assembles. We'll take our first question from Krish Bhanushali. Krish, please unmute your audio and go ahead with your question please. There is that person has dropped off. We'll take the next question from Sajal Kapoor. Sajal, please unmute your audio and go ahead with your question, please.

Sajal Kapoor : Excellent. Okay. So 2 questions. One is, given the increasing importance of integrated care models globally, how should investors think about the role of pharmacy within the broader Narayana ecosystem over time? And whether there is a scope for deeper economic integration of pharmacy within the listed platform, given how central it is to the overall care proposition? That's my first question.

Viren Shetty : By pharmacy, you mean retail pharmacy or you mean pharmacy as like a department?

Sajal Kapoor : So Hrudayalaya, where it's a partnership model, the listed entity doesn't own it outright today. But they are on our hospital premises and they use the brand, et cetera?

Viren Shetty : So we run pharmacies in all our hospitals and in all our clinics. We run it -- when it's in the hospital, it runs under the P&L of the hospital. When it's in the clinics, it runs under the NHIC for now. They fulfil the dispensing functions for people walking in, people who come with a prescription, people who walk into our clinics, avail services, we are able to offer them pharmacy services.

As part of the integrated care model, this is also something that's of tremendous value for patients. And for those who subscribe to our Arya plan, they get access to home delivery services, where on our own expense, we undertake the expense of taking the medicine to their house as well as doing pickups and so on.

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May 26, 2026

But we are not yet, at this point, thinking to create a separate stand-alone pharmacy vertical similar to, say, Apollo Pharmacy, which only does retail pharmacy in a branded franchise model on a stand-alone basis. For us, the pharmacy is the primary fulfilment center for our internal needs in the clinic and hospital.

Sajal Kapoor : Right. So Viren, the shareholders of the listed entity don't get the benefit. That's where I was coming to?

Viren Shetty : No, they do. All the pharmacies of all the

hospitals and all the clinics are part of the P&L of the NHL listed entity.

Sajal Kapoor : All right. That's helpful. That explains. Right. And then second question is, as Narayana expands into adjacent health care services over time, I mean, how does management think about the strategic and economic alignment between the listed platform and the newer ecosystem initiatives that leverage the Narayana brand, the clinical infrastructure or patient network globally.

If you can just elaborate what's the thought process, not just here and now, but let's say, a 5-year view because the group will be expanding into several ecosystem services because we want to be not just seen as a hospital, not as a physical hospital, but a technology-enabled ecosystem of preventive to maintenance to corrective health care, etcetera?

Viren Shetty : Yes. Sorry, Sorry, just to clarify for me, which adjacent businesses you're referring to?

Sajal Kapoor : In future, so, for instance, if we decide to expand the services to introduce something new above and beyond, let's say, hospitals above and beyond, let's say, Practice Plus here in the U.K. So I'm from

U.K., by the way, and so on, right? How -- what's the sort of overarching thought process? Because ours is a differentiated model, which is unlike other hospitals for the simple reason that we do insurance to conflict of interest was our primary objective to introduce the insurance services. That's again within the listed platform, so the shareholders get benefit. But is there a thought process? So I'll give you an example. There is a procurement-related party, which does the procurement on behalf of Narayana.

Viren Shetty : Sorry, is this Samyat Healthcare?

Sajal Kapoor : Yes, yes...

Viren Shetty : This is a subsidiary of the listed entity. All the related party companies that you see are owned by the listed company.

Sajal Kapoor : Okay. So it's not very clear in -- Viren, in the annual report. I think we need to just clarify a bit because I -- so I run a Substack channel, and I'm very active on Twitter. I get a lot of these queries because I've been a 10-year-old shareholder here in Narayana, right? So there's a...

Sandhya J: 100% subsidiary, Sajal, actually, this has been clarified in the annual report. It is a 100% subsidiary. What we can do, Sajal, is that given that many of these are little fundamentals, we can take a separate call with you from the IR team.

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Sajal Kapoor : That's very helpful, Sandhya...

Sandhya J: They will explain and answer all your queries.

Sajal Kapoor : Yes, I've got many more strategic questions, but in the interest of the time, I will drop you an e-mail and we can -- or the IR team rather, and then we can take it from there. Thank you so much for your time.

Moderator: Next question is from Prithviraj. Prithviraj, please unmute yourself and go ahead with your question, please.

Prithviraj : Yes. Hi, I have a couple of questions on India business. One, if you look at ARPP for the Bangalore cluster, I think it's north of 250,000. If you have to ignore NCR region, I don't think there's any other hospital chain that makes this kind of ARPP in any cluster. So we're just trying to understand what explains this probably a bit more detail on the kind of high-end surgeries that you people are doing at your Bangalore center. And also, how large is Narayana's Bangalore facility with respect to transplants, robotics, kind of advanced surgeries in India? As a follow-up, where is Kolkata cluster in this journey?

Viren Shetty : Yes. Dr. Rupert will answer this.

Dr. Emmanuel Rupert : So if you really -- we have been mentioning over the last 3, 4 quarters about the complexities of the cases that is being delivered at Bangalore. So if you see that we have gone very high in the robotic cardiac surgery, we do almost 100 cases per month. Many co

mpeting hospitals do probably that in 1

to 2 months. I mean, 1 to 2 in a year, we do it in a month. That's the scale at which we operate.

So are the percutaneous aortic valve reimplantation, which is suggested a couple of hours in the cath lab for an aortic valve replacement, which generally used to happen with the surgical program is happening in the cath lab nowadays, and we do a very large number. We did around 160 procedures of that nature in the Bangalore facility alone. And if you look at the other non-cardiac work, we do the highest number of paediatric bone marrow transplants as well. So the complexities of work that happens here is quite on the very high quaternary range, and that brings in the financial gains which you are talking about.

Kolkata journey - if you look at the kind of infrastructure that we have in Bangalore, it's extremely large facilities which we have to support the scale of the clinical work. Kolkata, we have limited in scale because we can't expand within that small campus of around 3.5 acres in RN Tagore Hospital. So that's where when we come up with the Rajarhat project, we will have many integrated quaternary care facility all in one place. That's where we are. If we have 45 beds in bone marrow transplant in Bangalore, we have 10 such beds in Kolkata. So that's the difference around 30% of the scale of the infrastructure is what we have in Kolkata compared to the Bangalore campus.

Prithviraj: Okay. Go to it. My second question on India business. If you look at the clinics and insurance losses, it has remained flat around INR 66 crore s loss in FY 26. I think the initial impression was that the losses

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will keep coming down as clinics will turn 3 years kind of maturity, etcetera. So how should we look at these losses for the next couple of years? And why is there a delay with respect to decline in losses?

Sandhya J: So it is a factor of the number of new clinics that we are setting up. If we have more clinics coming in, every clinic takes an 18-month period to turn around. So the clinics that we set up about 2 years back, they have broken even and the losses that you're seeing are mainly because of the corporate overheads that are being incurred to sustain the integrated care infrastructure.

Now as we add new clinics, which we intend to do in the coming year, we will have cash burns coming in the new clinics, though the leverage that we will get for the fixed costs will also increase. So therefore, some of the cash burn will moderate because of that. So you have to assume that a similar run rate of cash burn will continue on the clinics going into next year as well because we still have a significant ambition in terms of growth of clinics in our core geographies.

Viren Shetty : What we are doing is a little bit of consolidation and a few of the corporate overheads can be worked in a way that we share resources with the parent and we'll be demerging the clinic business into the core that should help with a lot of that. But we've seen a lot of positive aspects of running these clinics. We are accessing patients domestically that historically would never consider coming to Narayana. We've generated a lot of inpatient referrals of fairly advanced therapies for the main hospital. So they act as an excellent feeder for all the hospitals and build up that long-term relationship for patients who tomorrow will subscribe to our One Health plans insurance plans and then become Narayana Health customers.

Prithviraj: Got it. One final question on India business. If you look at the India business, say, hospitals margins, I think over the last couple of quarters margins have expanded significantly. Do you think there is still scope for further margin expansion in India hospital business?

Viren Shetty : On an absolute basis or a percentage basis?

Prithviraj: Percentage sir.

Viren Shetty : As you know, we don't give guidance on those. But a lot of the things we put in place that have brought us to this journey still have room for growth and there's still a fair amount of work that we've put in place that we'll start to see. But of course you have to balance that out against the new center losses that will start coming in from the mid - to end of this financial year and going forward. So one would temper the other.

Prithviraj: Understood. Fine. I will join back in the queue for international questions.

Moderator: Thank you. Next question is from Shashwat Singh . Shashwat please unmute and go ahead with your question please.

Shashwat Singh : Am I audible?

Moderator: Yes you are.

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Shashwat Singh : Thank you so much for the great care at the affordable prices that you're giving. Sir, as a customer and as a patient, we appreciate it. Sir, regarding the business updates, sir, when do we expect the breakeven to be achieved in the insurance business? And sir, on the U.K. side of things, are we planning to get into insurance there as well?

Viren Shetty : Sure. I'll answer the India insurance business and Anesh will answer for the U.K. business. For India and the insurance business, this is still the early days. We have a very reasonable control overall claims ratio. What's happening right now is that once you combine it with the expenses, it is what's causing the losses, but this is to be expected for any new business that is being built up here.

We've had a lot of positive momentum in growing our group business with small and medium enterprises. And we're doing a lot of things on automation to bring the overall admin cost down. So it definitely will start to reduce over the coming years. But at this point, we can't give a date for when the losses would subside. Anesh, would you be able to address whether we're planning in U.K.

Anesh Shetty : No, plans for that.

Viren Shetty : Yes. So just to clarify, for U.K., we're not planning to get into the insurance business. We're currently only operating insurance in India and the Cayman Islands.

Shashwat Singh : Okay, sir. And sir, regarding our pharmaceutical business -- the pharma business, sir, are we planning to white-label the basic formulas and selling out-of-patent drugs under our own brand name to expand our margins in that business by any chance?

Viren Shetty : No. Like I said, we don't have retail pharmacies. All our pharmacies are fulfilling internal demand. What we do have is a very large purchasing network and a formulary that is able to get very good deals with suppliers. There are certain antibiotics that we're talking with suppliers. These are still in the very early stages, but these wouldn't be branded per se. These are just unbranded generic medicines that are coming directly from the manufacturer to our facility. And these are meant mostly for inpatient activities, not for selling in the outside market.

Shashwat Singh : Okay, sir. And sir, why do we not consider that?

Viren Shetty : We don't have sufficient scale at which it makes sense. If a manufacturer has to put a dedicated line for you and the packaging and so on, that's only in your brand, they will not take returns. And there is a certain amount of volume guarantee you would need to give them, which from an outpatient perspective, does not make sense for us.

Shashwat Singh : Okay, sir. And sir, going forward, what would be the ARPOB growth that we are expecting?

Viren Shetty : Again, without giving forward guidance, we have seen healthy ARPOB growth over the past couple of years based on the efficiencies on reducing length of stay and changing the payer mix. We expect

to be able to continue the performance that we've had so far. But the one thing to note is that year-on-year, the rack rate and the price increases that we take hover in the low to m

id -single digits.

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Moderator: Shashwat, I request you to join back the queue.

Shashwat Singh: Sure, ma'am. Thank you.

Moderator: Thank you. We'll take our next question from Japreet Singh. Please go ahead.

Japreet Singh: So, my question is on the acquisition that you have done. So my question is what led you to believe that you will turn this assets and you will -- this acquisition will be value creation, not value destruction.

And my question is on this acquisition?

Viren Shetty: What is the question, Japreet?

Japreet Singh: I want to ask how will you turn this asset?

Viren Shetty: Sorry, how will we?

Japreet Singh: I mean this you have to increase our efficiencies and technology, increase the payer mix. What led you to believe that you will turn this acquisition into value creation?

Viren Shetty: Okay. Fine. Anesh, if you can just talk about the things.

Anesh Shetty: Yes. Thank you for your question, Japreet. So essentially, we have two prime areas of focus. The first is to increase the proportion of patients coming from private sources that is self-pay or private insurance. Those patients inherently pay more for the same service. The second is an implementation of our entire technology platform that will allow us to meaningfully lower the cost base for all operational processes, similar to what we've done in the Cayman Islands.

Japreet Singh: What is the purpose behind this acquisition motive.

Anesh Shetty: So once we had stabilized our operations in Cayman, we were looking for a second jurisdiction or a second country with a larger base and a larger potential because our software platform, our technology and the systems we had developed to make Cayman a very profitable operation are scalable in other markets as well. And outside India, in certain European, U.K., British mature markets, the acquisition multiples are quite reasonable relative to the opportunity we have to lower the cost base and improve earnings.

Japreet Singh: There are also some other risks like cultural difference like, culture in Narayana will be different to what practice plus culture and your resources will also get diverted, your focus will also change. That's obvious and integration risks are also there. So how do you see these risks?

Anesh Shetty: Yes, definitely, you are right. These are all things we have to be aware of and cautious about with any acquisition in a different geography, there are cultural integration risks, understanding local nuances.

These are things we have to work through.

Moderator: We'll take our next question from Raman. Raman, kindly unmute and go ahead with your question, please.

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Raman: Hello, sir. Can you hear me?

Moderator: Yes, Raman. Please go ahead.

Raman: Yes. So, my question is with respect to your U.K. business as well. So when we did the acquisition call, then the U.K. business reported EBITDA margin was around 7% to 8%. And now at the end of the Q4, for the quarter ending this -- for the previous quarter, the margins have increased to 10%. So can you just give me a ballpark figures on where did the cost cutting come from? And is this margin going forward sustainable?

Anesh Shetty: Sandhya, do you want to take that because I think we're not looking at apple-to-apple comparison in those numbers.

J. Sandhya: Yes. Actually, when we acquired, we only had 2 months of data. So the cost bookings were not complete. And we still are not fully grounded in terms of how our costs are coming through because we still have transition costs, et cetera, coming in the P&L. So in the first quarter 3, which was our first quarter, we did have slightly higher overhead costs coming in. And we also don't have a full handle of all the costs that were getting booked.

Now we have a slightly better handle

and most of the savings for us have come on the overhead cost

side. However, what did a few more quarters to be able to fully baseline and be able to give a consistent commentary on the movement of the costs because we still have many moving parts. We have not fully separated from the seller. IT separation is still underway and costs are still getting grounded for us. We also had some pre-deal costs, most of which we called out separately and explained, but there were

costs that were getting booked because the deal was in progress. So I do think that you should give us a few more quarters, and we'll be able to explain these cost movements with greater clarity.

Anesh Shetty : On a more sustainable steady-state basis, what we can say at a high level is that now that we are running the company as an independent entity, there are some costs and cost increases with regards to certain IT costs, vendor costs, etcetera, compared to when it was part of a broader company.

At the same time, now that it is part of NH, there are cost synergies as well that we bring to the table. So, as Sandhya mentioned, in a few quarters, the overs and unders should neutralize, and we will get a better sense of the trajectory improvement in earnings.

Raman: Understood, sir. Thank you for that. And my second question is with respect to the margins of the Indian business. That has been consistently increasing, like, let's say, for last year same quarter, you reported 21.5% margin, and now it's 25.1%.

So I just want to understand the incremental margin improvement in the Indian business. Is it because of mainly you're bringing in more technology, more automation, more robotics into the picture? Or is it because of the complex increase in revenue share from complex surgery or something else?

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Can you just give me where is this margin expansion coming from? And are we -- is this 25% a stable margin? Or can we expect a further room of margin improvement?

Viren Shetty : You already answered your own question. It is all the things that you mentioned, but Venkatesh can elaborate on what exactly we're doing to be able to do high-end procedures.

R. Venkatesh : Yes. So this is typically -- I mean, the increase in volume, which has happened year-on-year now is typically because we've seen the impact of our transformation initiatives infrastructurally. And also, while we have worked on performing more high-end procedures, alongside whether patients have opted for higher configuration beds, keeping our volumes and occupancy obviously intact. So the hard work which we have put in, in terms of the transformation and high-end procedures are actually kind of reflecting now on the bottom line.

This over and above that, we have also coupled it with technology, as you are raising this question through increased volume of robotic cardiac surgeries, through high-end transplant work across our flagship centers, primarily in Bangalore and a few other high-end procedures, which have also resulted in increased realization, leading to a higher revenue achievement and better margins.

So all these initiatives together have actually helped us in increasing realizations and improving on the bottom line. While another thing which you're saying about the sustainability of this, we are very confident in terms of the margins, and we can see that the gains are sustainable, but we also need to see that currently with a lot of headwinds, including the environment being volatile with several moving parts around us in terms of the cost of crude and the dollar. So obviously, we are very watchful while we are being optimistic in terms of sustaining the gains.

Raman: Understood, sir. Thank you.

Moderator: Thank you. Next question is from Aditya. Aditya, please unmute your phone and go ahead with your question, please.

Aditya: Yes. Thank you. First of all,

congratulations on the strong set of numbers. So my question is regarding the Raipur Hospital specific, given that you are committing significant capex there. So I just want to understand the current state of the hospital, like what are the occupancy levels, what's the specialty mix and payer mix? And in terms of capacity addition, what clinical capabilities or specialties we are building that doesn't currently exist at the facility?

Sandhya J: You're referring to the Raipur hospital, Aditya?

Aditya: Yes, ma'am.

Sandhya J: Okay. We have shared information relating to Eastern Peripheral, and we have shared information relating to our capex and expansion plan for Raipur. We don't share hospital-specific data. But what we can say is Raipur is a full-service hospital and all the capabilities which we have in the rest of the centers in NH are also available in Raipur.

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Viren Shetty : The only thing that we felt we had to expand on the existing facility is the existing building is quite old, one of the oldest hospital structure that we run, O&M model we have with the trust. And while we have refurbished the interiors a lot in the time that we've been there, it doesn't match up to what people expect when they go to other hospitals. So the new investment is adding another tower that can offer a much more differentiated experience for patients coming in.

Aditya: Got it. Thank you. That's it from my side.

Moderator: Thank you. Next question is from Akshay Thakur. Akshay, please unmute your connection and go

ahead with your question, please.

Akshay Thakur : Hello. Hi, team. Can you hear me. Am I audible?

Moderator: Yes, Akshay . Please go ahead.

Akshay Thakur: Yes. Thanks for taking my question and congratulations on good numbers. My question is related to the new capex, which you have announced. So this capex is on the higher end as compared to the old Narayana days. So my question is, so you are expanding in core areas like within the city limits, not like early days.

So we being a low -cost model, low volume player, we try to cut down our expenses on opex and capex front. Now the capex front has increased as compared to the previous model , so the only impact would be on the opex front. So, how do we plan to follow the same model, but at the different locations, which is on the higher expensive capex per bed ?

Viren Shetty : Yes. So, the capex is higher for 2 reasons. One is just naturally construction cost today is 60% higher than it was 5 years ago. These are post -pandemic -related cost increases that is beyond our control. So no matter where you are building, if you're building in the most small town in India or if you're building the largest capital of the country, the construction cost has gone up significantly. So that cost remains the same wherever it goes.

Cost of land acquisition, yes, that is much higher in the big cities, but that is a one -time cost. It will be amortized over the 100 -plus years that this hospital will be there. The most important value proposition we bring in as a low -cost, highly efficient provider of health care is that no matter where we operate, our pricing will be the lowest in the market for the similar levels of quality that you get in any of our facilities.

The things that we are putting in place in all the hospitals we're setting up to reduce the cost involve huge amount of automation, being able to eliminate a lot of the manual work, taking away all printing and paper, making the patient experience a seamless journey. So, less space inside the hospital is wasted for nonclinical activities.

Less people are required inside the hospital for doing nonclinical activities. And so all the space and attention that is there inside the hospital is dedicated entirely to the patients. So, that would increase

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the throughput, that would increase a lot of the flow that exists for the patient journey and thus, in turn, lower the operating cost and that we can pass on to our patients.

These already exist in all our hospitals. That's why we are the only listed group that has not added a single bed from the time of listing and still been able to deliver revenue and margin growth because all our effort has been on improving the way how hospitals function.

Akshay Thakur : That was helpful. So, primarily on the efficiency front and the tech front, that is how we are going to drive the same result with different tech. Is that correct?

Viren Shetty : Thanks, Akshay.

Akshay Thakur : Thank you that's all from me.

Moderator: Thank you. We have a follow -up question from Shaswat Singh. Shaswat please go ahead.

Shaswat Singh : So on the UK business, so, the doctor expenses, is it showed under some other heading because I'm unable to figure out where the doctor expense for the UK business is?

Anesh Shetty : Yes. No, we don't break it down into doctor and non doctor.

Shaswat Singh : Okay, so , because in our Indian business, we are giving out the profitability snapshot. It's not there for the UK business. Is there a reason why we are not giving that out?

Anesh Shetty : It will take us some time to decide what are the relevant metrics to display and discuss quarter -on-quarter. Having said that, the predominant method of engaging with doctors in the private sector, the UK is also very different from India, wherein most of the doctors are visiting consultants and their revenue is booked separately. That's not the case for PPG. That's the case for the other private providers, and we are moving towards some hybrid between that model where doctors submit their own invoices and reimburse themselves differently. We currently have an employed model, but we're working out a hybrid structure.

Shaswat Singh : Okay, sir. And sir, if I'm not mistaken, we took out the GBP 150 million loan for 7 years for the UK business acquisition, right?

Sandhya J: Yes, correct.

Shaswat Singh : So our profit numbers from the UK business is not sufficient. So how are we hedging it for the currency risk?

Sandhya J: The loan has been taken in GBP, and it is being serviced locally in the UK So there is no currency risk on this loan.

Shaswat Singh : But ma'am, profit numbers are not sufficient to pay back the loan, right?

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Sandhya J: At the moment, we believe that we have adequate cash flow to pay back the loan. Currently, you are not able to see the full effect of the PAT because there are a lot of transition -related costs that are getting booked in. It's not the steady -state number that you are able to see. In fact, in our deck, we have clearly called out, the movement between the numbers. On a cash flow basis, if you see cash PAT, we are able to service the loan. The net negative PAT you are seeing is because of the amortization of expenses, which is not a cash expenditure. So we are currently able to service.

Shaswat Singh : Okay. Ma'am could you give us a.

Moderator: Shaswat, I request you to join back the queue, please. Next question is from Rajit Aggarwal. Please unmute your connection and go ahead with your question please.

Rajit Aggarwal : Hi good afternoon. This question is related to UK P&L. Now, if I look at the Slide 21, it shows FY26 without UK and then a column FY 26. So I'm assuming the column FY 26 is with U K. Is that right?

Sandhya J: Correct.

Anesh Shetty : Yes, that's correct. I'm laughing because Sandhya and I had this debate before if it made sense to say without and with U K.

Rajit Aggarwal : And if I were to , so if I subtract the two columns, I'll get the UK numbers for Q3 and Q4?

Sandhya J: Yes. Q4, you will . For full year, you will get FY 26, which we have given. Th

is has five months of U K.

Rajit Aggarwal : Correct. And if I refer to the similar slide of your last call, then it had two months P&L.

Sandhya J: Yes.

Rajit Aggarwal : Right. And if I subtract those two , then I get this quarter's numbers for U K.

Anesh Shetty : Sandhya adjusted for the 6 or 9 days in November.

Rajit Aggarwal : Right. I mean that's a little bit of adjustment. But some of these numbers somehow don't make sense in this , so I'll give you an example. If you look at your doctors' expenses , the row doctors expenses here, it's INR 67 crores increase. So UK had INR 67 crores of doctors' expenses as per this Slide number 21 of current PP T.

Sandhya J: Yes. Actually, Rajit, some re -class es has happened. We re -classed some expenses because see, when we reported out in Q3, we didn't have a full handle of the way the P&L was being reported in the U K. Now we are slowly aligning. So for example, your specific example of doctor expenses, there is a re -class in the P&L from doctor expenses line to the manpower line because of which you are seeing the number to be not like -to-like.

Rajit Aggarwal : Right, which is okay. So I get that. Now I mean, if I were to include doctors' expenses and employee, excluding doctors' expenses, will that be a correct way to look at your total expenses for everyone?

Employees, I mean, including doctors and others, right?

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Sandhya J: Yes.

Rajit Aggarwal : So now if I compare that, those two numbers itself with what you had given in the last slide as a percentage of sales, that number seems to have gone down by 200 basis points.

Sandhya J: Correct. That is correct.

Rajit Aggarwal : So how has that been achieved?

J. Sandhya : So like I was explaining to the previous caller, because we took a part quarter handover, so the costs on the baseline of Q3 are not representative of the underlying cost structures, which is why we requested a couple of more quarters for us to be able to report these numbers more accurately and be able to explain the movements.

Rajit Aggarwal : So basically, we should just , we should not delve too much into these numbers.

Viren Shetty : No, you're sure. It's just that Q3 is not representative of a whole quarter and not representative of all the expenses. It's only when you have 3, 4 quarters of numbers to look at, can you see movement.

Rajit Aggarwal : But there's no movement in the total headcount, right?

Sandhya J: No, there is no movement in the total headcount, but the booking in Q3 was slightly on the higher side in percentage terms vis -a-vis revenue. The absolute cost has not changed. It is just that the booking , percentage terms, it is different because like I said, it was not even cut at a month. It was cut in the middle of a month. So we really couldn't exactly allocate the costs.

Rajit Aggarwal : Okay. So I guess, I mean, we should just wait it out to understand the absolute and percentage margins as well because it seems a lot of pieces are moving between heads also. Fair enough.

Sandhya J: Yes.

Rajit Aggarwal : So I guess , and in terms of net profit or net loss?

Moderator: Rajit, I request you to join back the queue, please, as we have participants waiting for their turn. Thank you. Next follow -up question is from Prithvi Raj. Please go ahead.

Prithvi Raj: Yes. So I have a couple of questions on Cayman Islands. One, if you look at the insurance business, it's surprising to see the extent of losses. So USD 5 million seems to be quite high. I think over the last couple of quarters, the initial guidance seems to be that maybe by Q1 FY 27, we might even reach breakeven. So trying to understand what exactly happened with the insurance business? And how

should we look at losses going forward?

Anesh Shetty : Yes hi Prithvi . So I think the first comment is in terms of the losses relative to the scale of the business.

So when we started in January 2025, initially selling policies in the open market, we didn't expect to reach by this time around annualized USD 60 million premium revenue rate. So the ramp -up in the

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insurance business has happened far quicker than we anticipated. We always knew that it would take a couple of quarters for the initial book to stabilize as well.

I think we've made great progress on growing the number of clients we have, the profile of the clients, most of the premium clients and the higher -paying clients are with us in the market.

Having said that, the quarterly losses are not where we want them to be, and we don't think this is going to be an ongoing thing aside from maybe we may have, I think, another quarter or 2 of losses in this range, but we have price increases kicking in, in June for about 30% to 35% of the accounts. The end of that period also gives us the opportunity to purge certain accounts we no longer want, which have been priced unfavorably. So, we should start seeing these come down significantly over the next 3 quarters or so.

Prithvi Raj: Is it possible for you to give the claims ratio here on a higher trend ?

Anesh Shetty : I would say the loss ratio, I mean, we don't disclose it specifically, but if you look at the CIMA statistics, you can find out. So it's approximately about 110% to 112% is the loss ratio, which is essentially claims paid divided by premiums collected net of reinsurance. So for a year 1 insurance book, that's not very bad. I mean it's not great, but it's not horrible. So with the price increases midyear as well as exiting some accounts, we hope that, that does settle in.

Prithvi Raj: Okay. One last question from my side on Cayman again. If you look at the hospital side, the scale -up has been really phenomenal from less than USD 30 million revenue to almost USD 50 million per quarter kind of run rate. Can we assume that the scale -up is largely done and from here on, the growth will slow down to mid -single digits for Cayman hospitals?

Or do you think still there is scope for a few more quarters of double -digit growth before it tapers down? And also, a follow -up, how much of this incremental growth came from the other Caribbean islands?

Anesh Shetty : Sure. So, before I answer your question, Prithvi, I do want to call out here for all the participants that we request people to analyze the Cayman business as one , and not disaggregate insurance and hospital. The whole idea behind the integrated care model is that the insurance helps drive steerage into the hospital and they work as a whole.

So, if you are going to see earnings, let's say, losses in, say, the insurance business in isolation, but contrast with very good growth in earnings and top line in the hospital business, it does give a bit of a confusing picture. So, it's always helpful to look at it as a consolidated whole because that's what we're trying to achieve.

To your specific question, yes, definitely, I think the significant growth in the hospital top line has been driven by two factors ; one, the new campus, which is a little old story now, but also because of One Health and all the benefits and synergies that Integrated Care play has. It will be in that market unsustainable to continue to see quarter -on-quarter double -digit growth in top line. That will be -- that's not physically possible.

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Also, given the claims experience in the insurance business, we are slowing down the new client addition. We do have to balance it out and observe for a few quarters. So, you're not going to be seeing the same pace of growth in the revenue. Of course, the hospital margin has been preserved pre Camana Bay Hospital, post Camana Bay and even now through Integrated Care. So, that is stable. But yes, you won

't see the same rate of growth anymore.

To your last question around what percentage is international? I think we don't get into those specifics, but international, especially some of the core markets where we've been investing for quite some time are starting to bear fruit. And we think once Cayman significantly slows down and the opportunity is exhausted, the growth will be driven by international, but we're not there yet, but someday soon.

Prithvi Raj: Thank you. That's all from my side .

Moderator: We have a follow -up question from Aditya. Aditya, please unmute your connection and go ahead with your question, please.

Aditya: Yes, just thank you so much for taking my question again . Just one small question, a bit digression

from NH, broader on the health care industry. There are a lot of new health care and open start-ups like Super health Even coming up with standardized pricing, smaller hospitals, lower capex, prepaid, and payvider model, which we are also doing. So, like Viren sir, just wanted to get your view on these models and are they sustainable? And can they scale up?

Viren Shetty : Are they sustainable . I mean that remains -- we'll see. The thing is these are all with a lot of start -ups. They bring very interesting viewpoints, and they are the closest to what we can get as almost a cost - free R&D. And so, it is good that the market is moving towards a direction where payers and providers work together and provide a more unified experience for customers because that's not happened in the Indian industry so far. So, in as much as these things become more popular, it increases the relative attractiveness of what we are trying to do.

Aditya: Got it. Thank you so much .

Moderator: Next follow -up question is from Akshay Thakur. Akshay, please unmute and go ahead with your question please.

Akshay Thakur : Hello, thanks for taking my question My question is on your hospital information system. I believe you also get some revenues from this part of your business, but primarily, it's for your own internal use. Can you throw some light on how important was this in bringing in the efficiency for your Cayman as well as your India business?

Viren Shetty : So, the business that we acquired in the UK runs several interconnected systems, none of which talk to each other very efficiently. So, there is a human layer that has to not just translate and type, but sometimes even print out and retype information between one software stack and another. You will be shocked at the number of hospitals, not just in the UK but across Europe and the US that have these .

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In India, we're very lucky to have access to a large group of engineers who are able to take this problem first-hand and be able to tackle it so that we cannot have to worry about silly things about how the hospital runs and how data should be entered. So, the problem that we are solving is data being in a single source of truth is fixed.

But what we do with that data, be able to go through the medical records and find out if patients are suffering from diseases that they don't know they are suffering from or working with the doctors to be able to do research on all the patients who are available. So, these are huge amounts of efficiencies that can be unlocked because of our AT HMA system. And a lot of other hospital groups have seen the benefit of this and want a similar system for themselves.

Akshay Thakur : Can you also share your plans for the clinics? How much do you want to expand? What capex to spend on those and strategy behind them?

Viren Shetty : Yes. The clinics, we are 11 clinics in Bangalore. We will be expanding in Bangalore. And this year, meaning FY27, we will also be expanding to Calcutta . We have a plan to double the number of clinics that we currently have, but the speed at which we do it would be balanced on the

other side against the

acquisition of those properties, negotiating the rent, doing the fit -outs and also maintaining a steady amount of cash losses in the business so that we don't go overboard. But the clinics are a very important channel for us to be able to be closer to our patients.

Akshay Thakur : Thank you so much.

Moderator: We have a follow -up question from Rajit Aggarwal. Rajit Please go ahead.

Rajit Aggarwal : Yes. So, coming back to the numbers. Now there seems to be a difference of about INR10 crores to INR12 crores in the net loss of UK entity between Slides 21 and 19. So, GBP, it says 1.9 million GBP net loss. And in rupees, if you look at, it's around INR 34 crores.

Sandhya J: Yes. See, the conversion may be the point because what happens is that when you report out, you report a conversion rate for the quarter.

Rajit Aggarwal : So, you would be doing conversion at the RBI reference rate, right?

Sandhya J: No. Actually, what happens is that we -- of course, we may be doing at the RBI reference rate, but we catch up for the year, the conversion rate. So, in quarter 4, you'll also have the catch -up effect of the Q3 conversion.

Rajit Aggarwal : Okay. Maybe I'll just come back to you offline again. My understanding was Q3 would be at RBI reference rate and Q4 at RBI reference rate and whatever the difference would be in.

Sandhya J: Yes, it has always caught up YTD actually...

Rajit Aggarwal : All right. I mean that's not such a big thing. And there is a INR10 crore s other income in UK entity, around INR 8 crores. Can you explain that other income, please?

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Sandhya J: The other income essentially comes from interest on cash that we are holding in the bank, some scrap

sale that could have happened. For the size of the UK business, I think it's like a rounding off number. Mostly it will have come from bank interest because we are having cash in the entity, and we'll be placing them in some sort of an instrument and earning interest on that.

Rajit Aggarwal : All right. If I may ask a very quick question on numbers again?

Anesh Shetty : Yes, please go ahead.

Rajit Aggarwal : Yes. On the acquisition cost in the last quarter, about INR 40 crores was booked in UK but that seems to have been now reclassified into ex -UK. Is that correct?

Sandhya J: So, yes, so there are a lot of reclass items, Rajit. I think what we will do is we will set up time with you separately. We can explain the numbers to you so that you are able to see them clearly . You can reach out to us. You have our e -mail ID. Our IR team can actually explain to you all these items on a call because these are like small items, and I think we are holding everybody else on this.

Rajit Aggarwal : Right, ma'am. Understood. I'll come back to you. Thank you .

Moderator: We have a follow -up question from Sajal Kapoor. Sajal, please go ahead .

Sajal Kapoor: Yes, thank you for the opportunity again. Sandhya or anyone, I mean, question is on the UK operations and the overall debt that we have. I mean, what is the current thinking around the free cash? So, you will get operating cash flow. I think it's a wrong idea to compare PAT and think the PAT is paying the loan.

Ultimately, it will be the operating cash -- stroke the free cash, right? Because there will be some cash requirement in the maintenance and making the changes that we want to make to get the operational efficiencies like so, Viren was explaining the paper work and the manual effort. And he's quite right. I mean this is a problem not just specific to U.K., but wider Europe and U.S. as well.

So, the question is, over the next 5 to 7 years, what is the thought process in terms of reducing the debt or swapping it over and showing the material improvement in the operations and backing it up with the operating cash flow. What is the broad thought process as on

today? Of course, that may change tomorrow.

Sandhya J: Yes, Sajal. So, it is a leveraged buyout. So, the entire acquisition debt is on the books of the target. And therefore, the free cash flow that is being generated by the target, a significant portion of it will be used to pay the debt in the books over the next 7 years.

Sajal Kapoor : Okay , that's helpful Thank you so much.

Moderator: Thank you. We'll take one last question from Sashwat Singh.

Moderator: Sashwat, can you please unmute again?

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Sashwat Singh : Yes. So , ma'am, NHS pays once every month, that is on 26th, right?

Anesh Shetty : Are you talking about the NHS in the U.K.?

Sashwat Singh : Yes.

Anesh Shetty : No, no. It's -- that may be the payroll date, but we as a contracted vendor, we don't follow that cycle.

Sashwat Singh : And in how many days do they pay us? Is there a cash flow understanding that I can get, please?

Anesh Shetty : Sure. There are 2 kinds of payments. The bulk of payments will come advance quarterly for the anticipated work ahead of that. There's a small percentage of payments that they do after about 30 to 45 days.

Sashwat Singh : Okay. So that help's sir. That is all. Thank you.

Moderator: Thank you. We will now take the text questions. Over to you.

Viren Shetty : Nishant, will read them out.

Nishant Singh : First question is, please share what you think the SG&A would be as a percentage in the FY 2027, given the U.K. Group acquisition?

Sandhya J: Yes. So , in health care, it's not SG&A, but it is more overheads as we would call it. So, U.K. comes in at an overhead structure, which is lower than the group overhead structure. So, we believe that whatever numbers you're seeing right now, so without U.K., we we re operating overall, all overheads put together at about 28%. And with U.K., we'll see a 23% number.

Nishant Singh : So, the next question is -- while the PPG acquisition provides a great secondary currency hedge and scale, the baseline EBITDA margin has dropped by over 500 basis points this quarter. What is the precise road map, synergy extraction plan and timeline to bring the U.K. assets OPM closer to our core India trading standard?

Sandhya J: Yes. Just one -- just quickly on the 500 bps, almost 200 bps dilution has come because of the costs relating to the acquisition itself. Those are one -timers in nature, and those will not repeat. So, the effective dilution is about 300 bps. So normalized margin around 22%.

And Anesh, I think, spoke about the various plans that we have to improve the payer mix and improve the mix. U.K. will not come to the levels of India or Cayman. Each market has its own profit profile.

And U.K., we will try to catch up more closer to where the market is than where the group is.

Nishant Singh : Next question is ARPOB. Question is about ARPOB in Q4 FY '26 and full year FY '26. What is the occupancy level in quarter 4 and the full year?

Sandhya J: See, we don't report out occupancy, but we typically operate between 60% to 65% in terms of occupancy. We do not believe that either ARPOB or occupancy is a correct measure of performance.
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The right measure is ARPIP, which we report and also throughput, which is evident from the way we are able to do incremental volumes through the same facility.

Nishant Singh : And the other part of the question is asking about EBITDA margins for Cayman business?

Sandhya J: We don't specifically report out EBITDA margin for Cayman business, but I think all the numbers are available. India EBITDA -- overall group EBITDA is available. India is available, U.K. is available. I think it is possible for you to work that number if it's interesting for you to understand.

Nishant Singh : The next question is the other income stand-alone for Q4 is INR127 crores. What

is it comprised of?

And how does that translate to INR25 crores at the consol level?

Sandhya J: That INR127 crores has INR 94 crores of dividend that has been received from Cayman to India. So, because of that, you are seeing that number not coming in consol -- consol that intercompany gets eliminated. The rest of it is the real other income for the group.

Nishant Singh : On Slide number 15 of the IR deck, India ops for FY '26, in spite of a 10.2% Y-o-Y growth, the other expenses in percent stays at 18% of revenue, why is operating leverage not showing up here?

Sandhya J: See, actually, with the 10% revenue growth, our overheads have gone up by about 8%. There are a few factors to this. There is a certain part of the overheads, which is purely variable in nature, and therefore, that moves in tandem with the revenue. Now in addition, it is a very high inflationary situation that we are working with. Power costs have gone up across the board and minimum wages have gone up across the board.

So, there are a lot of underlying cost inflation, which we are absorbing in our overheads with the leverage that is being created with the revenue growth. In addition, there are some one-time costs in there because we are going through a certain merger process. So, there are legal stamp duty and other costs in them, which will normalize in the next year. But largely, we are seeing a very high inflationary situation, which is taking away the operating leverage we otherwise would have been able to generate in the current environment.

Nishant Singh : The next question is what will be the capex for next 2 years for Narayana, bifurcated and maintenance plus growth? Is there any capex requirement for international assets for Cayman and U.K. PPG? If yes, the quantum, please?

Sandhya J: So, for the India capex, I think it's part of the IR deck itself. We have split out the capex and shown the details. There is no additional capex planned in Cayman. It is too early for us to talk about U.K.

So, when we have greater information about our capex plan for U.K., we will share.

Nishant Singh : FY '26 reasons for hospital discharges across regions and IP volume footfall drop, but ARPP is still showing Y-o-Y growth.

Sandhya J: Venkatesh, do you want to take that question?

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R. Venkatesh : Yes, yes. So, we have been kind of prioritizing on our case mix and also working on a few things to focus on more advanced procedures, which we already discussed a while back. So, what has happened is this has led to a good increase in realizations and revenue. And as a result of which, the ARPP is still growing. But these are done keeping the volumes intact.

Obviously, the volumes have stagnated, but because of the optimization of case mix and doing advanced procedure, we've been able to increase on the realizations and the revenue. Also, we had to play a fine balance between serving customers across payers and also trying to manage the receivables with certain payers. But going forward, we will also, through our continued investments in clinics, digital marketing, insurances, we will start seeing gains in volumes too in the coming quarters as we keep building on our realizations and revenues on and on in subsequent quarters.

Sandhya J: Thank you, Venkatesh.

Nishant Singh : The other part of the question was -- private pay was 7% of PPG revenue at acquisition. Where is it now? And what is the realistic 2-year target? Maybe Anesh can take this.

Anesh Shetty : Yes, sure. It's still early days, but we have seen an increase in absolute terms of private revenue. We are making a lot of progress with our relationship with the private insurers as well as visiting consultants. We hope the benefits will play in over the next 3 to 6 quarters or so. Realistic 2-year target, still early days to see. We may have a figure to share in a couple

of quarters.

Nishant Singh : Yes. So, the next is on clinics. Could you provide some color around how insurance and clinic business

and the concept of providing integrated health care has helped us to drive growth for our business as a whole? Like if you could quantify or provide some metrics that have significantly improved.

Viren Shetty : Ever since we took a call on improving our domestic focus and deemphasizing on growing international revenues, the clinic business has been the sole driver of the increased referrals to our hospitals from the cities in which we operate. So the entire replacement of international business with domestic business would have been a lot harder if we didn't have the clinics to be a place where patients will be the first point of contact to see an NH doctor. And should they require any procedure, we refer to a hospital for treatment.

We do expect the growth to come in line. In fact, going forward, the vast amount of patient preference for being treated will move away from the hospital towards the clinic. And so a large part of our expansion in the cities where we operate will be underpinned by first running clinics, gauging what the doctor engagement model will be there, getting good sense of the patients and their clinical requirements, and then building a hospital to service that.

The clinic will be a very important part of our business going forward to accommodate changing patient preferences. But going forward, it will be harder for us to say what is attributable to clinic alone and what is not simply because a huge amount of the manpower and expenses and doctor talent will be shared between the two.

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So, doctors will be splitting time between the hospital -- between the multiple hospitals in the city and the multiple clinics that we run. So the attributability will become harder and harder as we go forward. So, from this year onwards, FY27, the clinic business will be rolled in with the overall hospital business because that is its best reflection of the contribution to the performance of those hospitals.

Nishant Singh : Next question is Kolkata being one of the core geography with 26% revenue share that has grown only at 5%, 6%. Is it because of the lower international patient? If yes, how much is the international patient share in Kolkata geography? Do we expect the growth to kind of come in line with the Bangalore growth?

Sandhya J: Venkatesh?

R. Venkatesh : See, when it comes to international marketing, we are not doing any active marketing around international patients and neither do we have any type of price differentiation. But we welcome any patient in the system coming into the system. It's basically because of the uncertainties around international marketing, which actually made us focus more on the domestic patients and convert focus more on domestic and get the numbers replaced for sick patients.

This journey has been a very fruitful and successful journey where we have more or less compensated for these numbers without any dent in the revenue and the EBITDA levels. Then again, a little bit of prioritization on case mix was also a journey which was well addressed without again any dent in the volumes and revenue. So we have set up a very strong base and consistent -- strong and consistent base as we speak.

Now with the investment in the digital marketing with, as Viren had mentioned earlier also, with the integrated care and the clinics coming in with the insurance and also trying to strengthen the clinical team across the region in quaternary care and tertiary care, we will start seeing the volumes grow up in the future. So with this base and incremental volumes, I'm sure we will be able to cover up this growth deficit, which we have seen over the last few quarters successfully in the coming 2 to 3 quarters here in Eastern region.

Nishant Singh

h : The next question is on plans to expand our footprint and patient volumes for the North and Western clusters.

Viren Shetty : No plans as of now. We have the existing cluster in the North and West. We will start expanding in SRCC Hospital in Mumbai and adding the adult programs in FY27. So there is sufficient headroom within the existing setup to be able to add patients. Infrastructure for either geography is not the priority for now, we're focusing on South and East.

Nishant Singh : Next question is, in which areas of specialty profile do you see AI/ML help you going forward to replicate what we have done with ATHMA and Medha ?

Viren Shetty : Our radiology department is completely powered by AI. A lot of the radiologists use software that implements image guidance AI for helping them make the diagnosis. So this is something that's been very standard in the medical field for a long time. There are a lot of efforts that our Medha team is

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putting in to use AI to improve a lot of the patient services , to detect anomalies in clinical data, provide risk score for our doctors to help choose which patients in the ICU are having trouble or not. So this

will just become part and parcel of the overall way in which a hospital functions. AI is not to be considered separate from all the transformation efforts. It is an integral part of it.

Nishant Singh : The next question is FY26 greenfield capex is only INR 109 crores against INR 424 crores planned, 75% to 74% miss . What specifically caused the slippage? And does the FY28 commissioning timeline for Rajarhat, HSR, Raipur, Bangalore still hold?

Viren Shetty : Lot of the project has started. The miss was due to a lot of the election -related issues in that it was impossible to get construction workers to come for a large amount of time. A few delays in commissioning the project for not getting permissions and so on. The FY '28 goal still remains the case, and a lot of this work can pick up now that things have normalized.

Nishant Singh: The other question is the -- I think it means that when you say EBITDA maintained, does it mean you'll maintain margin of 20%? This is at the consol level?

Sandhya J: Yes, we are not giving any guidance, but what we have explained is that the 20% margin, which has come at a consol level has one -timer impact on it because of the acquisition costs relating to U.K., which normalizes to slightly over 22%. So that is the base at which we are starting, and we have to work with improving on that.

Nishant Singh : The next is based on the disclosures in Slide 22 of the deck, the total capex for projects proposed INR 3,000 crores. Same slide also mentions greenfield organic capex is INR 460 crores. All projects are expected to be commissioned by FY '28, '29. Does it mean FY '28, '29, we'll have INR 1,000 crores each?

Viren Shetty : That's correct.

Nishant Singh : The next question is, what is the long -term target of top line growth for 2, 3 years?

Viren Shetty : Again, we're not giving guidance, but we're trying to maintain the pace of growth in our business. There will be a jump when new hospitals get added.

Viren Shetty : The next question, Northern cluster grew 7%, 5% in FY '26. With the NCR competitive intensity, is there a rethink on the North strategy or do you expect organic recovery in how soon?

Viren Shetty : So yes, it is no surprise to us and to the investors that North has been extremely challenging region for us. We are one of the latest entrants to latest entrants to this market, and it is extremely competitive. We have carved a fair niche for ourselves, both as Dharamshila as a very leading cancer hospital doing a large amount of bone marrow transplants and robotic surgeries and Gurgaon being a very domestically community -focused hospital doing high -end work, but it is not sufficient because the other hospital

s have much more aggressive ramp -up strategies.

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As of now, we will continue the way we are, keep our cost advantage there, add more patients and be able to offer good services for the customers who come to us.

Viren Shetty: So the question is on what is the status of Mumbai on EBITDA profitability? Sandhya.

Sandhya J: So, Mumbai has broken even in Quarter 4. We are working towards the adult program starting in Mumbai. We are in stages of discussion with the trust for approval, and it's in advanced stages of conversation. We do believe that we will be able to turn around the profitability profile of the Mumbai hospital once the adult program comes in.

As far as the next question, which is looking at the FY '26 consolidated PAT, it is been diluted by U.K. So return on equity and return on capital are being diluted as well. So you want to understand the 3 -year IRR or acquisition case.

So when we did our acquisition call, we had given a very detailed rationale and the business case for the acquisition, the valuation metrics and what levers we think are available for us to be able to create value with this acquisition. So, I would request you to refer to the transcript of that call in which a very detailed explanation is available.

At a very broad level, we are going through an expansion phase right now. We have put in USD 100 million capex and came in for expansion. We're putting in INR 3,000 crores in India. So overall, we are going through a phase where we will be diluting ROCE for growth. And eventually, this will come back because the mode of acquisition which we have chosen , mode of investment which we have chosen are longer -term ROCE accretive.

Most of them are greenfield in nature and most of them bet on the underlying business -- the cost of the business coming in with operating leverage. So we do believe that eventually the ROCE and ROE will recover on all 3 businesses put together, but we have to go through that phase to fuel growth and fuel expansion.

Viren Shetty : Next question -- impact of oncology revenue due to -- I assume by OPD, he means diagnosis

Viren Shetty : Outpatient department coming down 5% to 6%. Okay. So, this is a pure impact. Were we impacted by the same? No, we did not see any impact on oncology outpatient volumes.

Sandhya J: Why is the EBITDA margin coming down for the quarter has been answered in detail earlier also. How do you plan to expand in Kolkata has also been explained as part of our IR deck. We have given our Kolkata expansion plans.

Viren Shetty : Yes. So I think we've answered all the questions on the chat. If the Chorus team can conclude.
Moderator: Thank you, everyone, for joining the call and for your continued interest in Narayana Health. We appreciate your participation and support. Should you have any further queries, please feel free to reach out to the Investor Relations team. Thank you, and have a good day .